

WHISTLEBLOWER RETALIATION EVIDENCE

Largo Laboratory - Kaiser Permanente

EXECUTIVE SUMMARY

This document presents chronological evidence of retaliation against Ugochi L. Ndubuisi for protected whistleblowing activity. The timeline shows:

1. July 14, 2025: Director orders investigation
2. September 29, 2025: Analysis provided and acknowledged
3. October 3, 2025: Same analysis shared with staff (protected activity)
4. October 3, 2025: Four censorship demands in 8 hours
5. October 8, 2025: Final Written Warning (first discipline in 9 months)

Maryland OSHA Deadline: November 7, 2025

CRITICAL TIMELINE

Date/Time	Event	Significance
July 14, 2025	Investigation Ordered	Director orders investigation into second shift
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Oct 3, 4:30pm	Censorship #4	Fourth deletion demand
Oct 8, 2025	Final Warning	First discipline ever - clear retaliation

PATTERN OF RETALIATION - KEY EVIDENCE

- 1. Protected Activity:** Reported workplace safety concerns affecting patient care (41.23% failure rate)
- 2. Temporal Proximity:** Only 5 days between whistleblowing and Final Written Warning
- 3. Disparate Treatment:** Same analysis praised by director on Sept 29, punished when shared Oct 3
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- 6. Public Interest:** Patient safety at Level 3 Trauma facility serving community

Filing Information:

- Maryland OSHA Deadline: November 7, 2025
- Protected under Maryland Occupational Safety and Health Act
- Document Generated: October 14, 2025

The screenshot shows a Microsoft Teams chat window on a Mac. The browser address bar at the top indicates the URL is `teams.microsoft.com/v2/`. The chat is with a contact named Nathaniel Burmeister. The message history shows a discussion about team performance and an investigation. The interface includes a sidebar with navigation options like Activity, Chat, Calendar, and Calls, and a list of teams and channels at the bottom.

Chat History:

- Monday, July 14**
- Nathaniel Burmeister 7/14 11:03 AM: Does your injury prevent you from observing the second shift team and taking notes about their productivity? I would expect that you could watch the activity in the lab from a chair/desk. From the data and from reports, the second shift team is still struggling to keep up with productivity. Whenever we have a "Guest" tech (float or from another site) we have MUCH better performance. Leads me to believe that the regular team is underperforming.
- 7/14 11:07 AM: The regular team is underperforming. They heavily rely on top performers or night shift to do all the work. I can observe their activities from one location I just cant go back and forth between the CDU, AUC or MOB. But I can tell you that they are underperforming and not being attentive. I wanted see if I can start a compliance investigation
- 7/14 11:14 AM: I see this as an overall cultural issue that needs to be investigated
- Nathaniel Burmeister 7/14 11:16 AM: Ok, I'll give you some time to investigate. I'd like to know your plan on how you will address. Lets chat about it on Friday. I'm going to set up a calendar invite.
- 7/14 11:18 AM: Ok!
- I have some concerns. I do think Tracy and Sam needs to be on separate shifts. Tracy and Sam are my main concerns. Albert underperforms because he doesnt want to be the only one working hard. Albert continuously discusses how efficient he works with Emily which led to my initial conclusion. That Sam and Tracy do not pull as much weight as the other

Teams and channels:

- Largo Lab Team Chat!
- John and Nathaniel
- John F Ekpe
- Largo Leads
- Dream Team
- Moonlight Microscope
- Nathaniel Burmeister
- Largo Client Services
- 2nd Reviewers
- Inventory Team
- Workflows
- Shanthi A Hayes
- Janan and Tear

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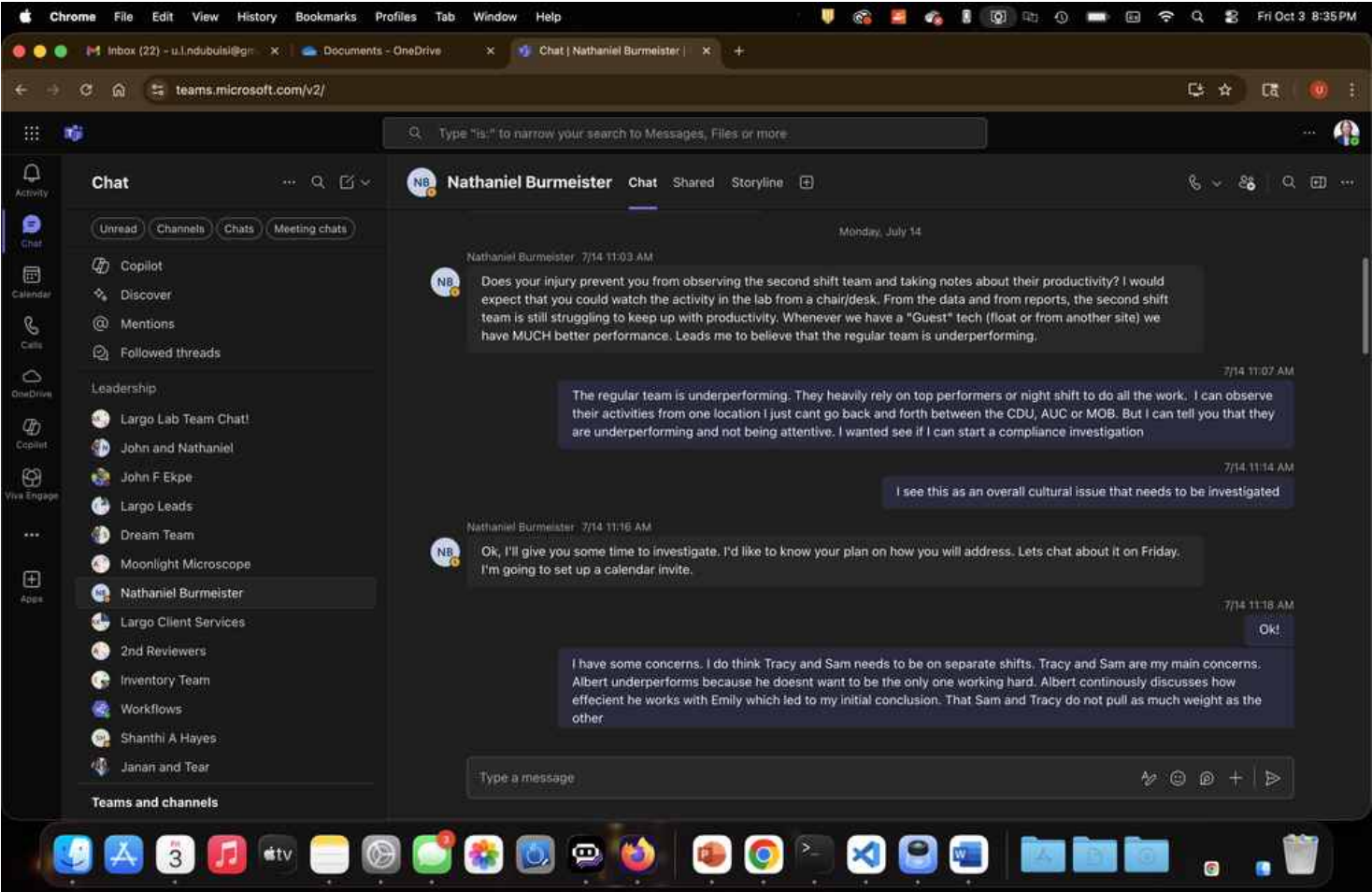
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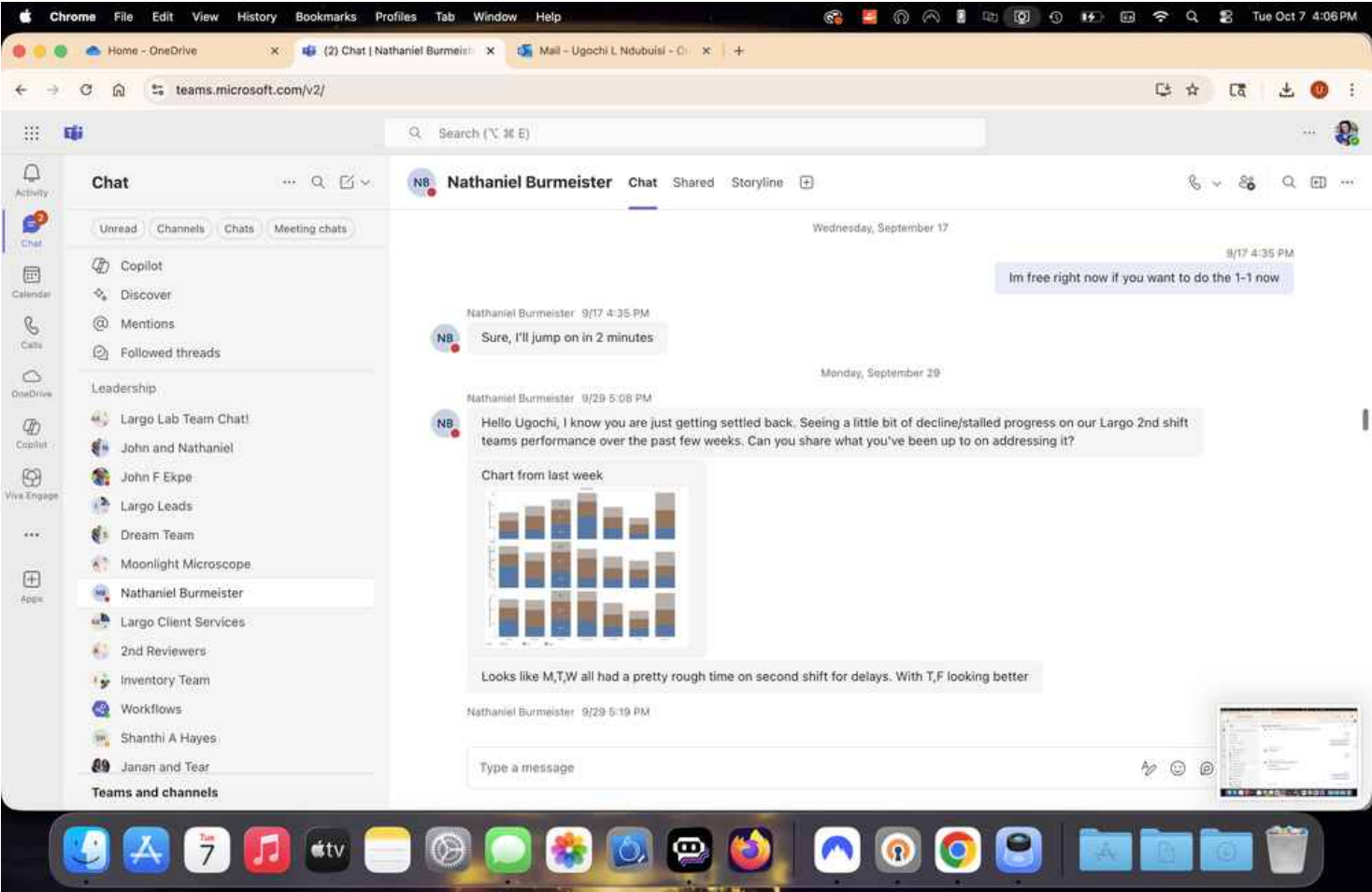
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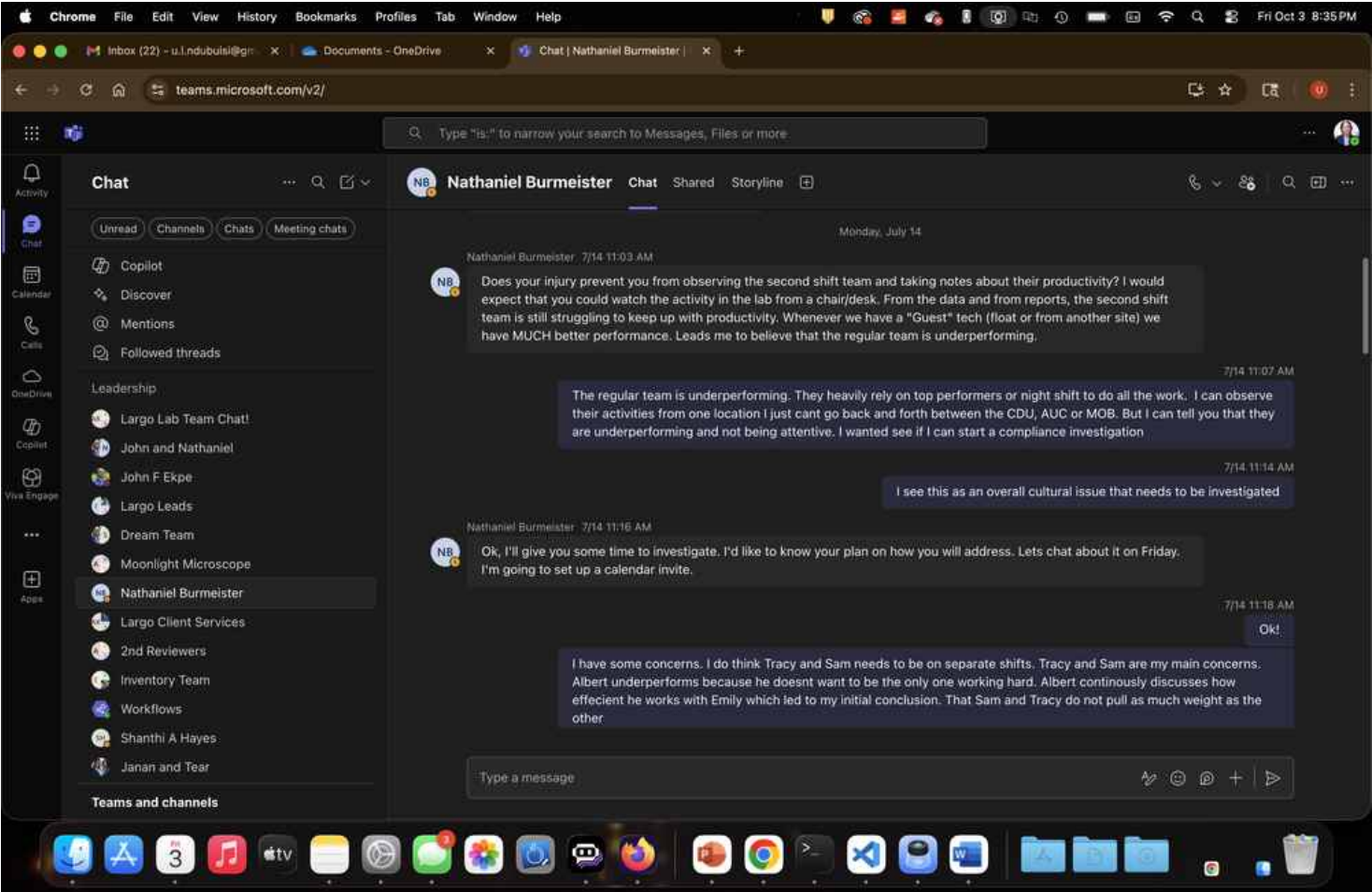
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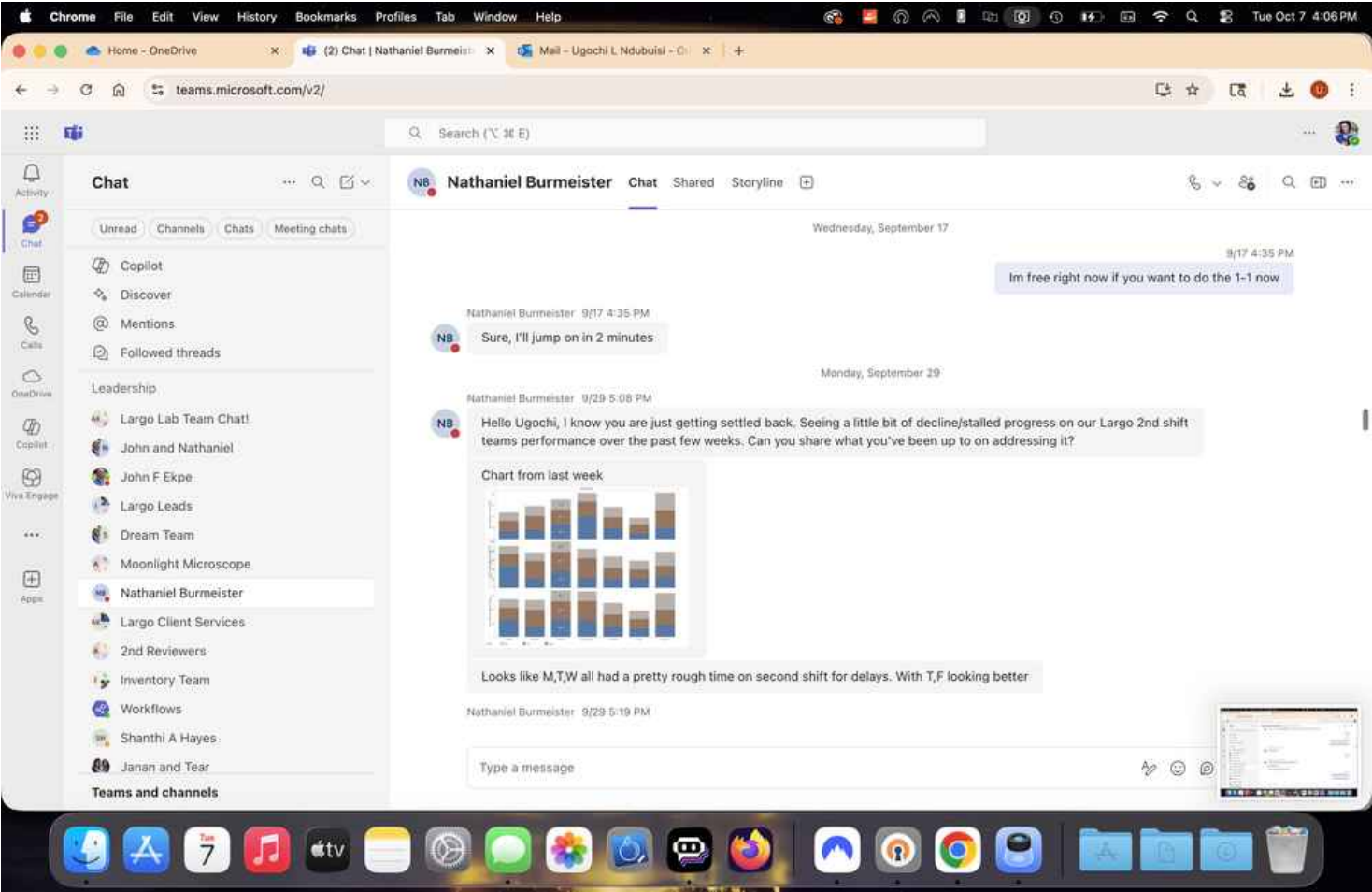
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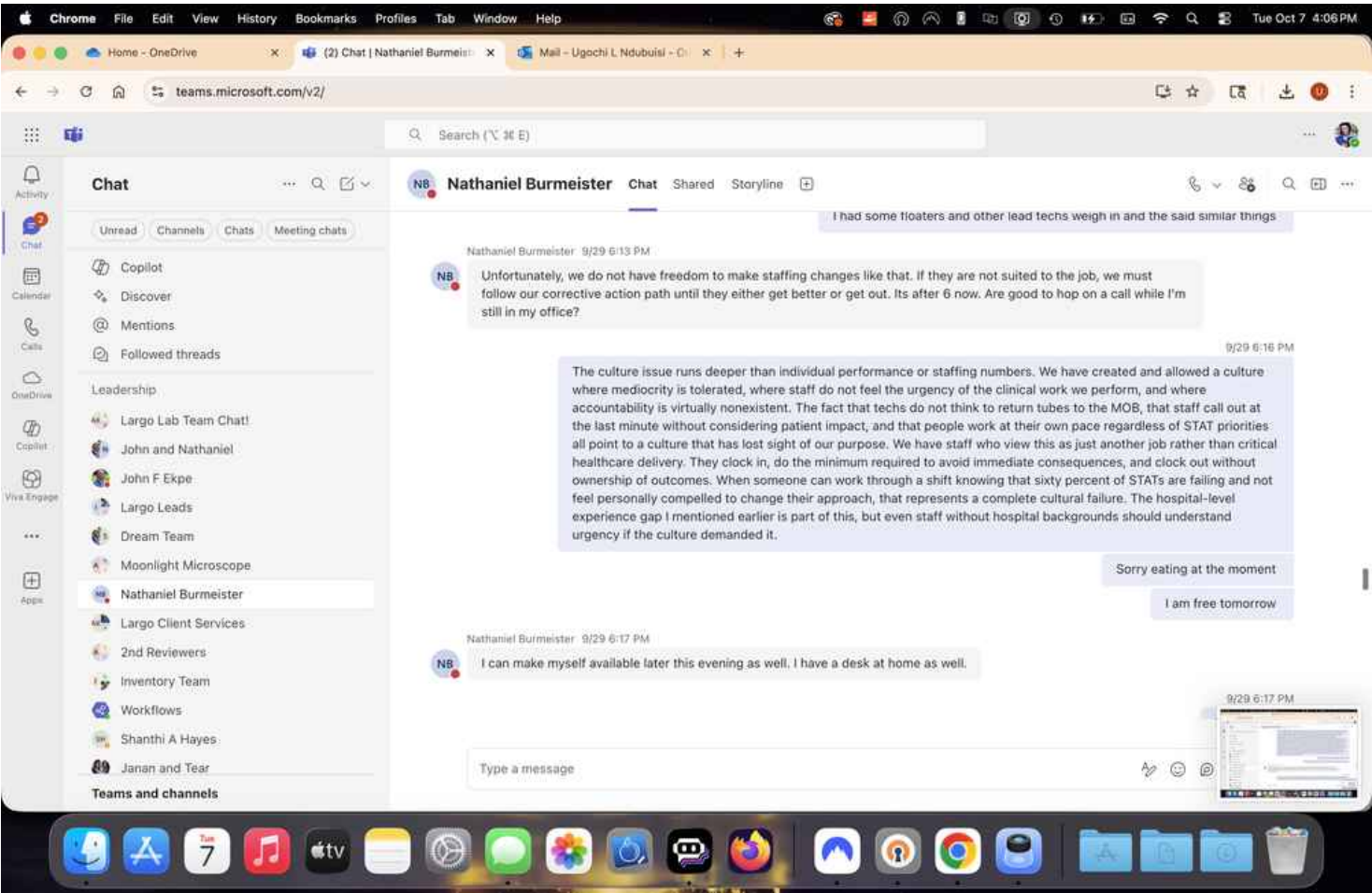
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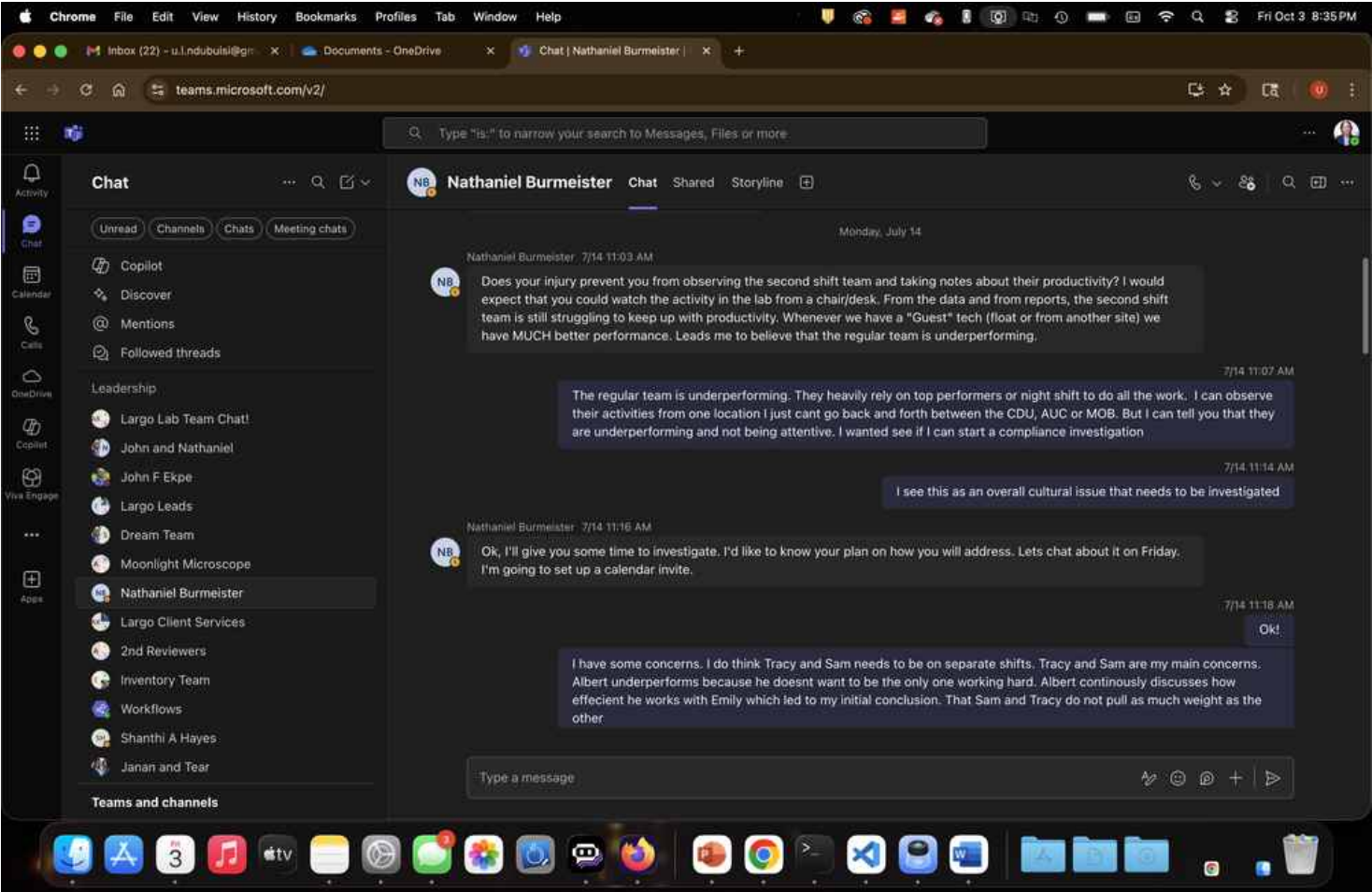
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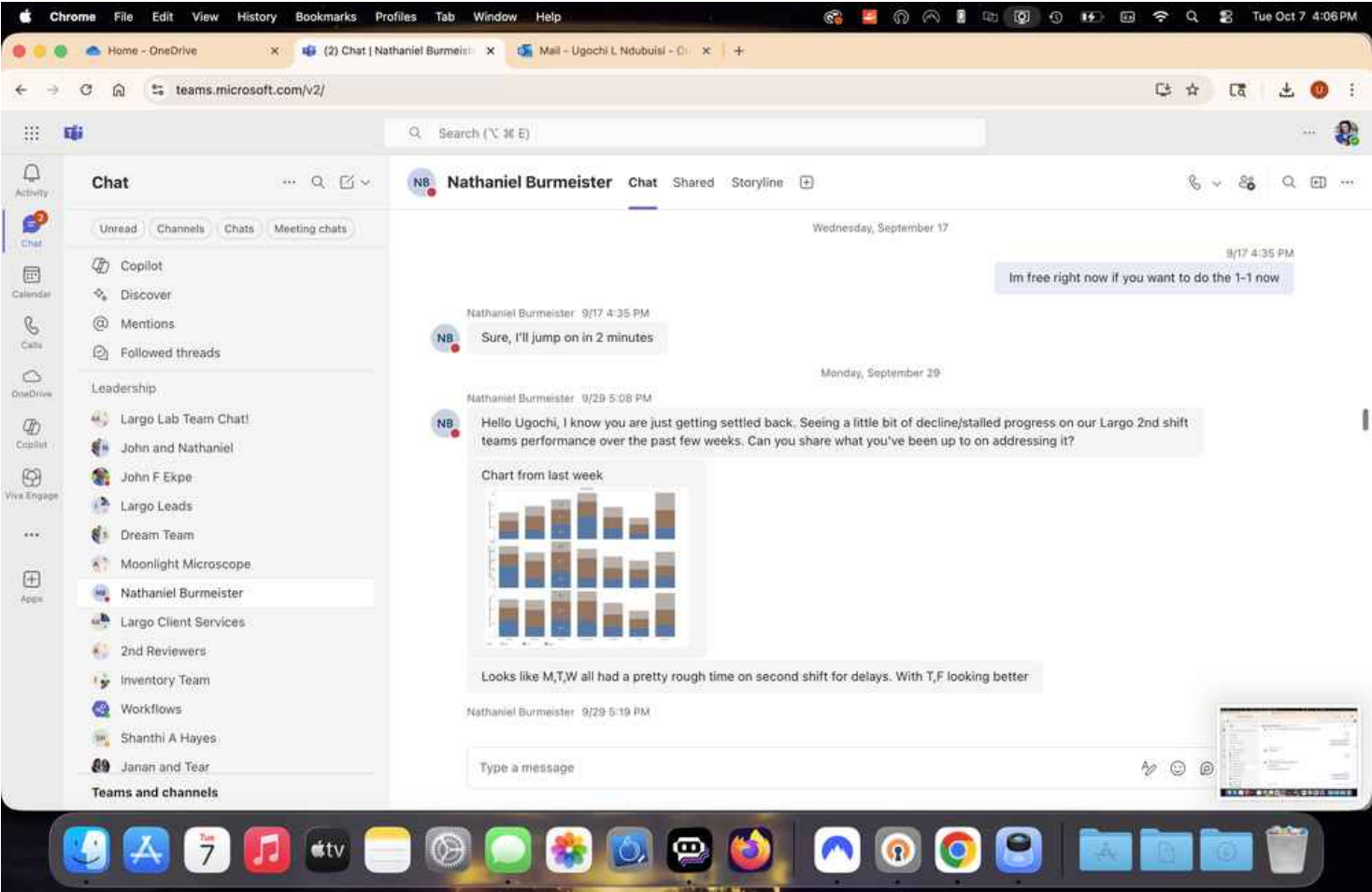
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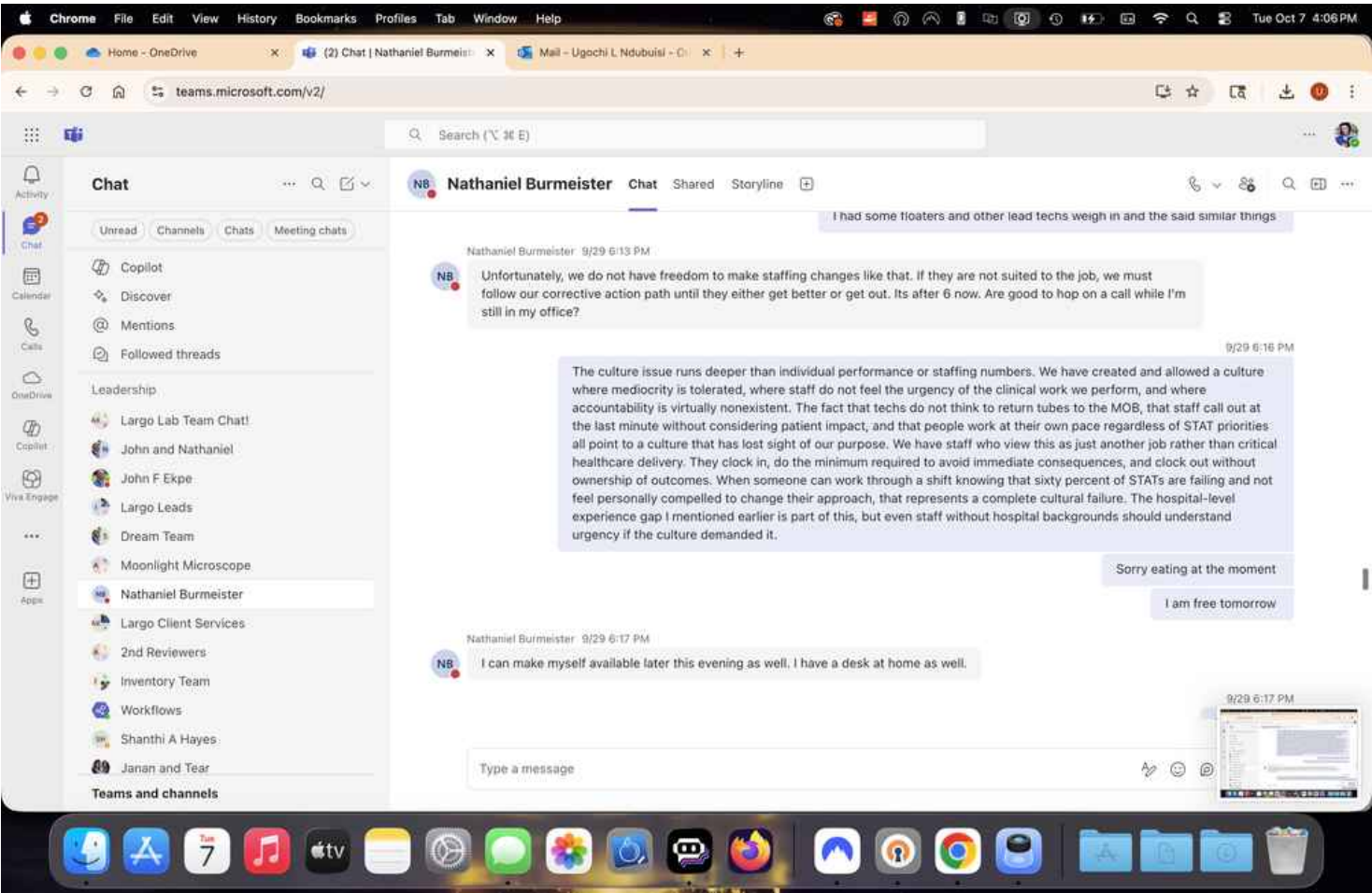
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The screenshot shows a Microsoft Teams chat window on a Mac. The browser address bar at the top indicates the URL is teams.microsoft.com/v2/. The chat is titled "Meeting Started: OPS Huddle Daily". The chat history shows a conversation with Nathaniel Burmeister. The messages are as follows:

- Nathaniel Burmeister (9/29 6:13 PM): Unfortunately, we do not have freedom to make staffing changes like that. If they are not suited to the job, we must follow our corrective action path until they either get better or get out. Its after 6 now. Are good to hop on a call while I'm still in my office?
- (9/29 6:16 PM): The culture issue runs deeper than individual performance or staffing numbers. We have created and allowed a culture where mediocrity is tolerated, where staff do not feel the urgency of the clinical work we perform, and where accountability is virtually nonexistent. The fact that techs do not think to return tubes to the MOB, that staff call out at the last minute without considering patient impact, and that people work at their own pace regardless of STAT priorities all point to a culture that has lost sight of our purpose. We have staff who view this as just another job rather than critical healthcare delivery. They clock in, do the minimum required to avoid immediate consequences, and clock out without ownership of outcomes. When someone can work through a shift knowing that sixty percent of STATs are failing and not feel personally compelled to change their approach, that represents a complete cultural failure. The hospital-level experience gap I mentioned earlier is part of this, but even staff without hospital backgrounds should understand urgency if the culture demanded it.
- (9/29 6:16 PM): Sorry eating at the moment
- (9/29 6:16 PM): I am free tomorrow
- Nathaniel Burmeister (9/29 6:17 PM): I can make myself available later this evening as well. I have a desk at home as well.

The interface includes a sidebar on the left with navigation options like Activity, Chat, Calendar, and a list of teams and channels. The chat input area at the bottom shows "Type a message".

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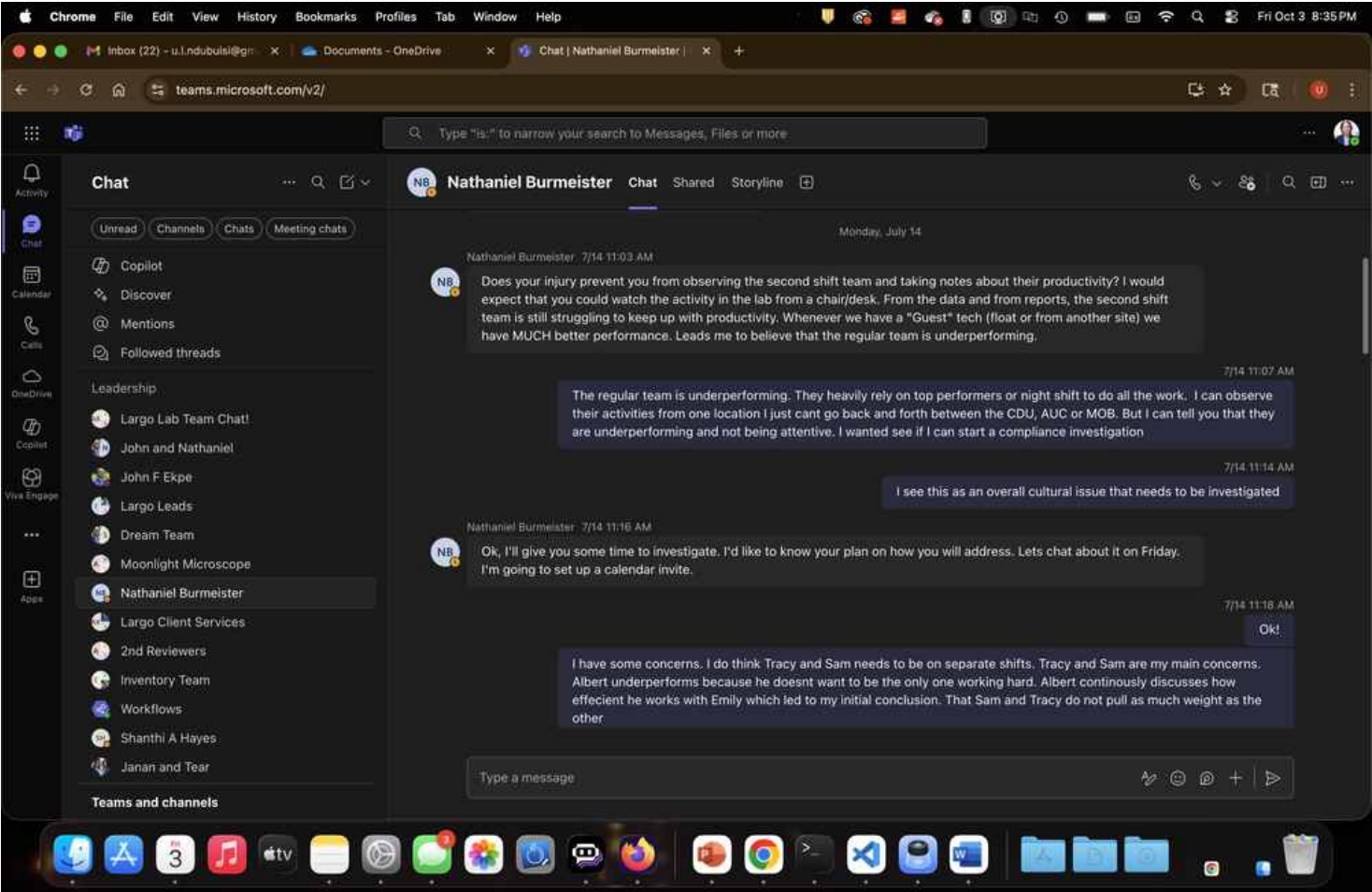
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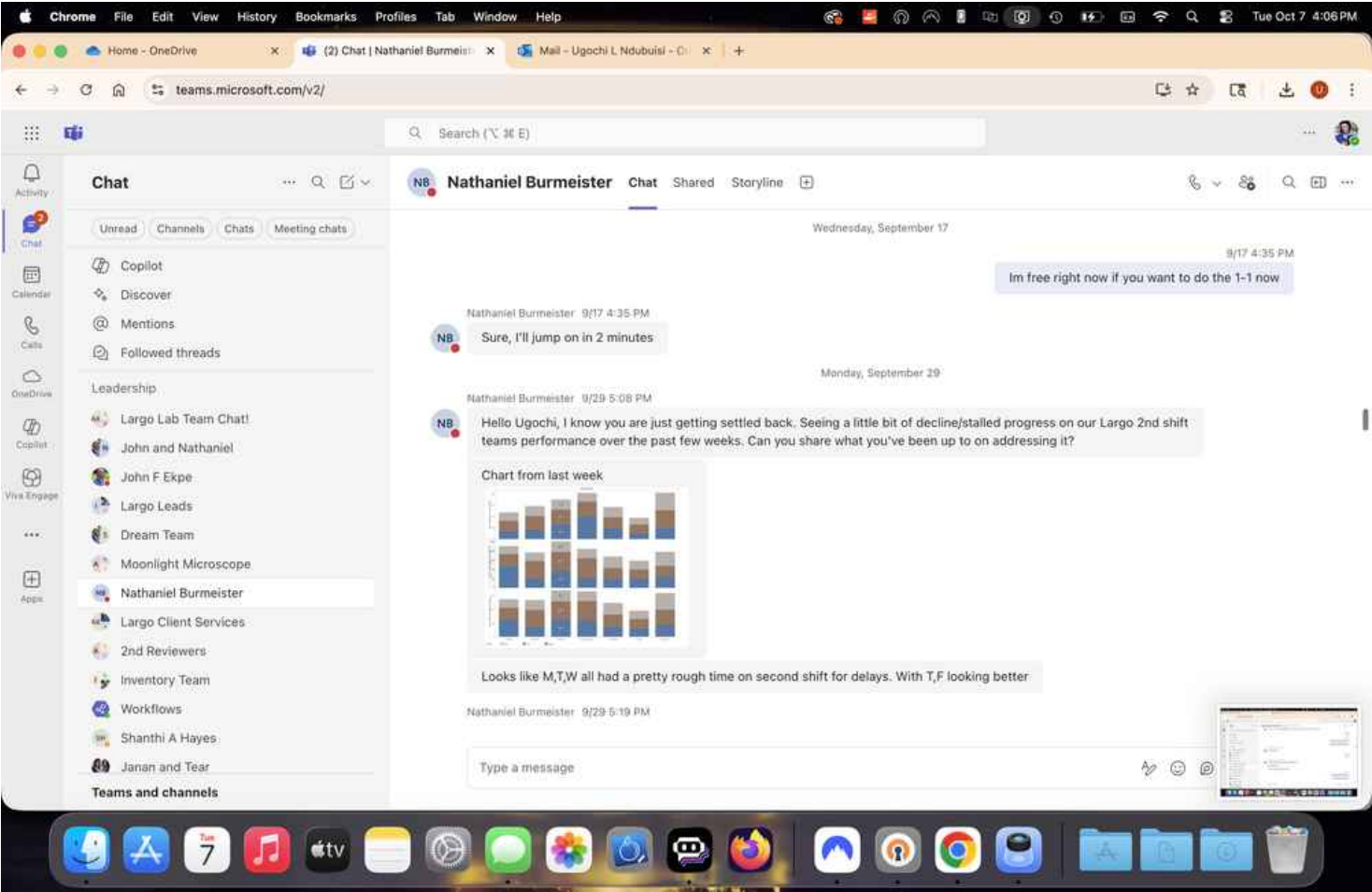
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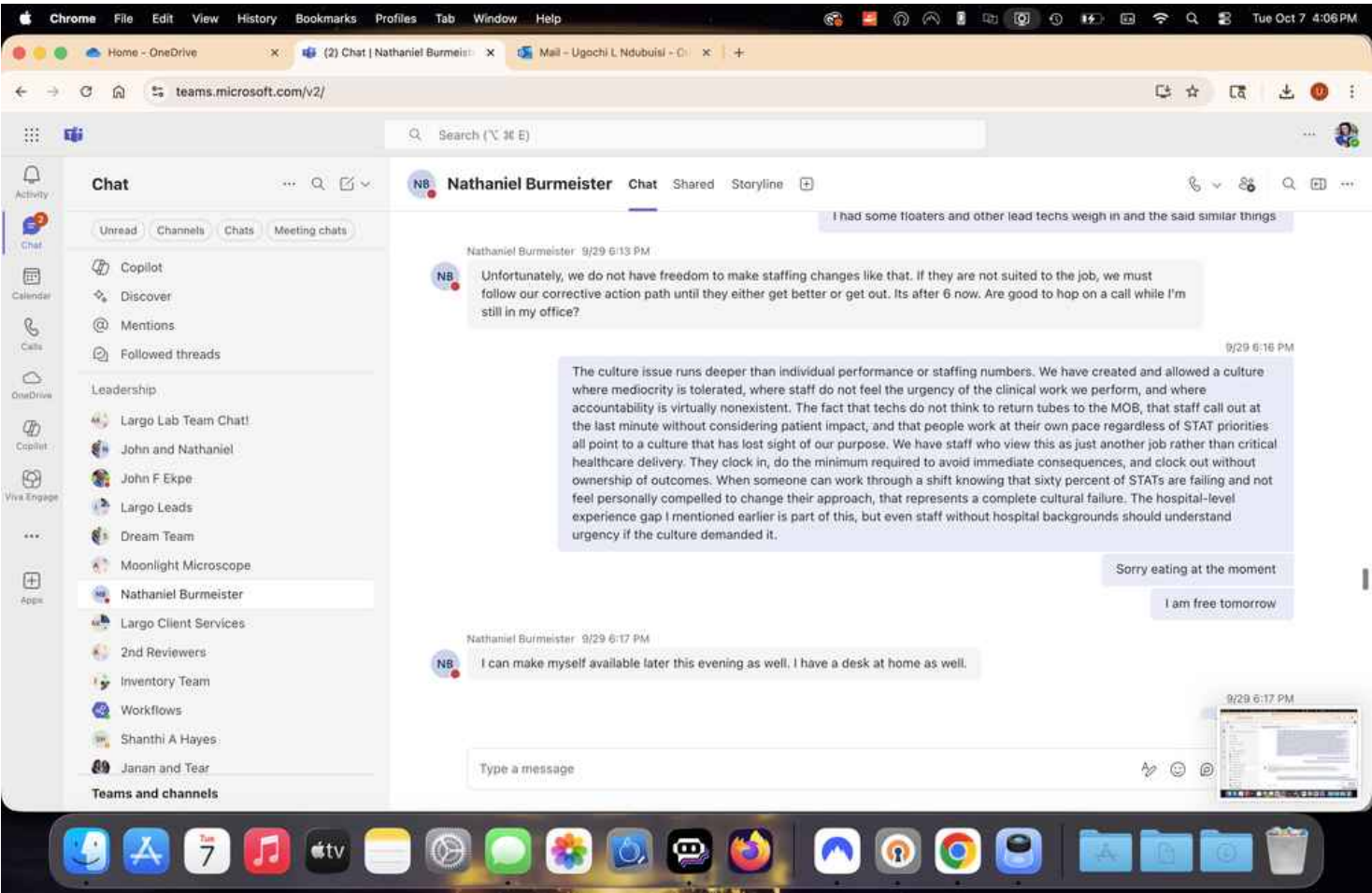
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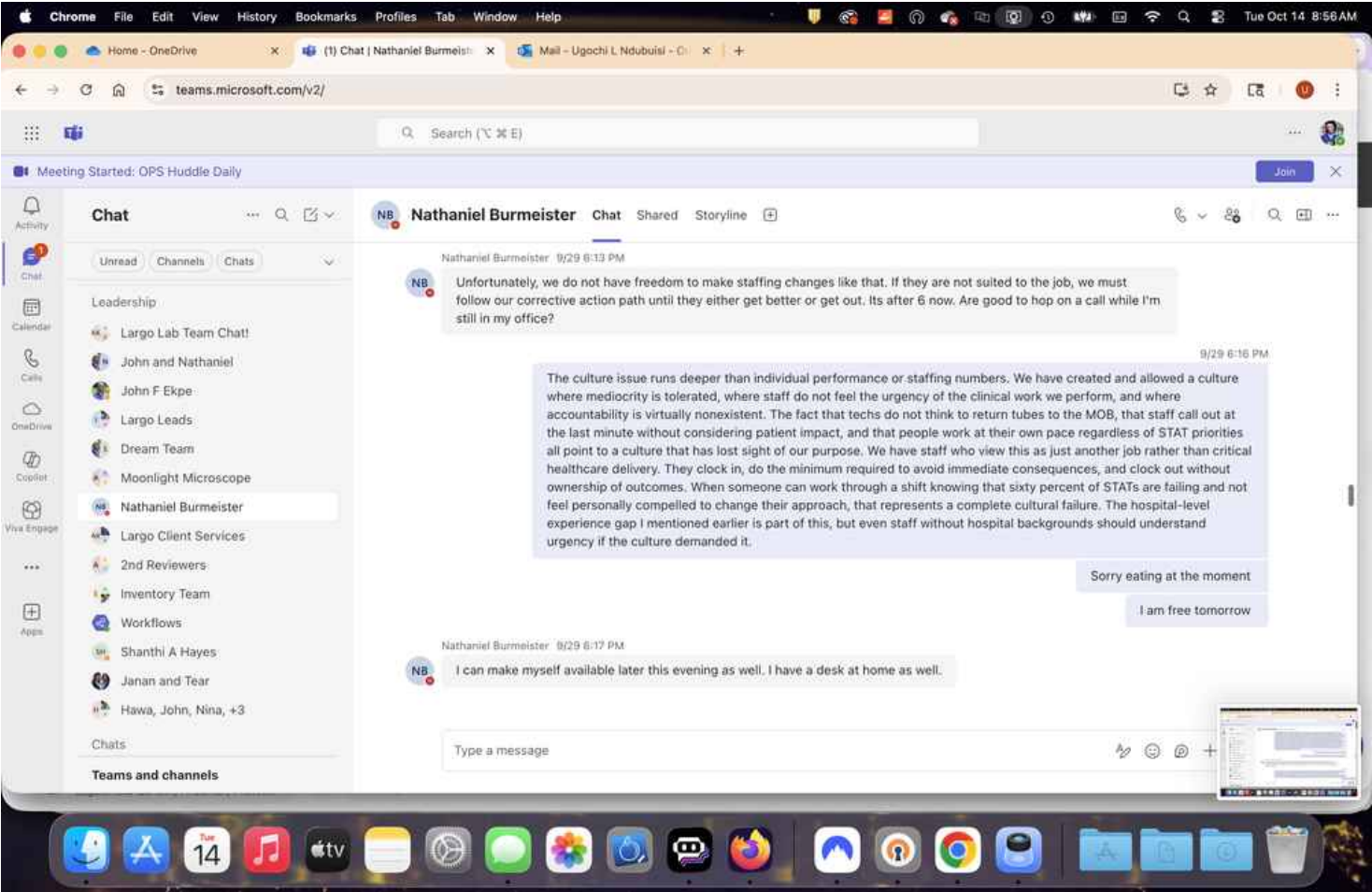
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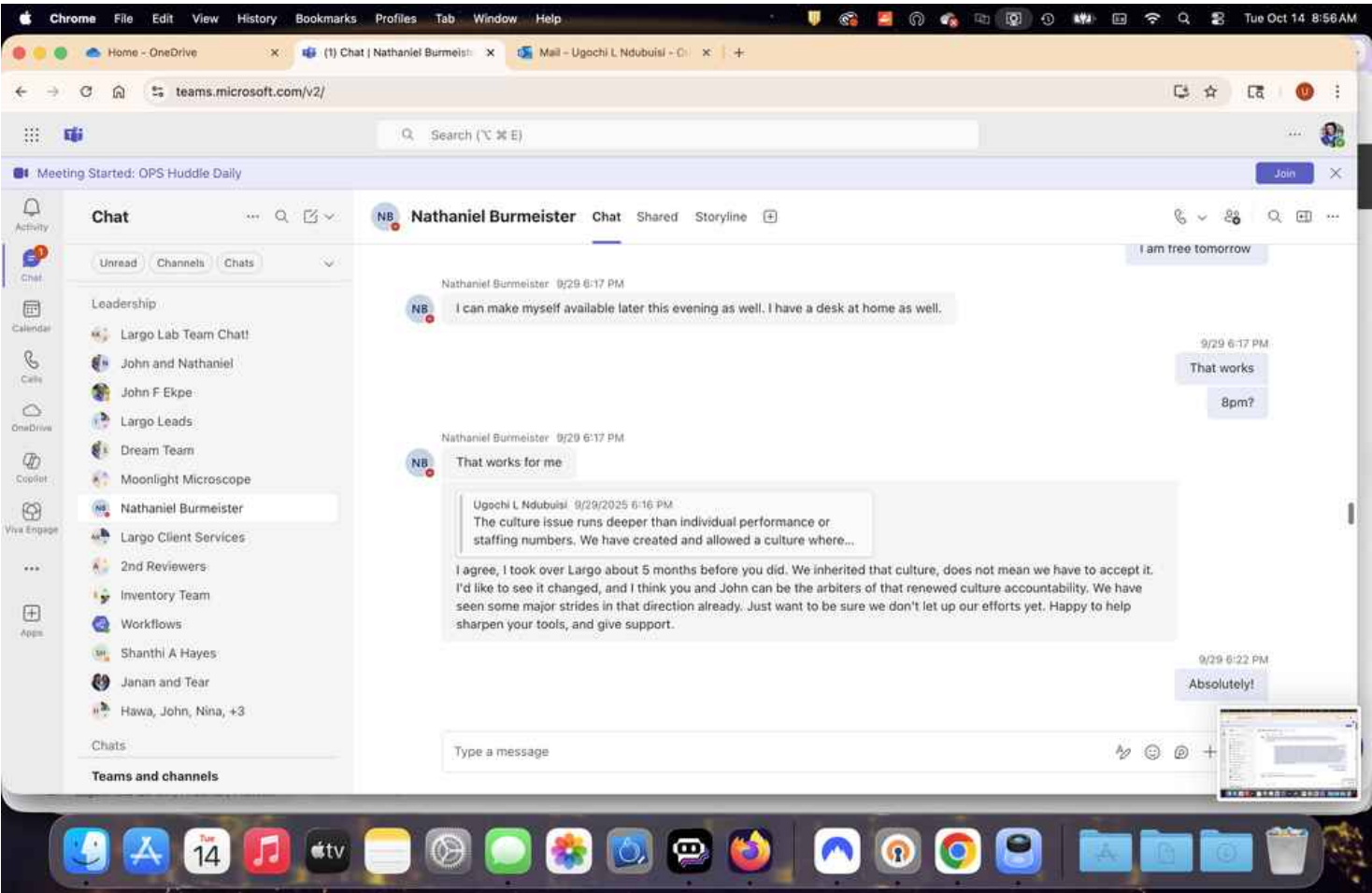
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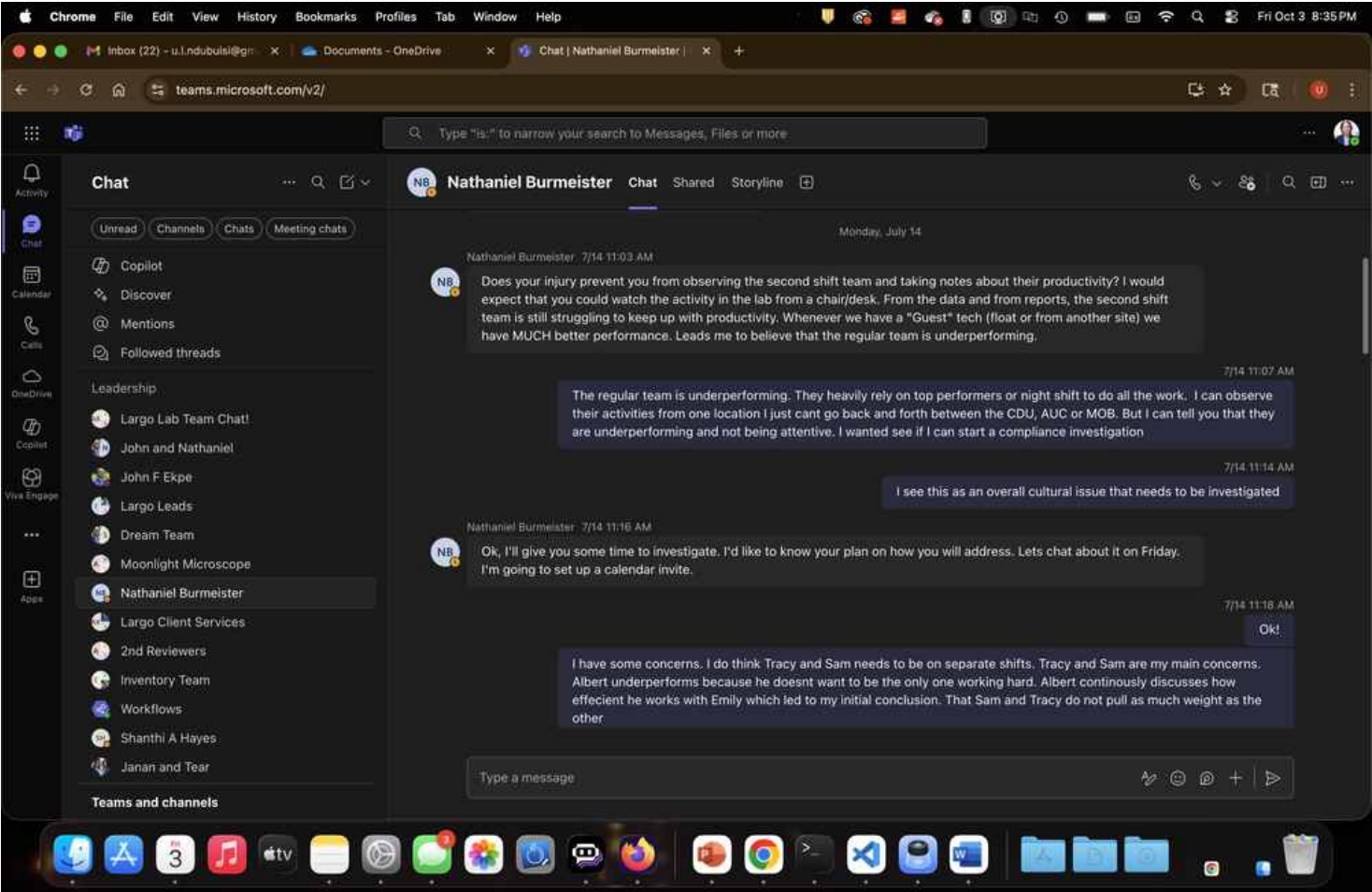
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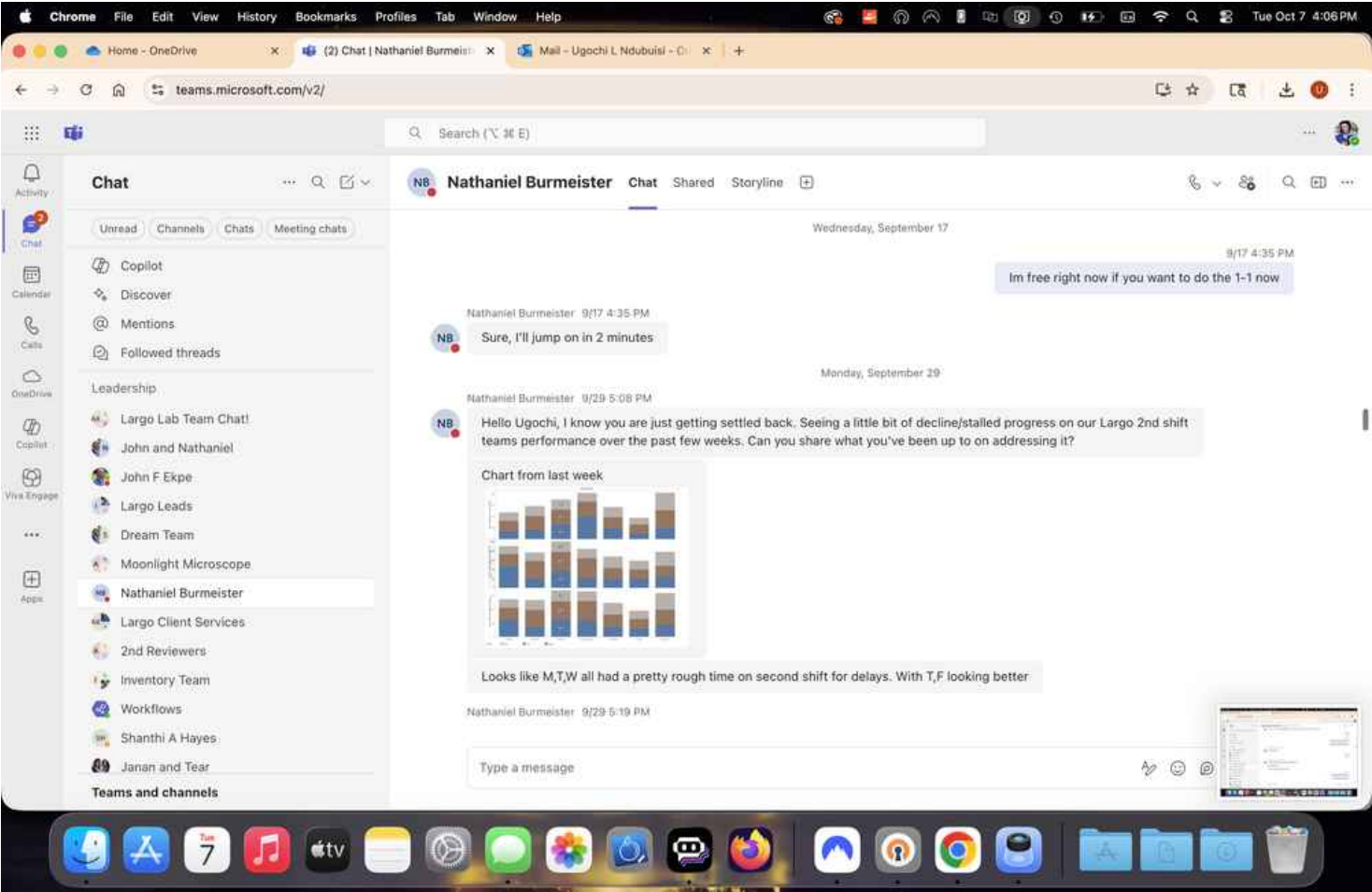
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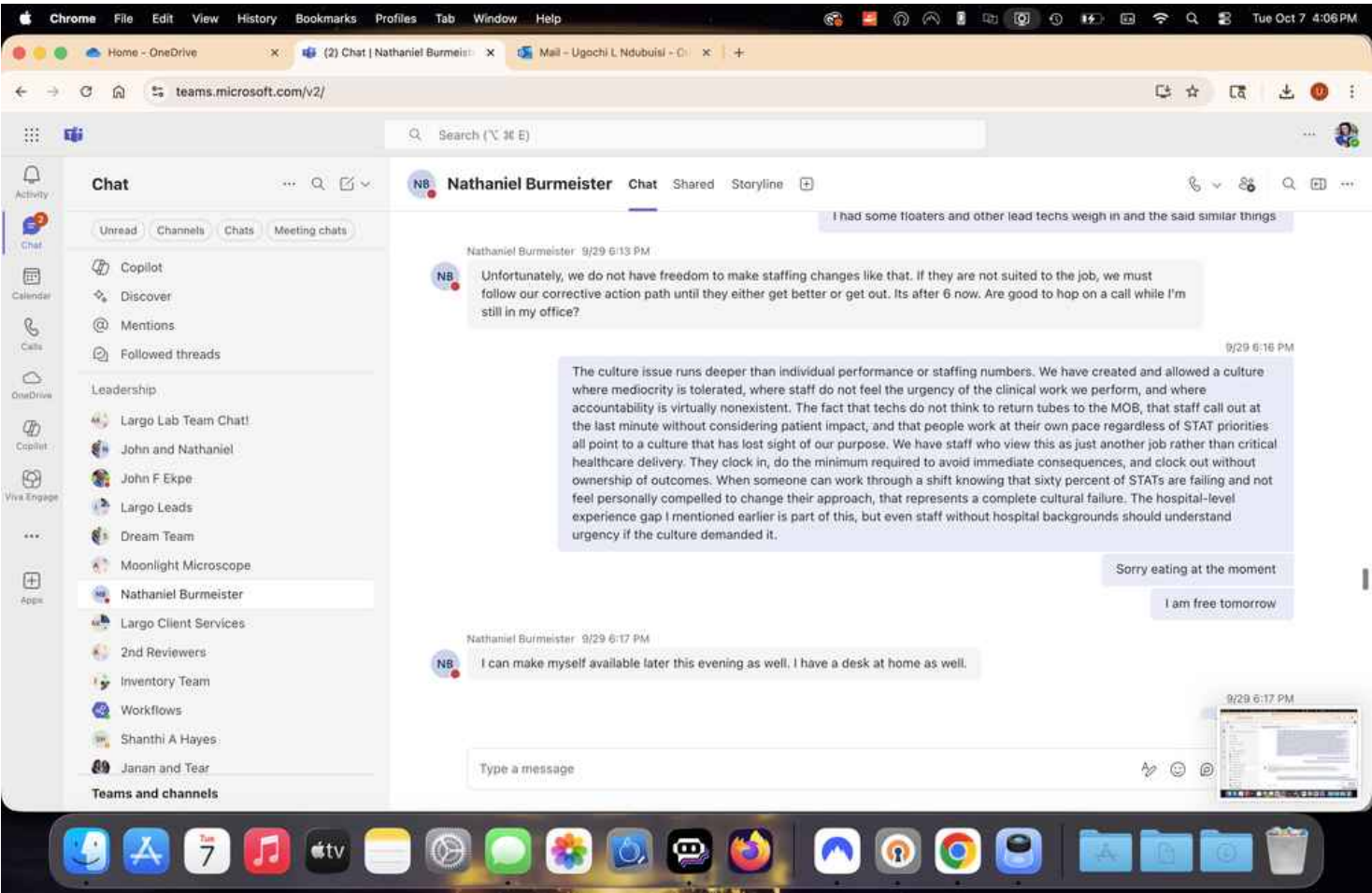
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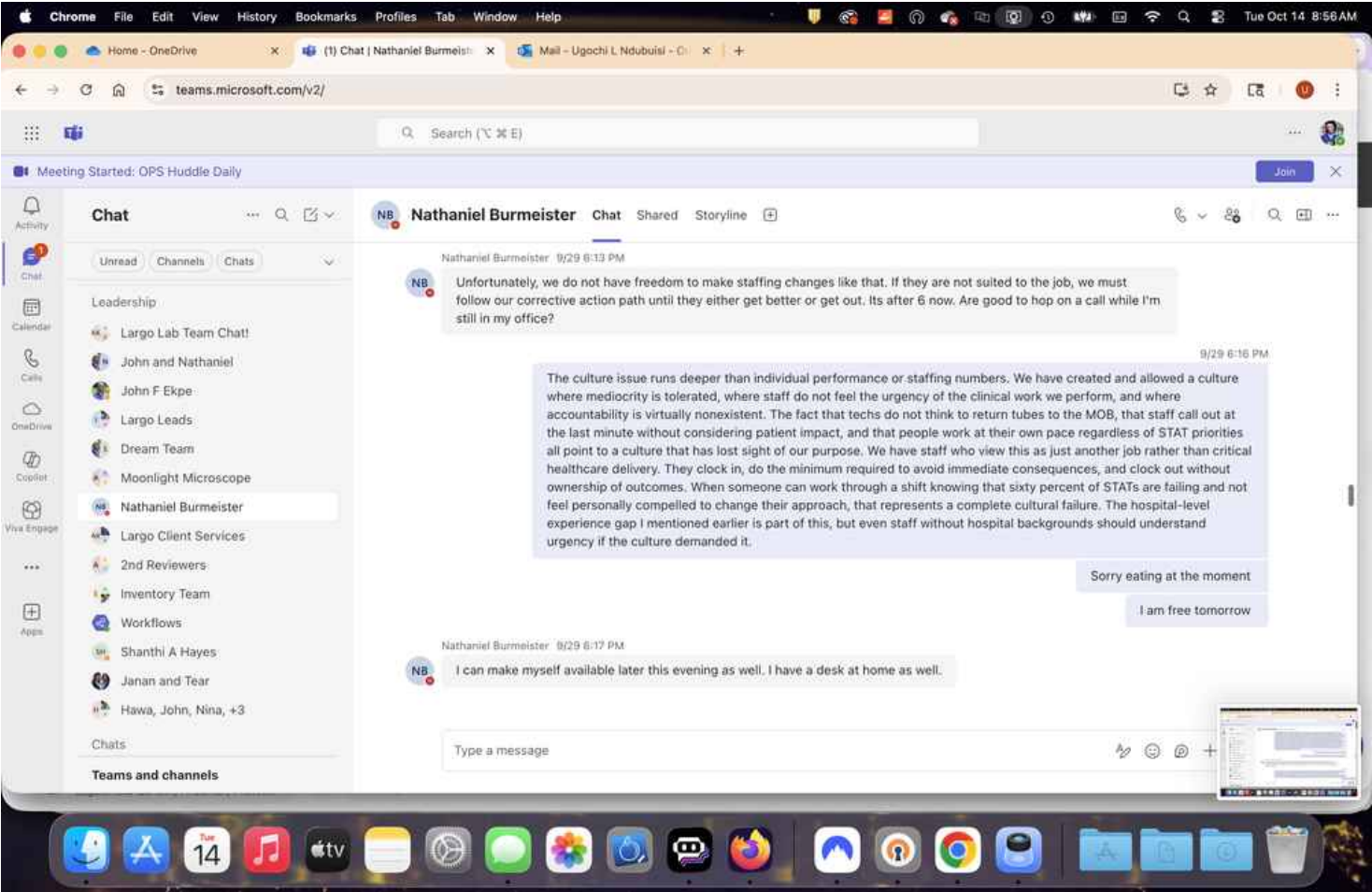
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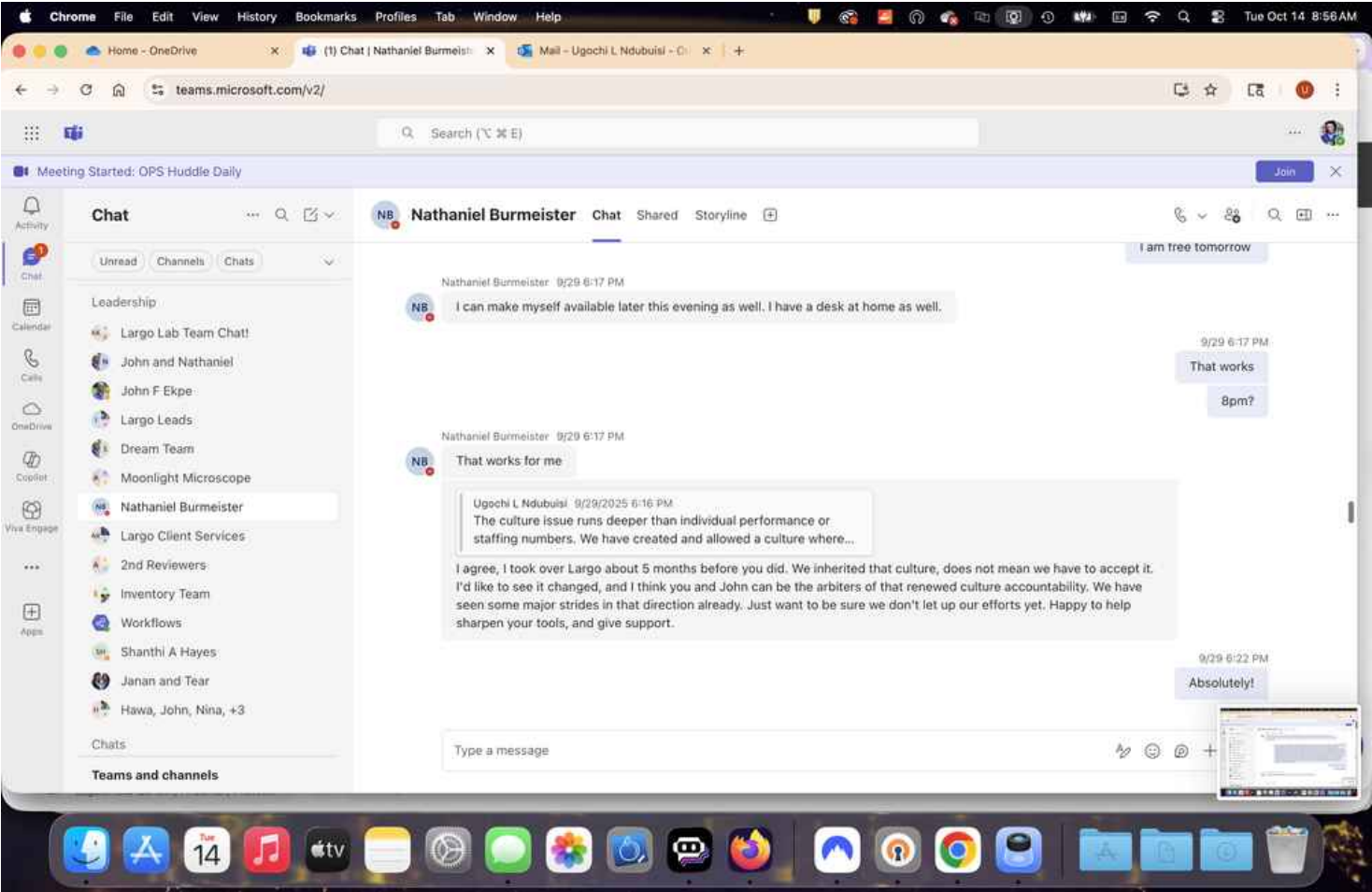


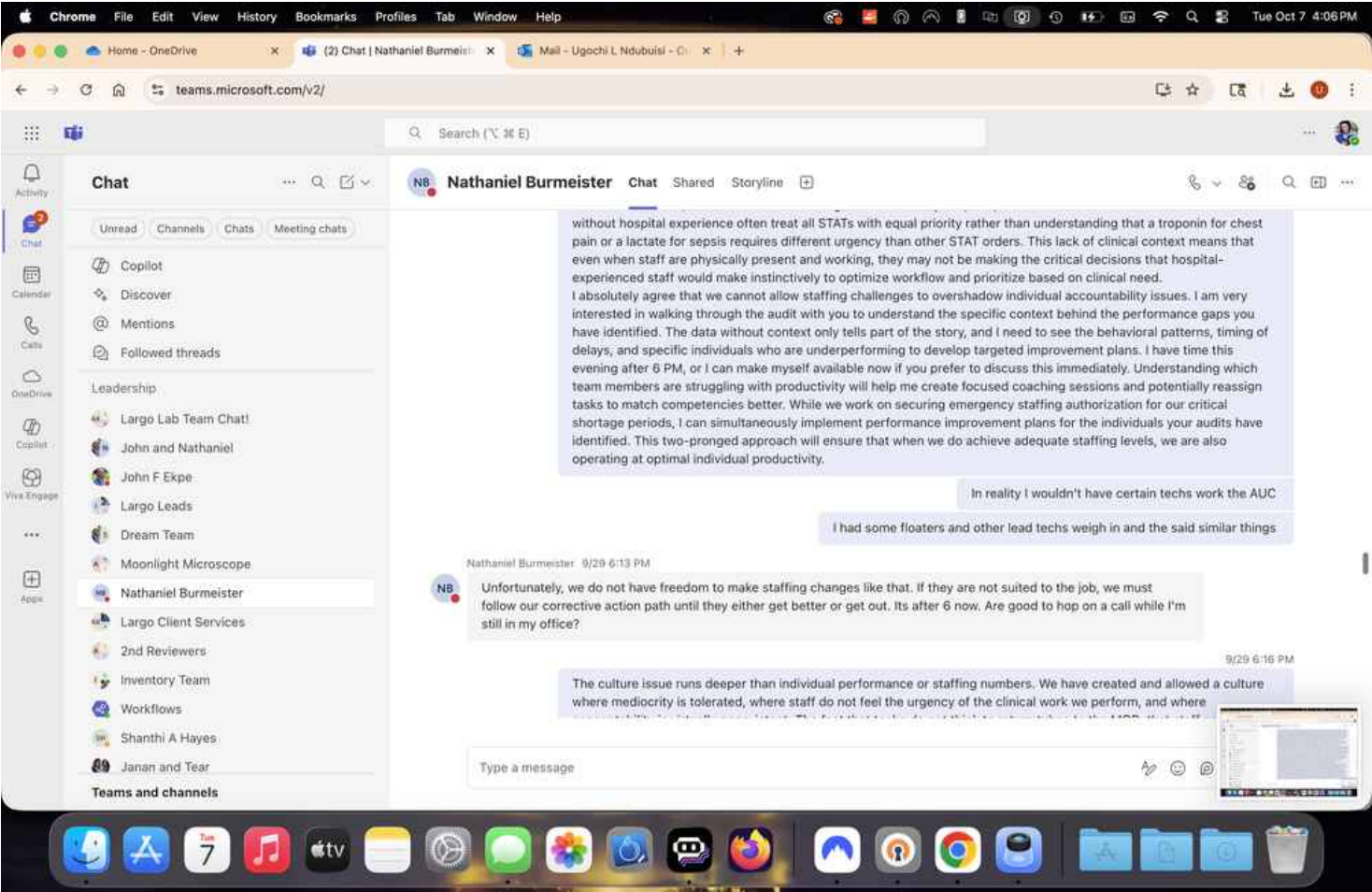




Evidence 4: Sept_29_Analysis_01_Screenshot 2025-10-14 at 8.56.10AM







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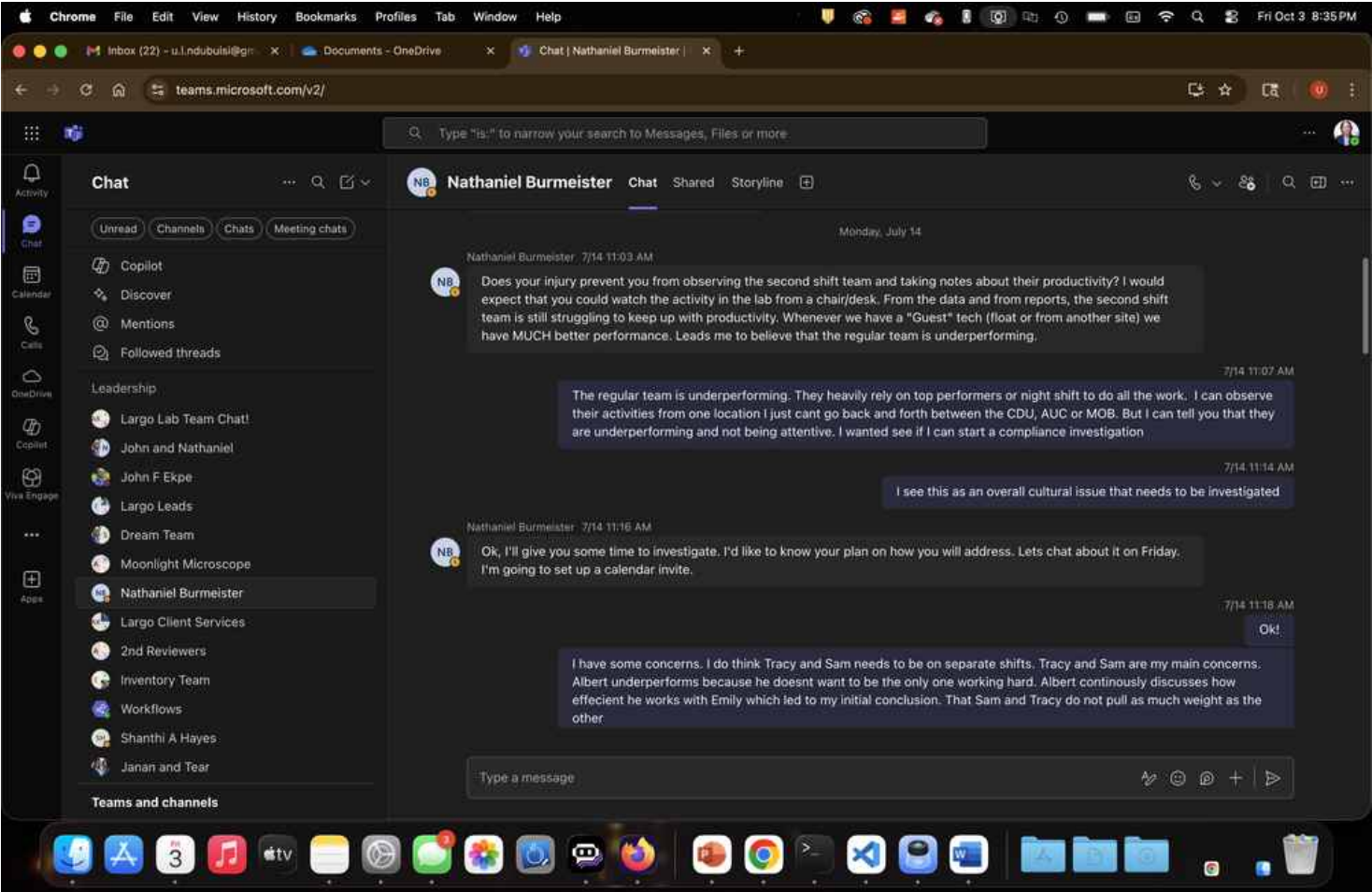
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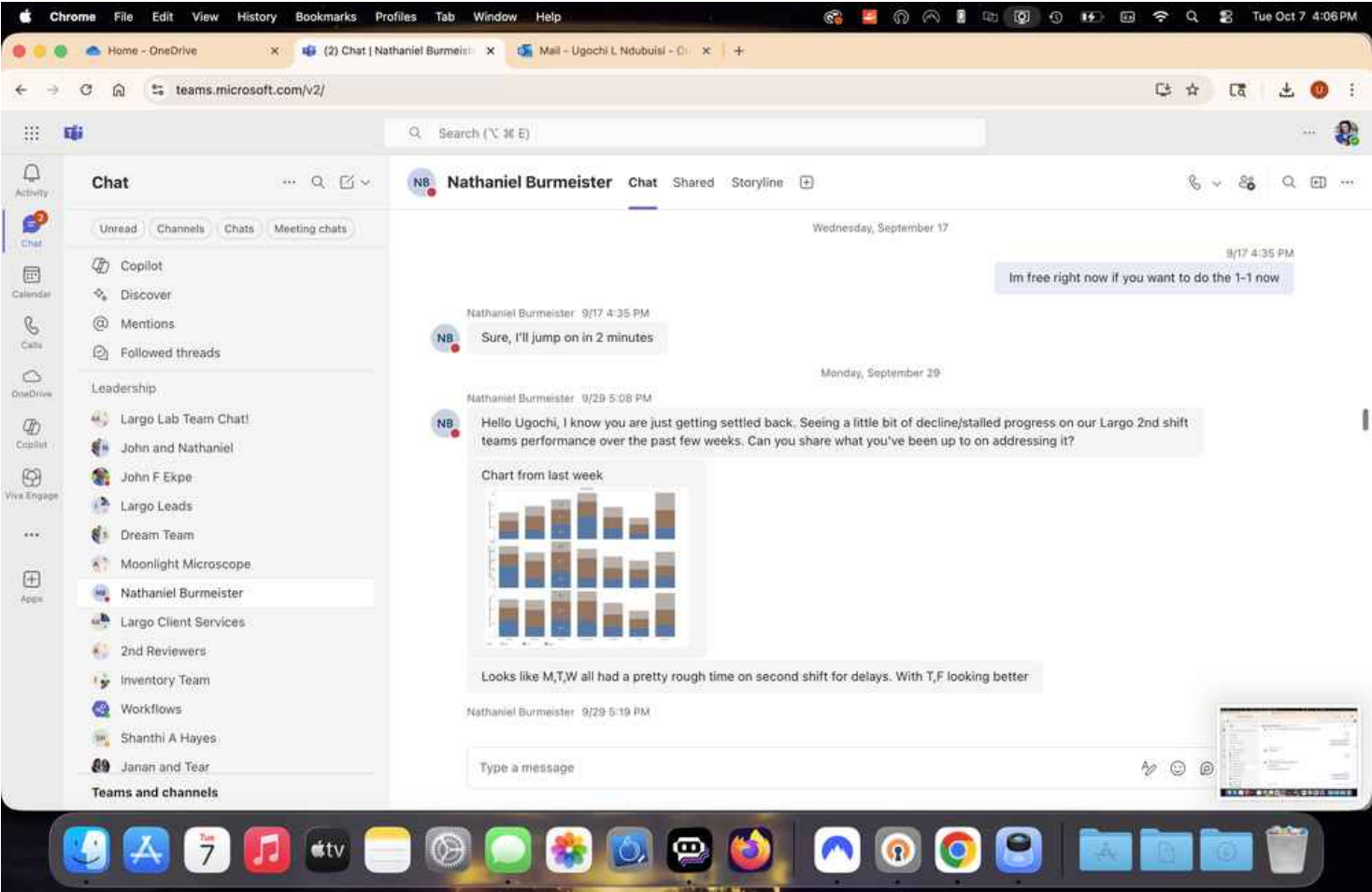
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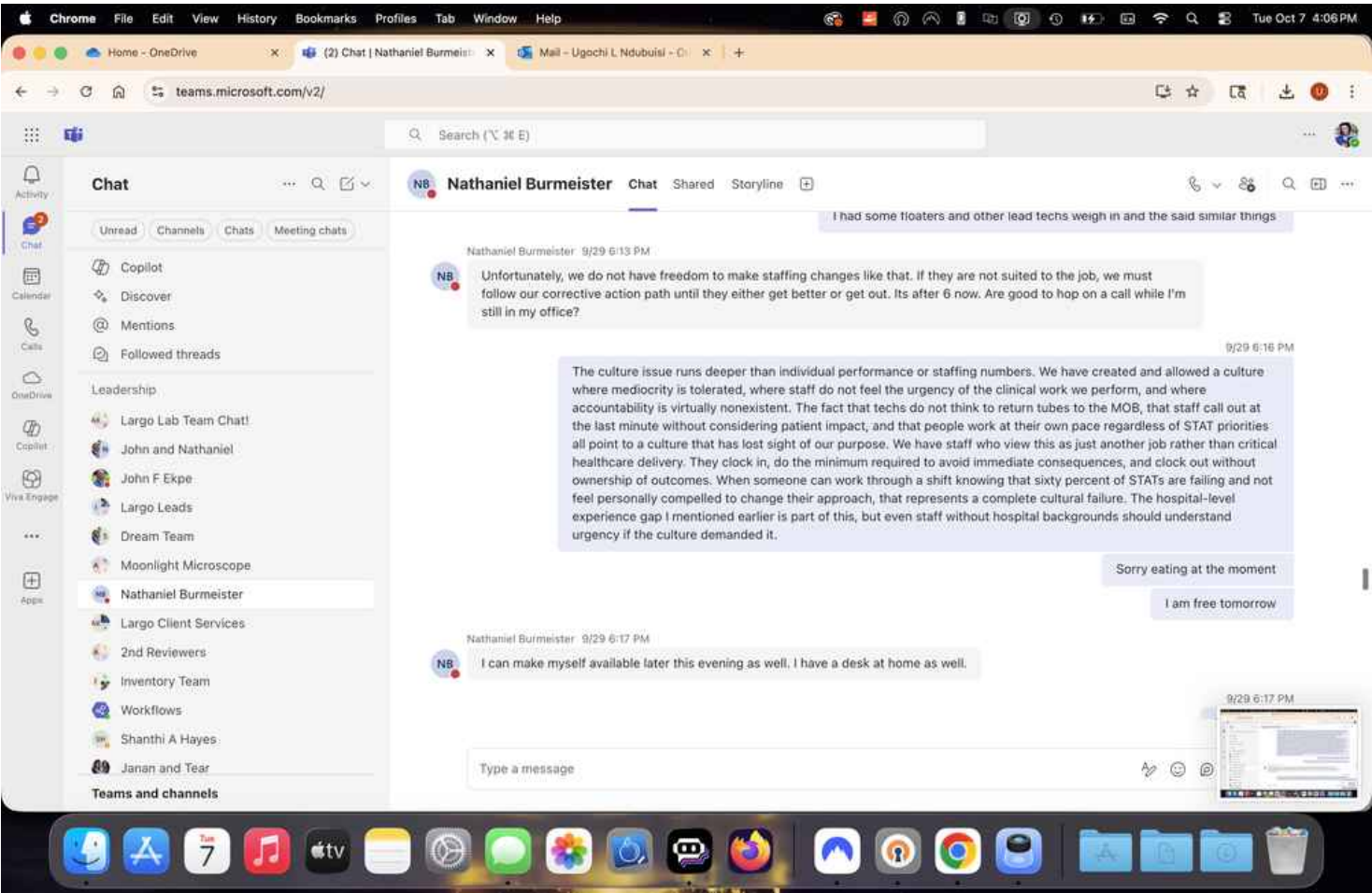
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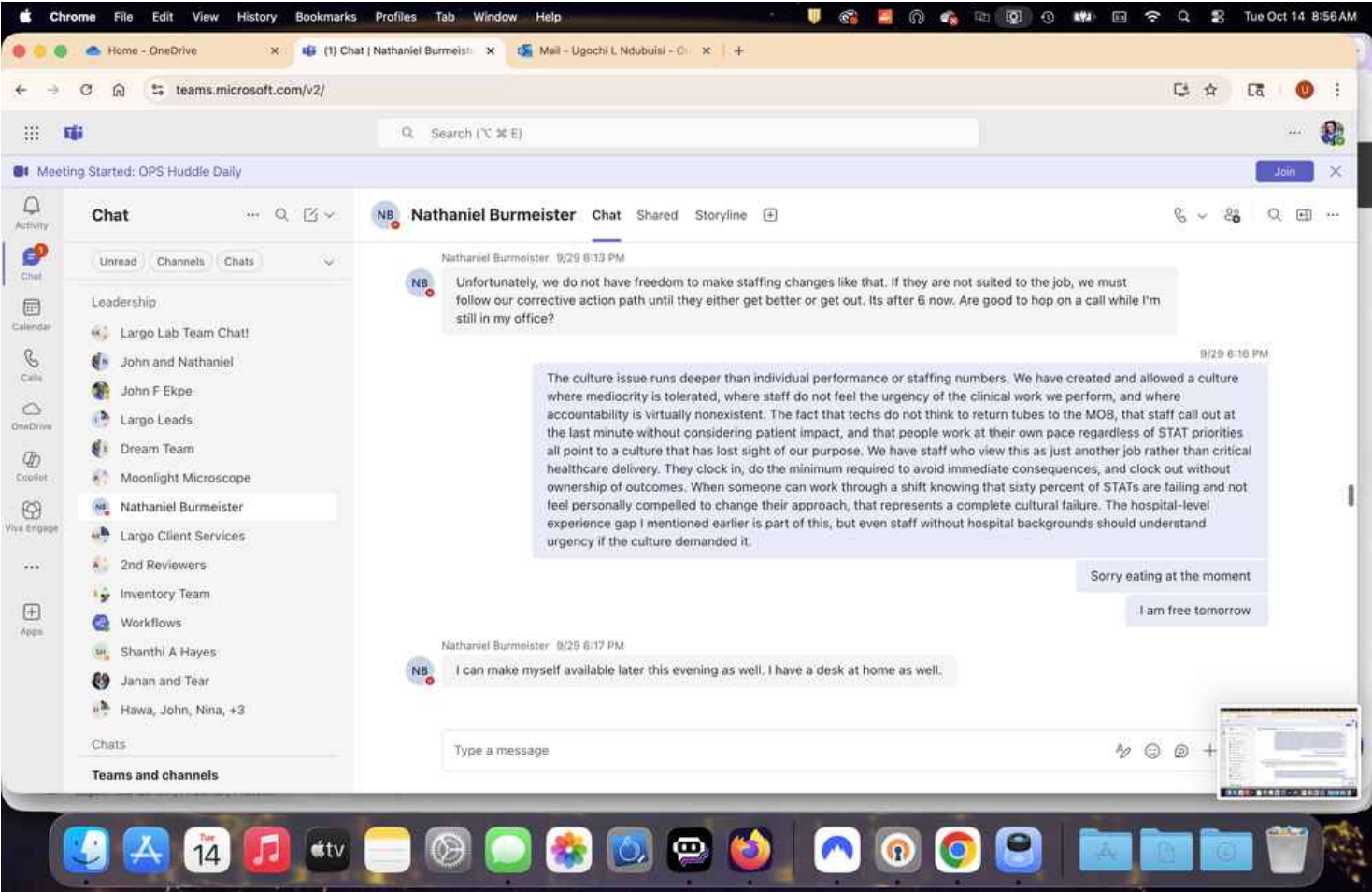
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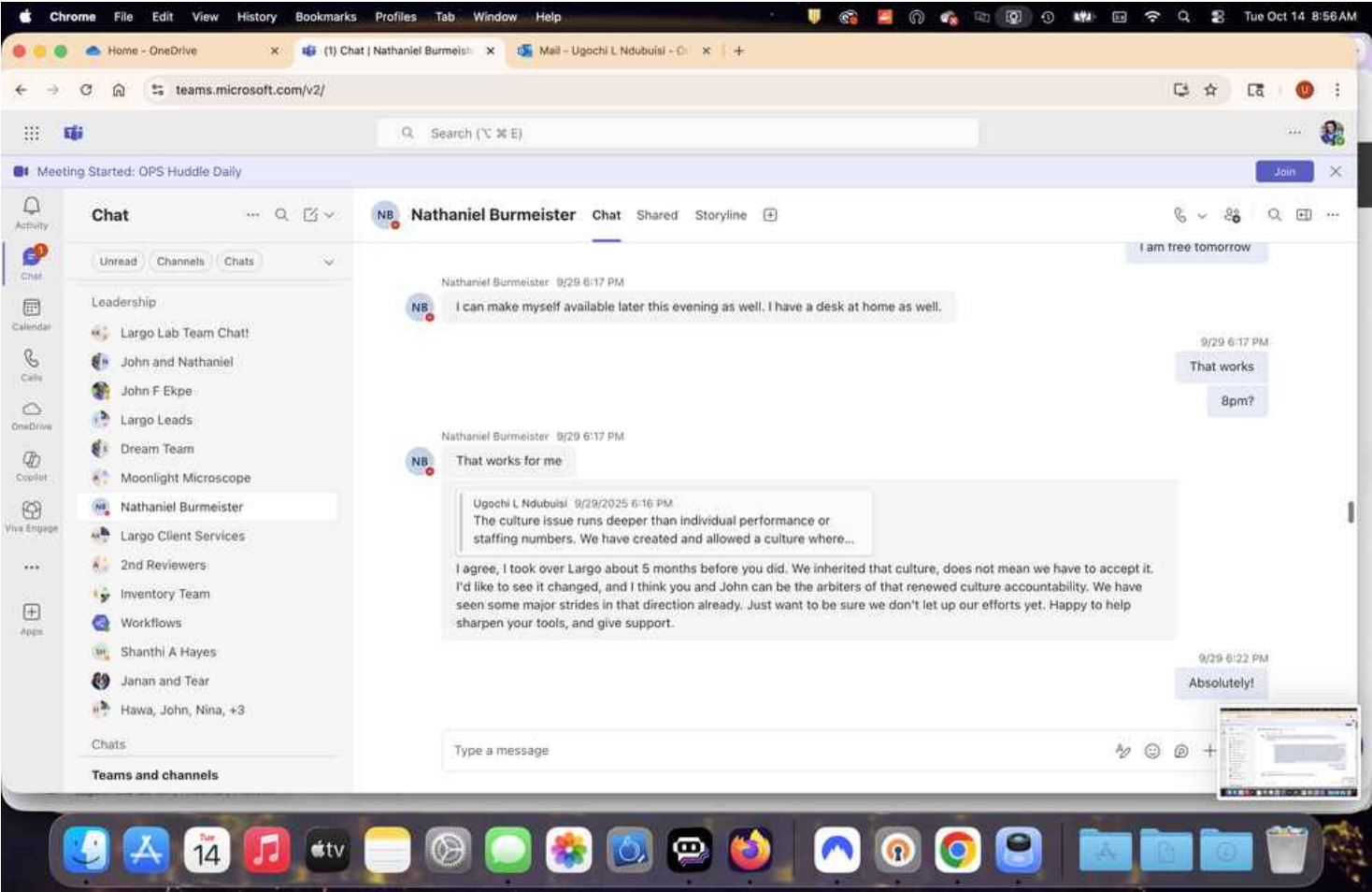


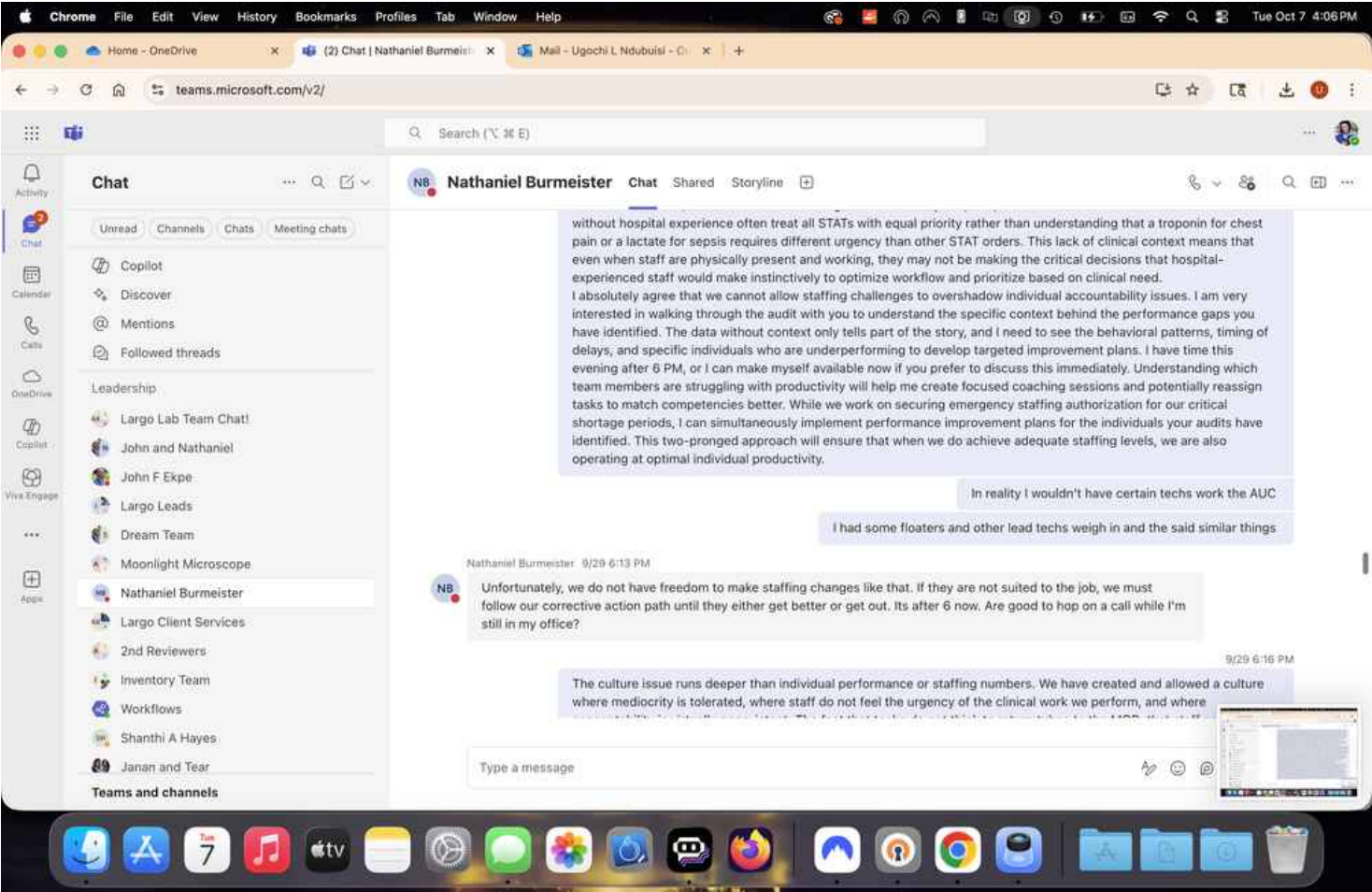


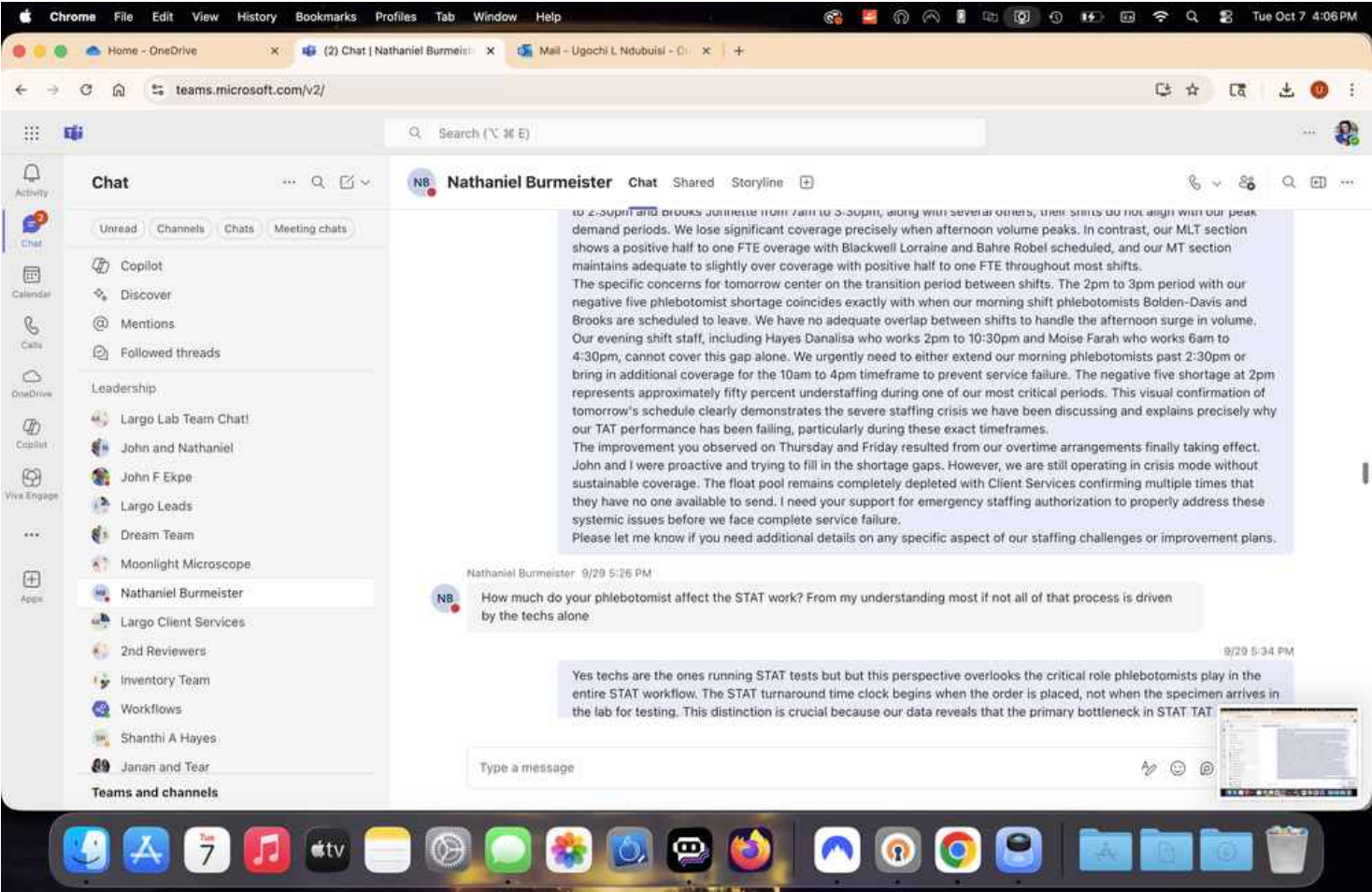


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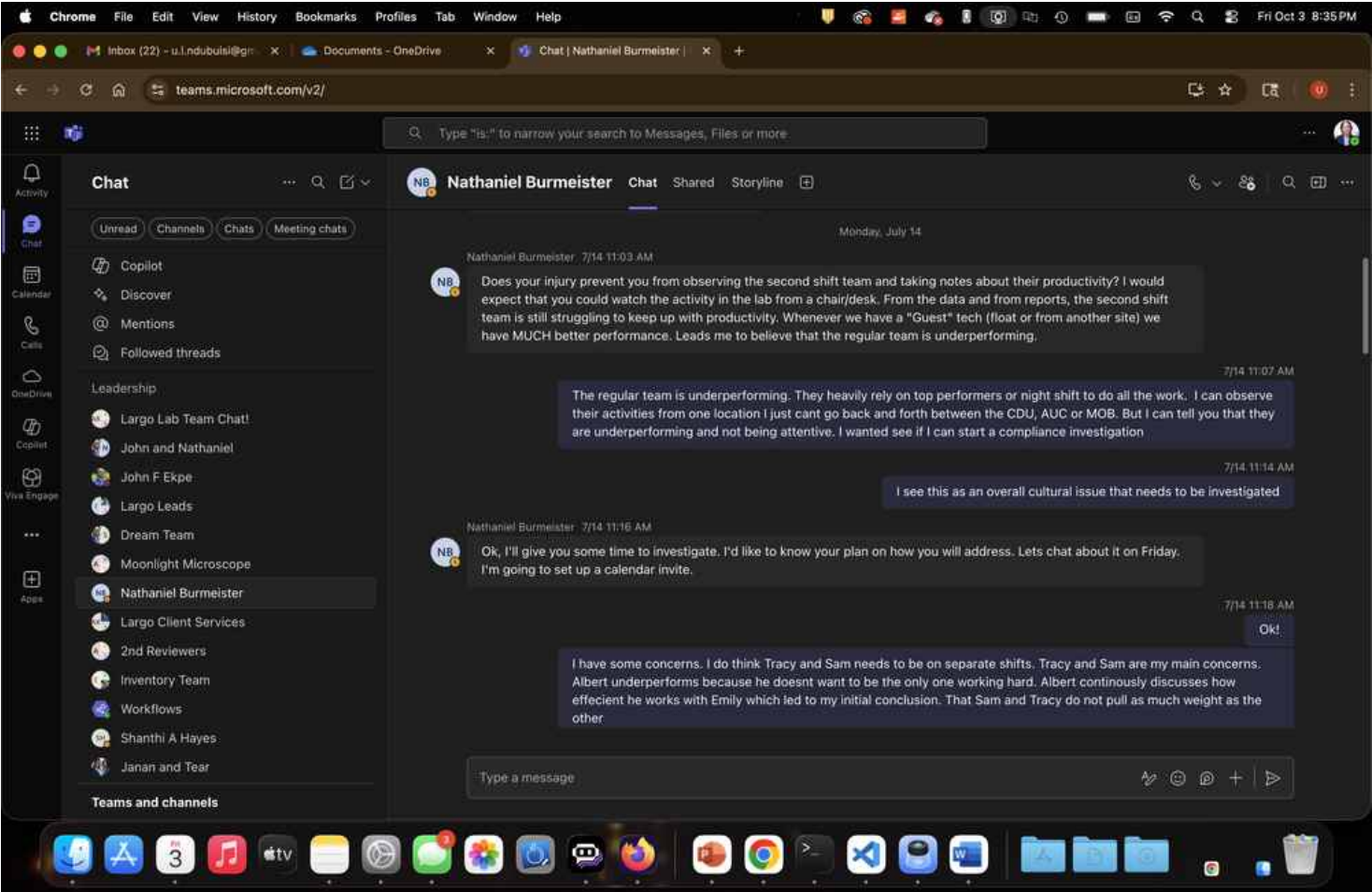
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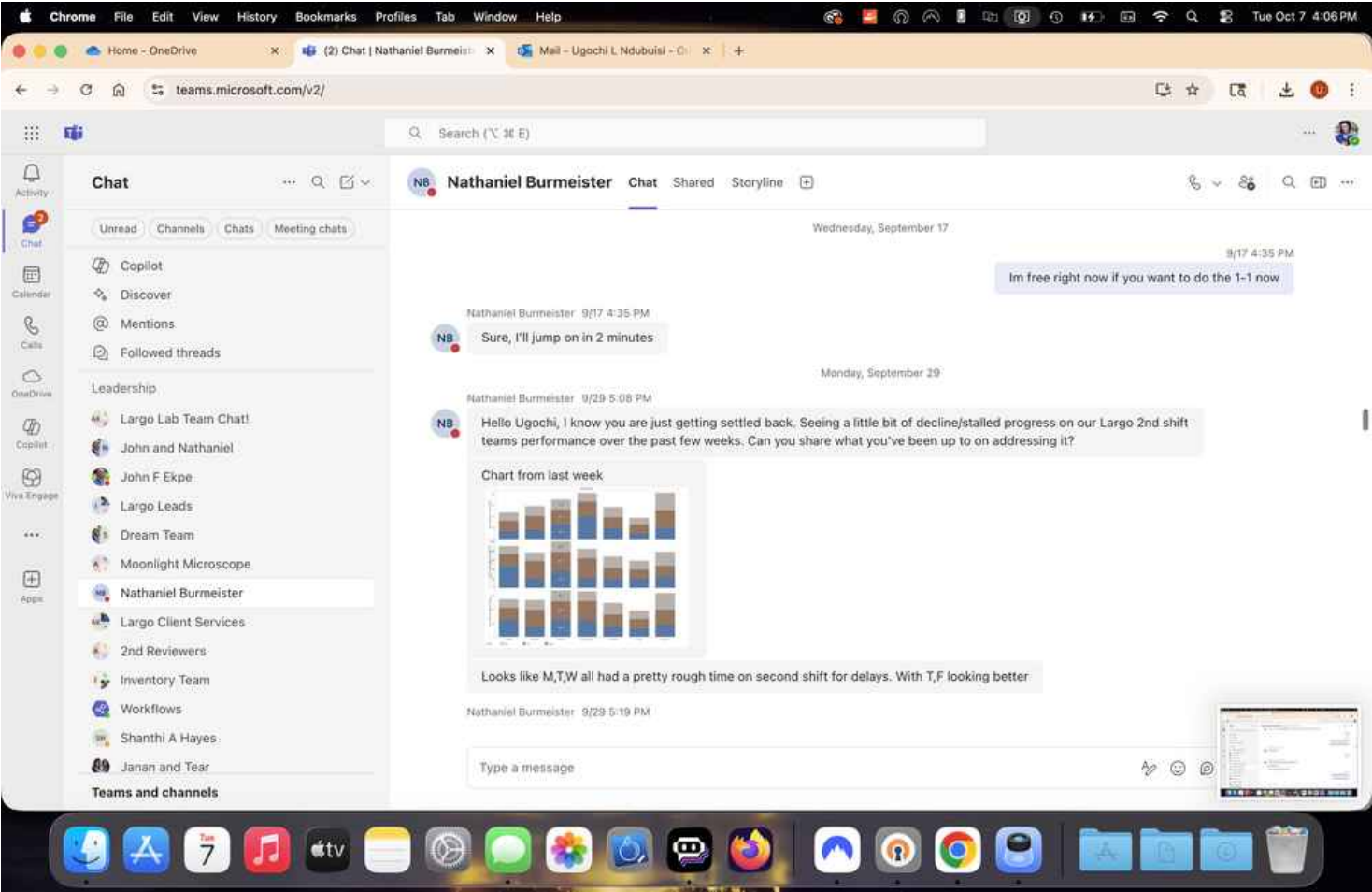
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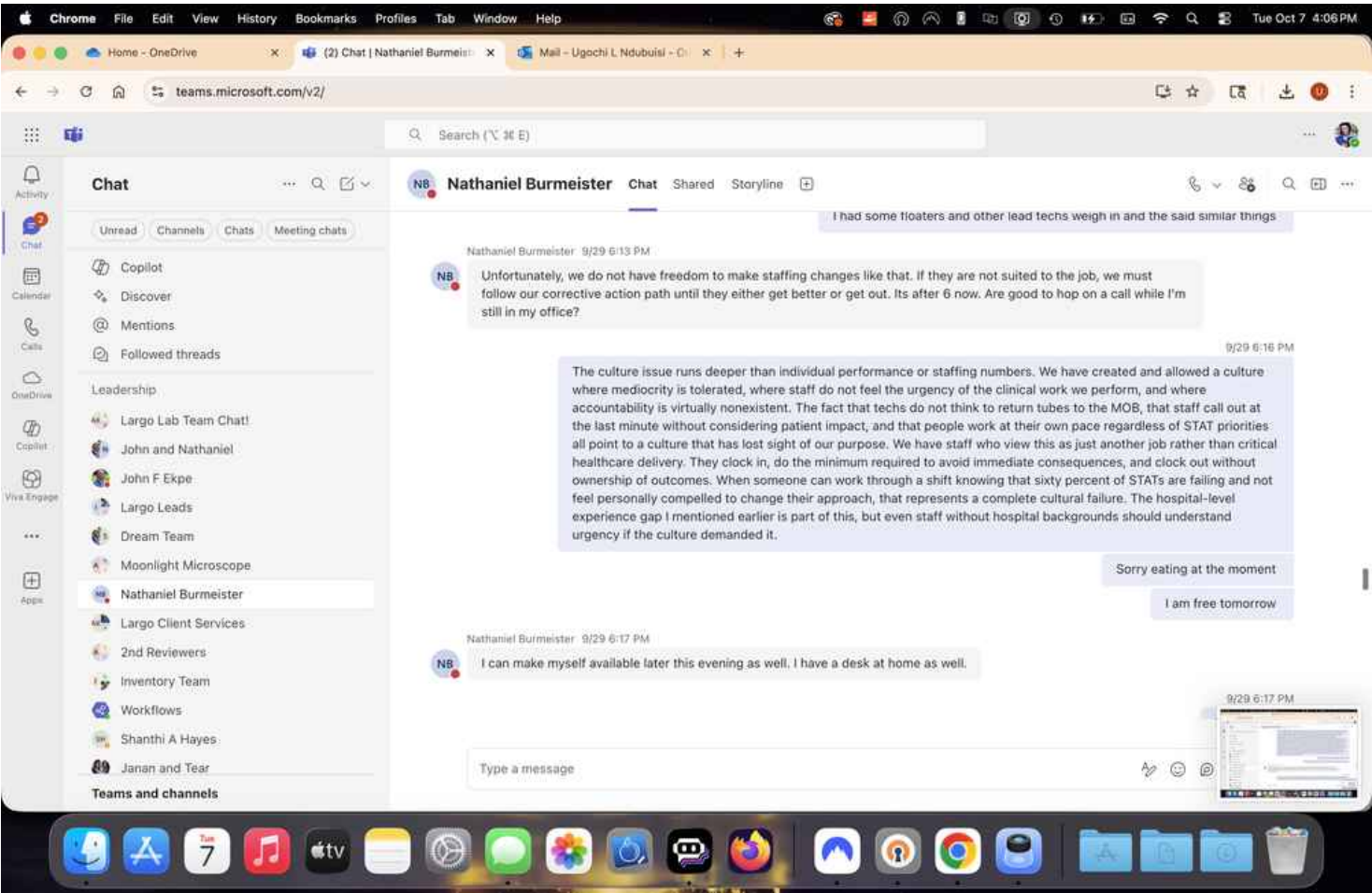
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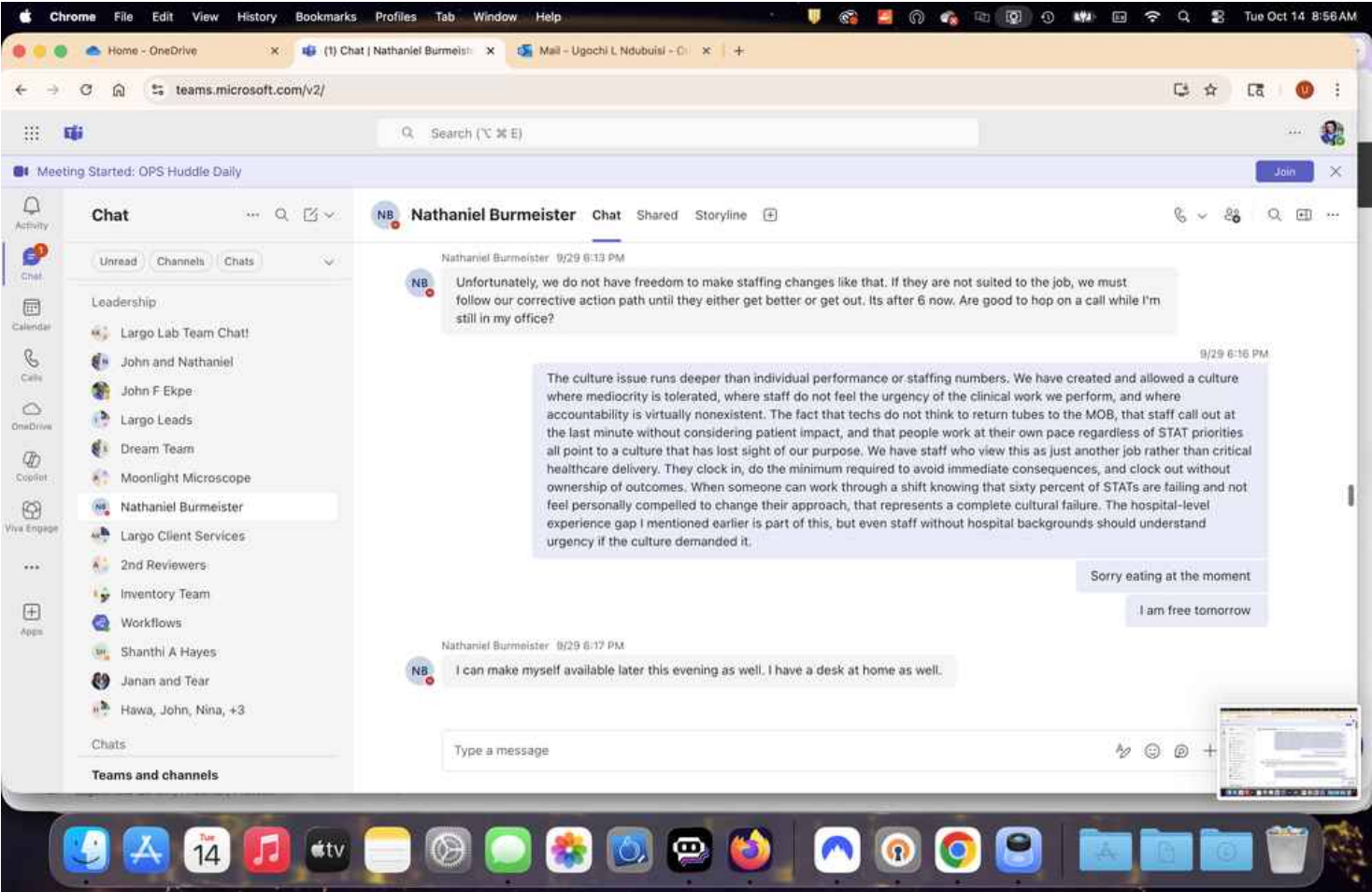
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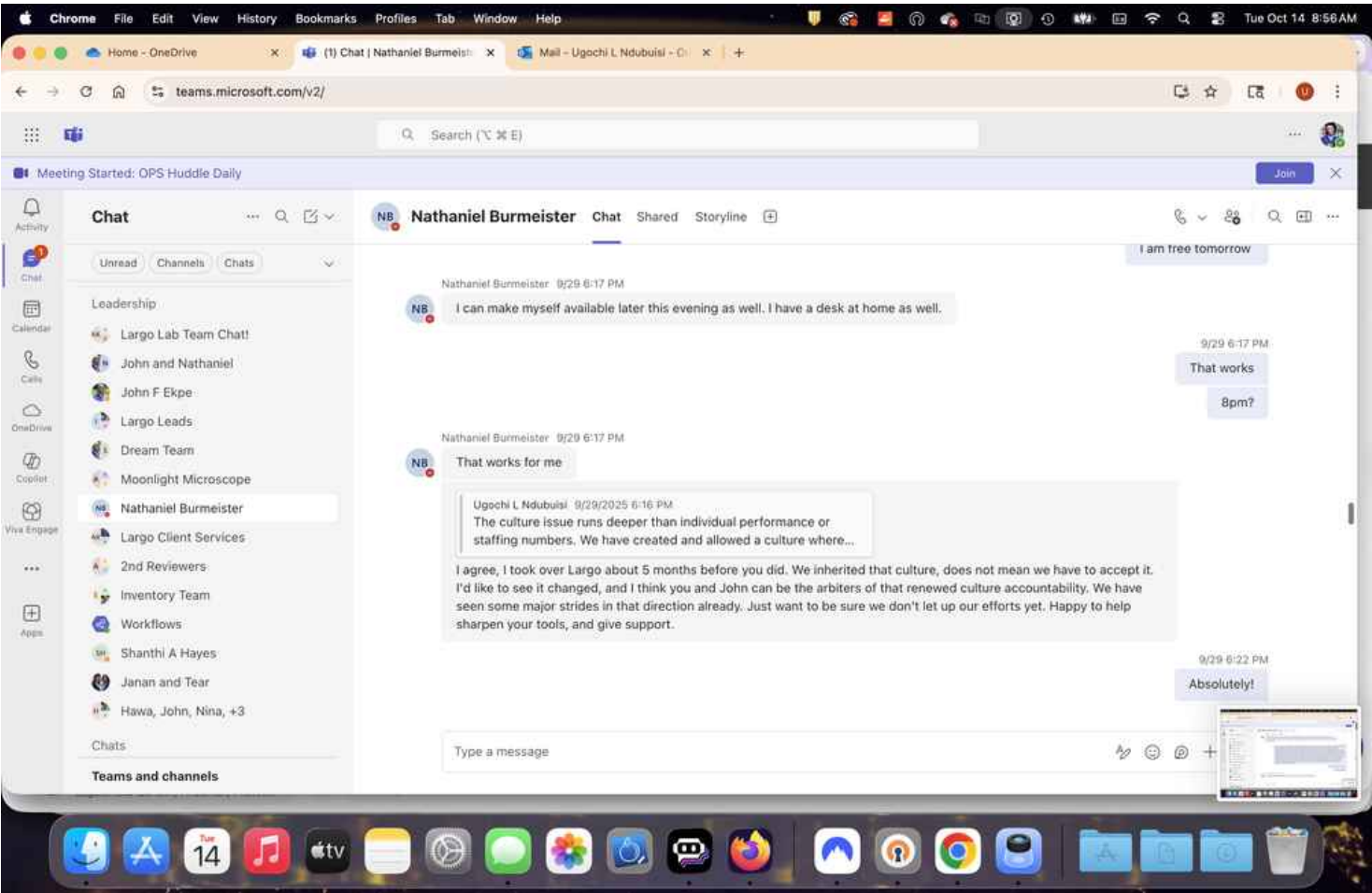


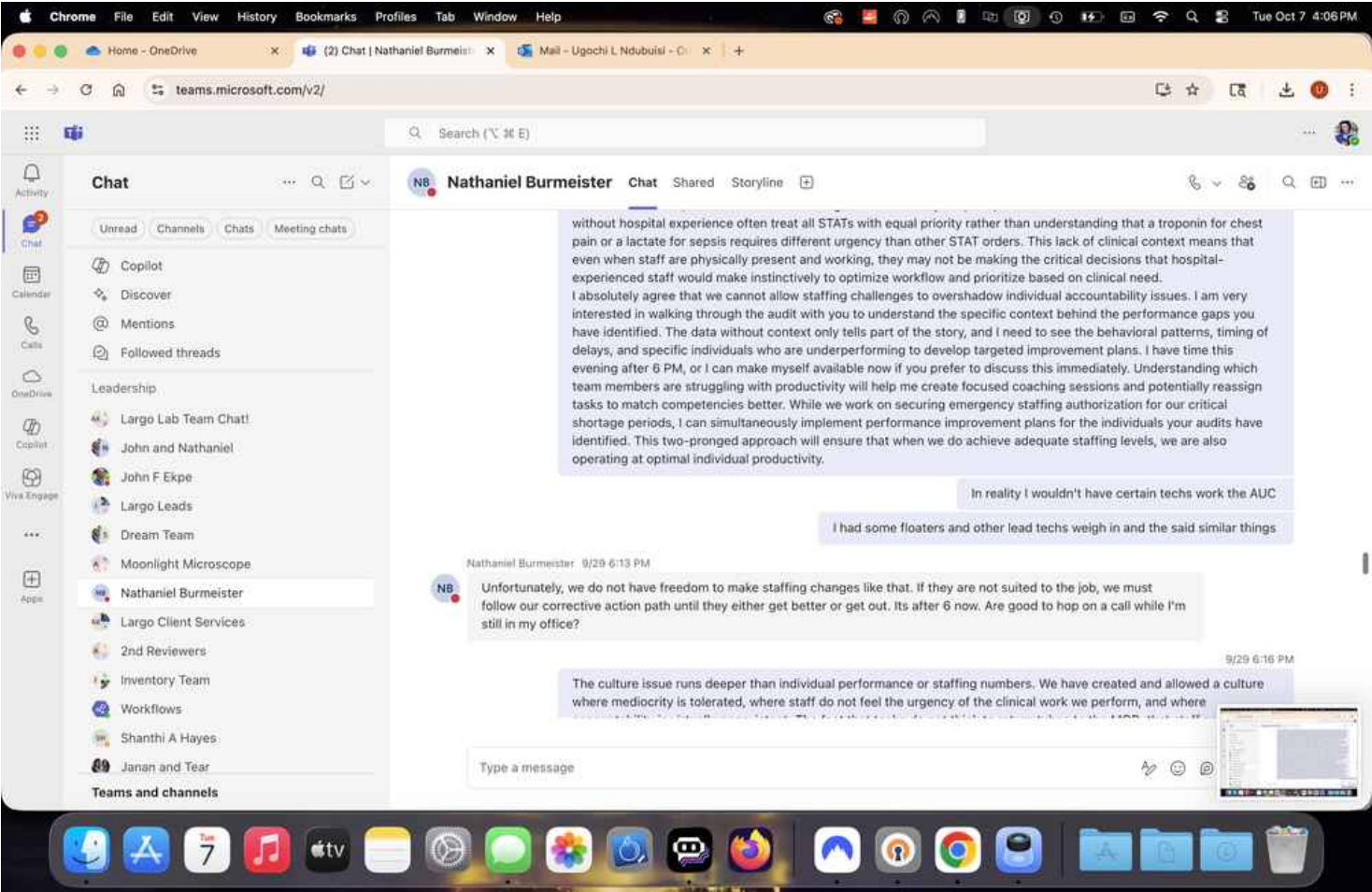




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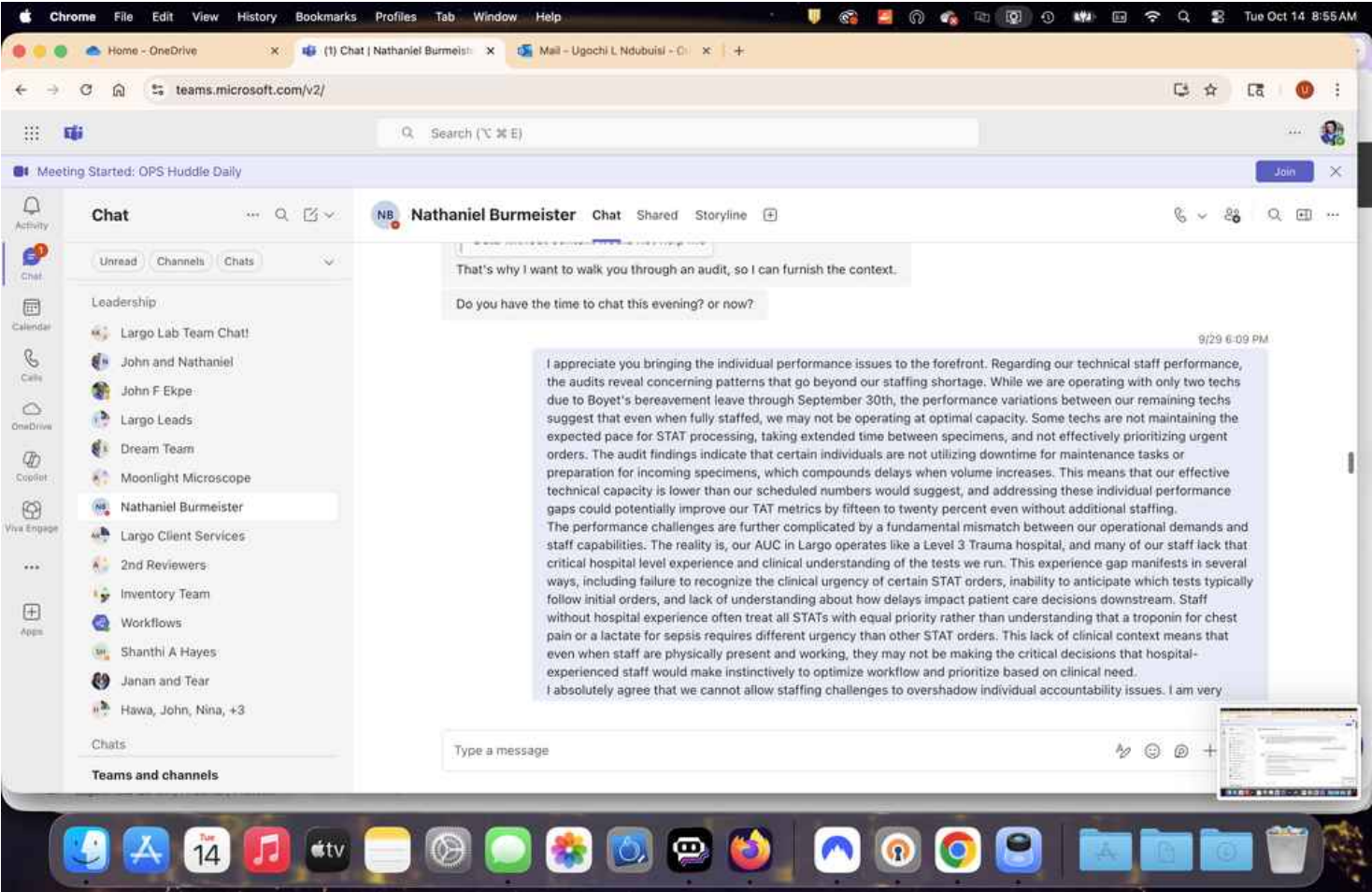




The screenshot shows a Microsoft Teams chat window on a Mac. The browser tabs at the top include 'Home - OneDrive', '(2) Chat | Nathaniel Burmeister', and 'Mail - Ugochi L. Ndubuisi'. The Teams interface has a left sidebar with navigation icons for Activity, Chat, Calendar, Calls, OneDrive, Copilot, Viva Engage, and Apps. The main chat area is titled 'Nathaniel Burmeister' and shows a message history. The messages are as follows:

- Message 1 (Nathaniel Burmeister):** A detailed staffing report for tomorrow. It mentions that 2-3:30pm and Brooks join from 3am to 3:30pm, but their shifts don't align with peak demand periods. It notes a positive half to one FTE overage with Blackwell Lorraine and Bahre Robel, but the MT section maintains adequate to slightly over coverage. It also mentions a negative five phlebotomist shortage coinciding with the morning shift phlebotomists Bolden-Davis and Brooks leaving. The report concludes with a need to extend morning phlebotomists past 2:30pm or bring in additional coverage for the 10am to 4pm timeframe to prevent service failure.
- Message 2 (Nathaniel Burmeister):** 'How much do your phlebotomist affect the STAT work? From my understanding most if not all of that process is driven by the techs alone'.
- Message 3 (Unread):** 'Yes techs are the ones running STAT tests but but this perspective overlooks the critical role phlebotomists play in the entire STAT workflow. The STAT turnaround time clock begins when the order is placed, not when the specimen arrives in the lab for testing. This distinction is crucial because our data reveals that the primary bottleneck in STAT TAT'.

The bottom of the screen shows a Mac dock with various application icons including Safari, App Store, Calendar, Music, Apple TV, Photos, Messages, Photos, Firefox, and others.



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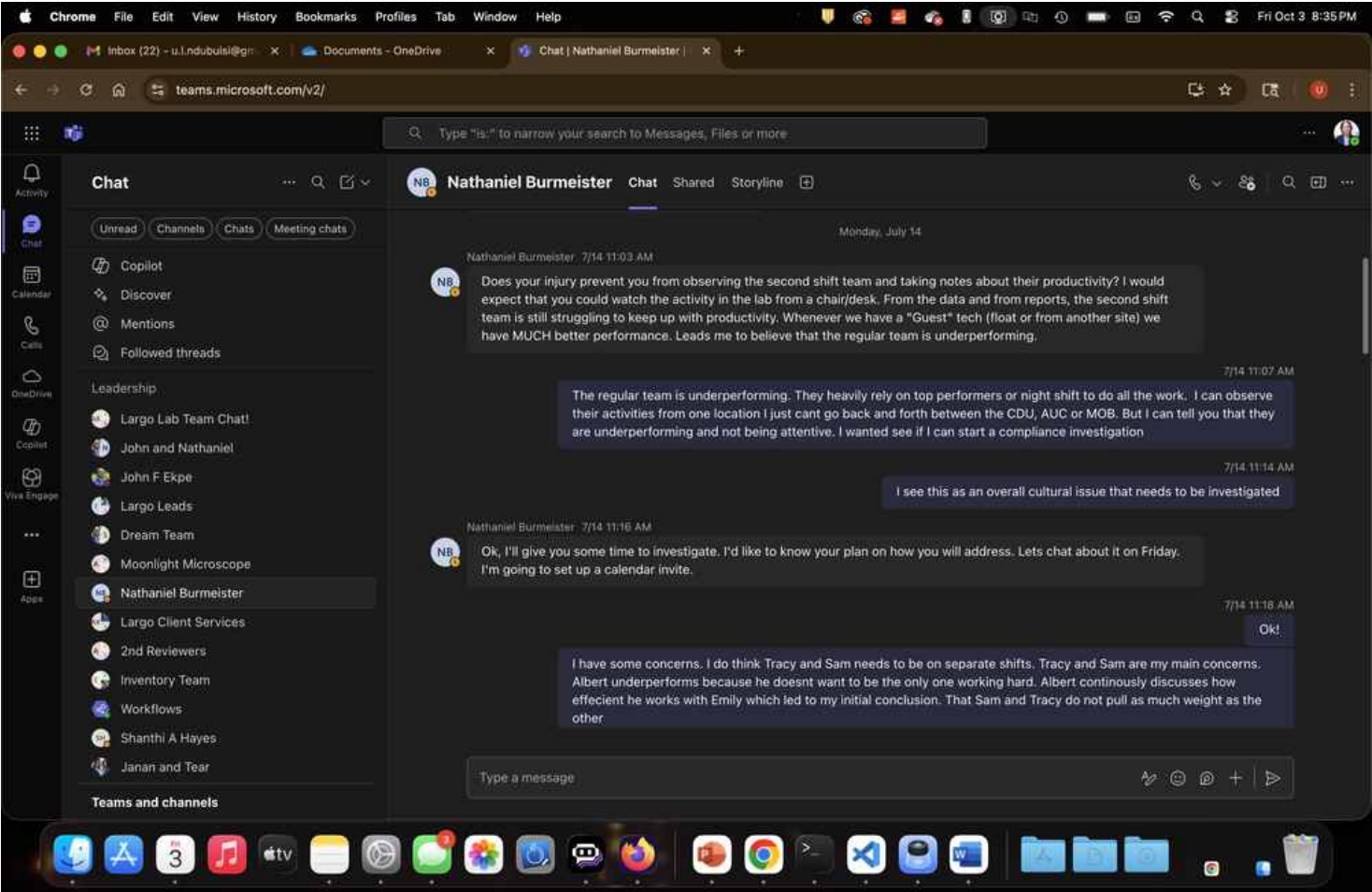
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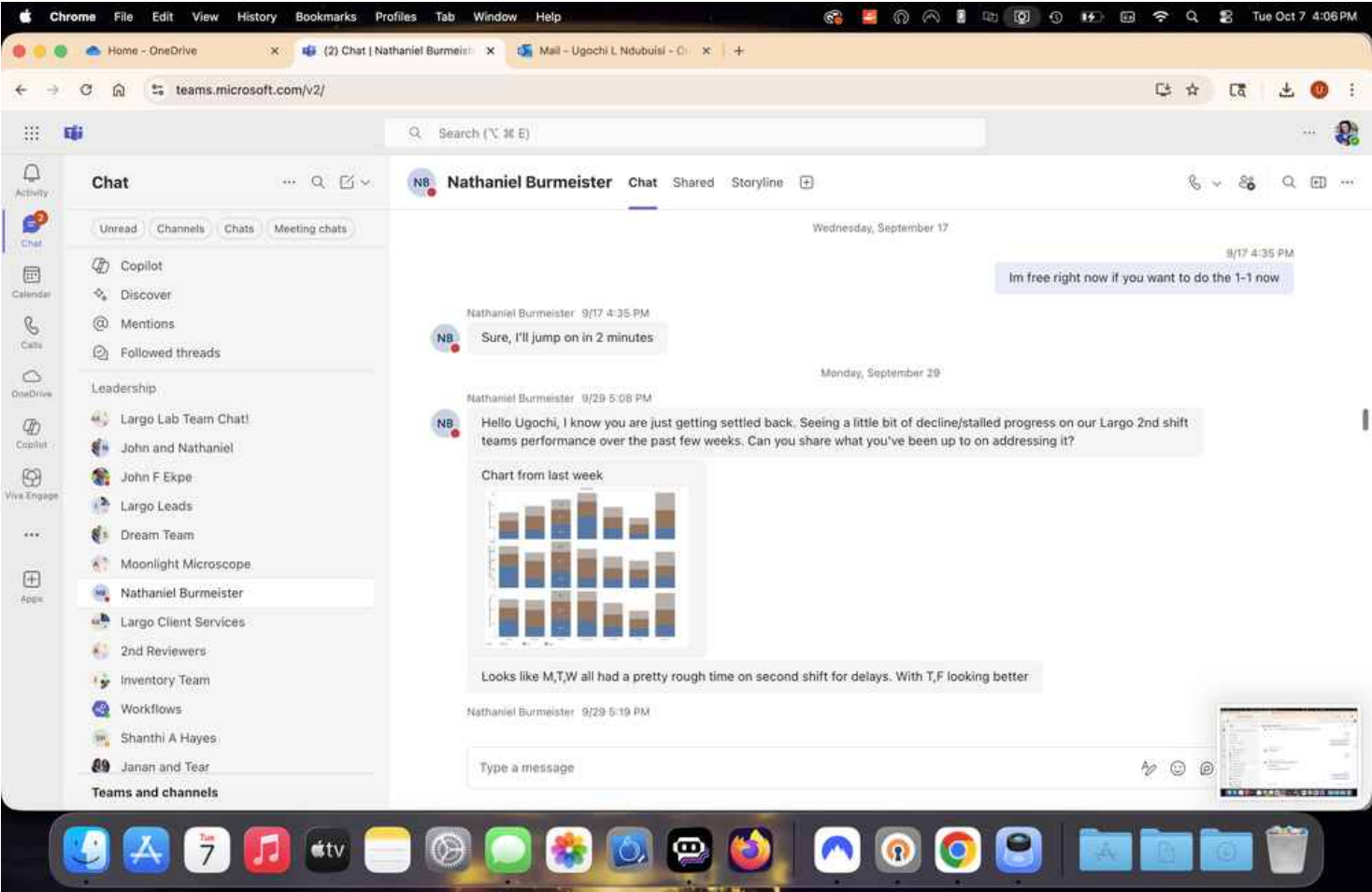
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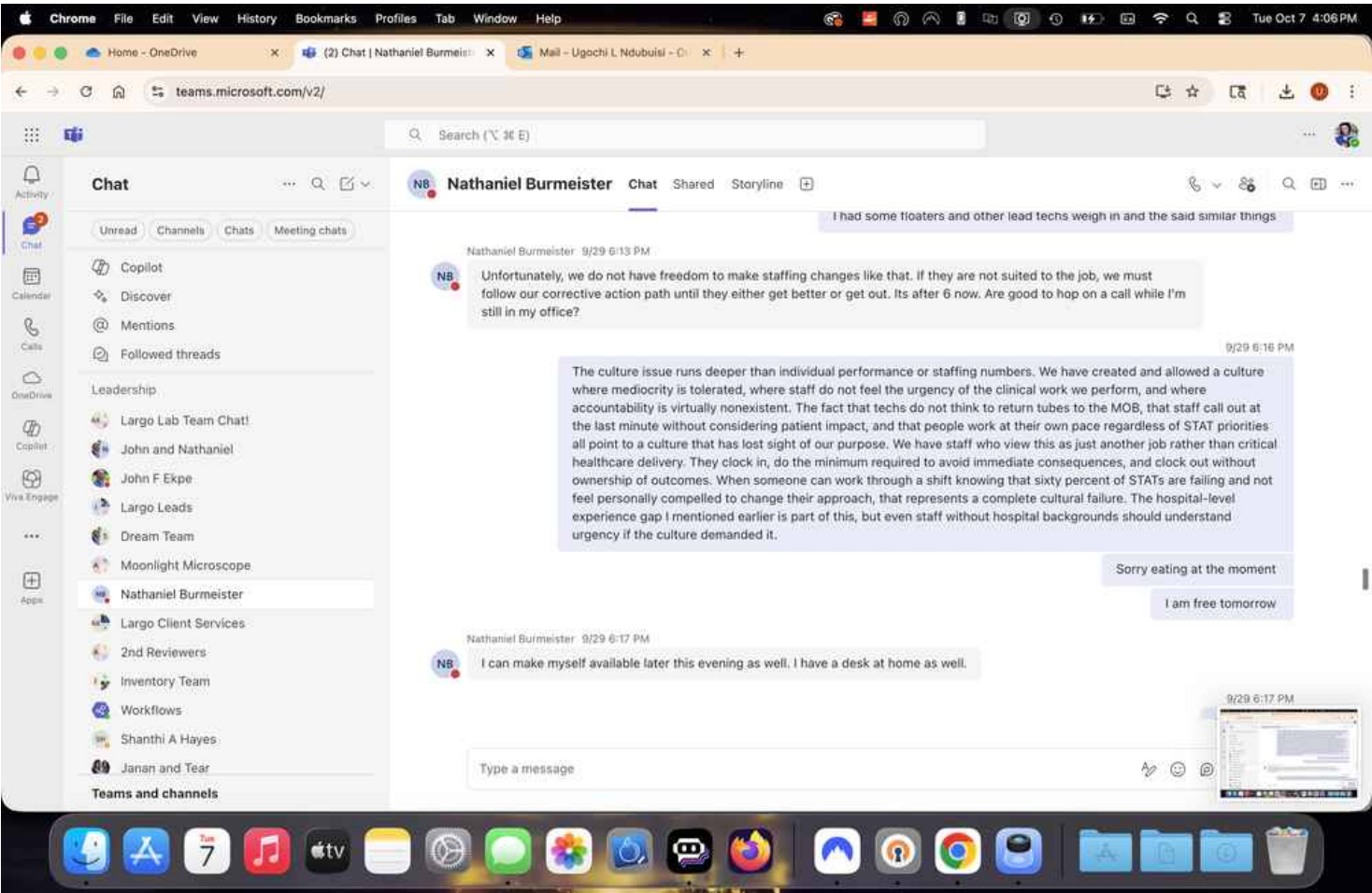
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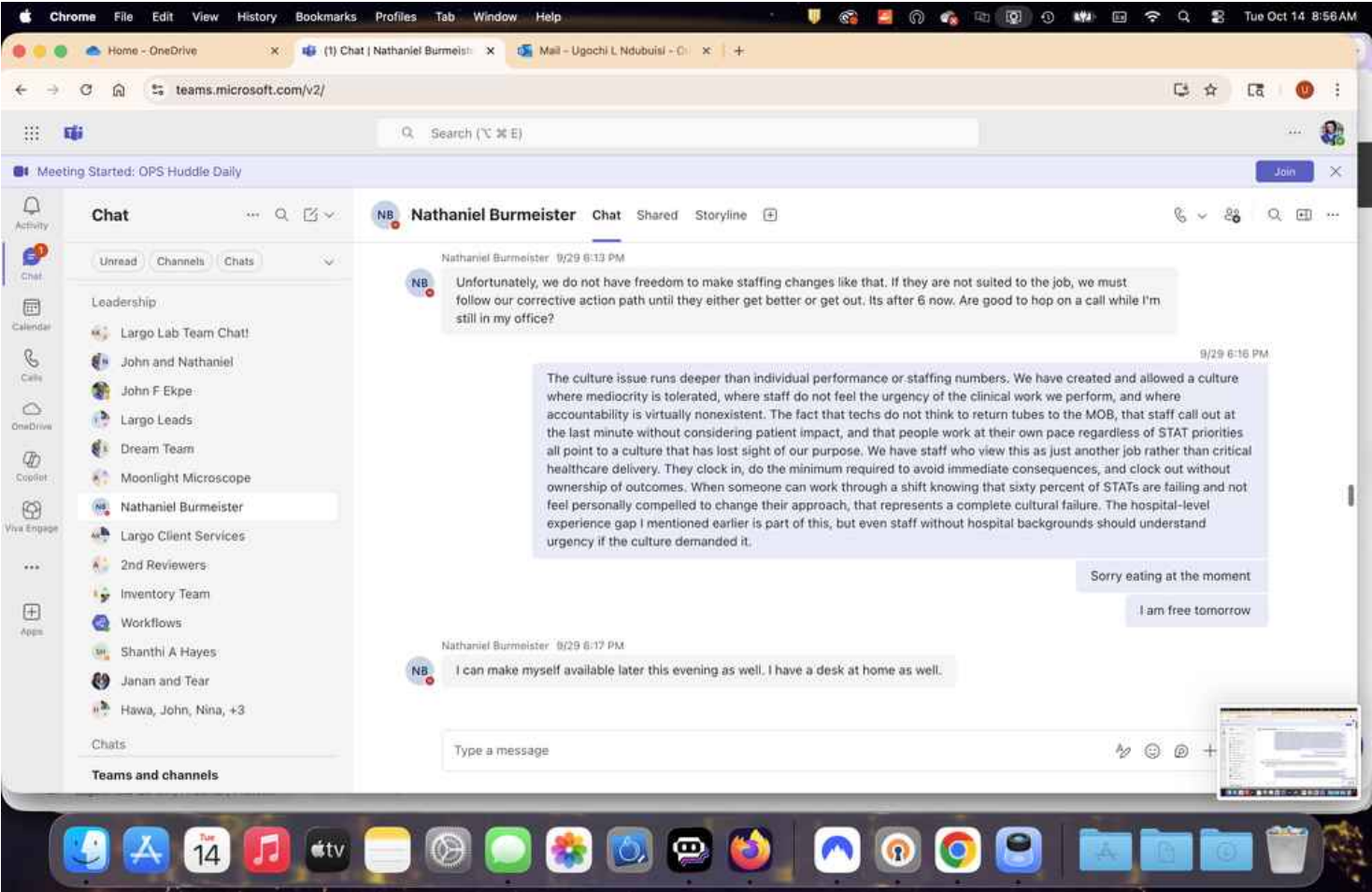
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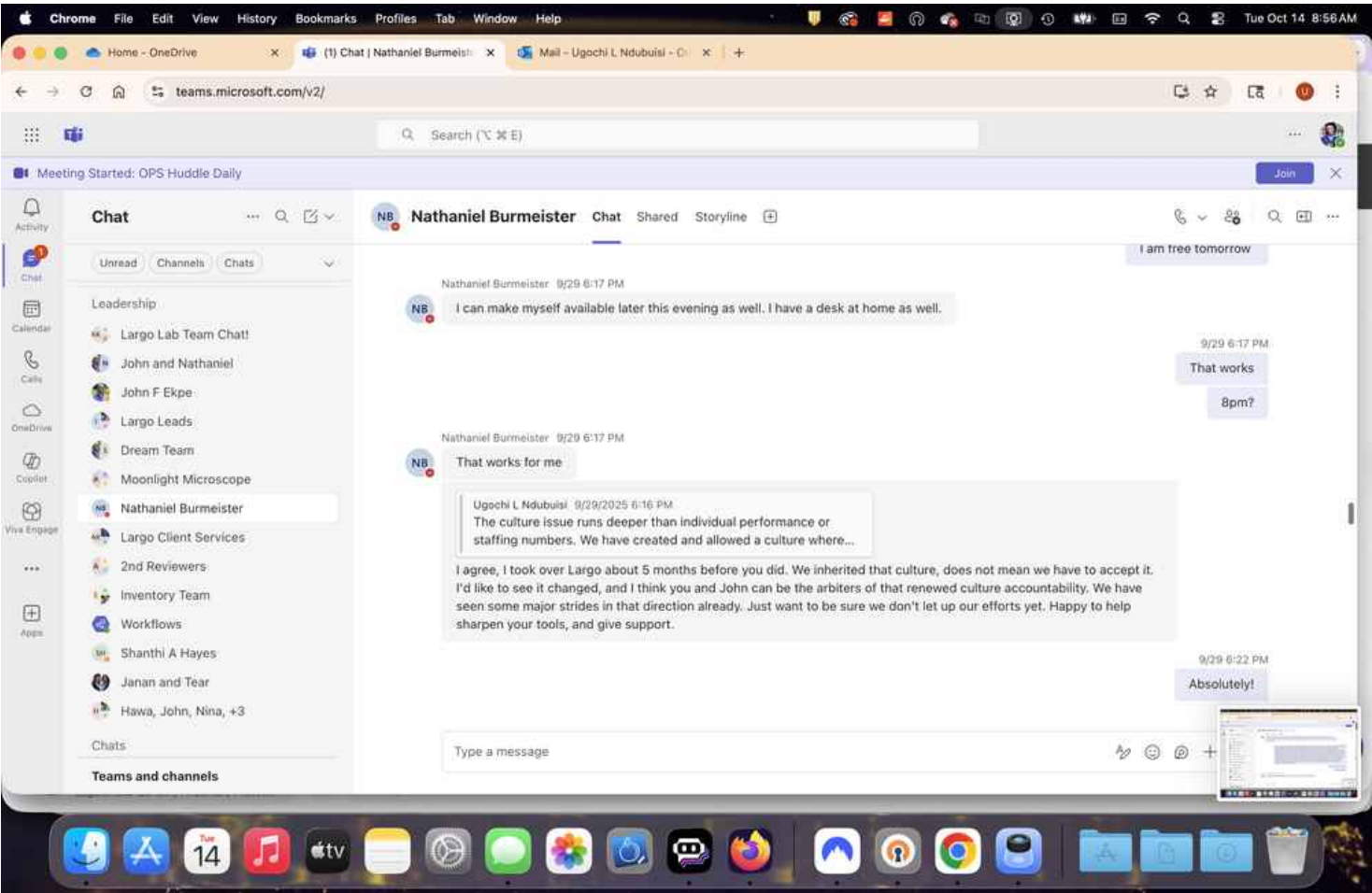
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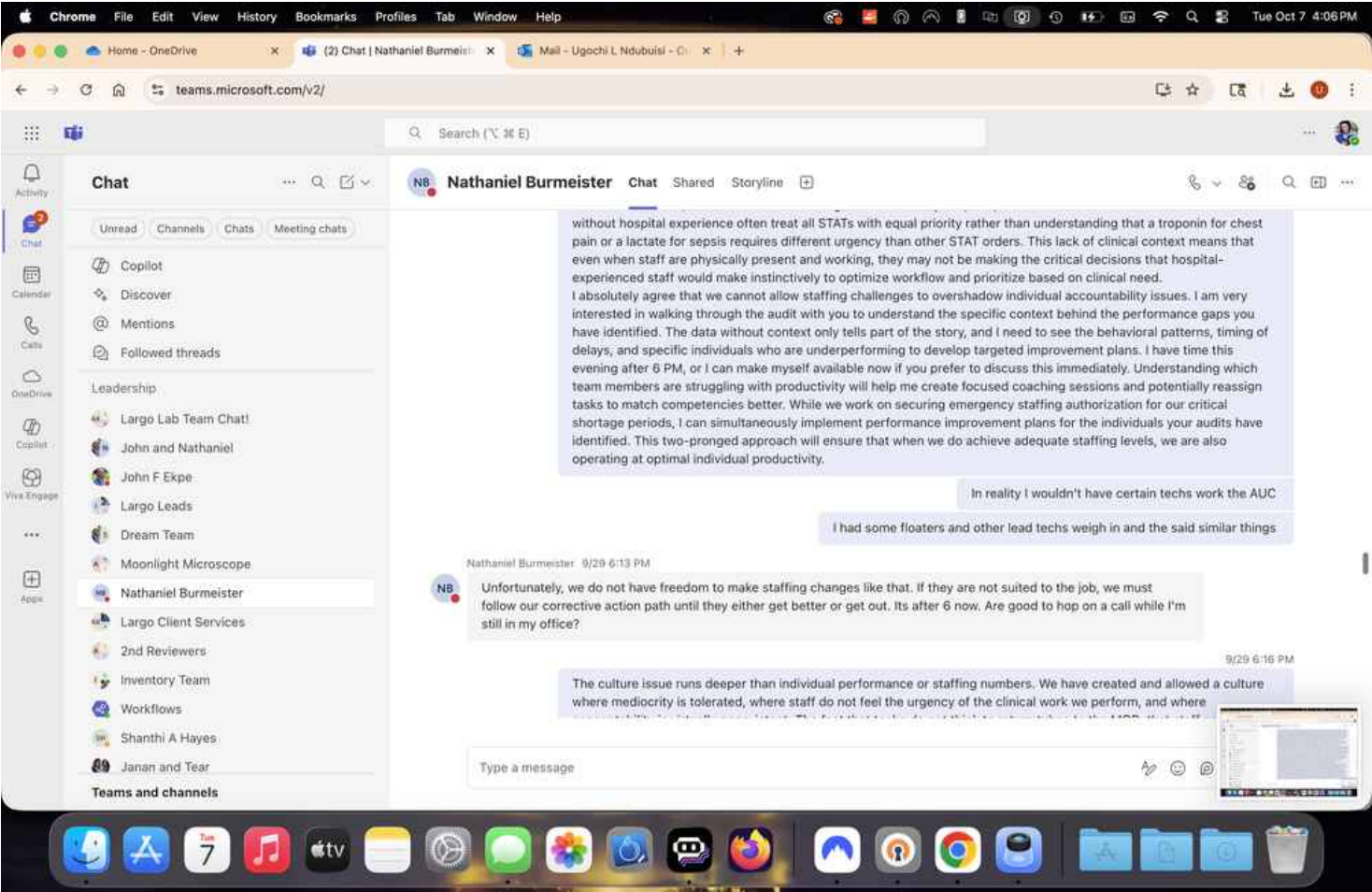


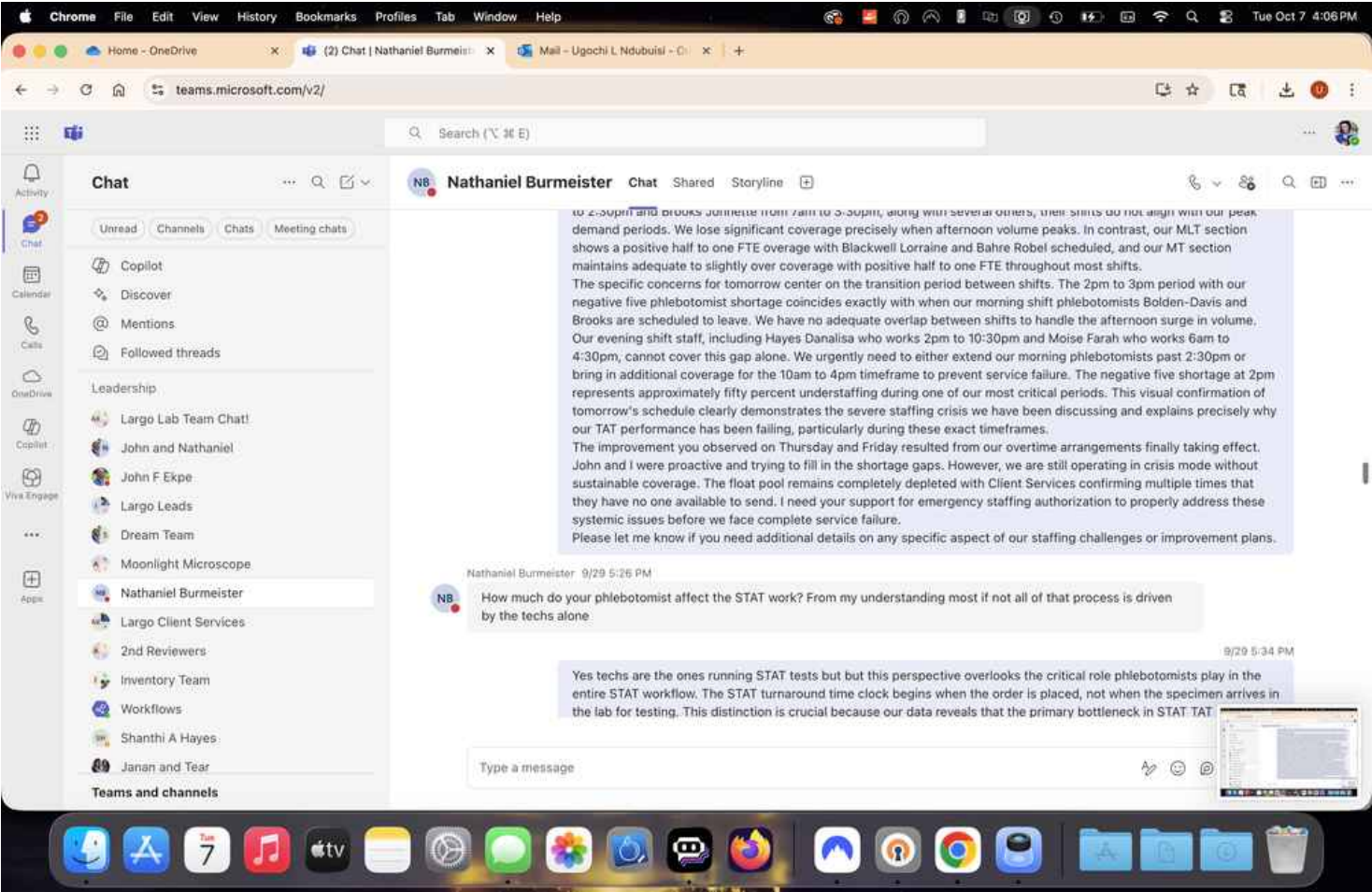


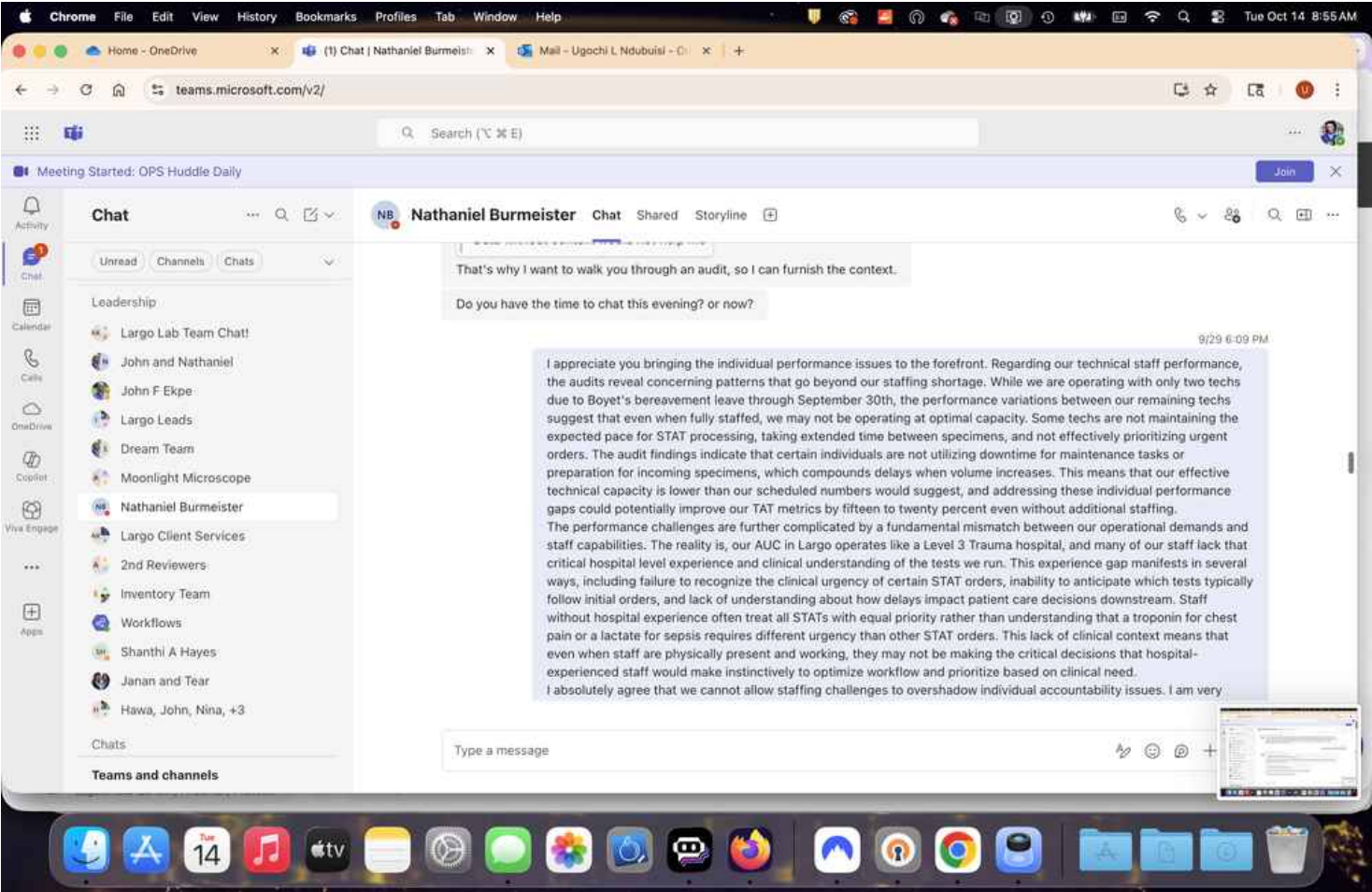


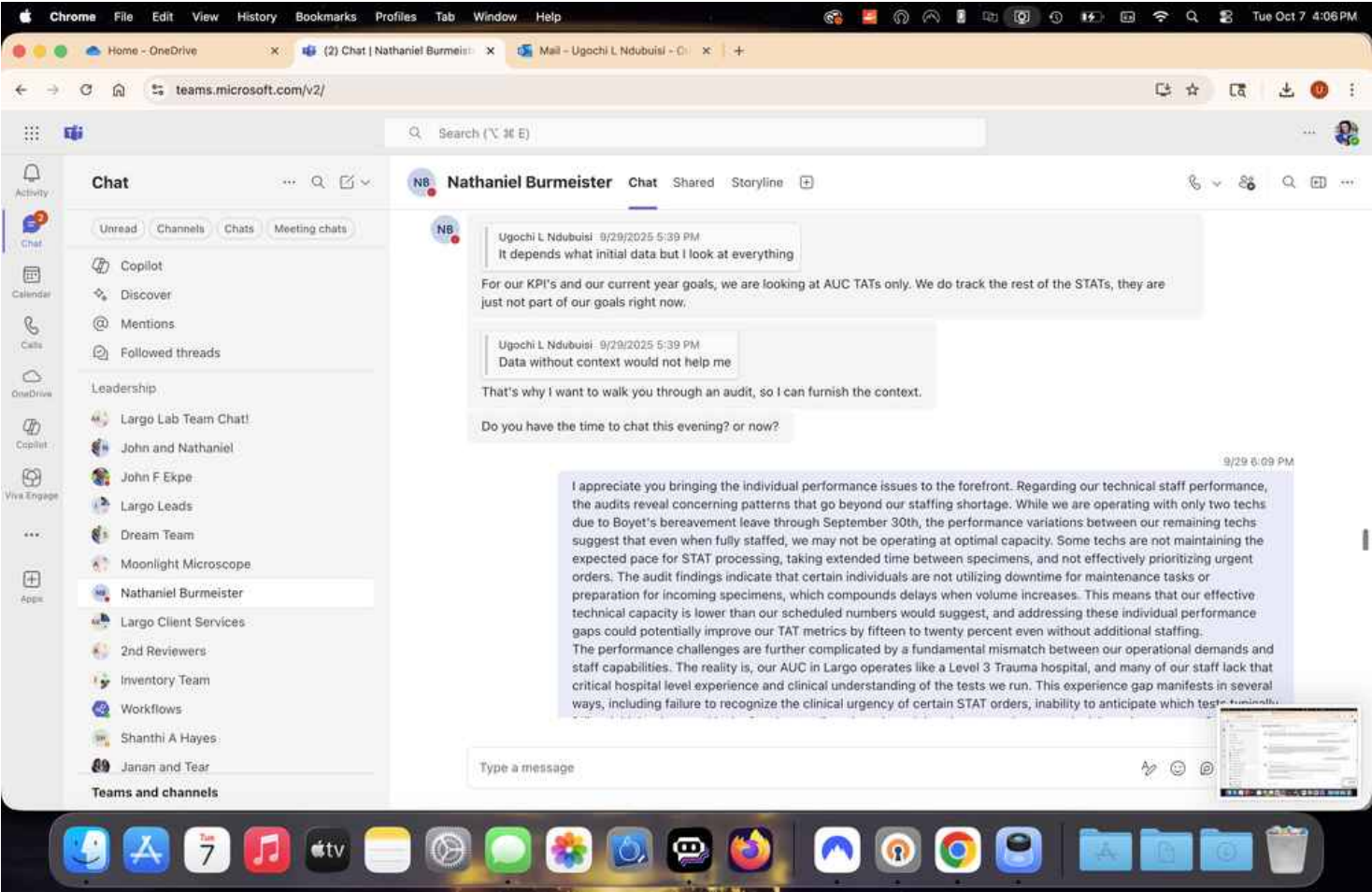












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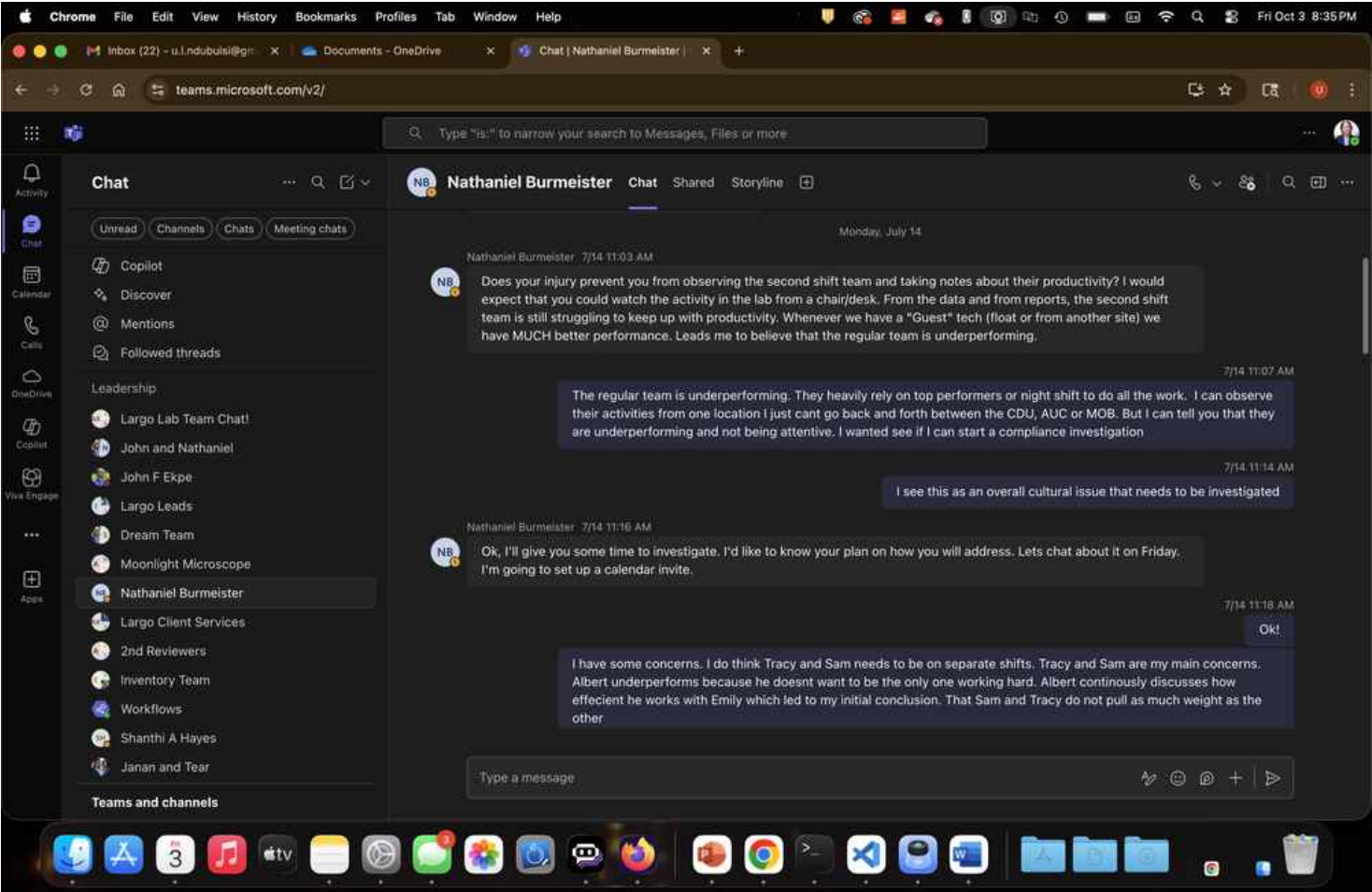
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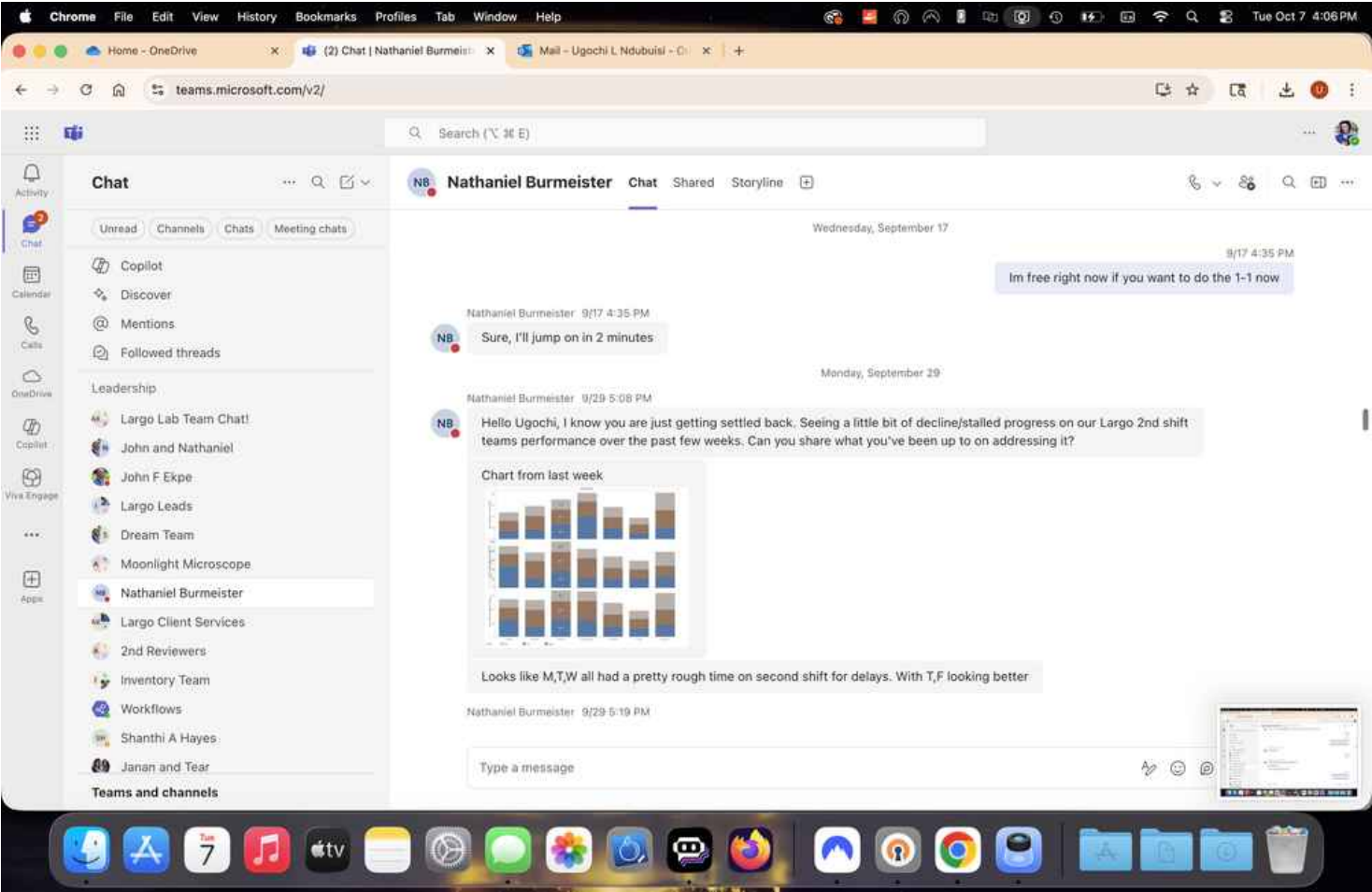
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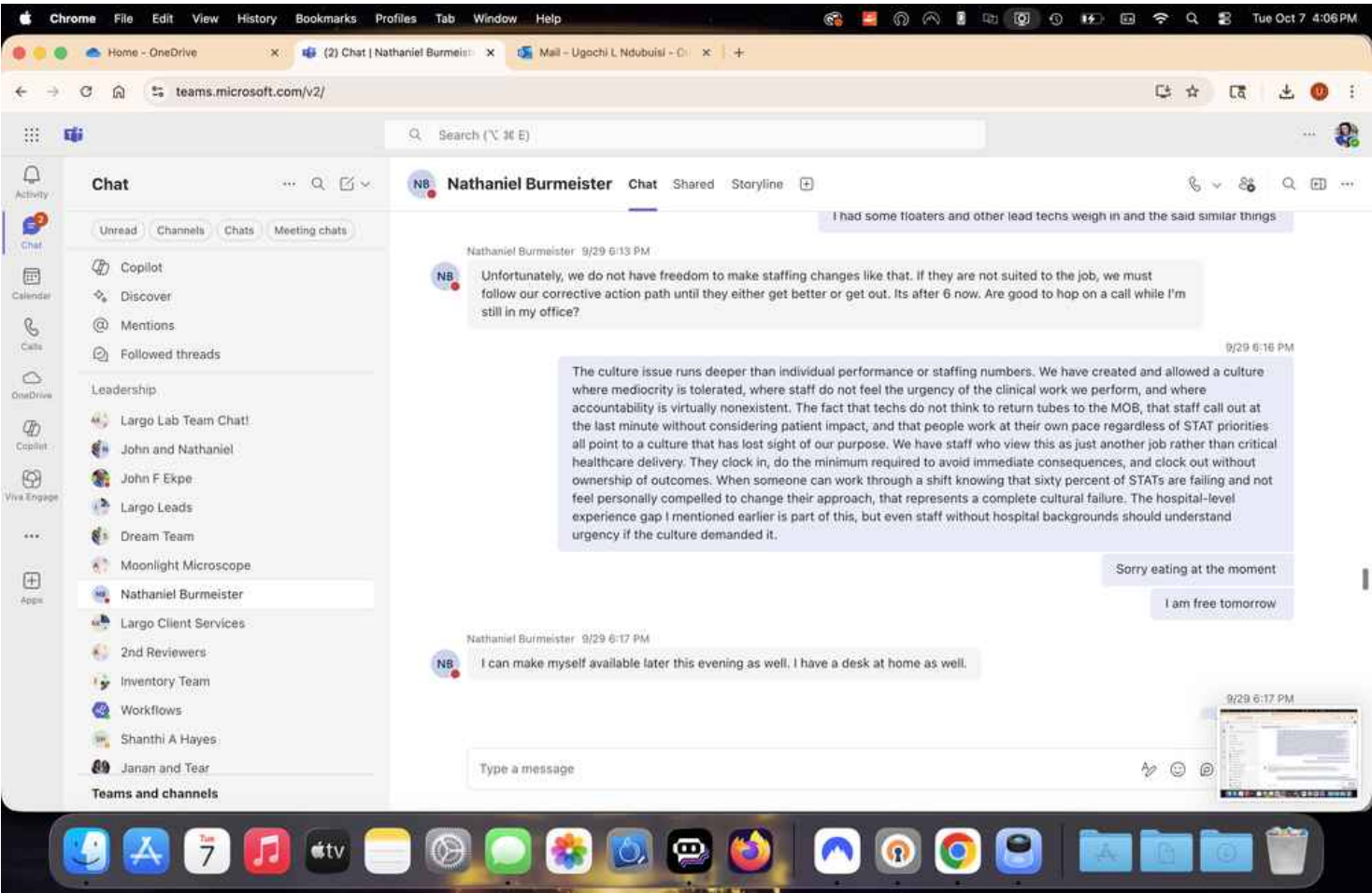
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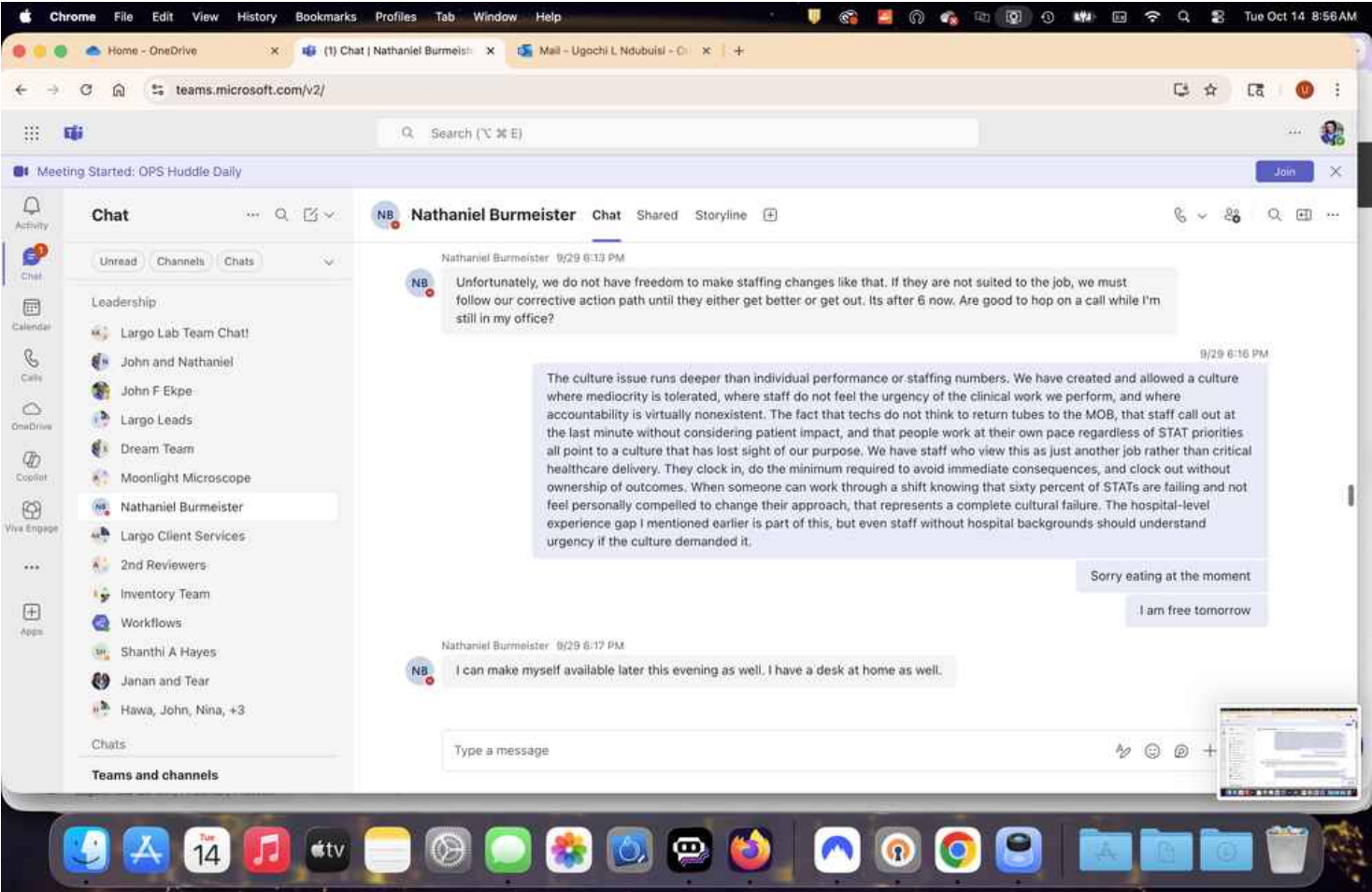
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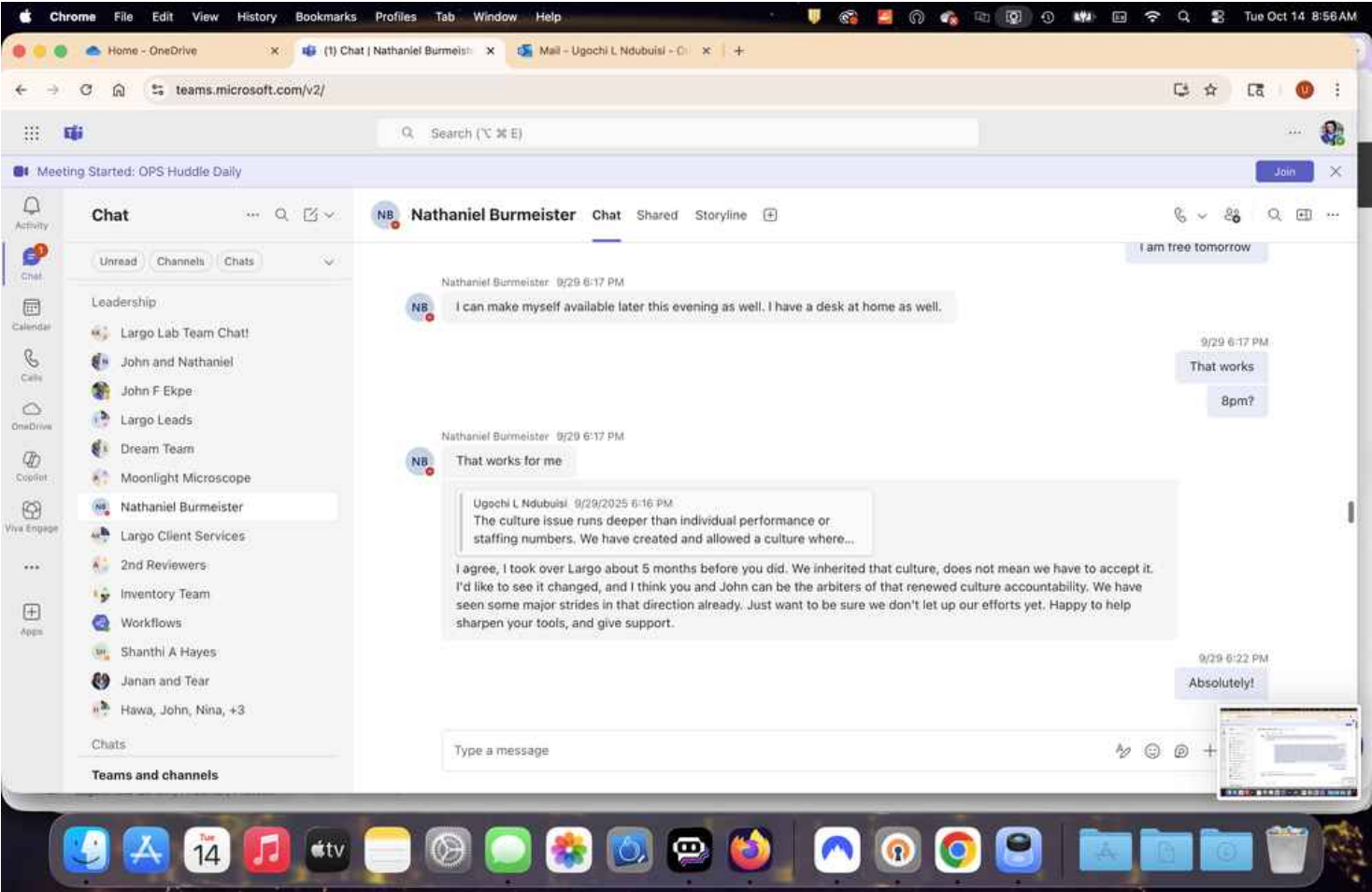
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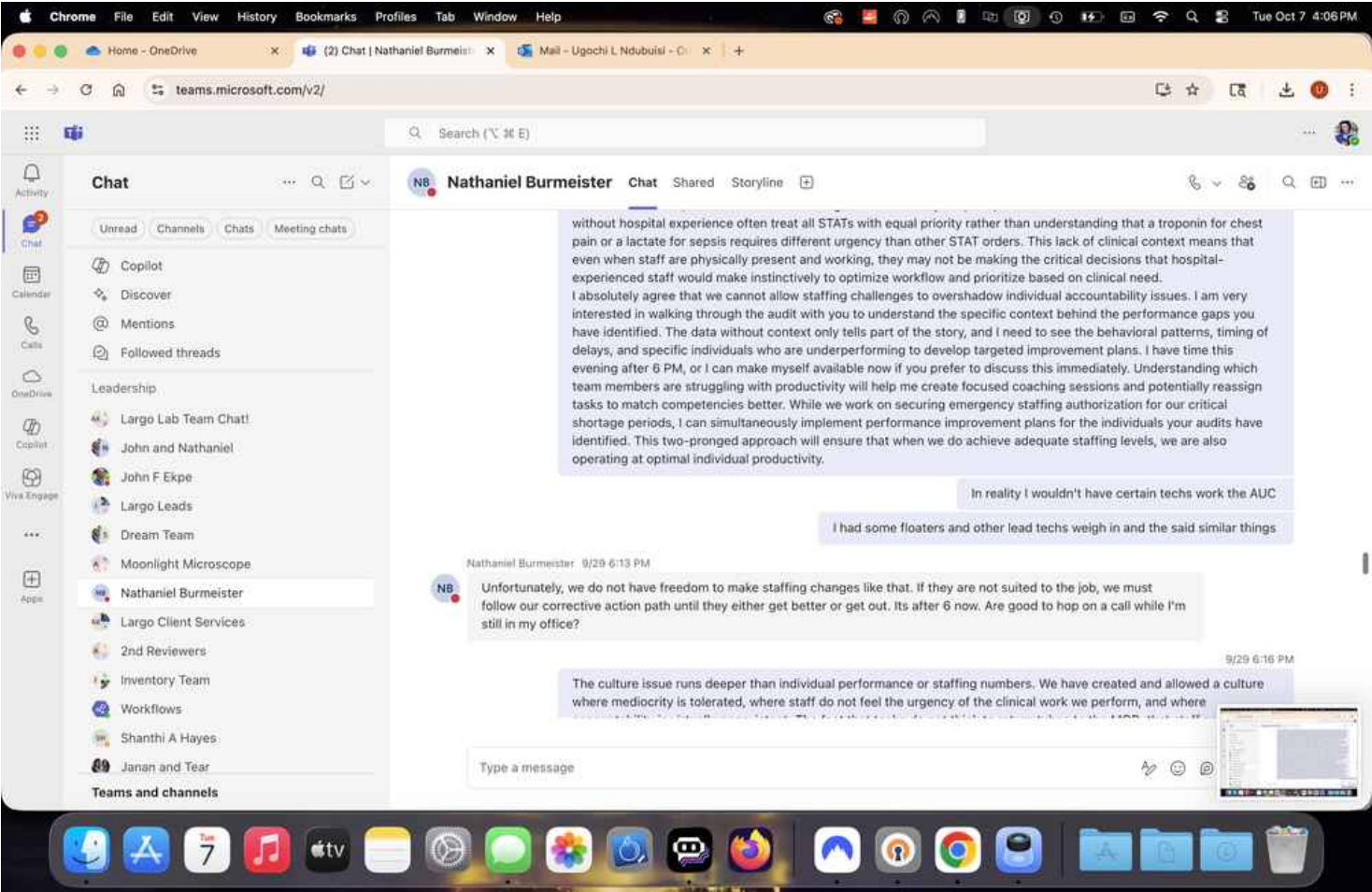


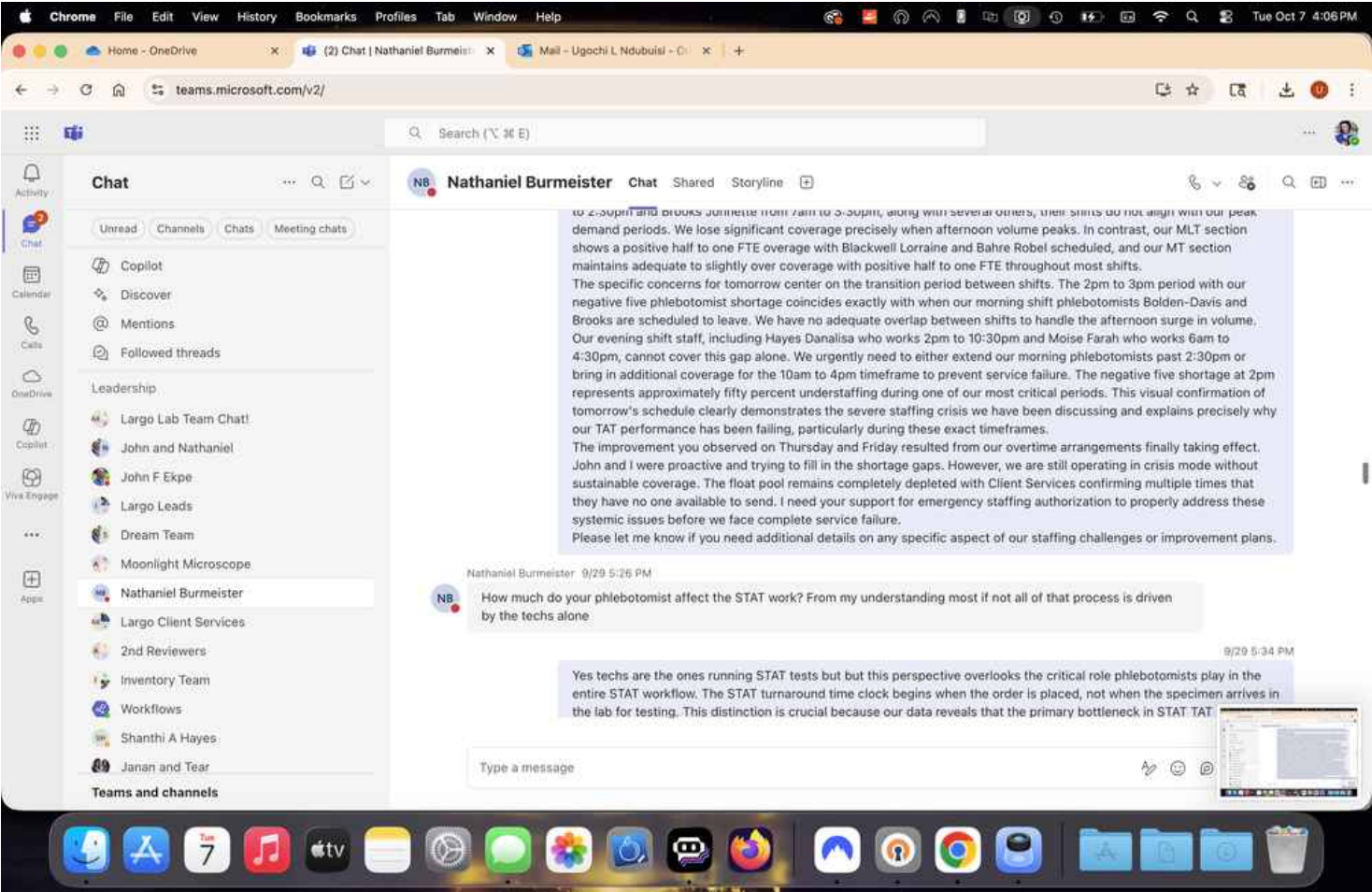


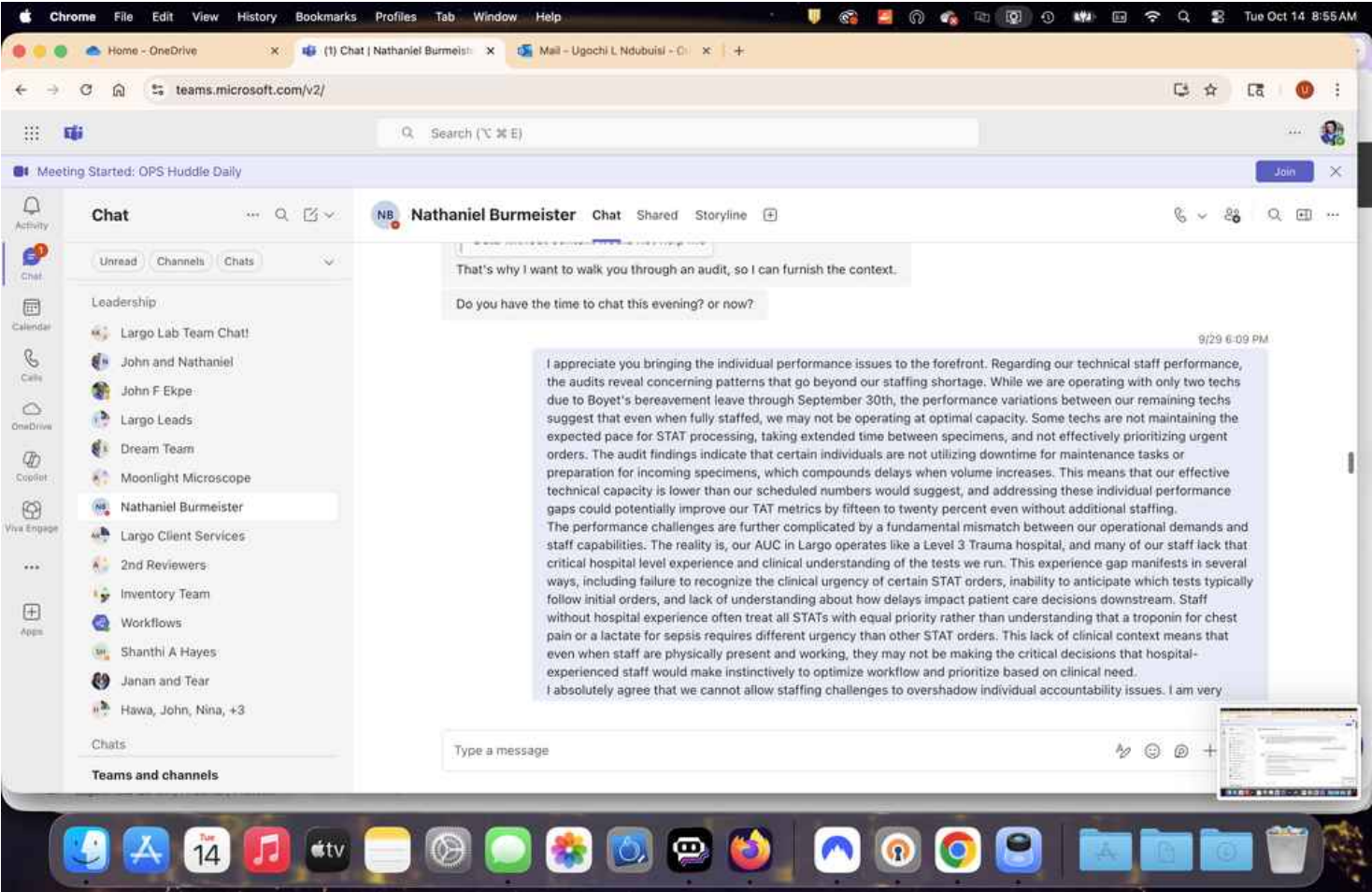


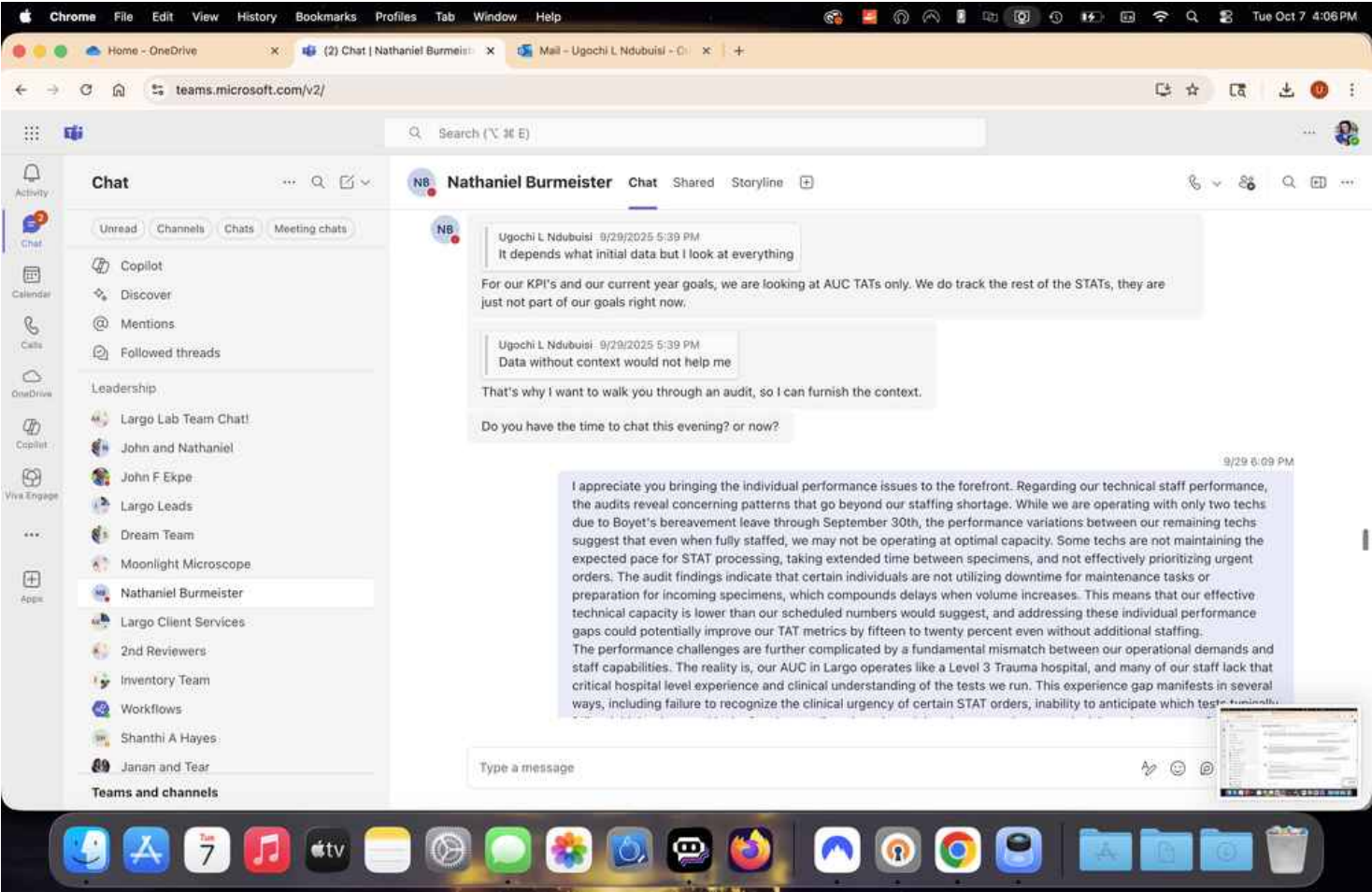


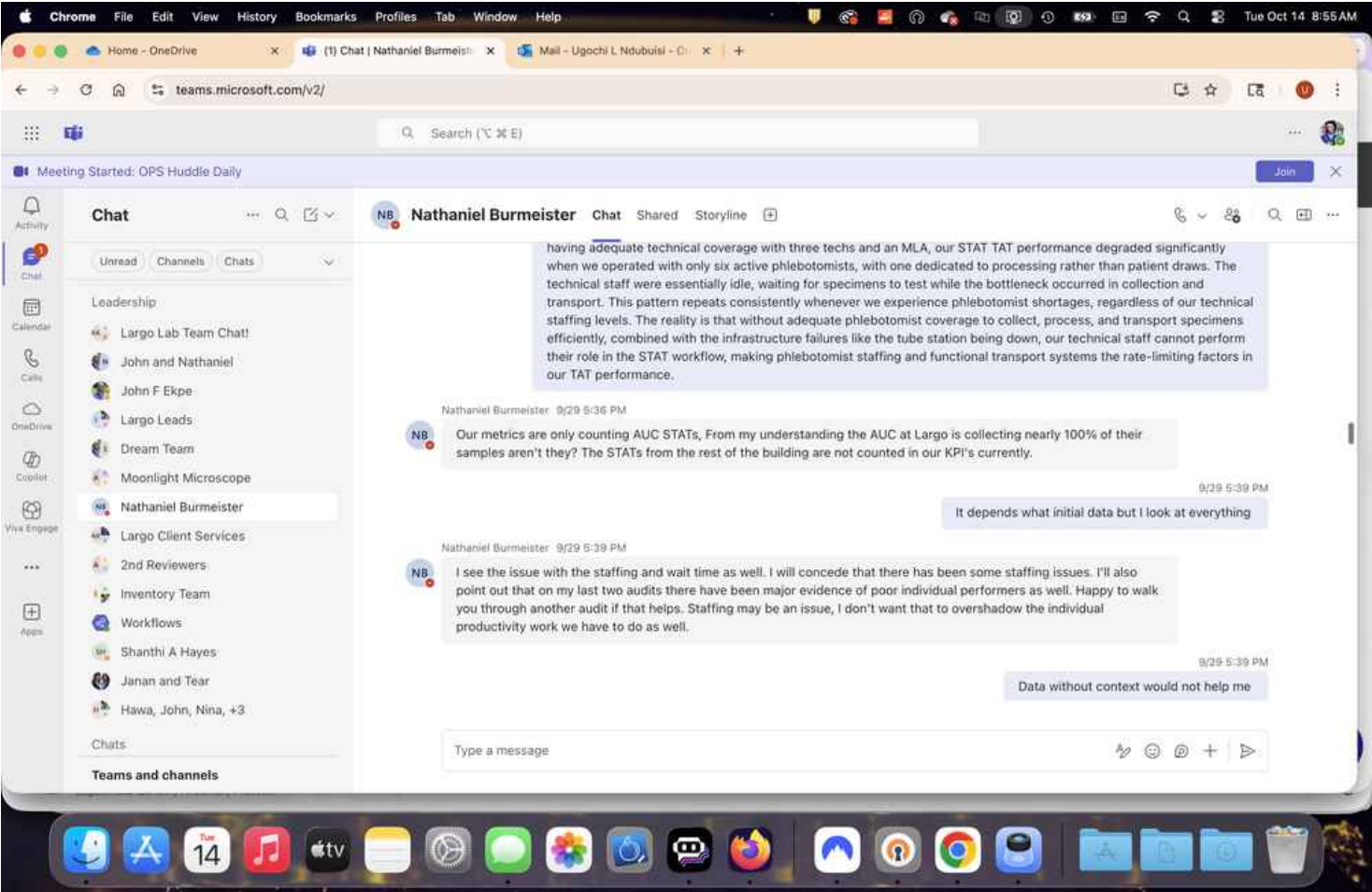












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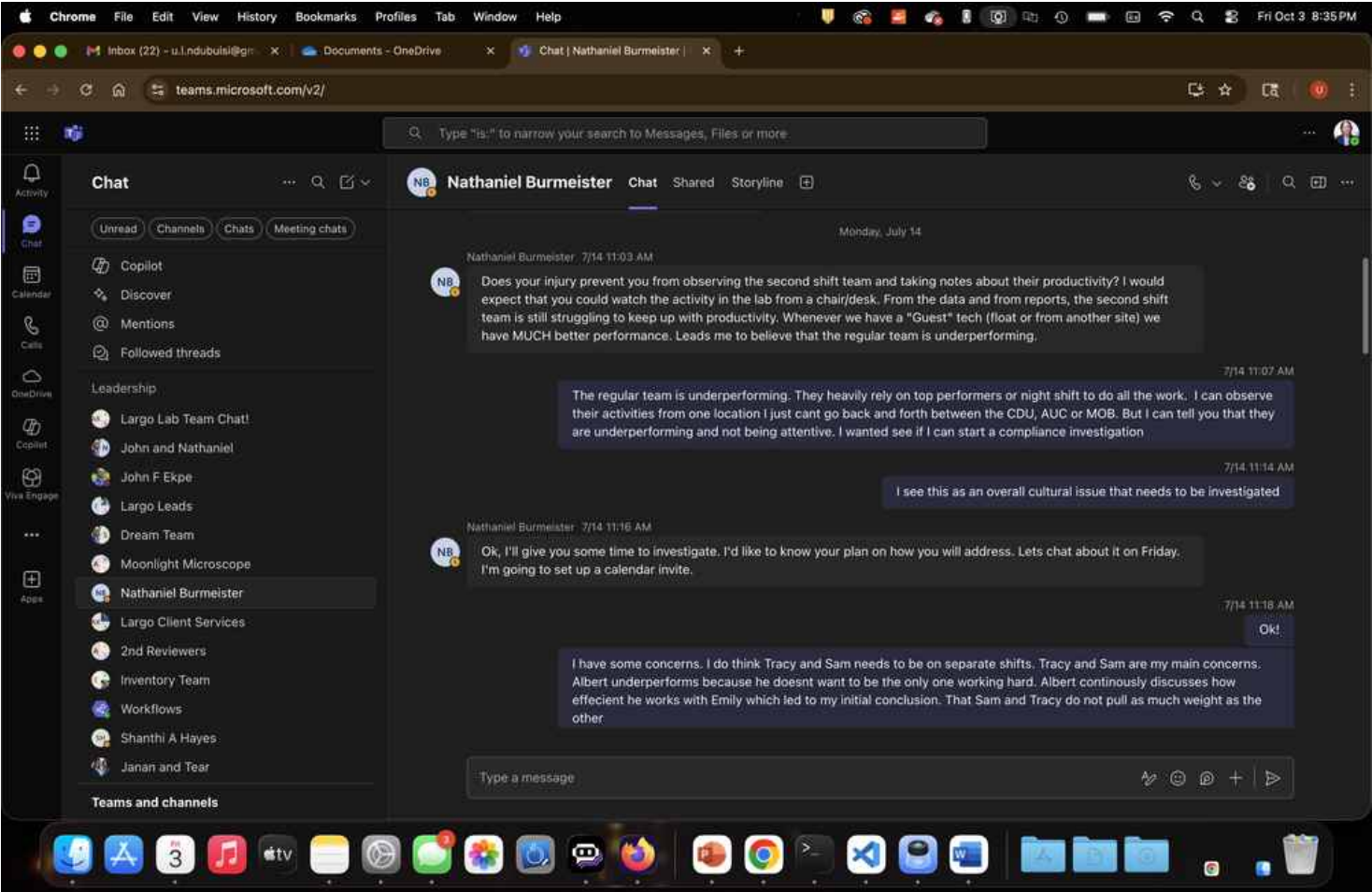
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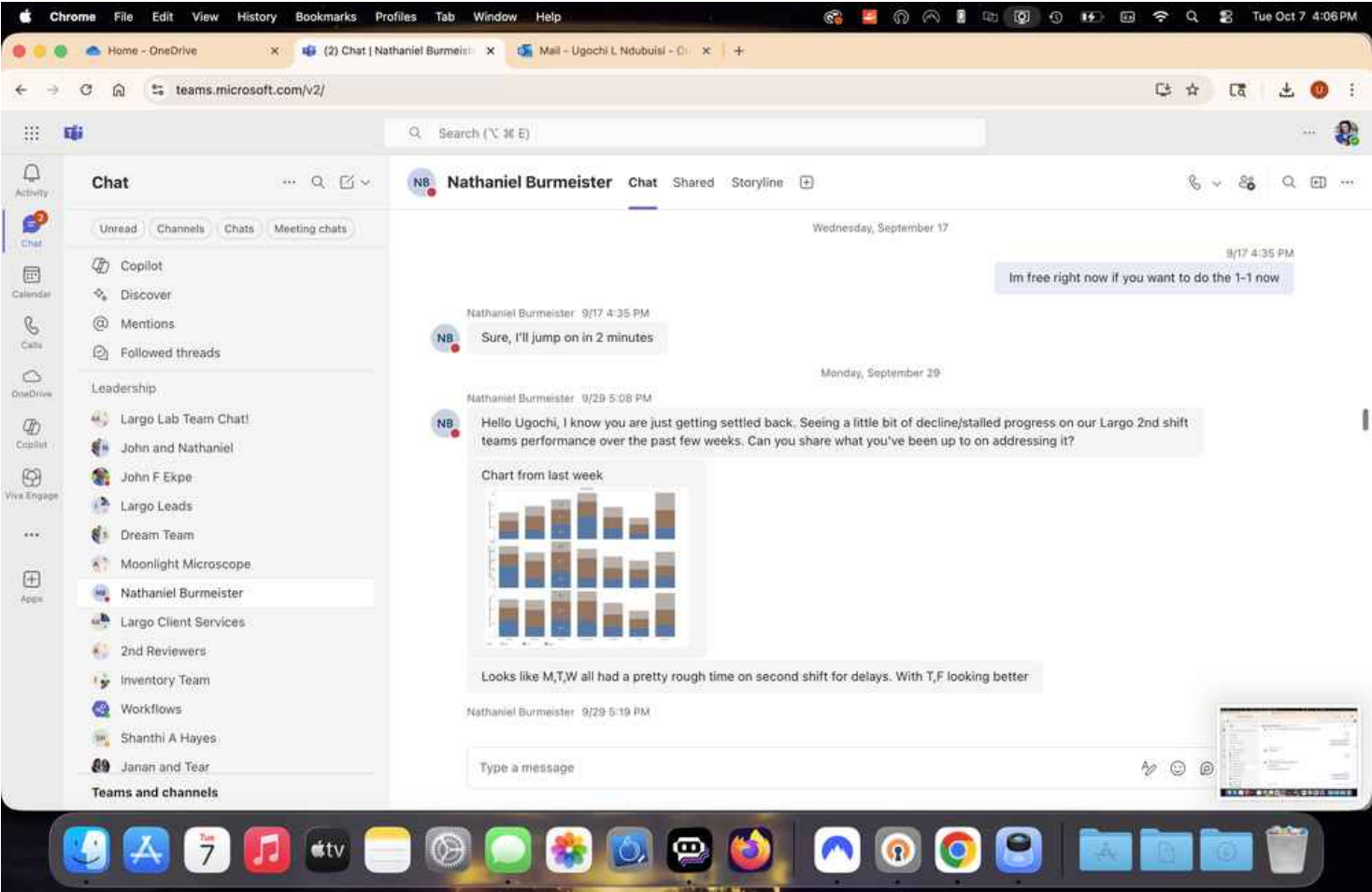
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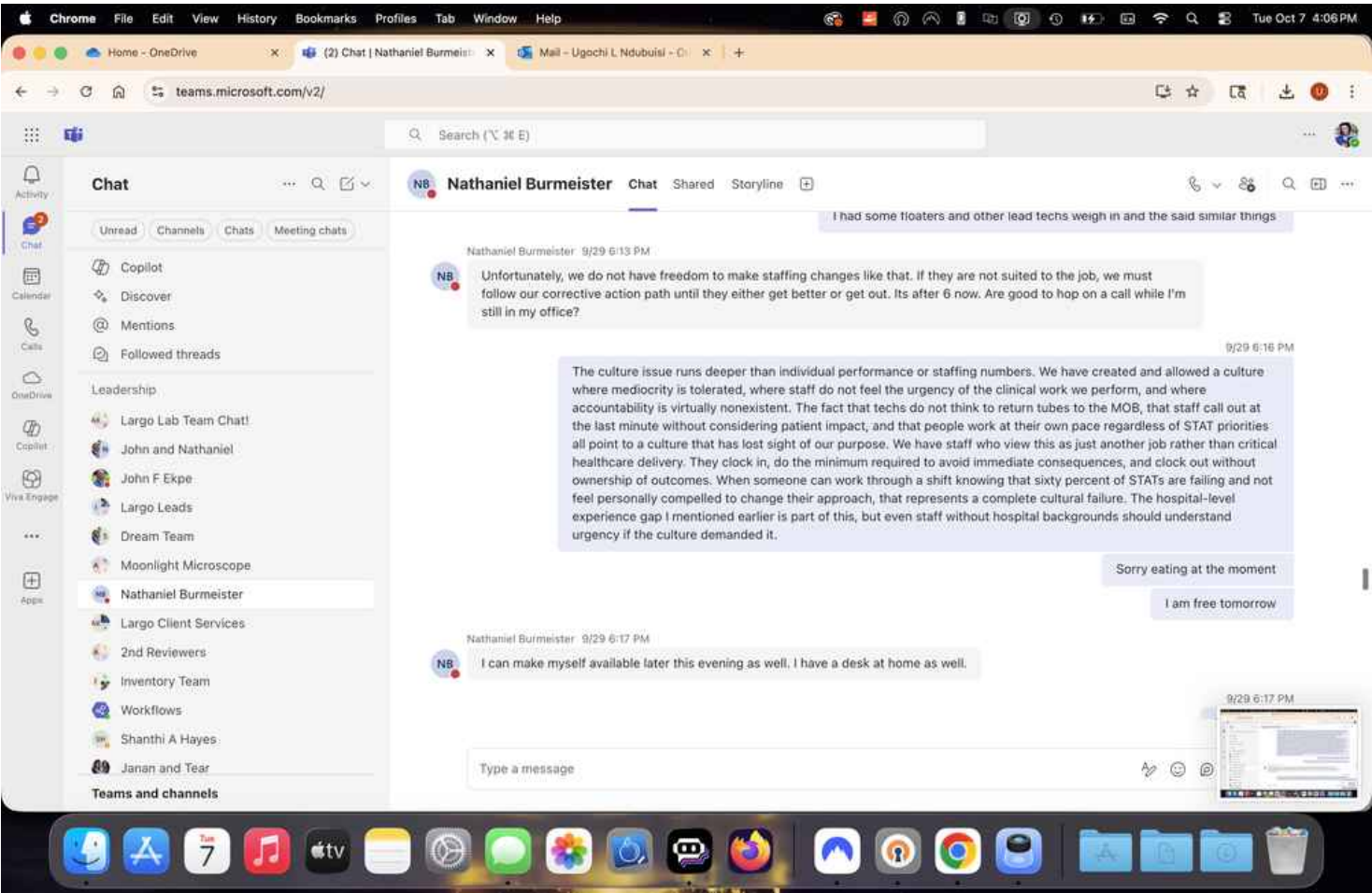
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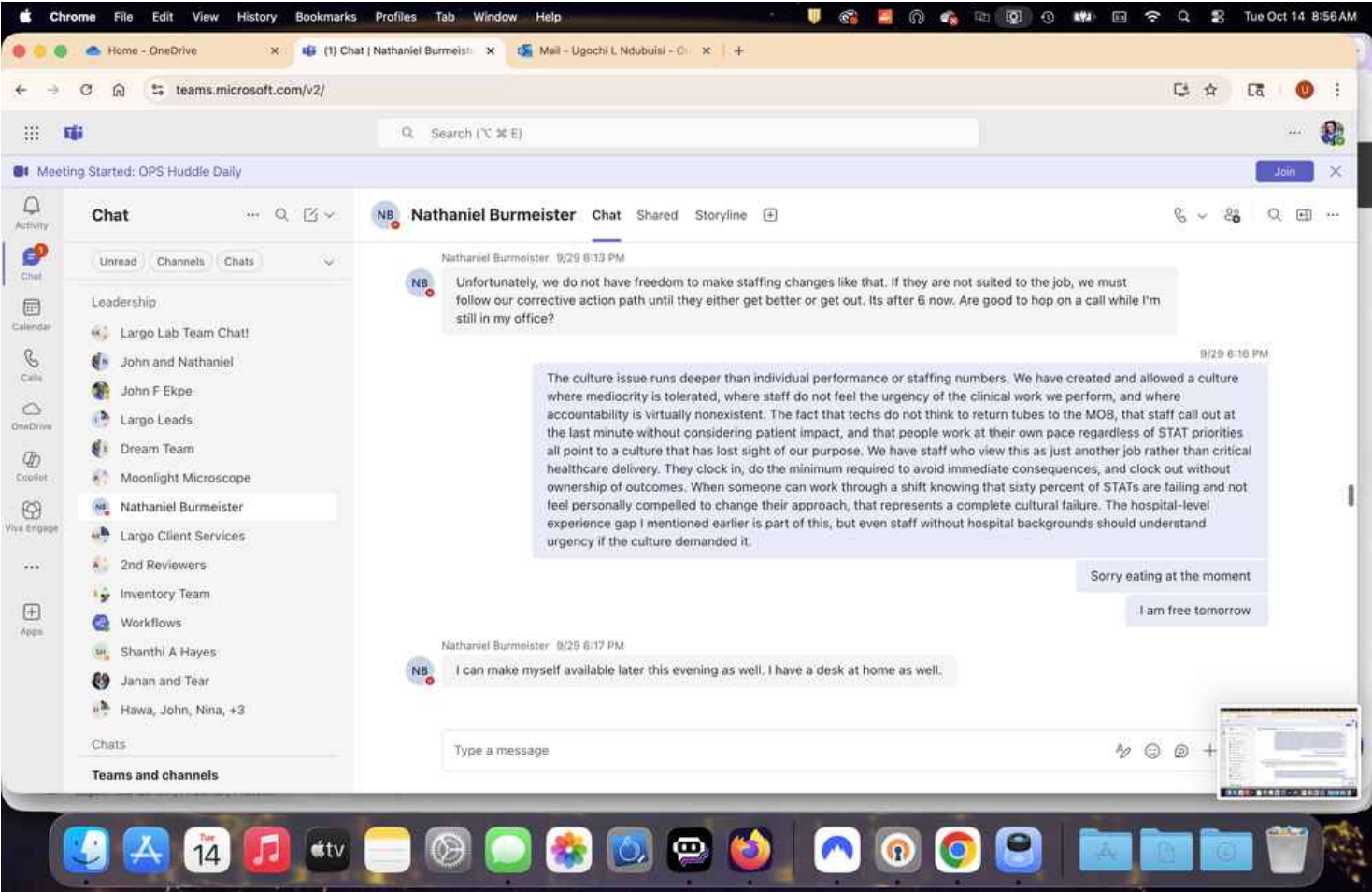
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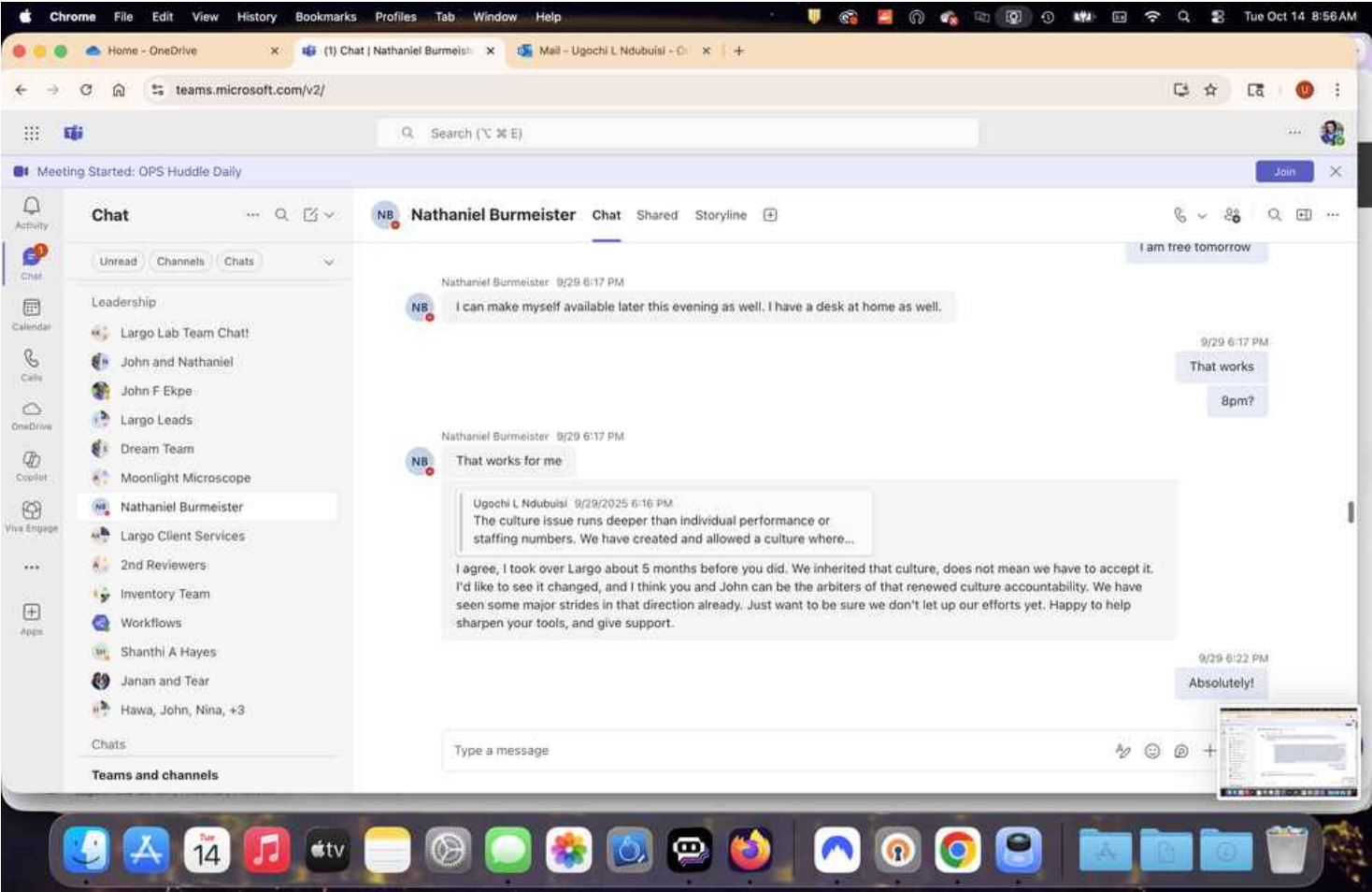


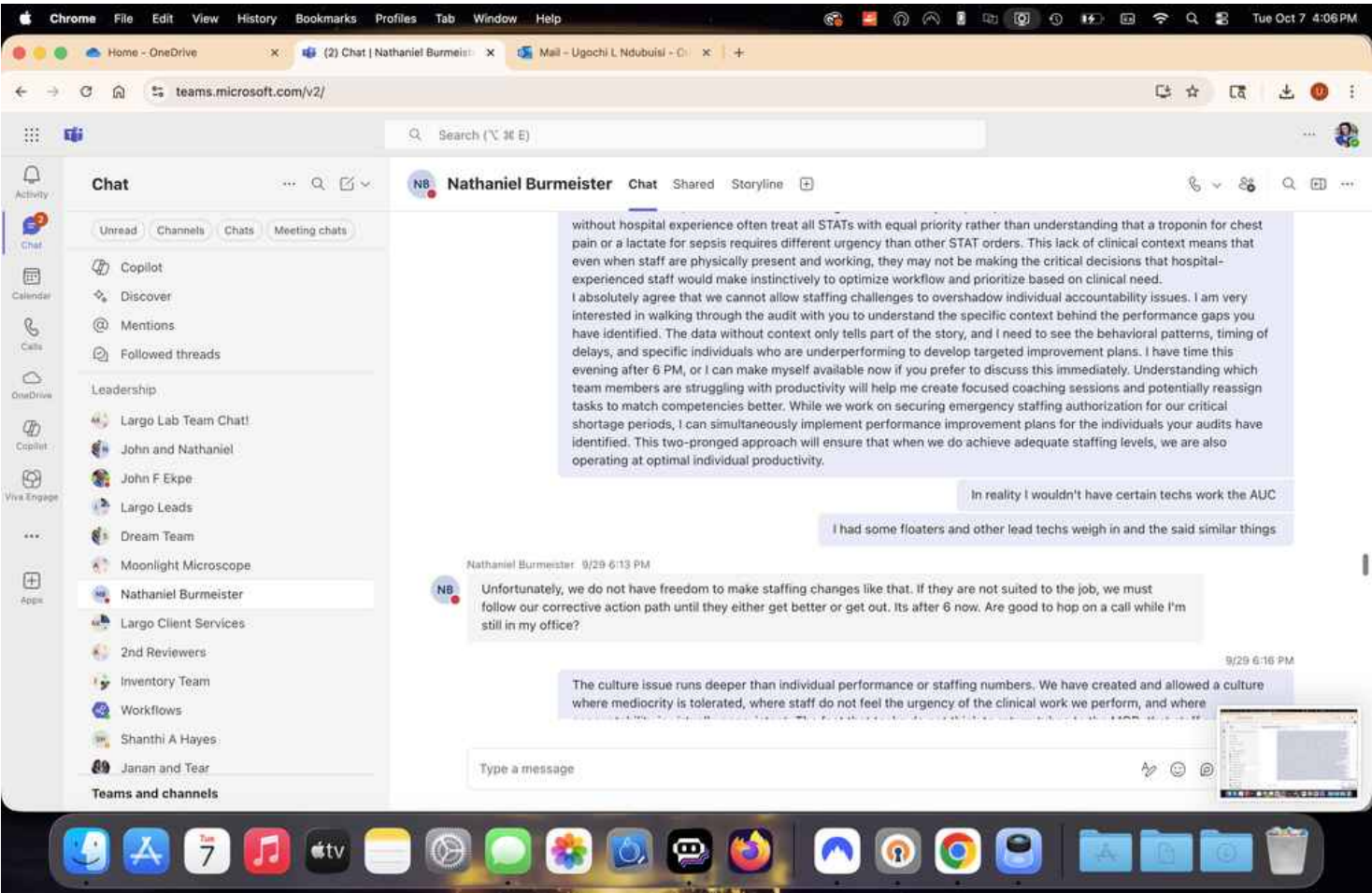


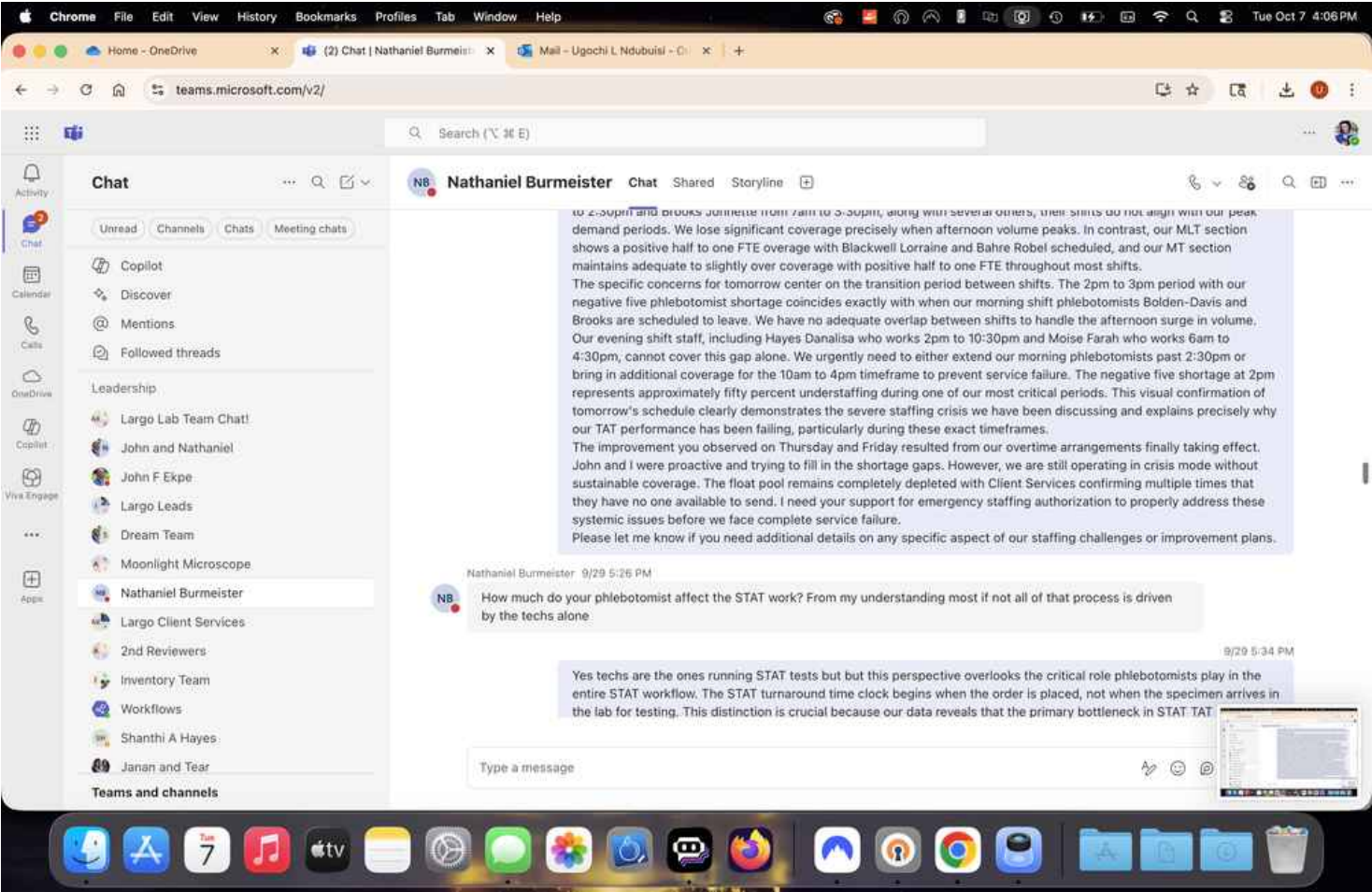


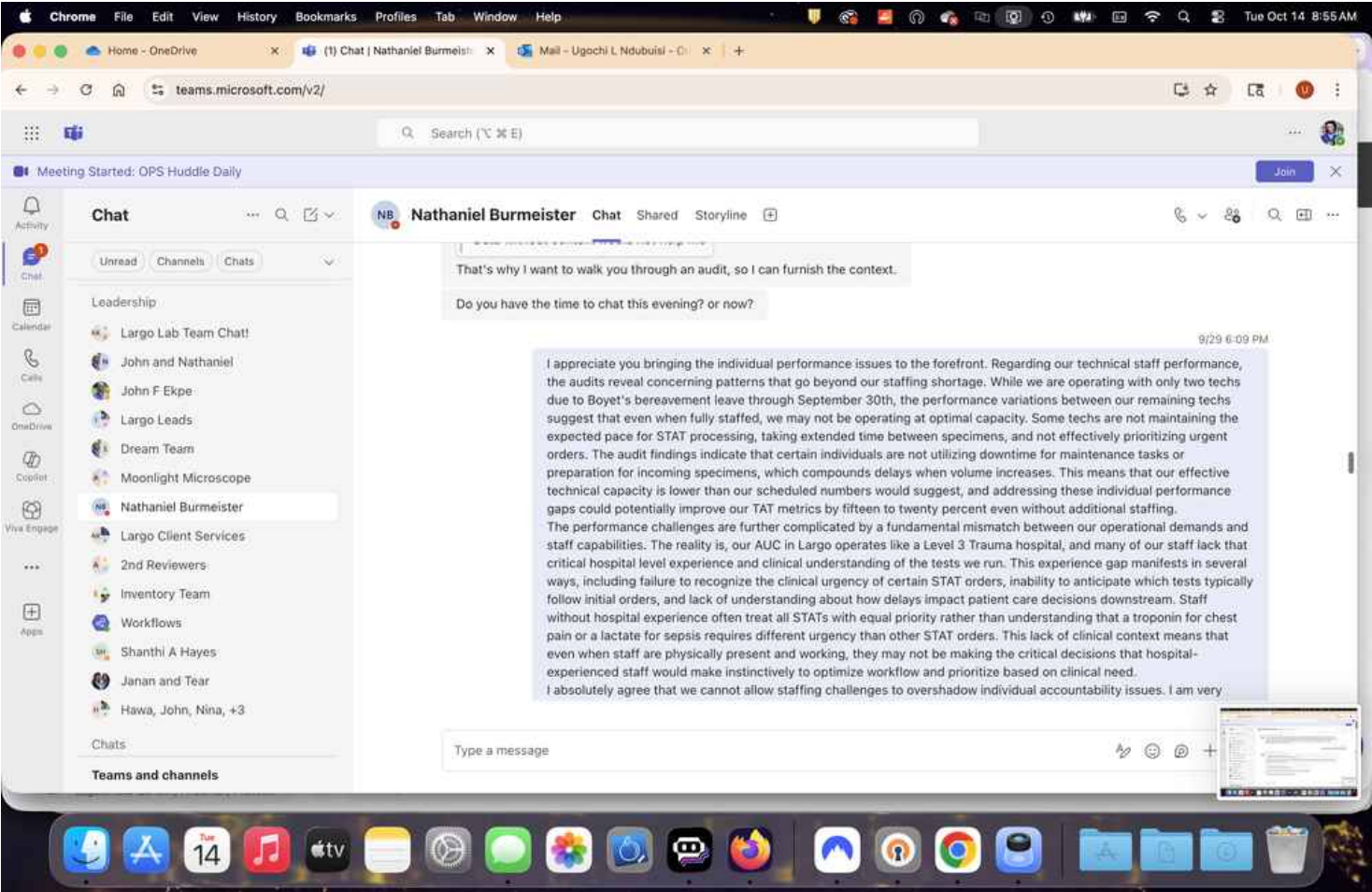
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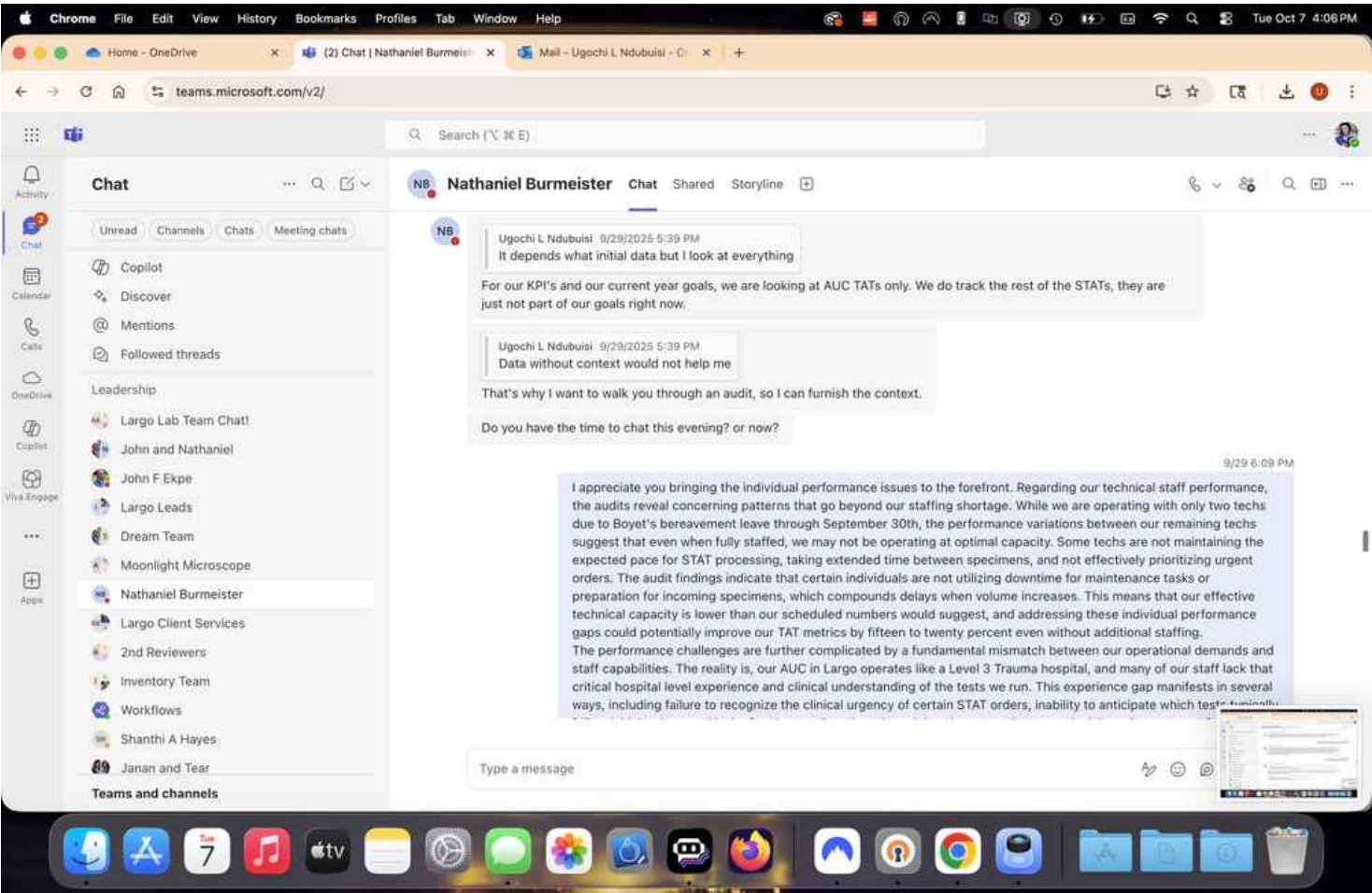


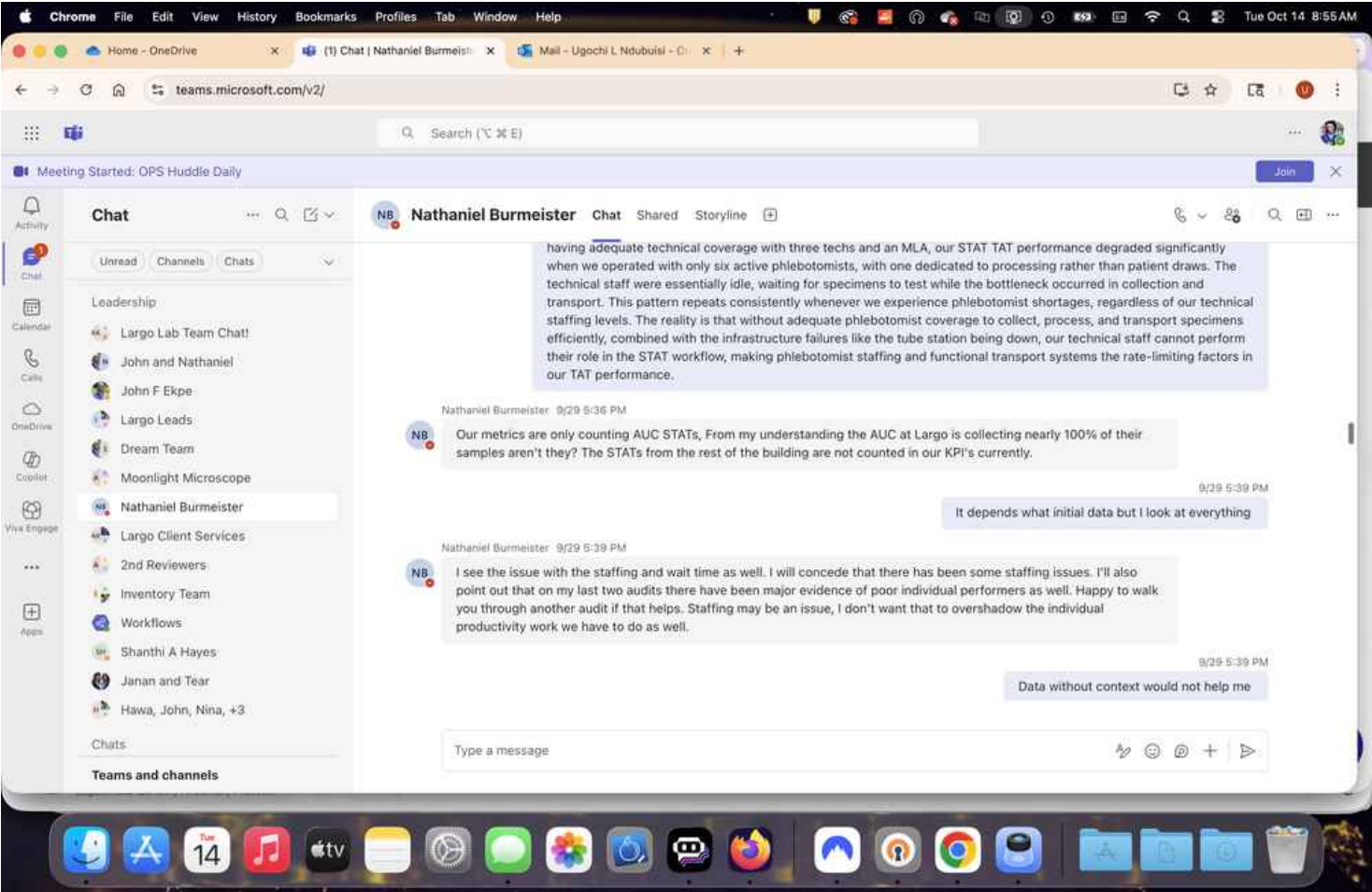


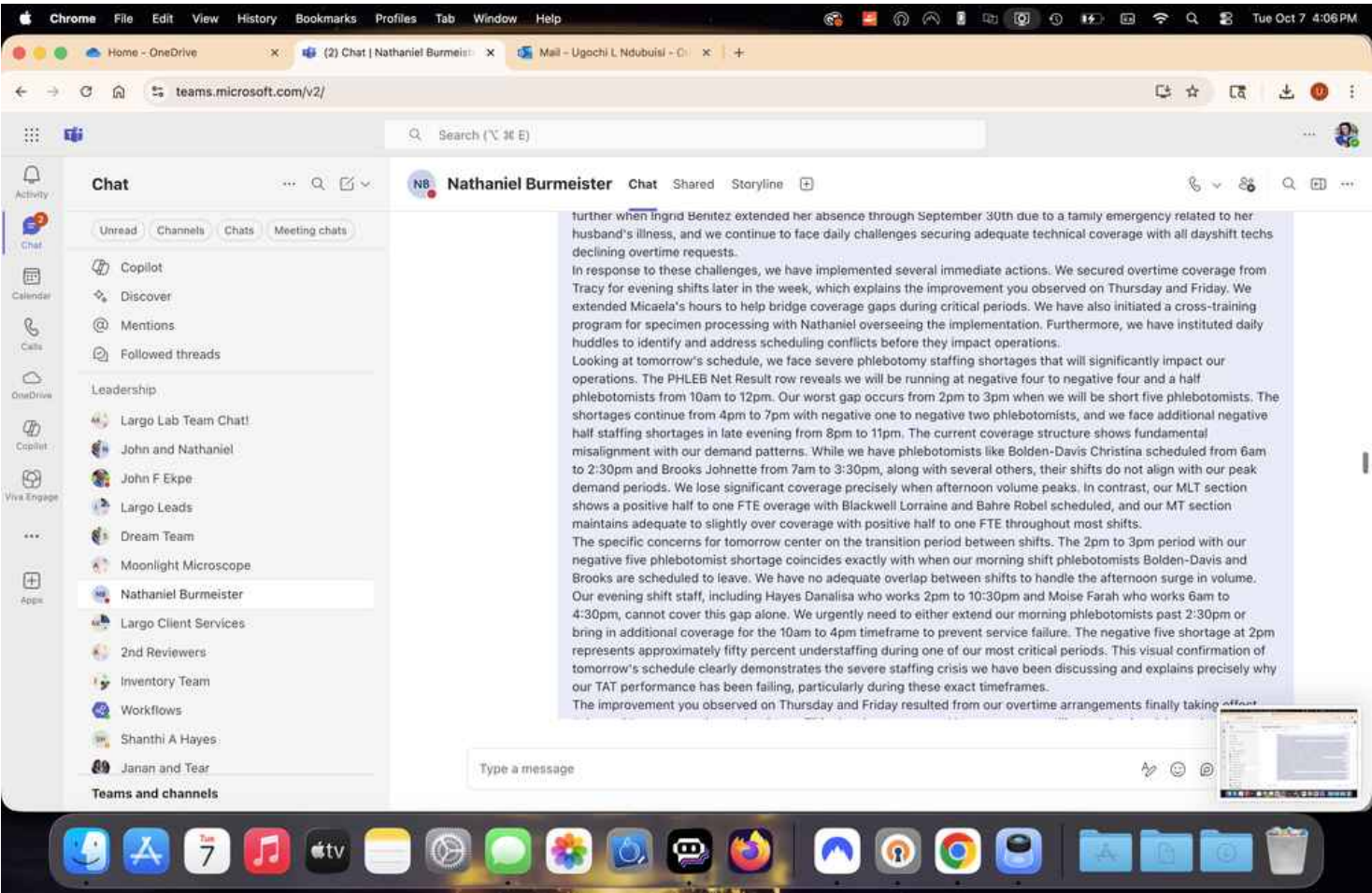












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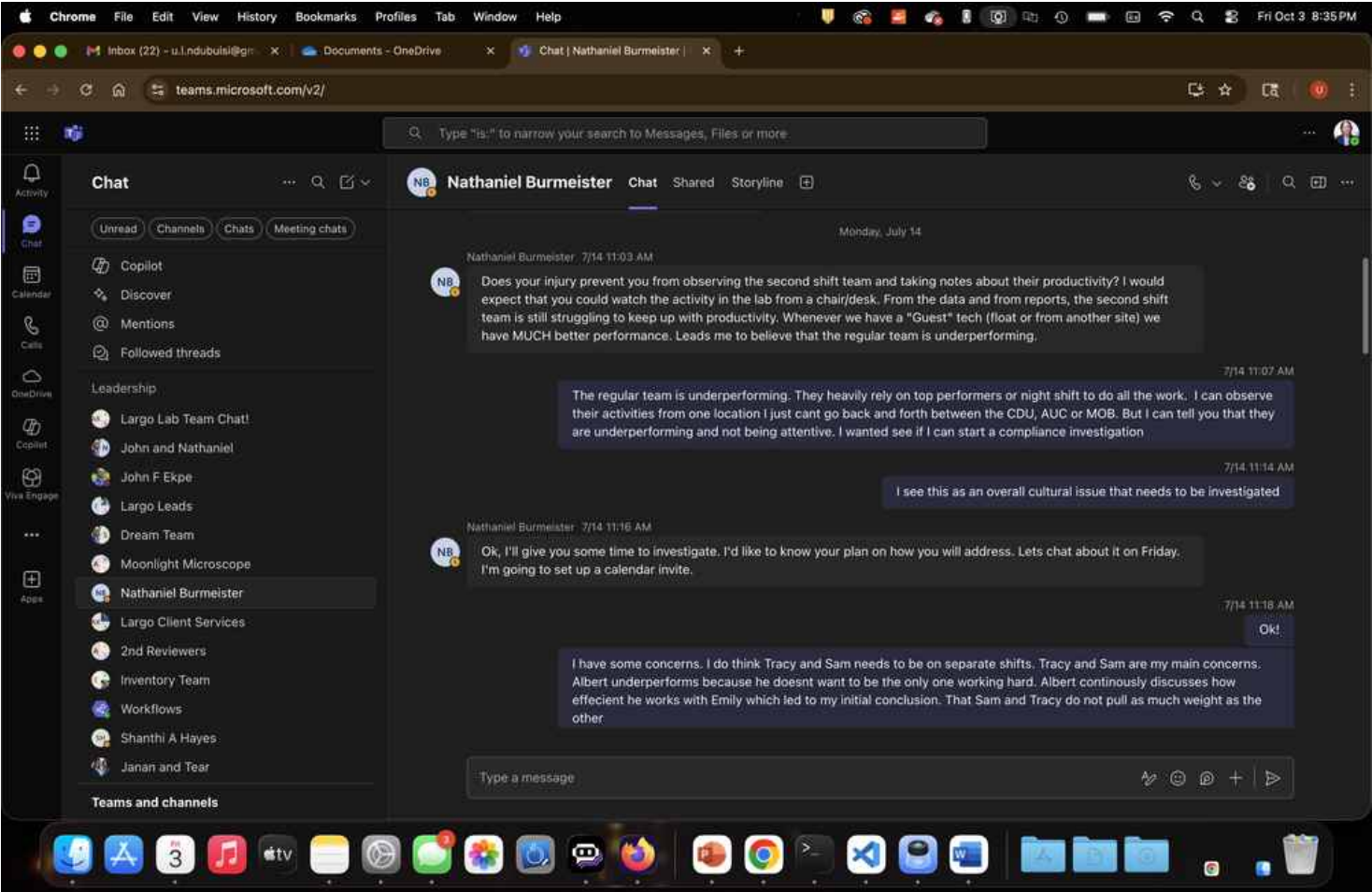
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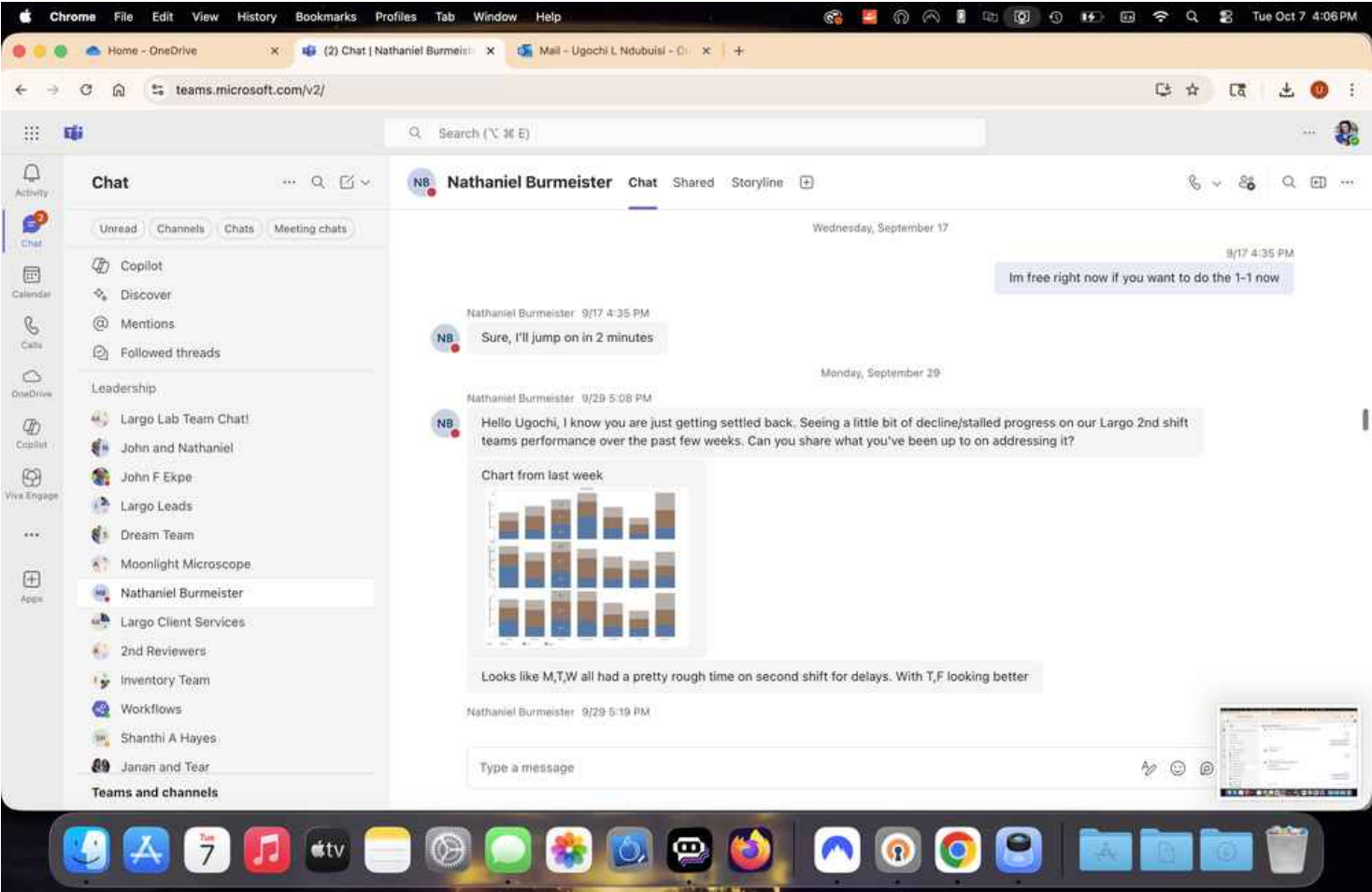
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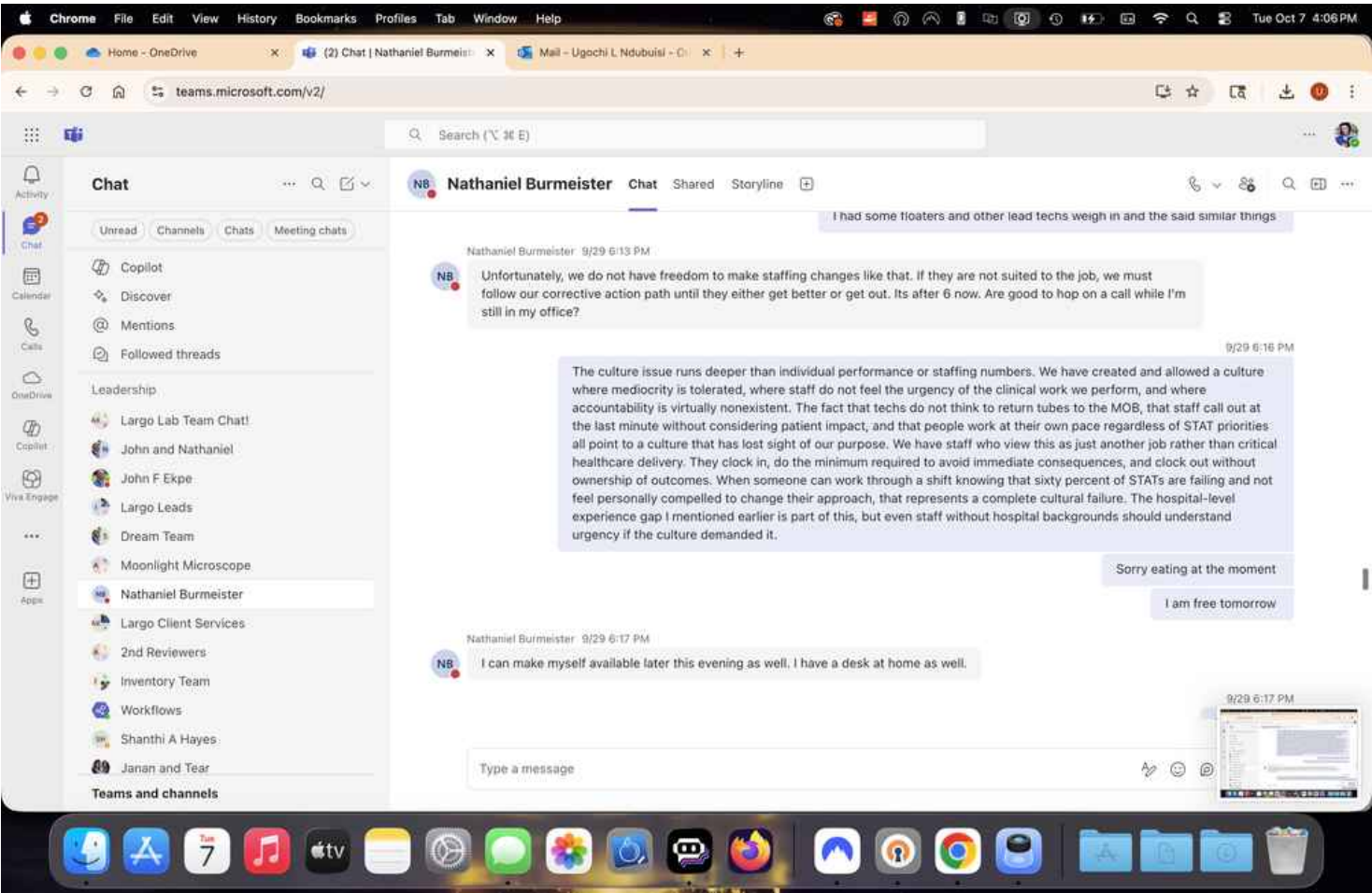
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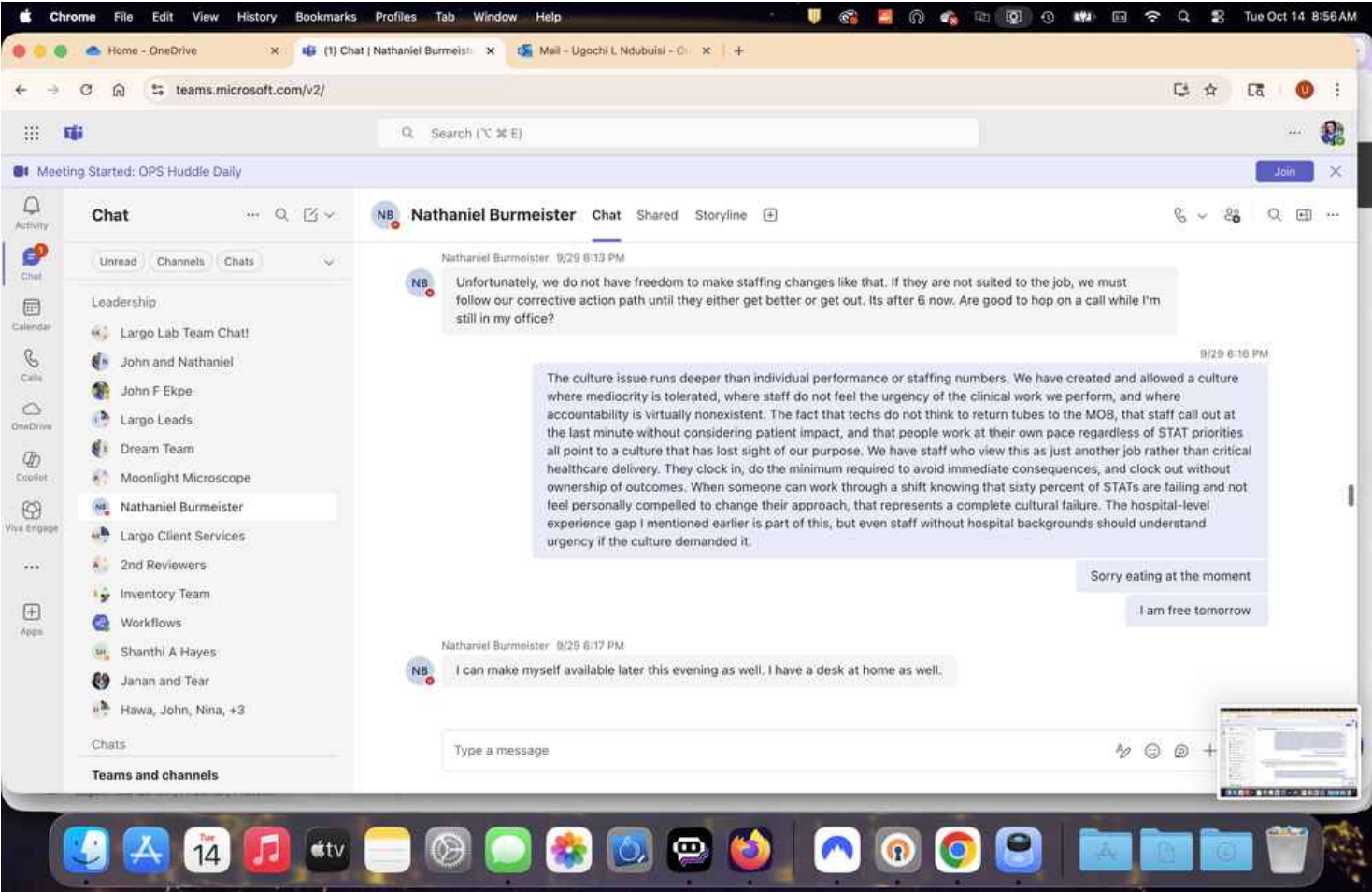
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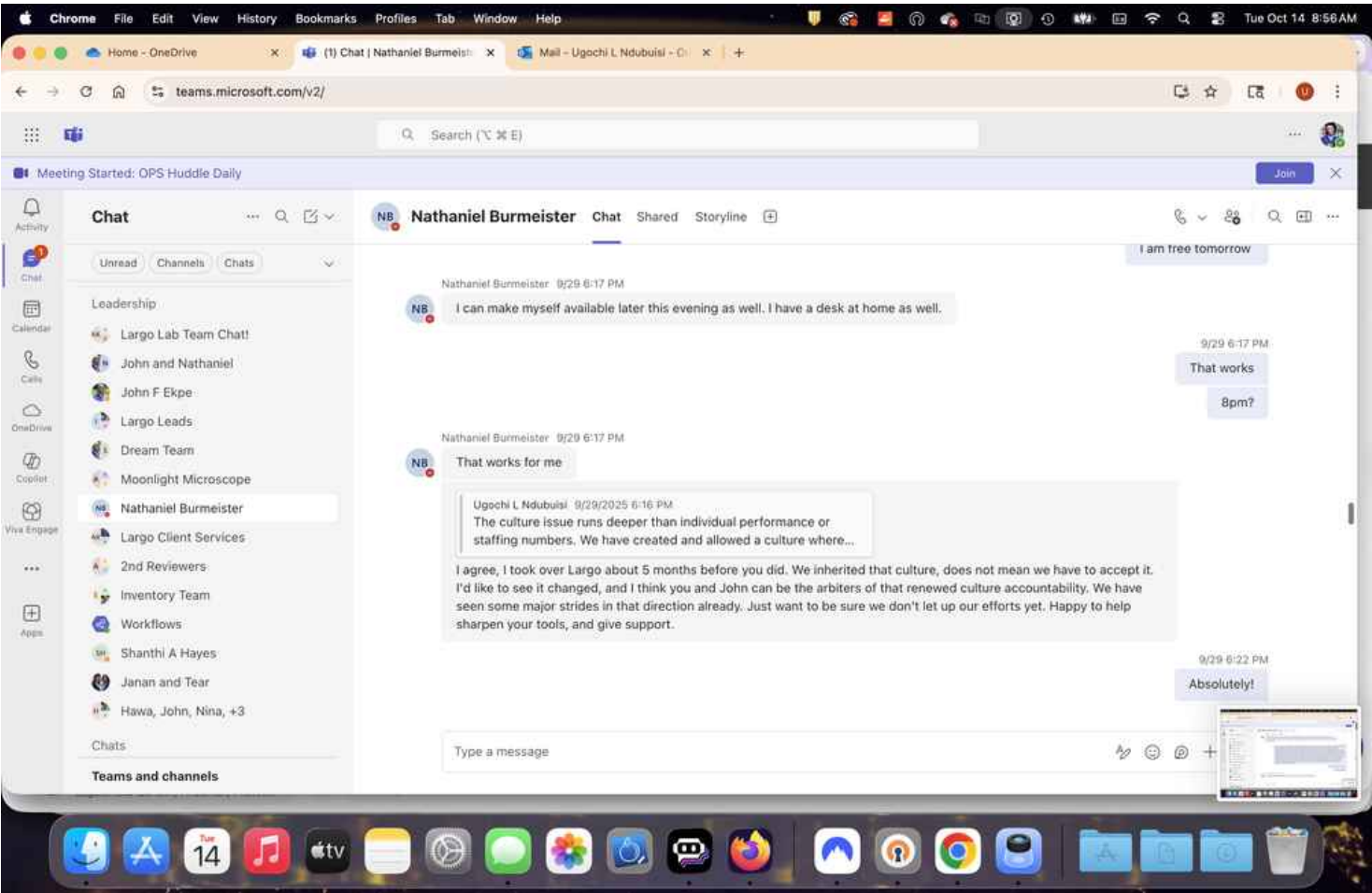


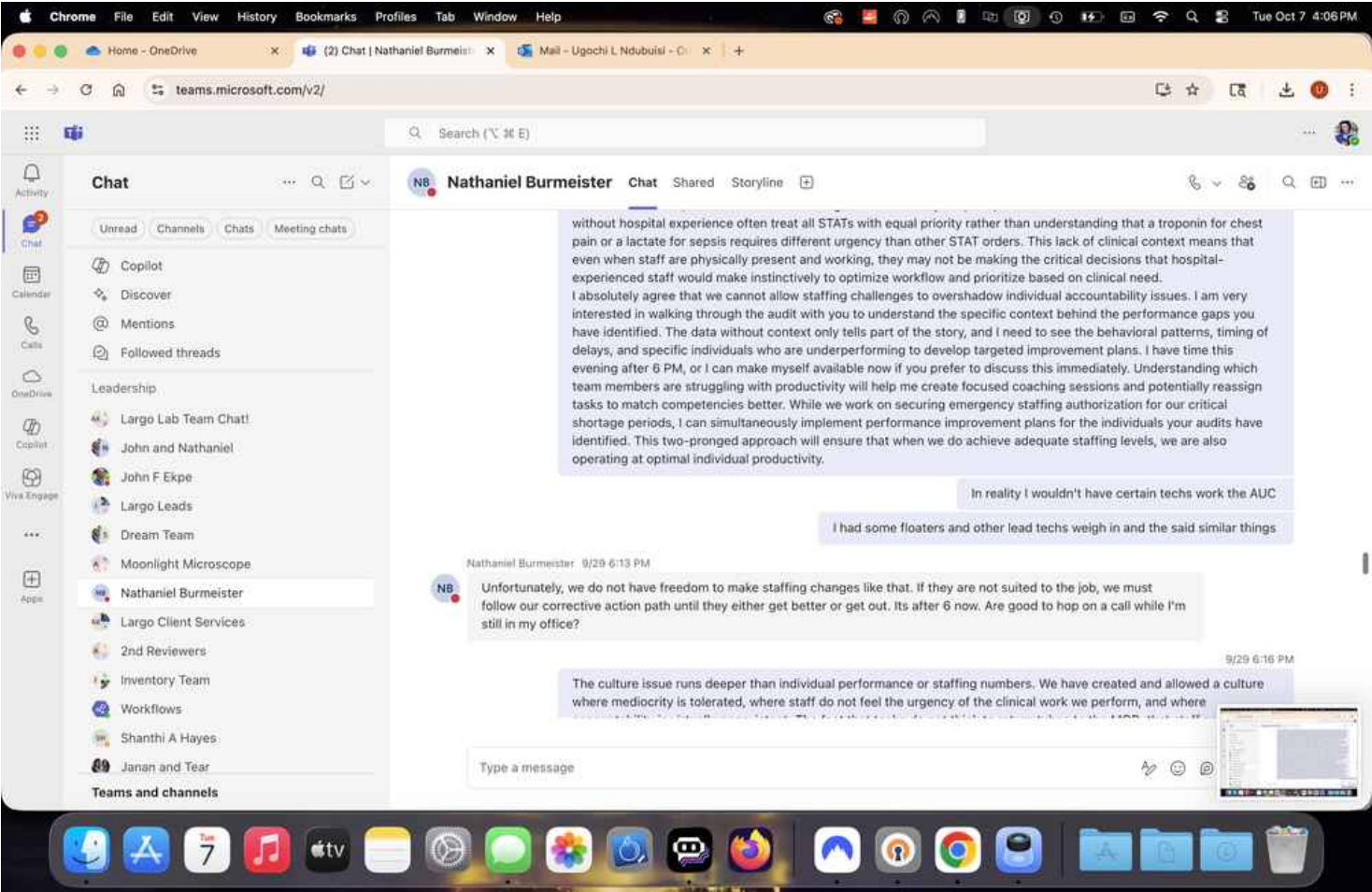


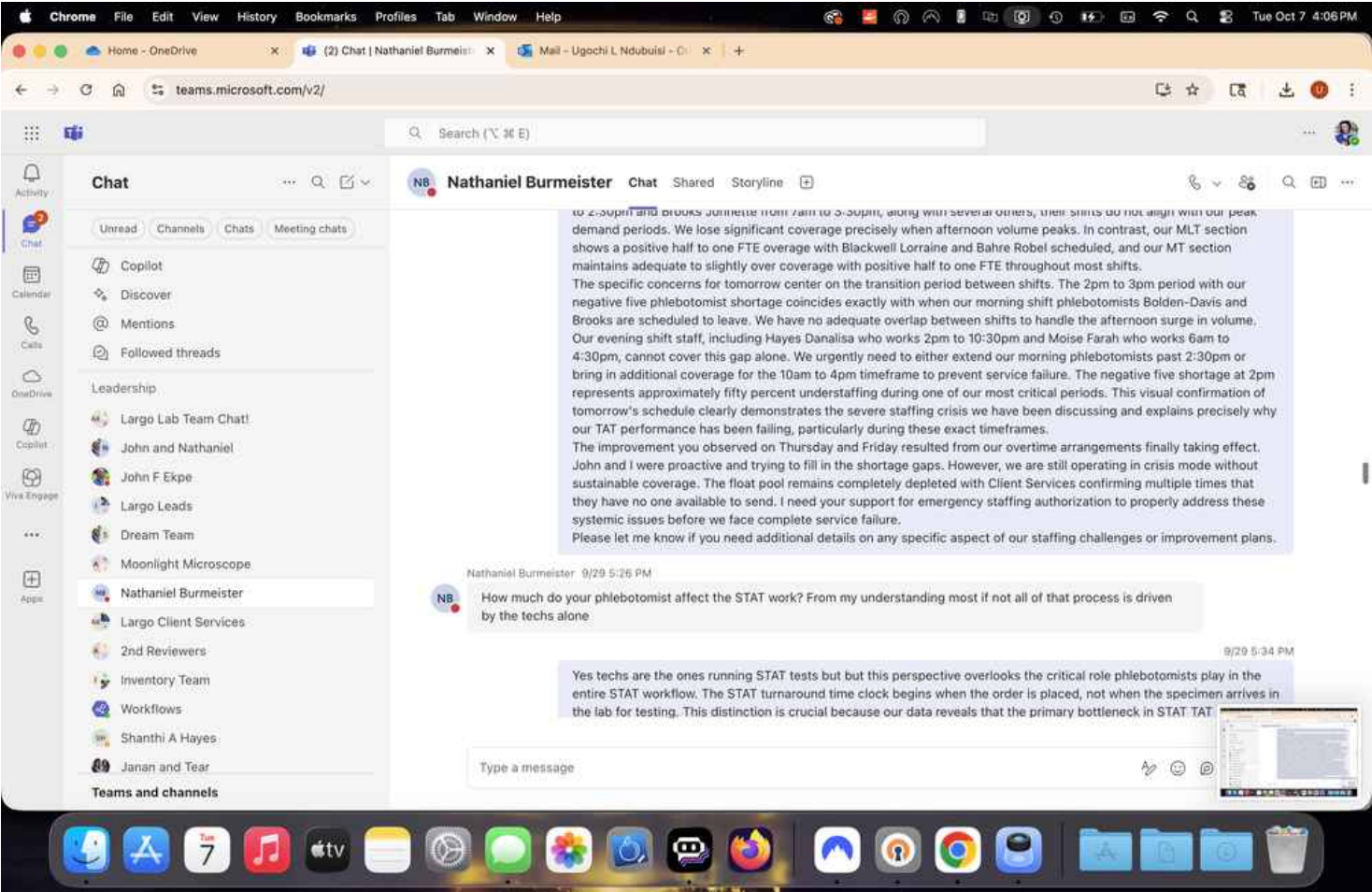


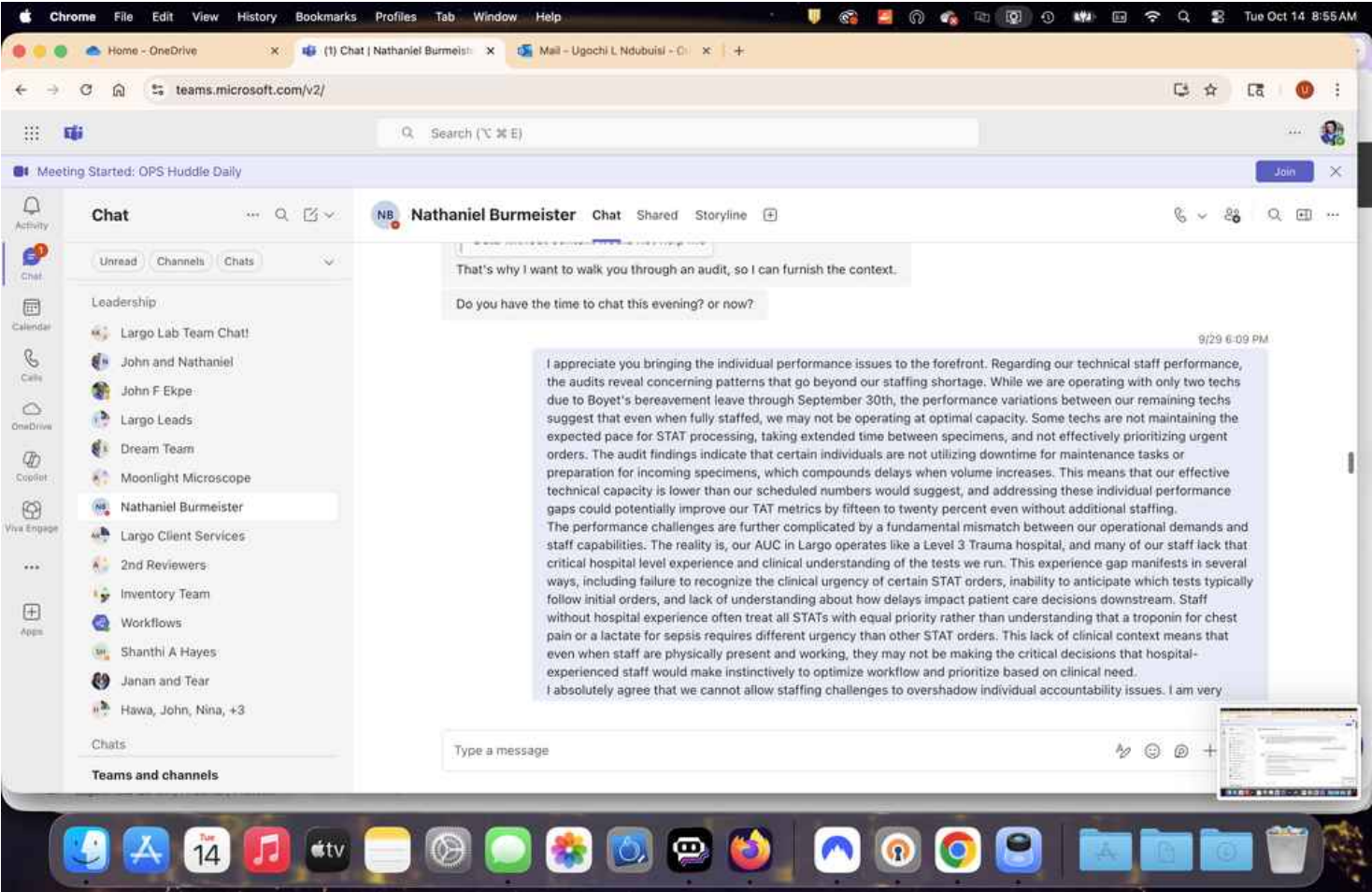
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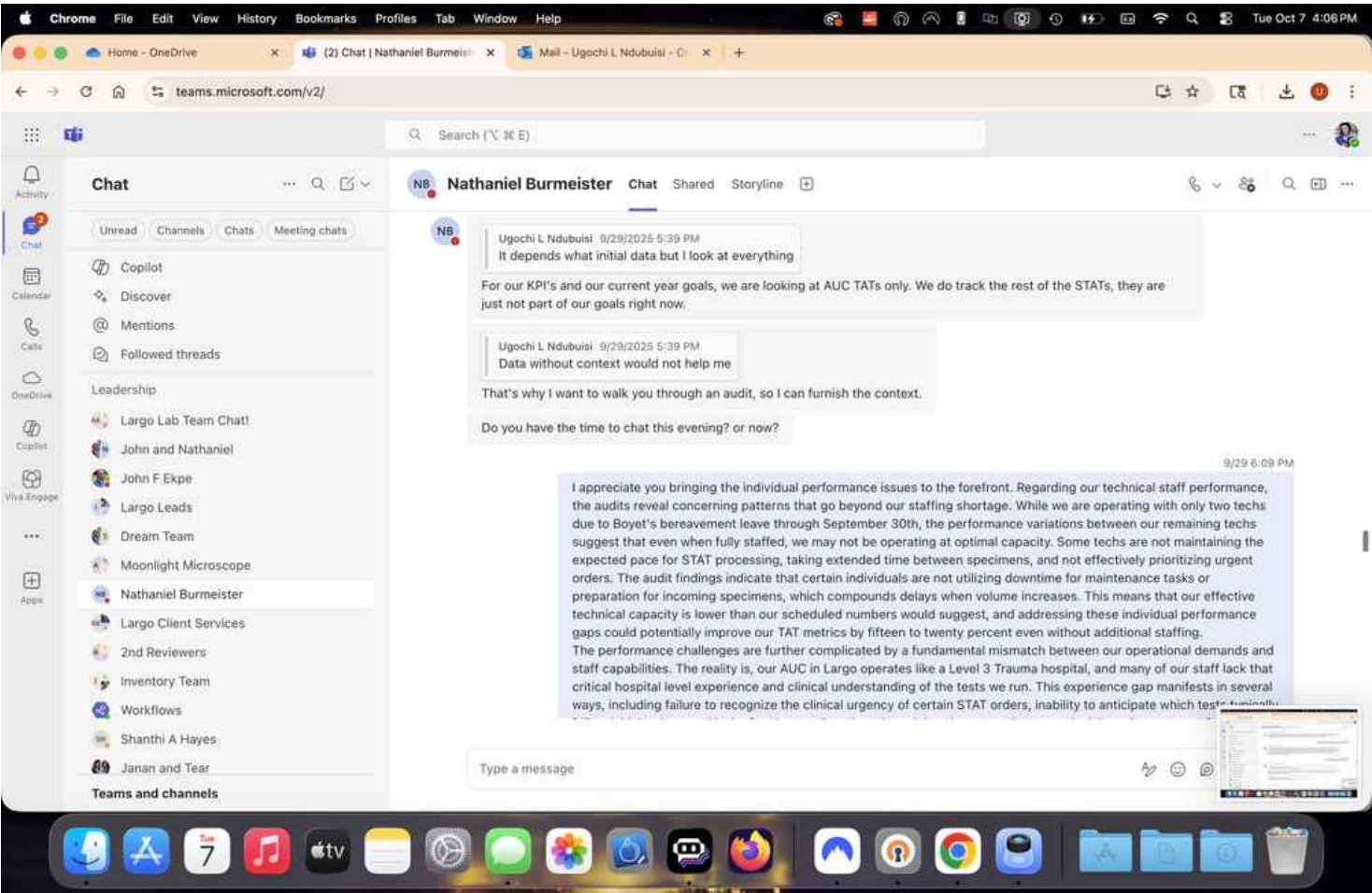


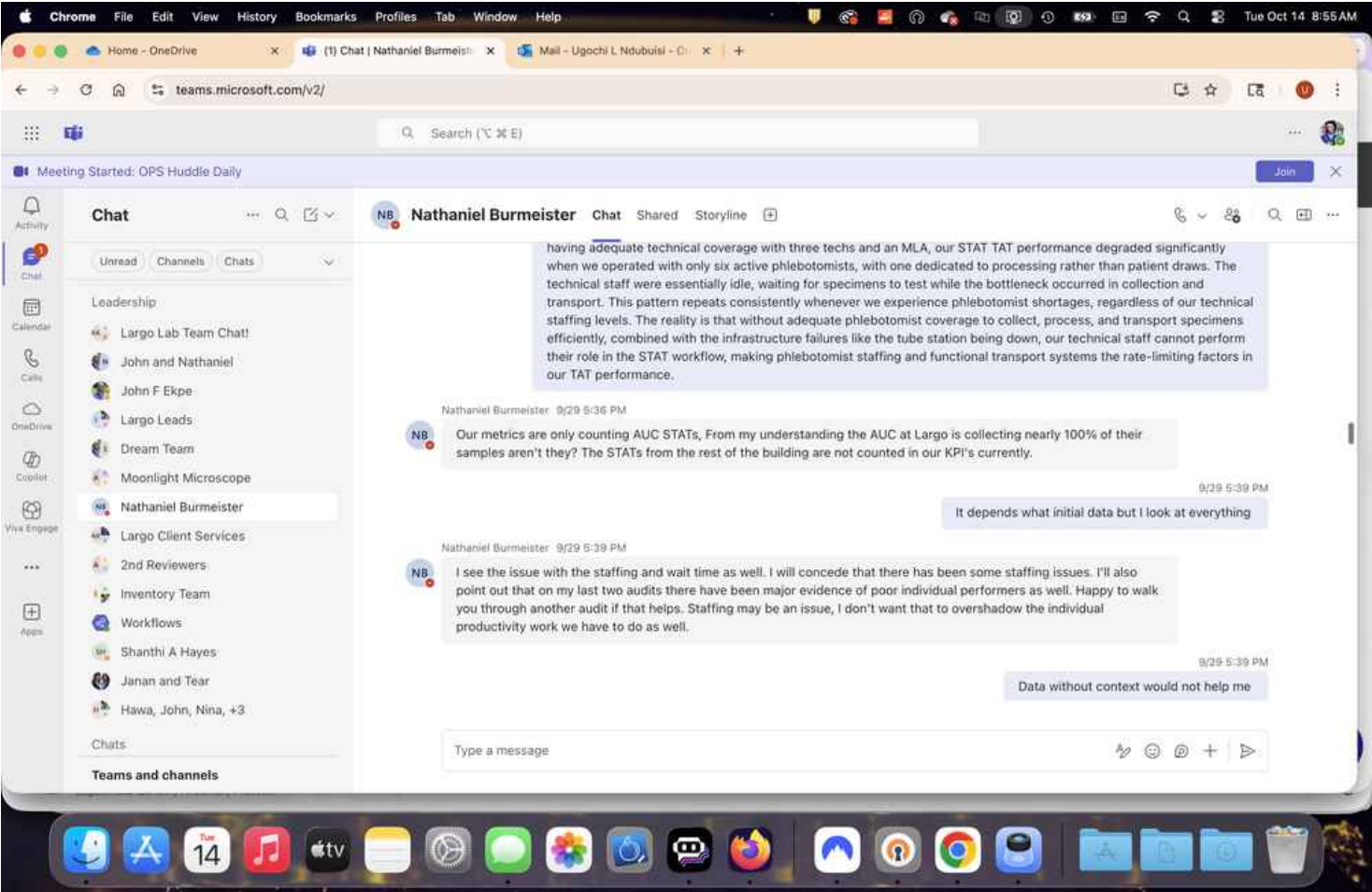


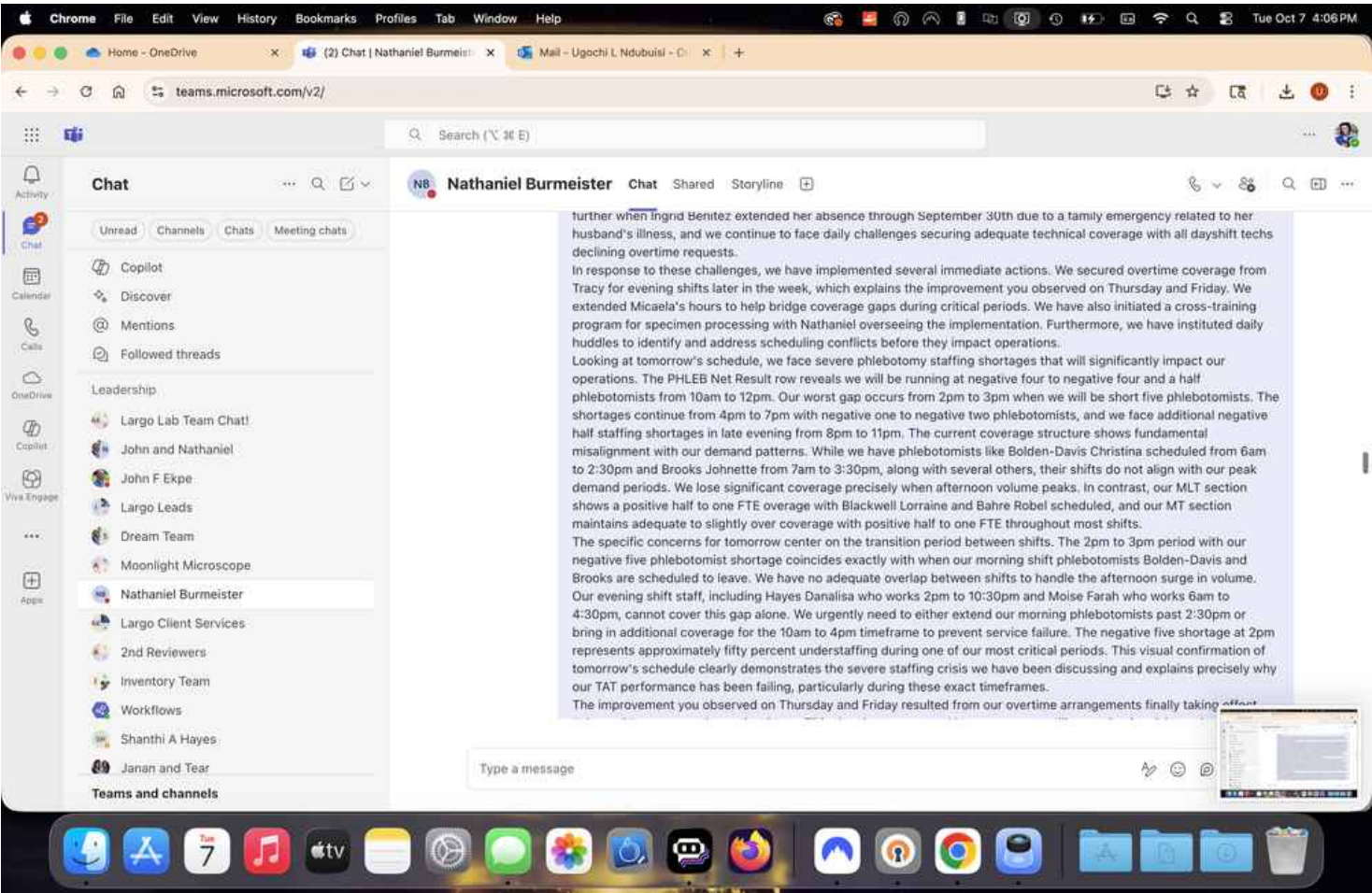


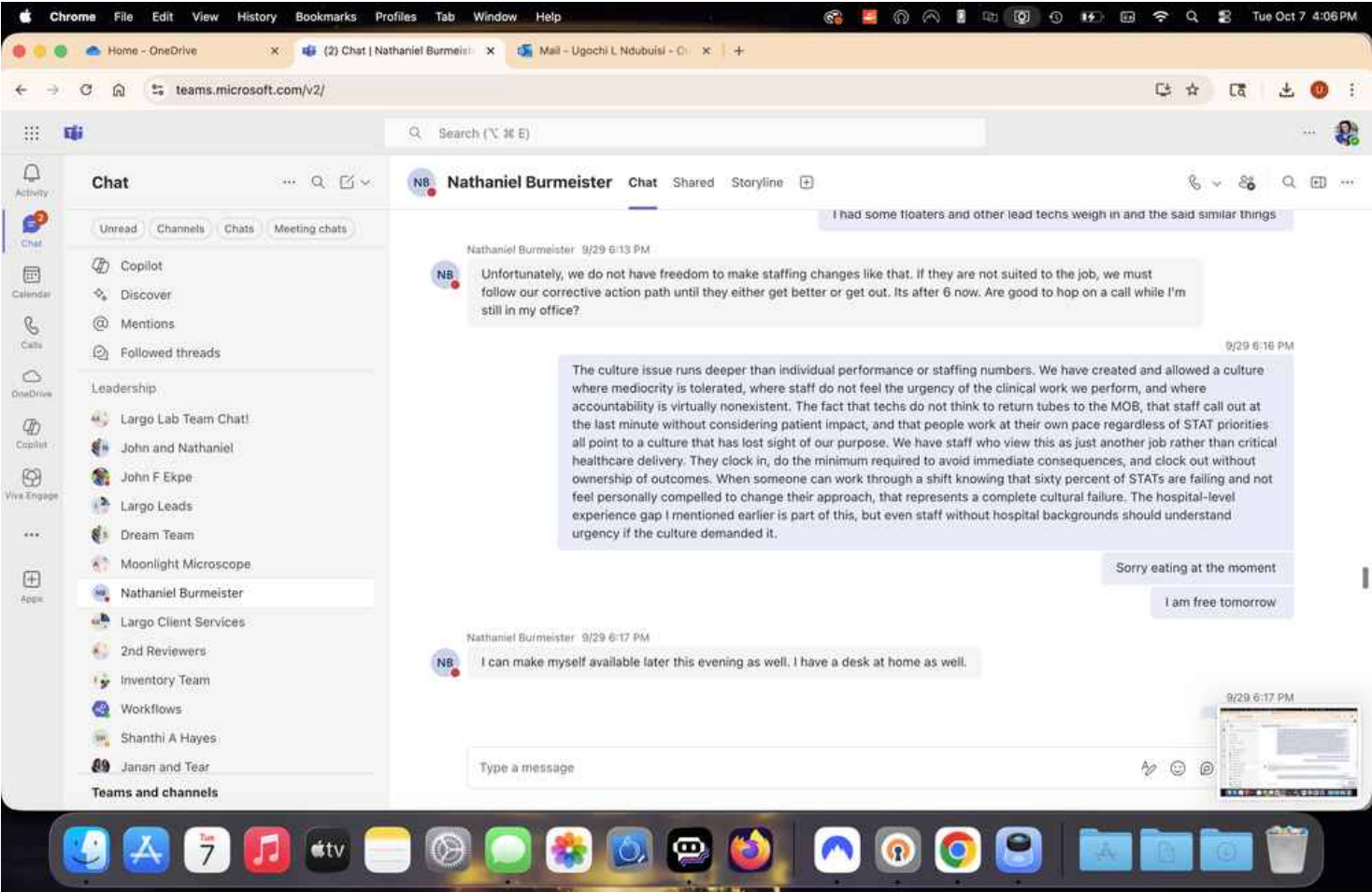












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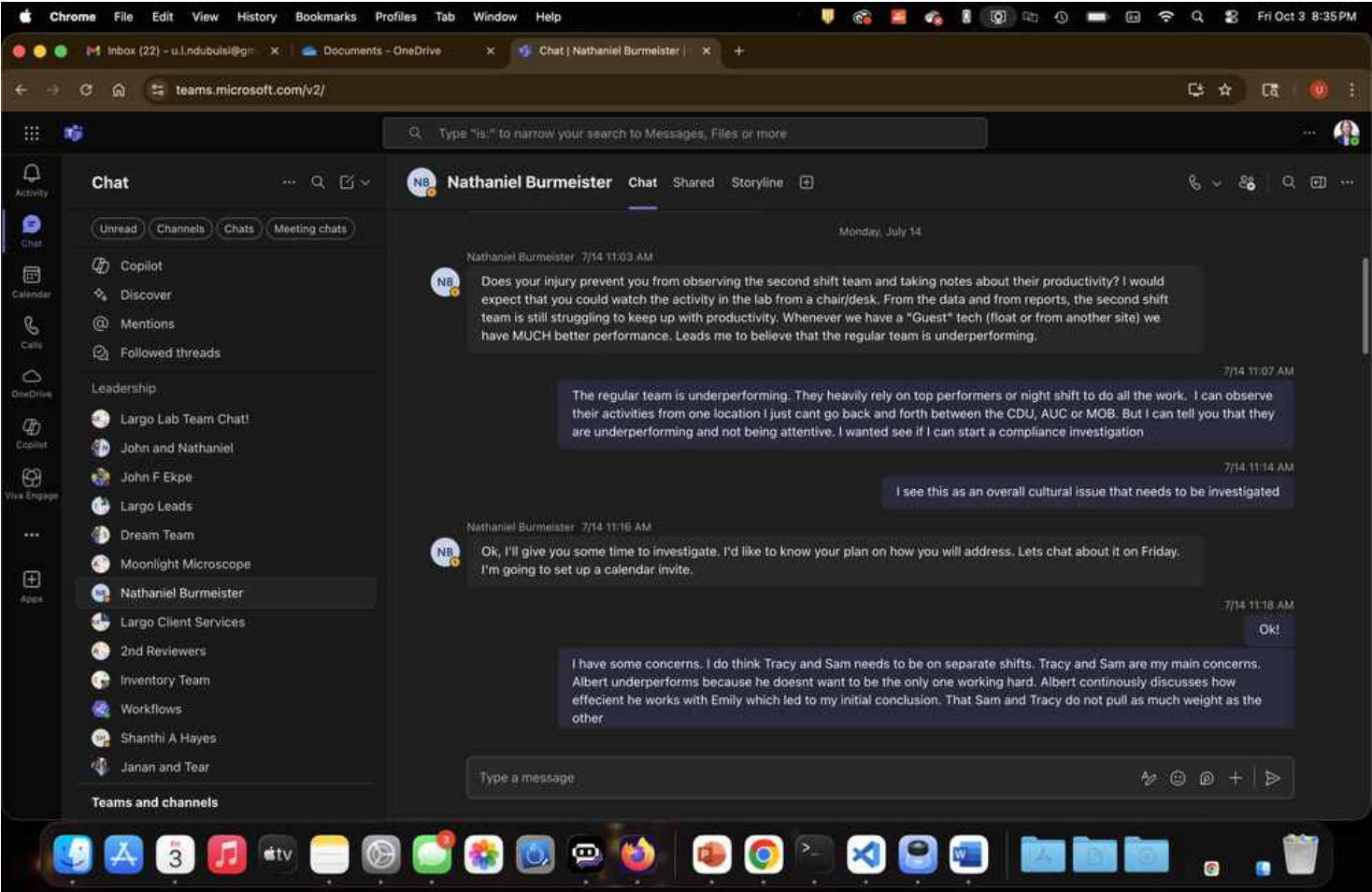
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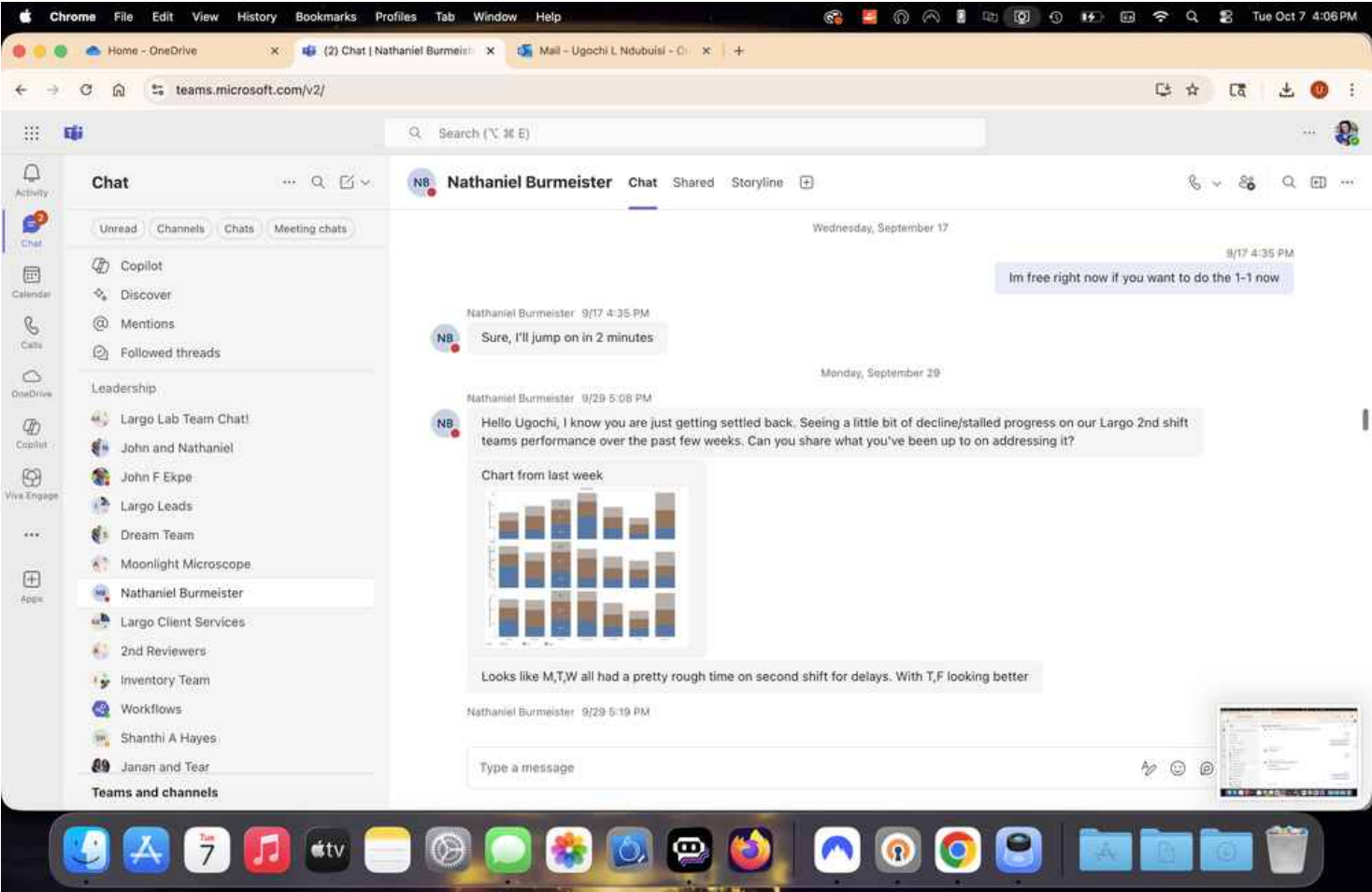
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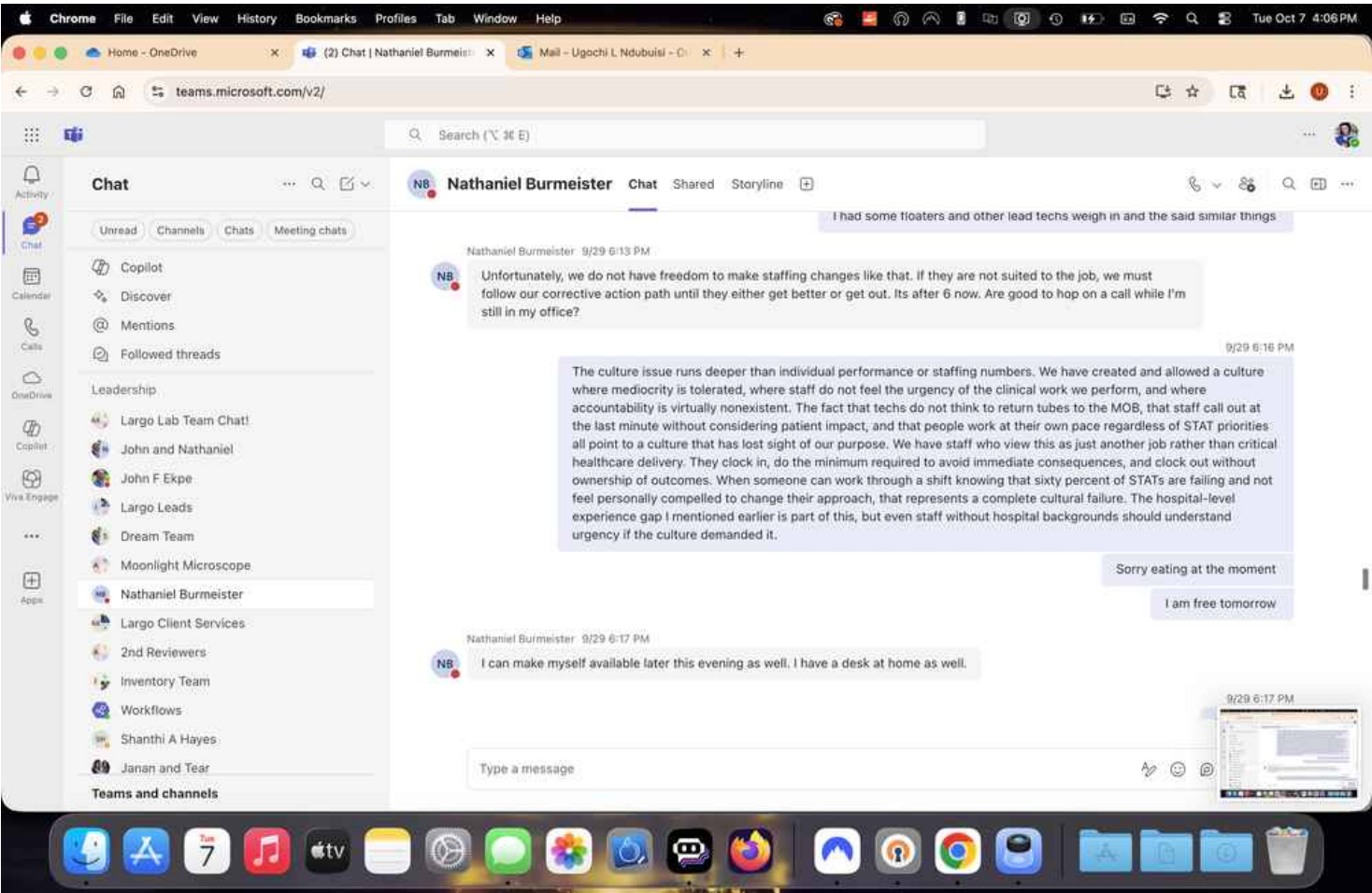
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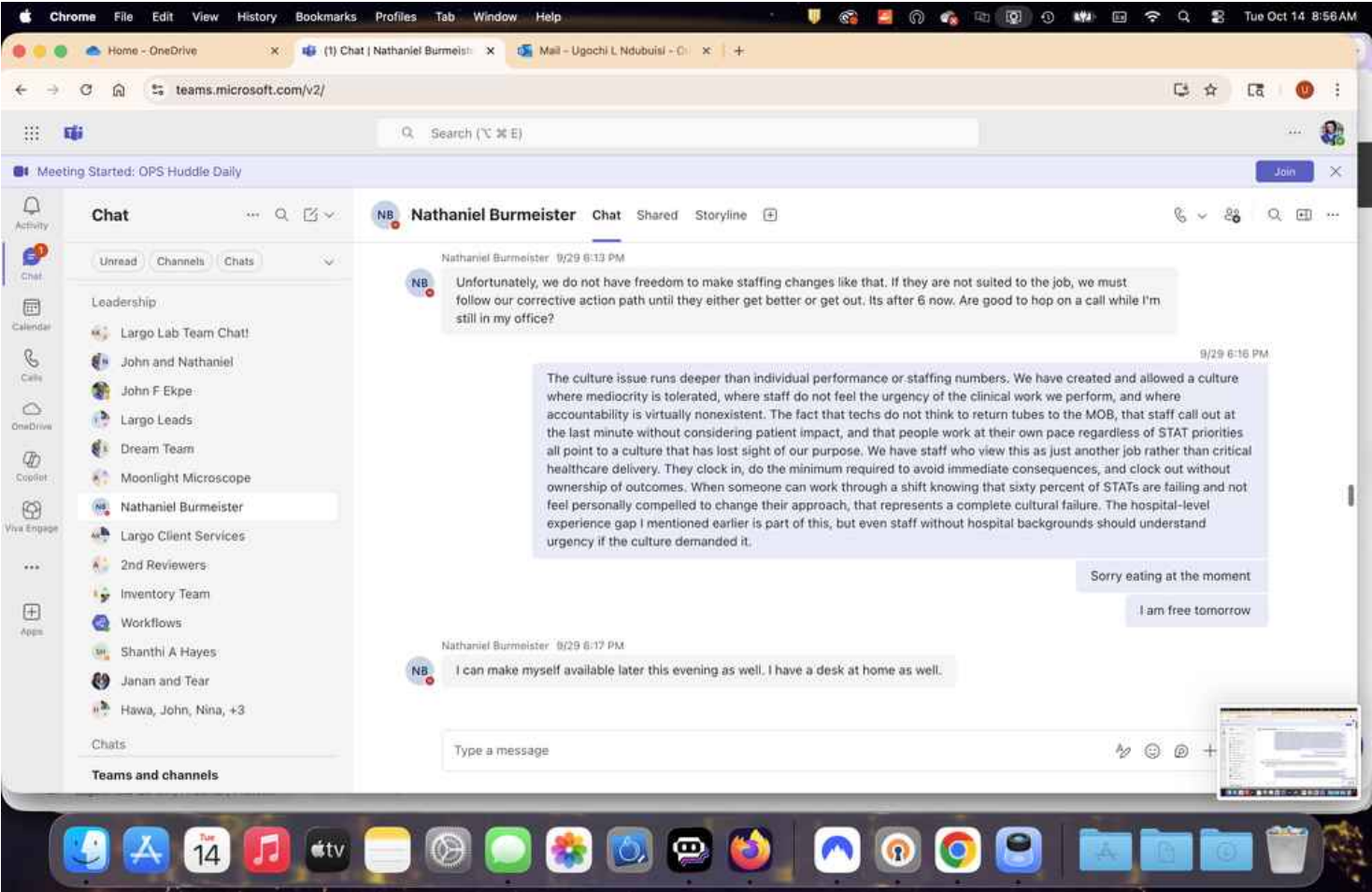
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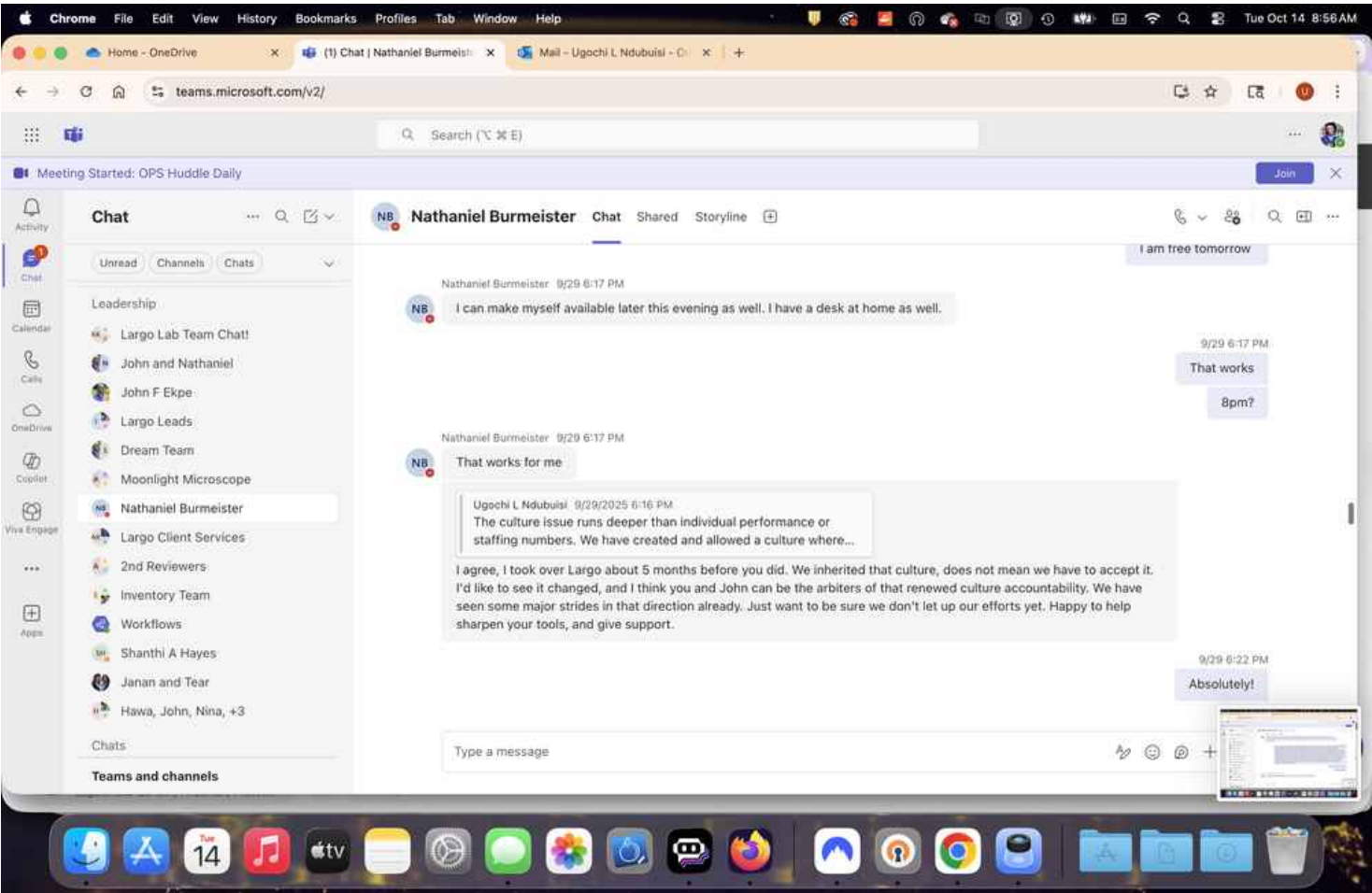


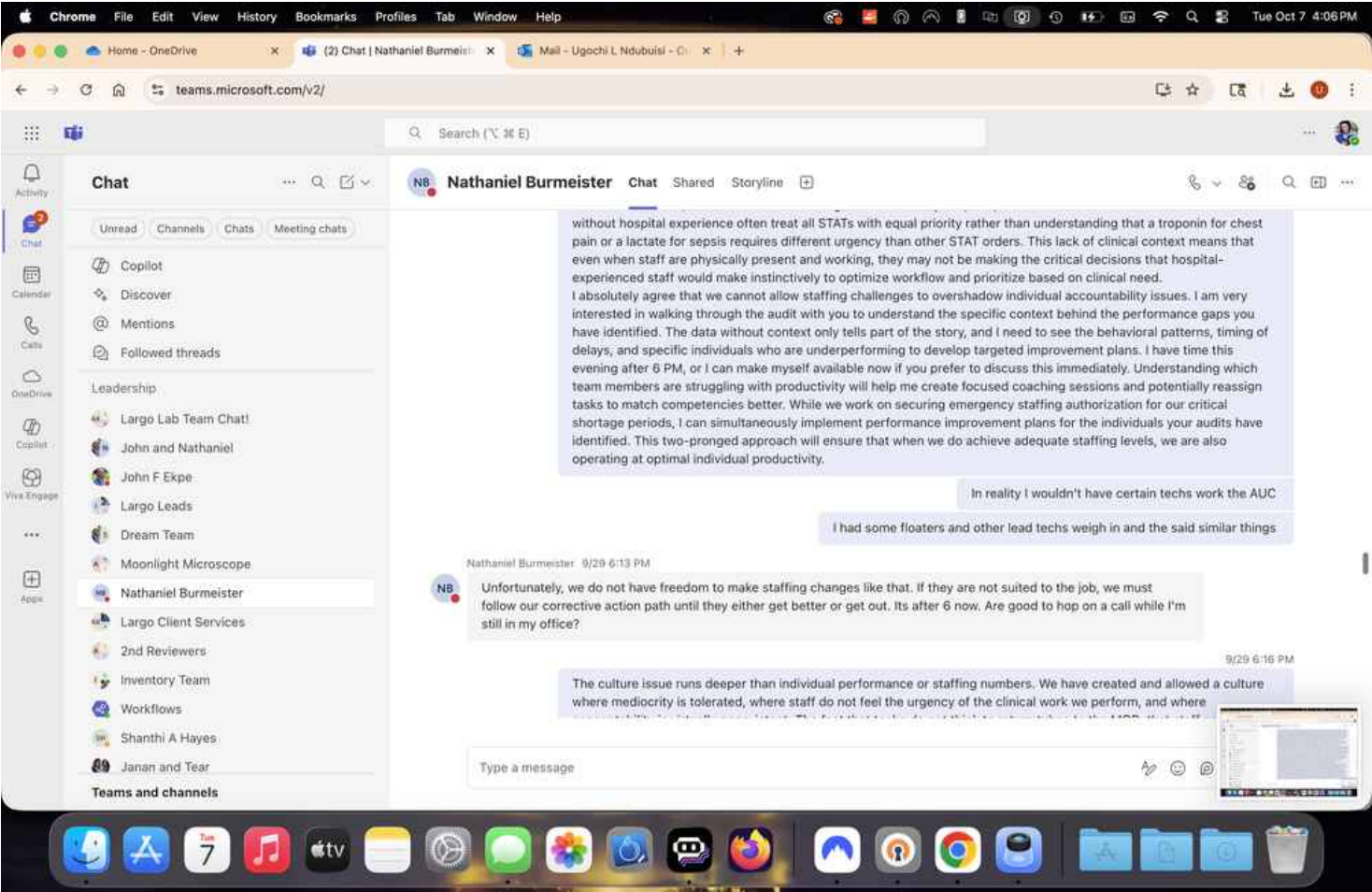


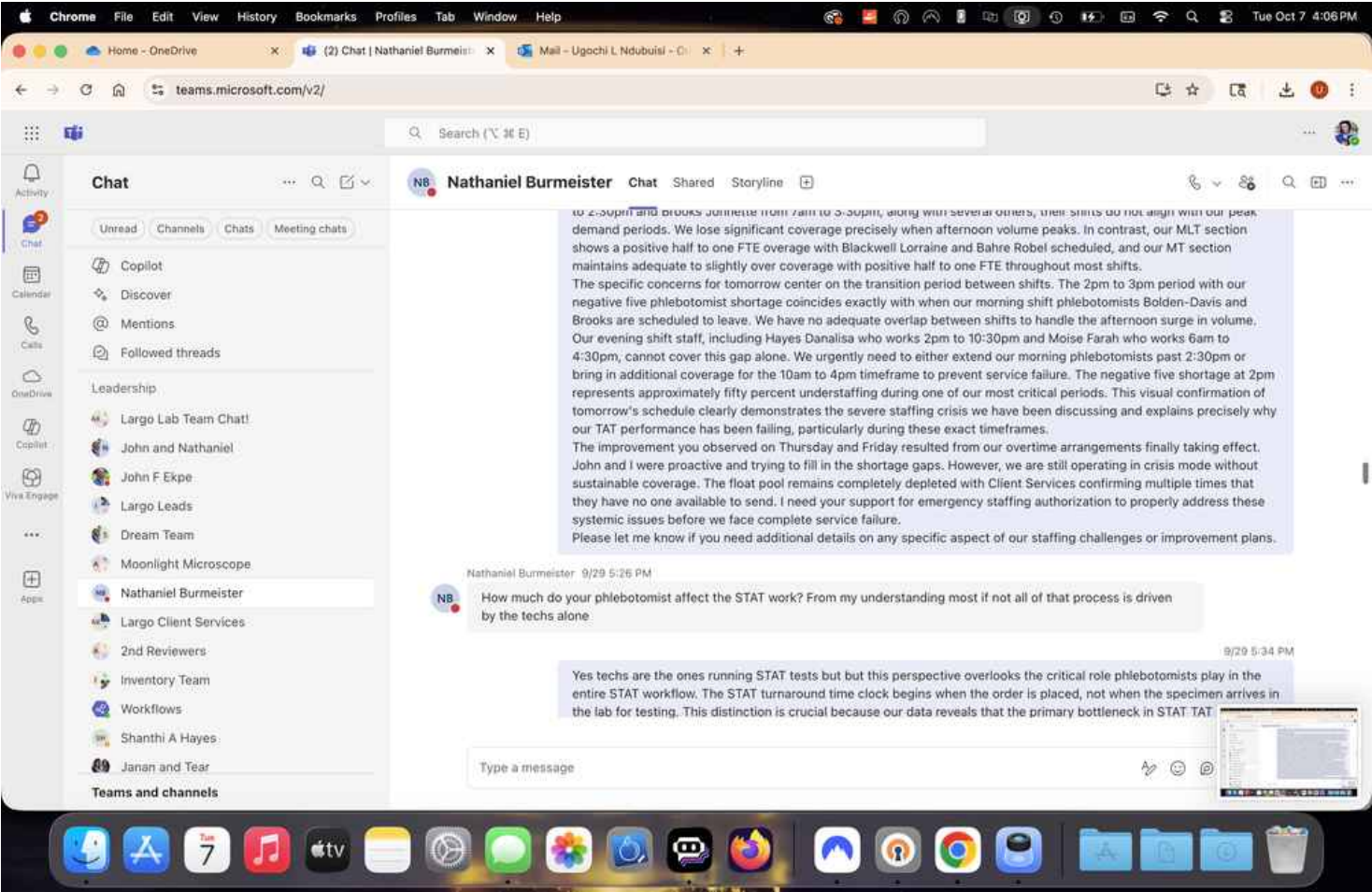


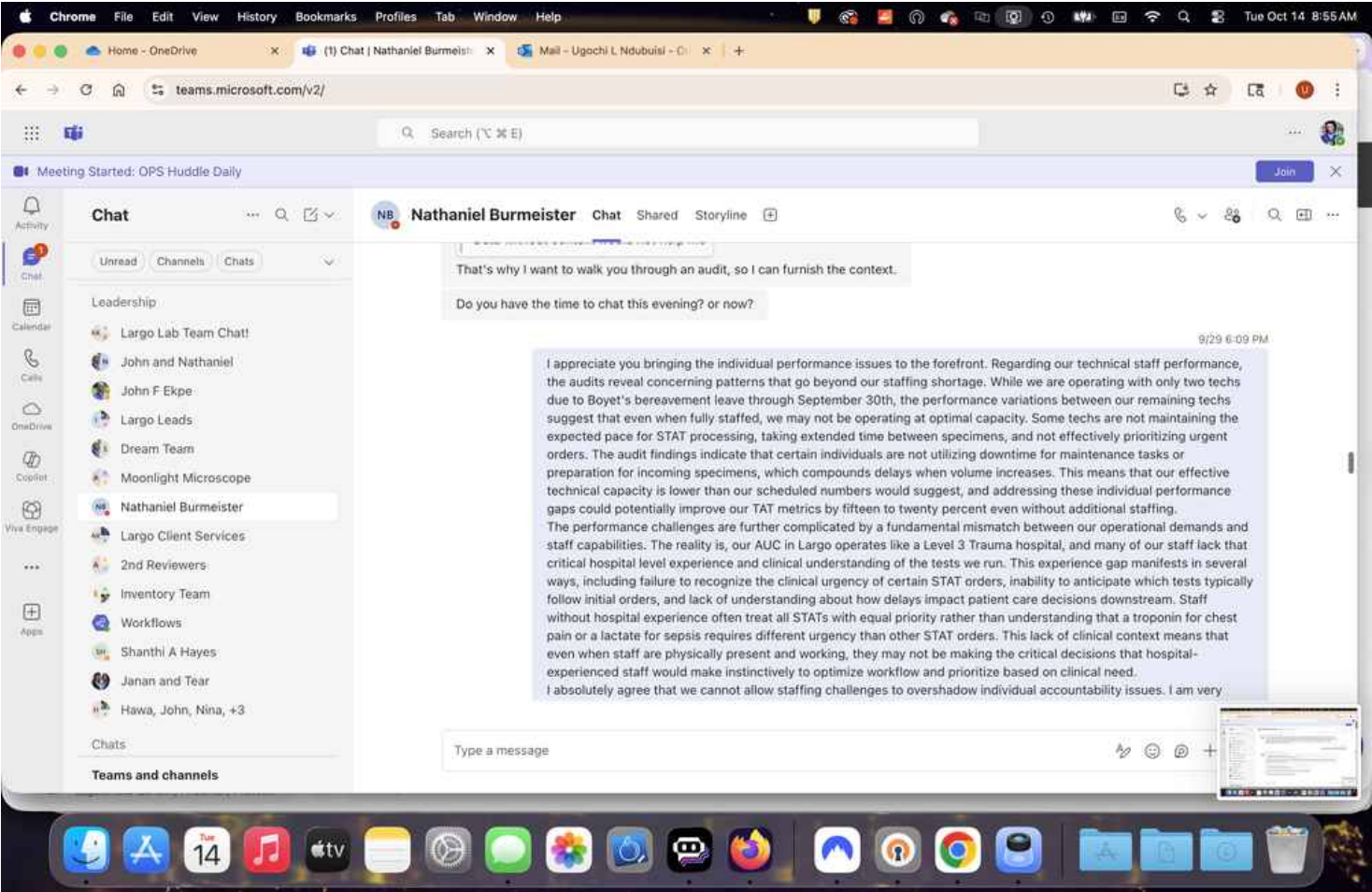
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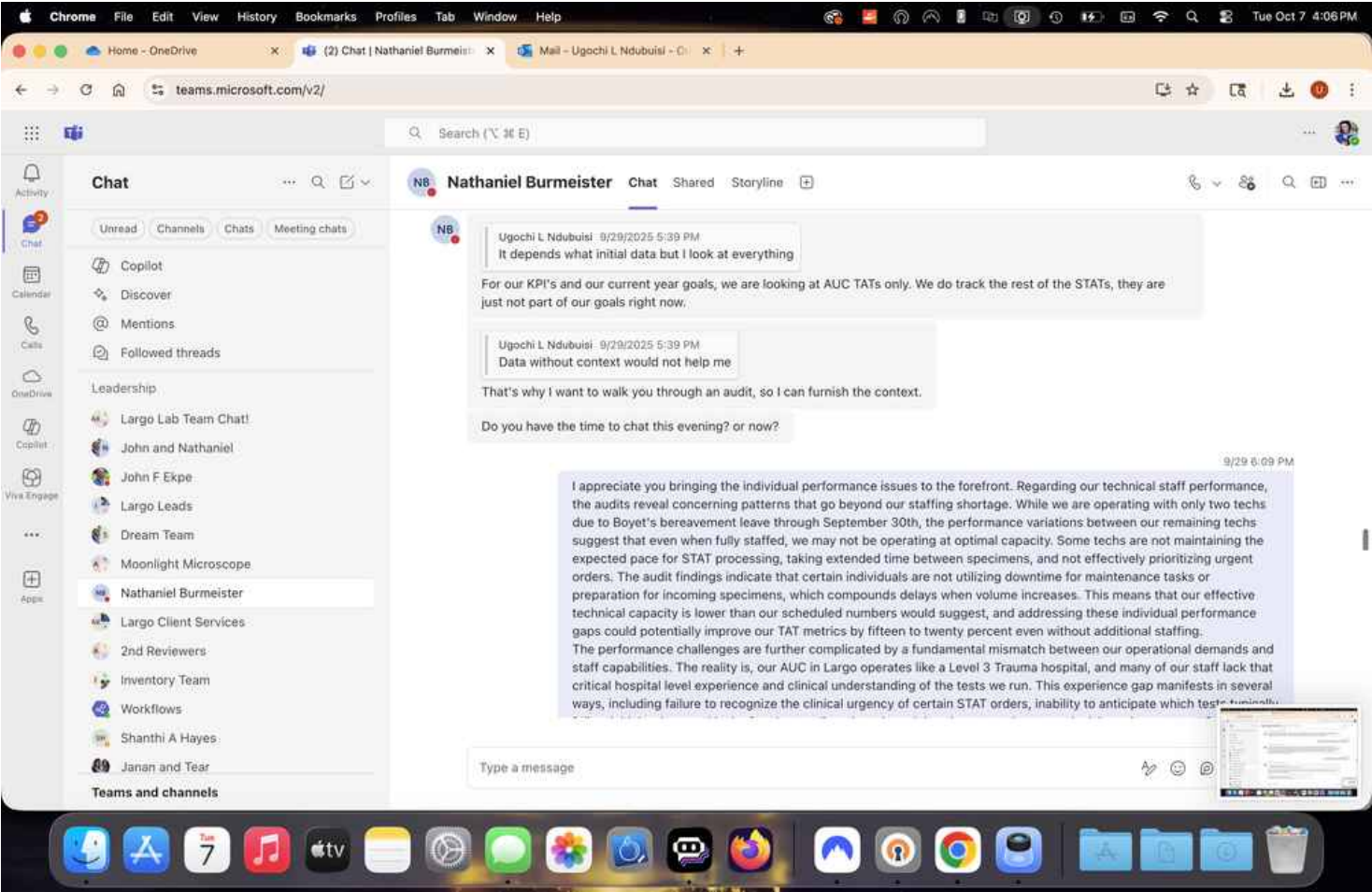


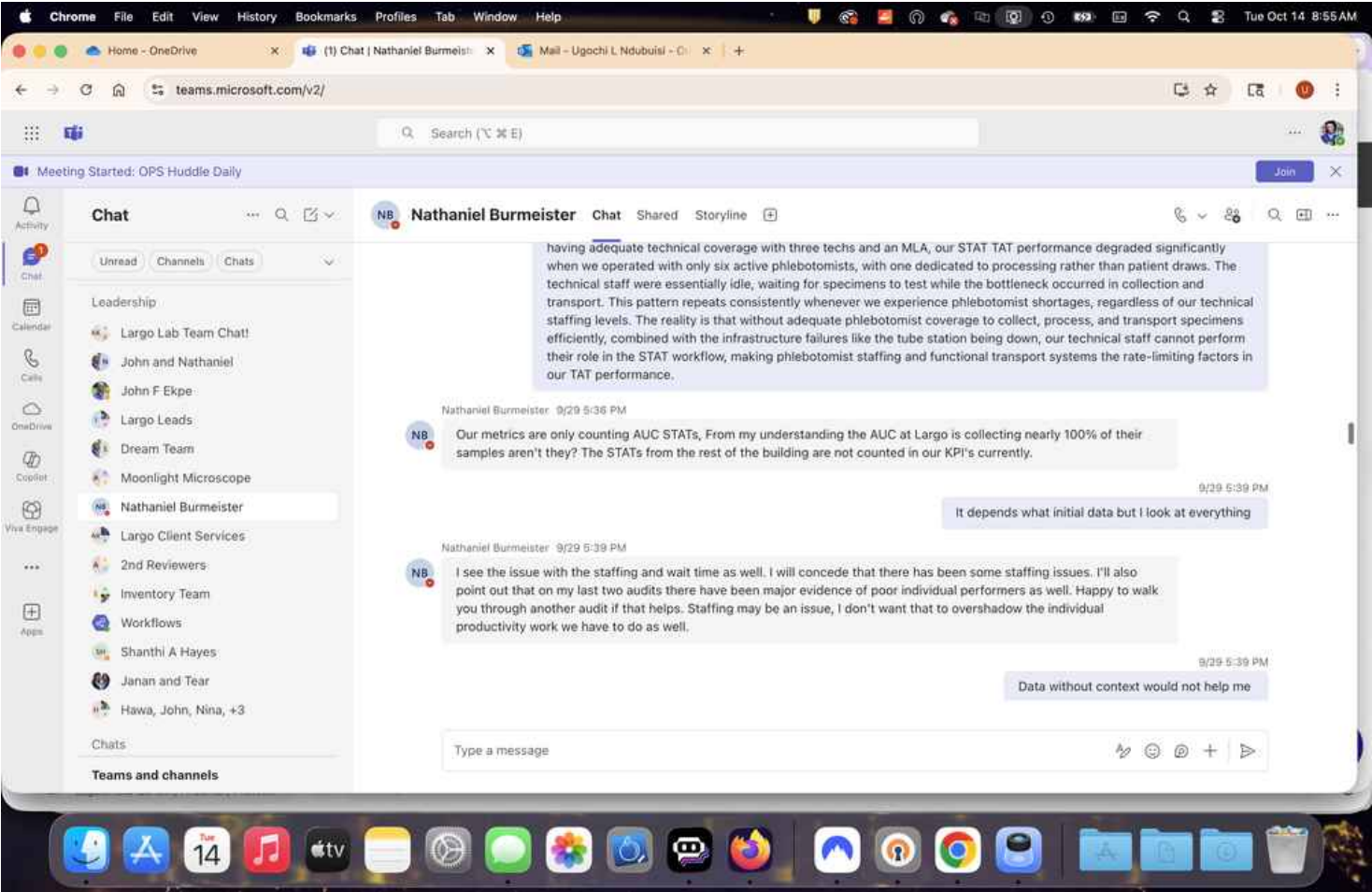


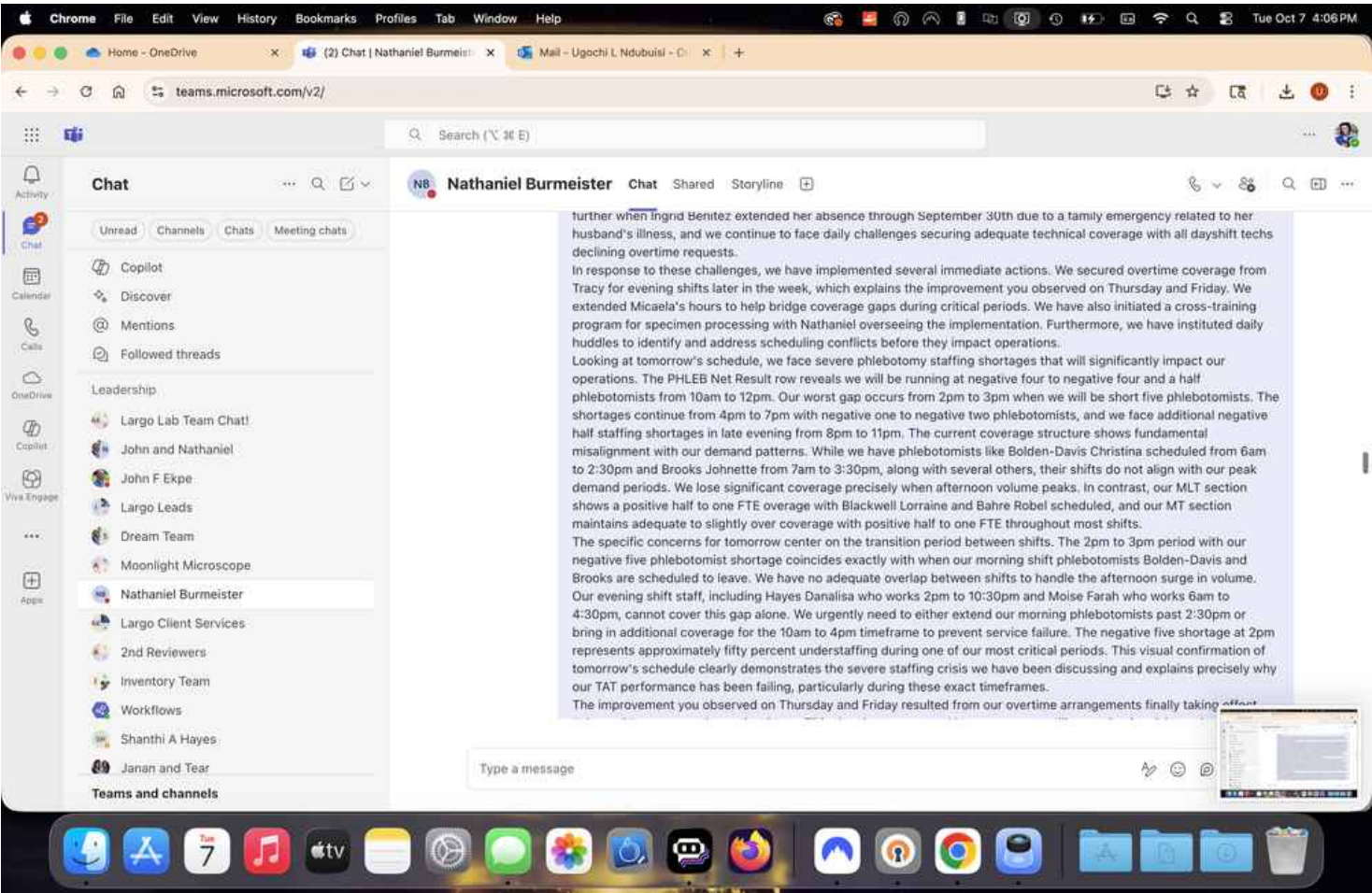


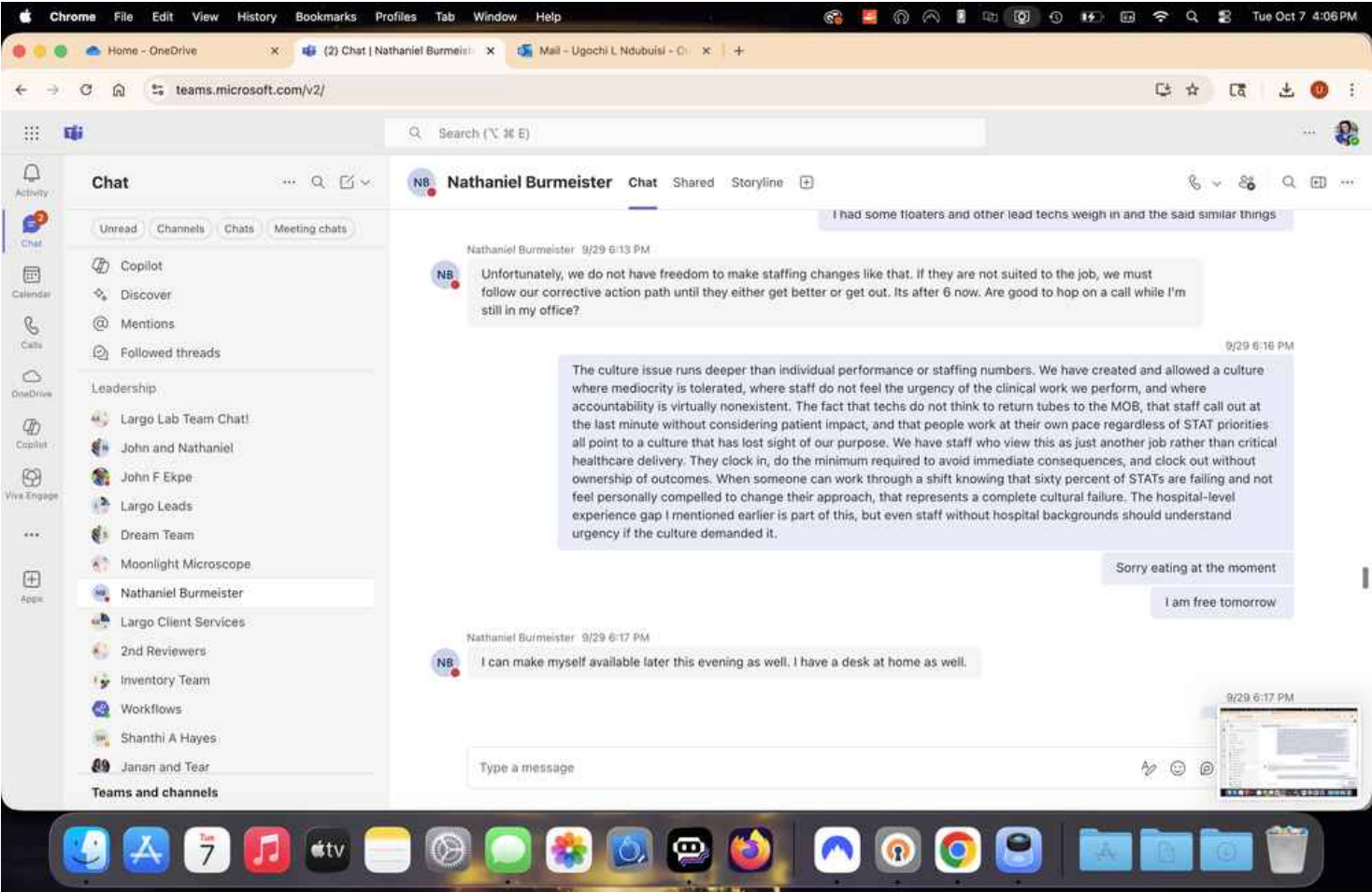




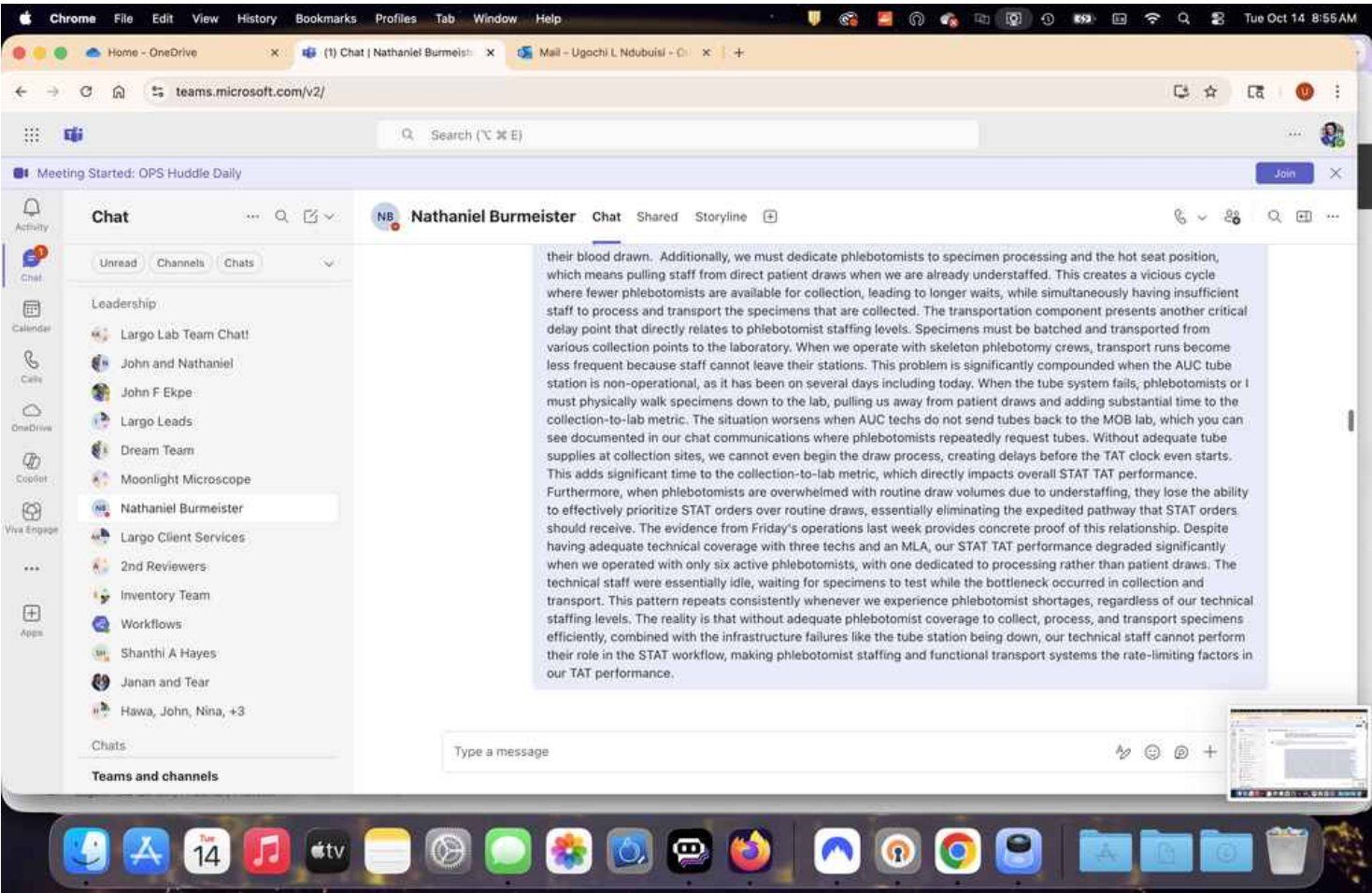








Evidence 13: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.55.37AM



WHISTLEBLOWER RETALIATION EVIDENCE

Largo Laboratory - Kaiser Permanente

EXECUTIVE SUMMARY

This document presents chronological evidence of retaliation against Ugochi L. Ndubuisi for protected whistleblowing activity. The timeline shows:

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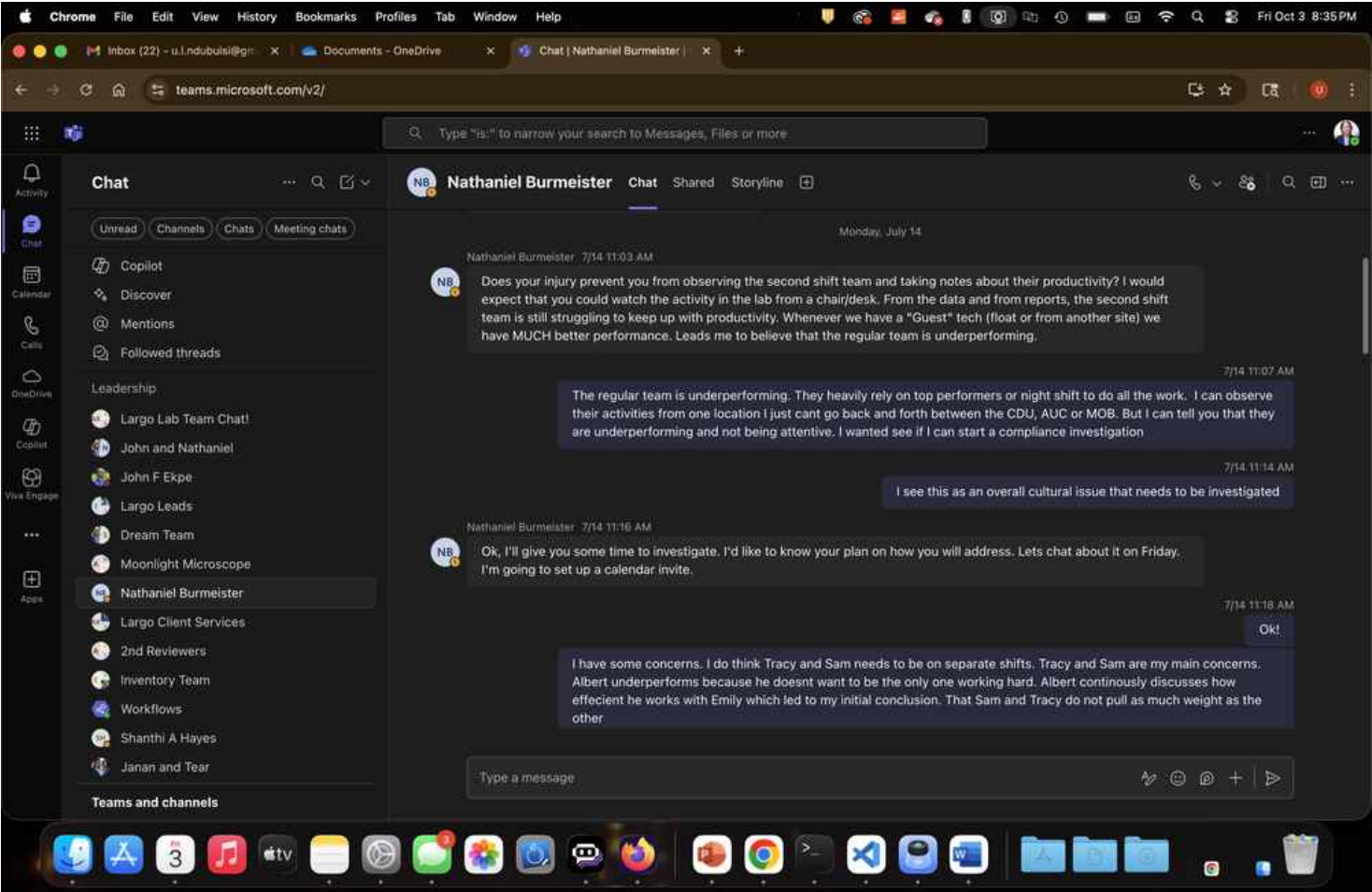
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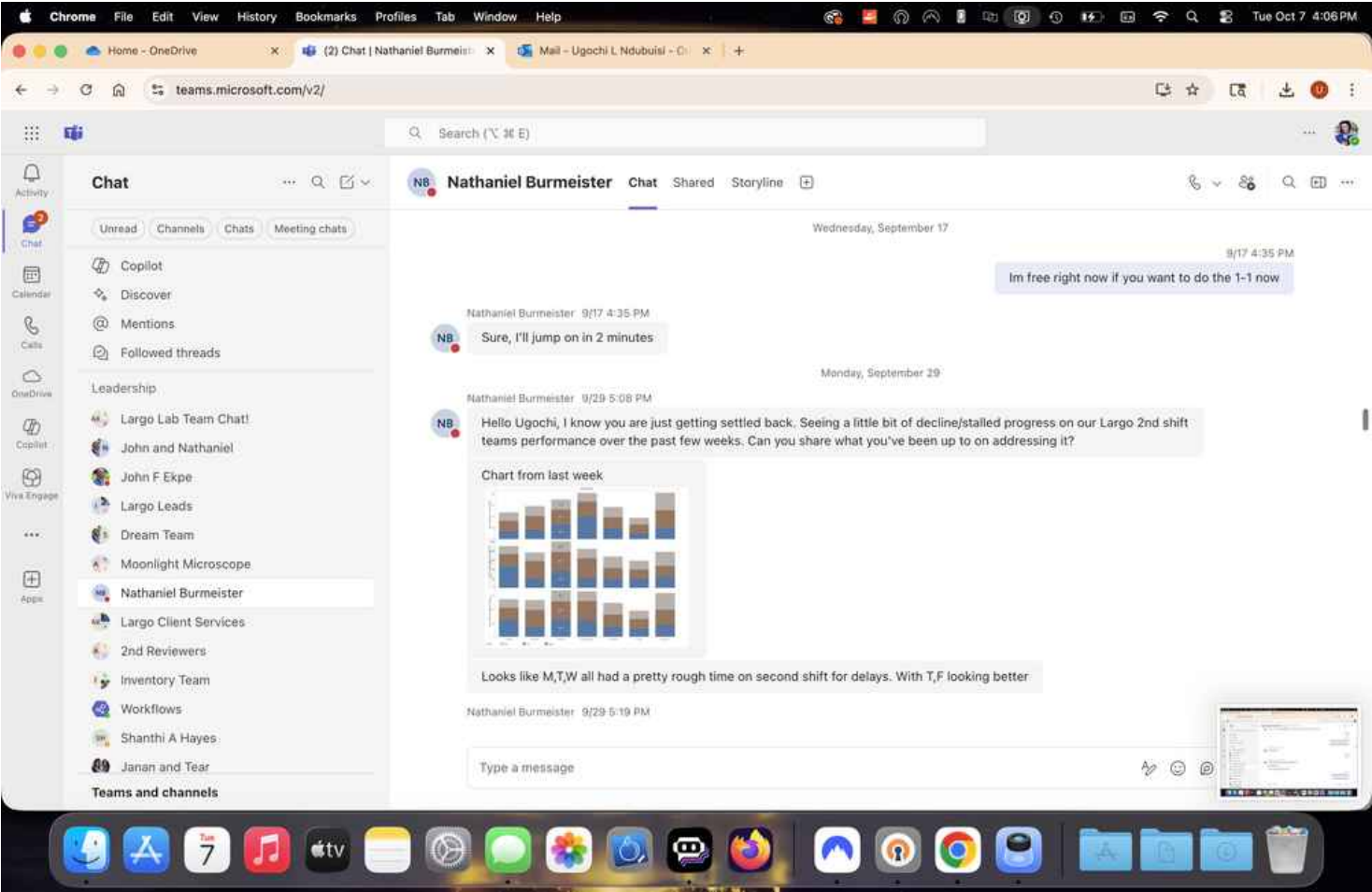
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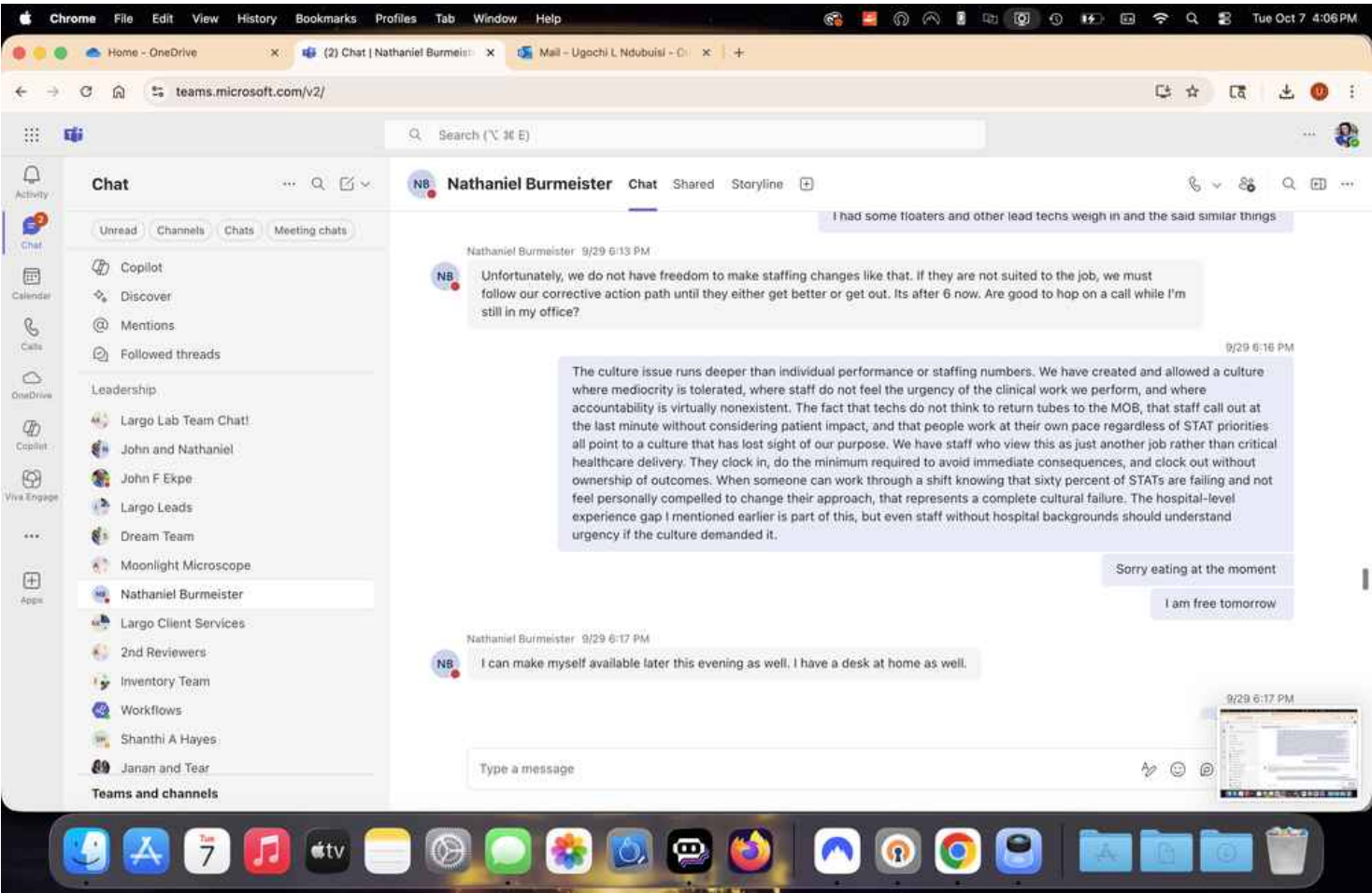
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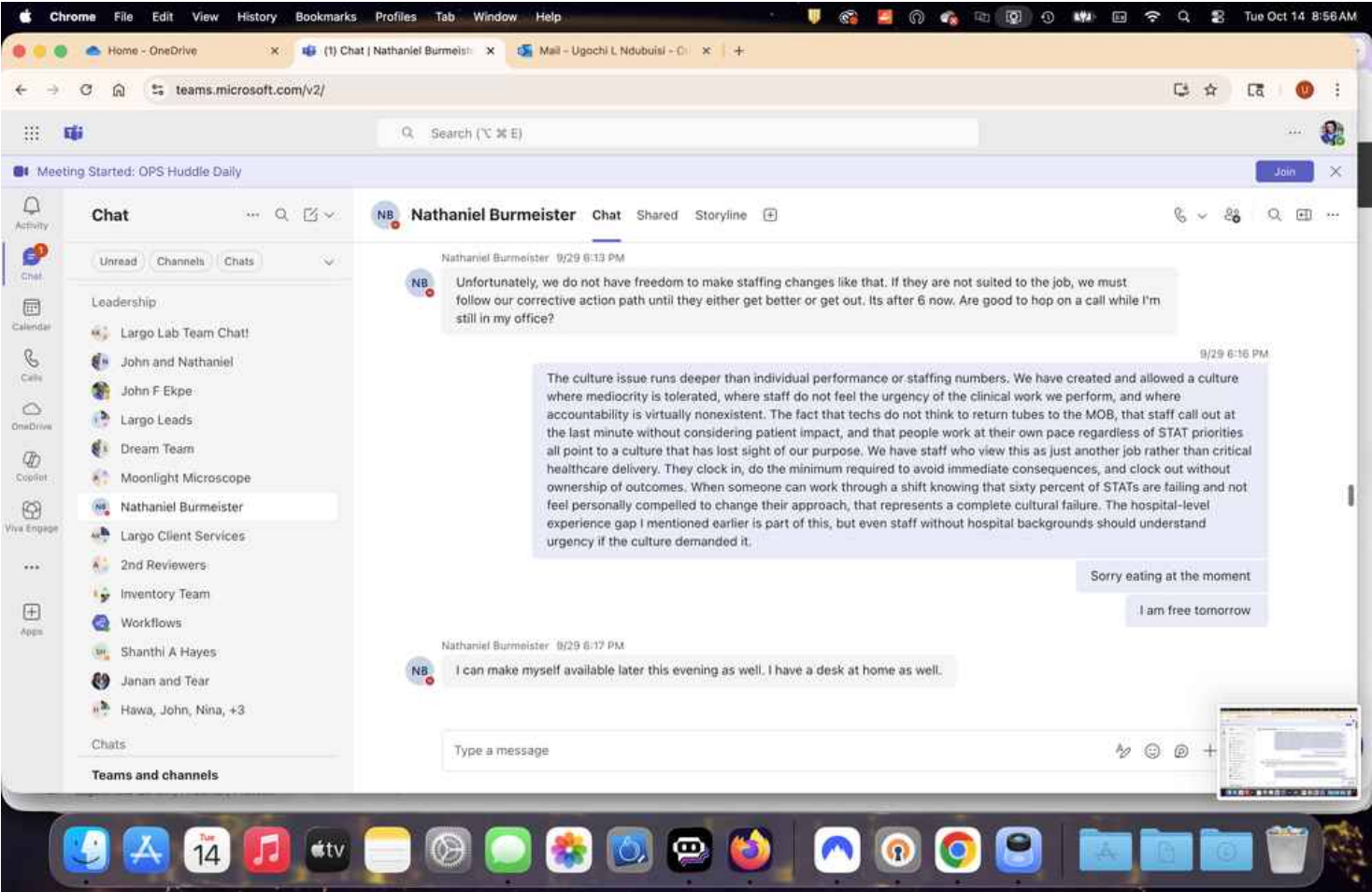
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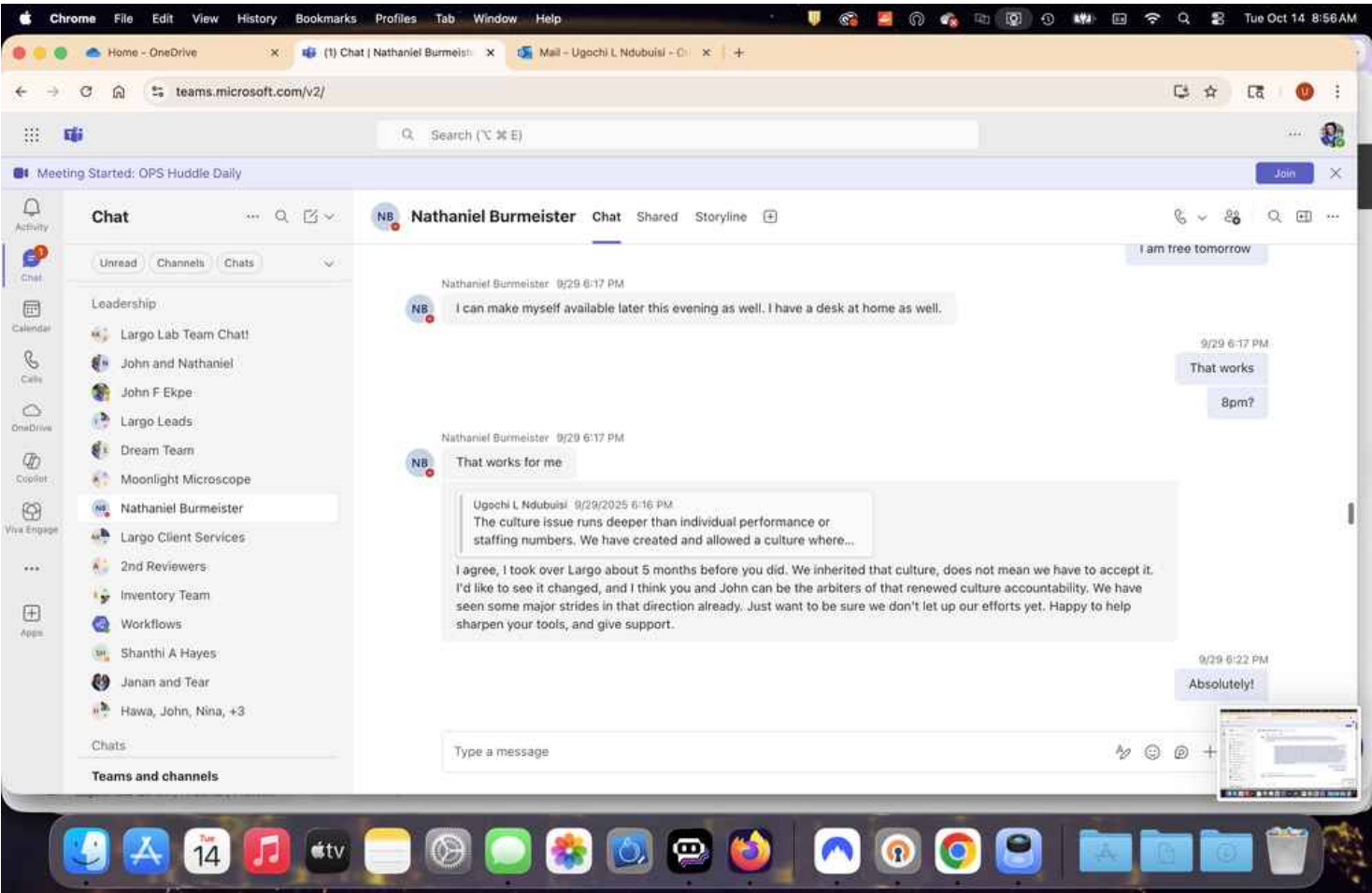
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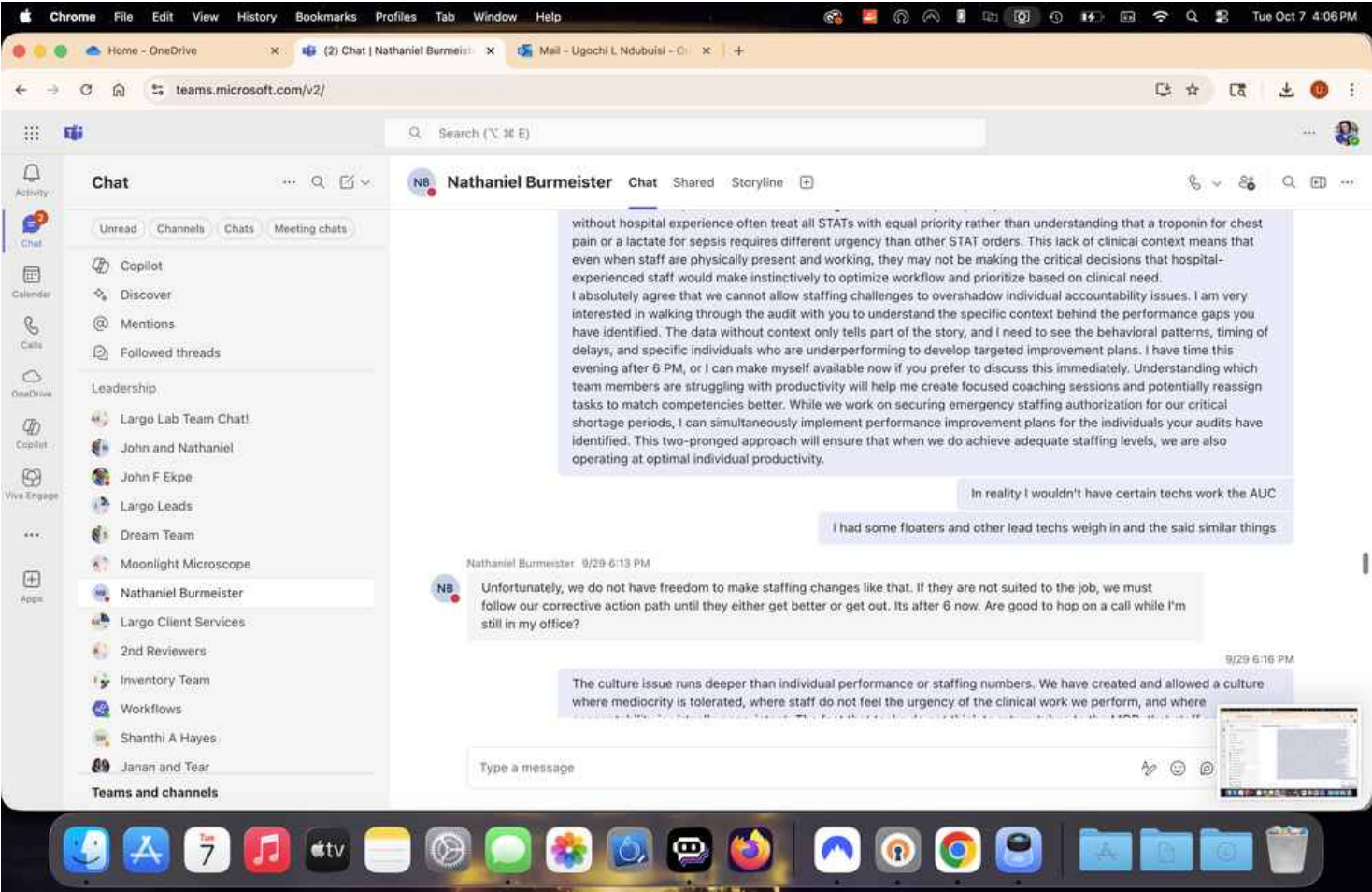


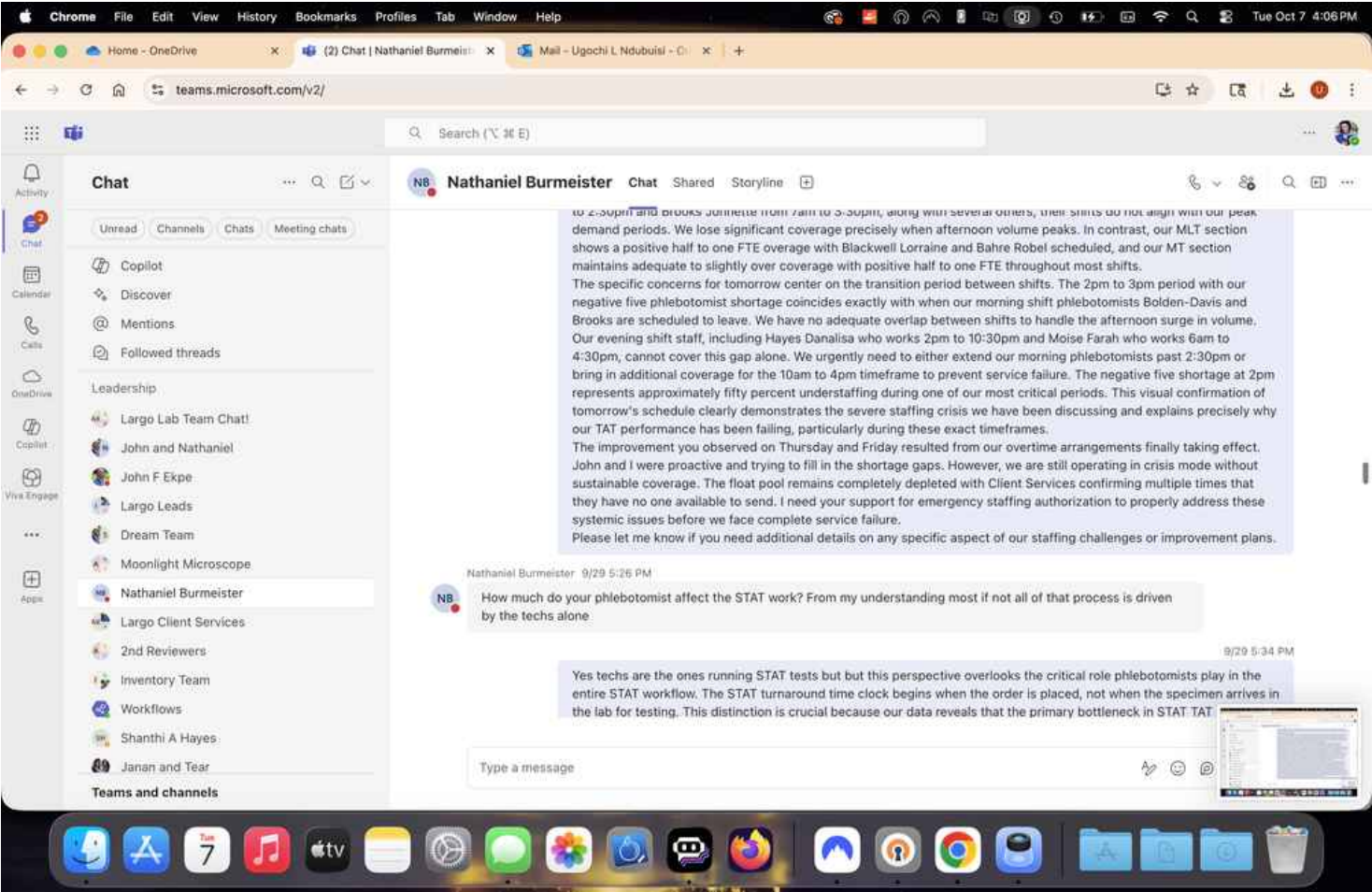


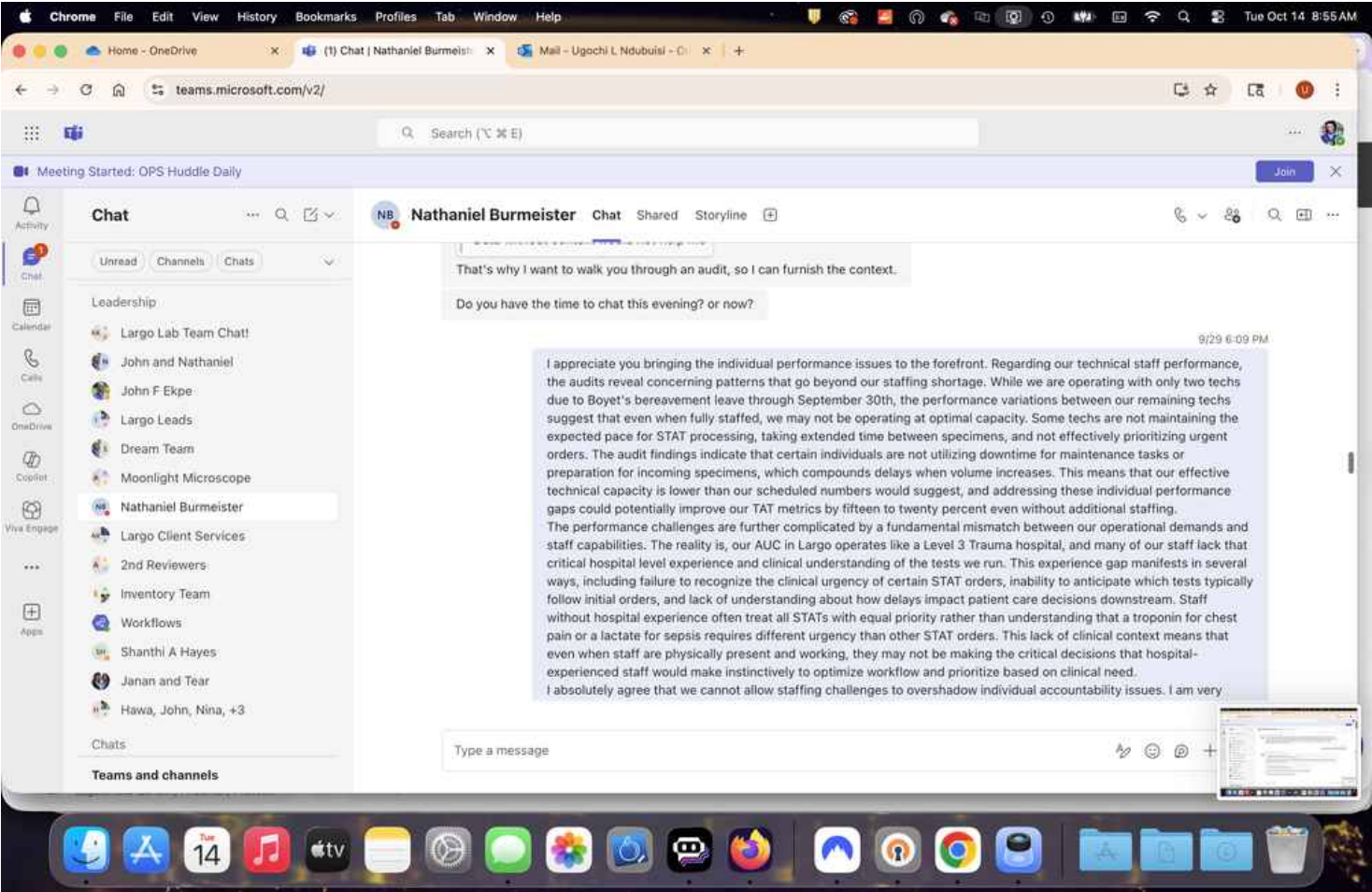


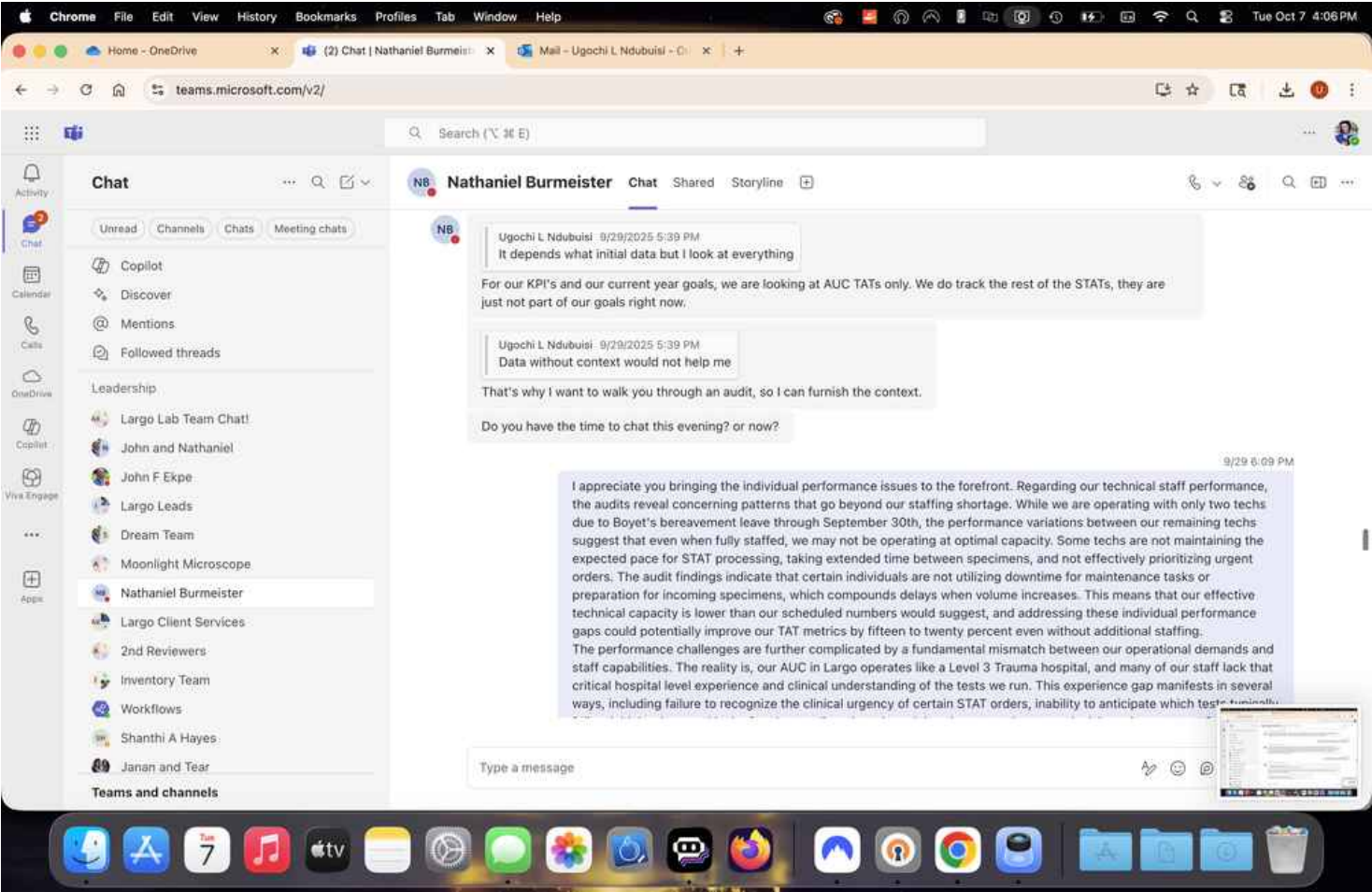


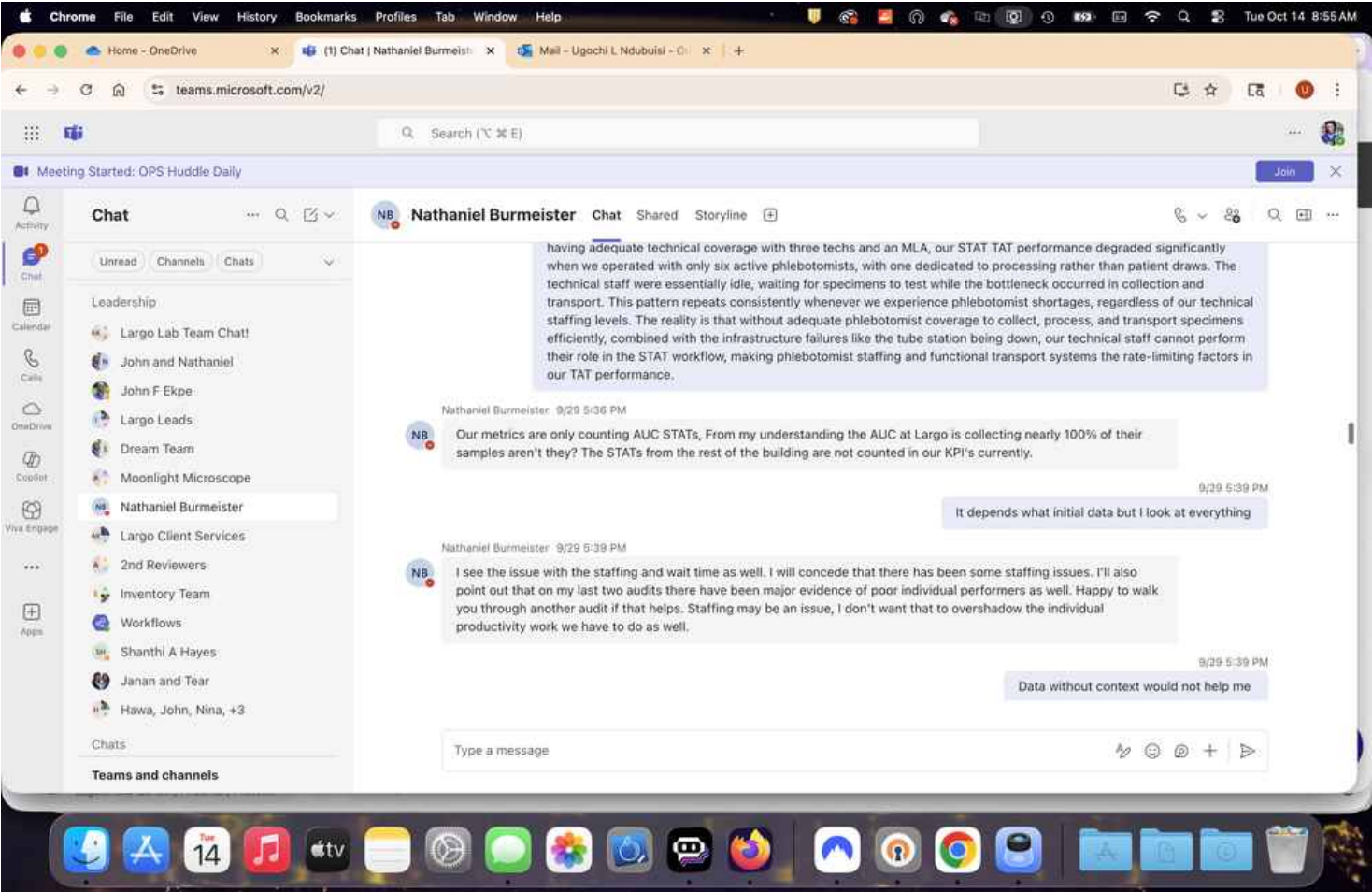


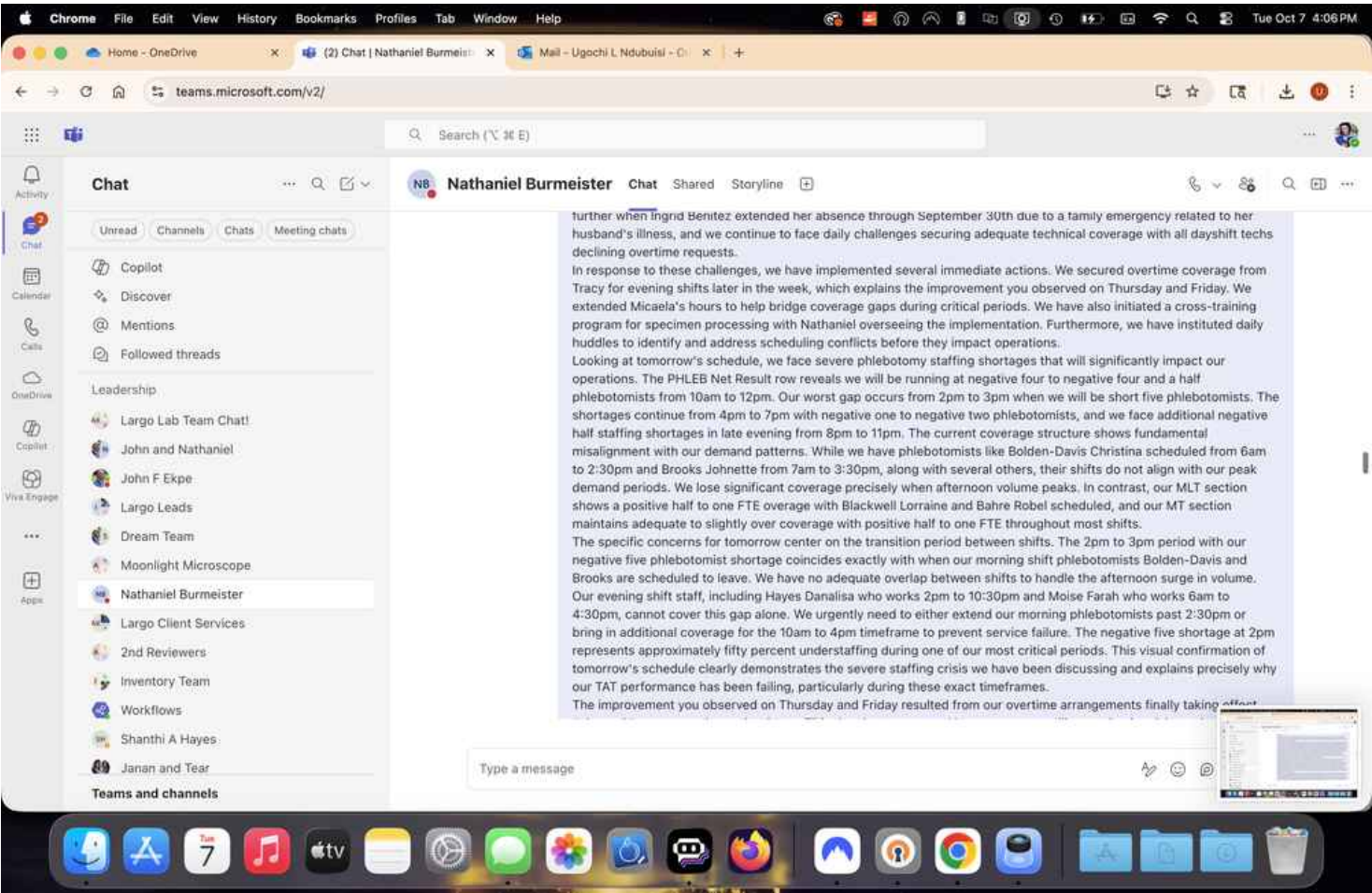


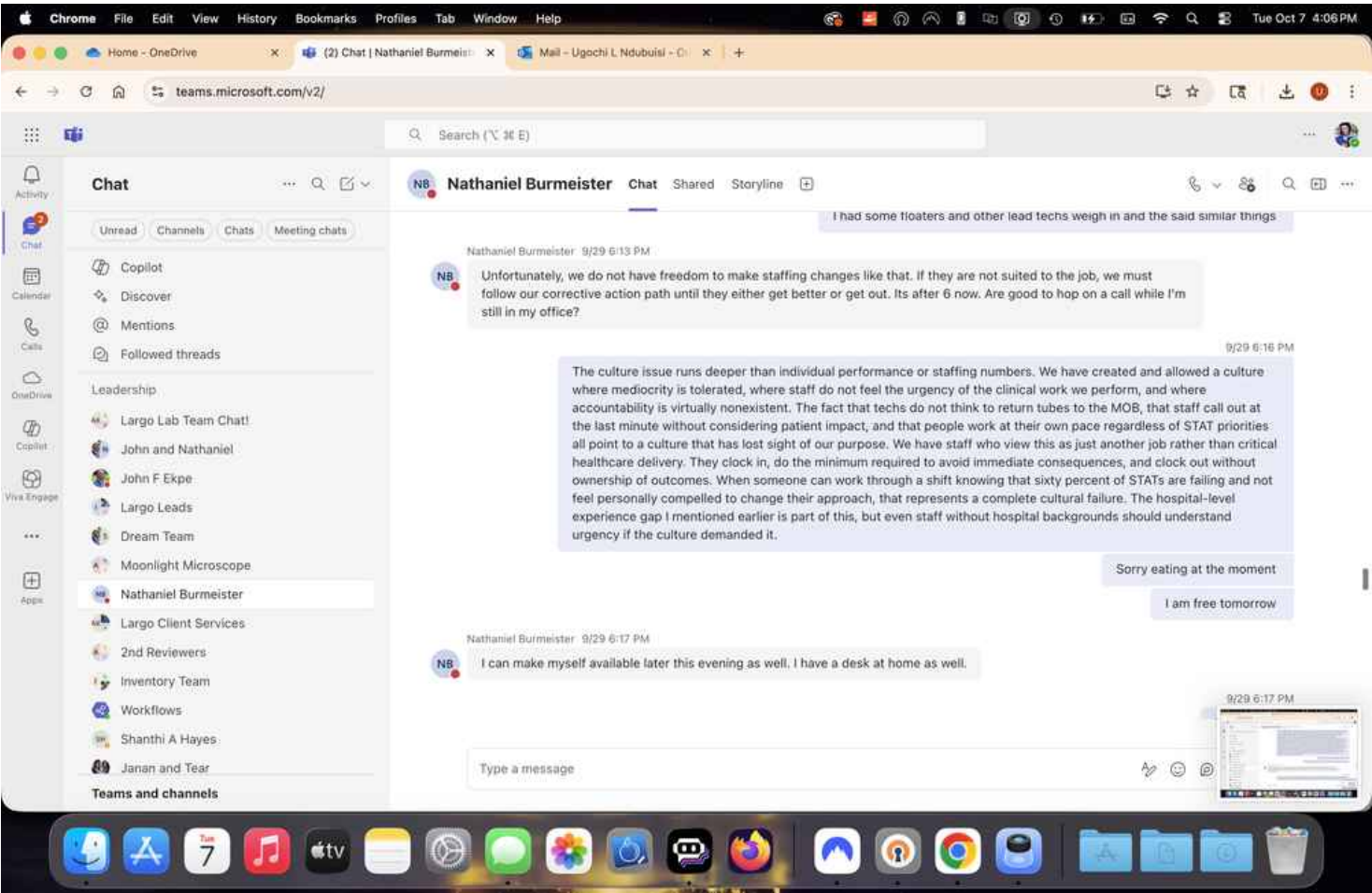




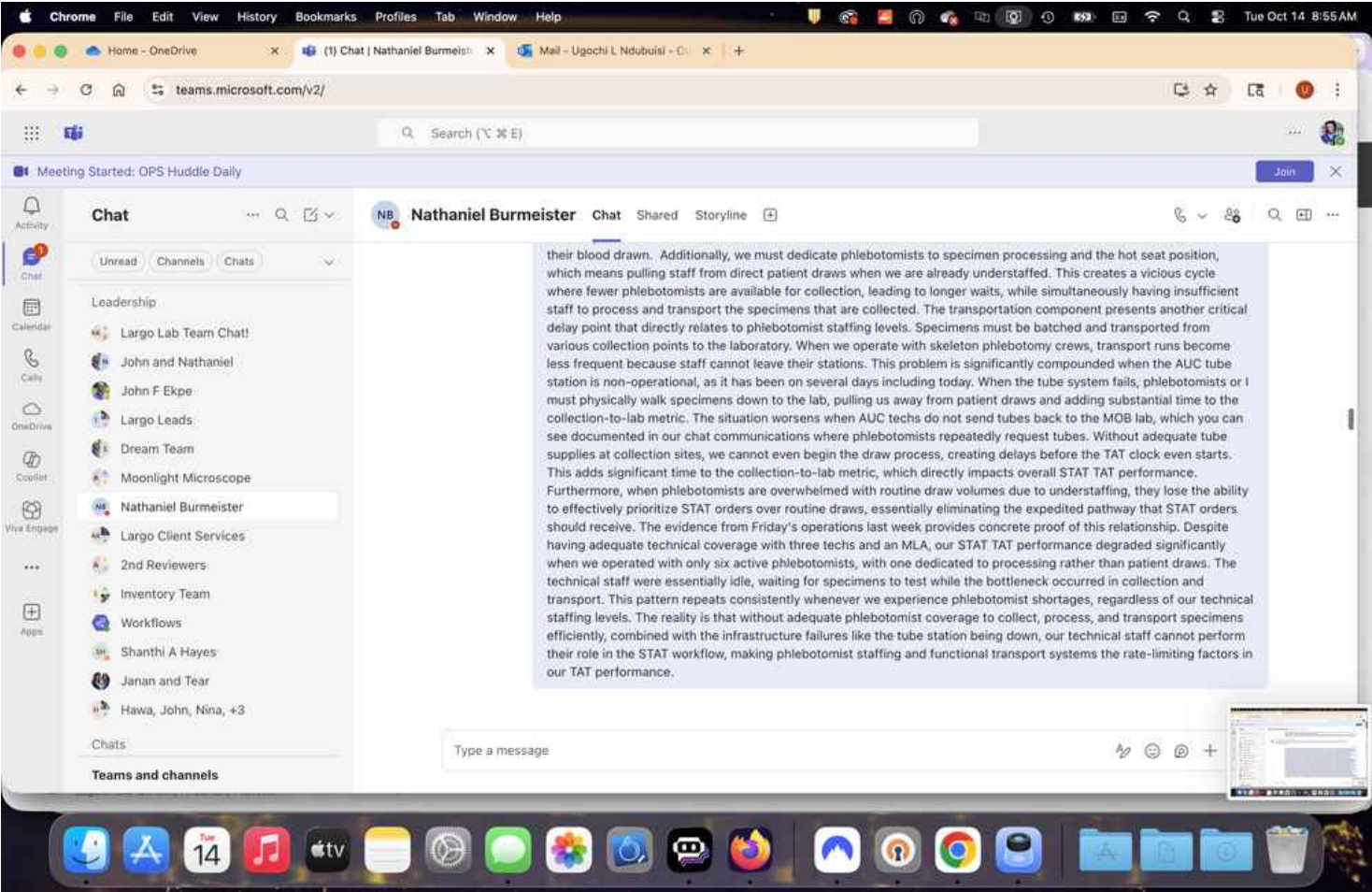




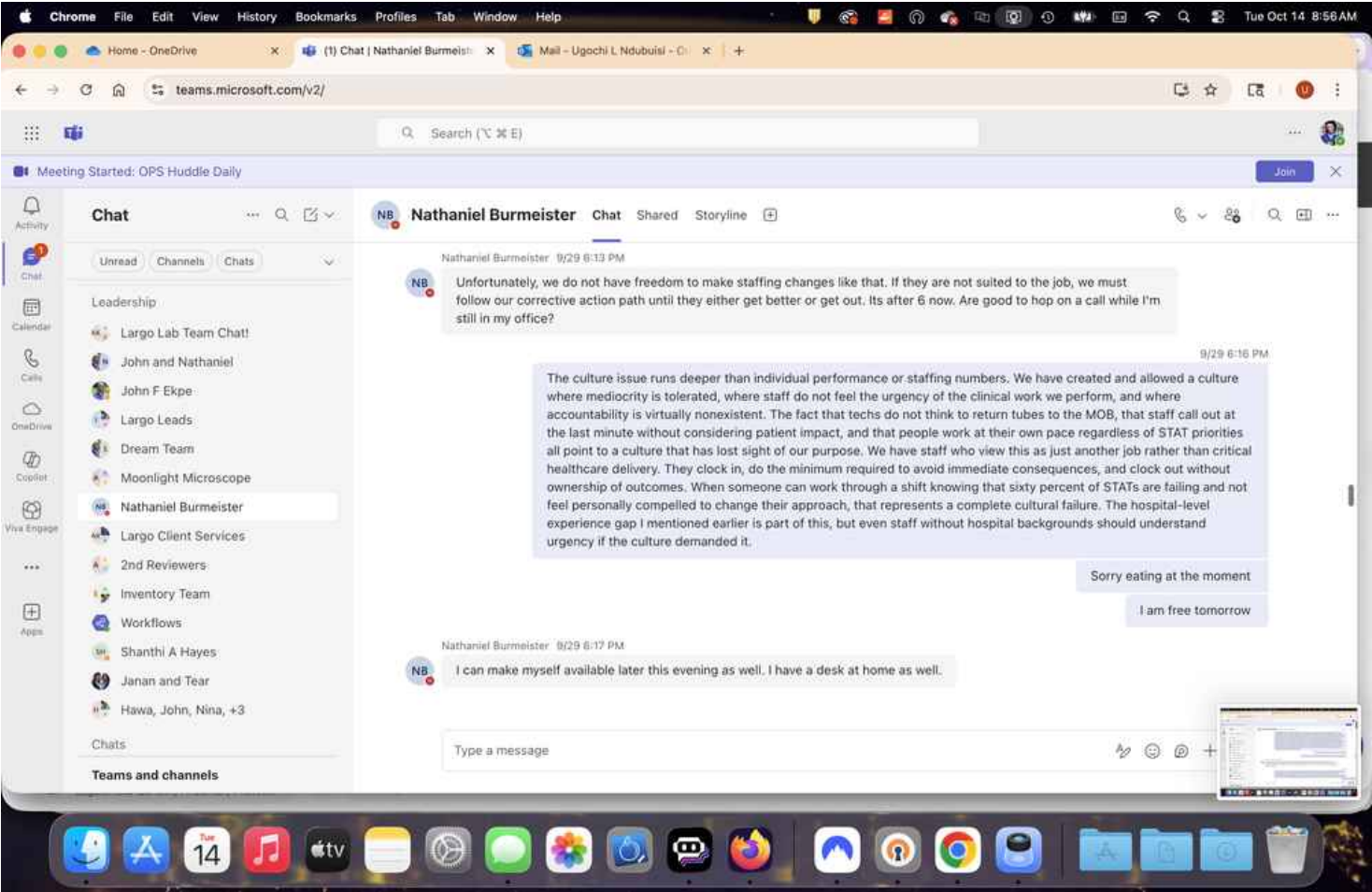




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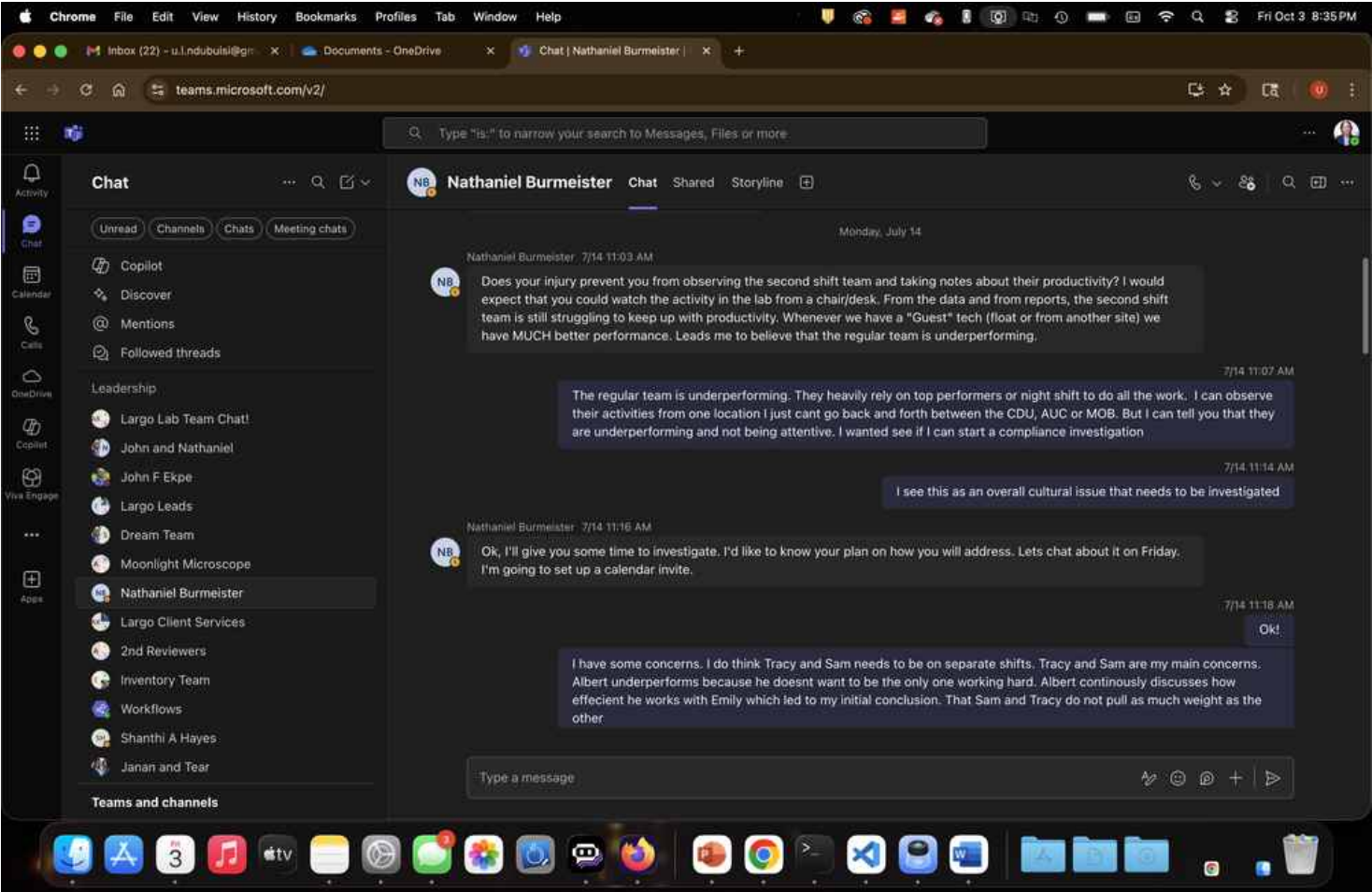
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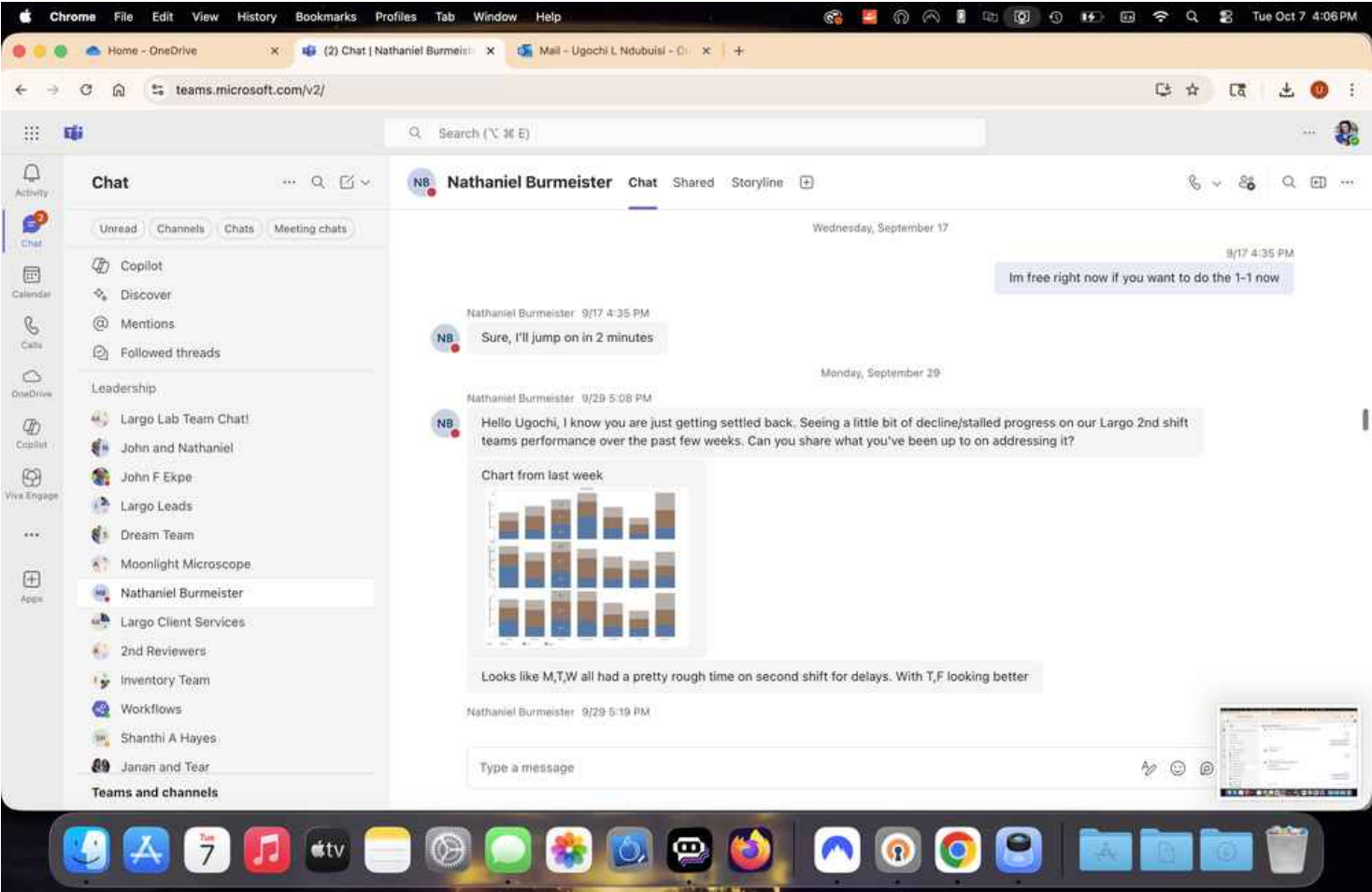
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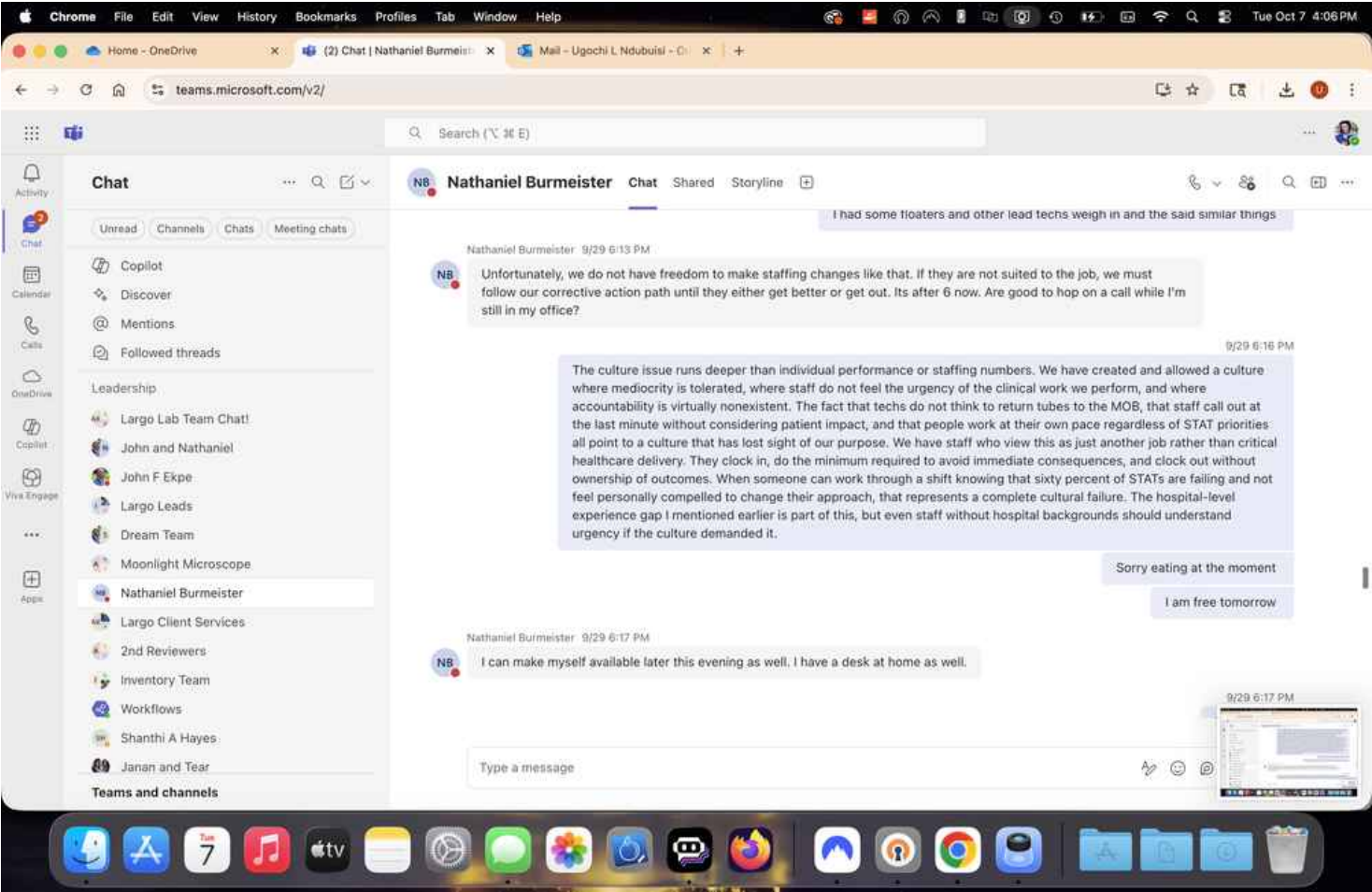
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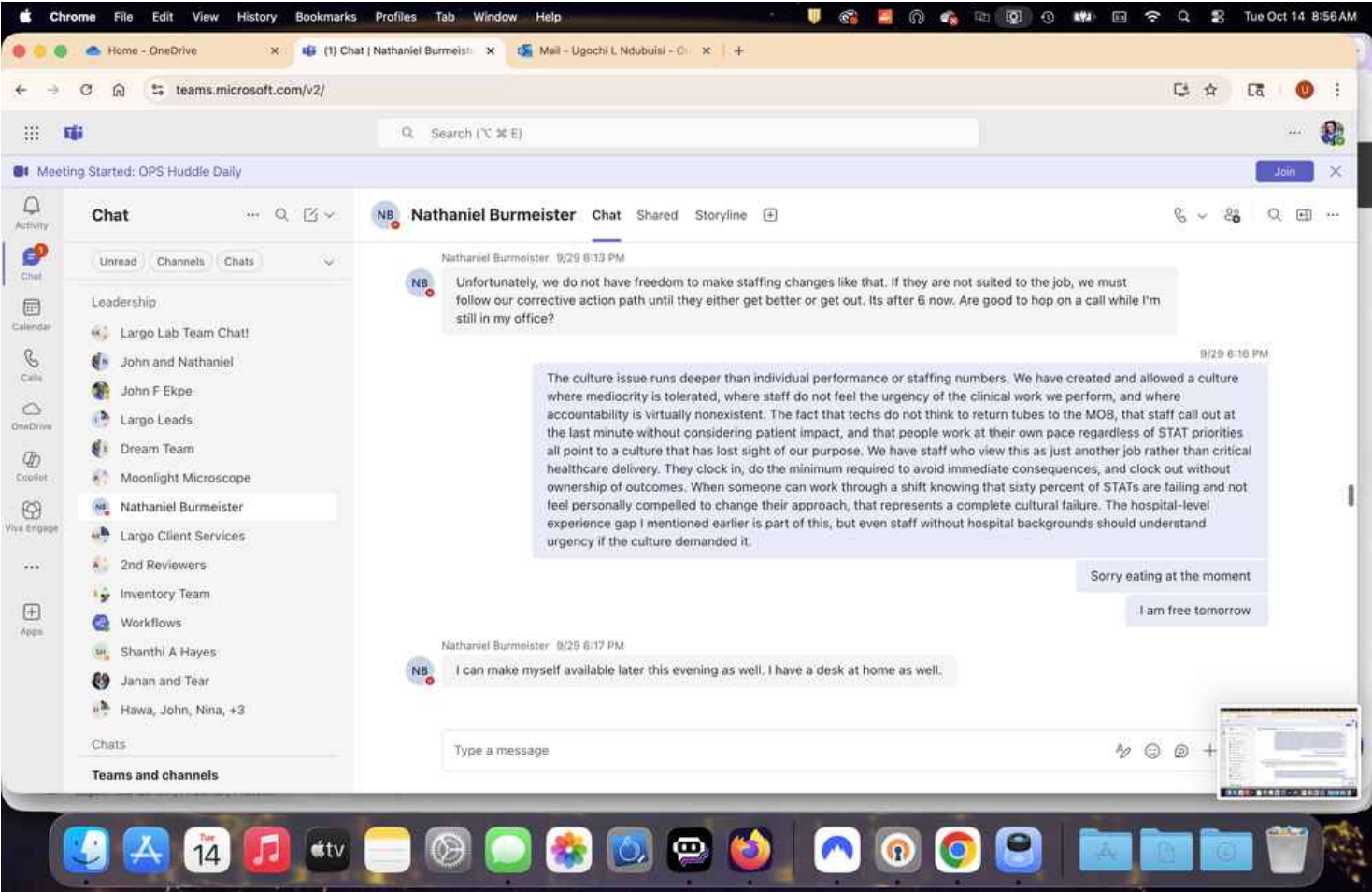
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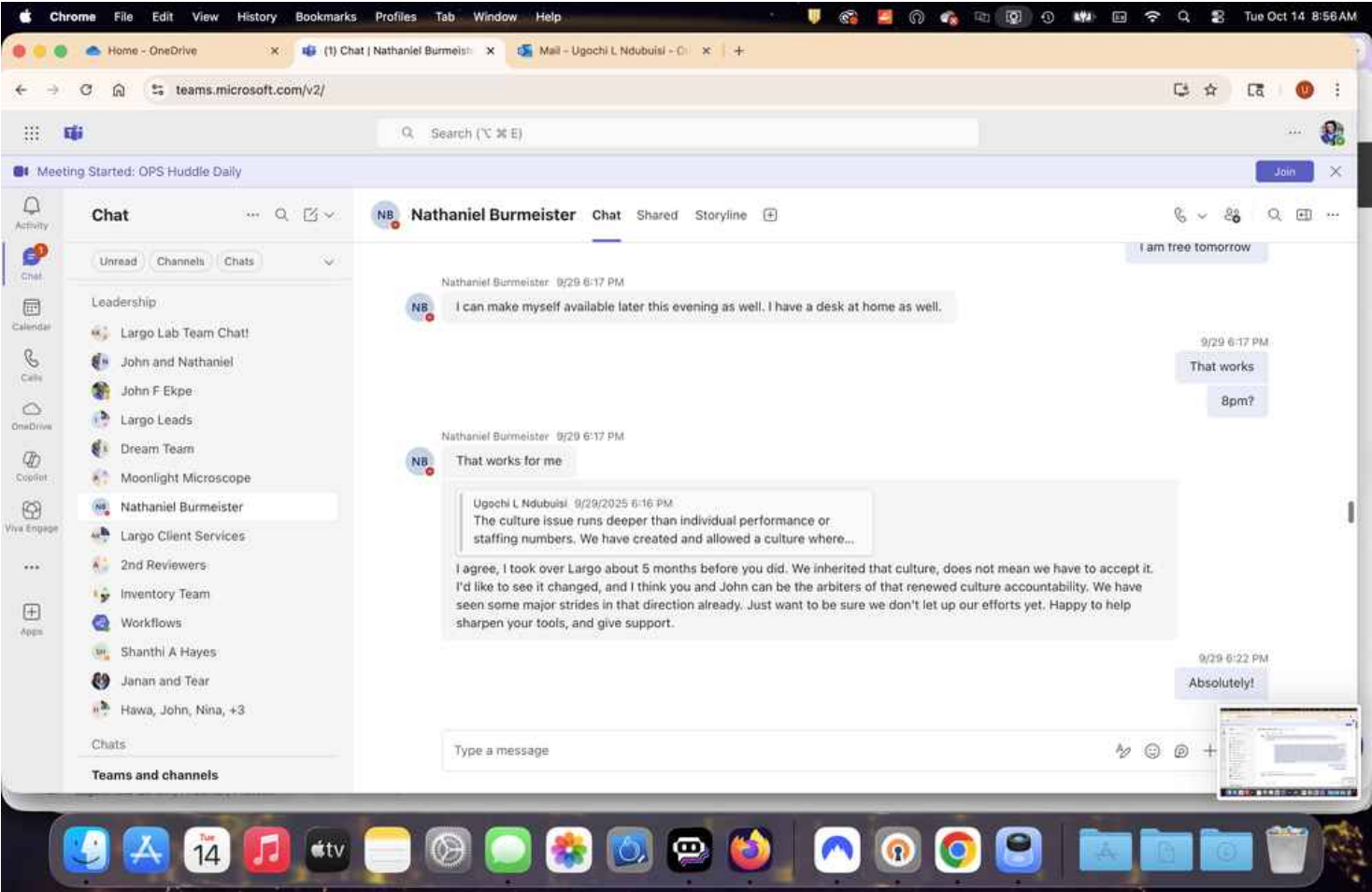
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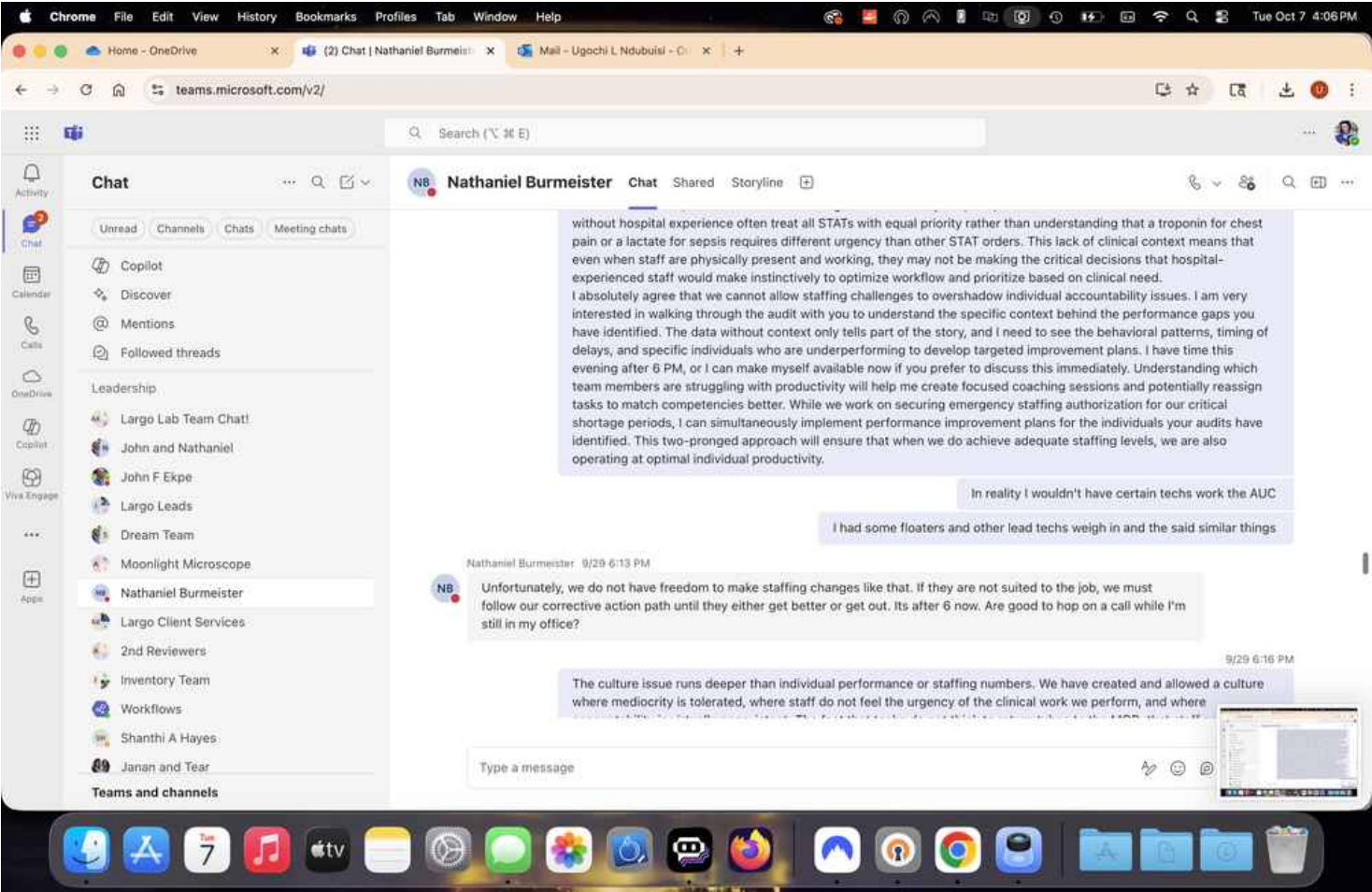


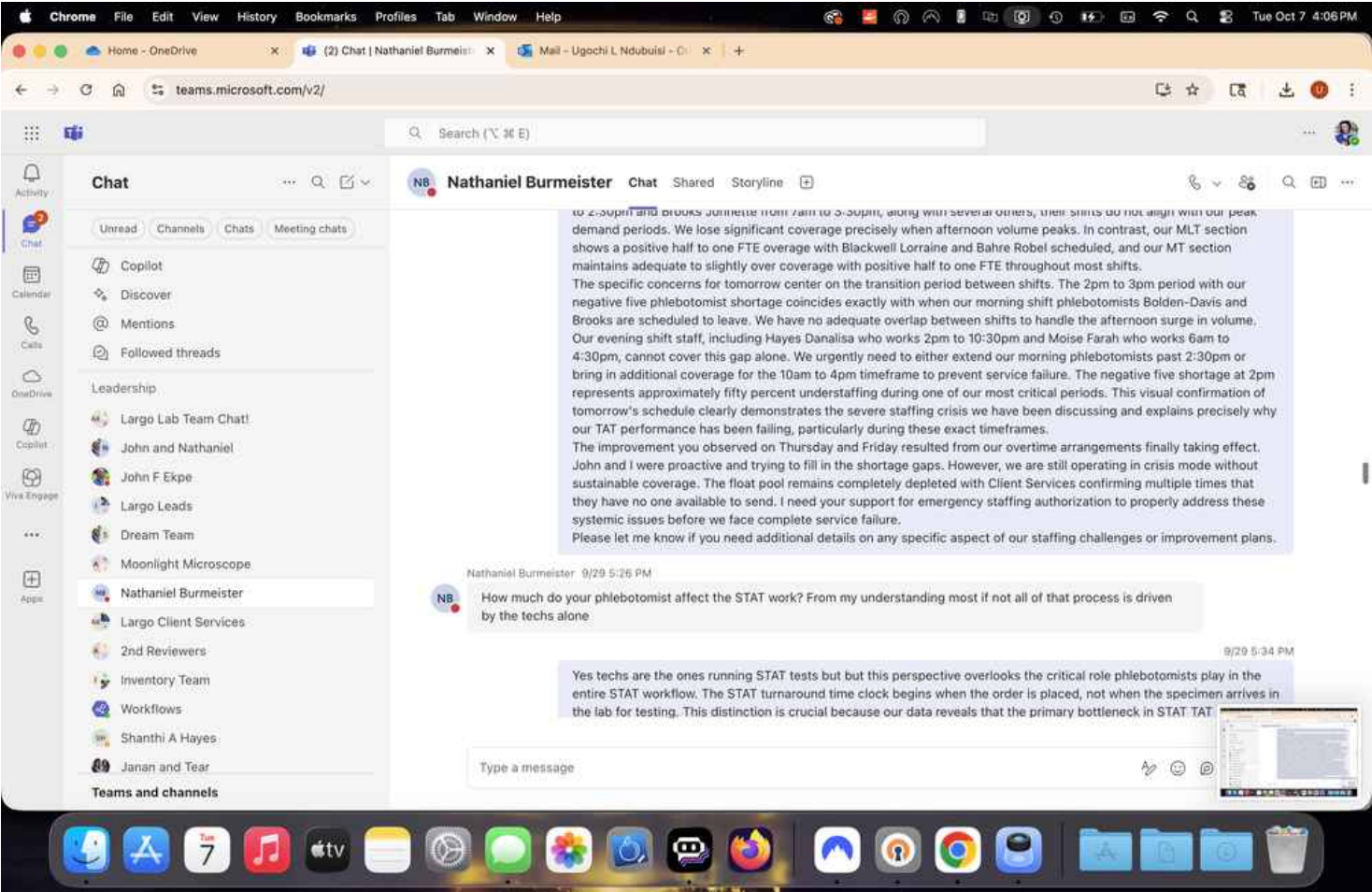




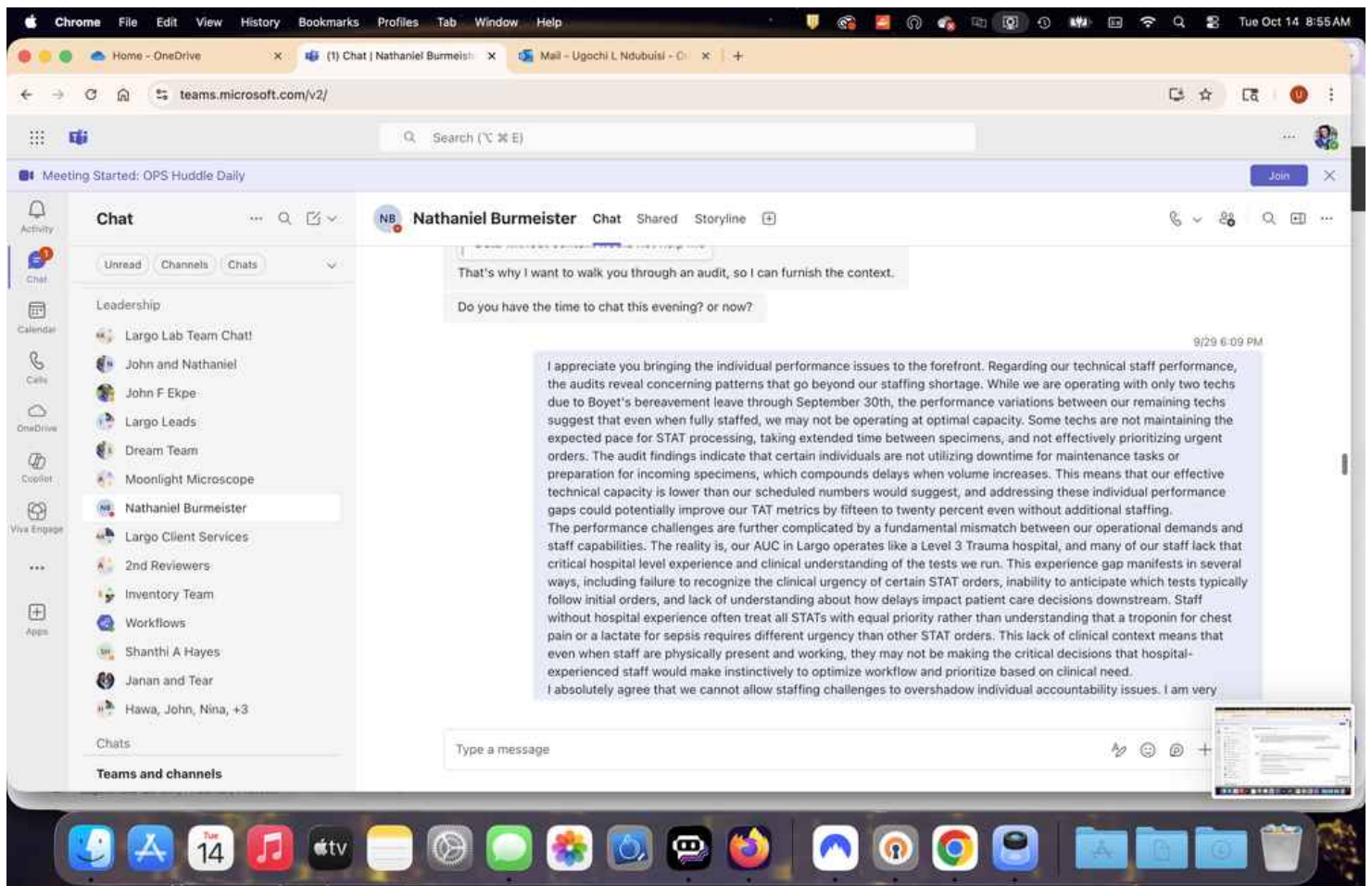


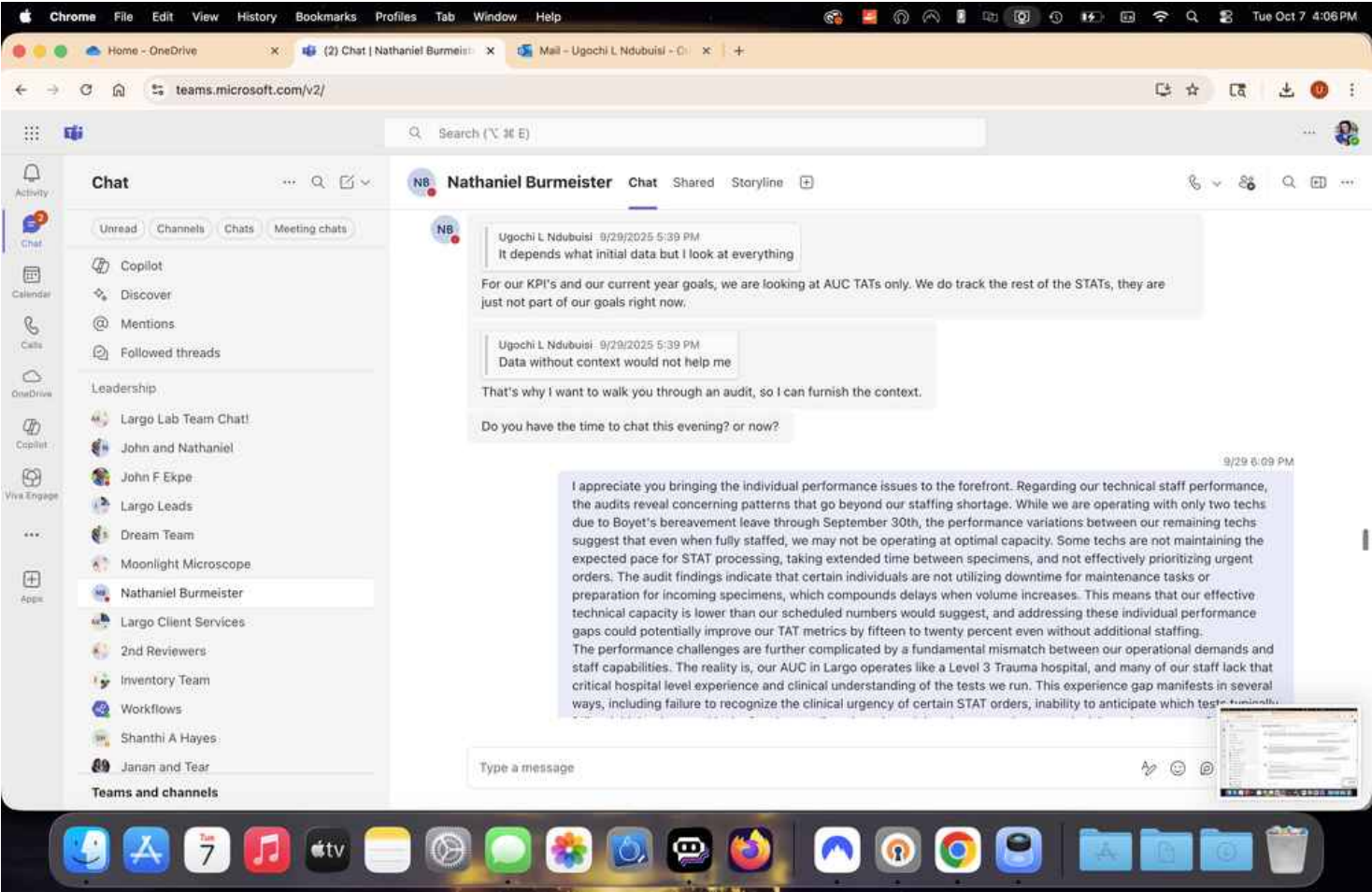


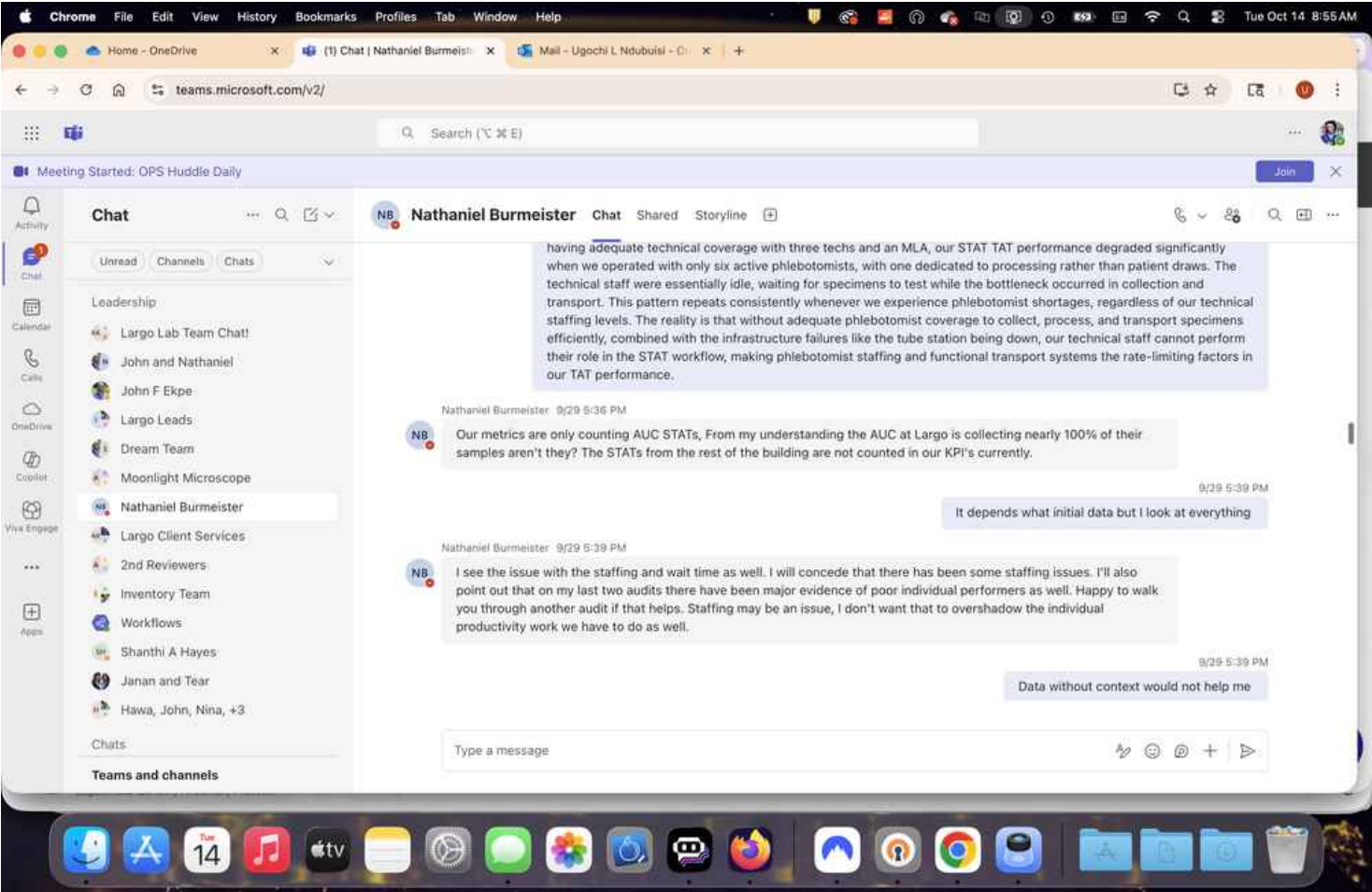


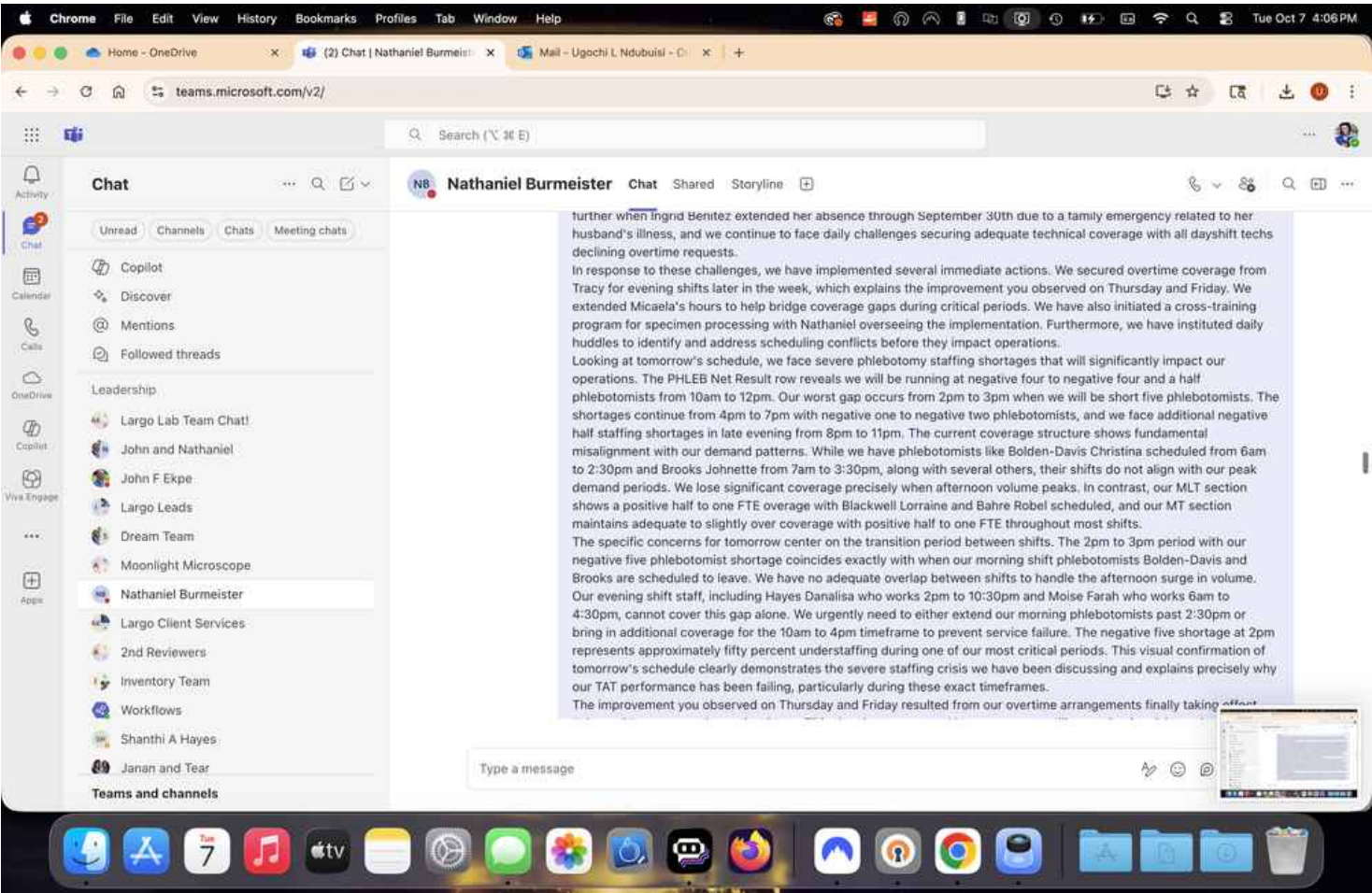


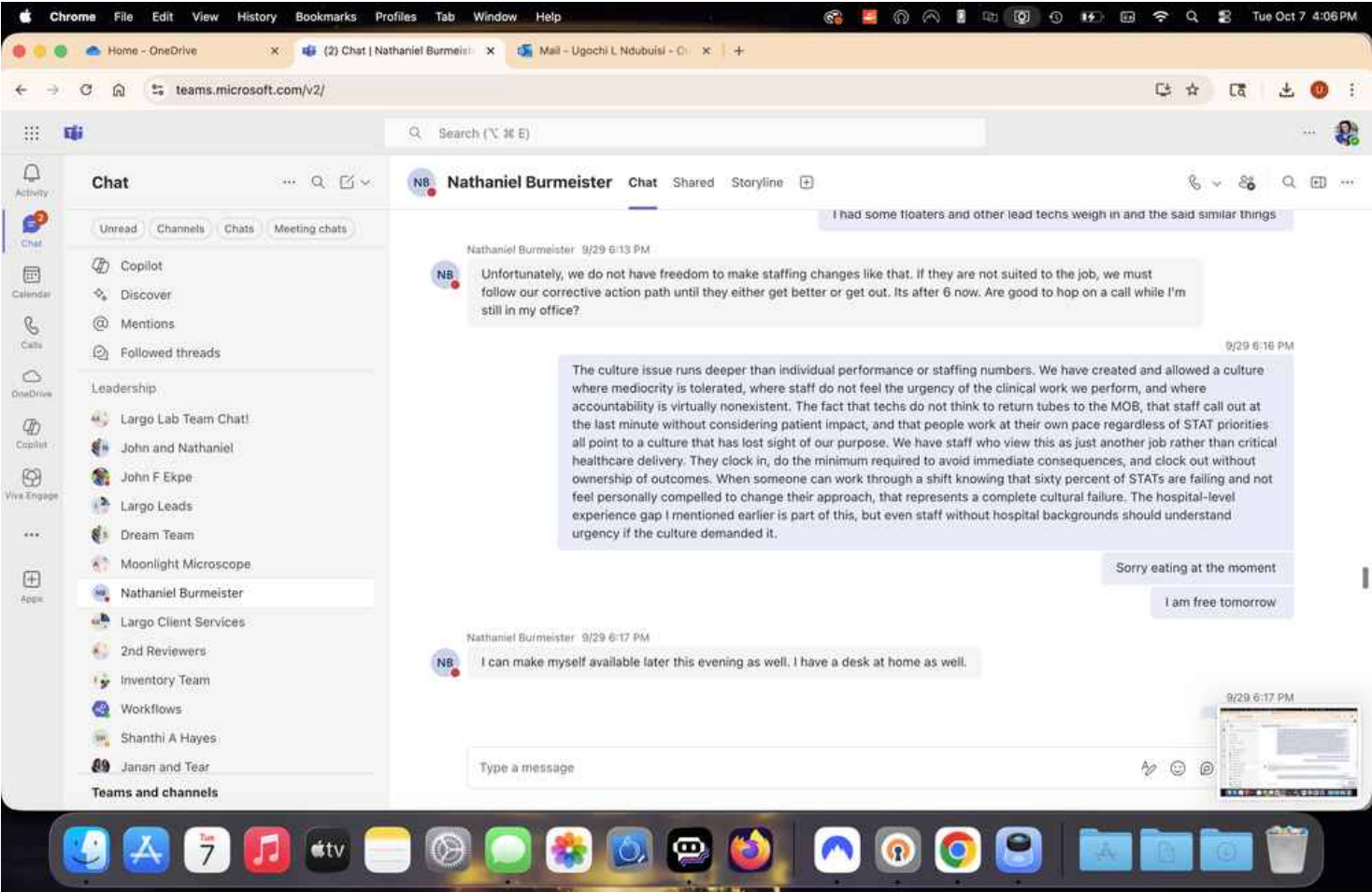
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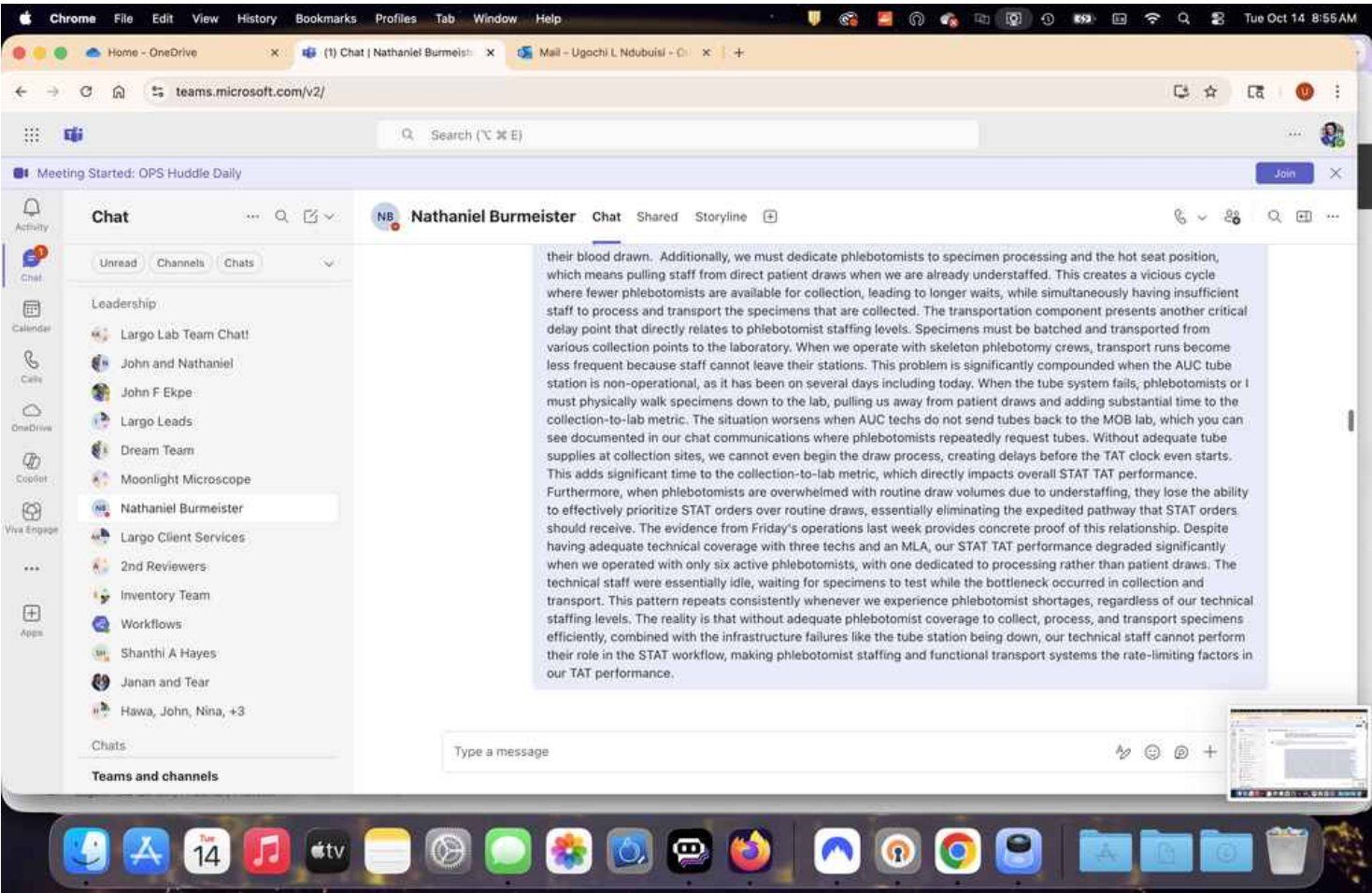




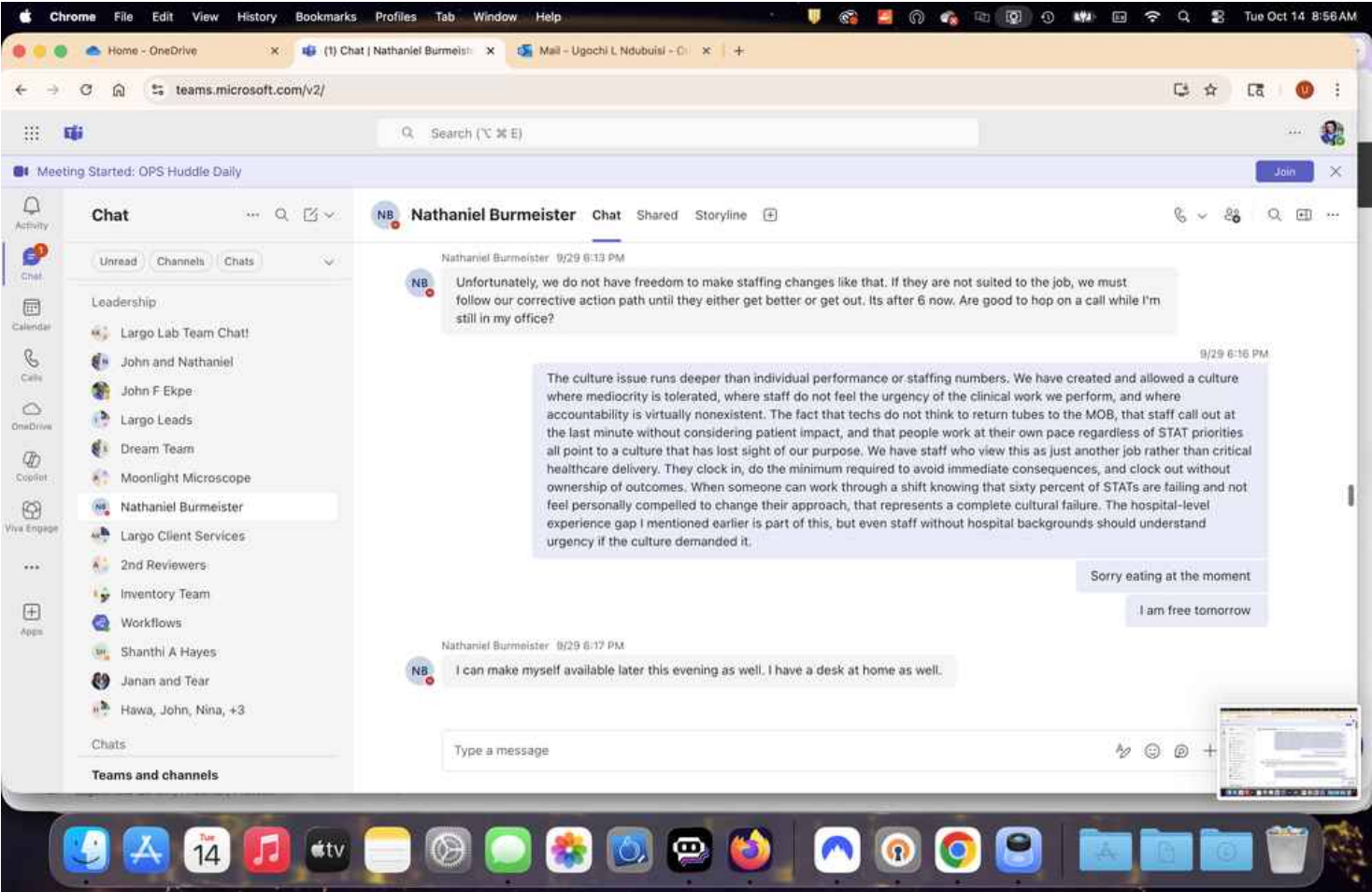


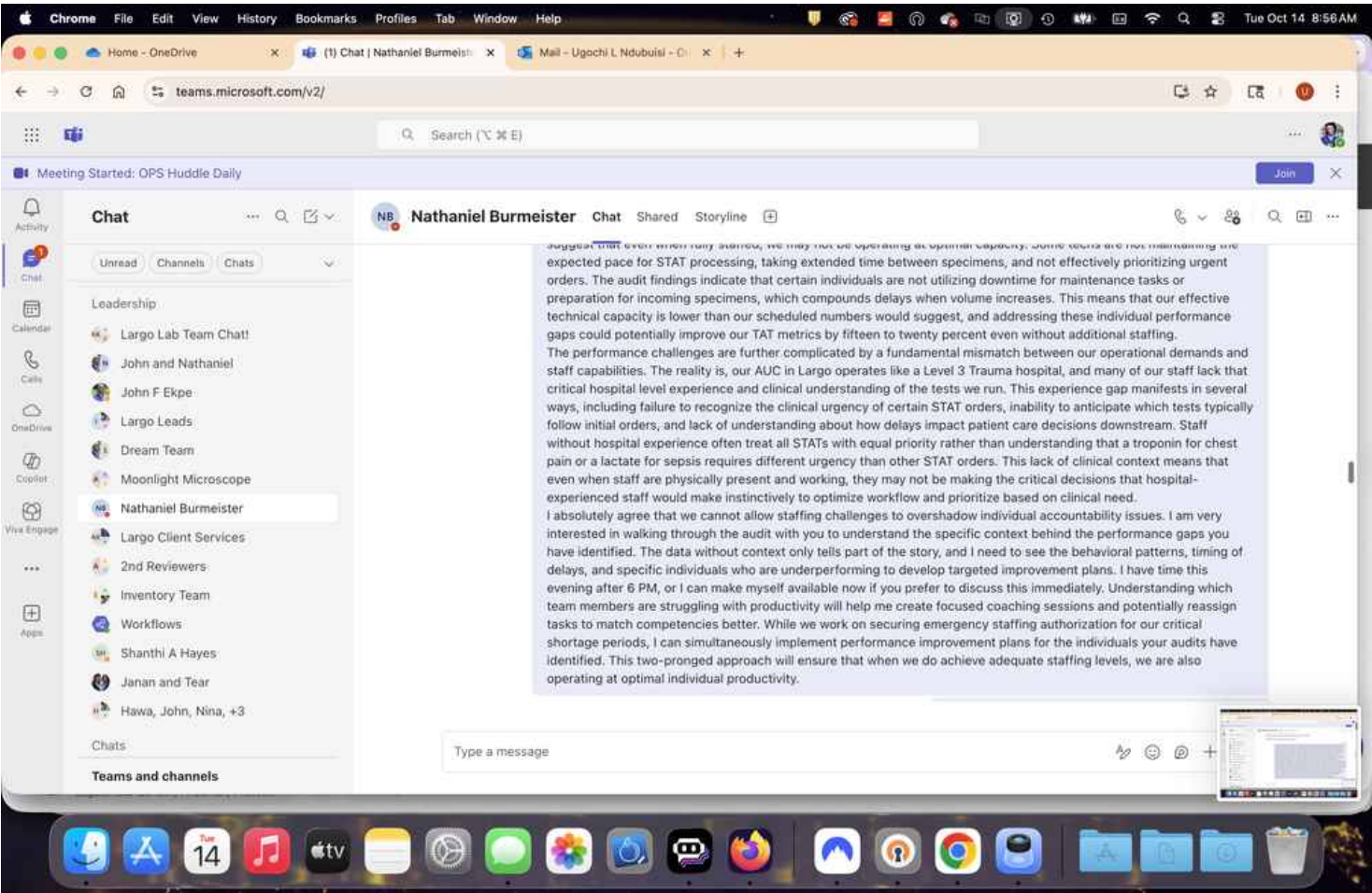






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WHISTLEBLOWER RETALIATION EVIDENCE

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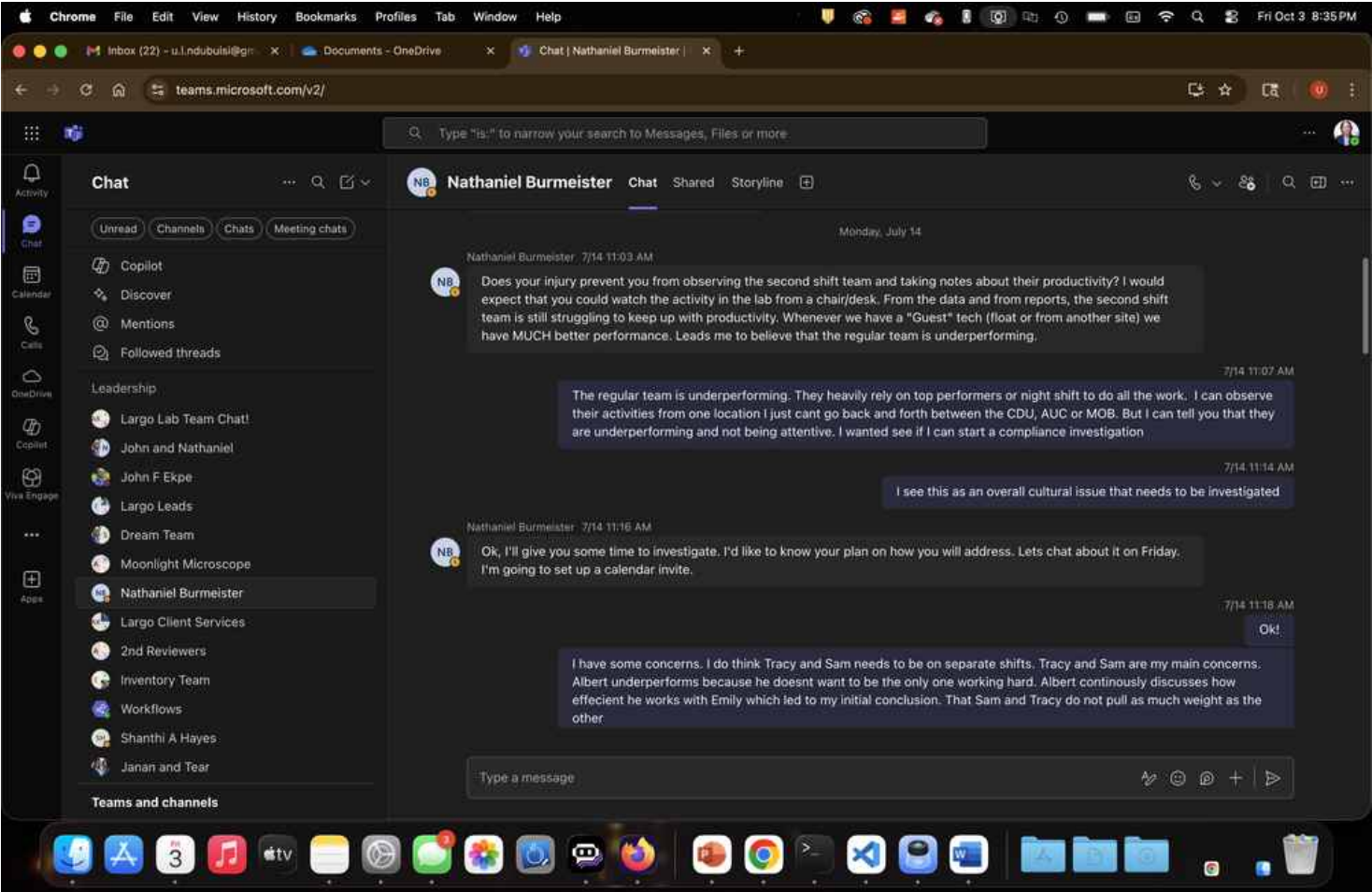
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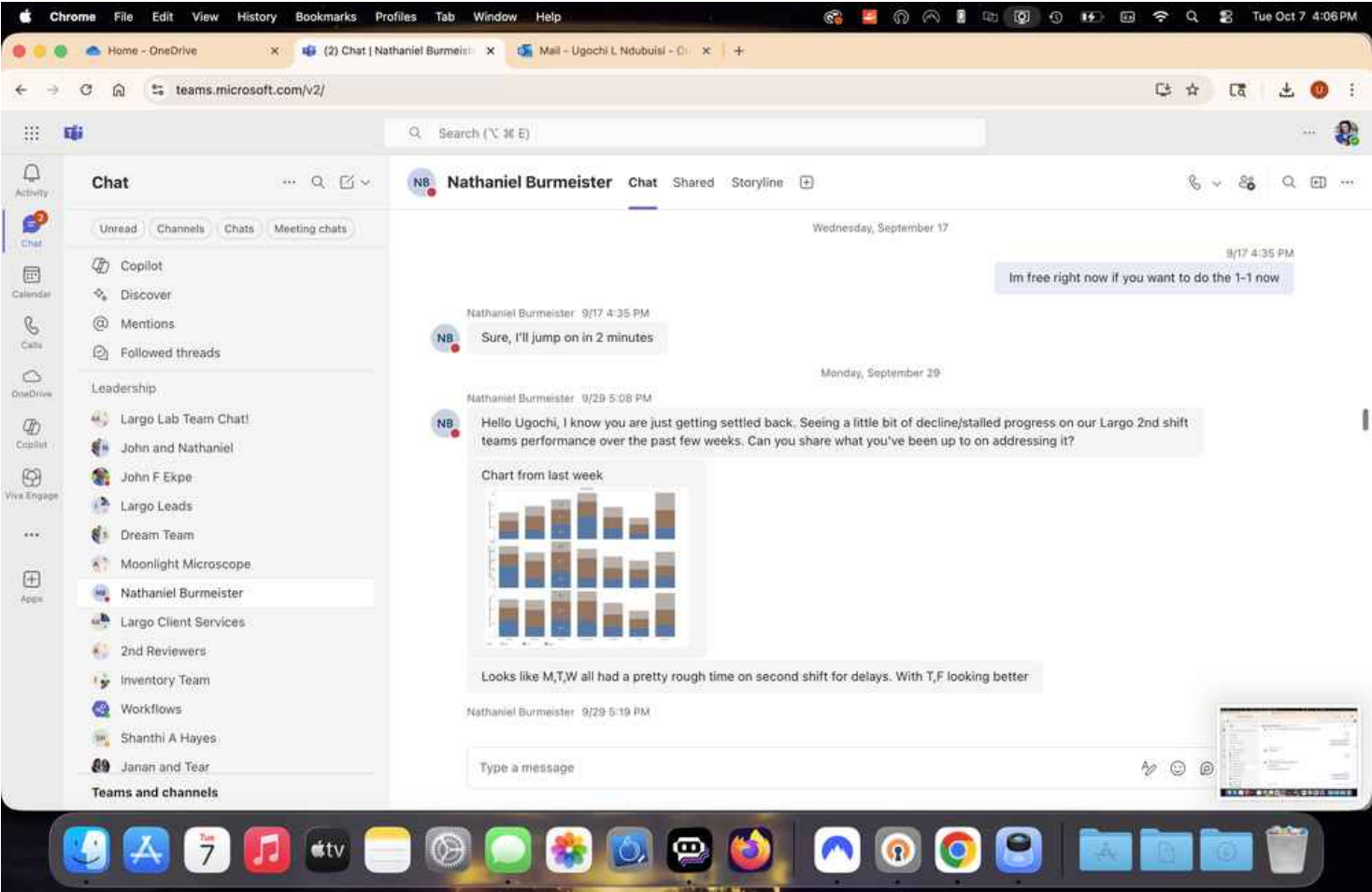
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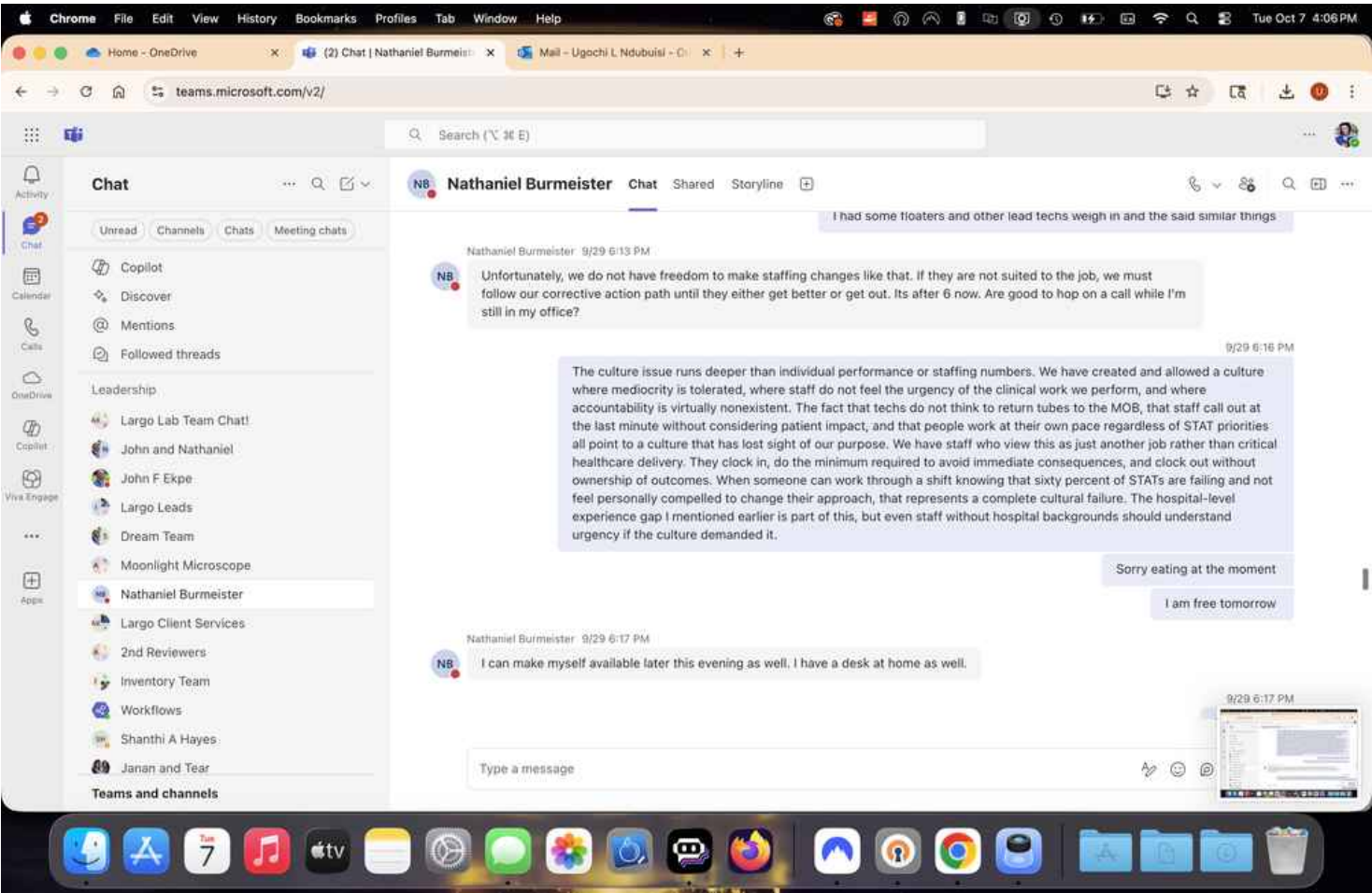
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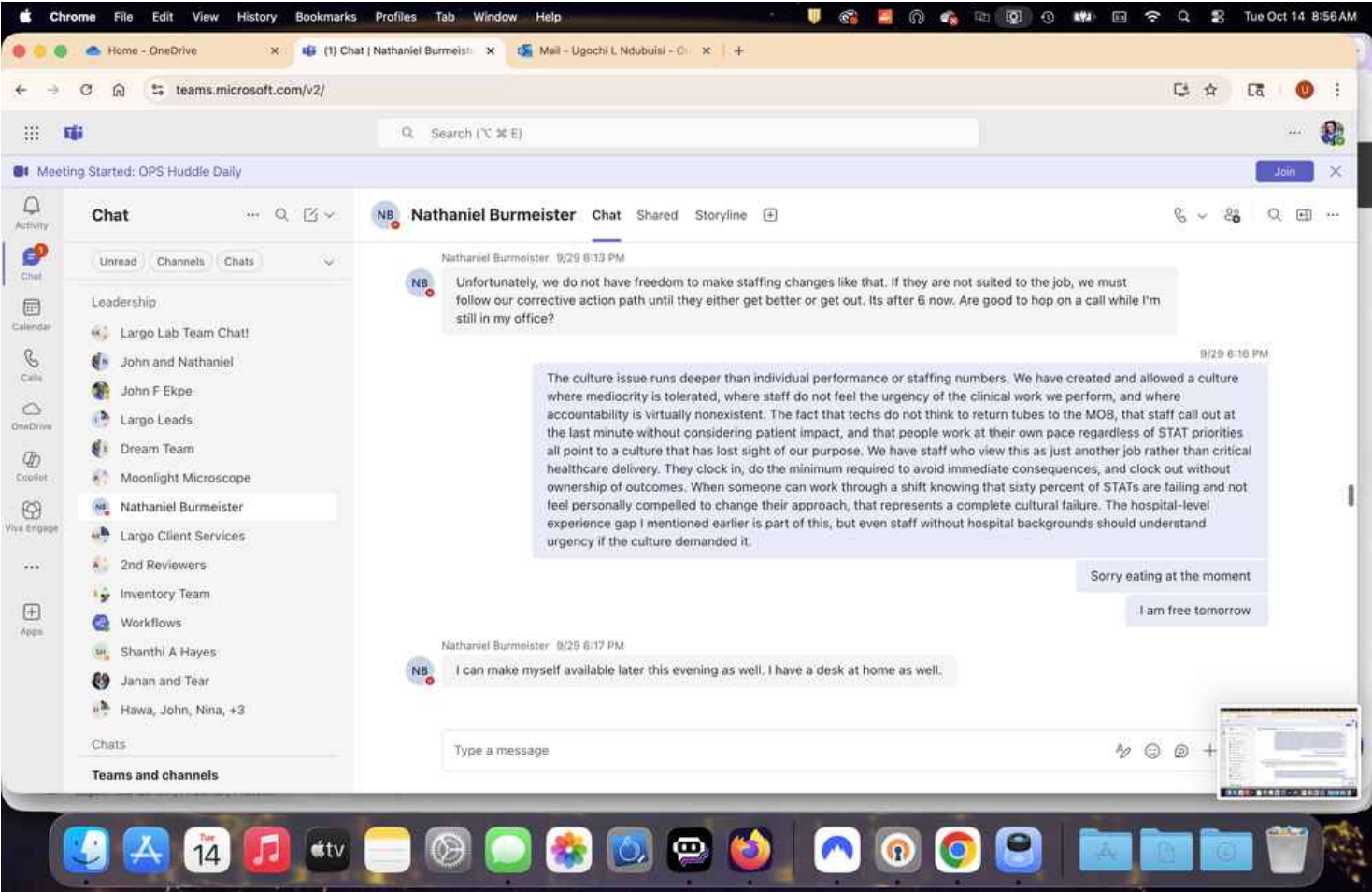
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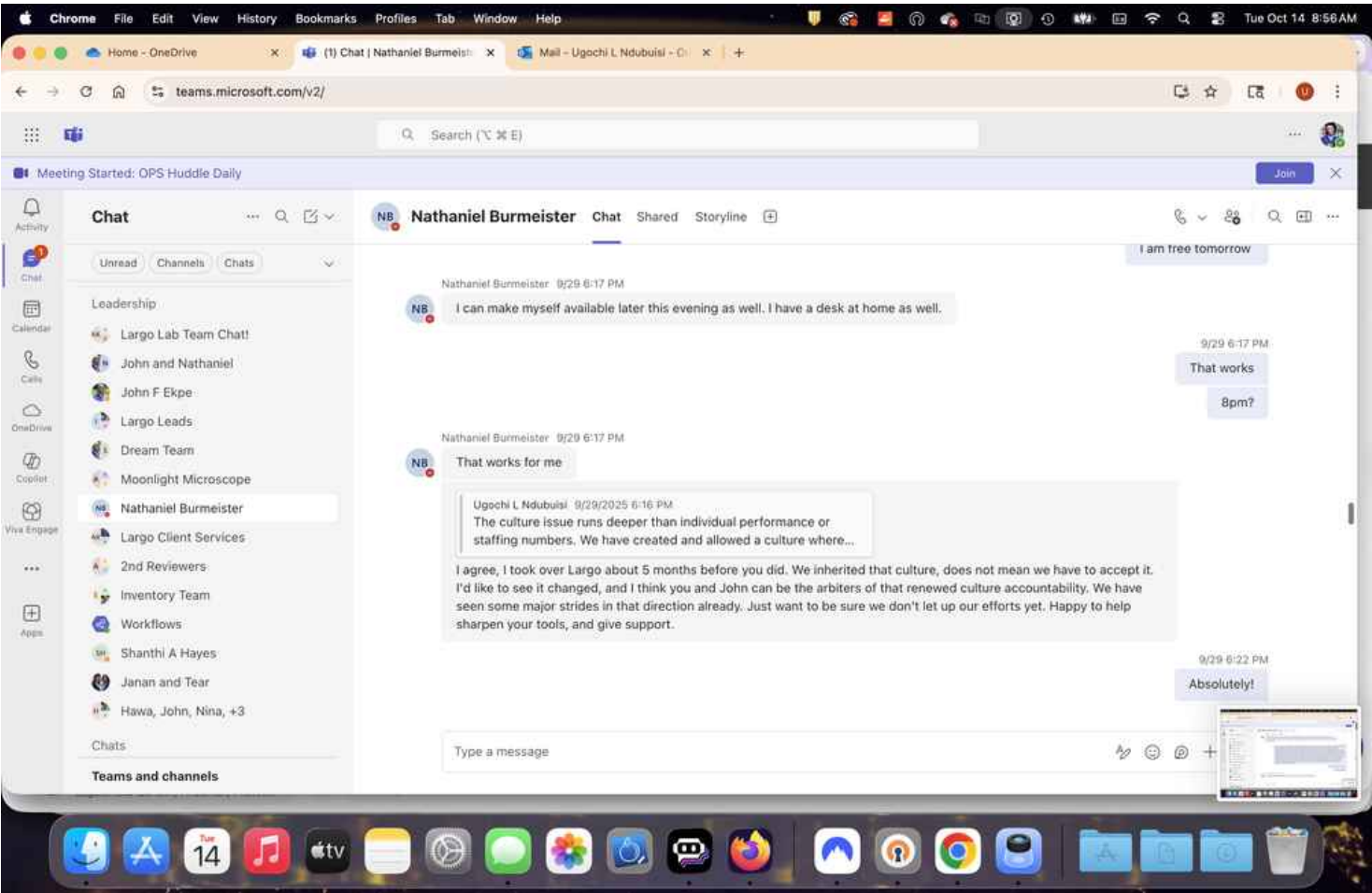
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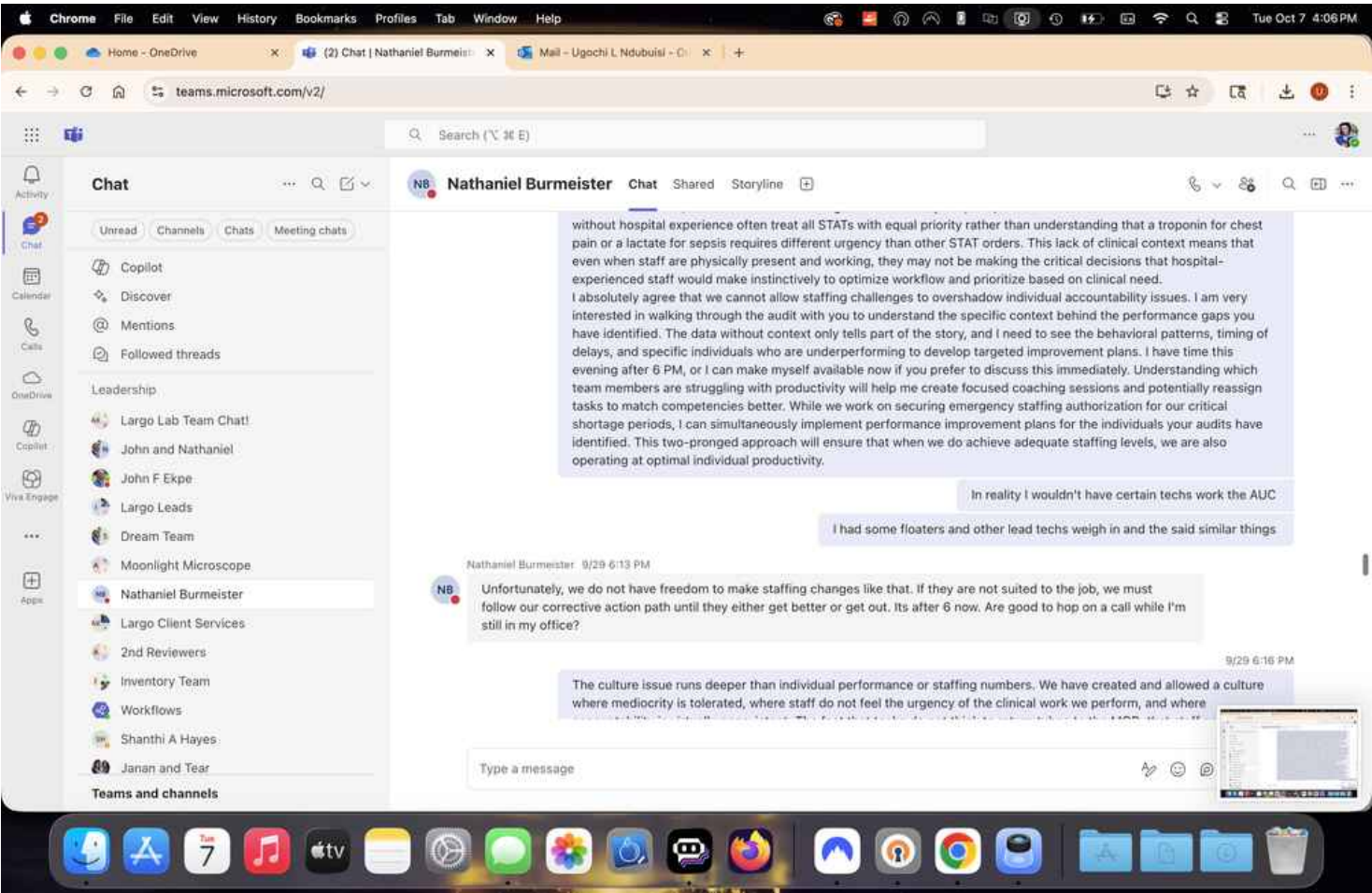


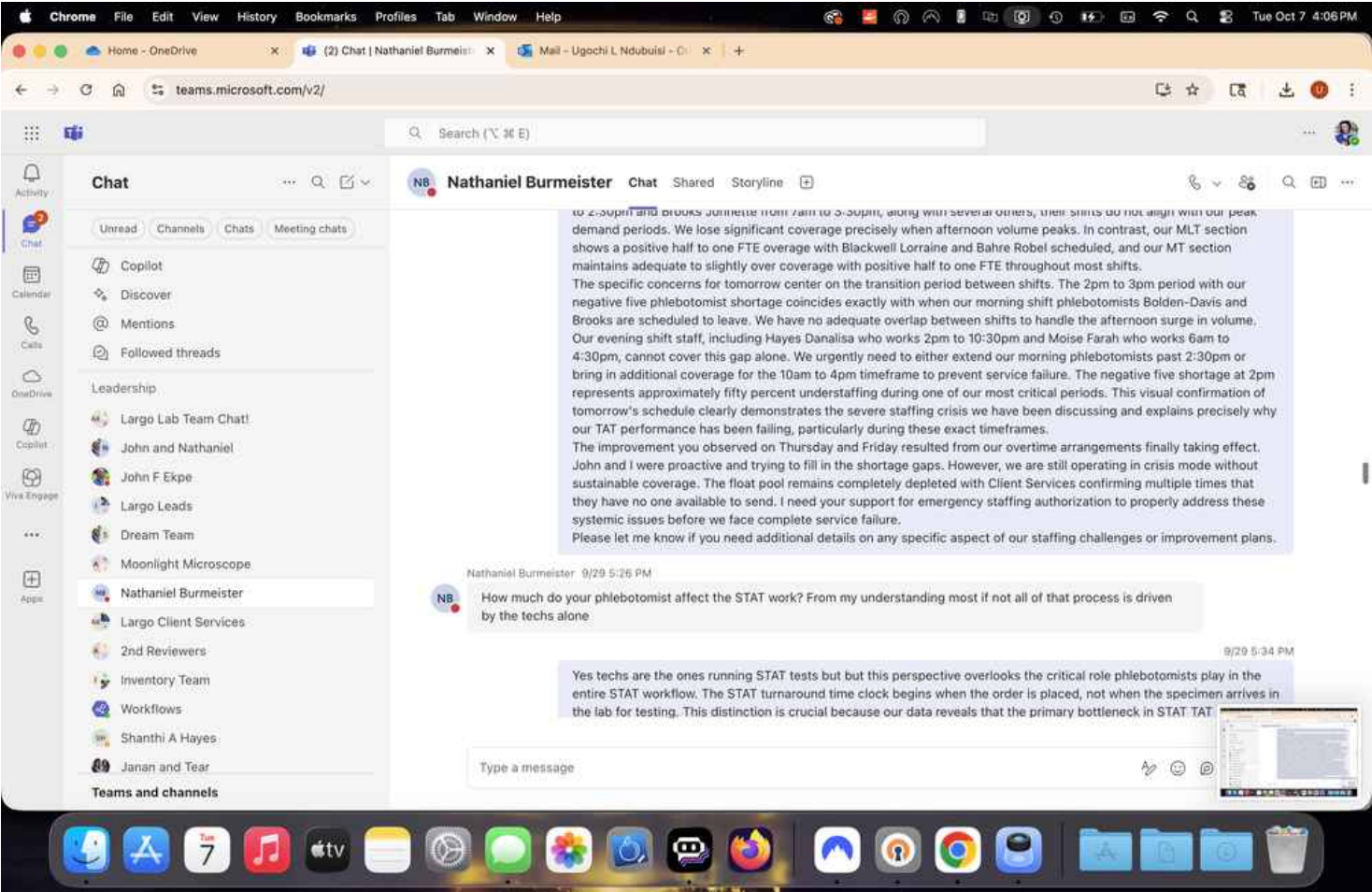


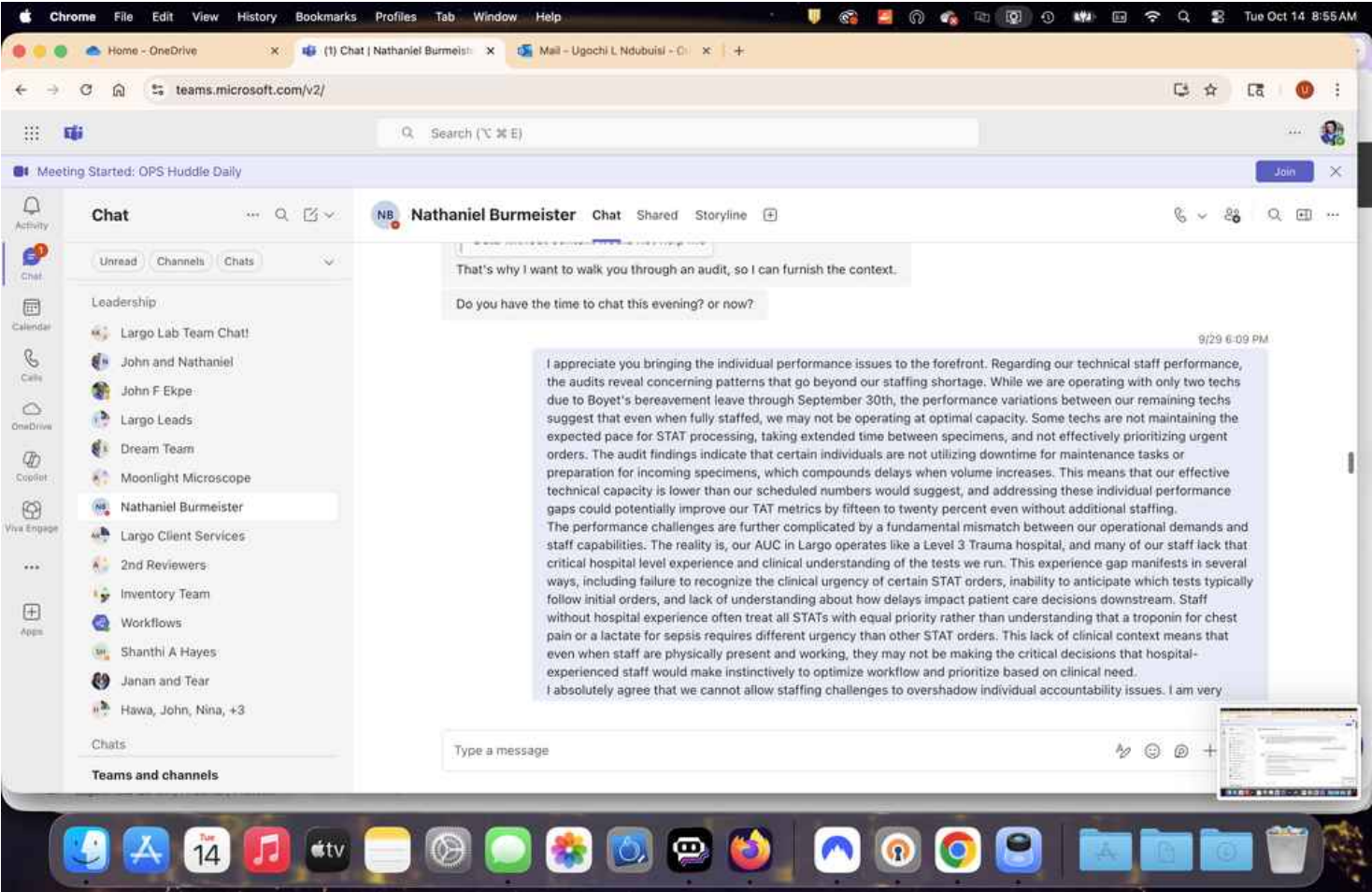


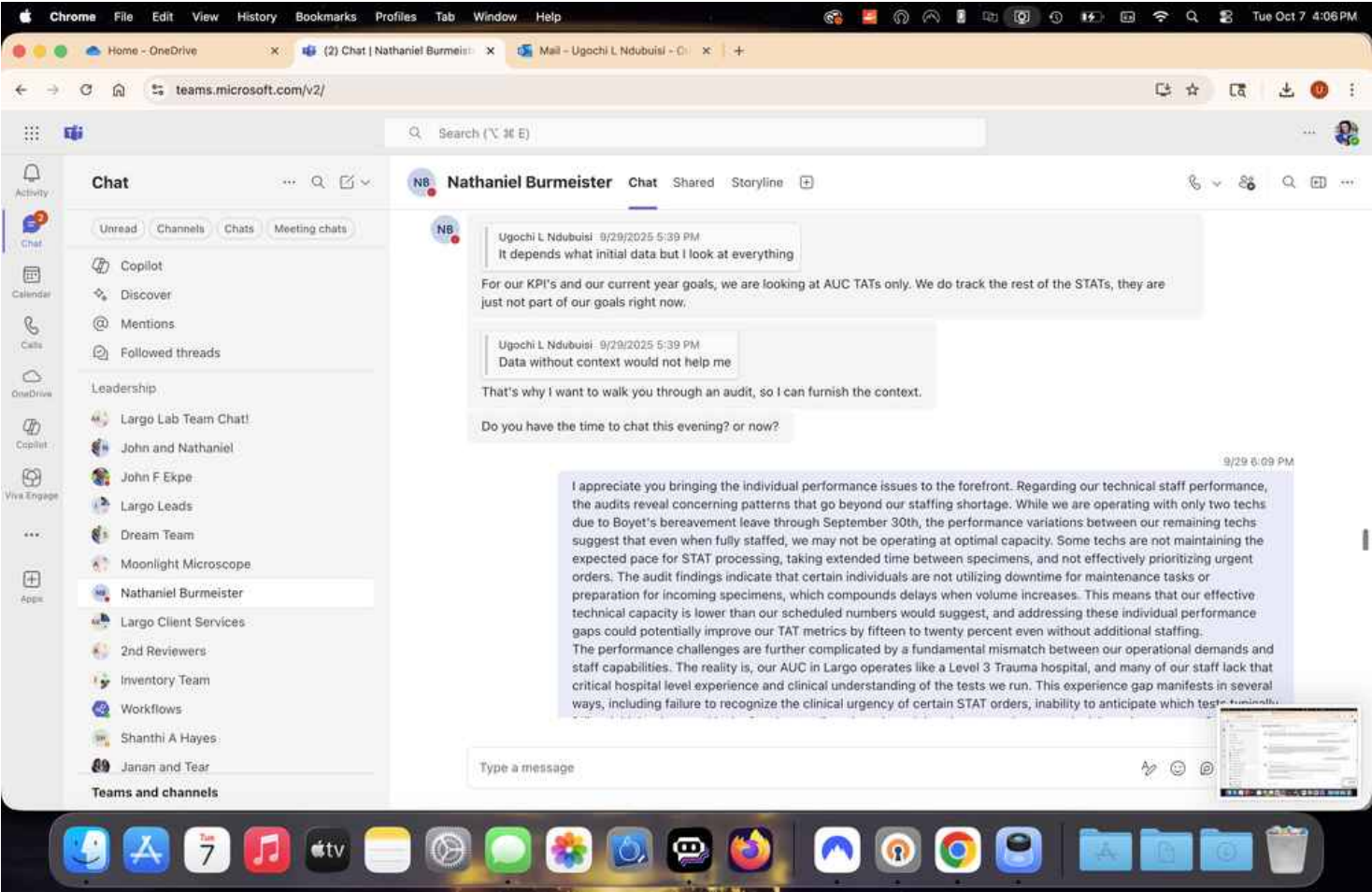


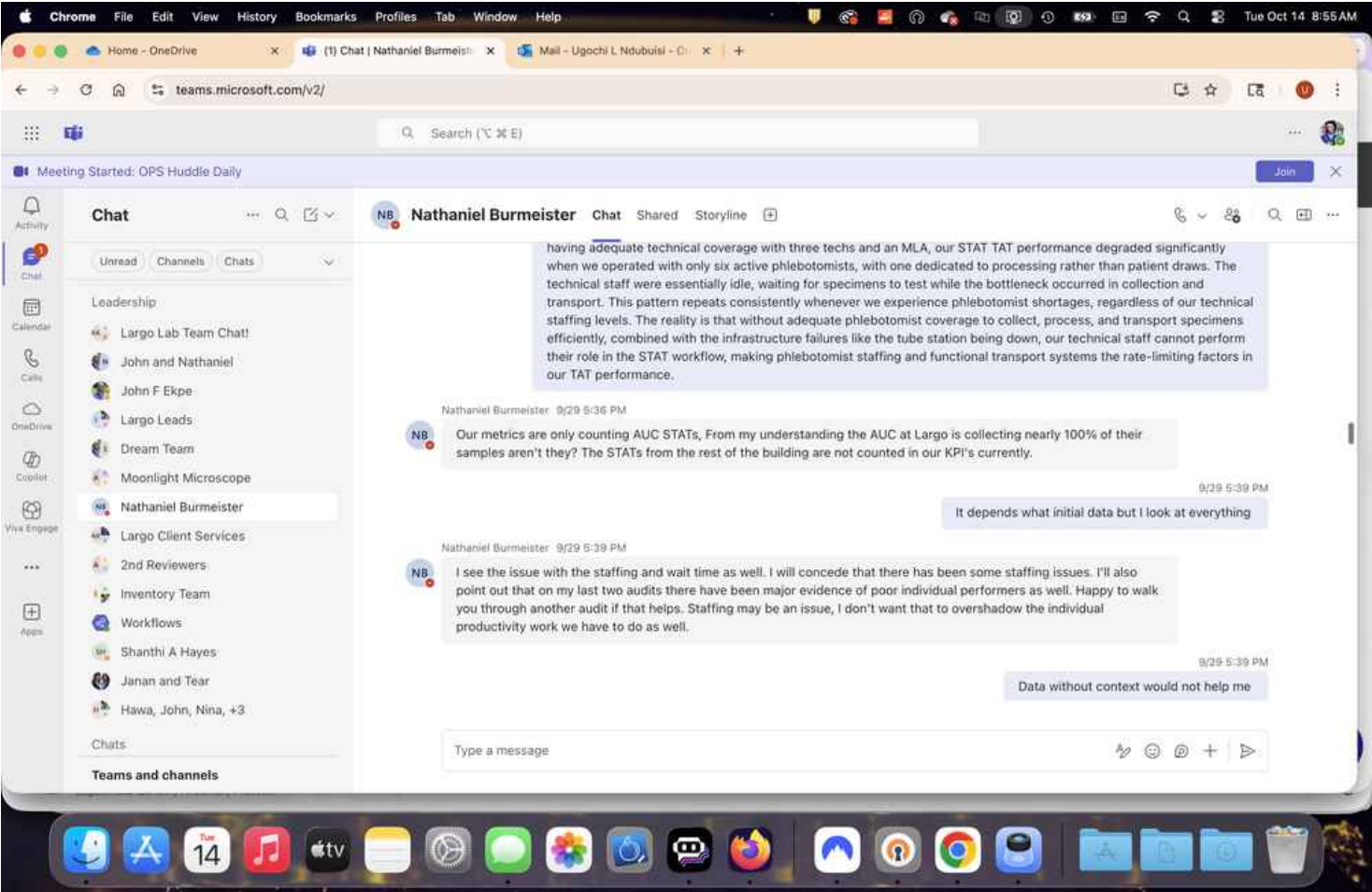


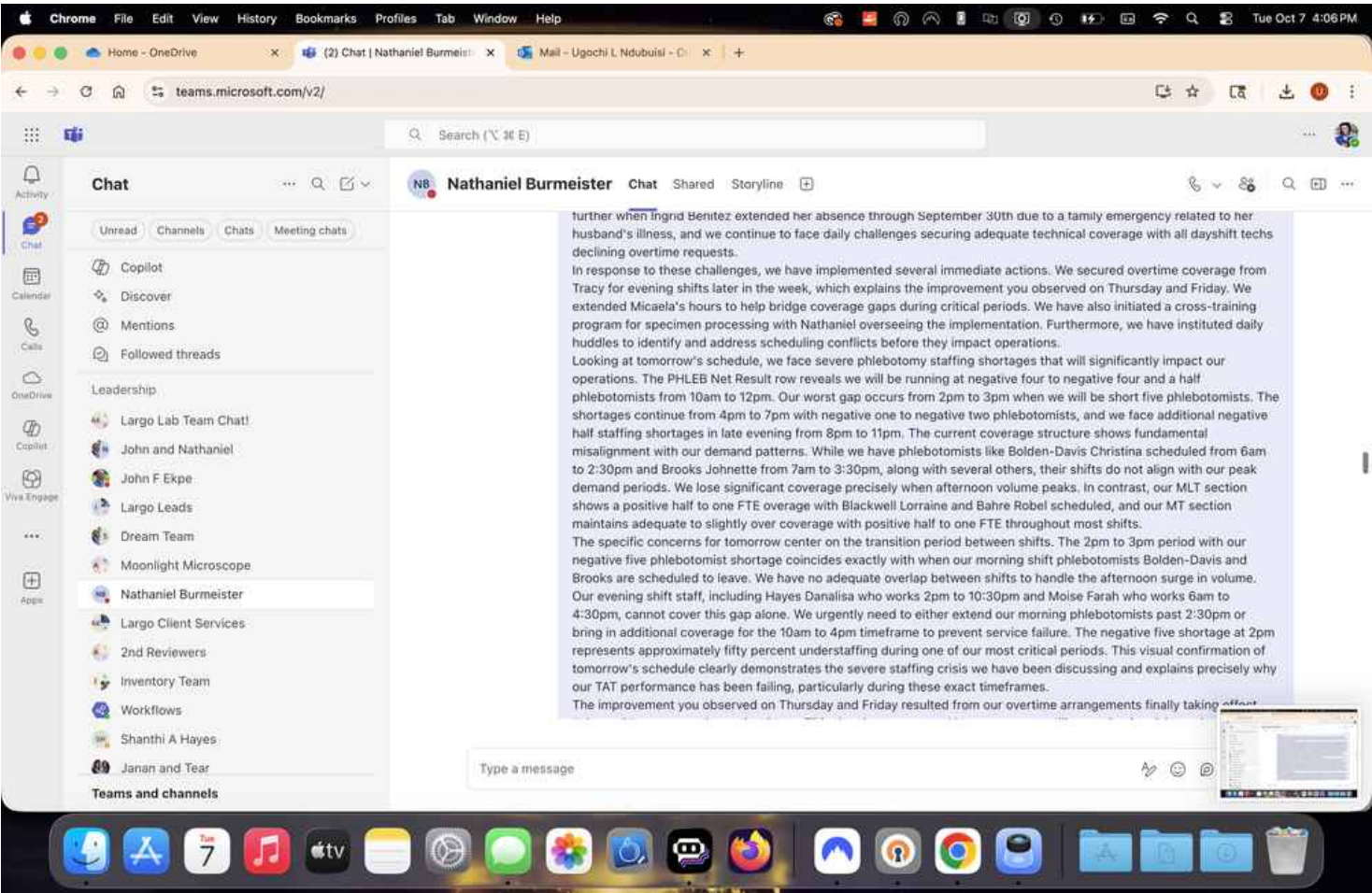


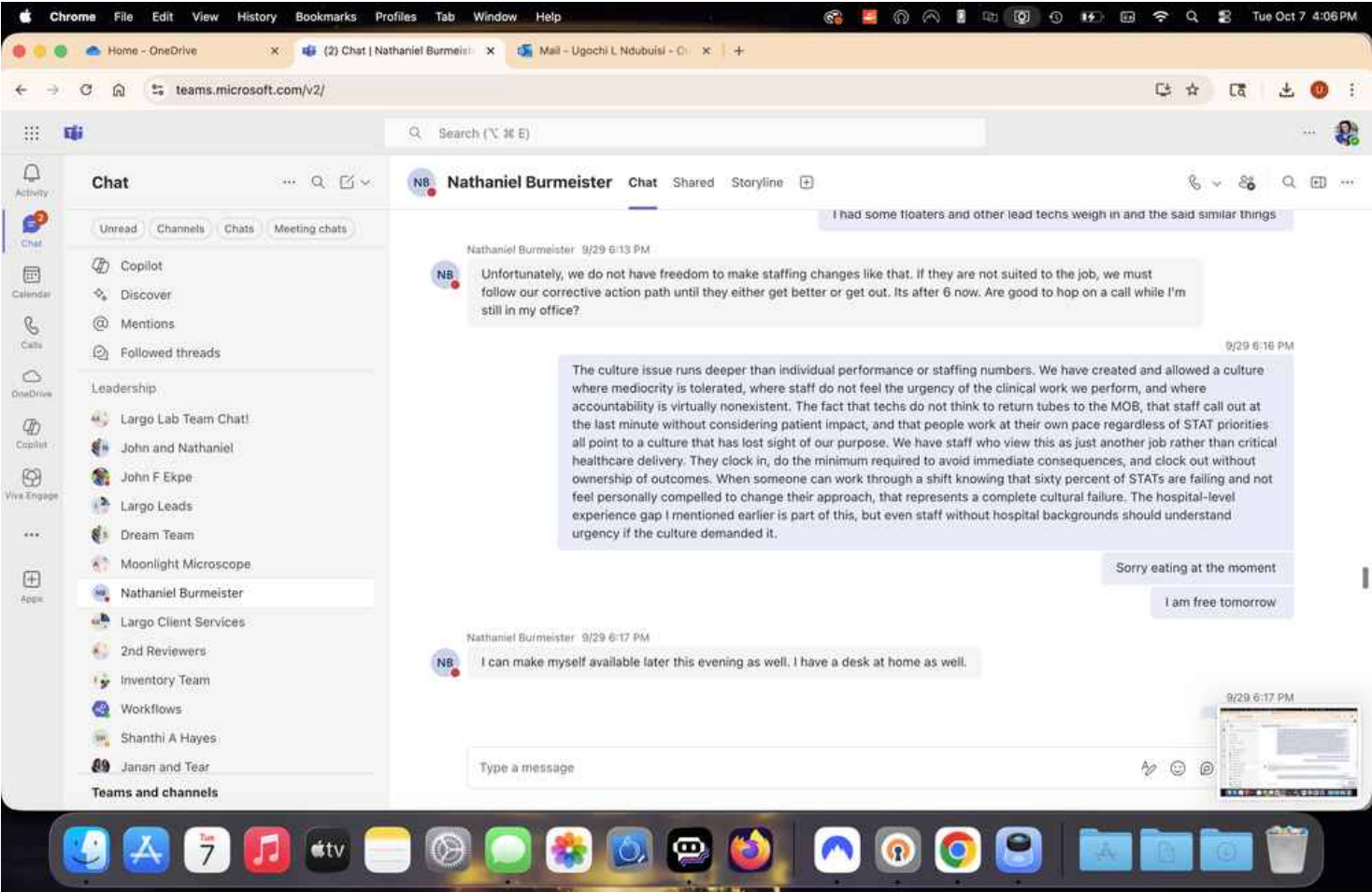




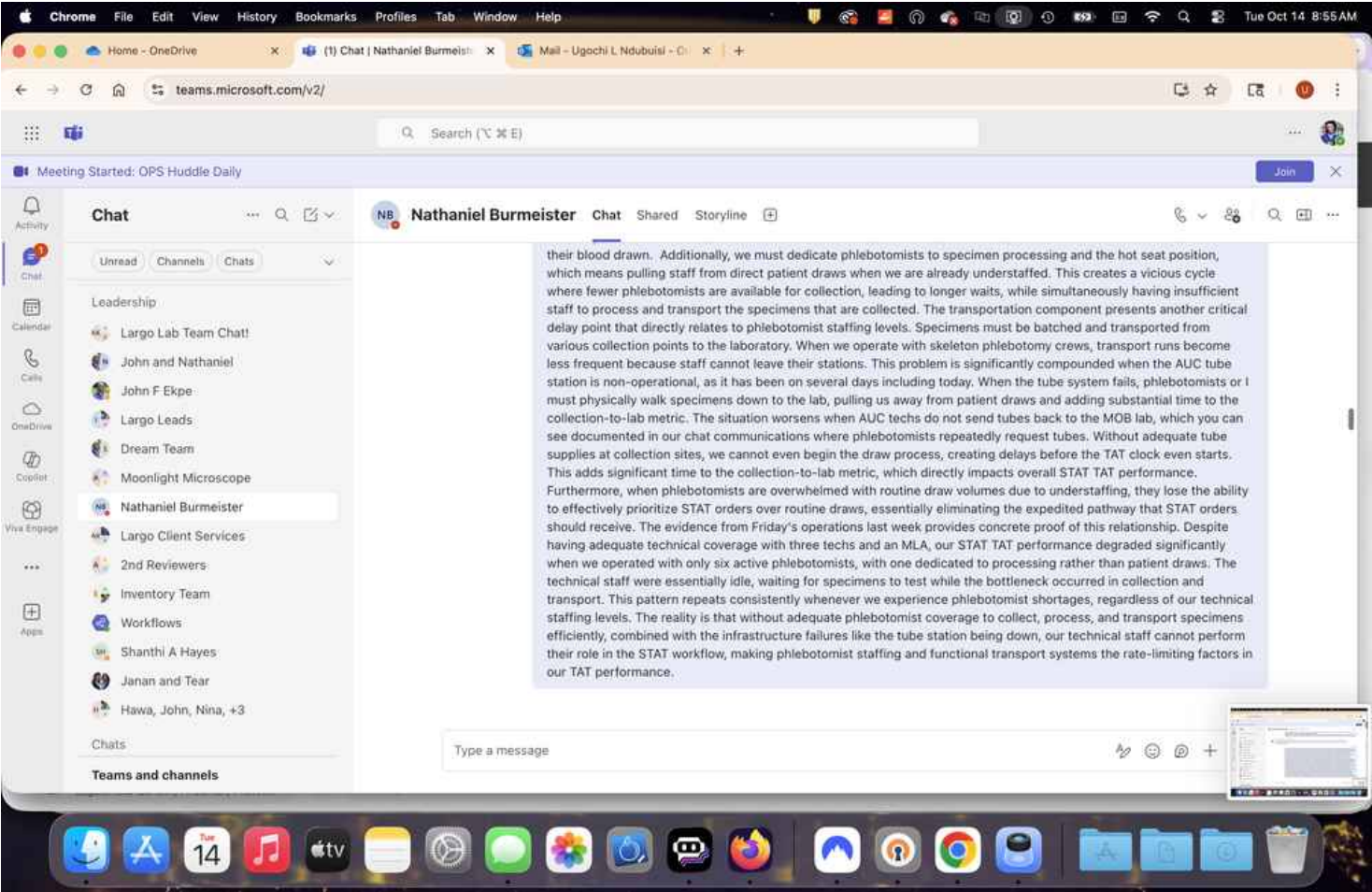




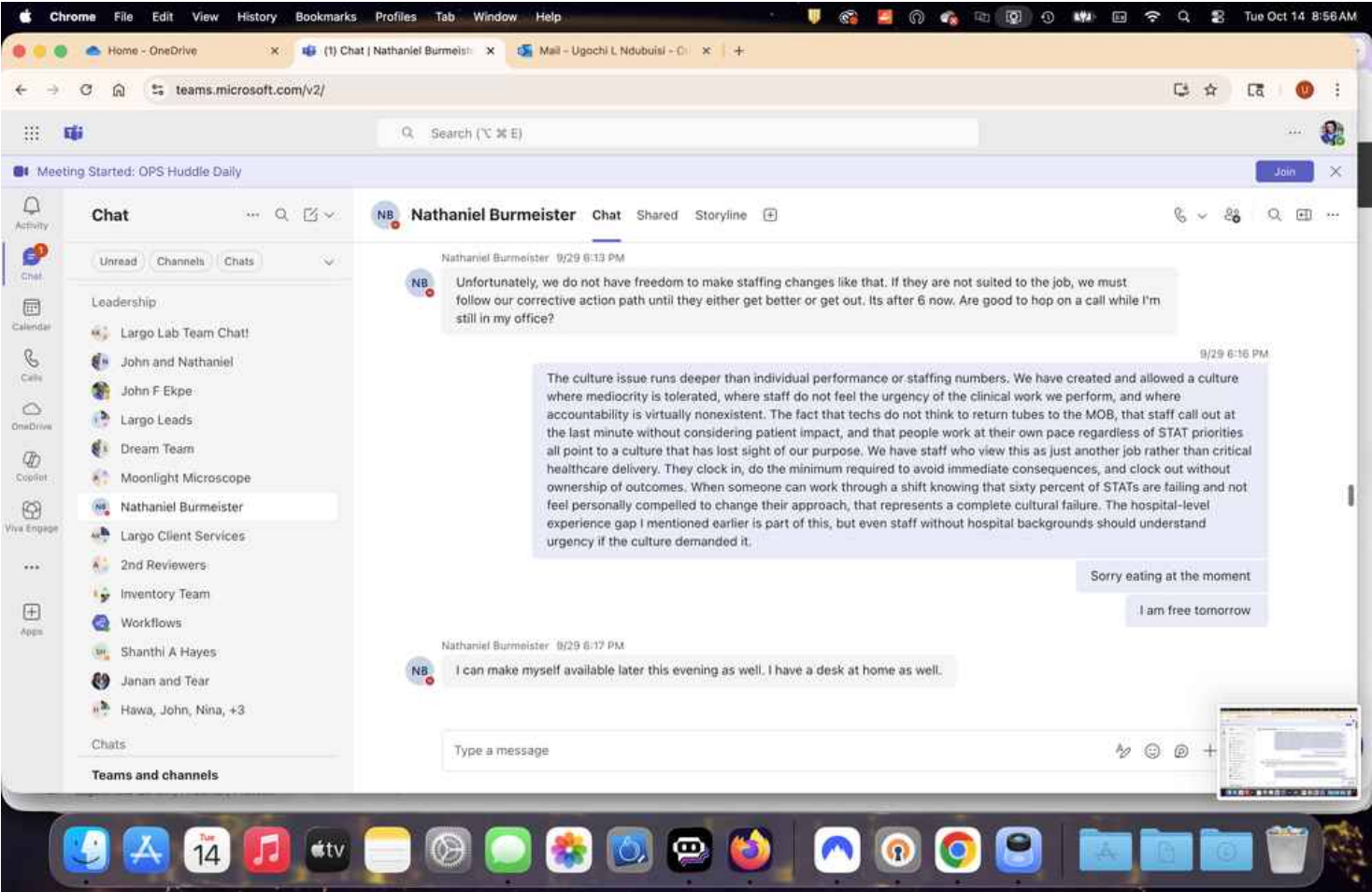


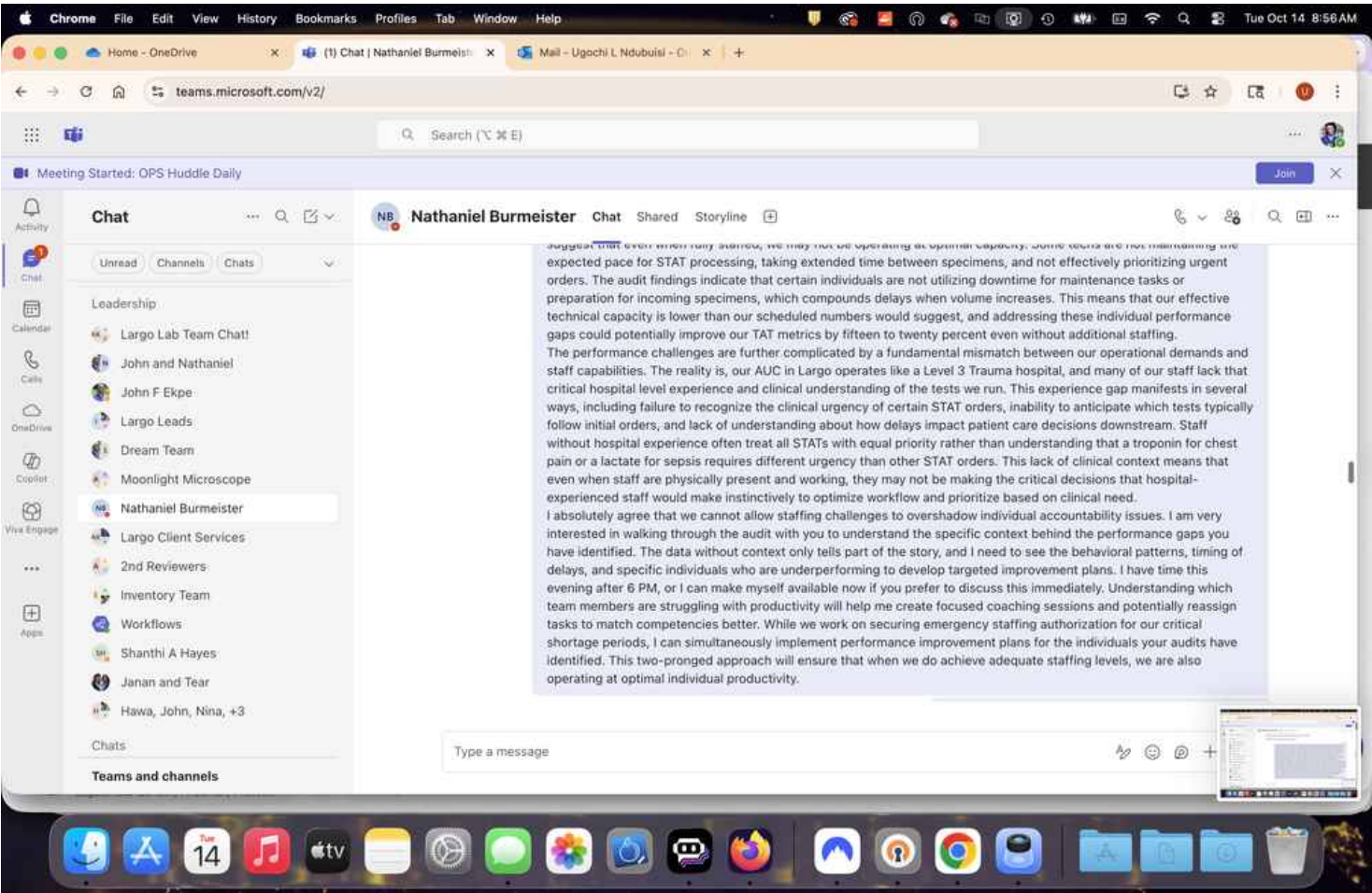


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The screenshot shows a Microsoft Teams chat interface on a Mac. The top of the screen displays the macOS menu bar with various system icons and the time 'Tue Oct 14 8:55 AM'. The browser's address bar shows the URL 'teams.microsoft.com/v2/'. The chat window is titled 'Meeting Started: OPS Huddle Daily'. On the left sidebar, there's a list of 'Teams and channels' including 'Largo Lab Team Chat!', 'John and Nathaniel', 'John F Ekpe', 'Largo Leads', 'Dream Team', 'Moonlight Microscope', 'Nathaniel Burmeister', 'Largo Client Services', '2nd Reviewers', 'Inventory Team', 'Workflows', 'Shanthi A Hayes', 'Janan and Tear', and 'Hawa, John, Nina, +3'. The main chat area shows a conversation between Nathaniel Burmeister and another user. Nathaniel's message at 9/29 5:26 PM asks about the impact of phlebotomists on STAT work. The response at 9/29 5:34 PM provides a detailed explanation of the STAT workflow, highlighting a major bottleneck in the collection-to-lab phase and the challenges posed by staffing shortages.

Chat History:

- Nathaniel Burmeister** 9/29 5:26 PM: How much do your phlebotomist affect the STAT work? From my understanding most if not all of that process is driven by the techs alone
- [User]** 9/29 5:34 PM: Yes techs are the ones running STAT tests but but this perspective overlooks the critical role phlebotomists play in the entire STAT workflow. The STAT turnaround time clock begins when the order is placed, not when the specimen arrives in the lab for testing. This distinction is crucial because our data reveals that the primary bottleneck in STAT TAT performance occurs during the collection-to-lab phase, which is entirely dependent on phlebotomist availability and efficiency. The data from last week demonstrates this clearly. During the 8am hour, which shows our worst TAT performance, the collection-to-lab time averaged an astronomical 3,489 minutes, while the in-lab-to-result time remained relatively stable at 142 minutes. This massive discrepancy indicates that samples are experiencing significant delays before they ever reach the techs who would perform the actual testing. Even with adequate technical staff ready to process STAT orders, they cannot test specimens that have not been collected and delivered to the lab. When we experience phlebotomist shortages like the negative five staffing gap at 2pm tomorrow, multiple cascading failures occur throughout the STAT workflow. Each phlebotomist can realistically handle only four to six patients per hour when following proper collection procedures. With half the required staff, patients experience extended wait times just to have their blood drawn. Additionally, we must dedicate phlebotomists to specimen processing and the hot seat position, which means pulling staff from direct patient draws when we are already understaffed. This creates a vicious cycle where fewer phlebotomists are available for collection, leading to longer waits, while simultaneously having insufficient staff to process and transport the specimens that are collected. The transportation component presents another critical

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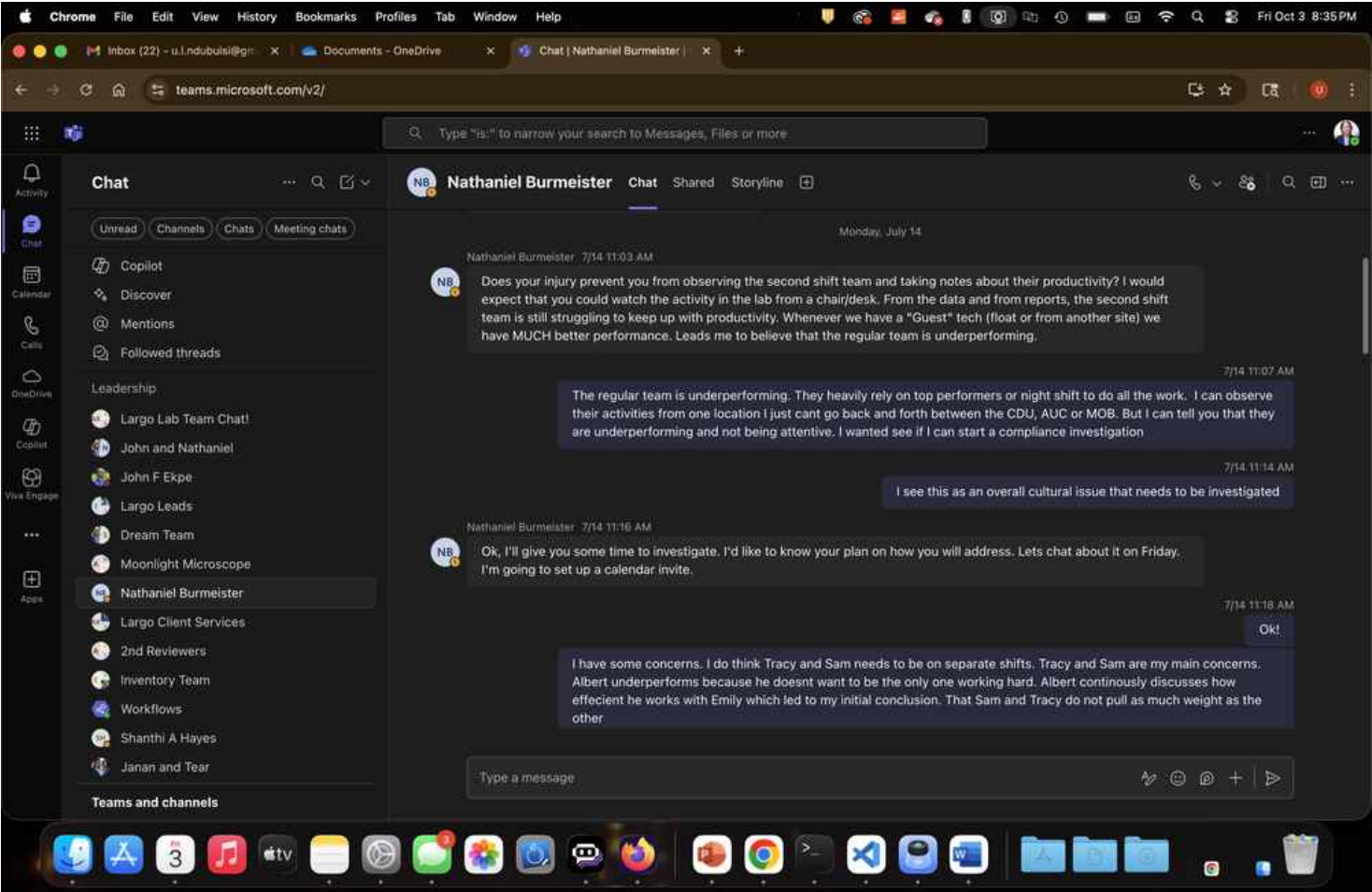
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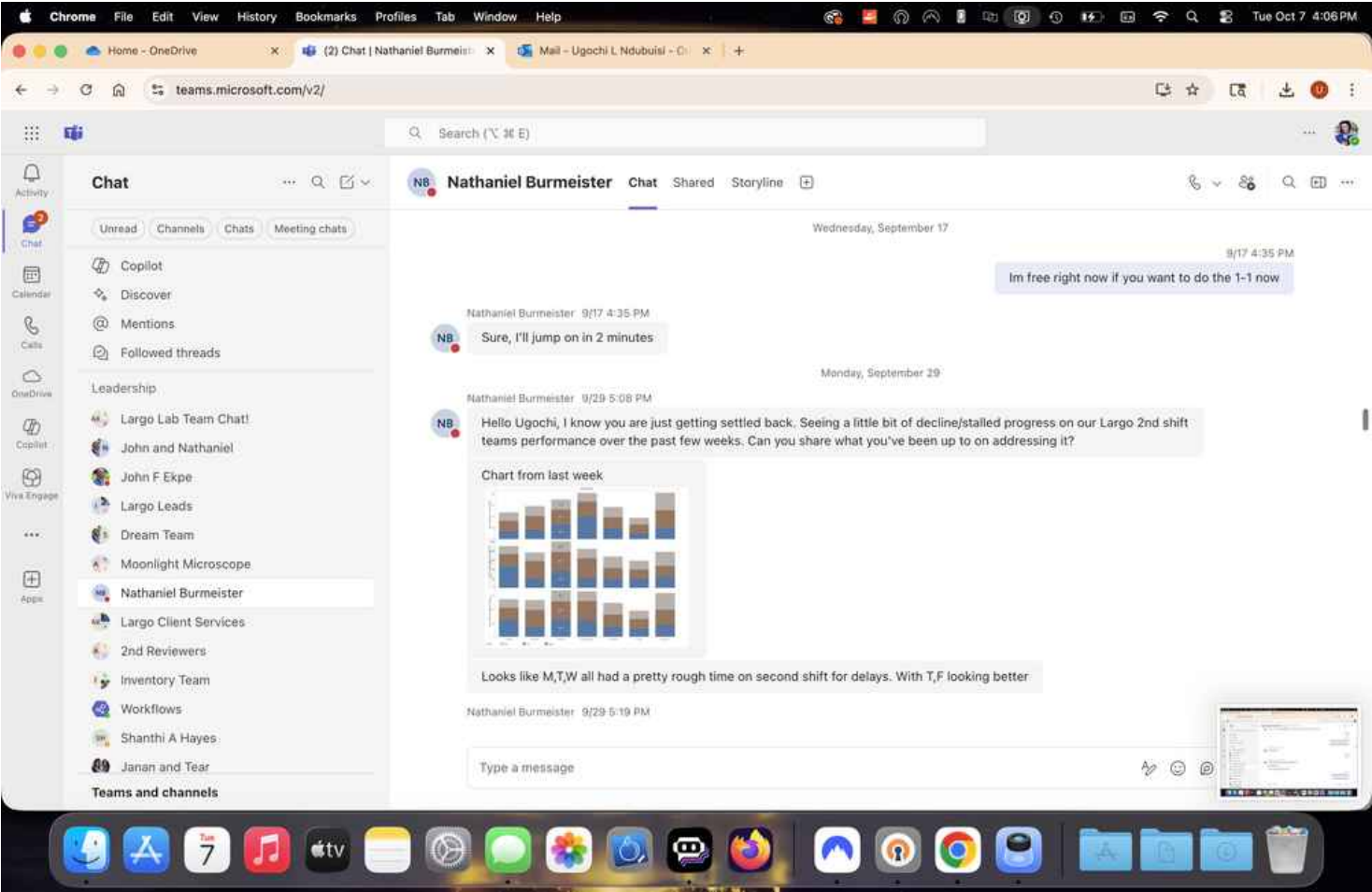
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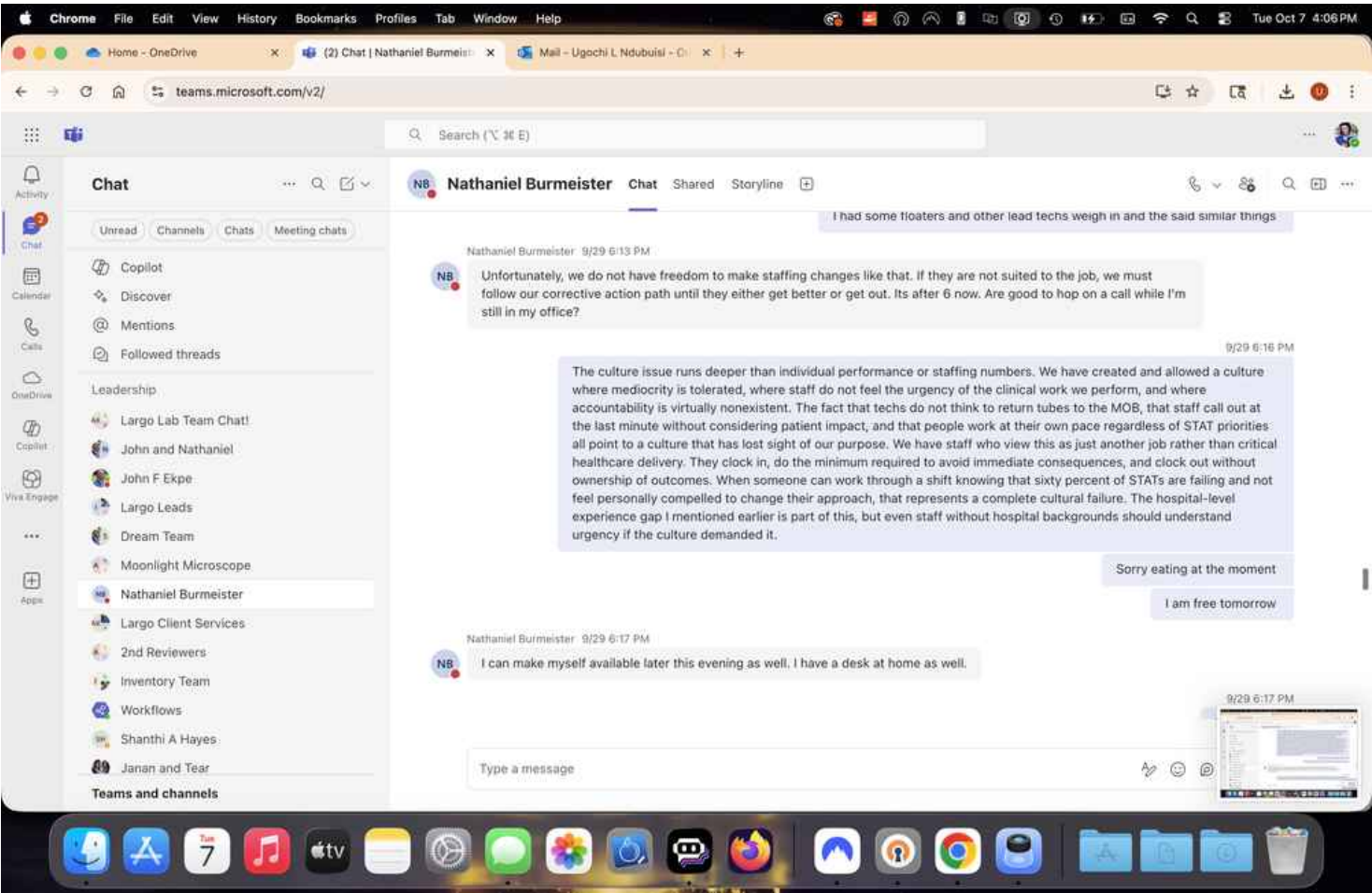
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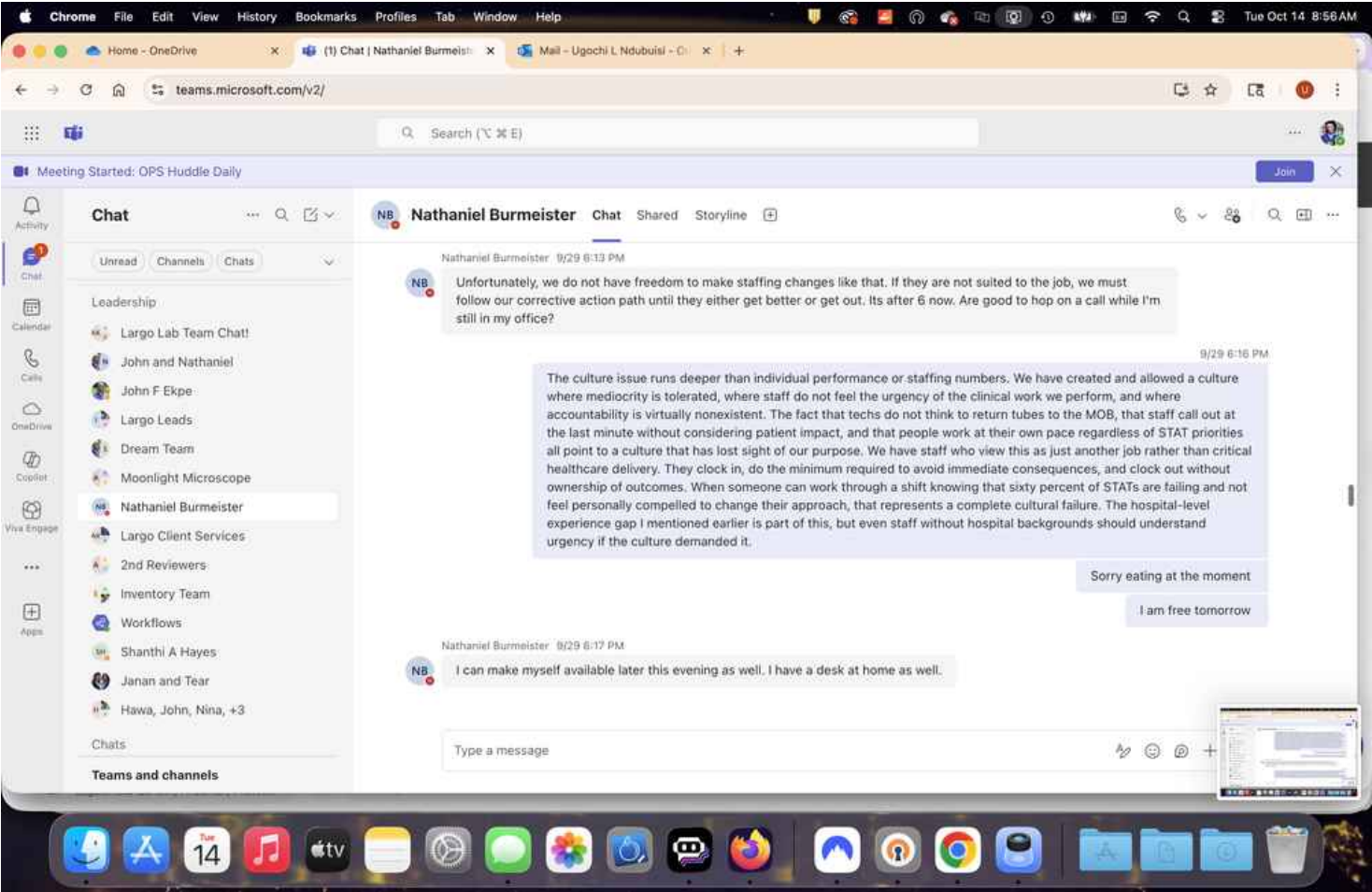
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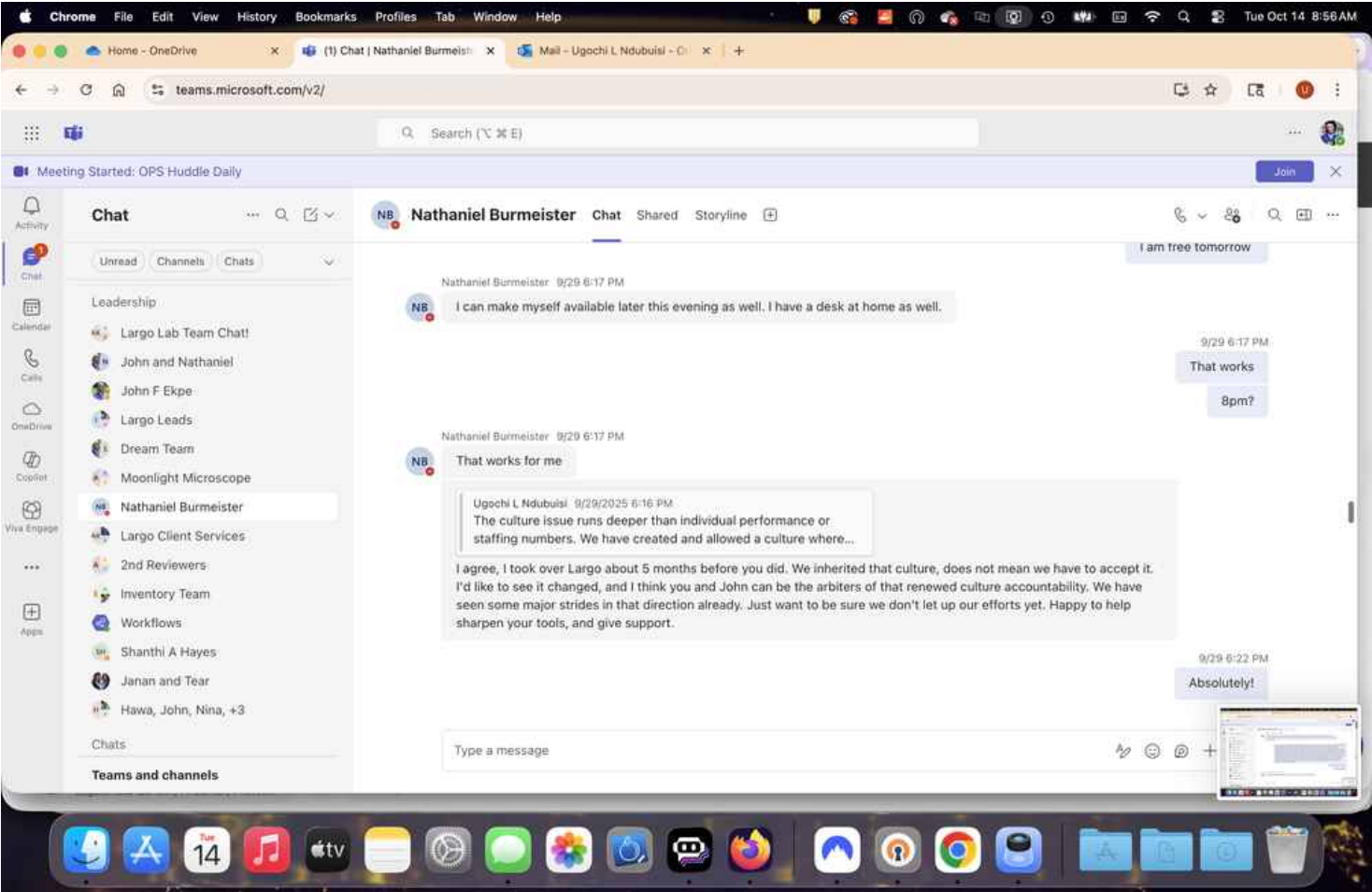
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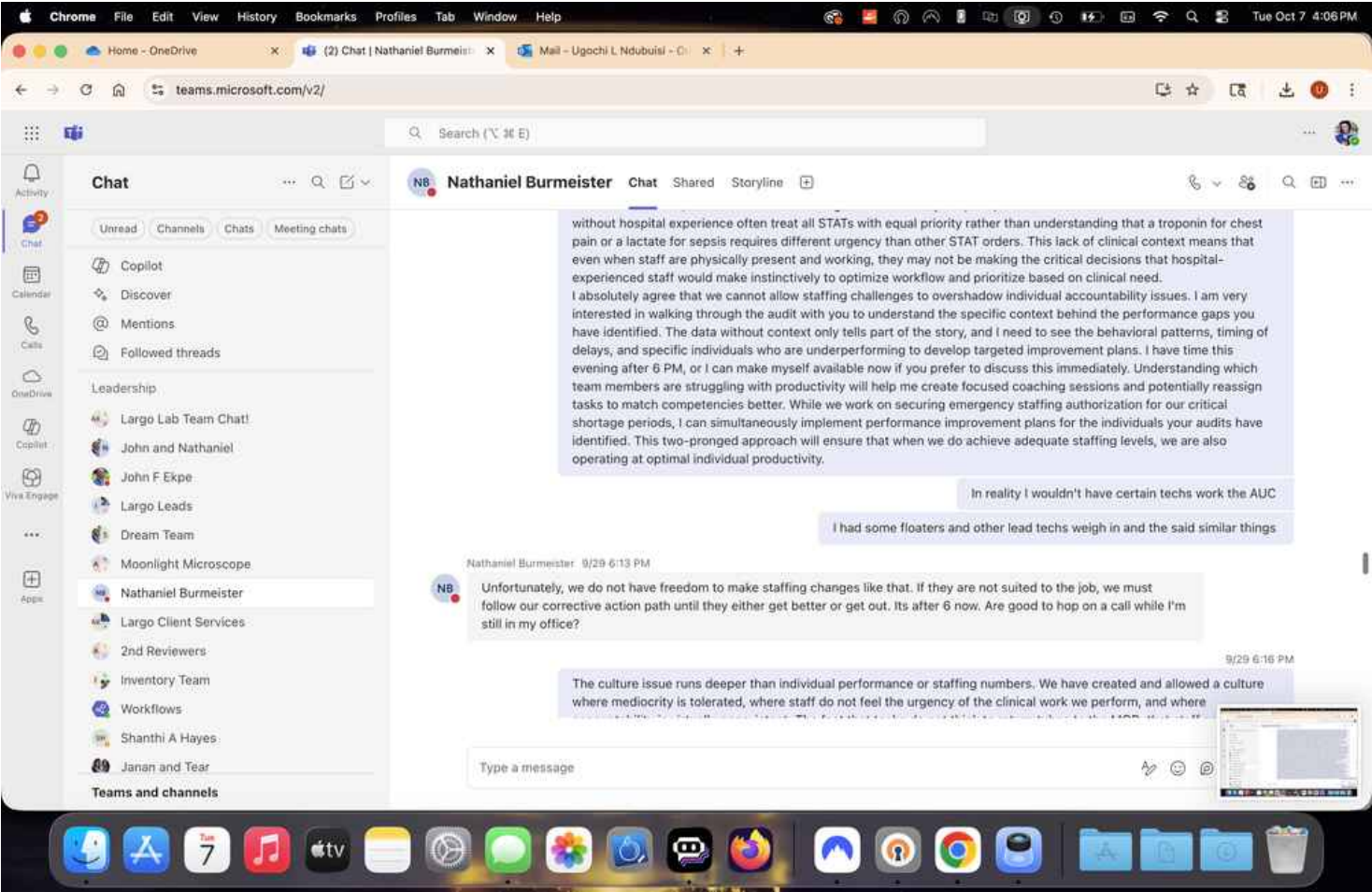


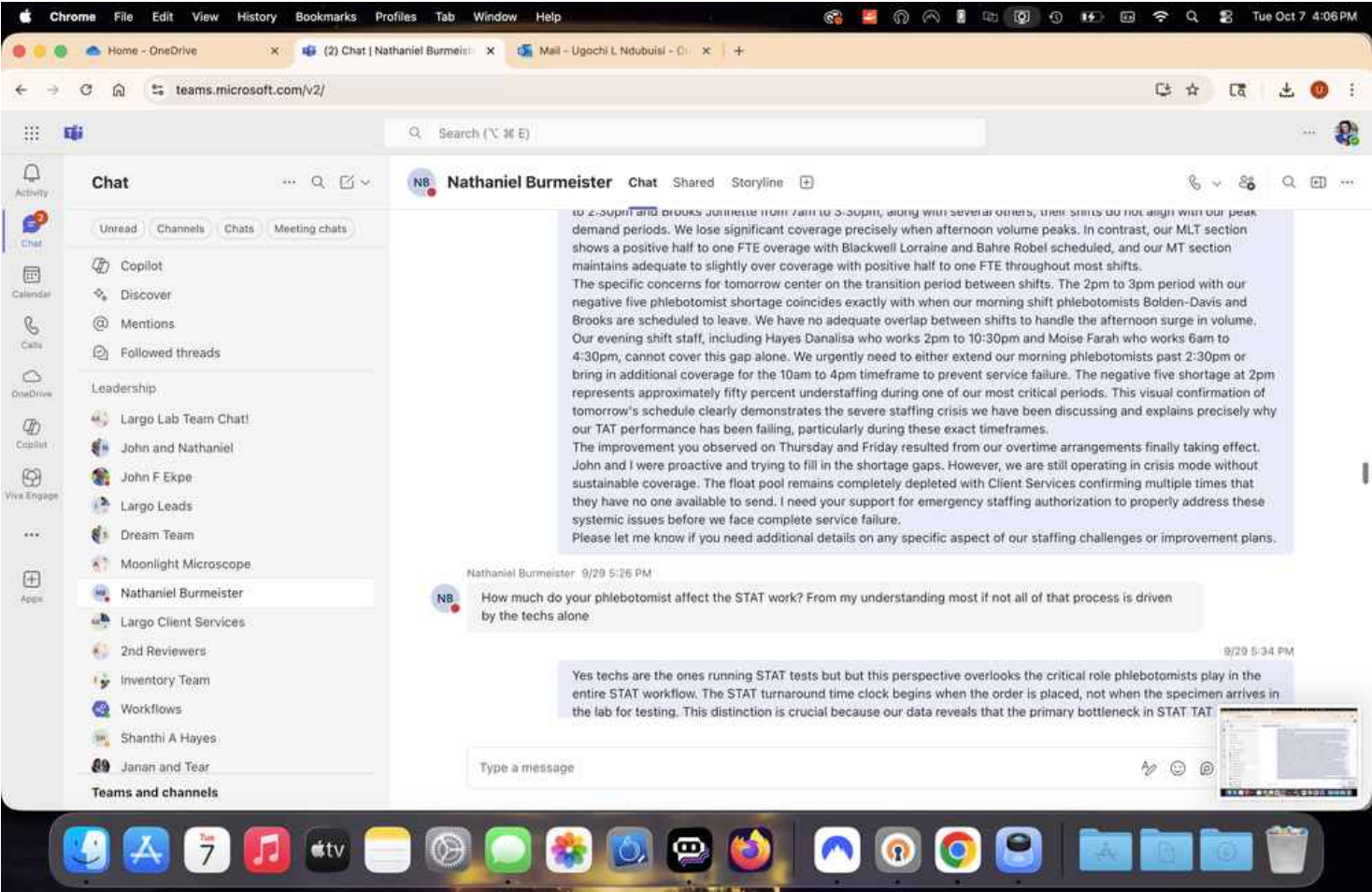


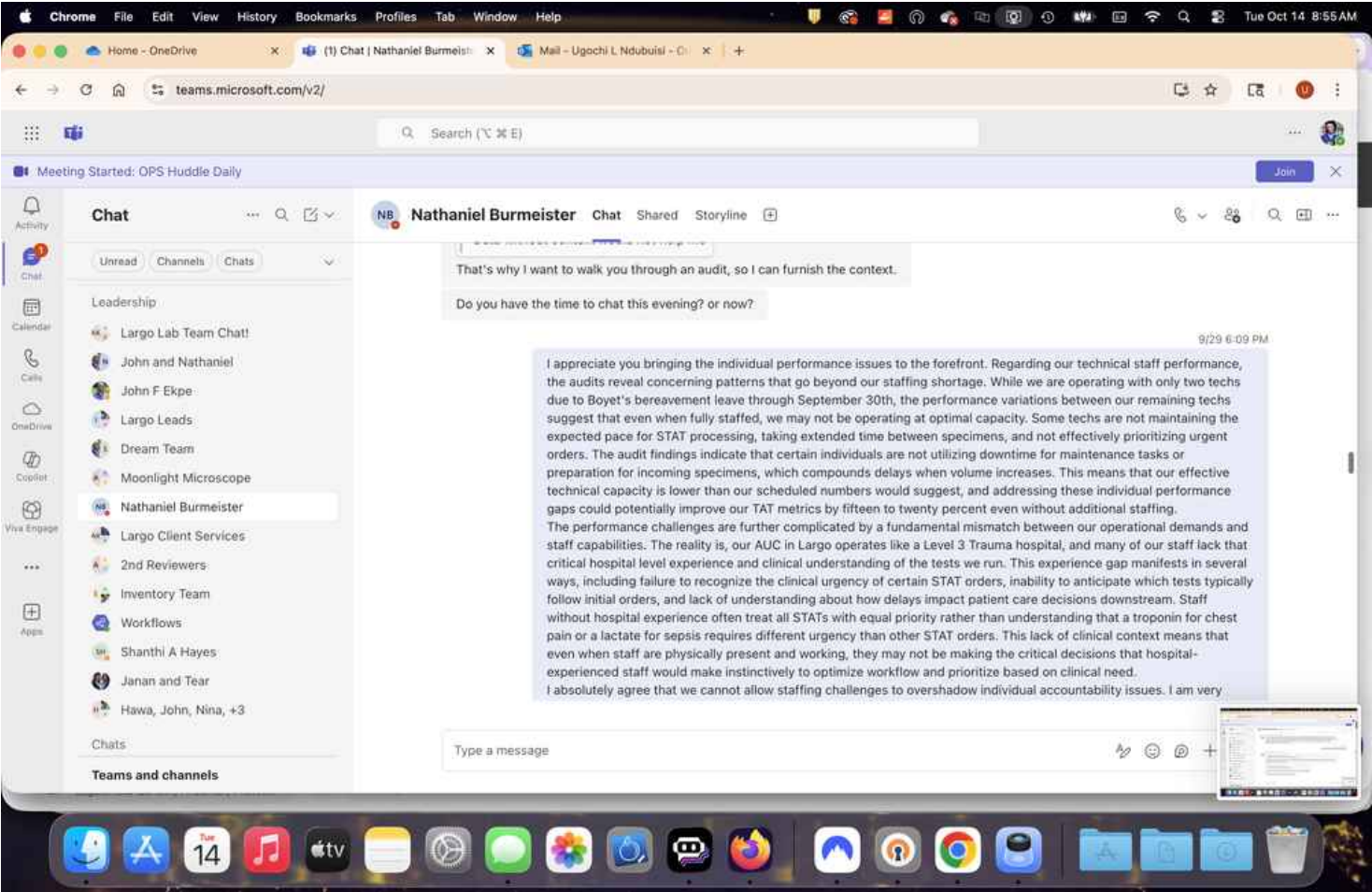


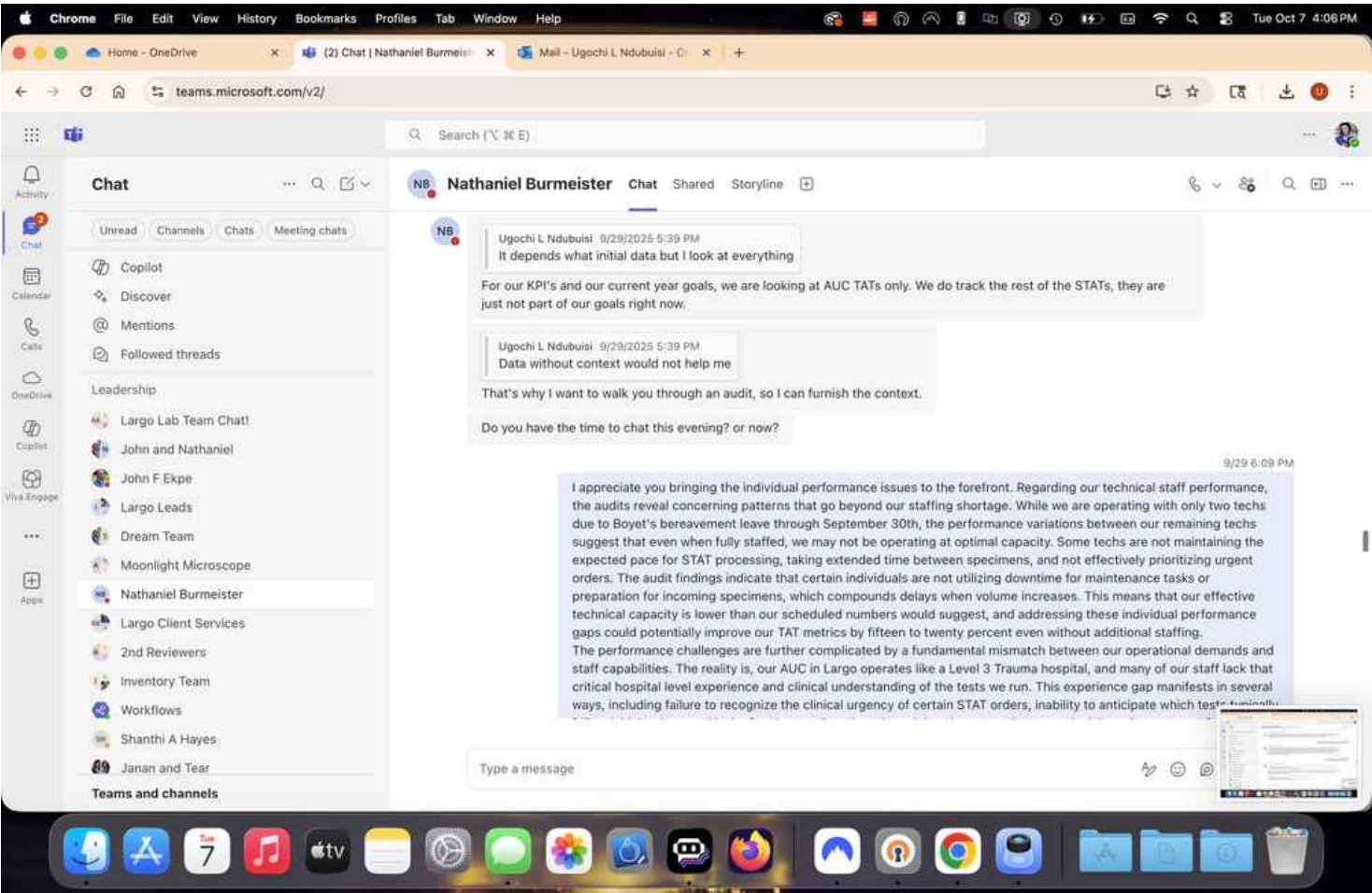


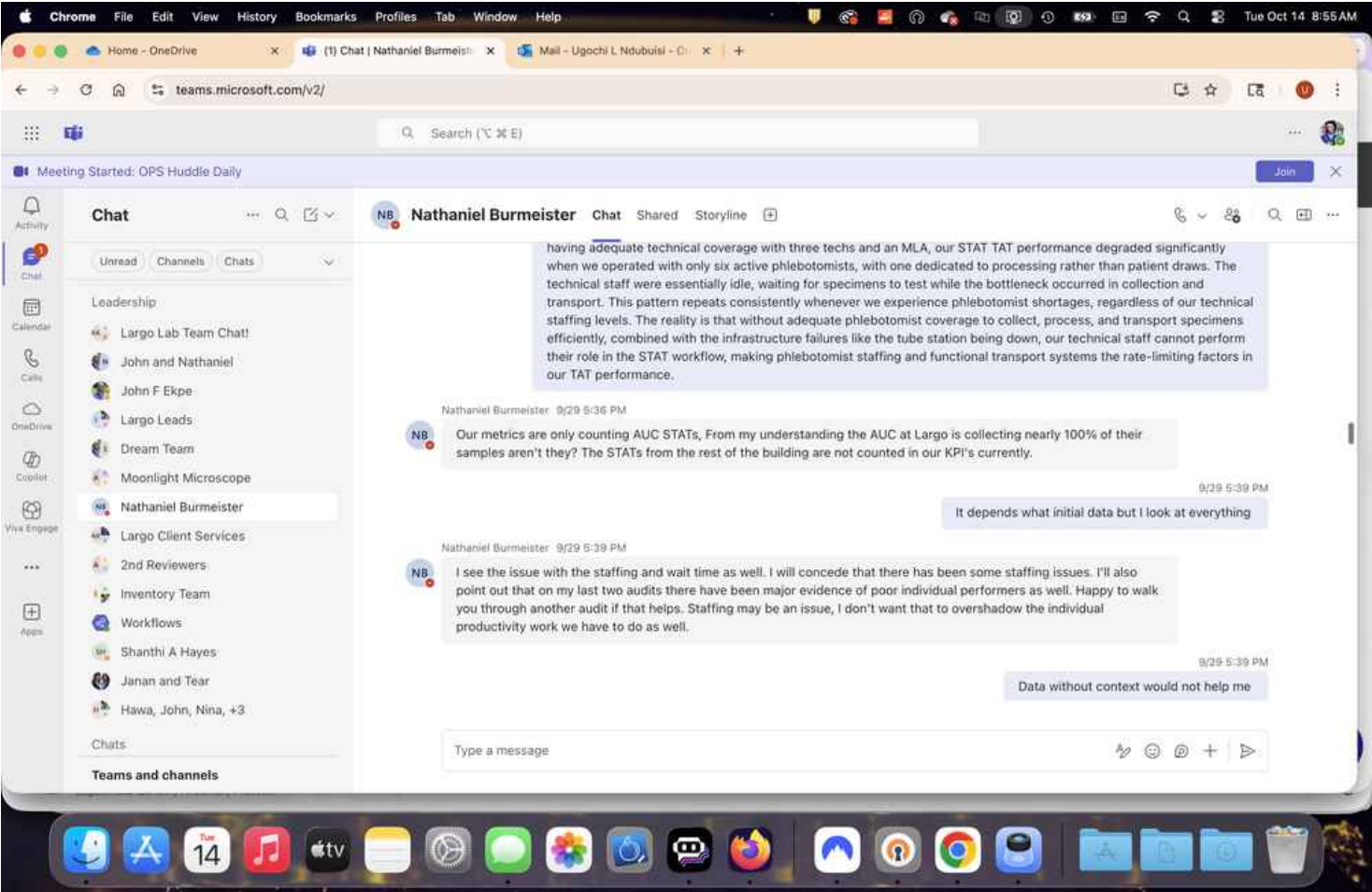


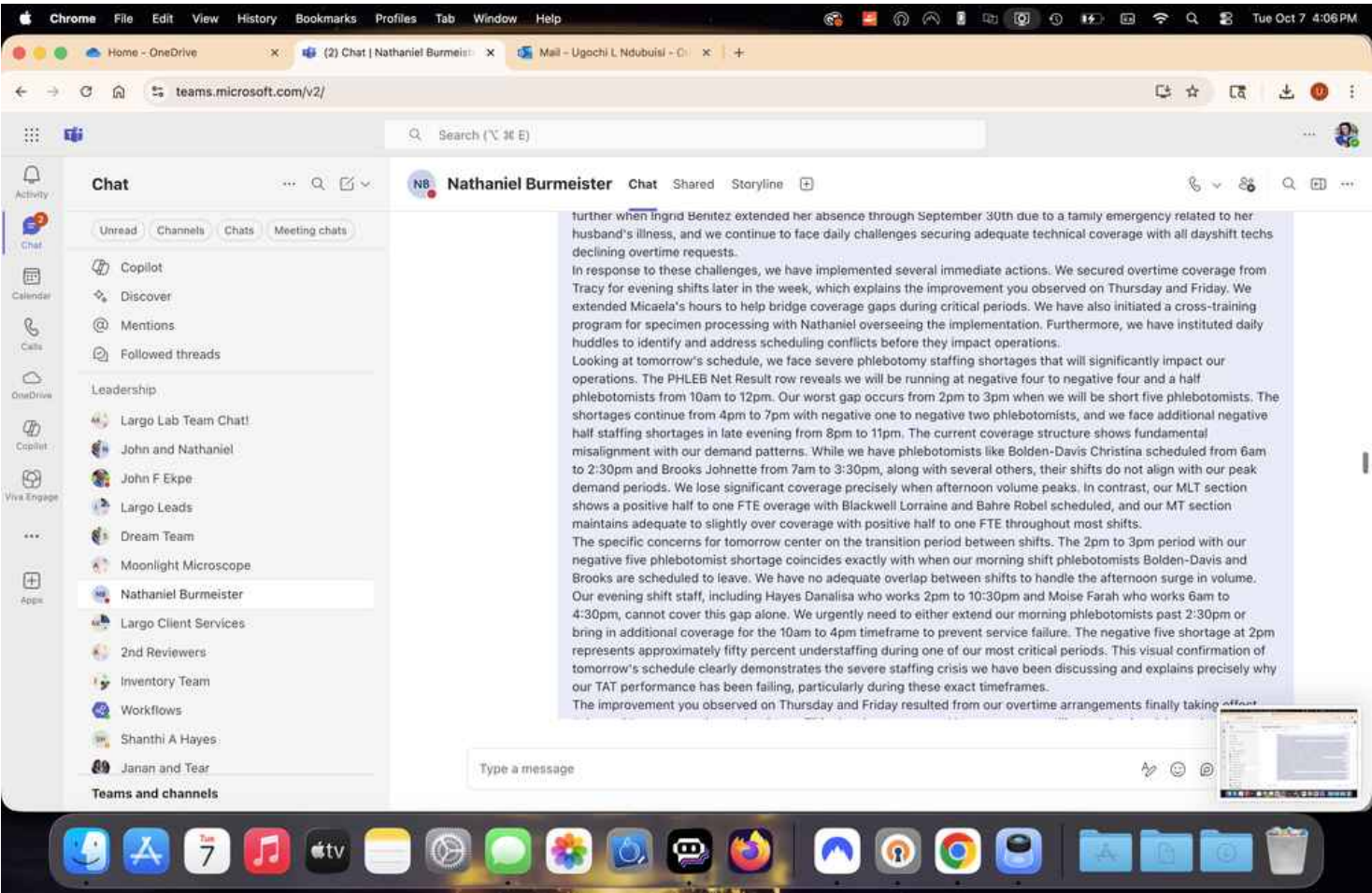


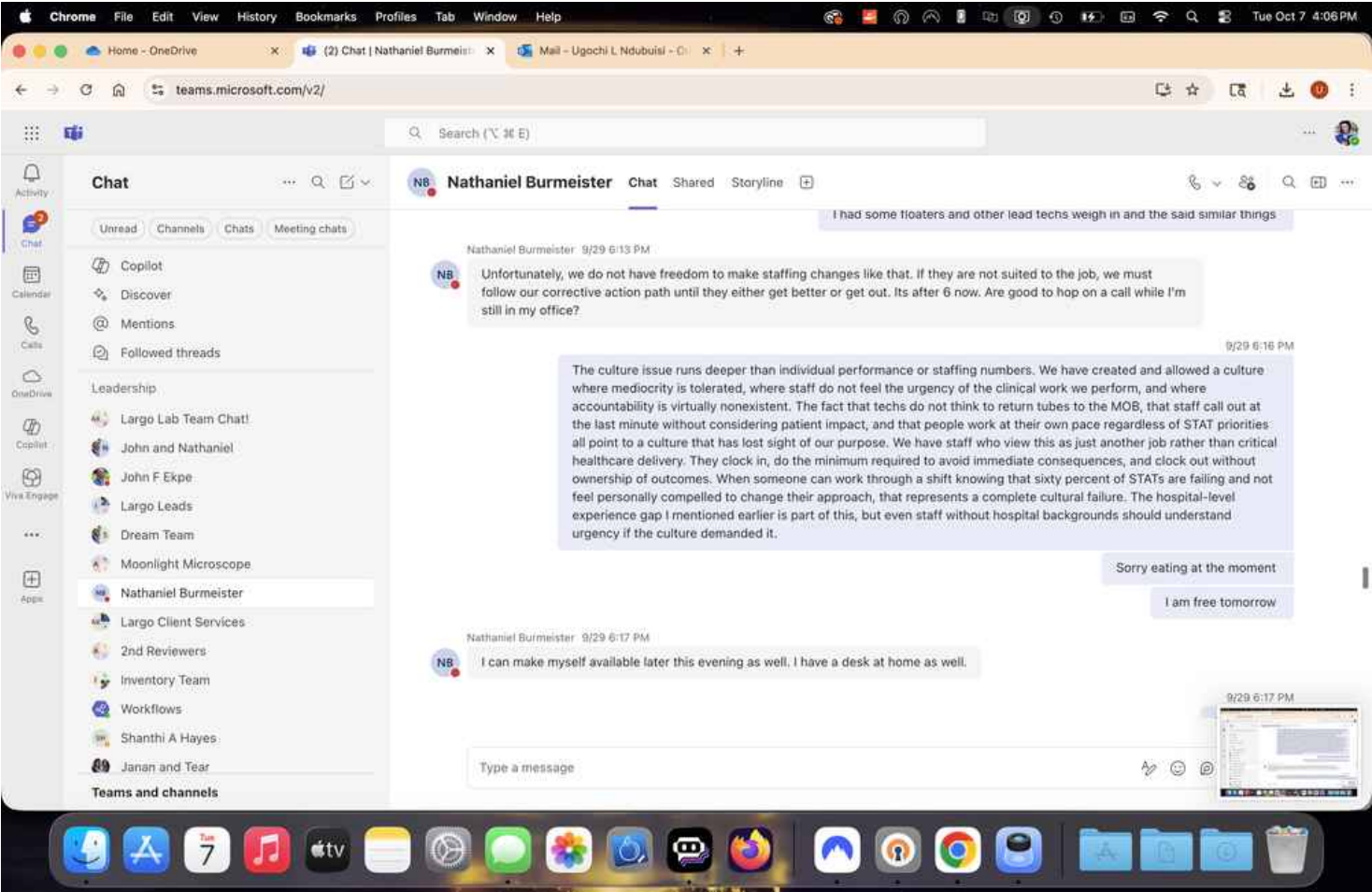




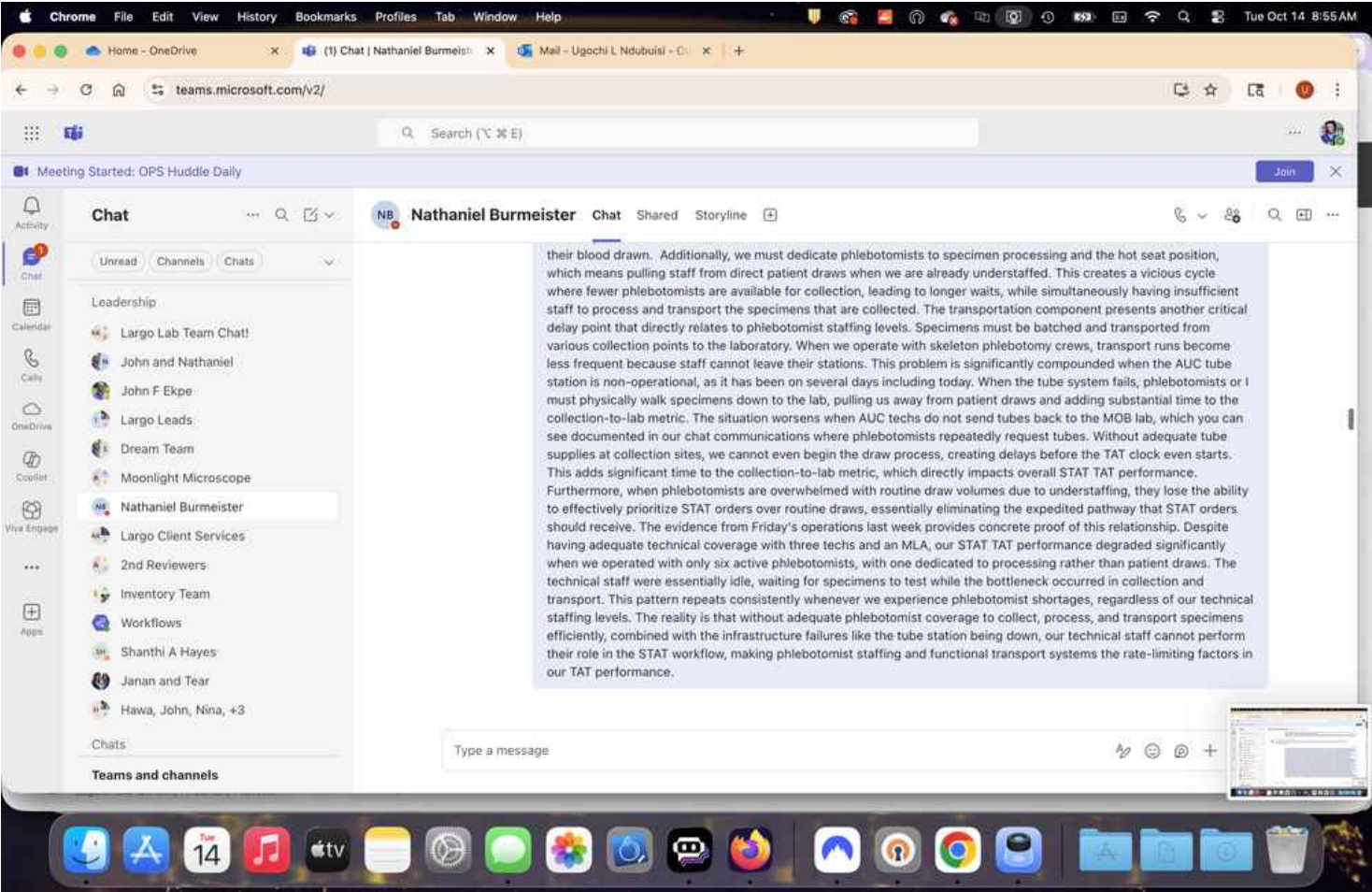




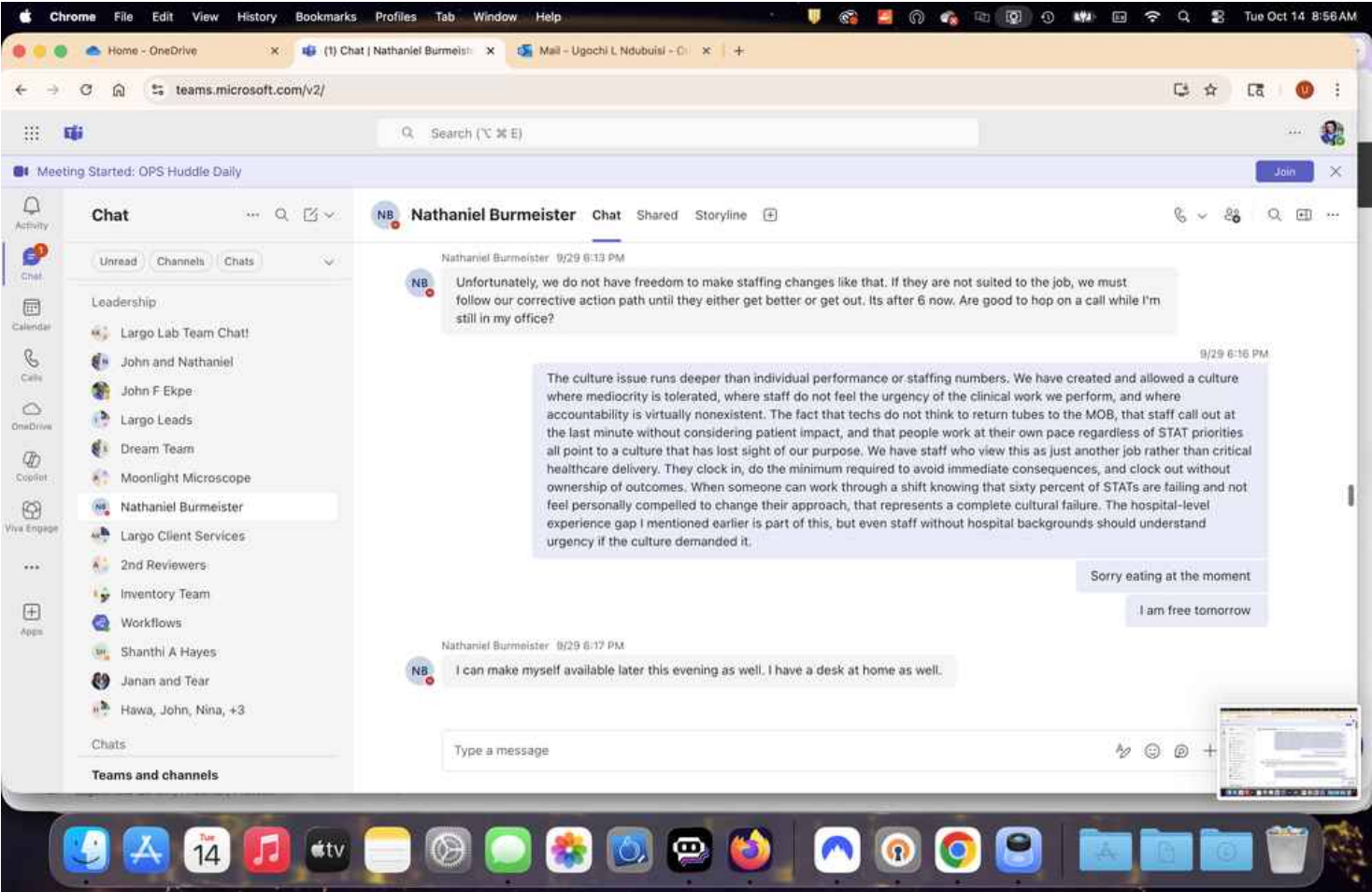


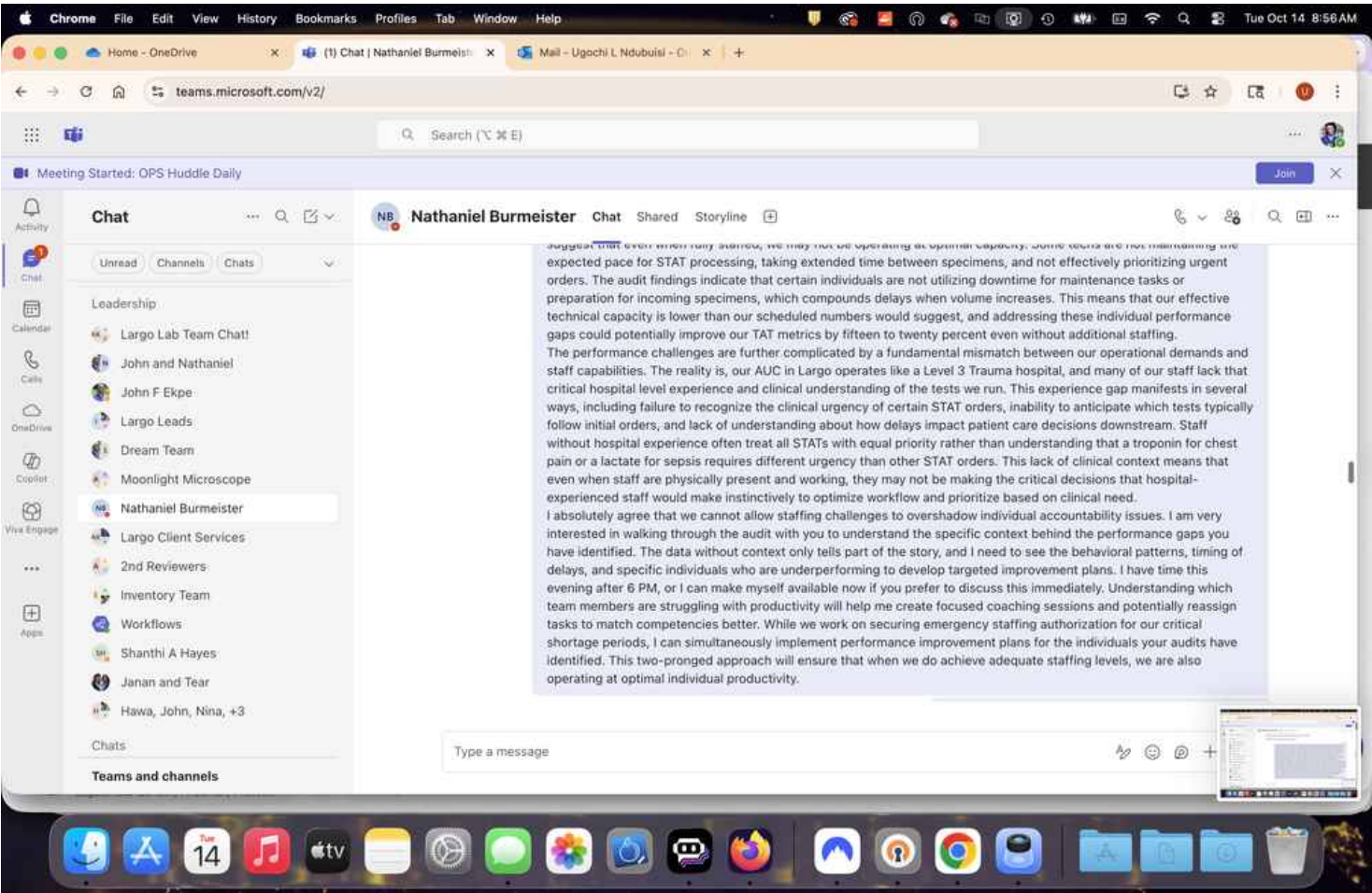


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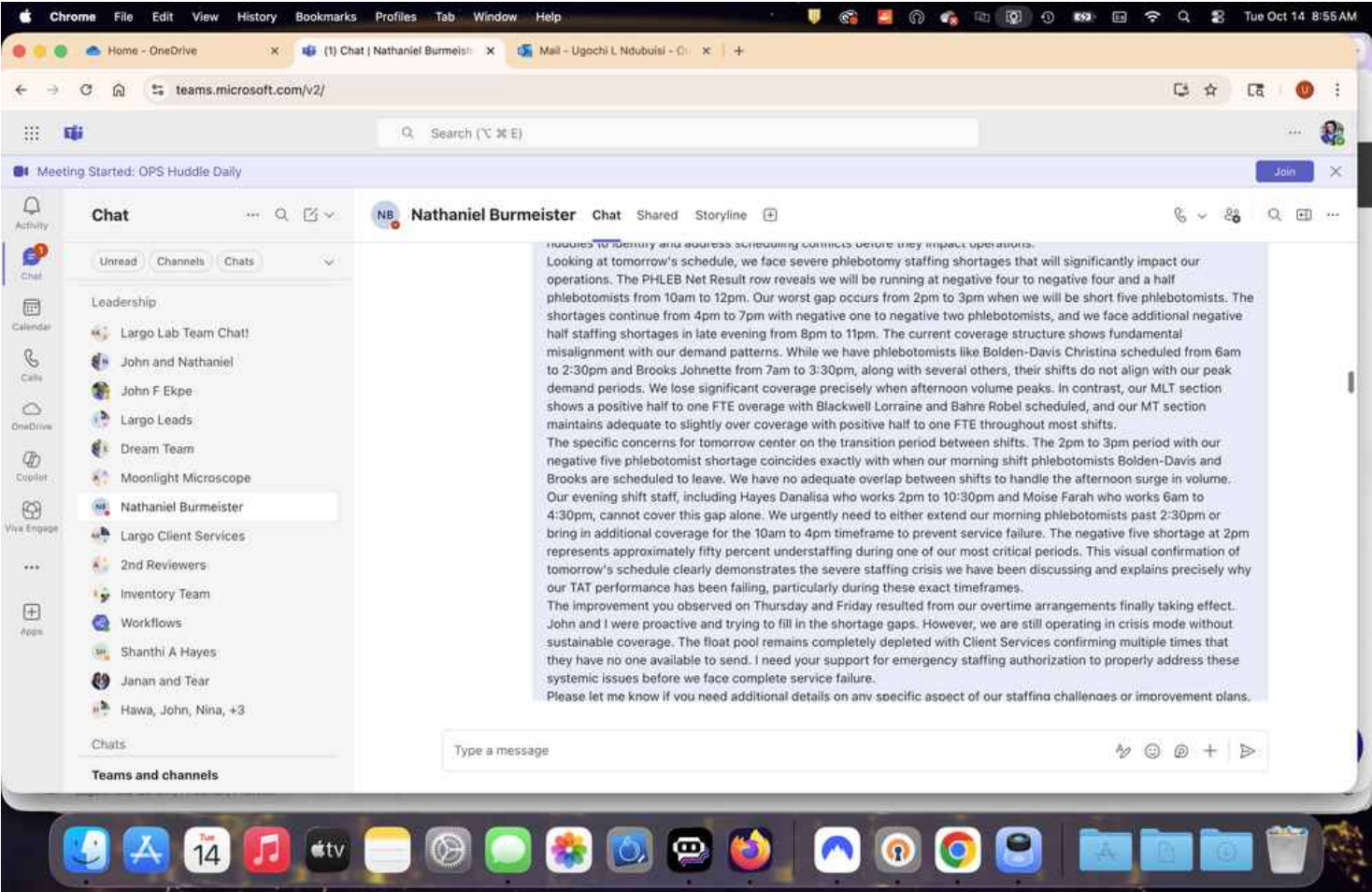
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Largo Laboratory - Kaiser Permanente

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Maryland OSHA Deadline: November 7, 2025

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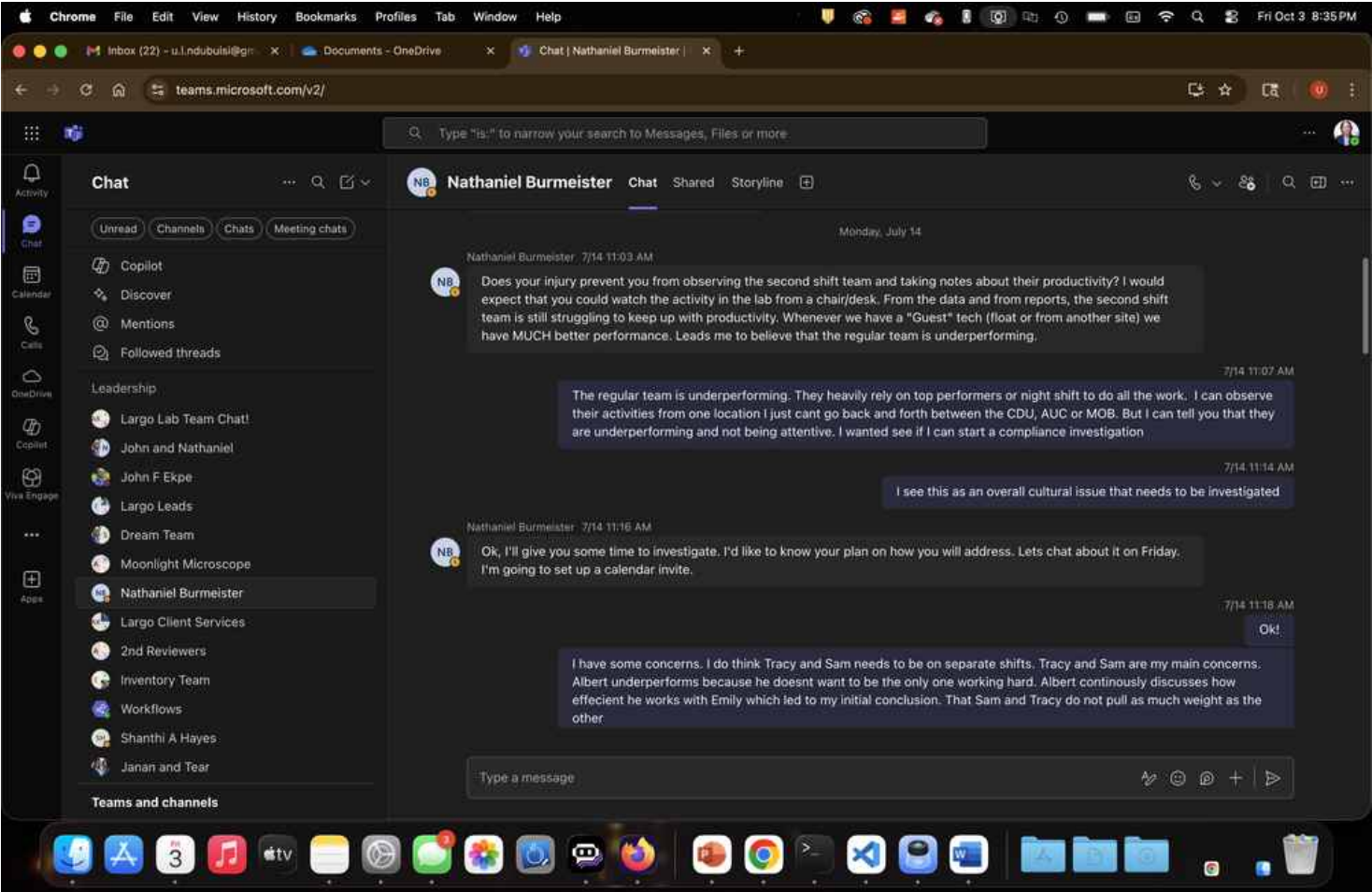
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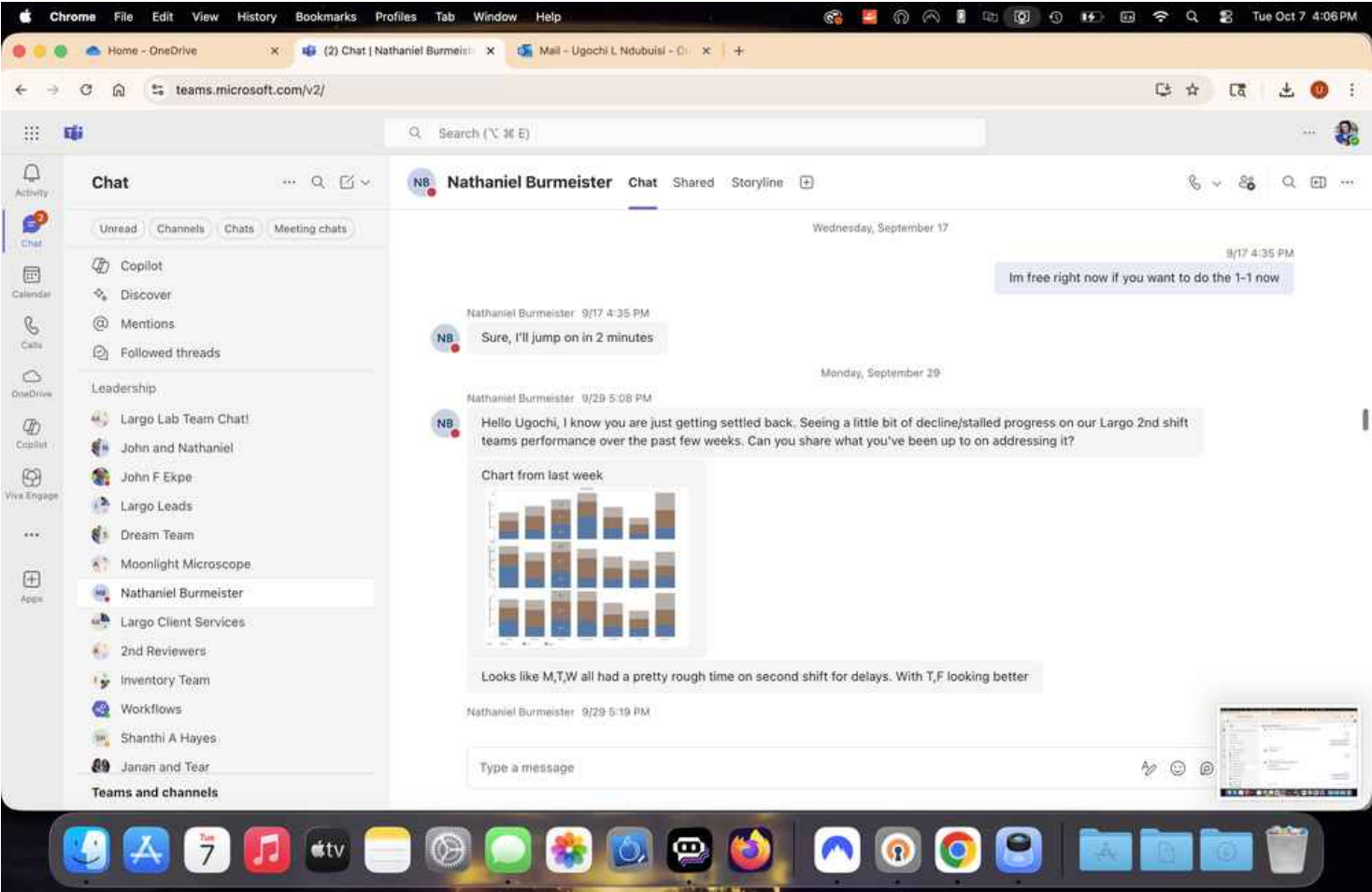
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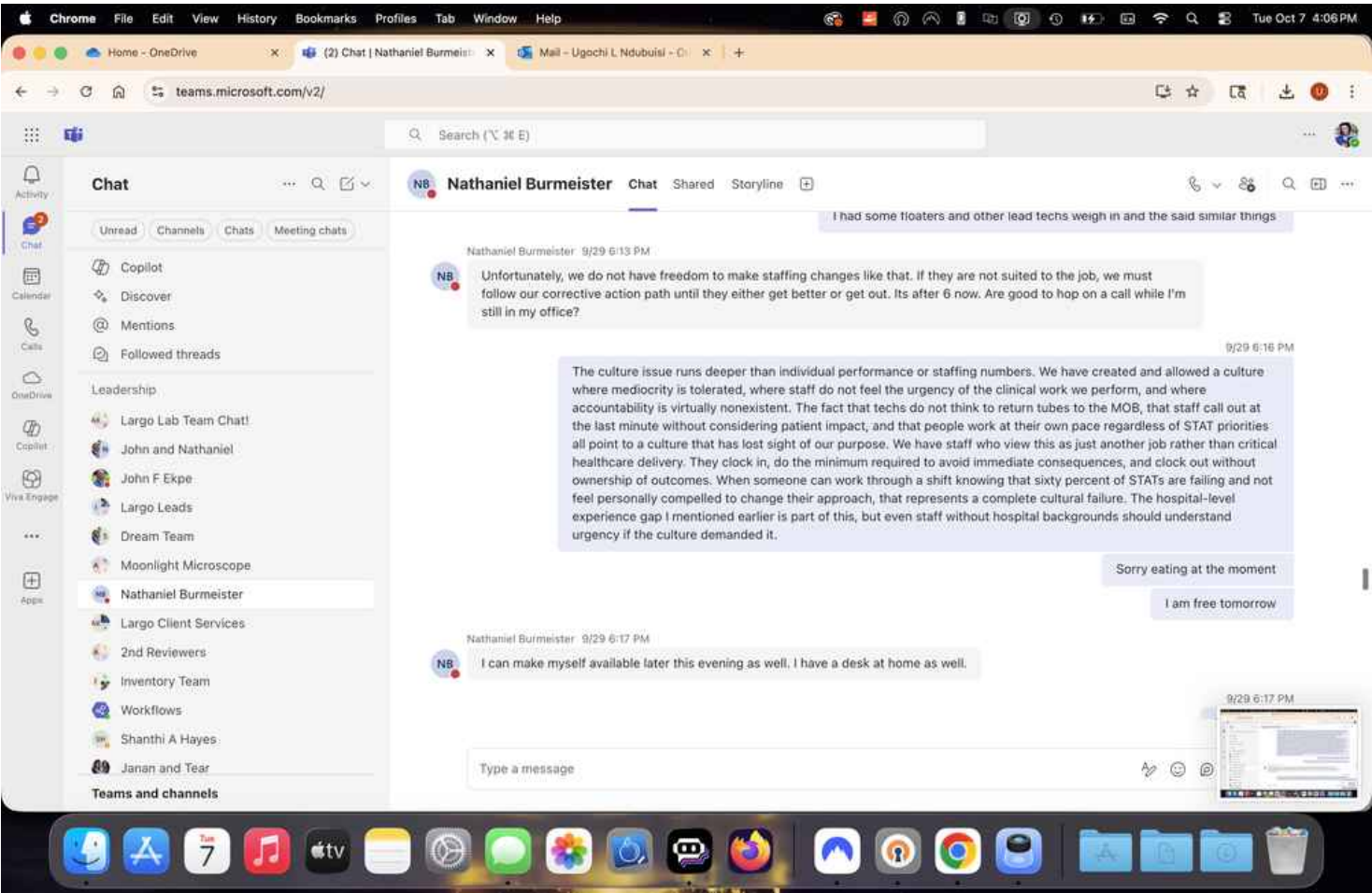
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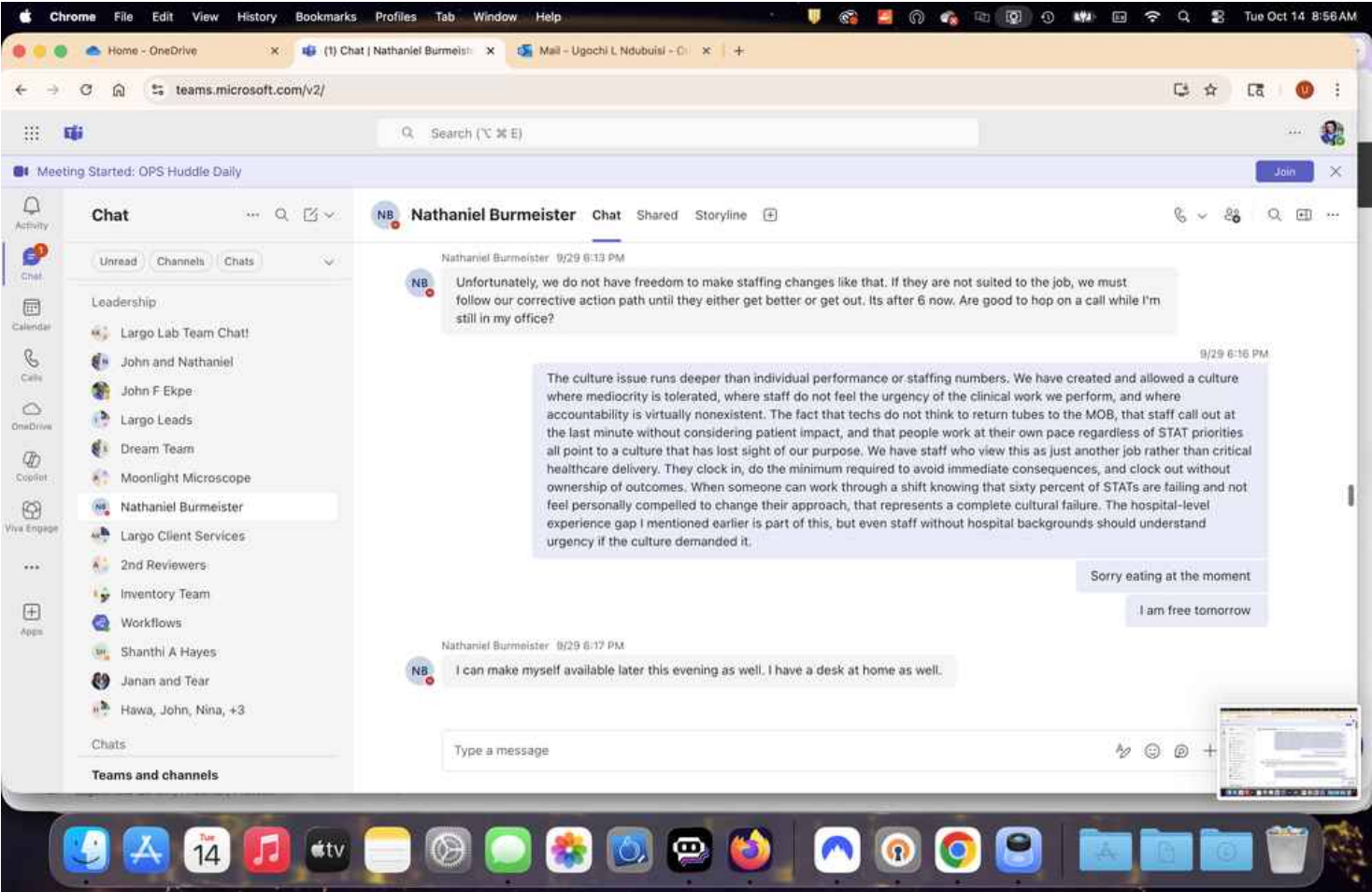
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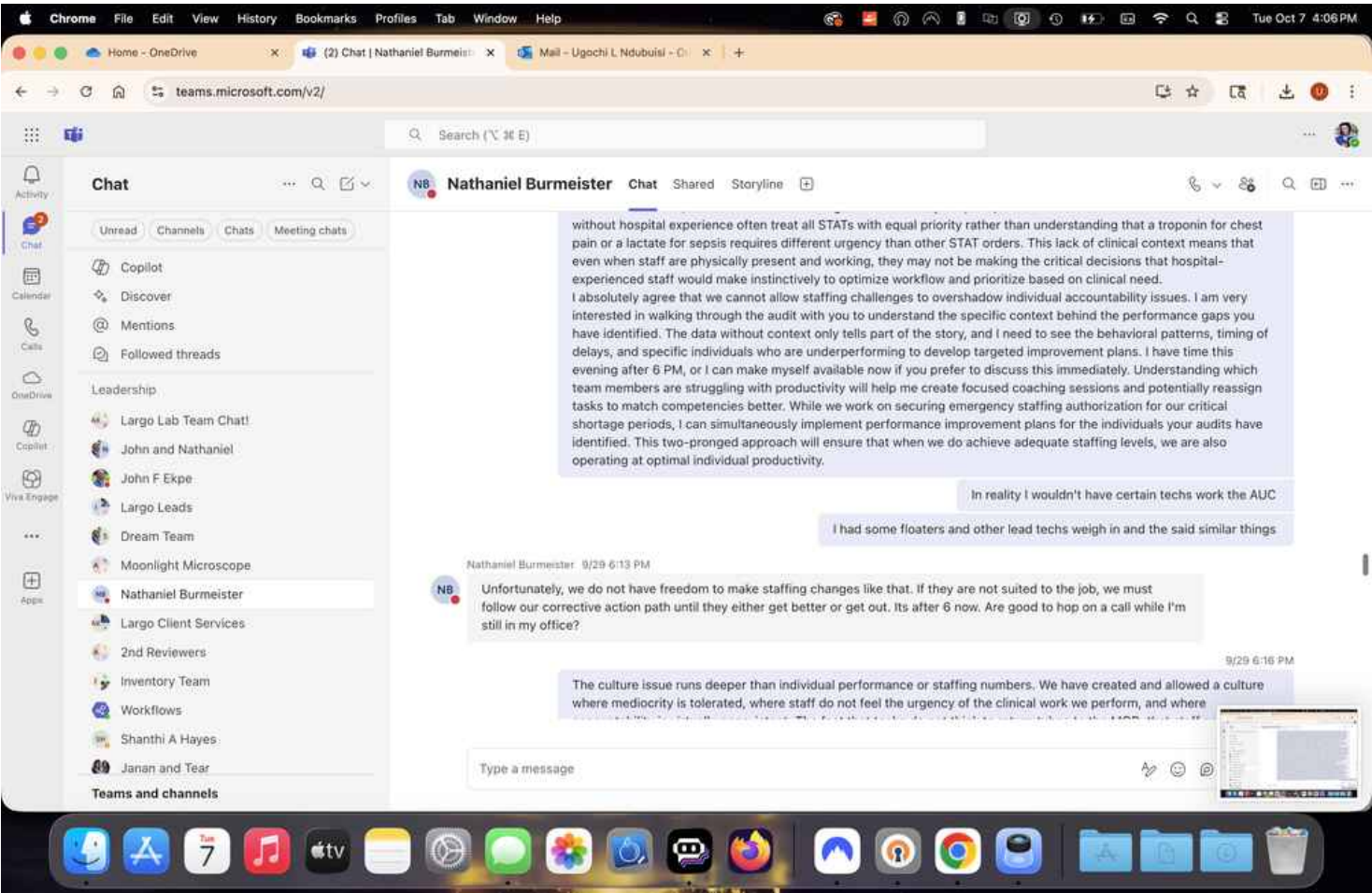


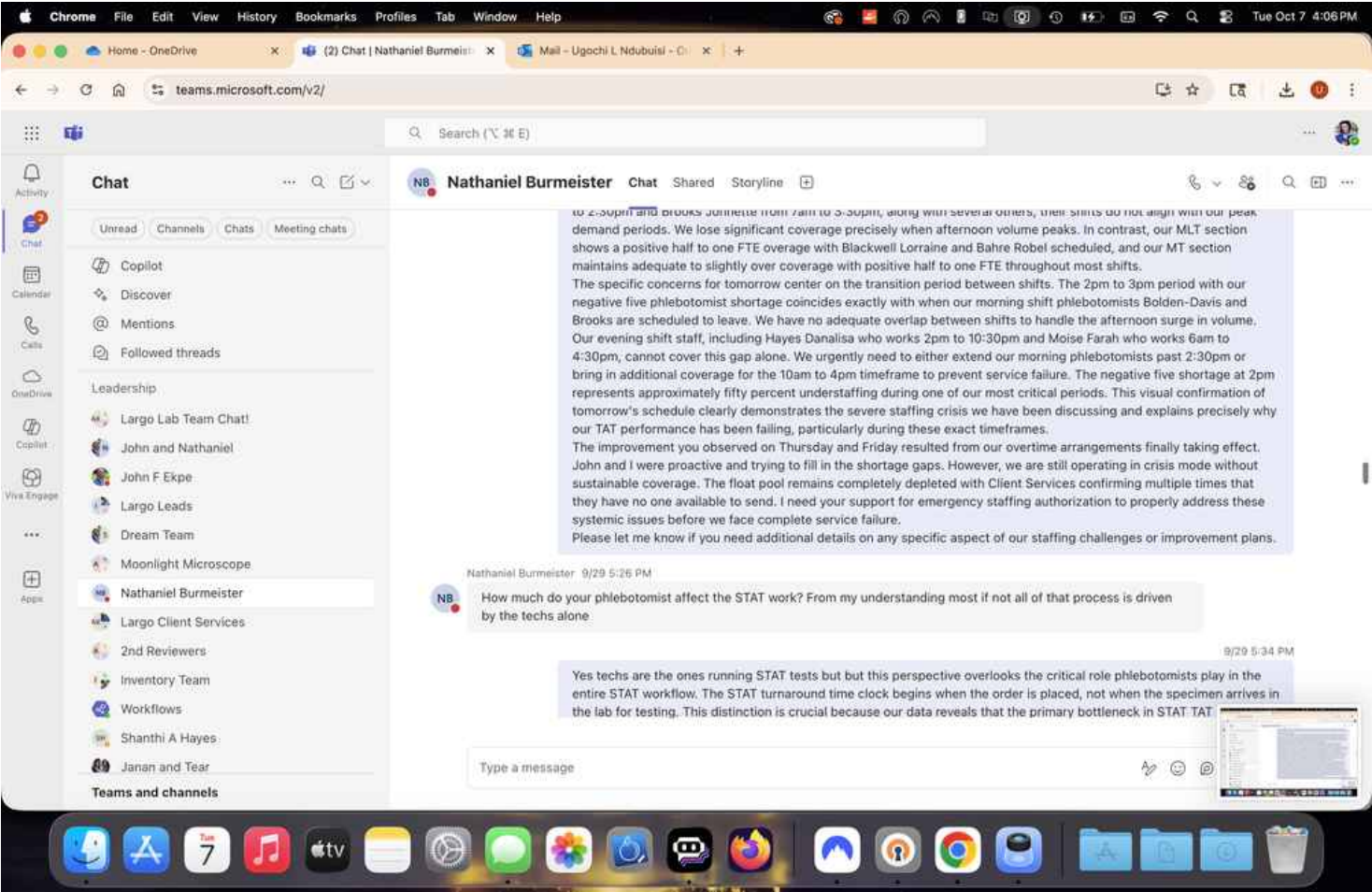
The screenshot shows a Microsoft Teams chat window on a Mac. The chat is titled "Nathaniel Burmeister" and is part of a meeting titled "OPS Huddle Daily". The chat history shows a conversation about a culture issue. The chat is part of a meeting titled "OPS Huddle Daily". The chat history includes messages from Nathaniel Burmeister and Ugochi L. Ndubuisi. The chat is part of a meeting titled "OPS Huddle Daily".

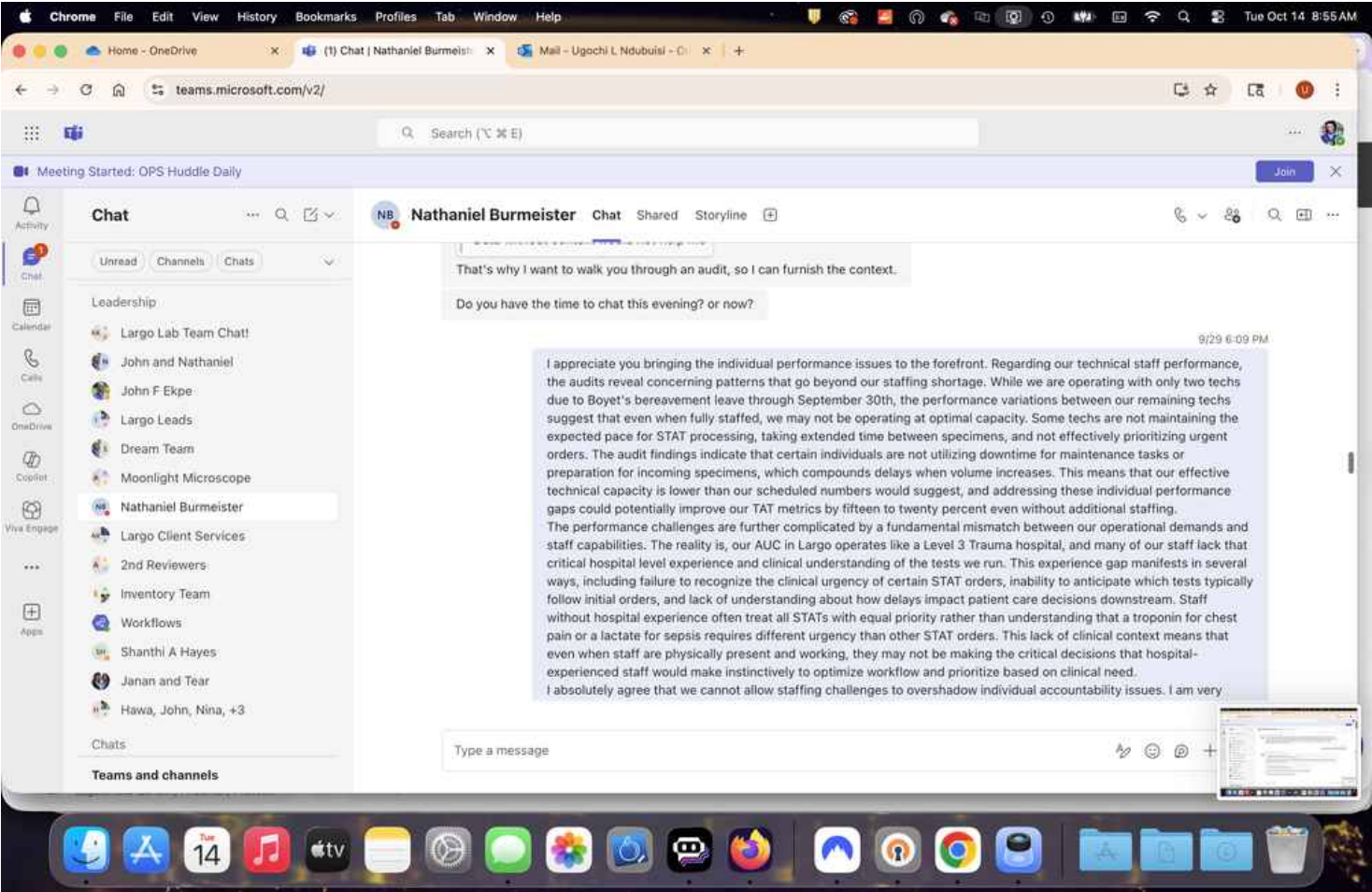
Chat History:

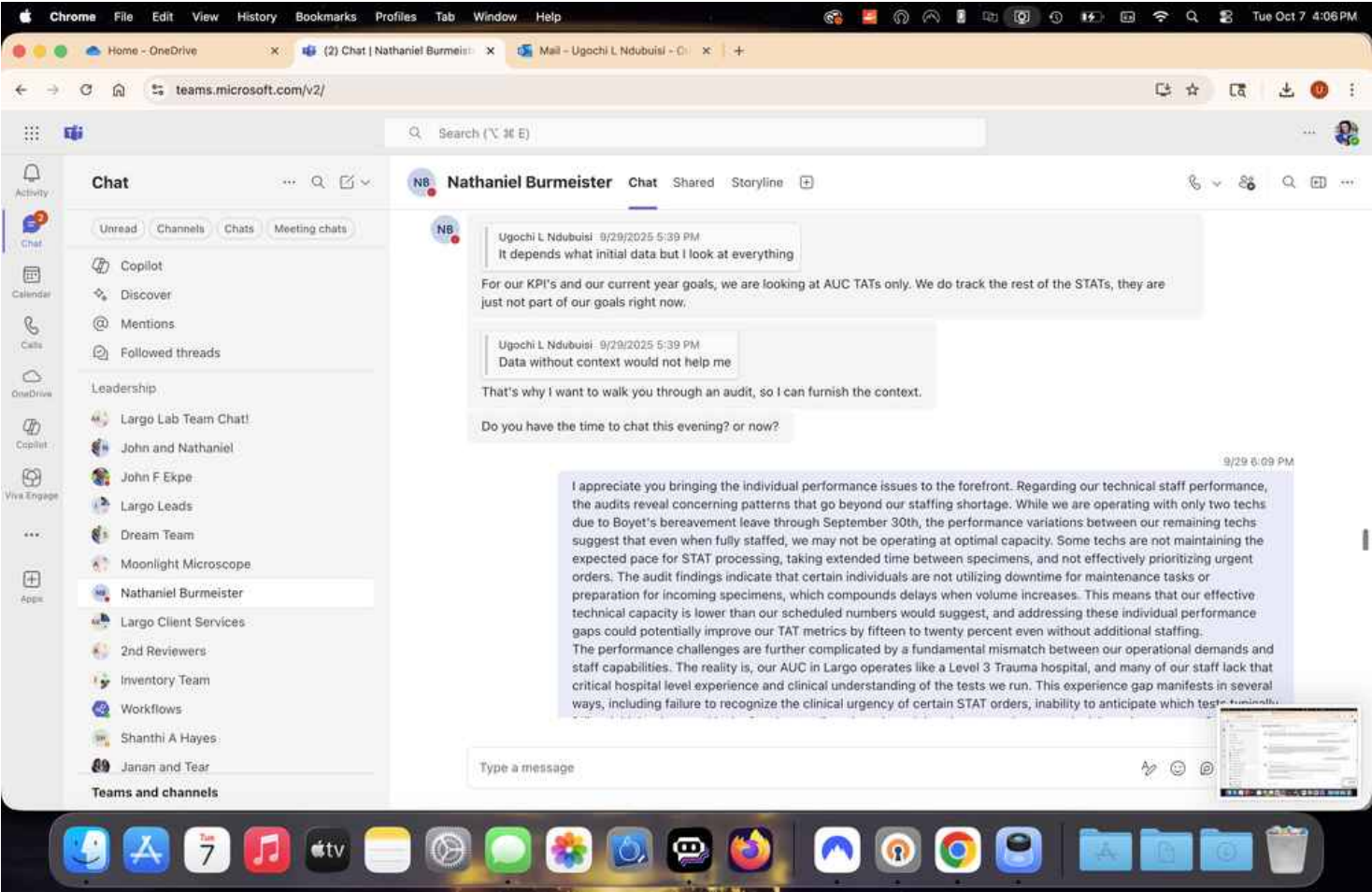
- Nathaniel Burmeister** 9/29 6:17 PM: I can make myself available later this evening as well. I have a desk at home as well.
- Ugochi L. Ndubuisi** 9/29/2025 6:16 PM: The culture issue runs deeper than individual performance or staffing numbers. We have created and allowed a culture where...
- Nathaniel Burmeister** 9/29 6:17 PM: That works for me
- Ugochi L. Ndubuisi** 9/29 6:22 PM: Absolutely!

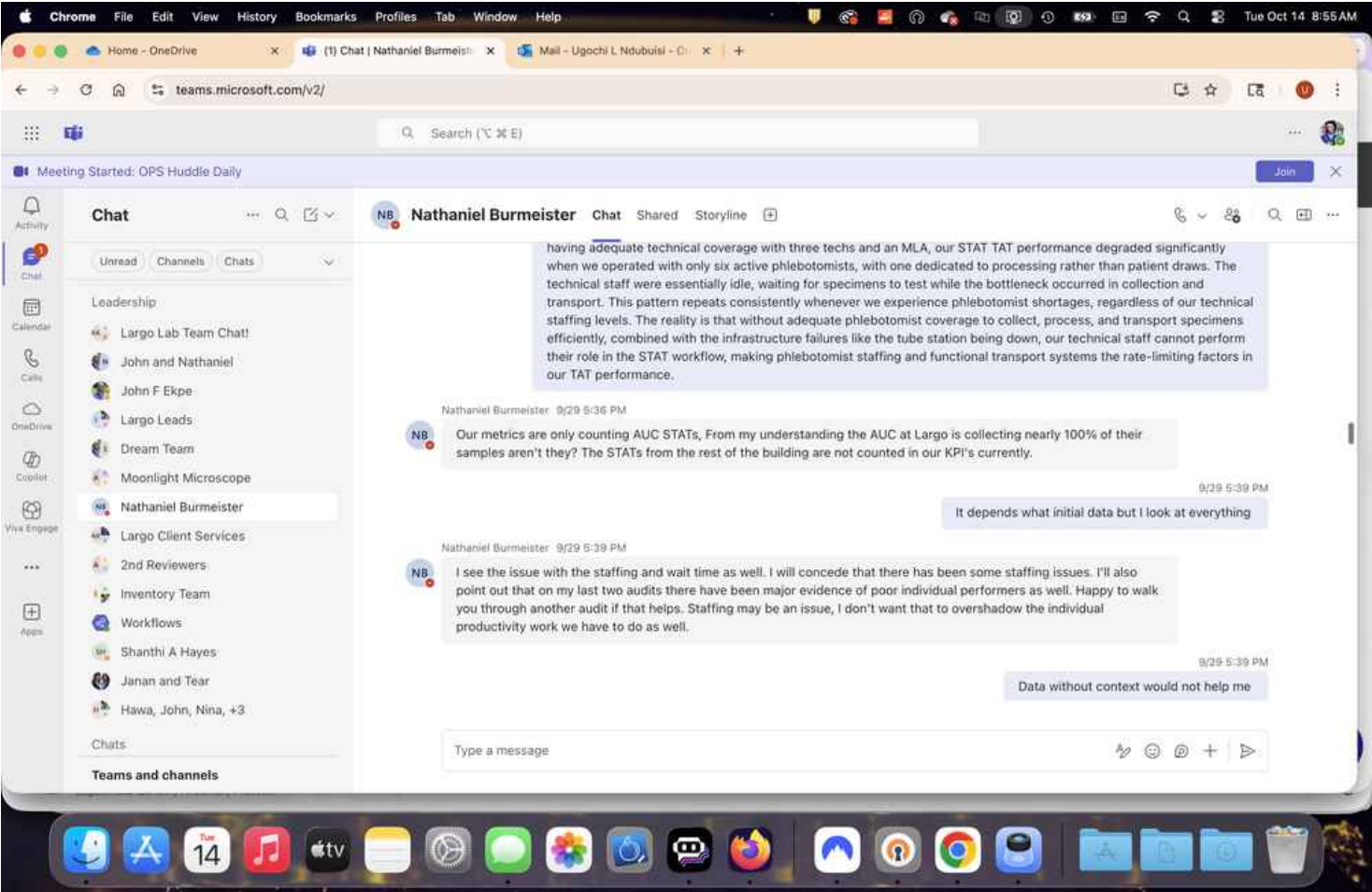
Mac Dock (from left to right): Safari, App Store, Calendar, Music, Apple TV, Photos, Reminders, Messages, Notes, Maps, WhatsApp, Telegram, Firefox, Brave, Chrome, Slack, OneDrive, File Explorer, Mail, Trash.

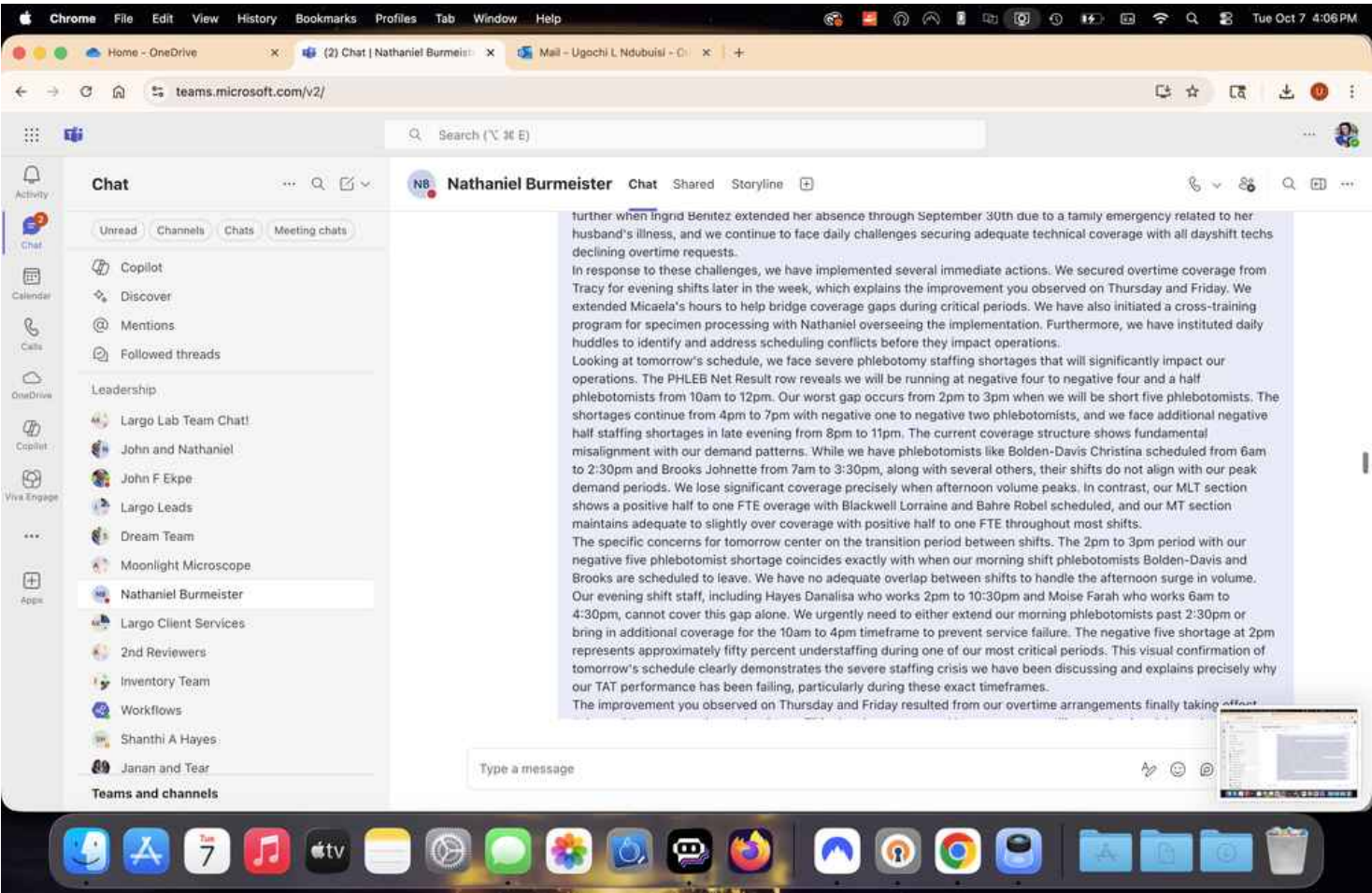


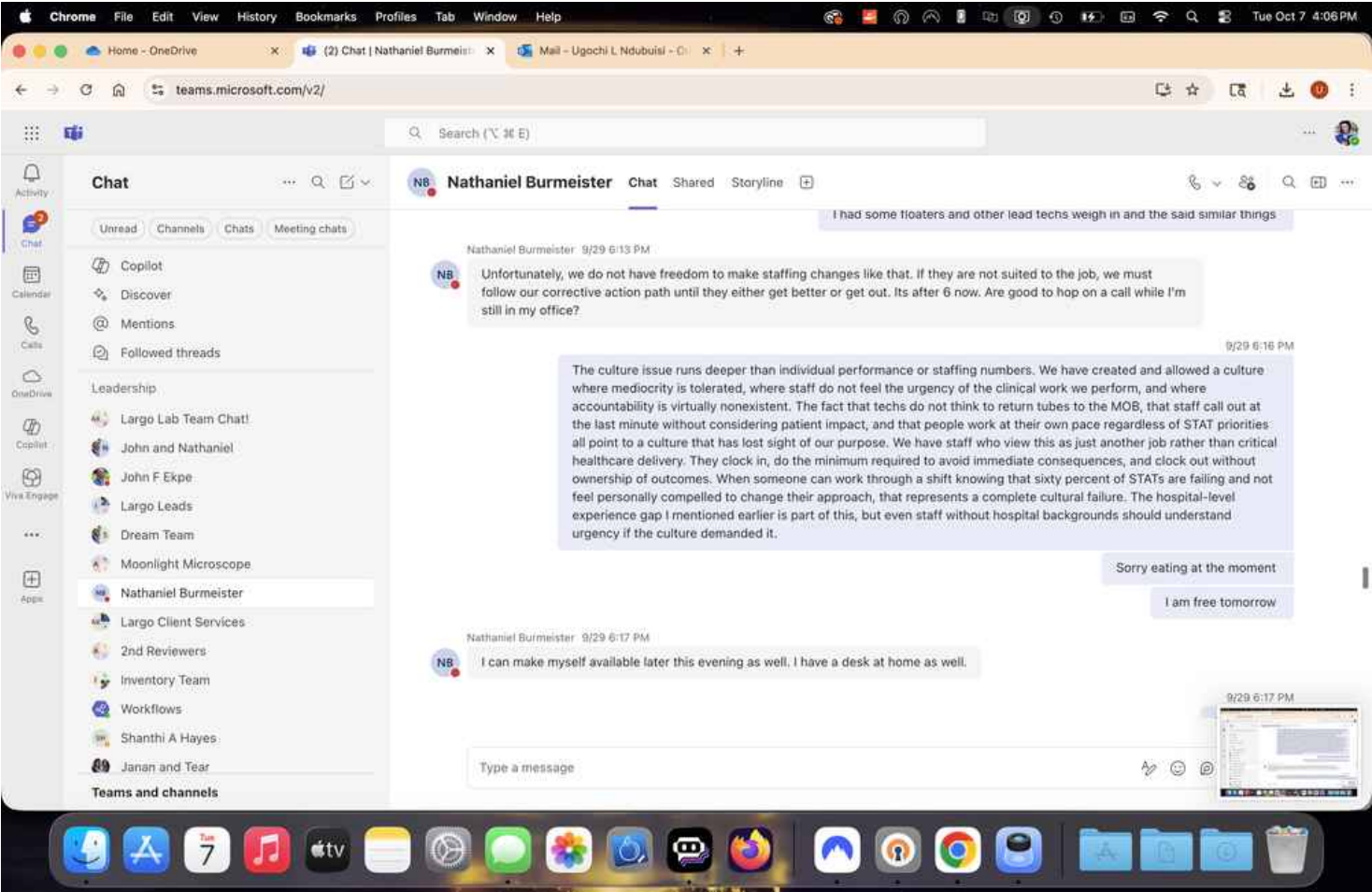




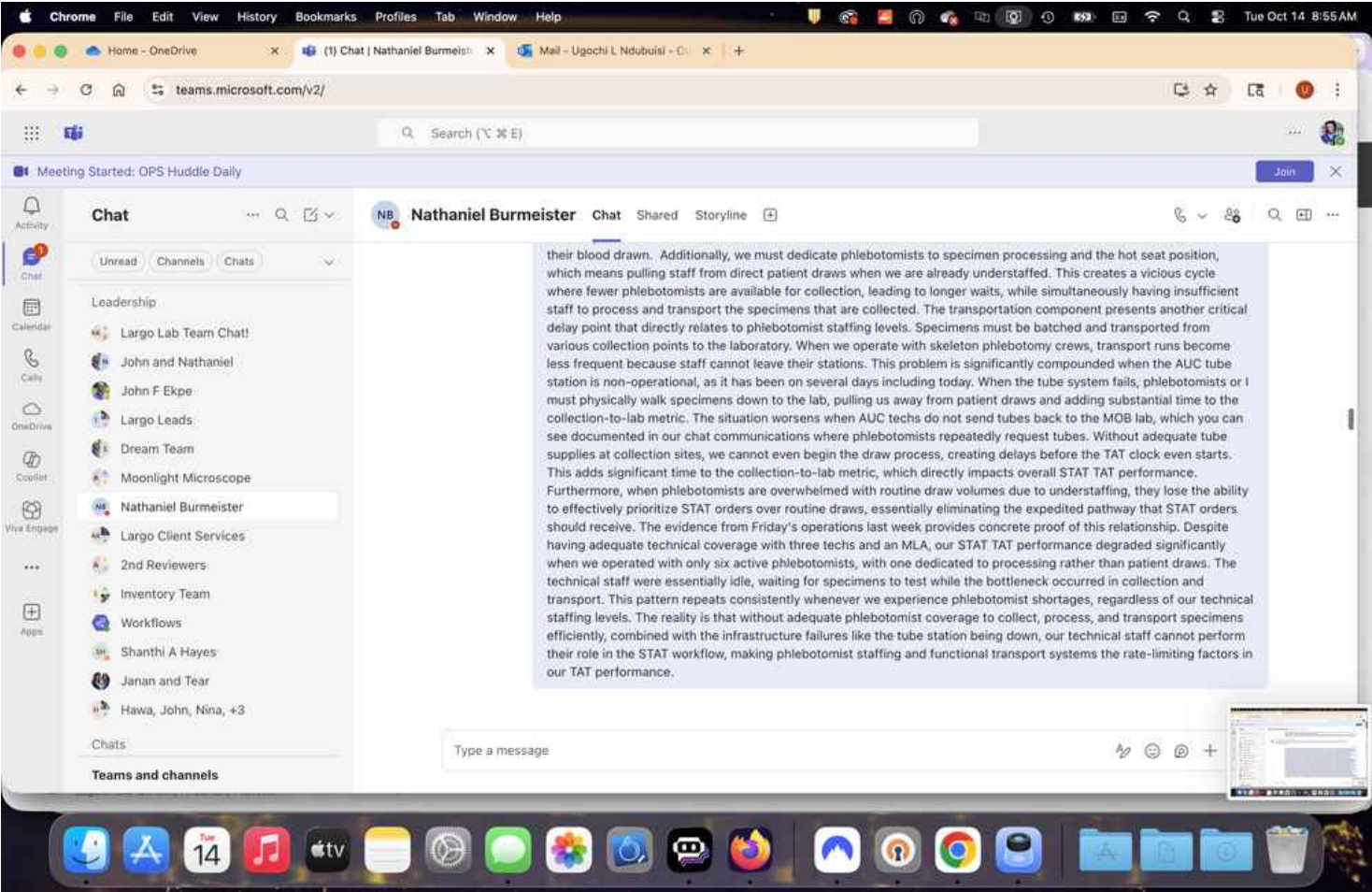




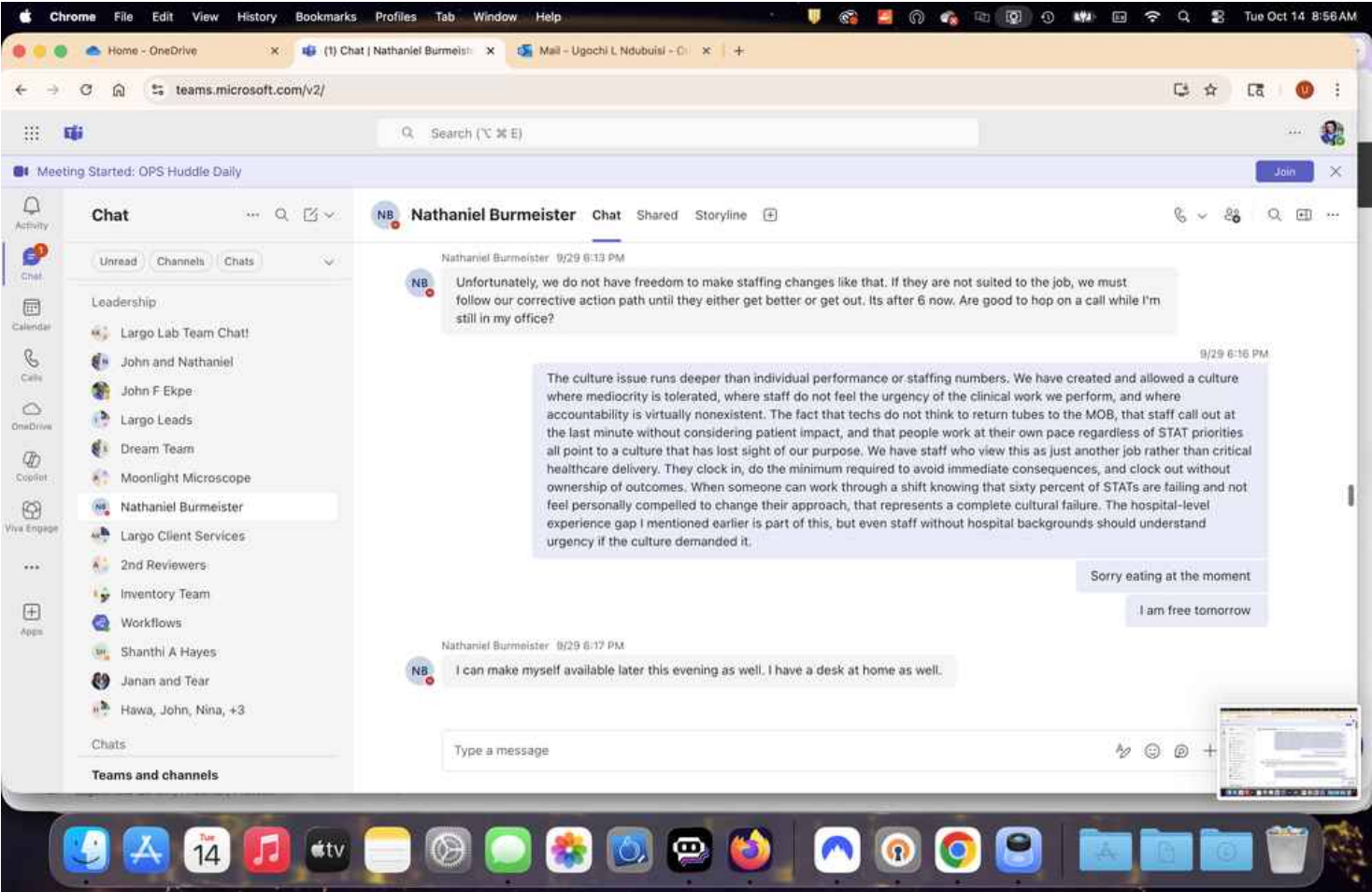


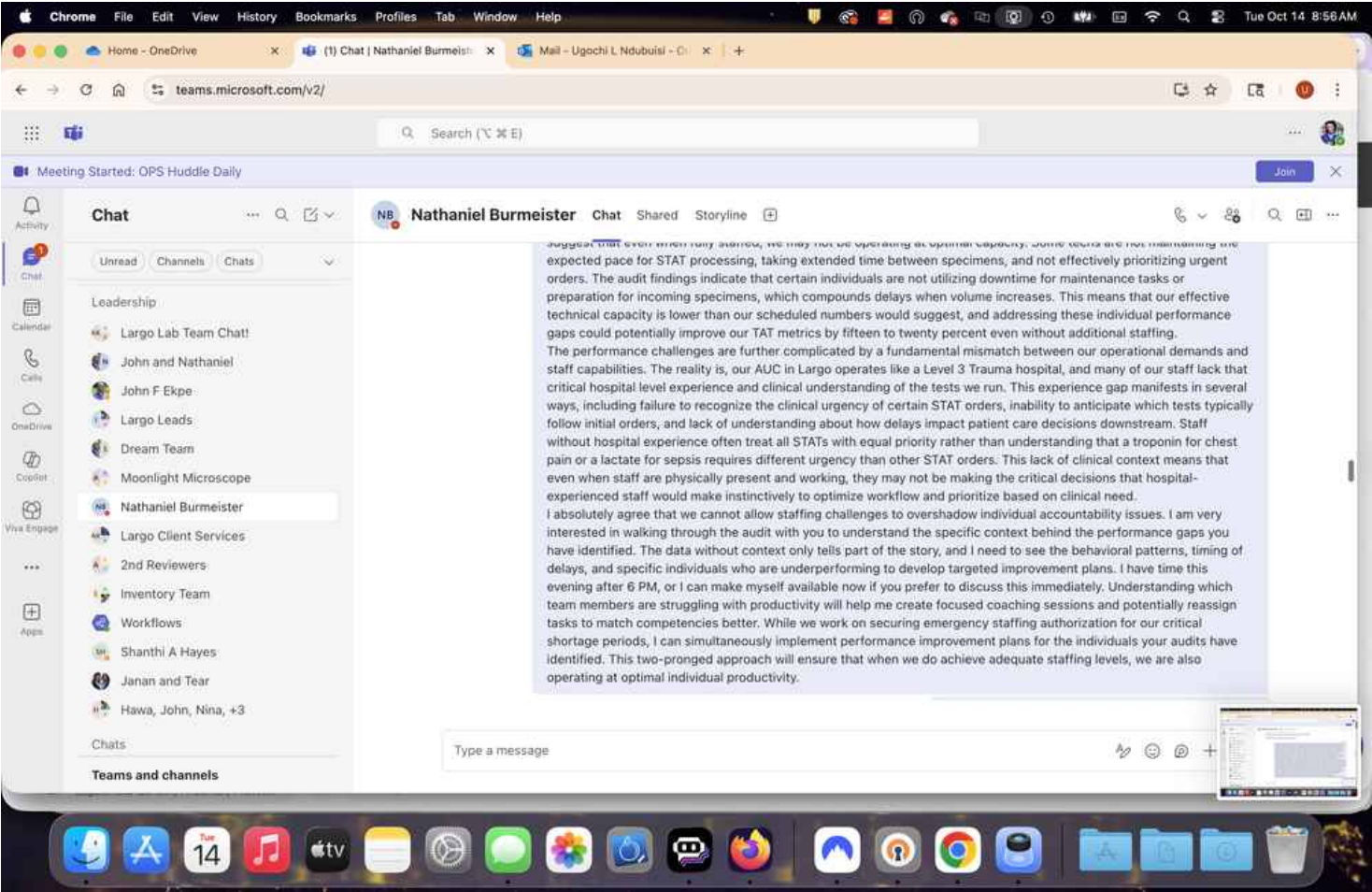


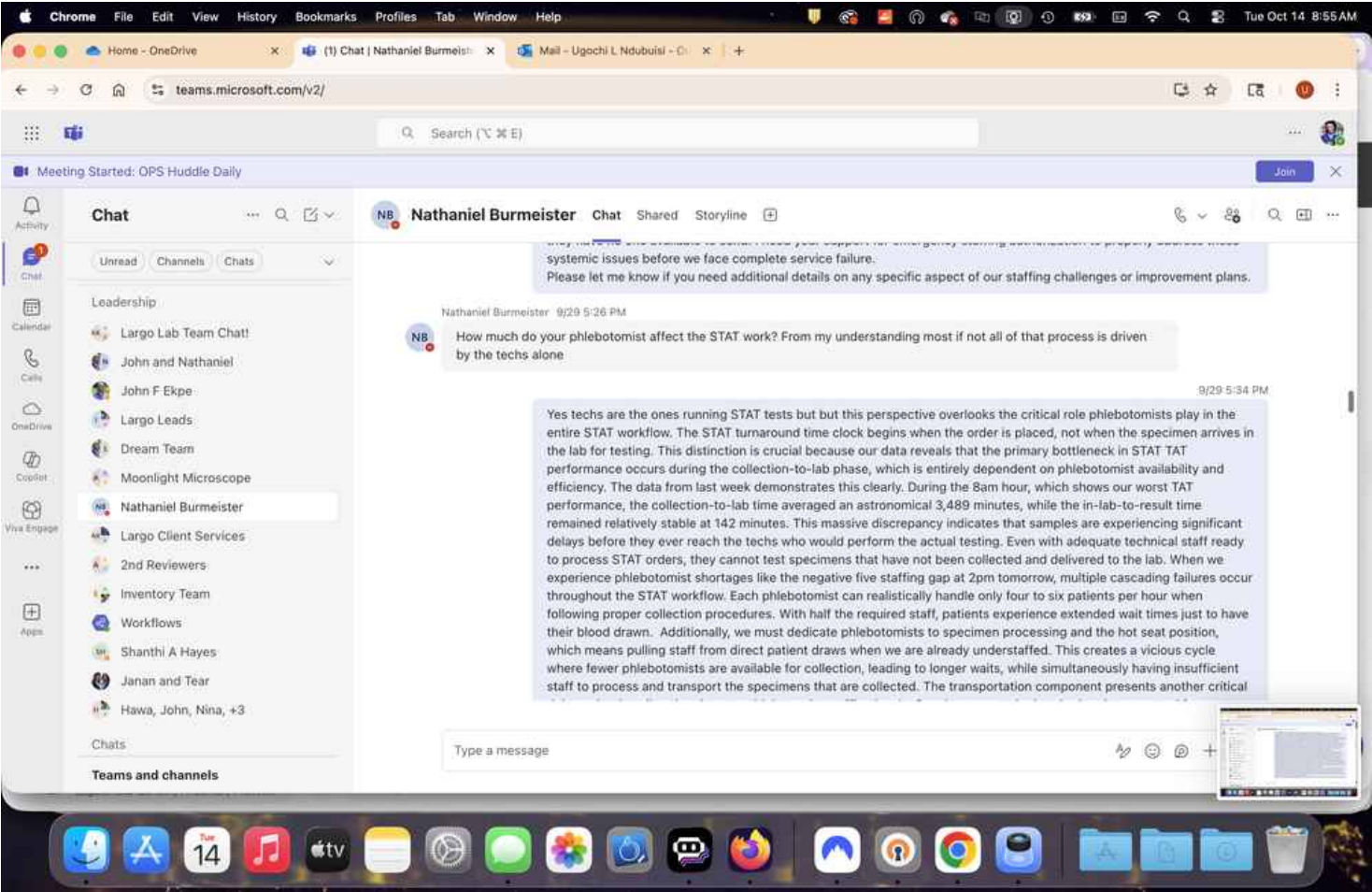
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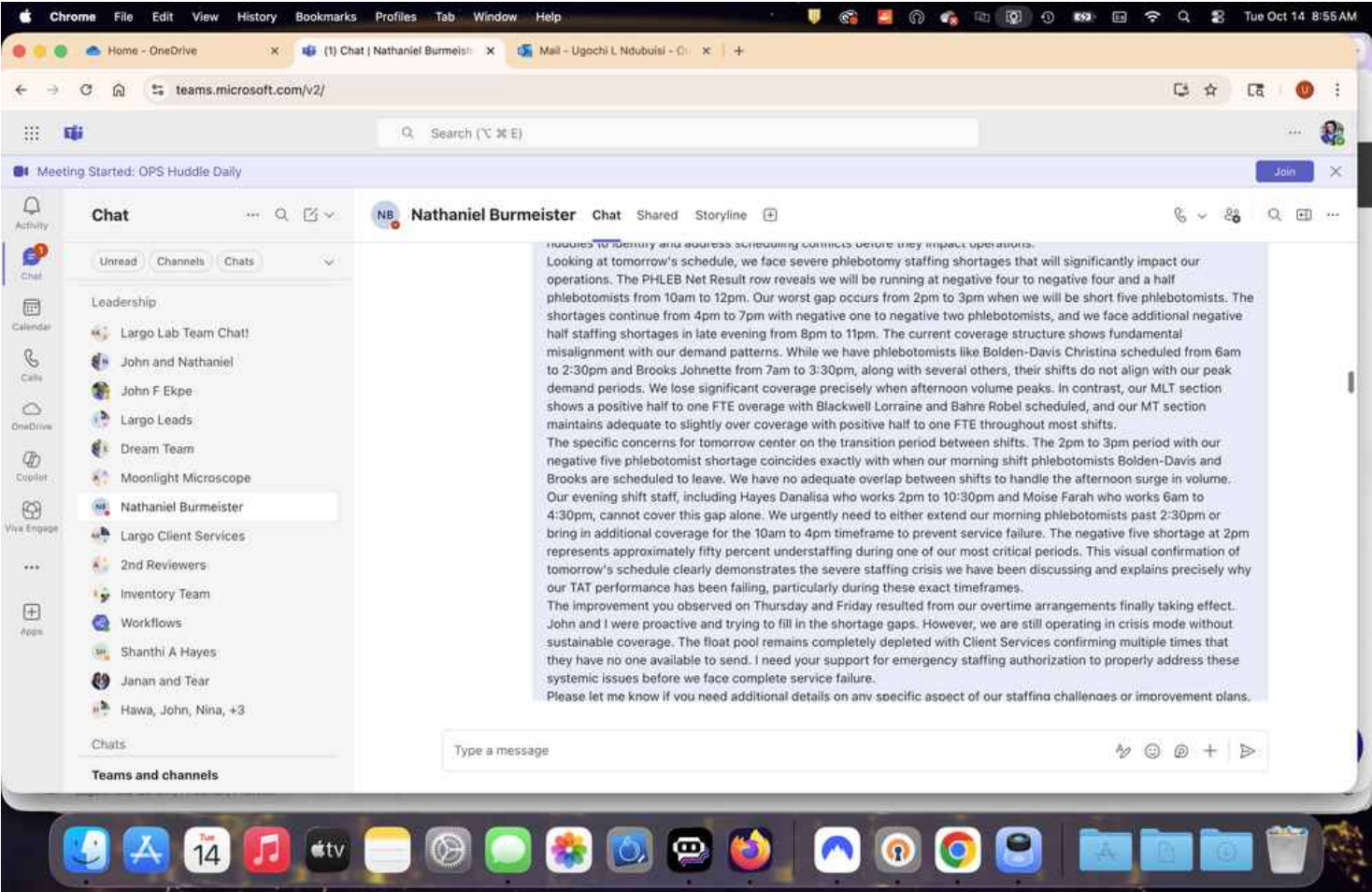
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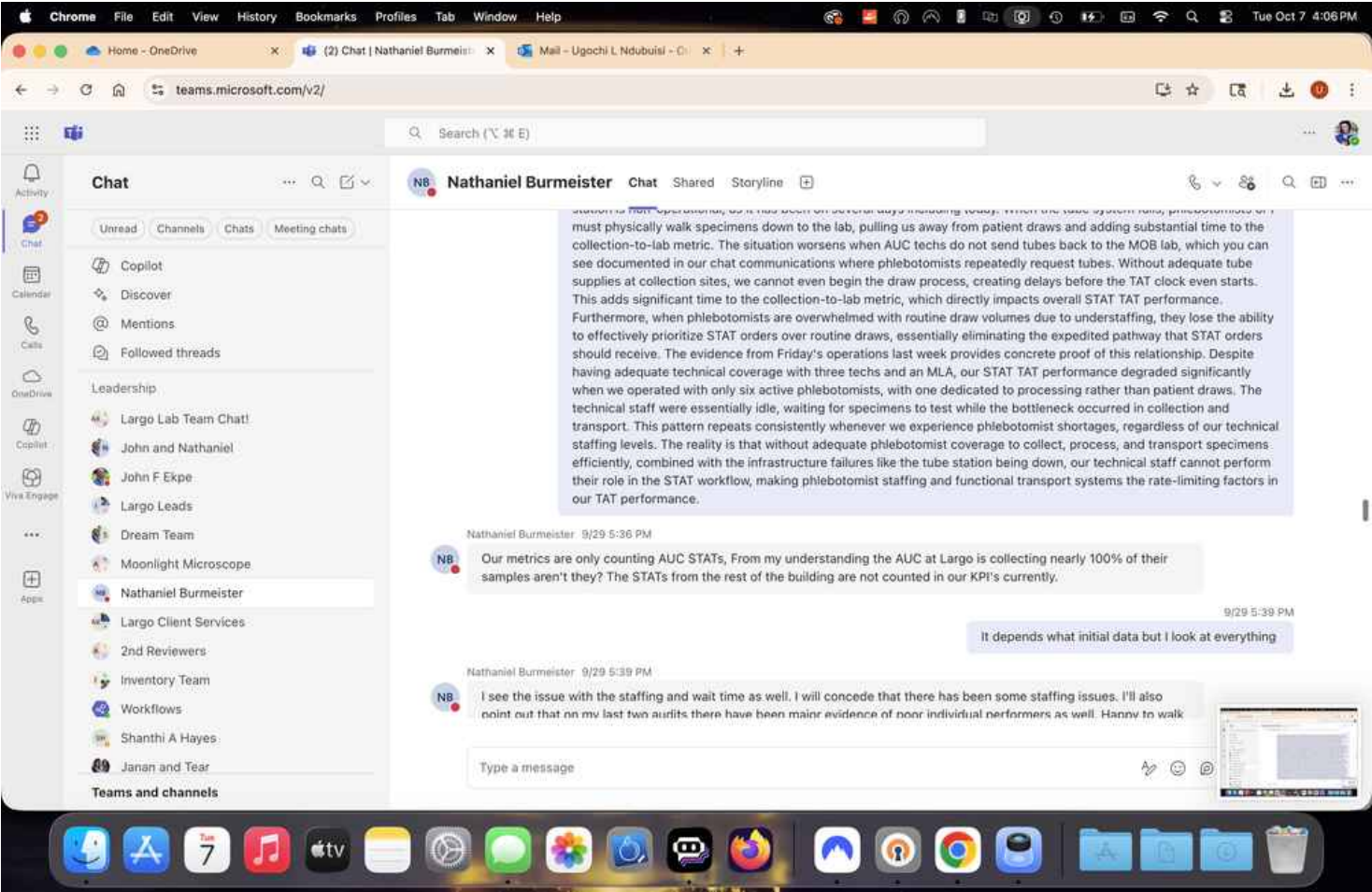






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Largo Laboratory - Kaiser Permanente

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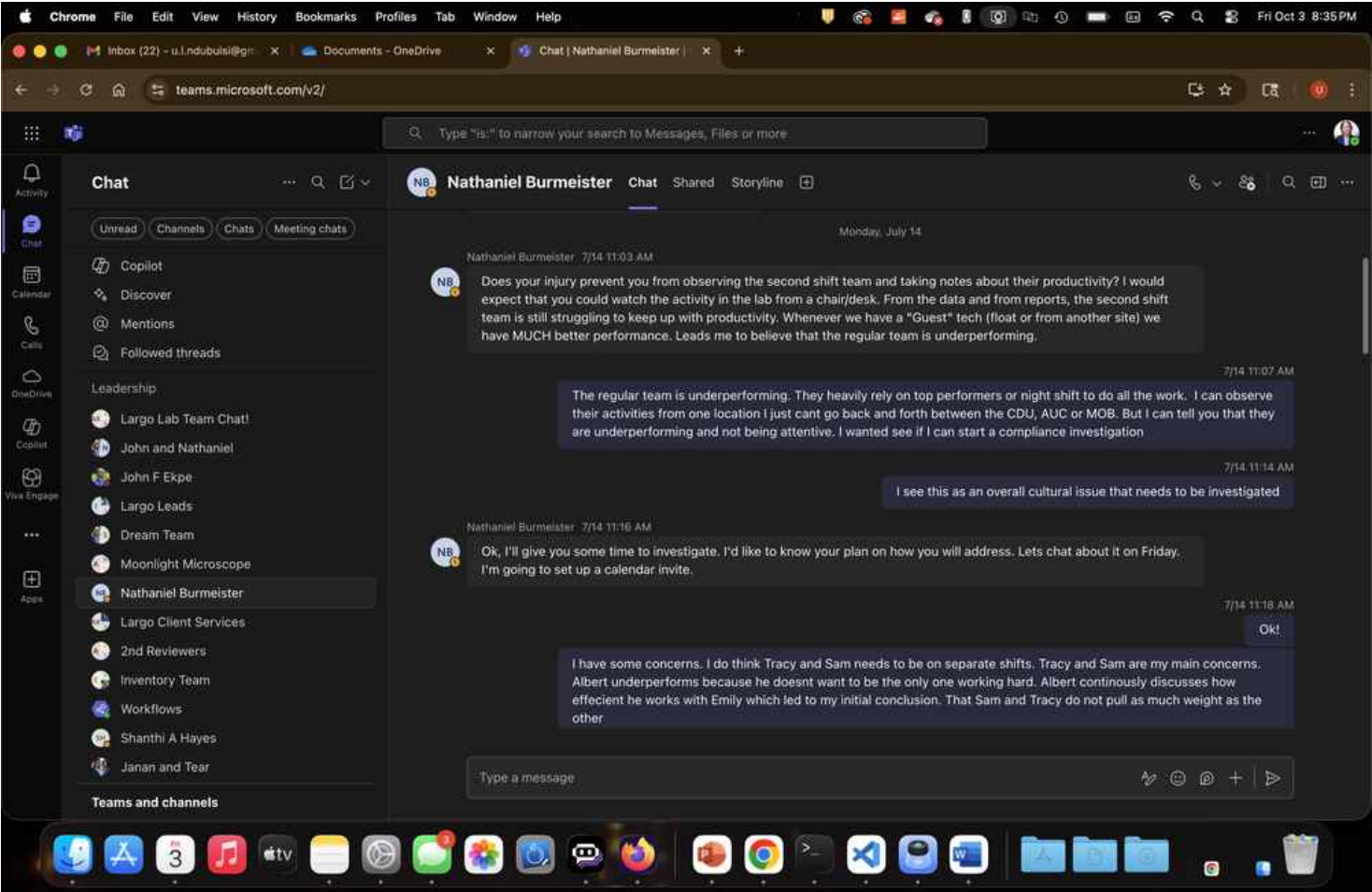
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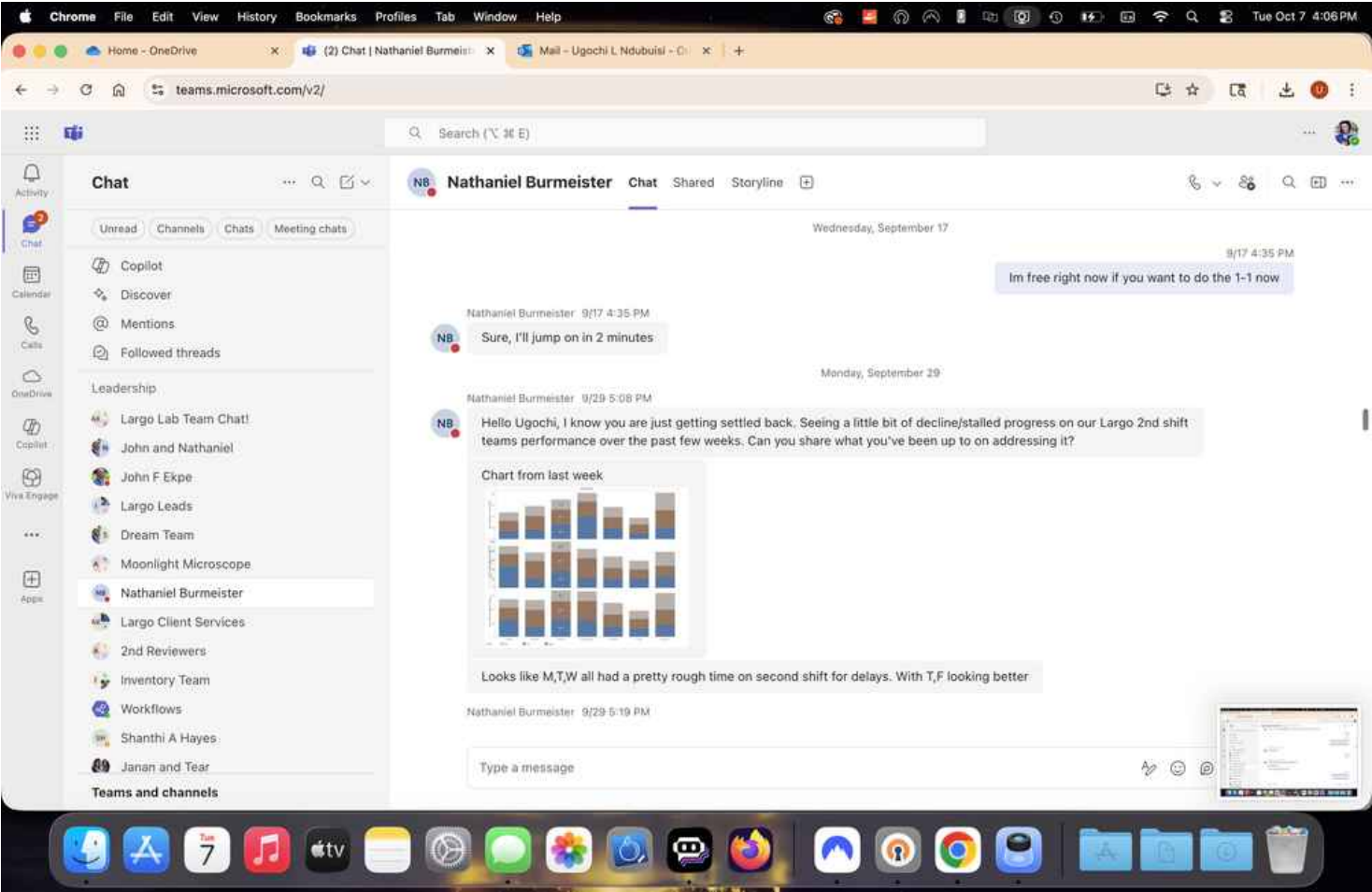
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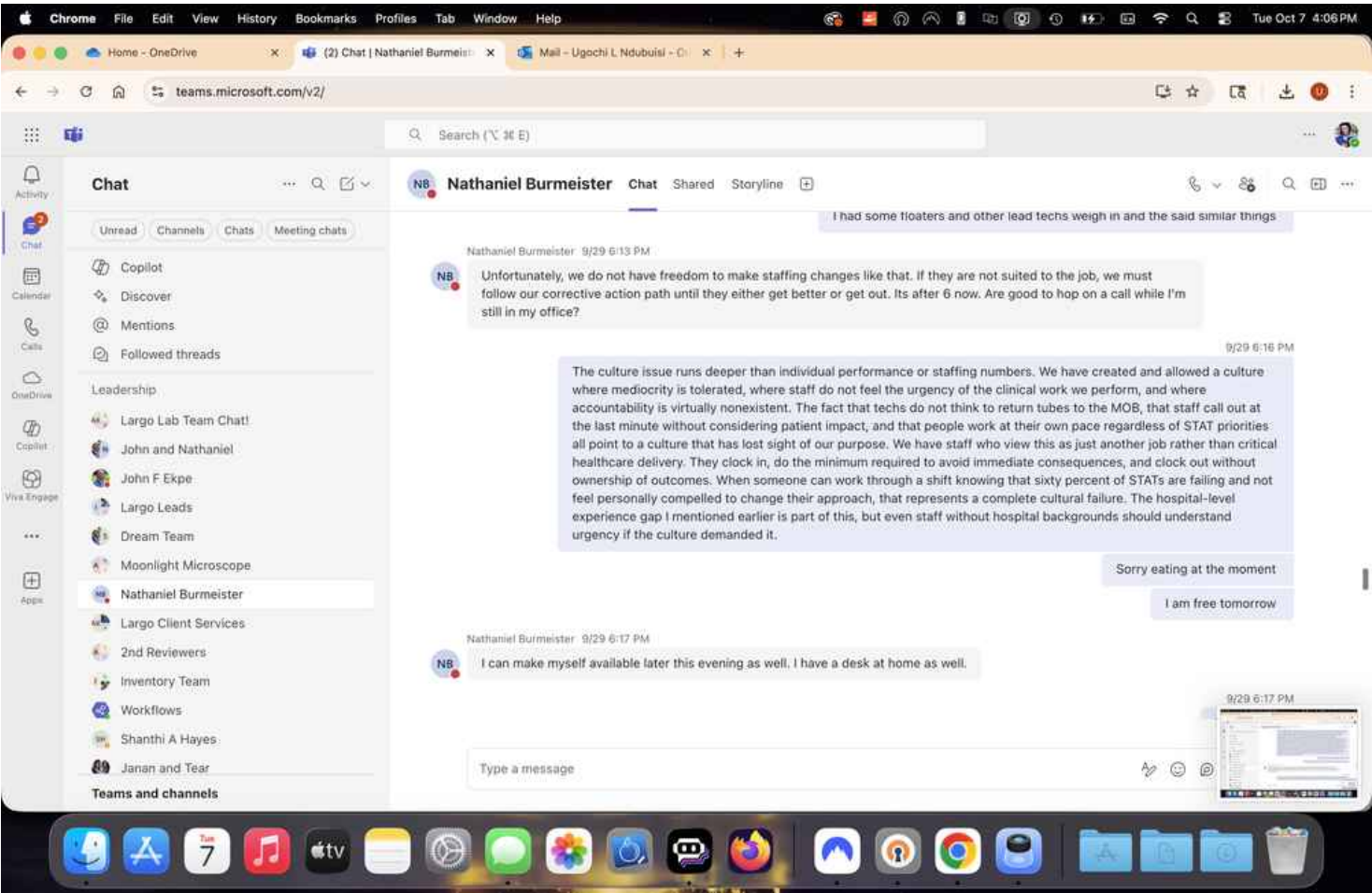
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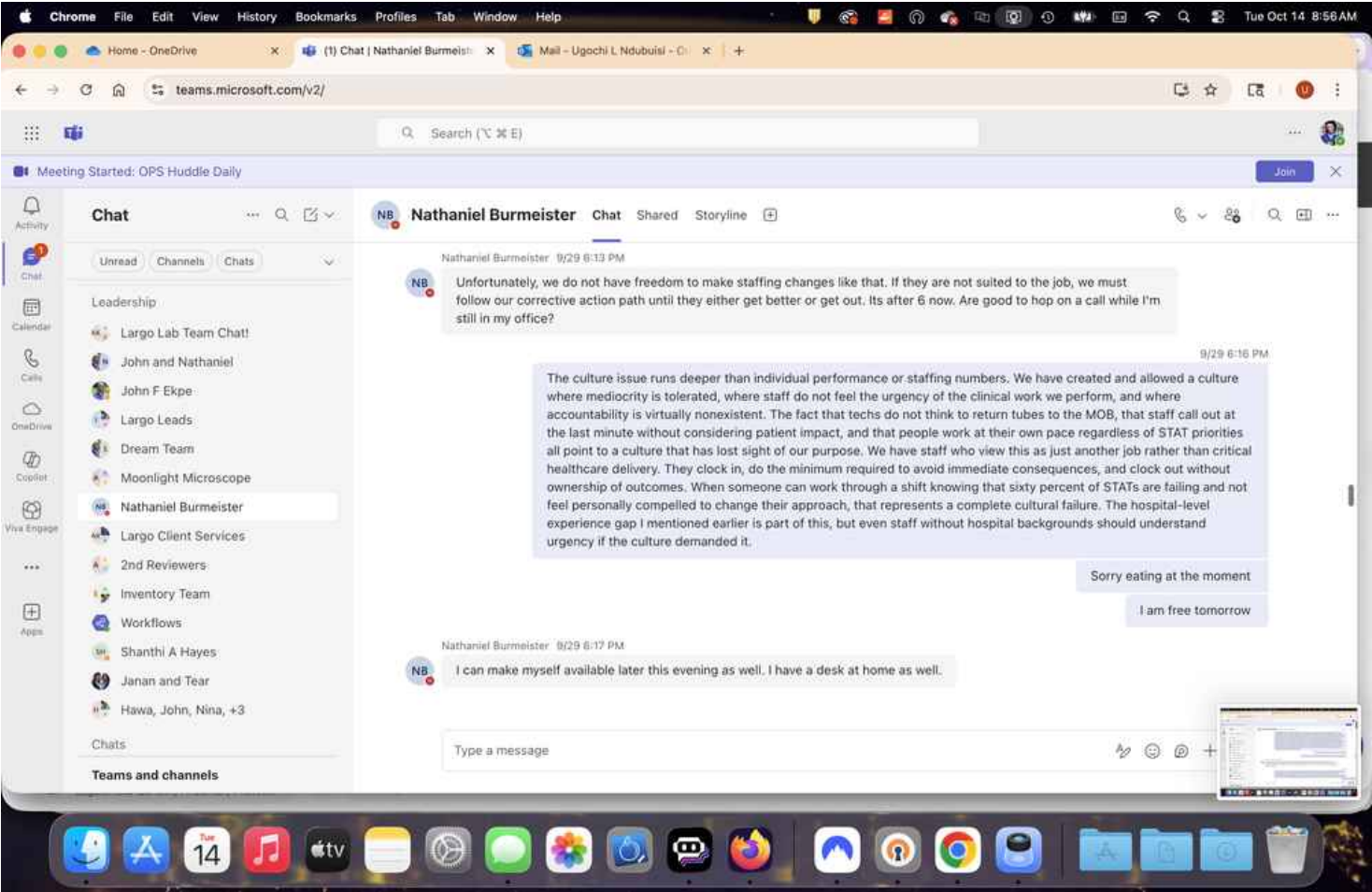
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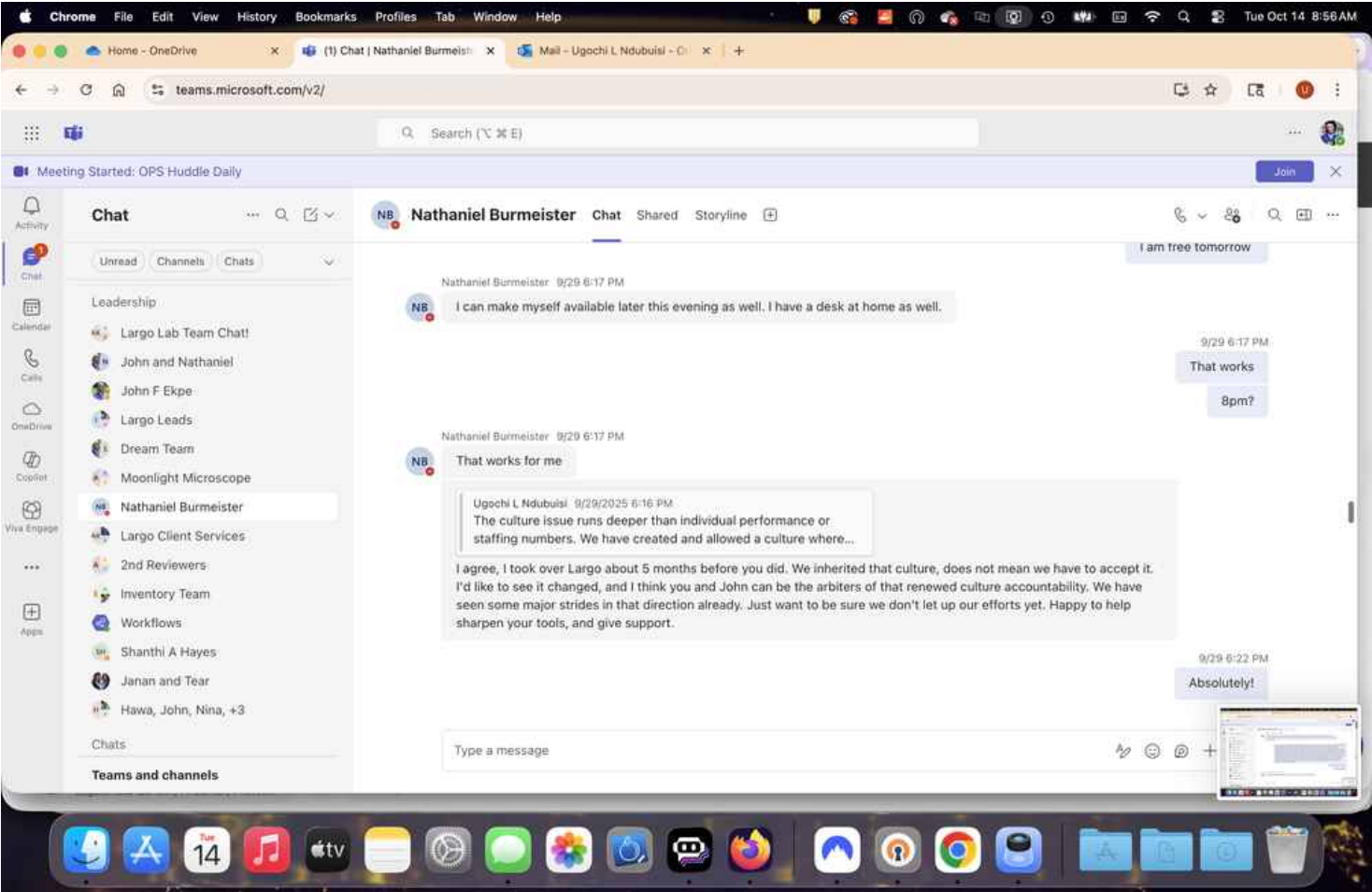


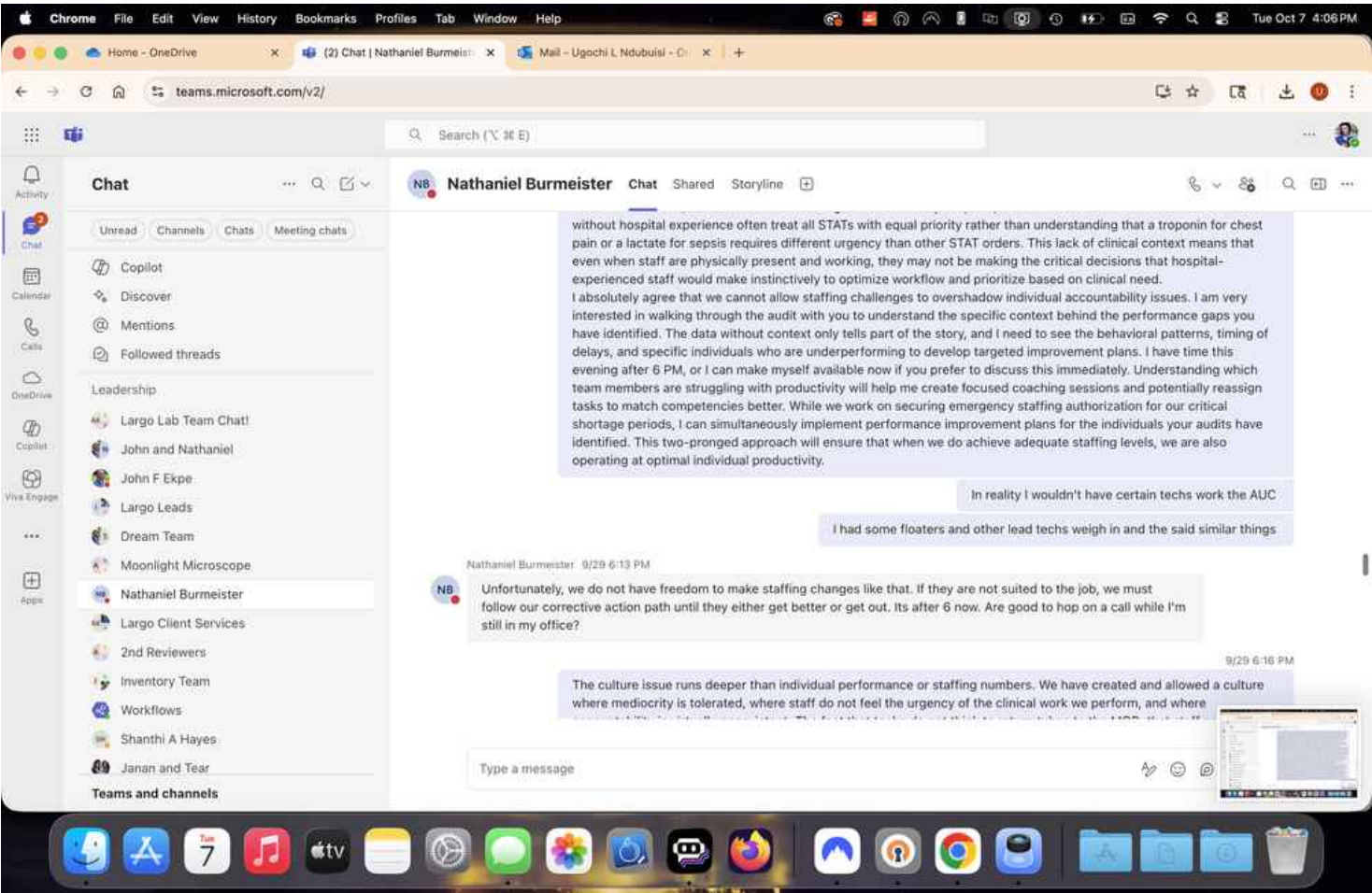


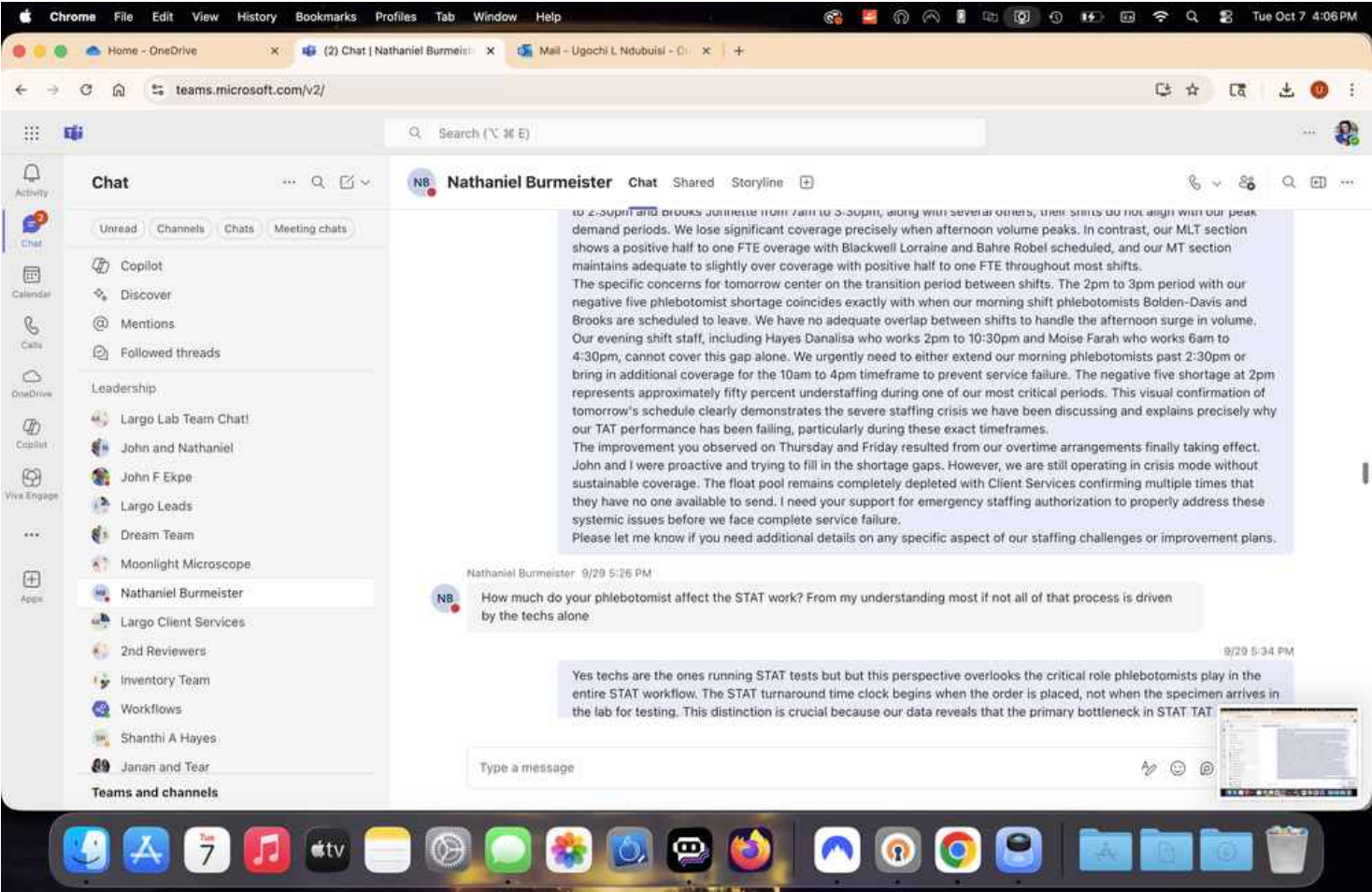


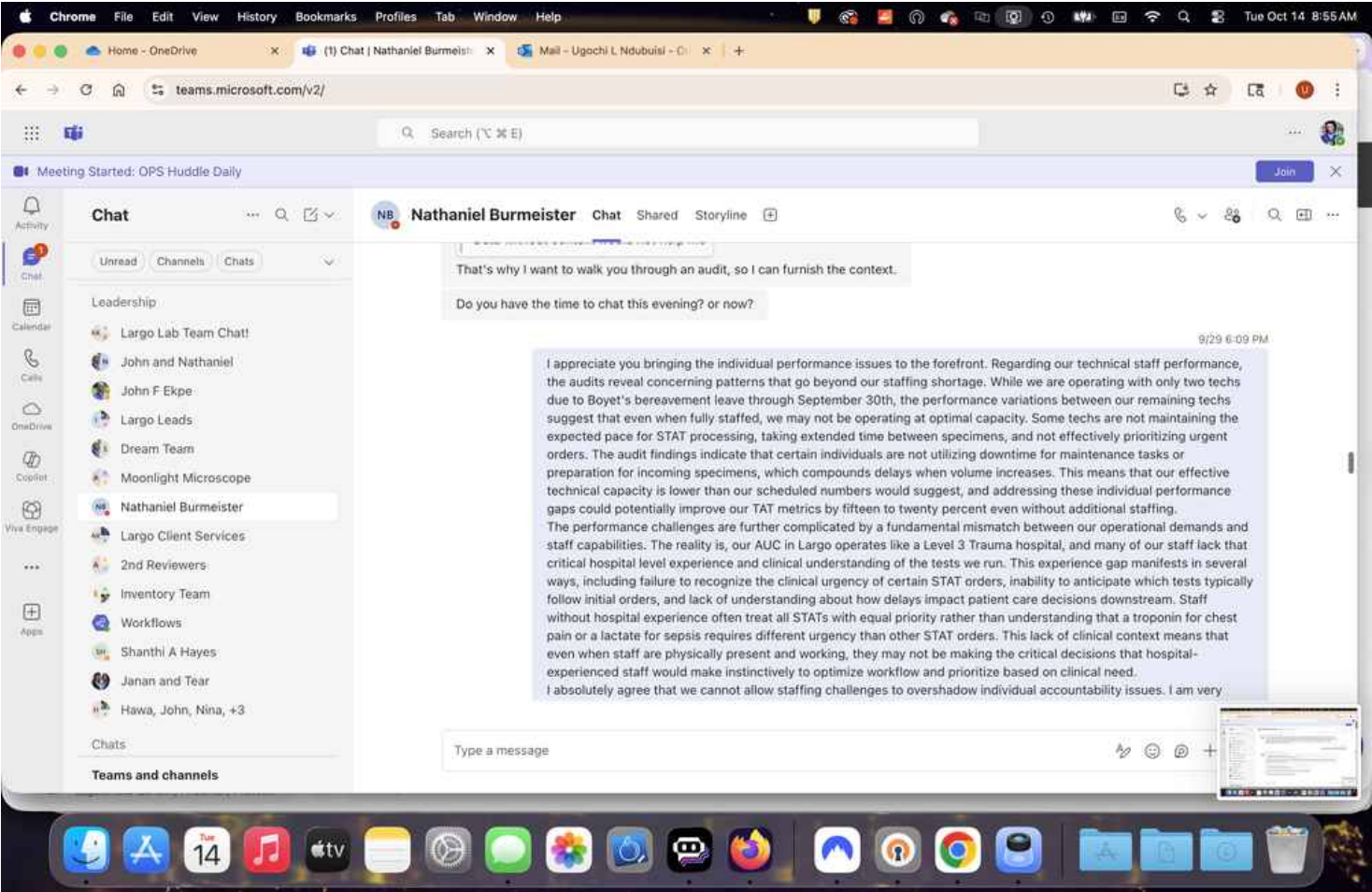
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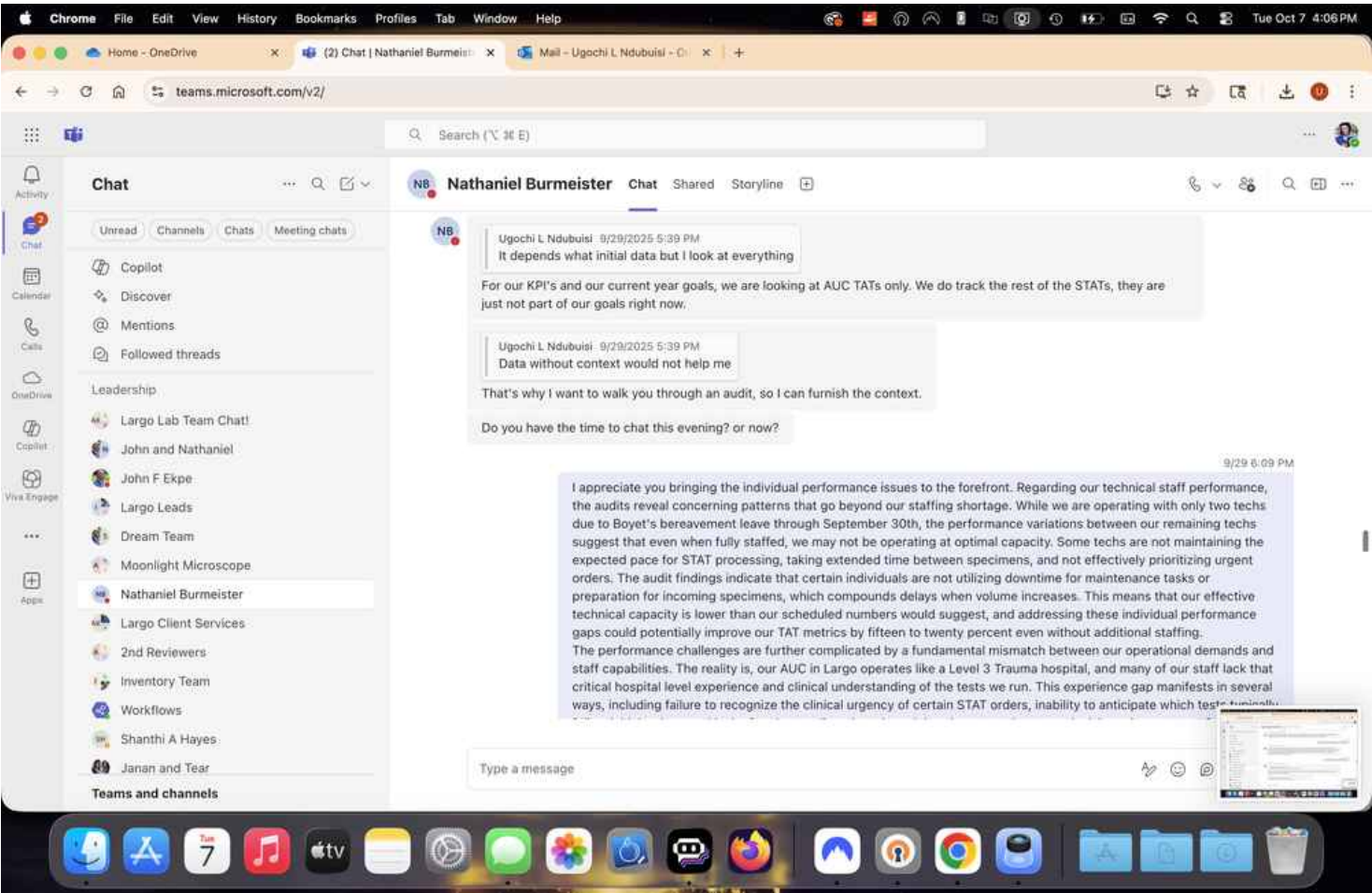


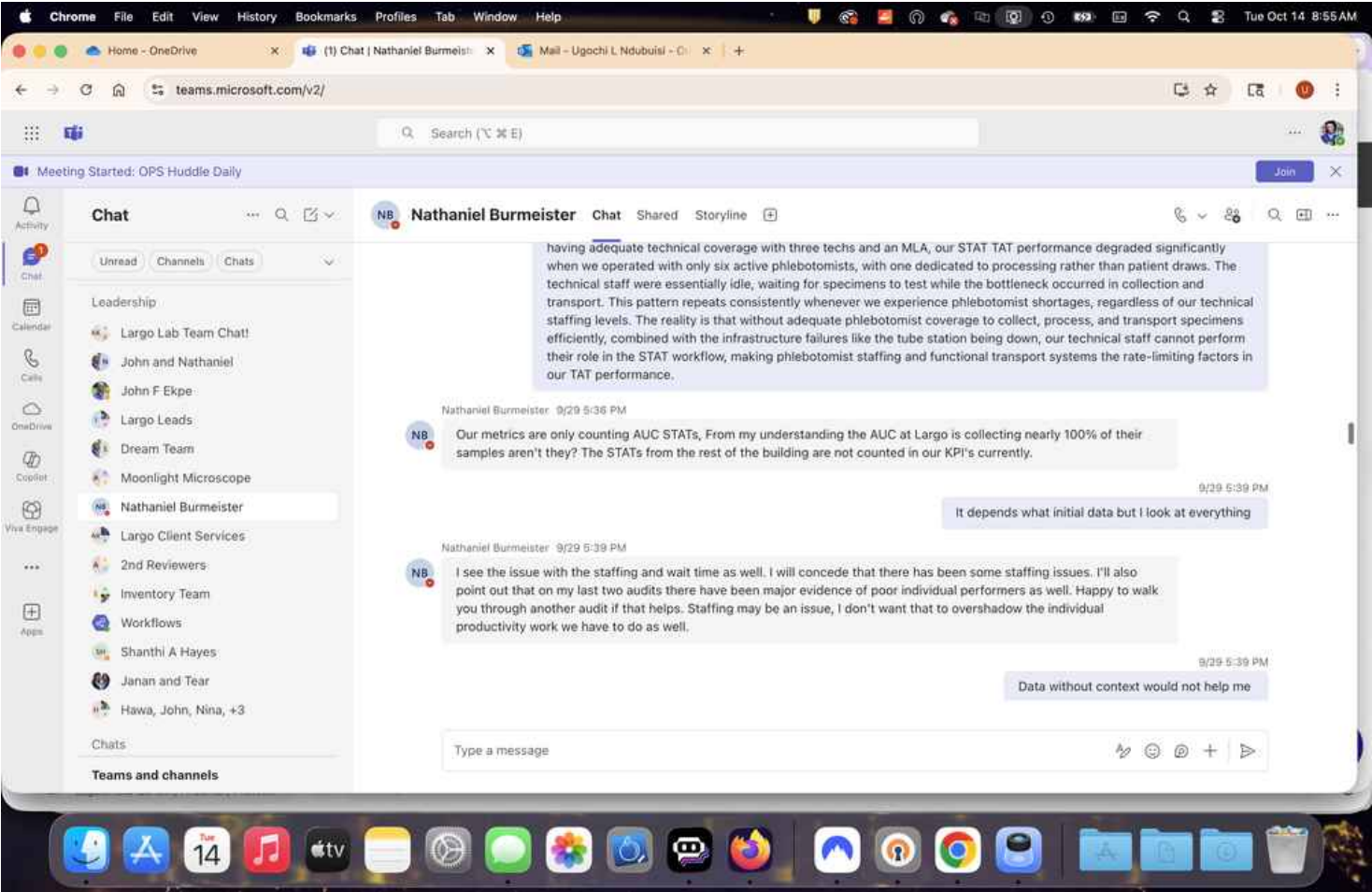


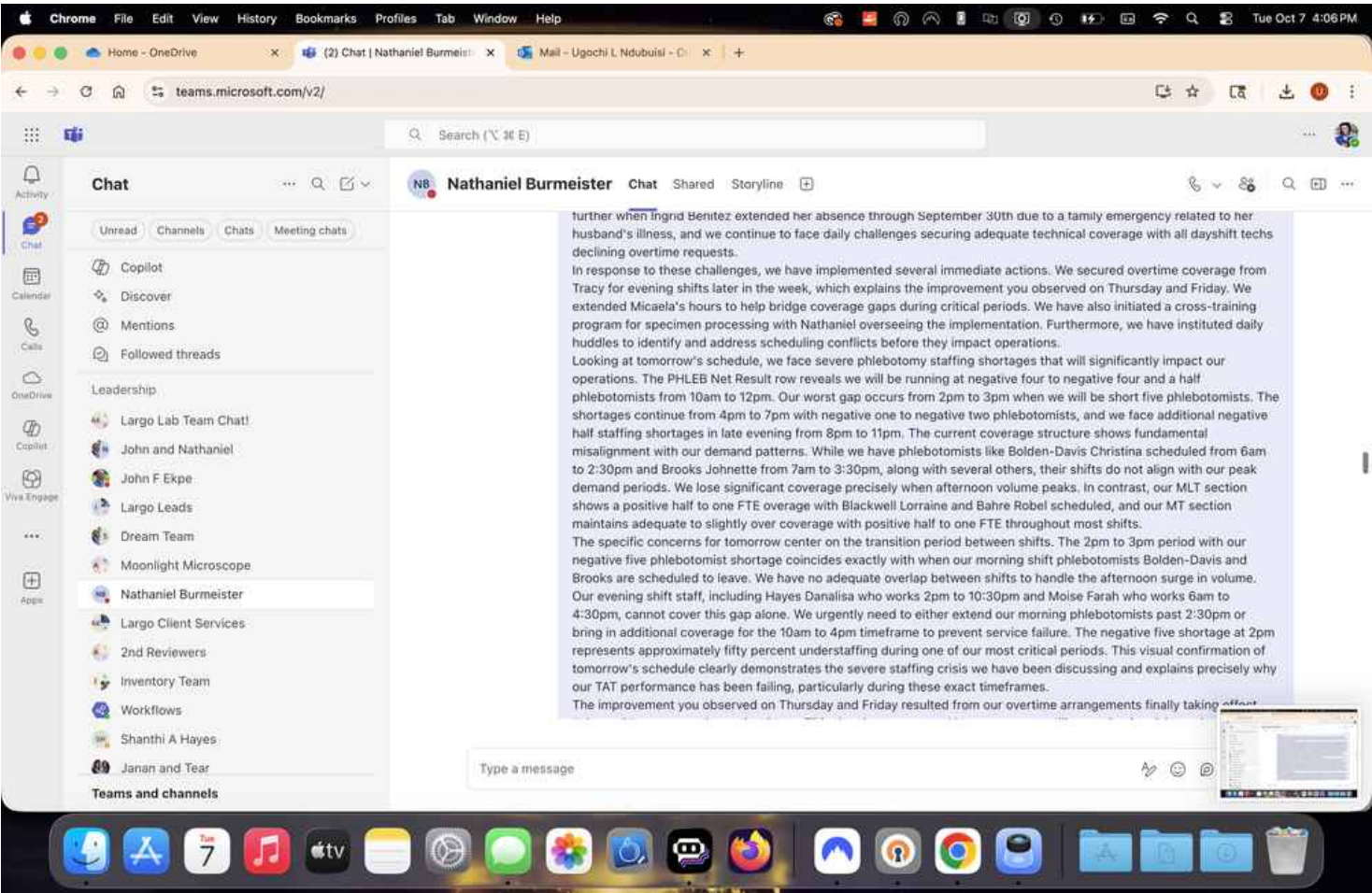


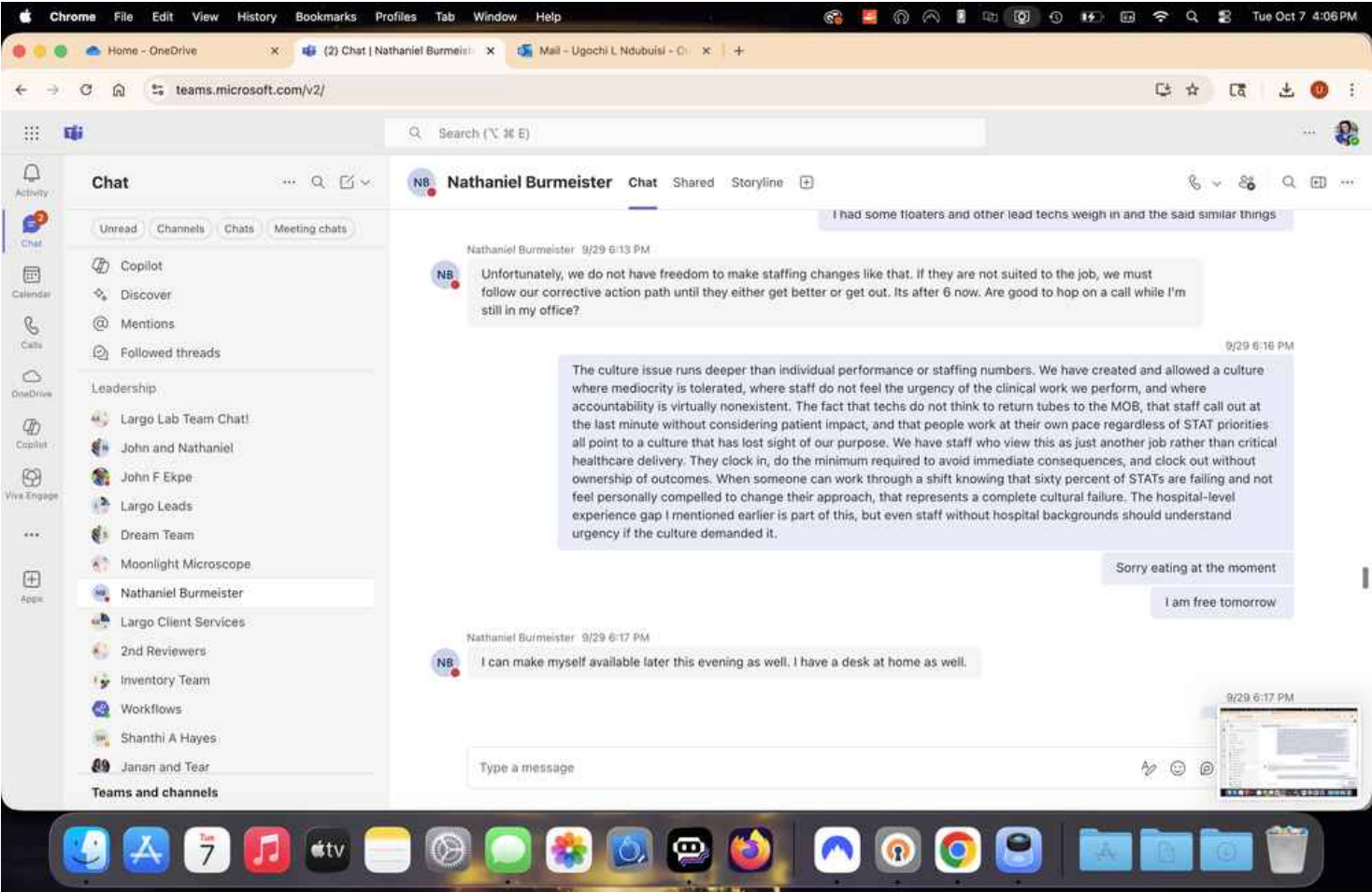




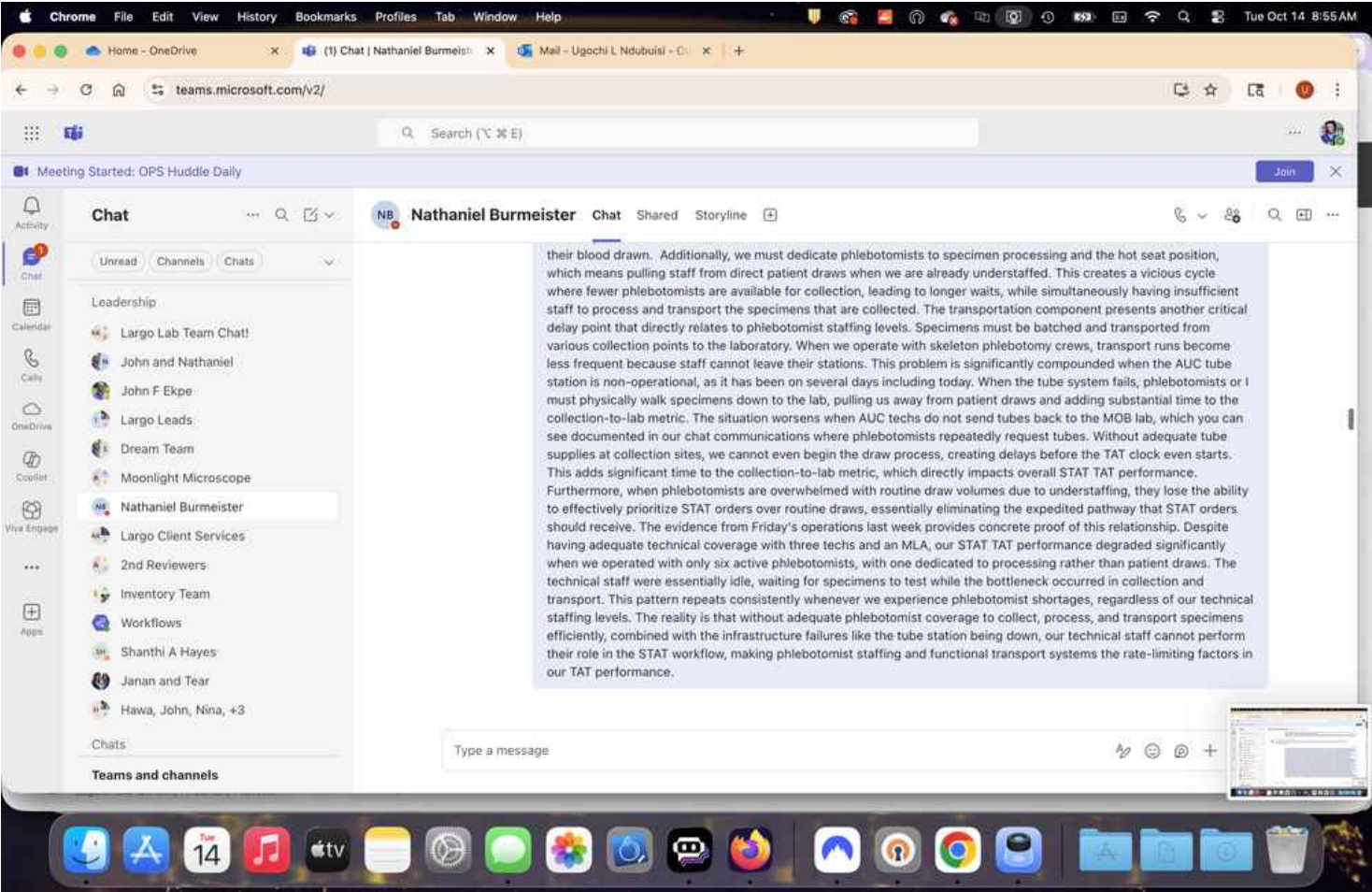




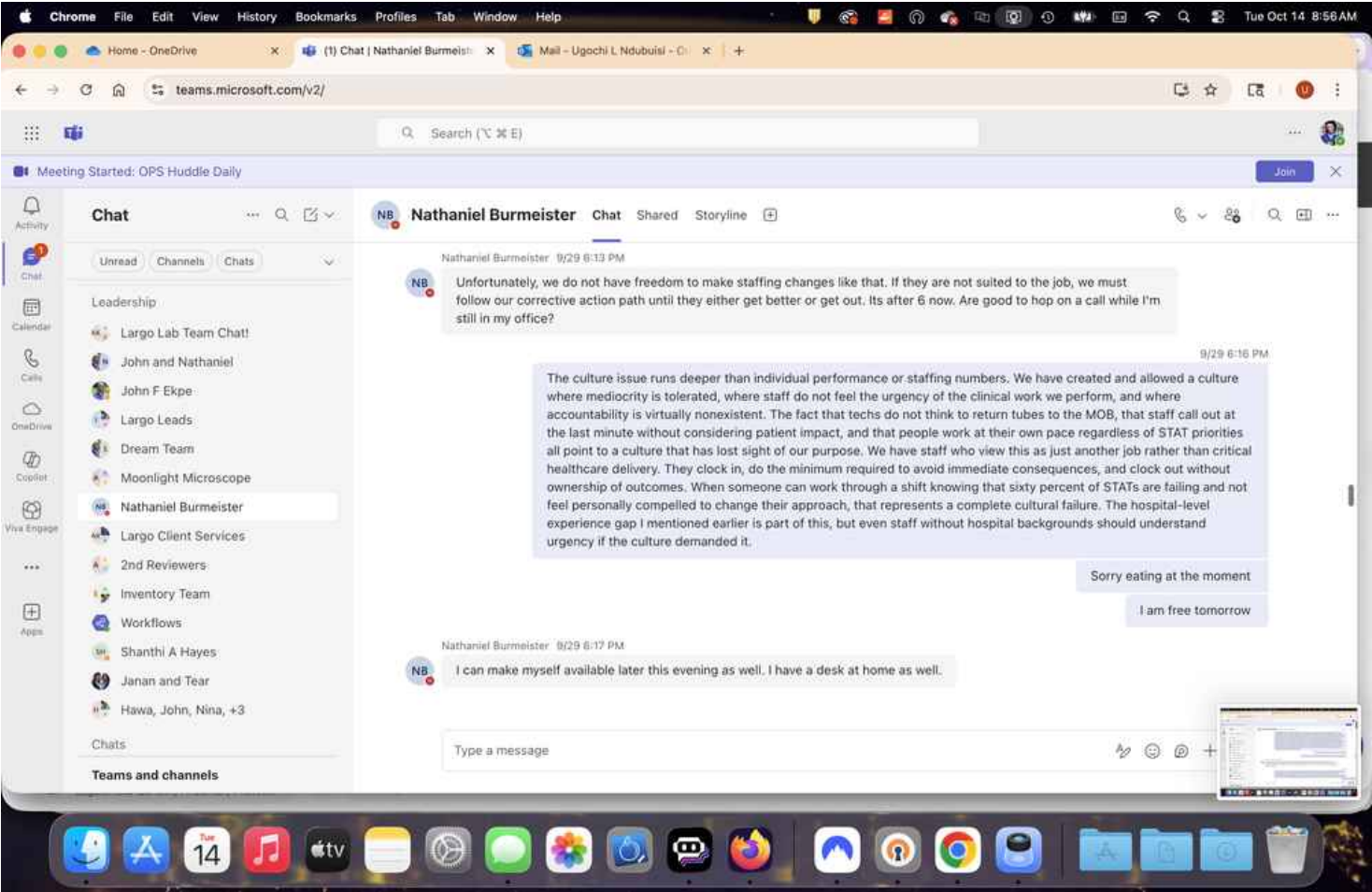


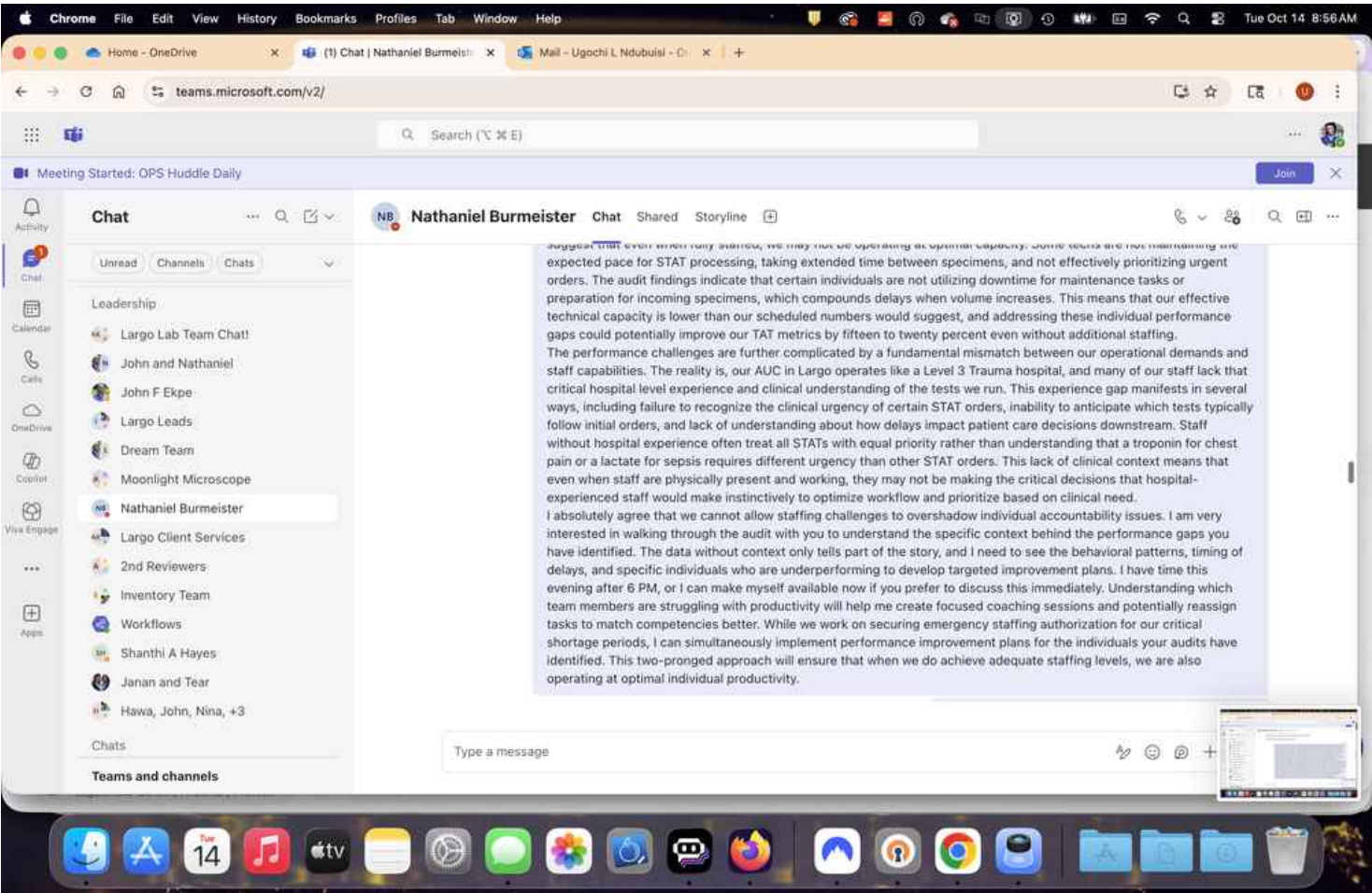


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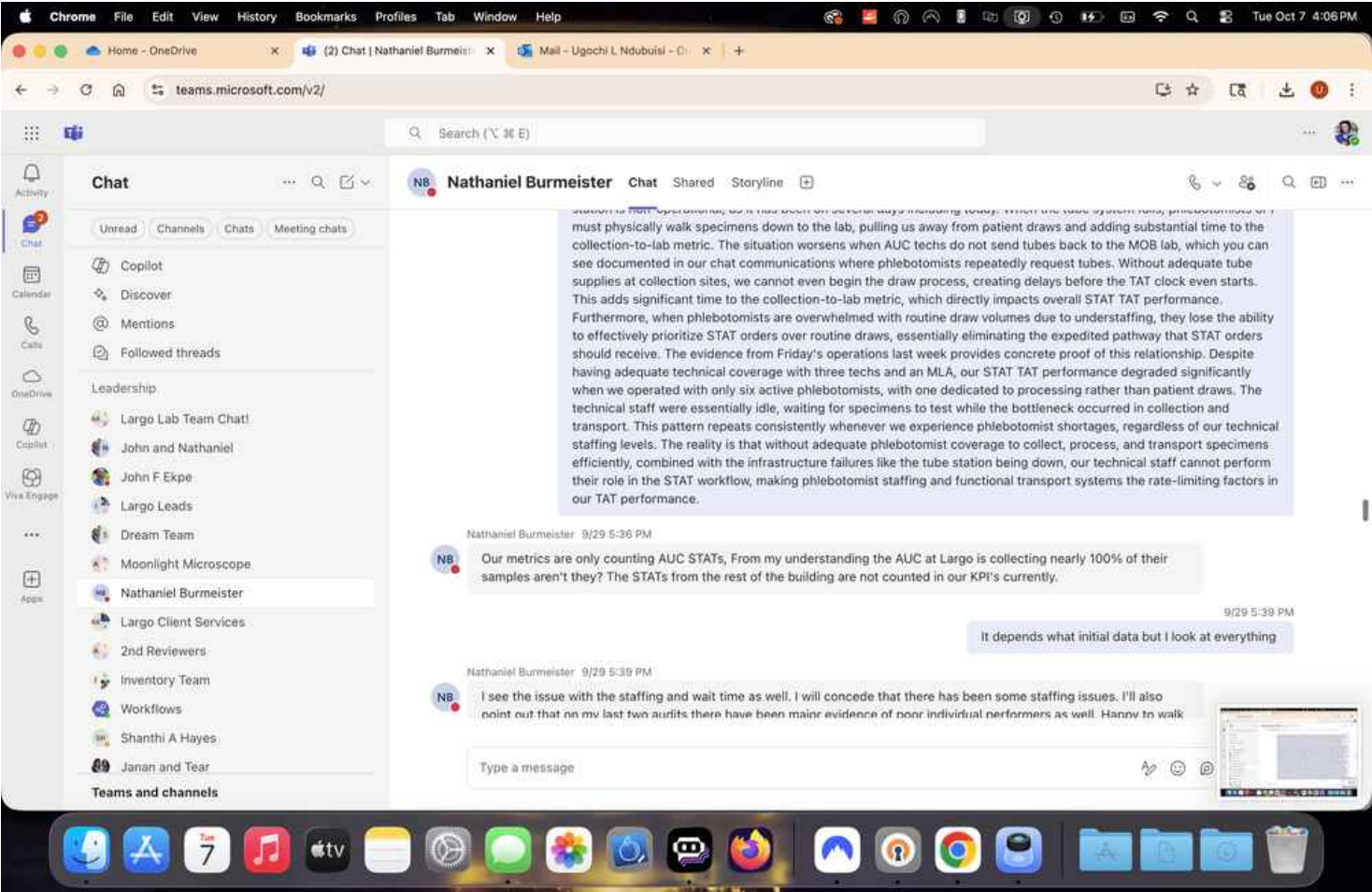


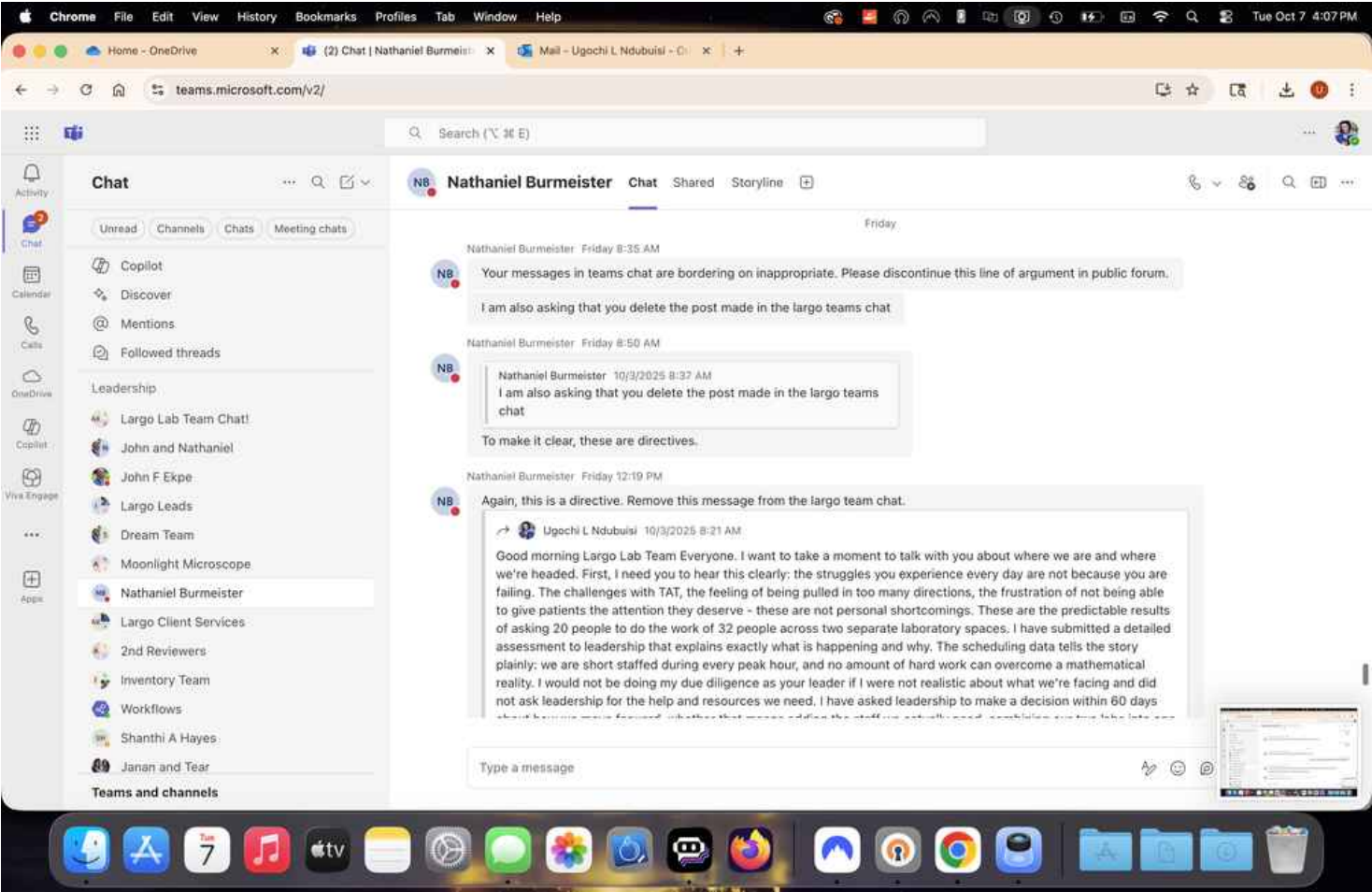
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The screenshot shows a Microsoft Teams chat window on a Mac. The chat is with Nathaniel Burmeister. The message content is highlighted in blue. The Mac dock is visible at the bottom.

Message Content:

...need to identify and address scheduling conflicts before they impact operations. Looking at tomorrow's schedule, we face severe phlebotomy staffing shortages that will significantly impact our operations. The PHLEB Net Result row reveals we will be running at negative four to negative four and a half phlebotomists from 10am to 12pm. Our worst gap occurs from 2pm to 3pm when we will be short five phlebotomists. The shortages continue from 4pm to 7pm with negative one to negative two phlebotomists, and we face additional negative half staffing shortages in late evening from 8pm to 11pm. The current coverage structure shows fundamental misalignment with our demand patterns. While we have phlebotomists like Bolden-Davis Christina scheduled from 6am to 2:30pm and Brooks Johnette from 7am to 3:30pm, along with several others, their shifts do not align with our peak demand periods. We lose significant coverage precisely when afternoon volume peaks. In contrast, our MLT section shows a positive half to one FTE coverage with Blackwell Lorraine and Bahre Robel scheduled, and our MT section maintains adequate to slightly over coverage with positive half to one FTE throughout most shifts. The specific concerns for tomorrow center on the transition period between shifts. The 2pm to 3pm period with our negative five phlebotomist shortage coincides exactly with when our morning shift phlebotomists Bolden-Davis and Brooks are scheduled to leave. We have no adequate overlap between shifts to handle the afternoon surge in volume. Our evening shift staff, including Hayes Danalisa who works 2pm to 10:30pm and Moise Farah who works 6am to 4:30pm, cannot cover this gap alone. We urgently need to either extend our morning phlebotomists past 2:30pm or bring in additional coverage for the 10am to 4pm timeframe to prevent service failure. The negative five shortage at 2pm represents approximately fifty percent understaffing during one of our most critical periods. This visual confirmation of tomorrow's schedule clearly demonstrates the severe staffing crisis we have been discussing and explains precisely why our TAT performance has been failing, particularly during these exact timeframes. The improvement you observed on Thursday and Friday resulted from our overtime arrangements finally taking effect. John and I were proactive and trying to fill in the shortage gaps. However, we are still operating in crisis mode without sustainable coverage. The float pool remains completely depleted with Client Services confirming multiple times that they have no one available to send. I need your support for emergency staffing authorization to properly address these systemic issues before we face complete service failure. Please let me know if you need additional details on any specific aspect of our staffing challenges or improvement plans.





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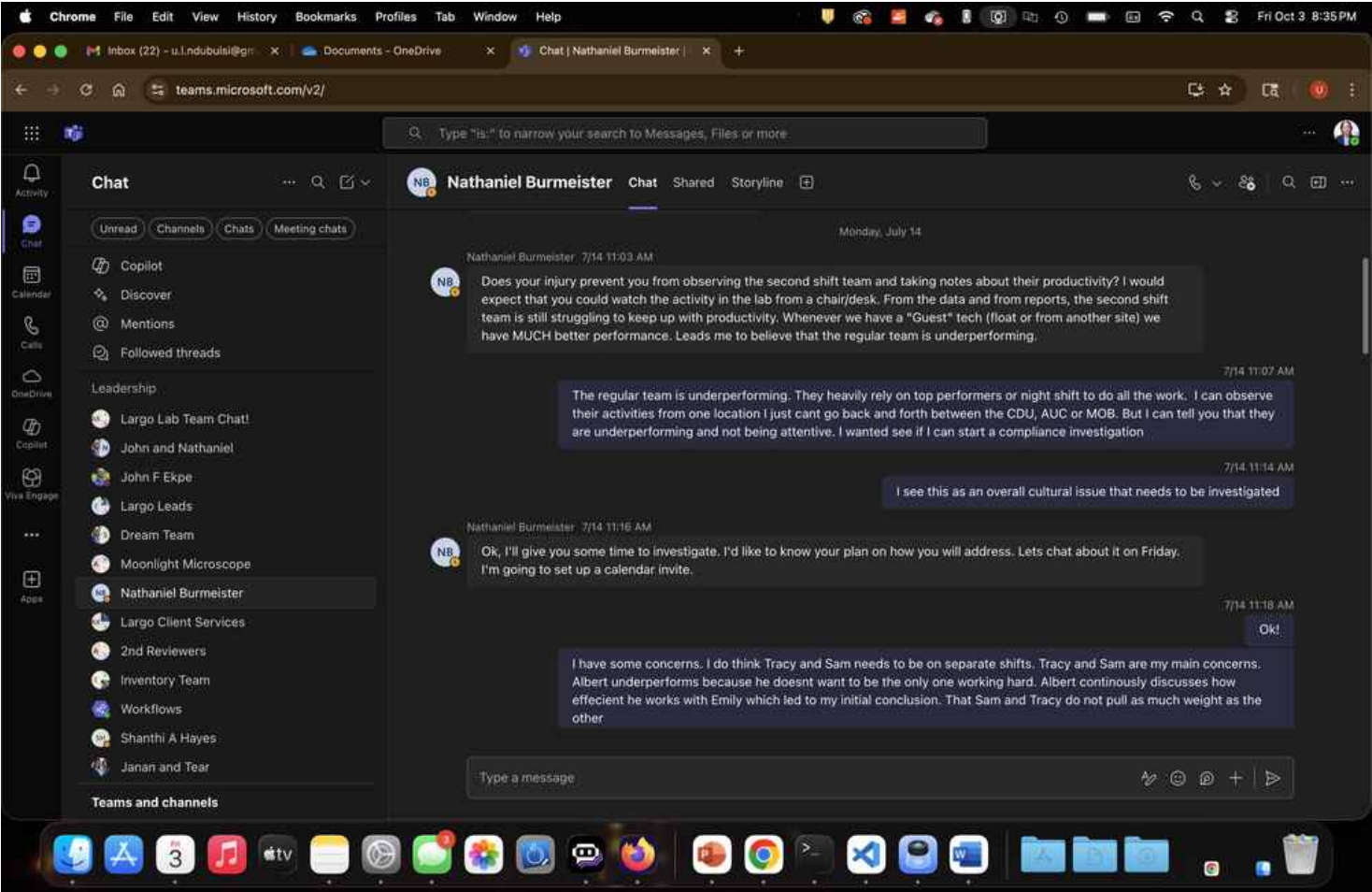
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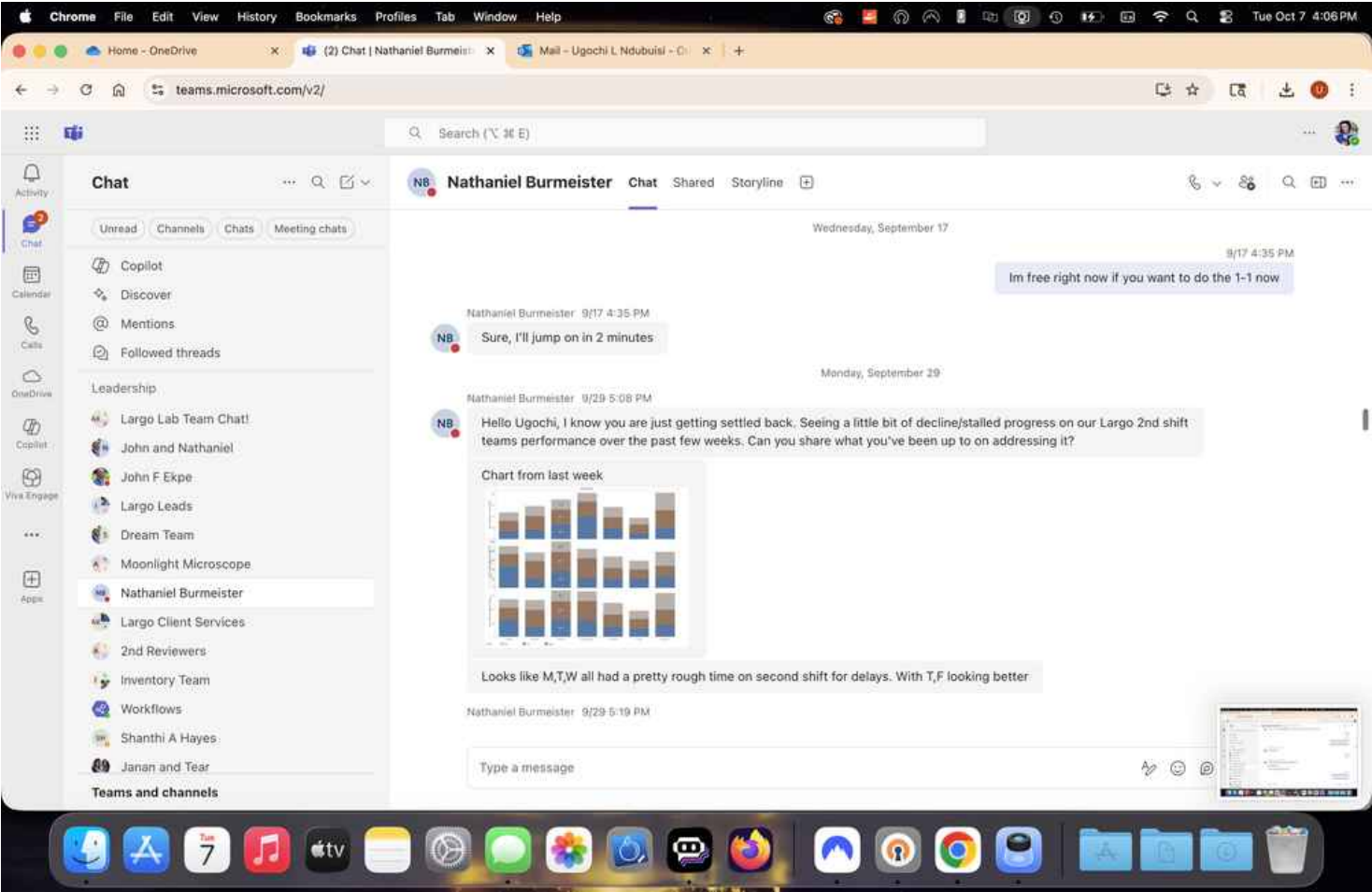
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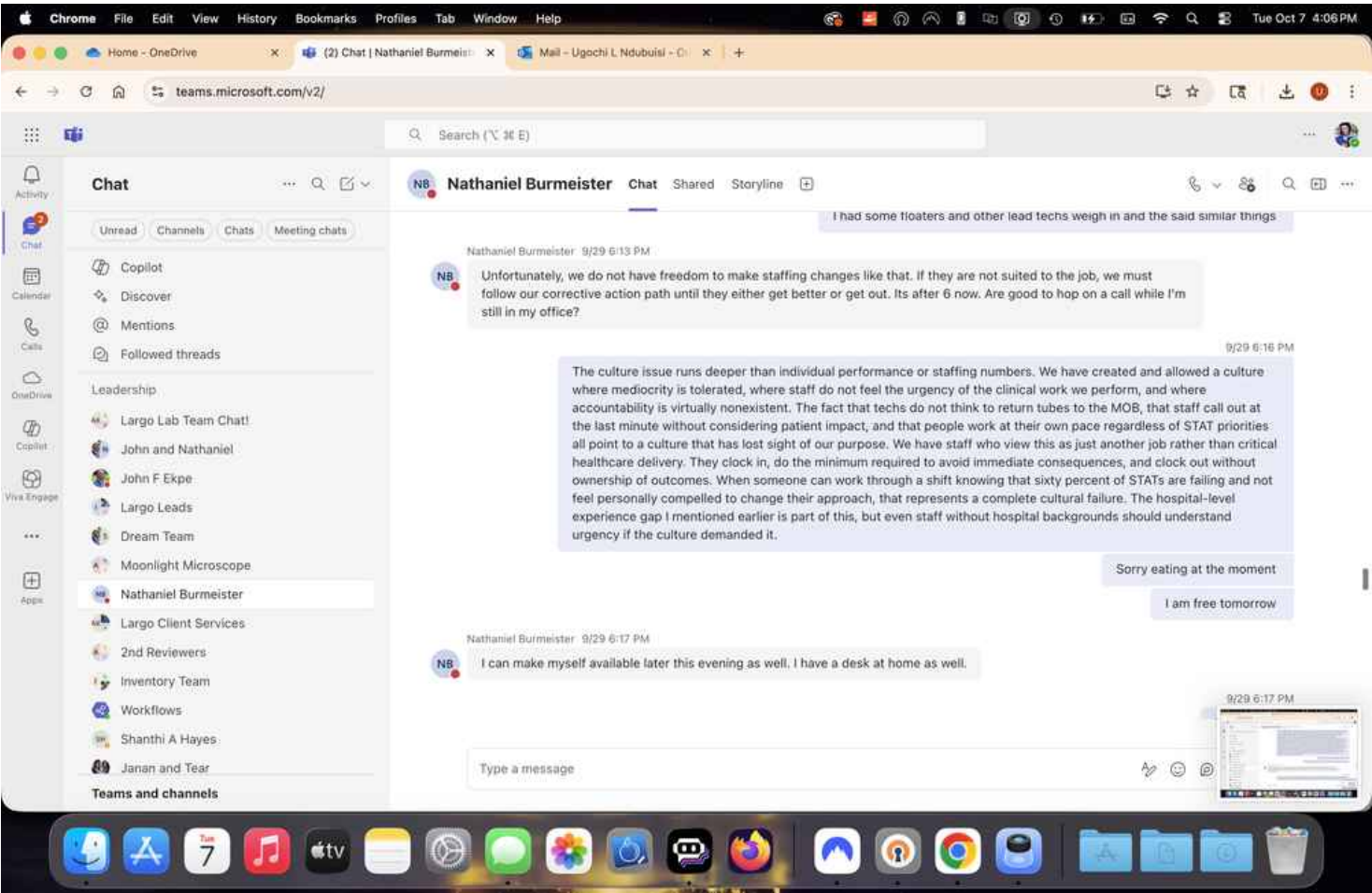
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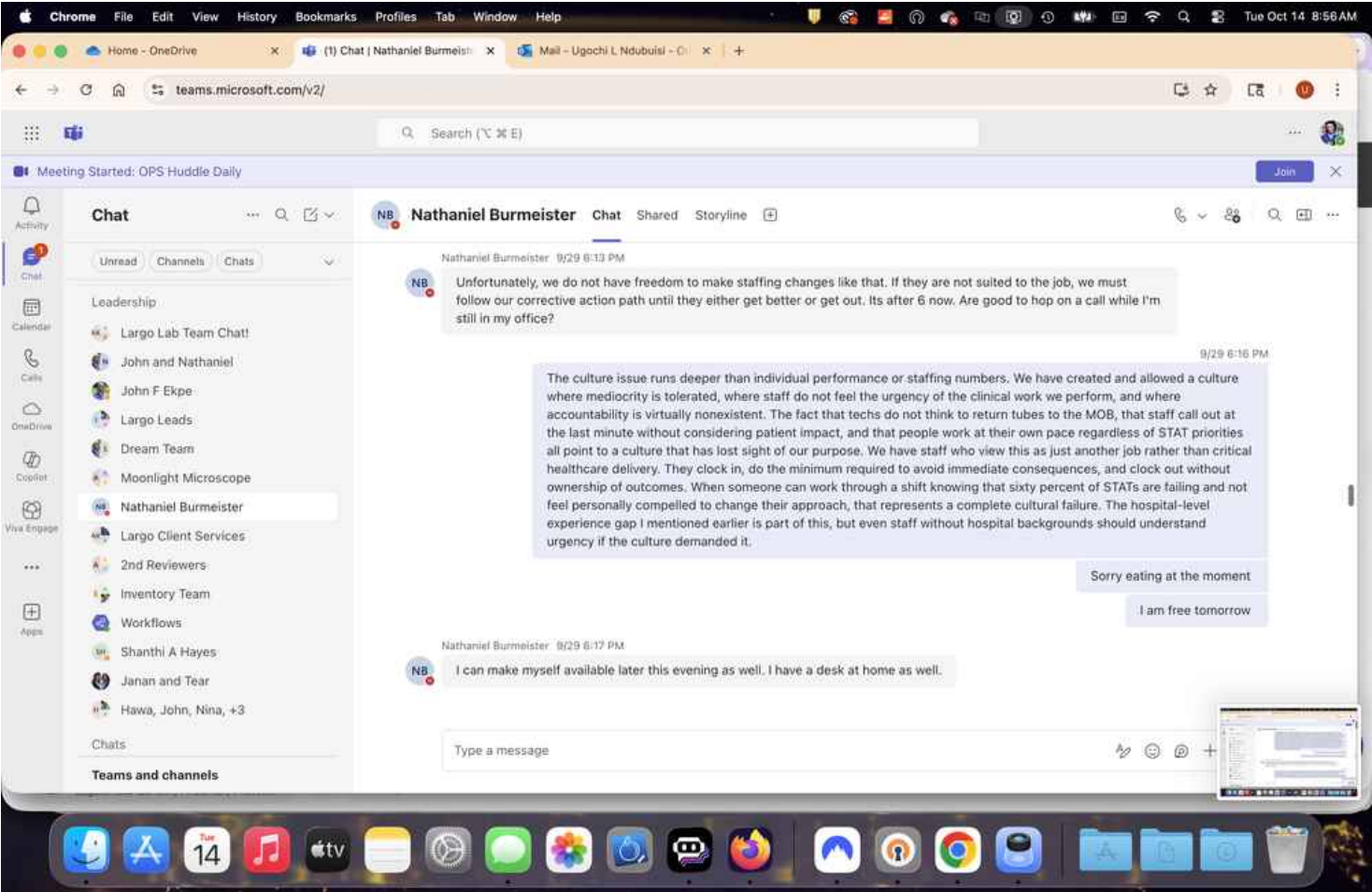
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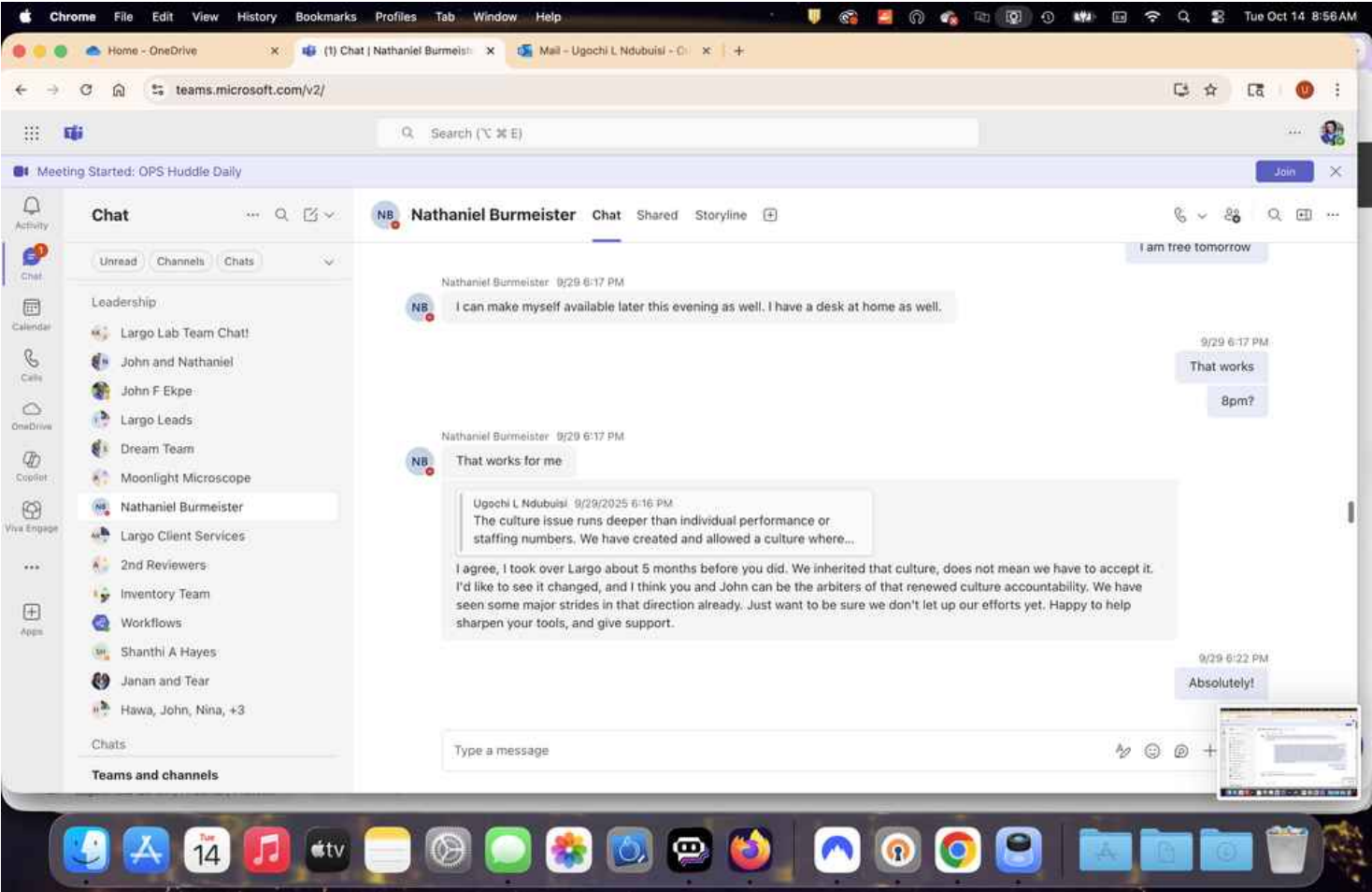


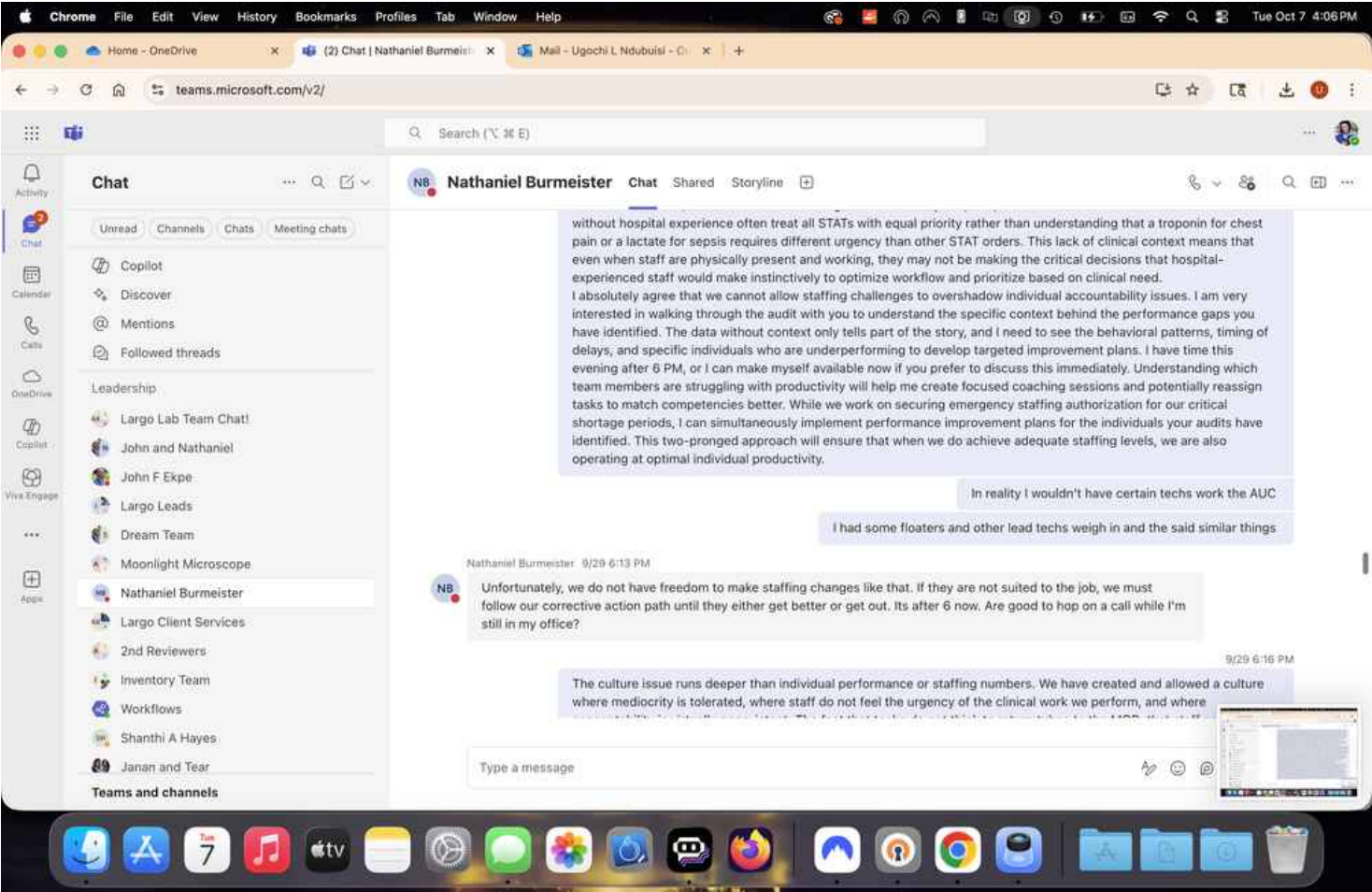


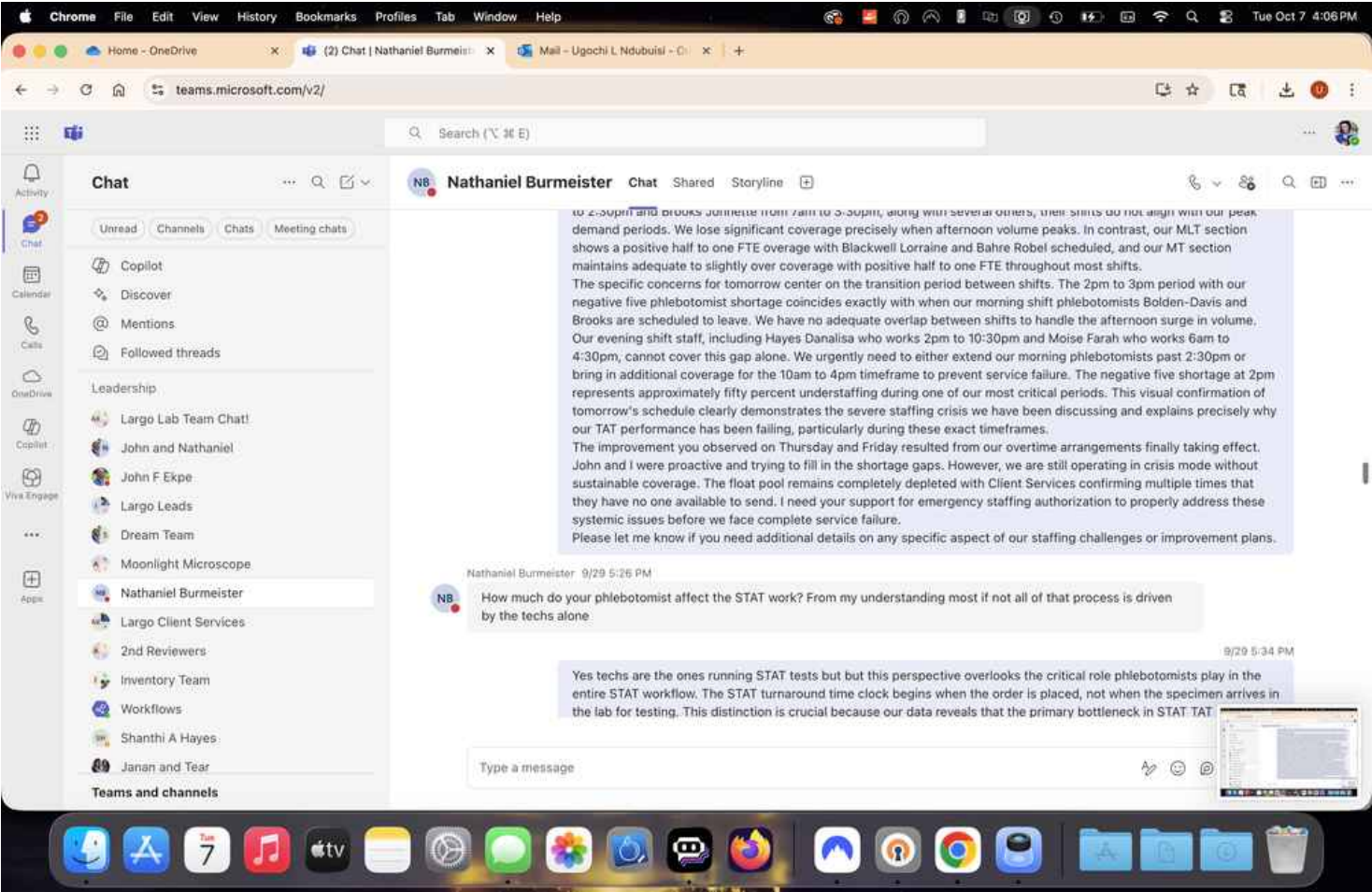


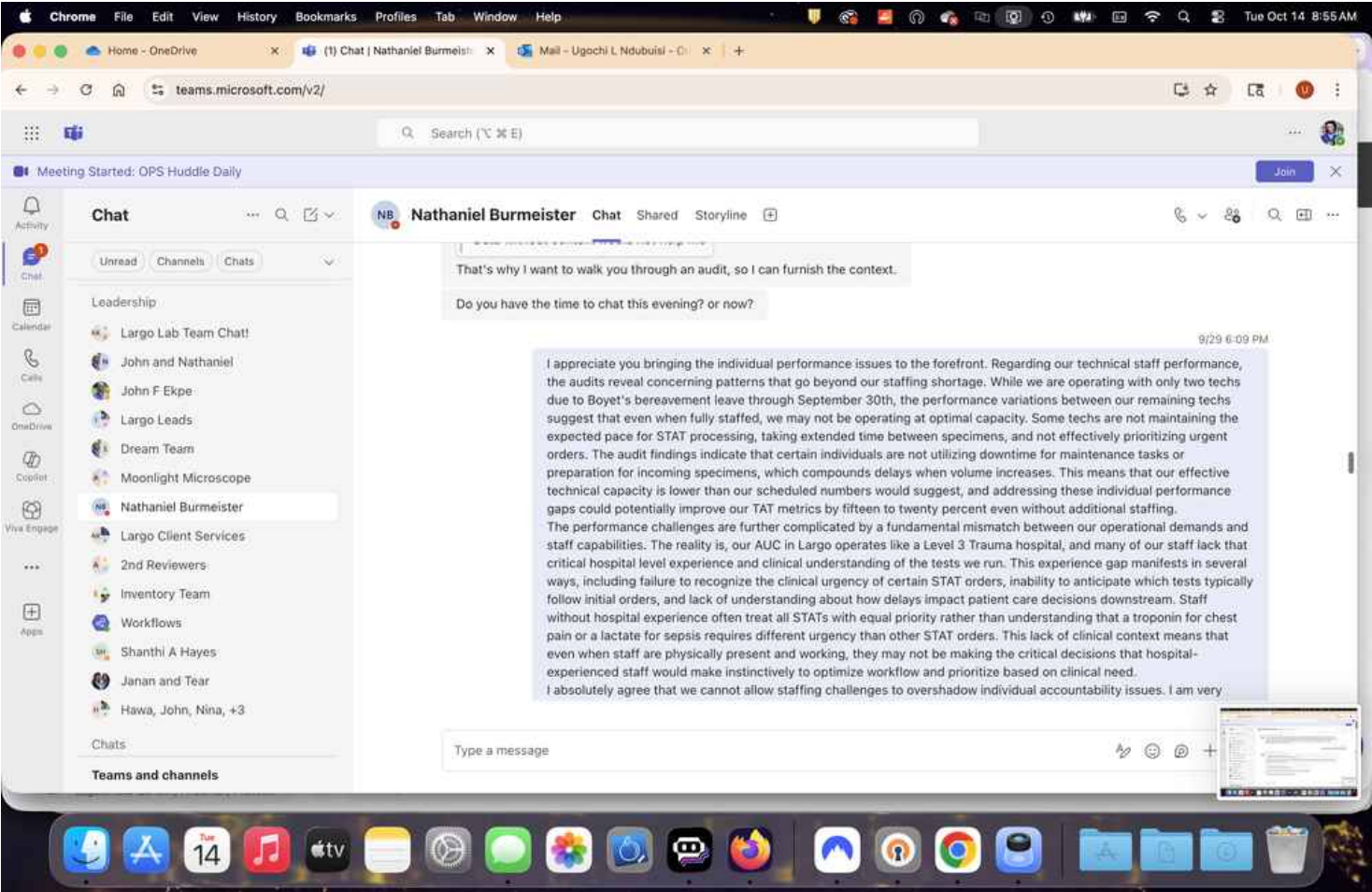
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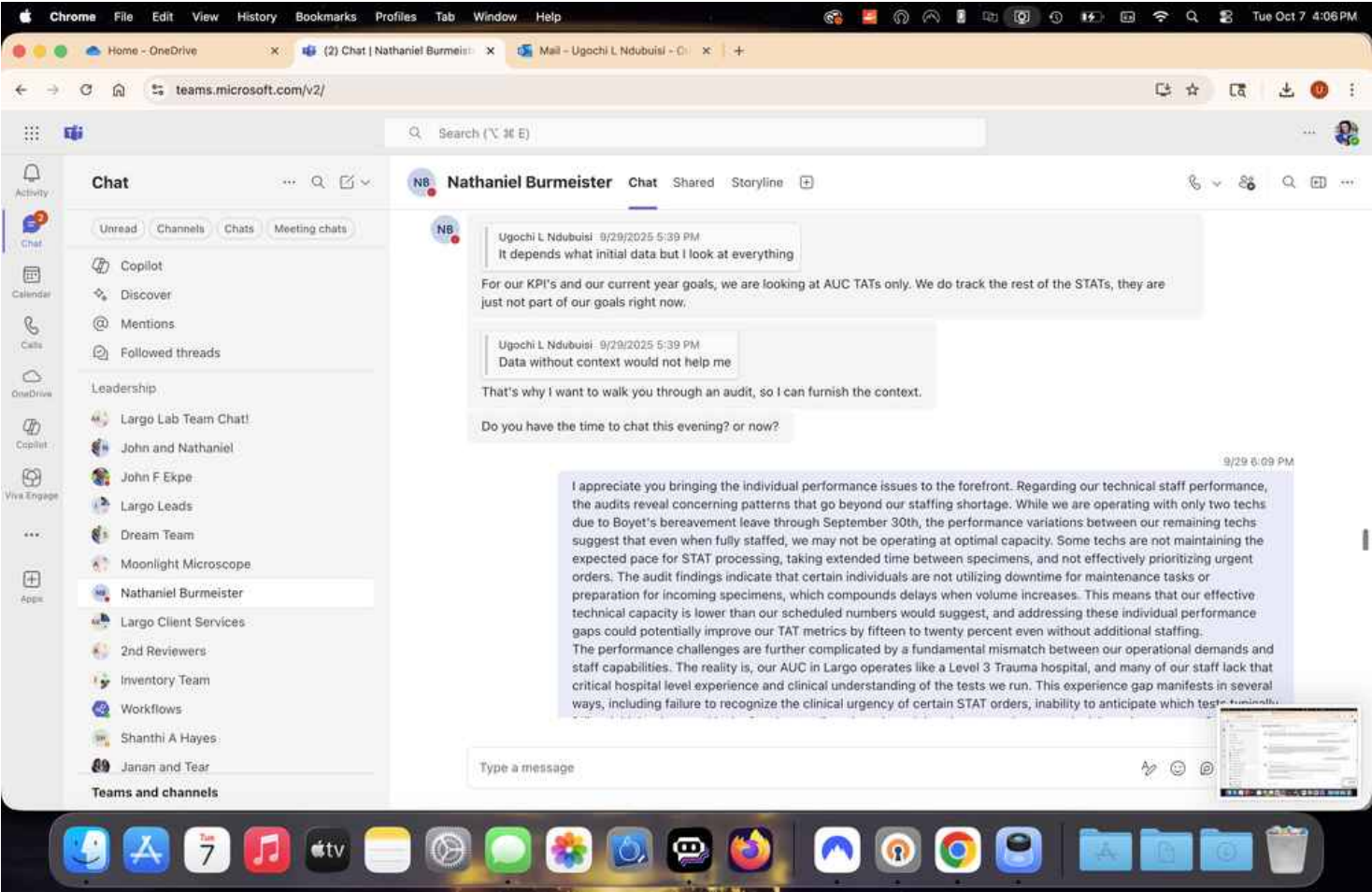


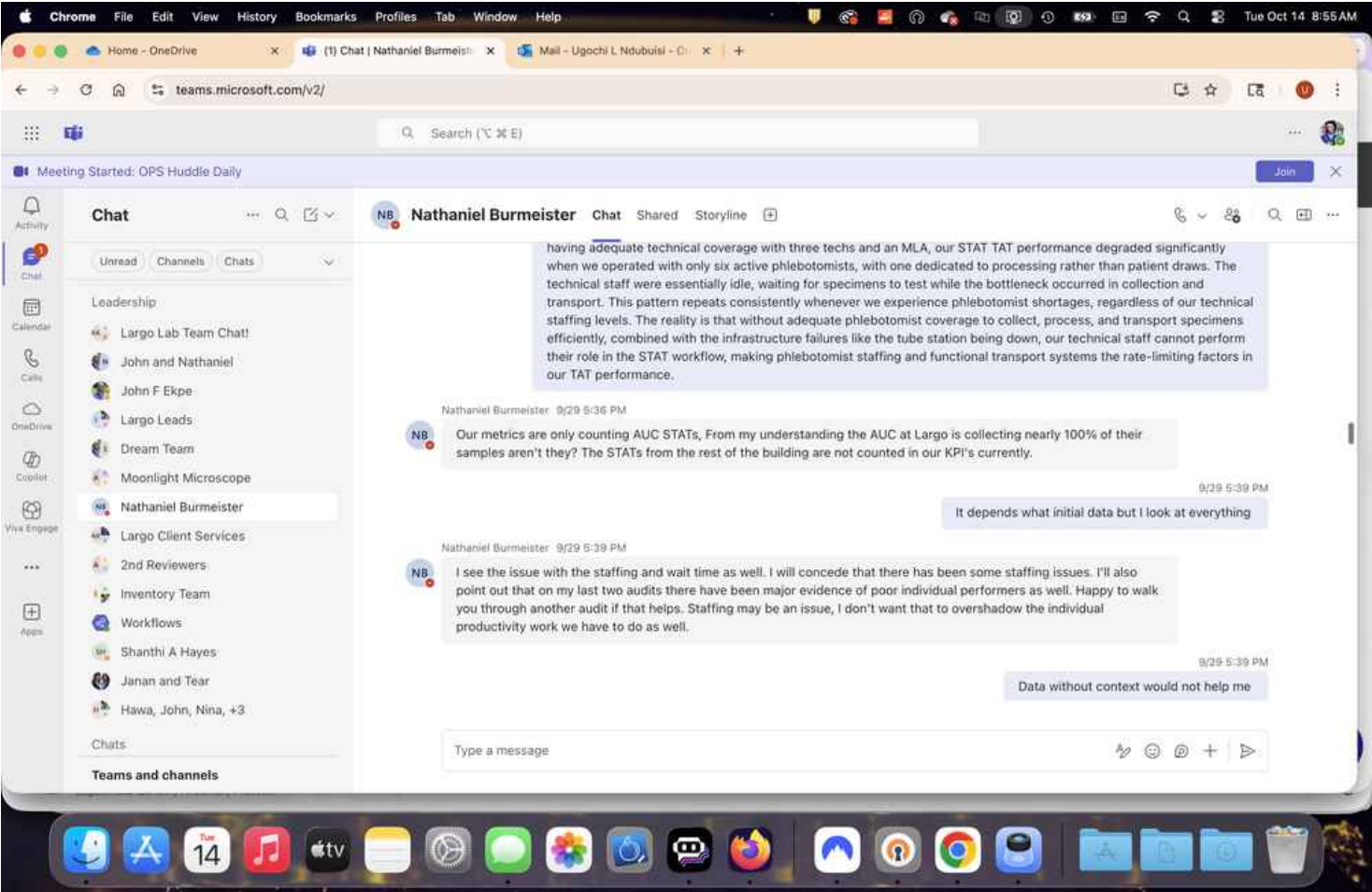


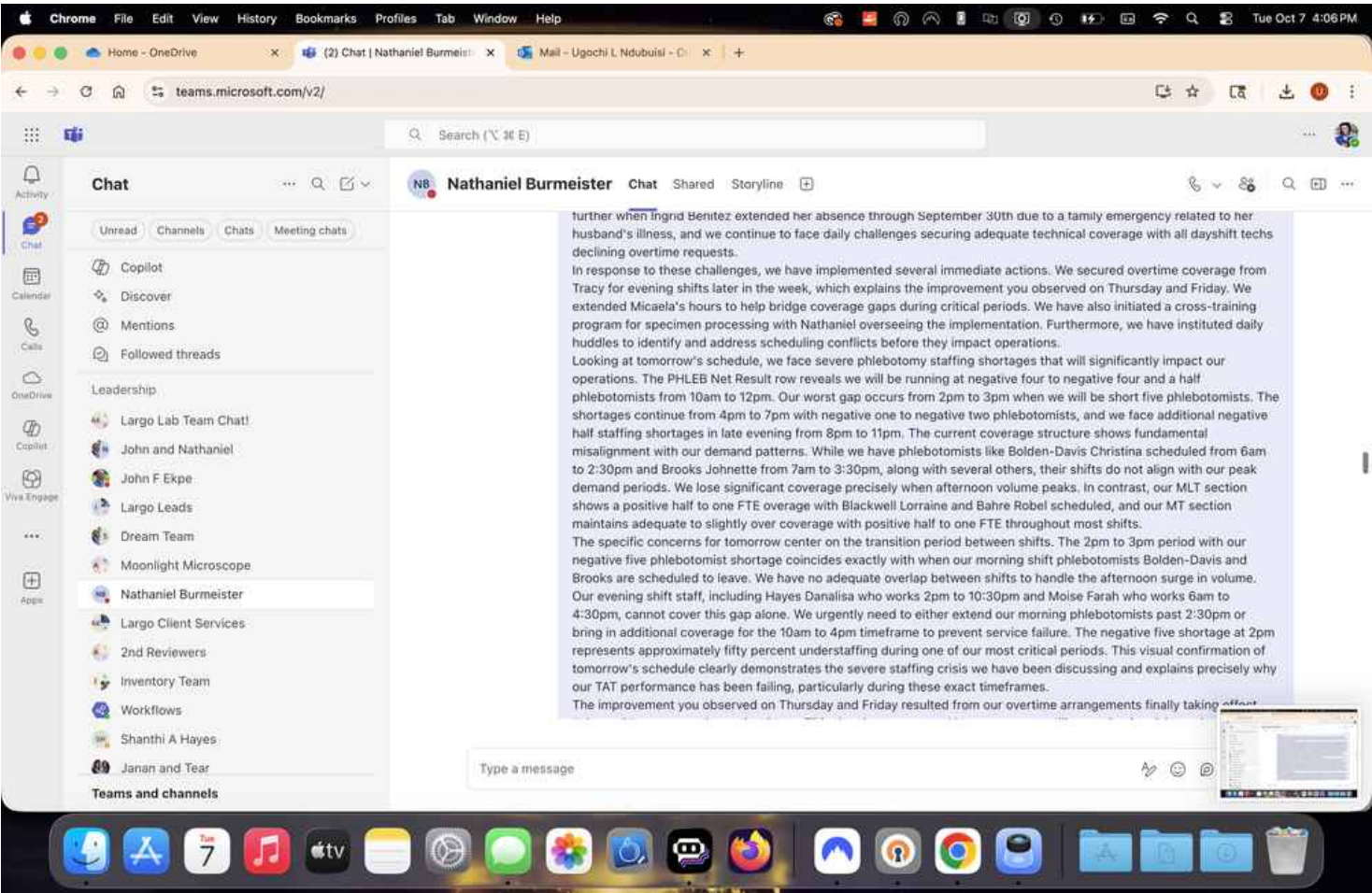


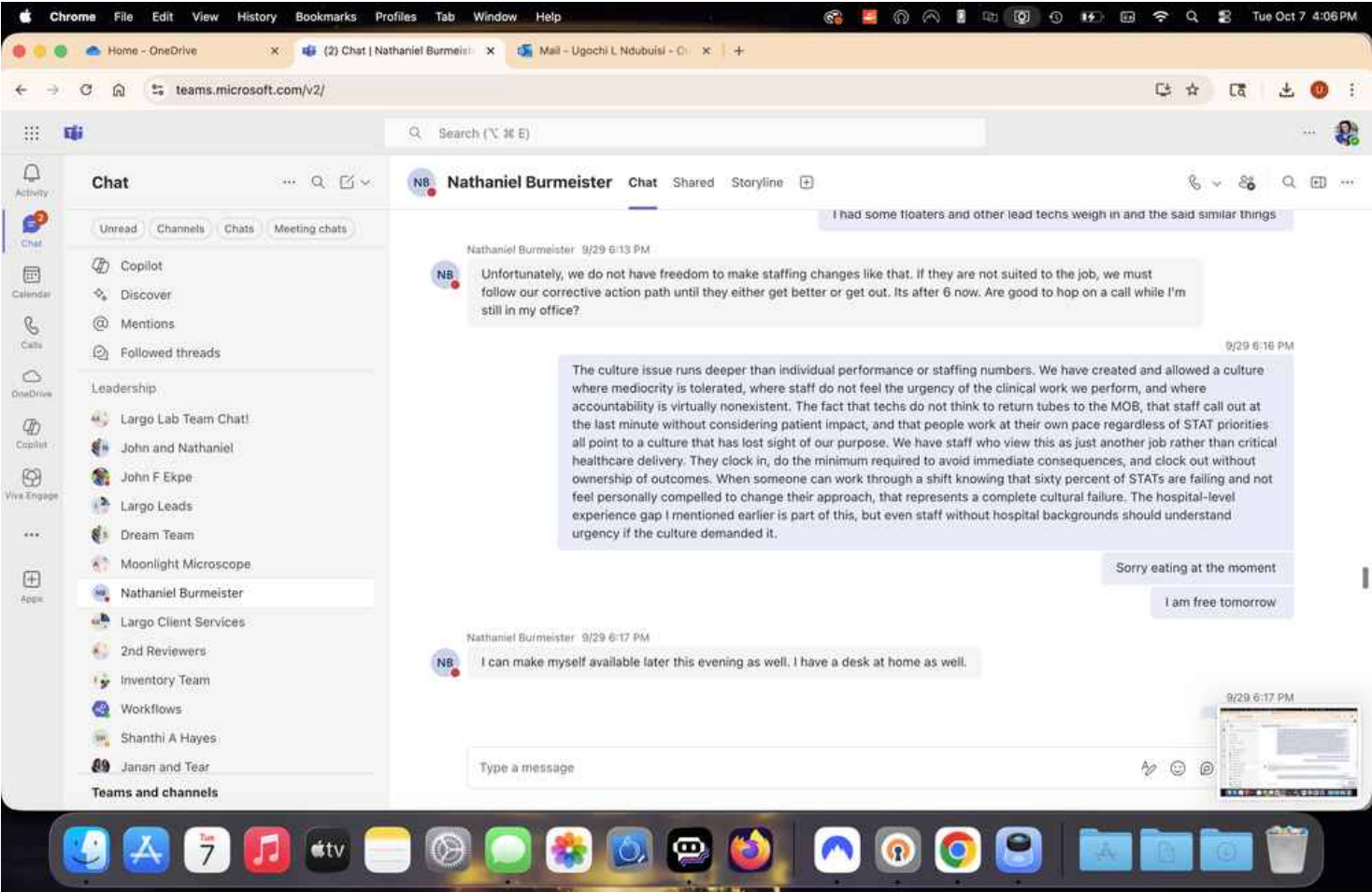




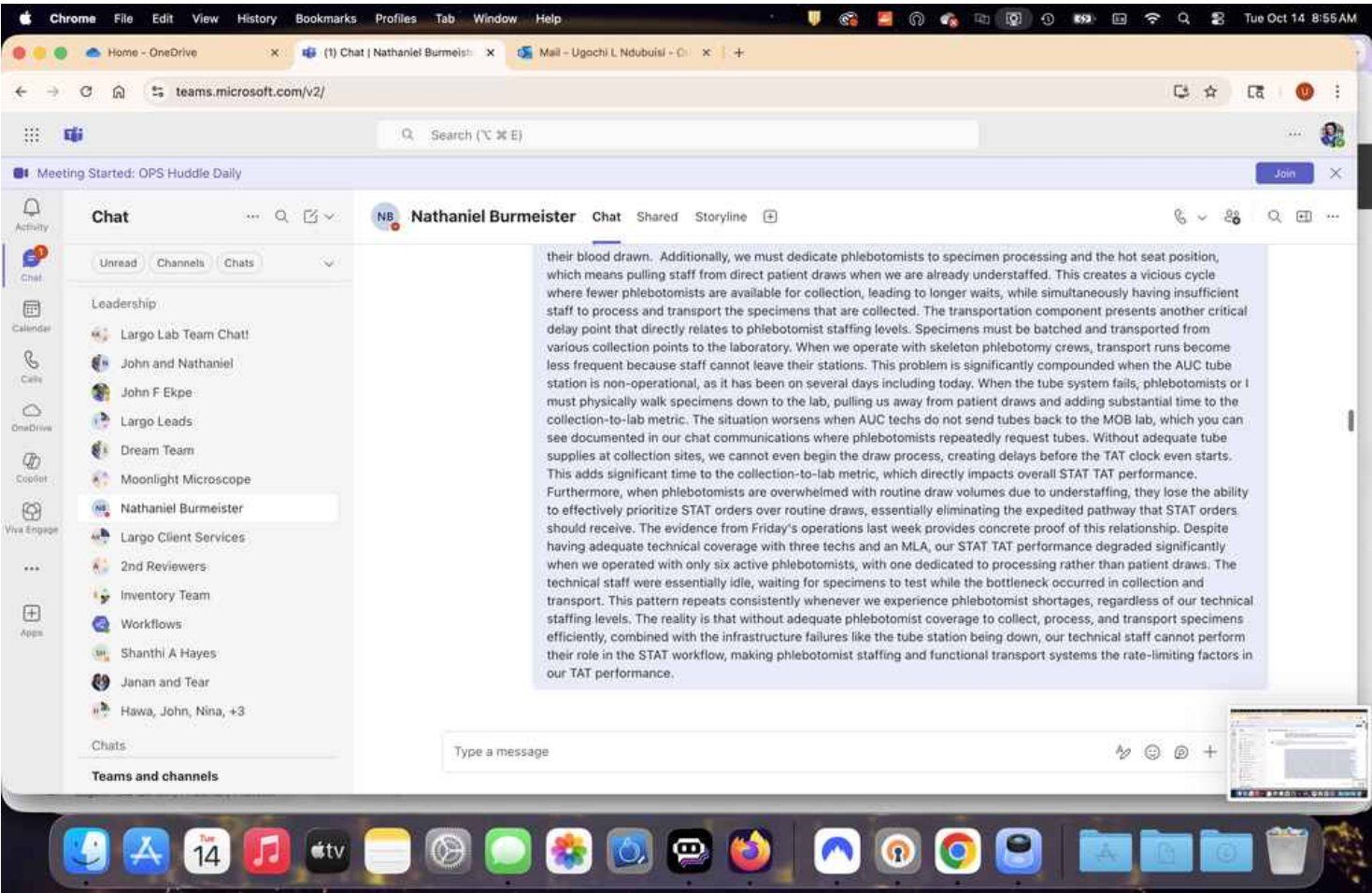




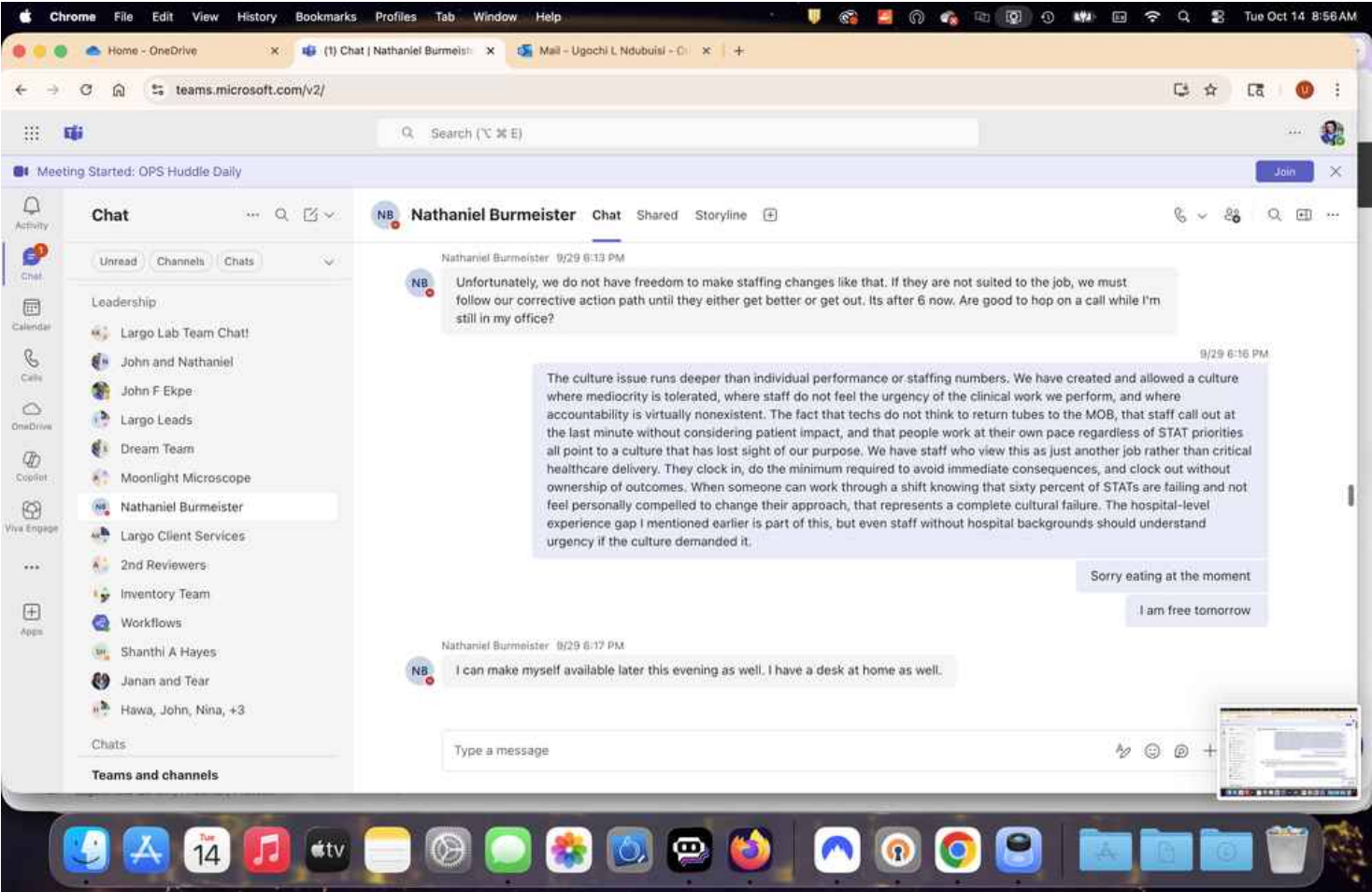


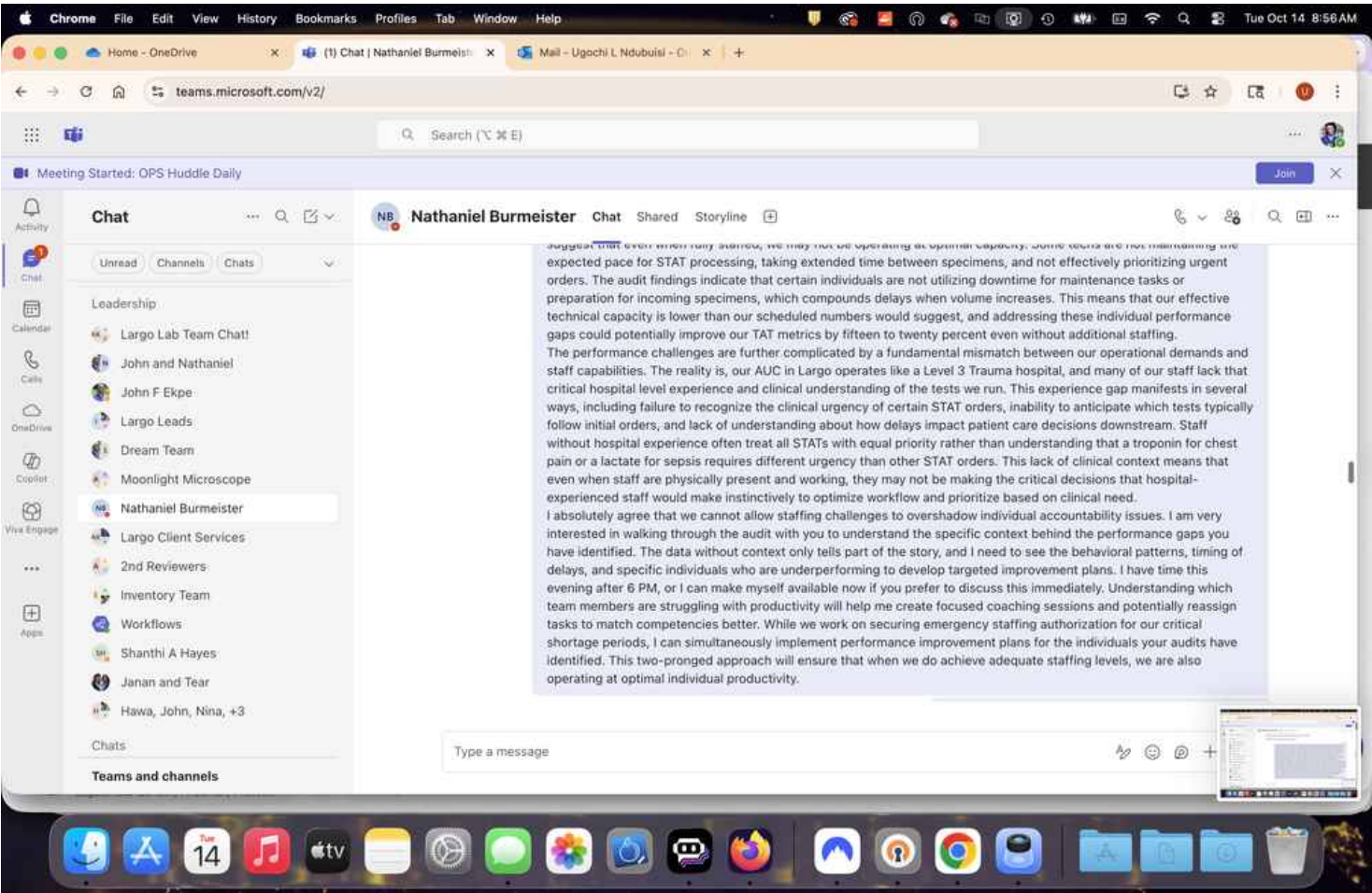


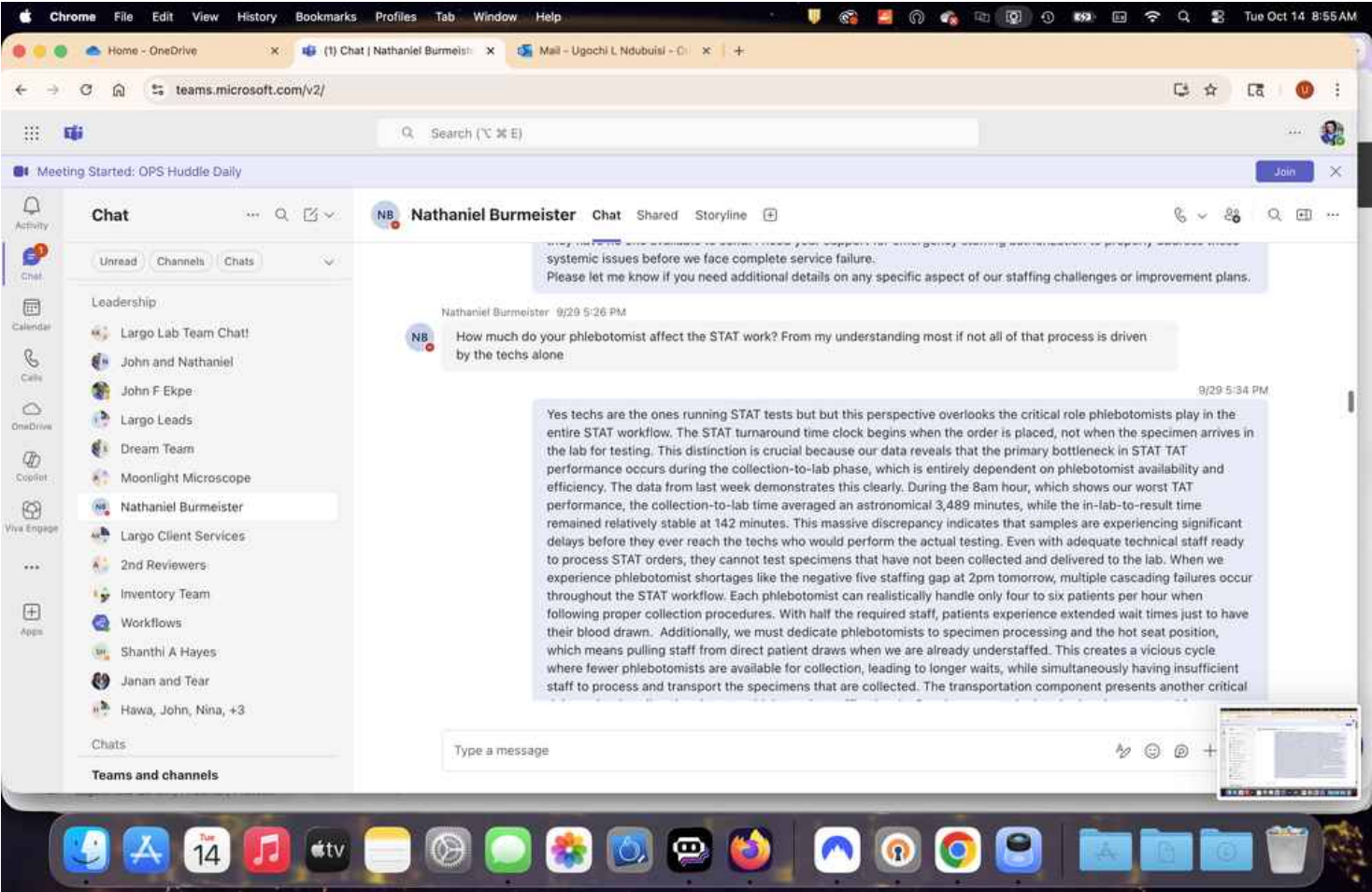
Evidence 13: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.55.37AM



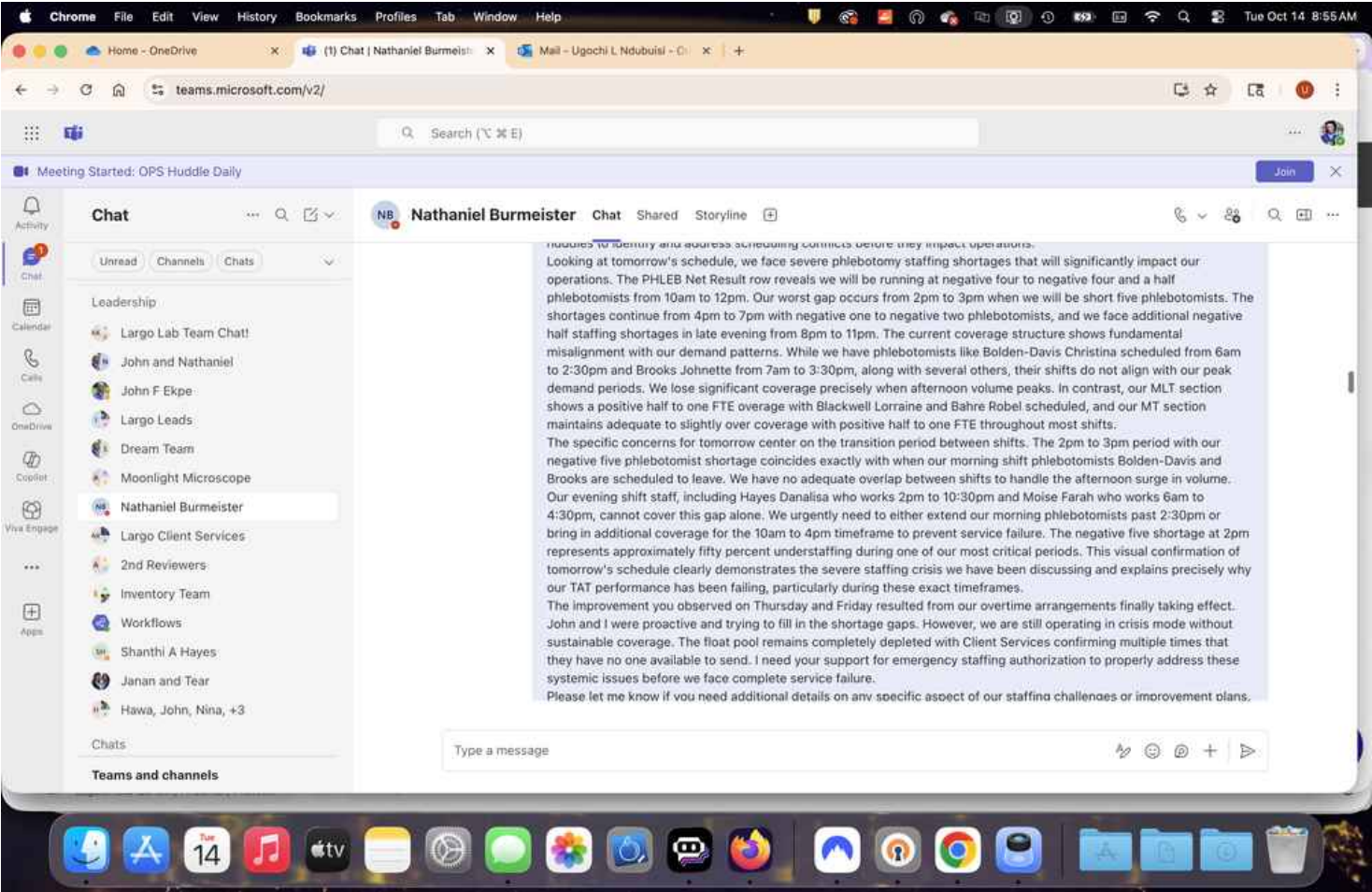
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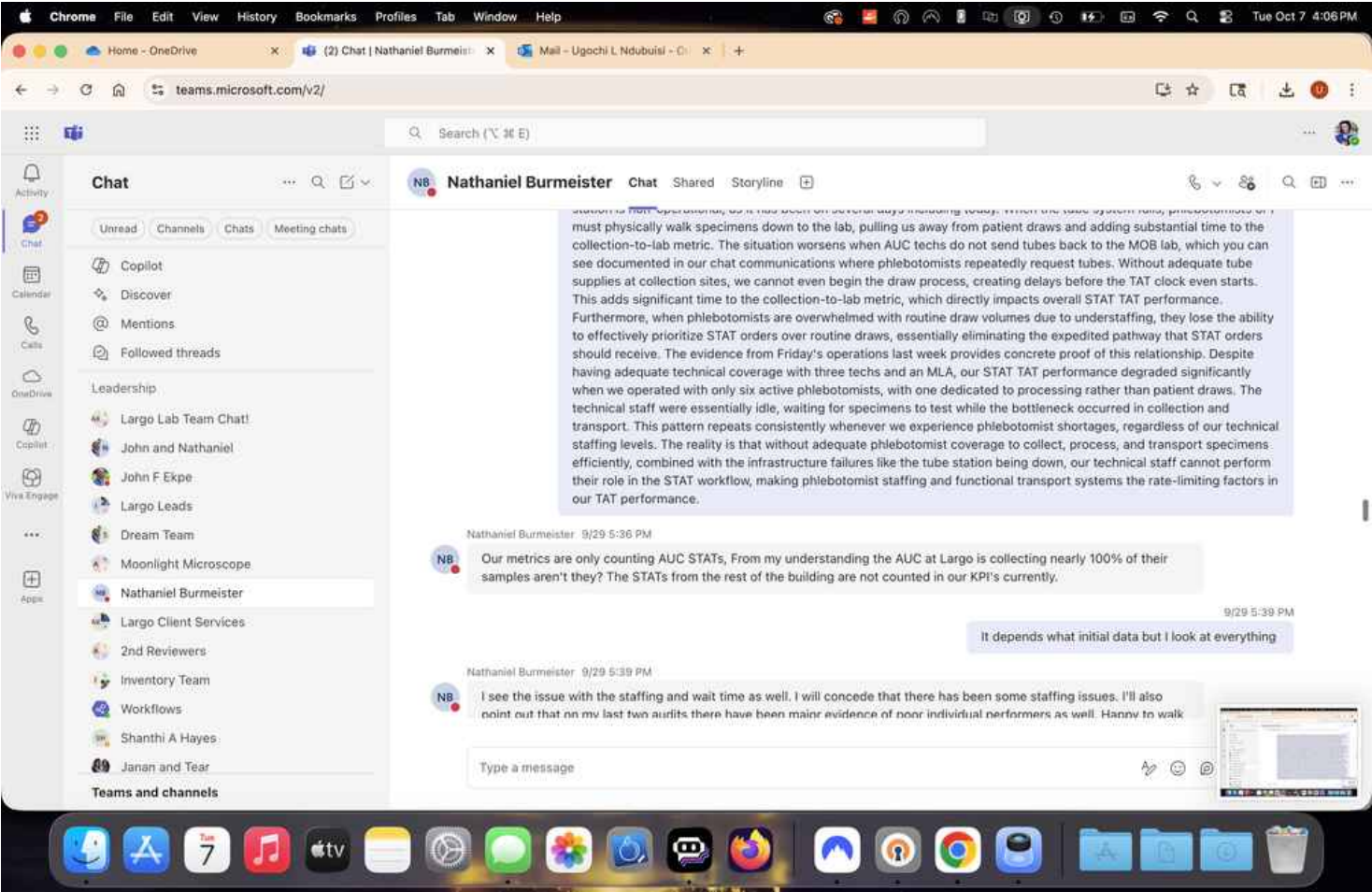


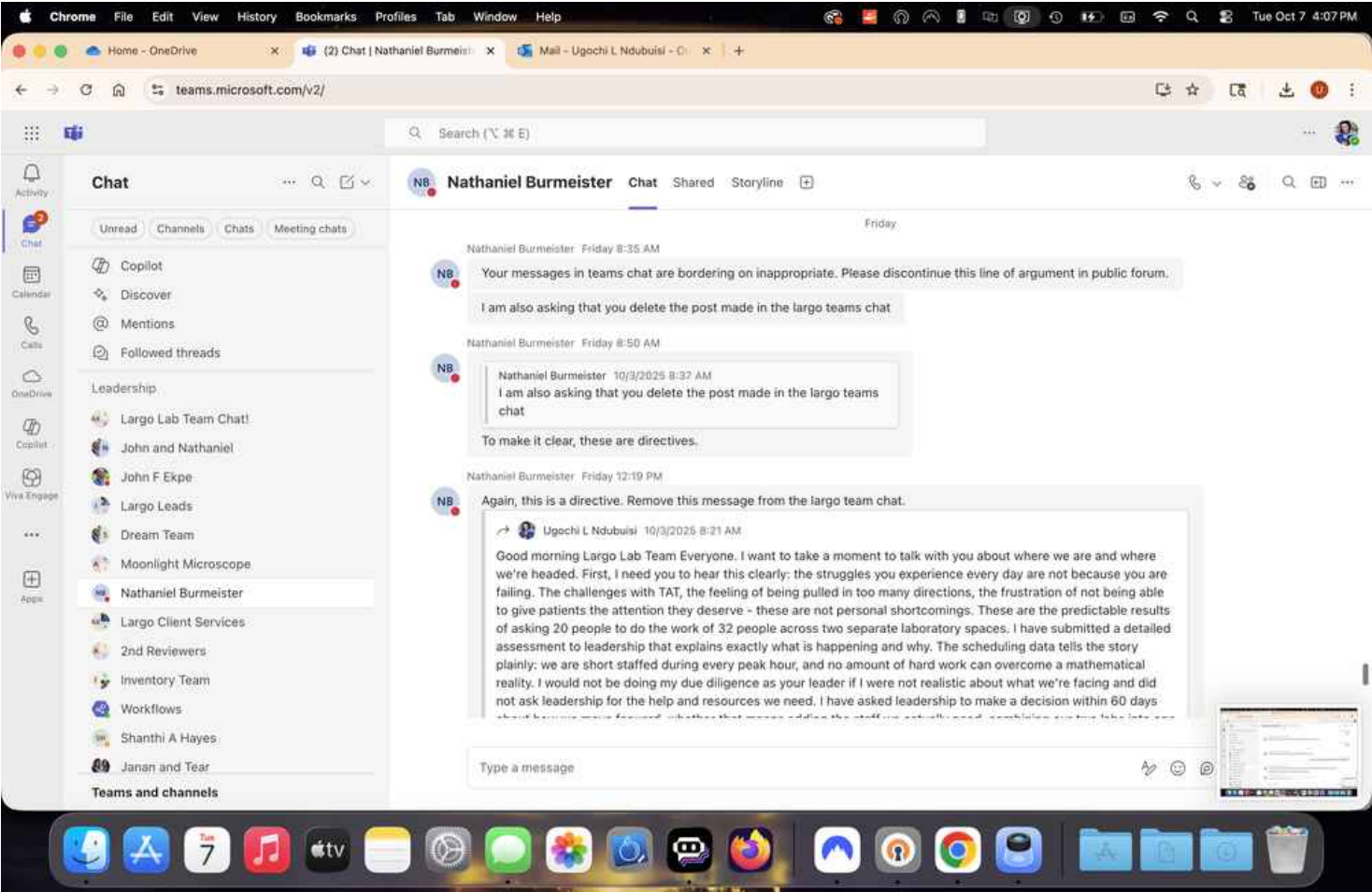


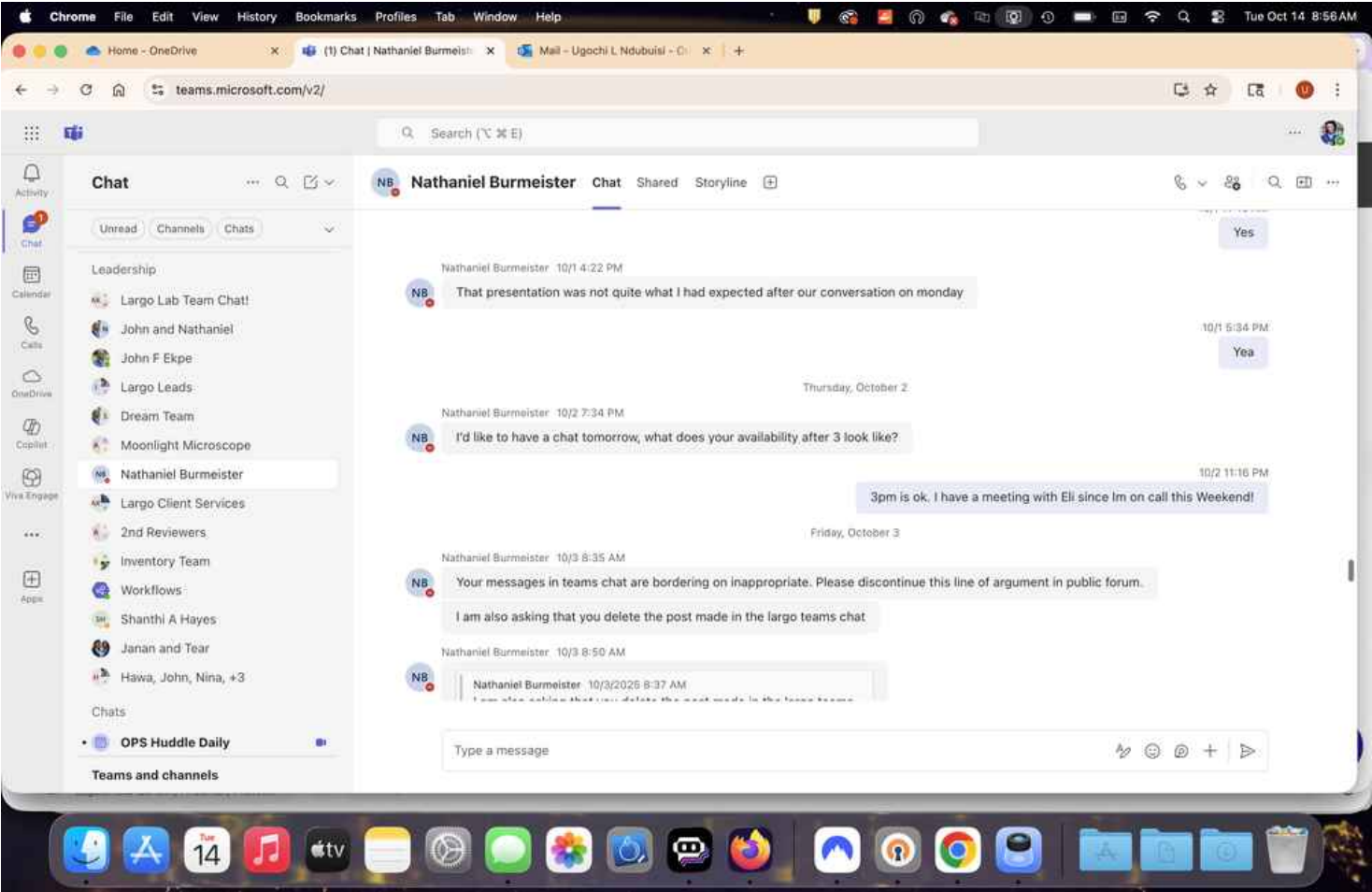


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WHISTLEBLOWER RETALIATION EVIDENCE

Largo Laboratory - Kaiser Permanente

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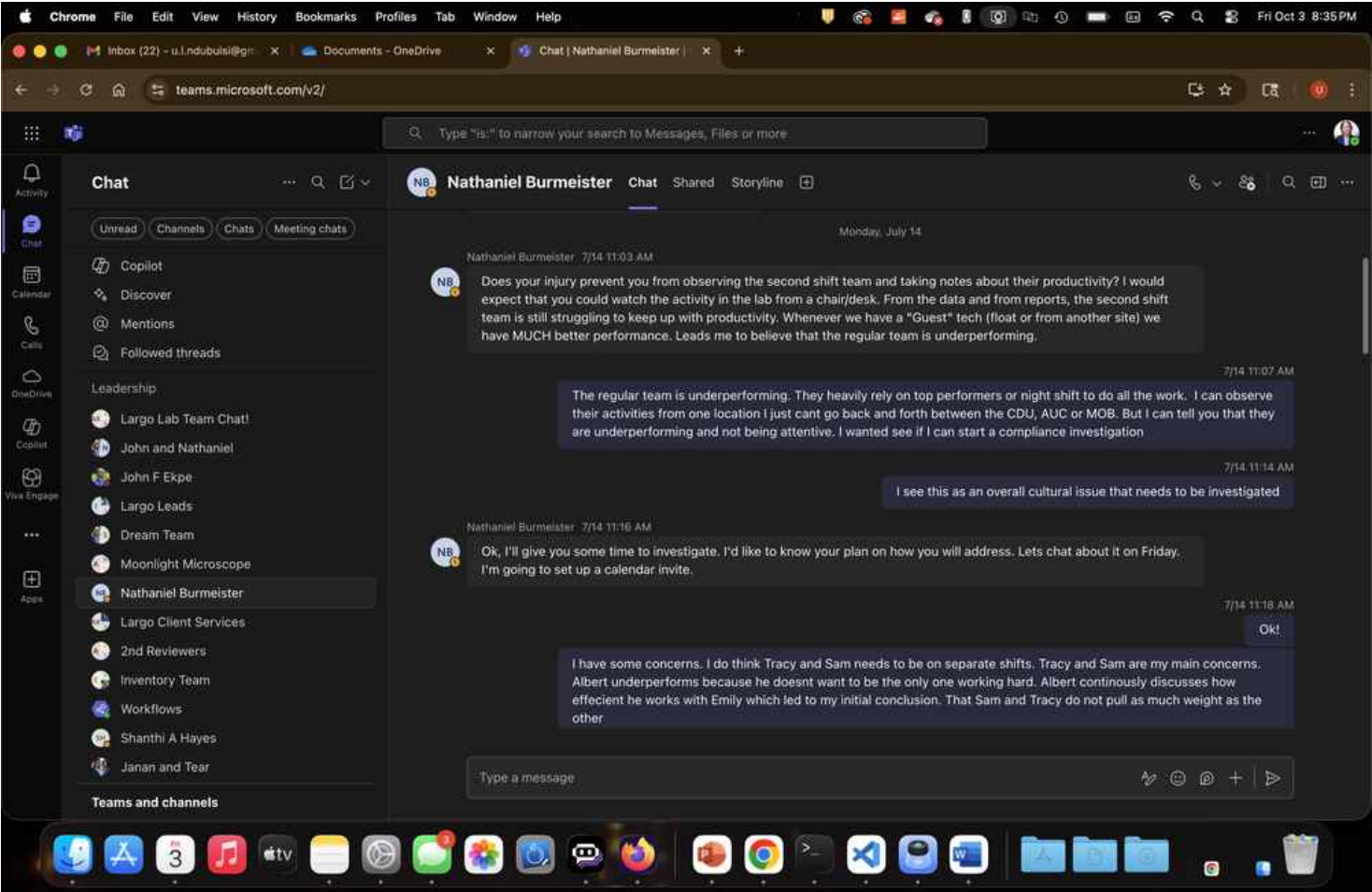
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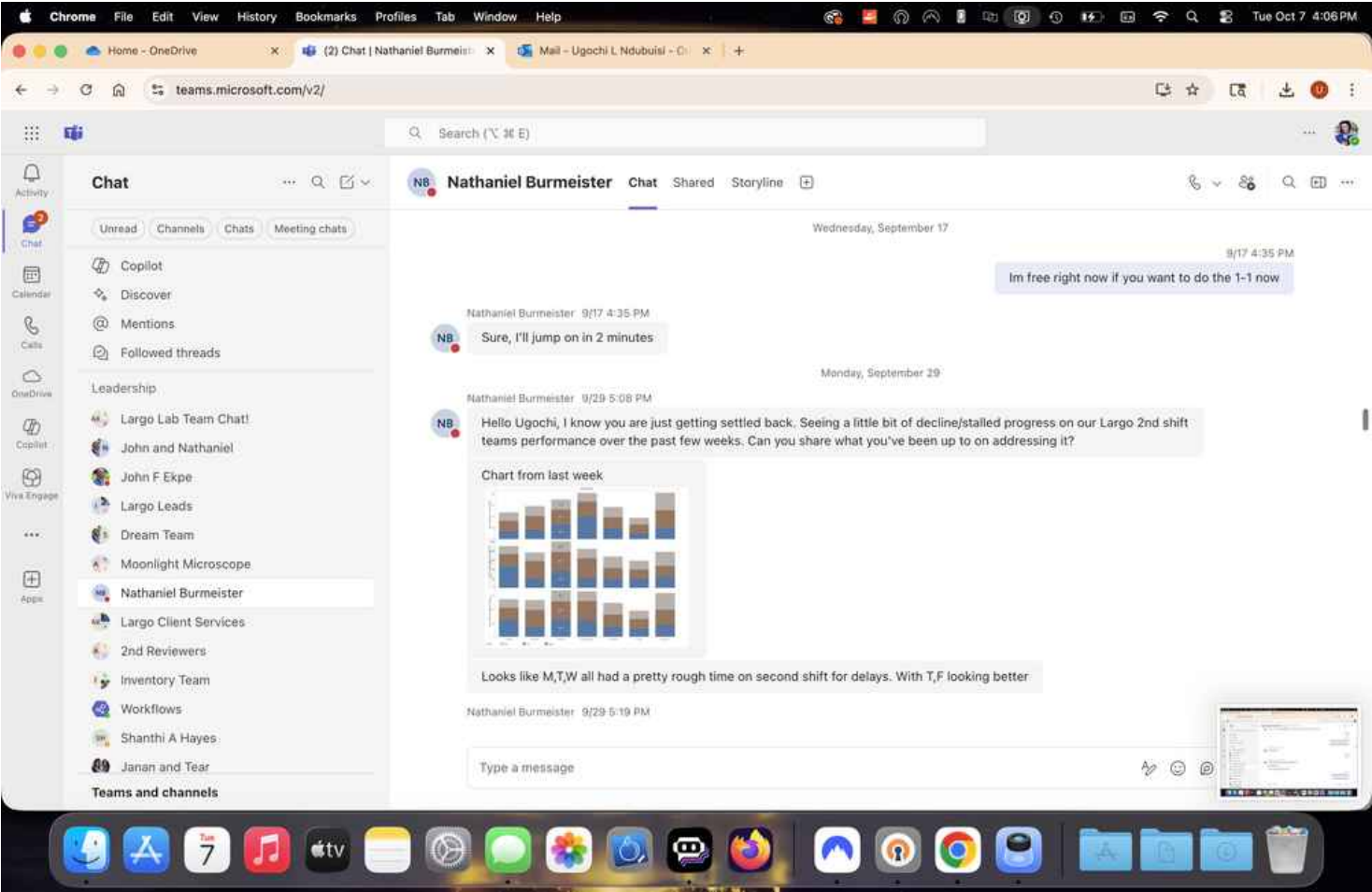
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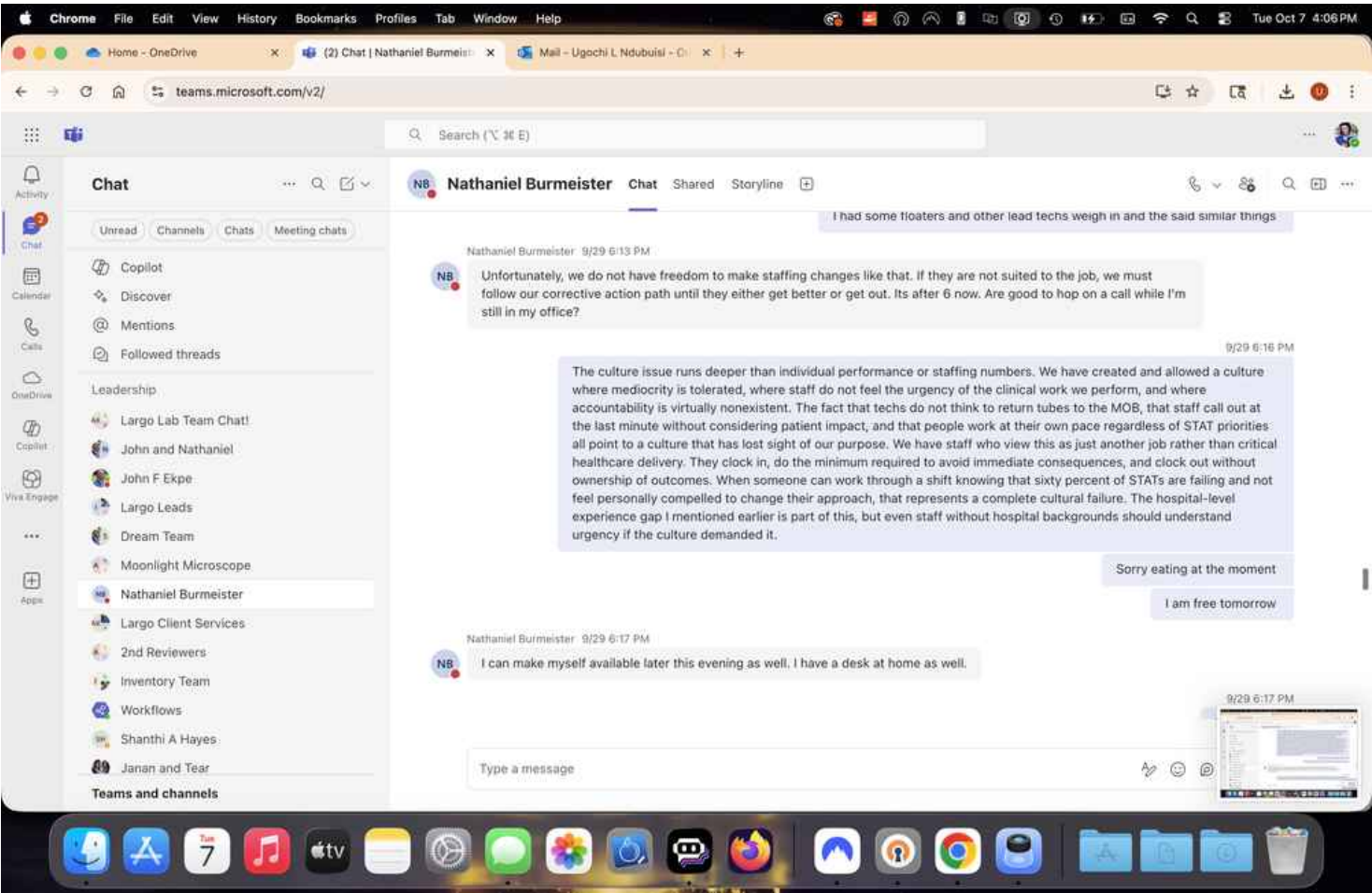
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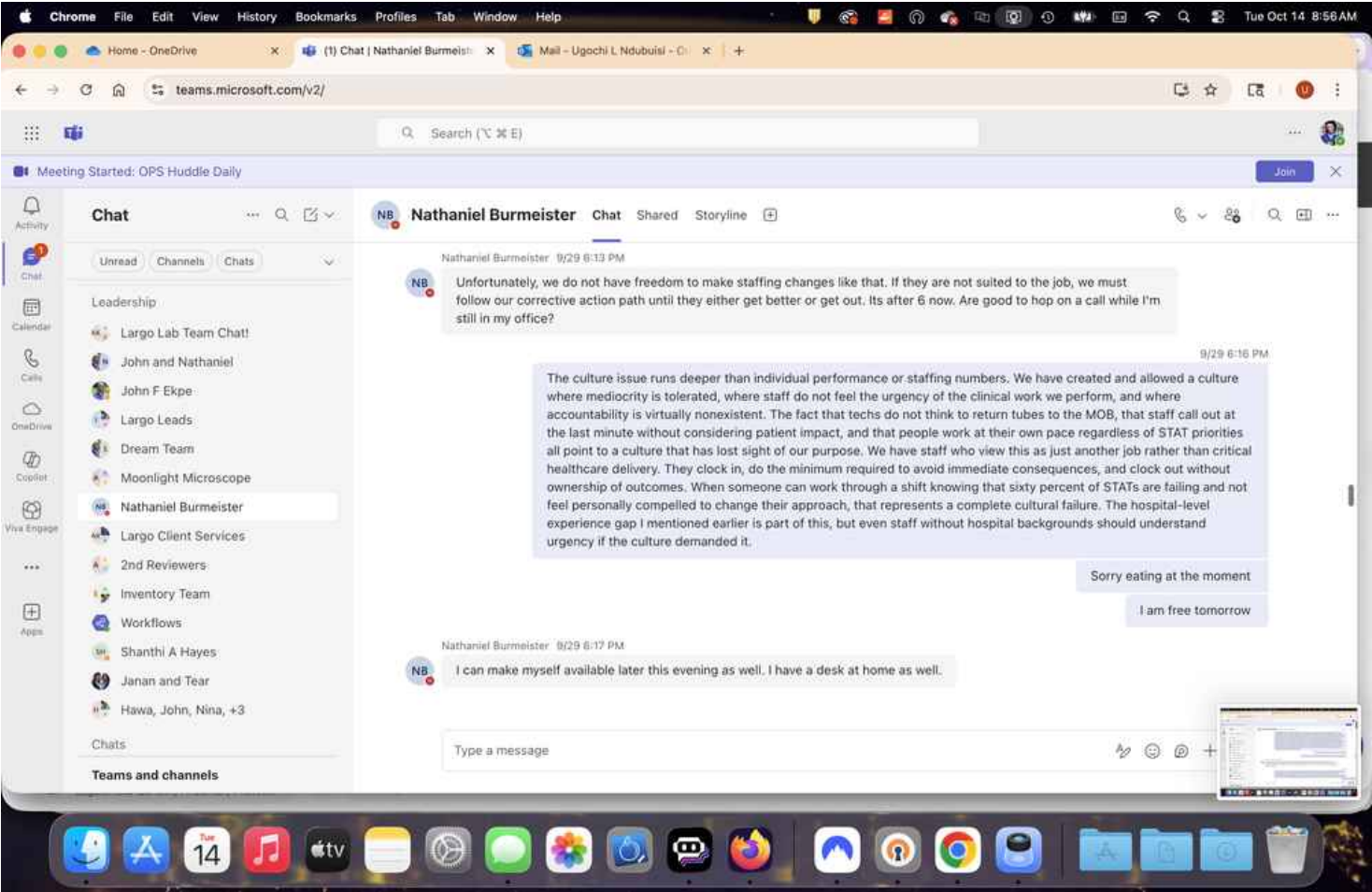
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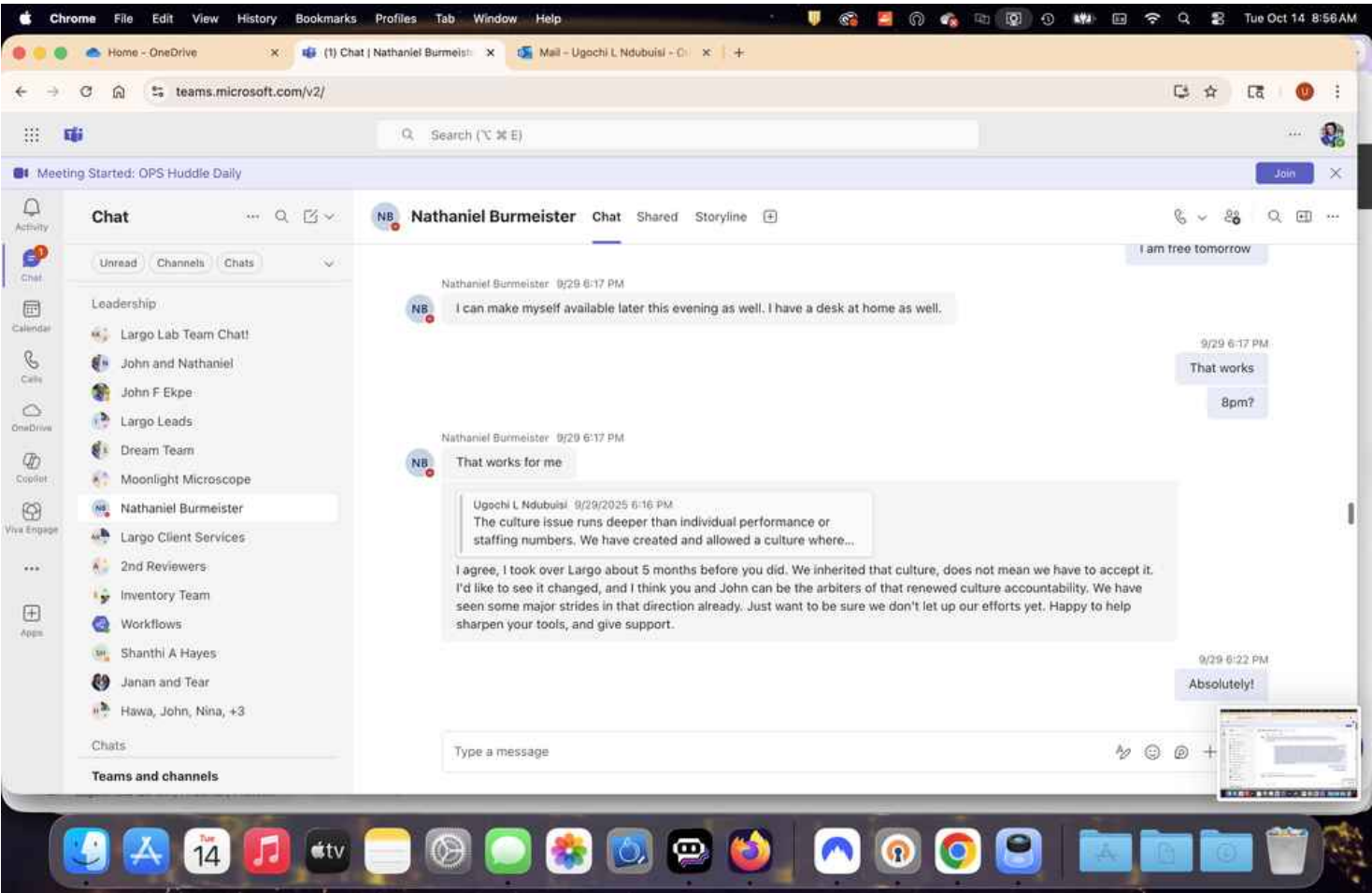


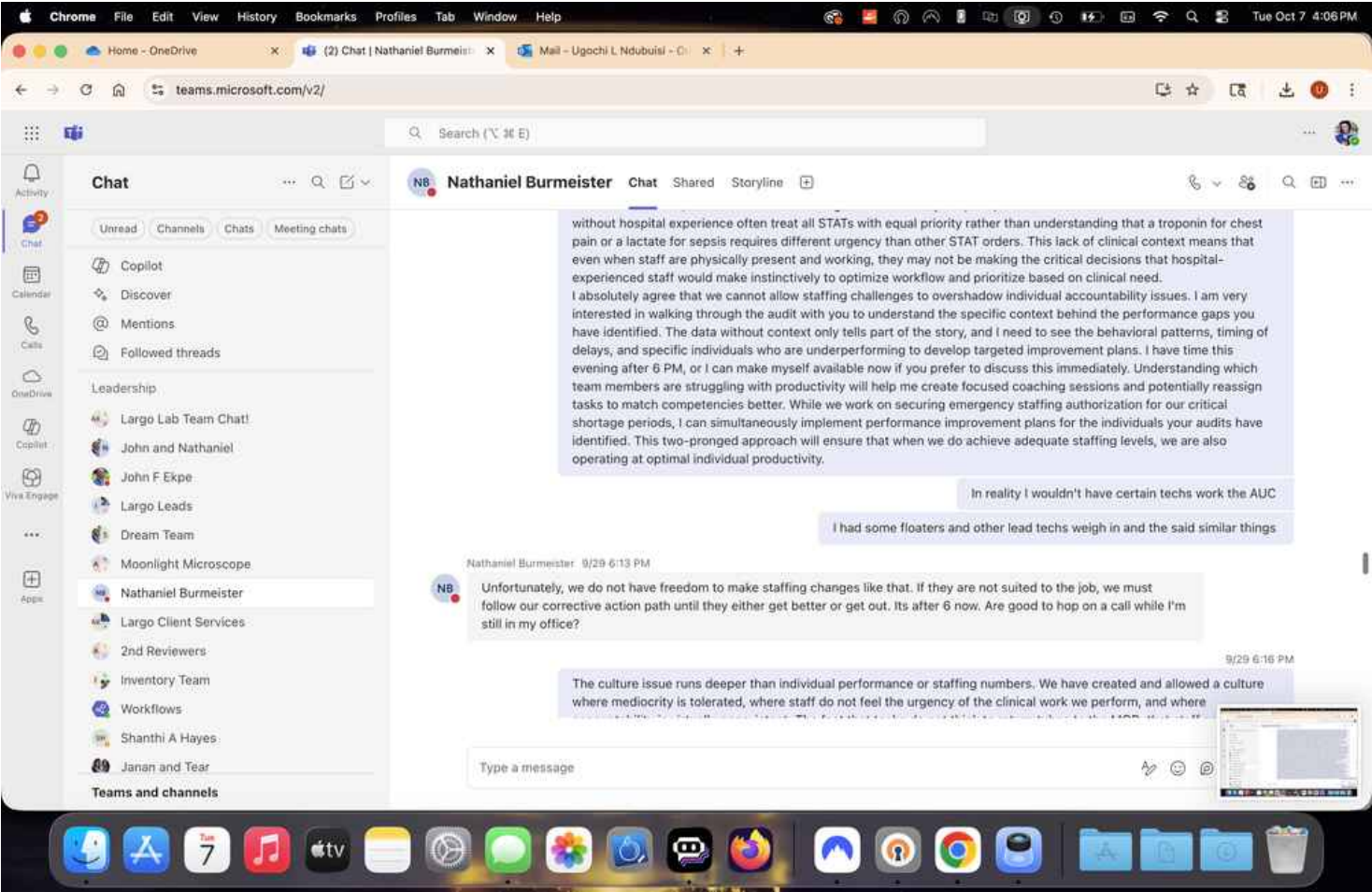


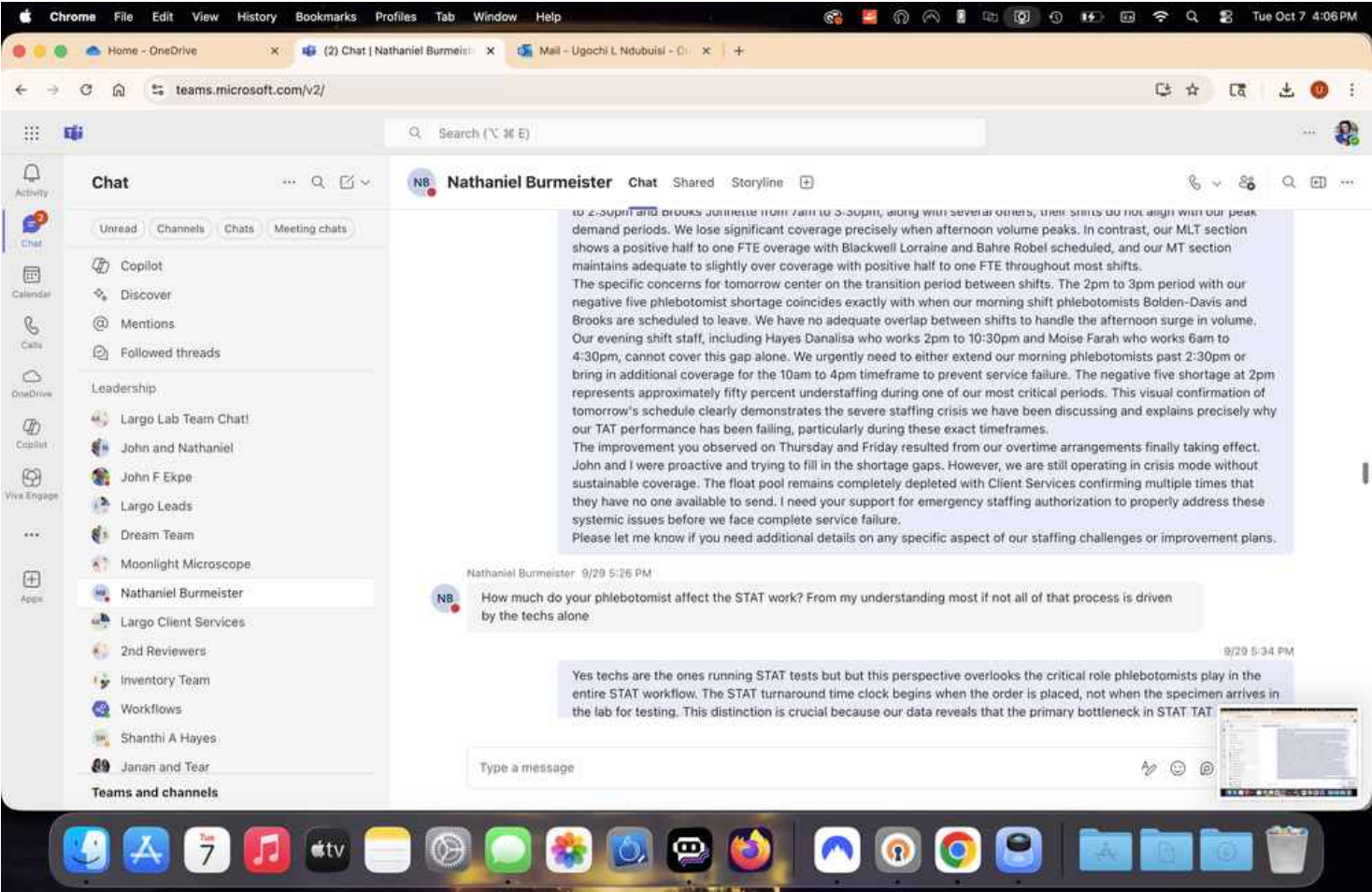


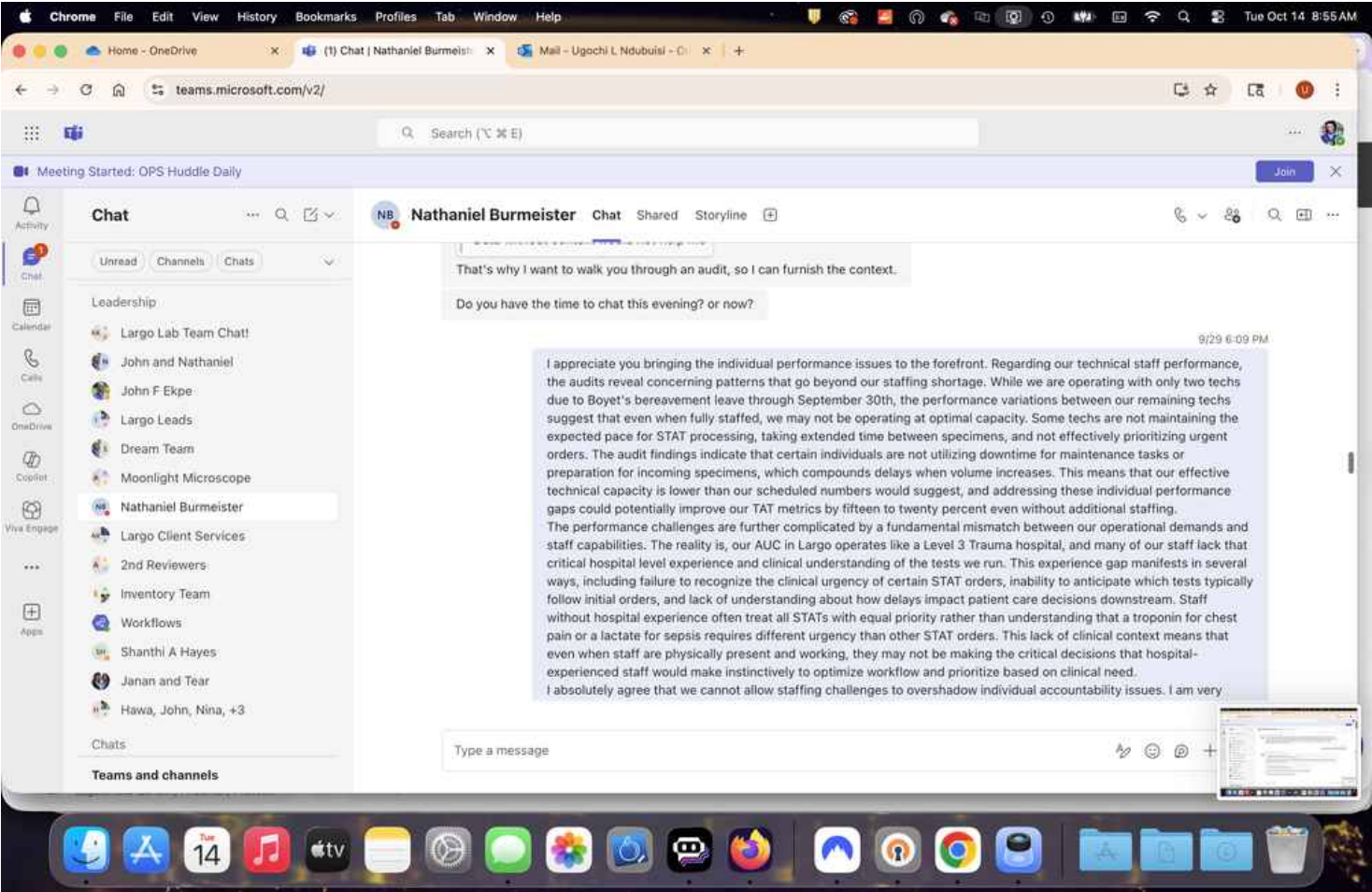
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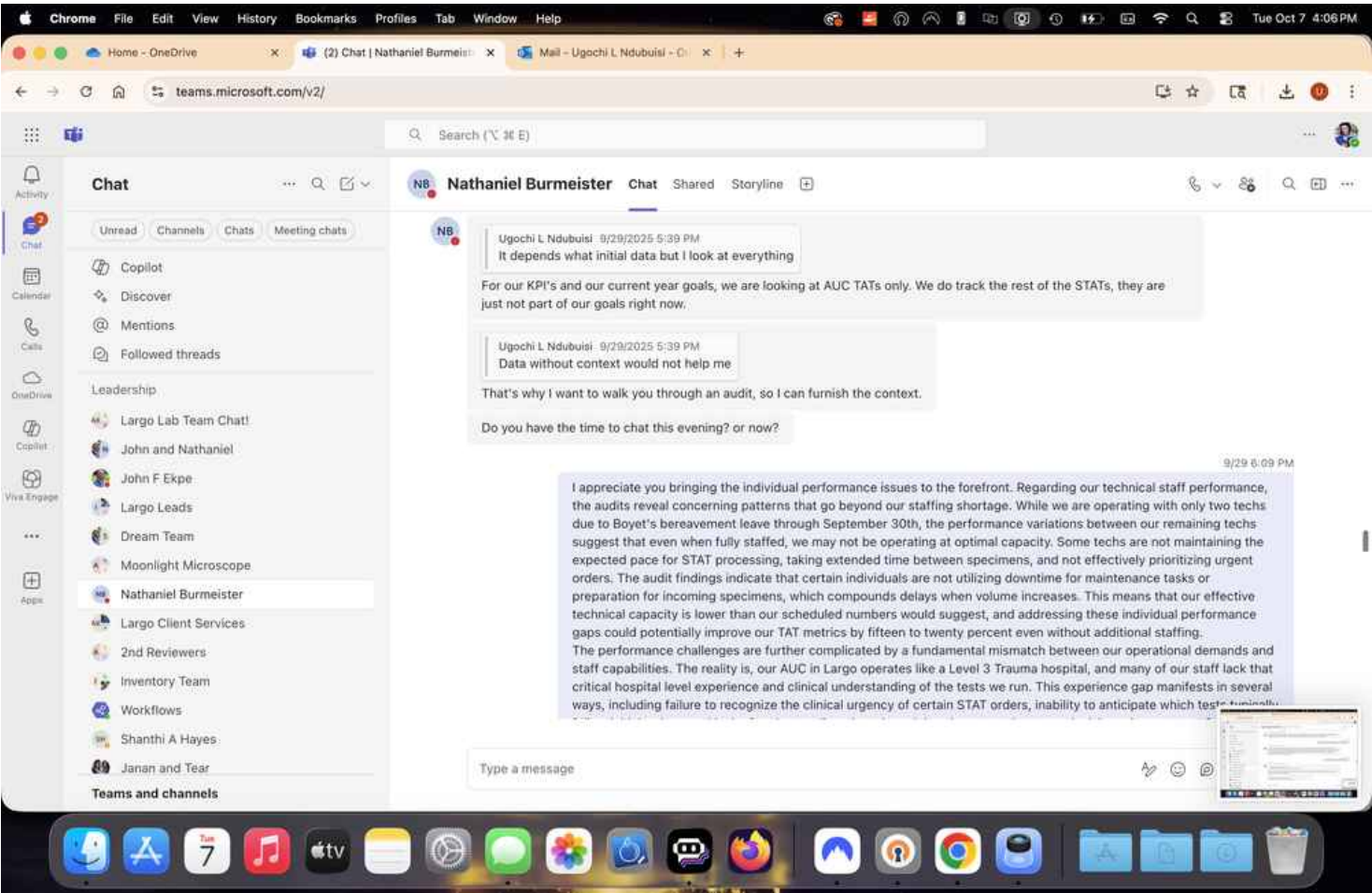


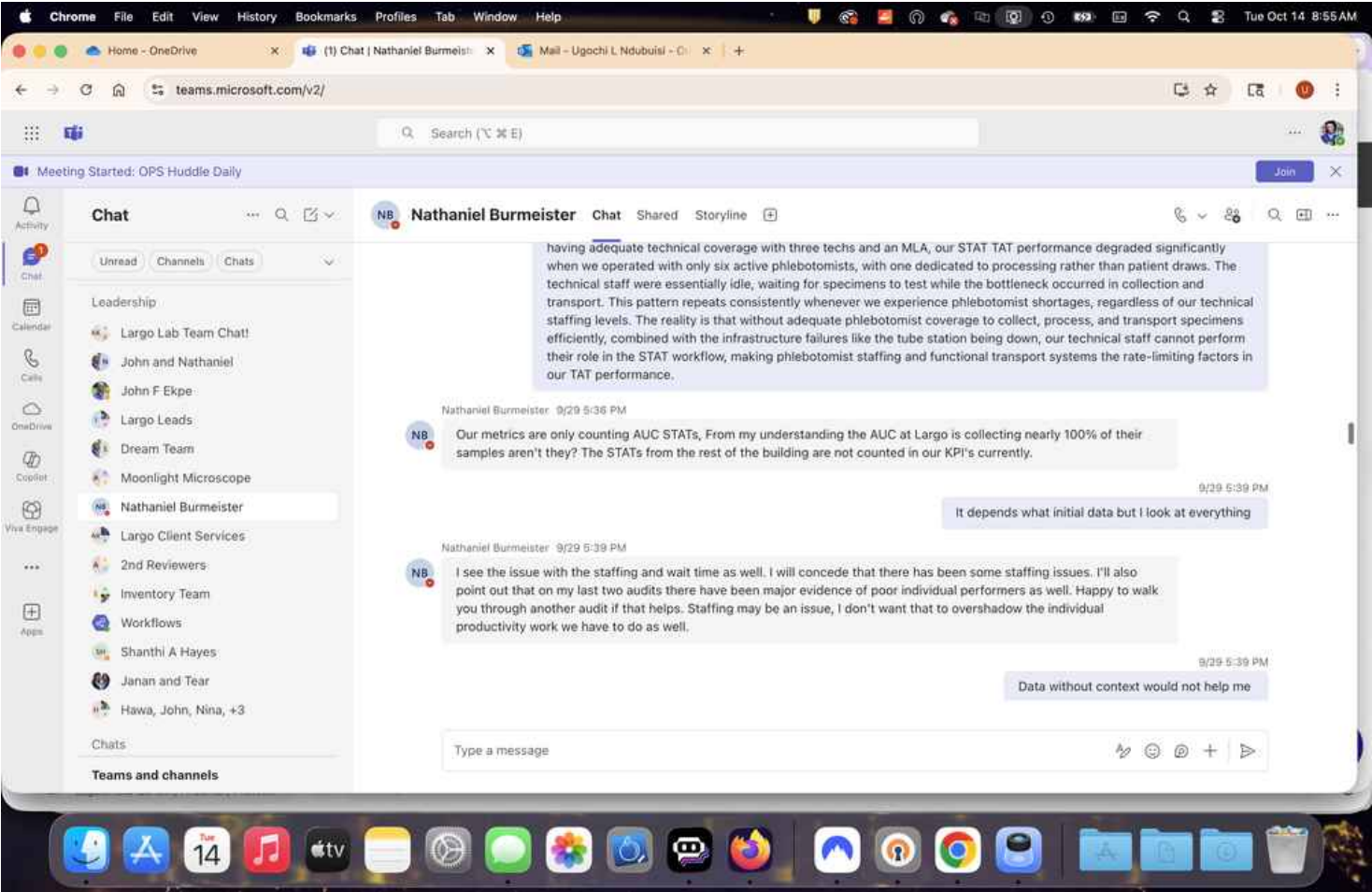


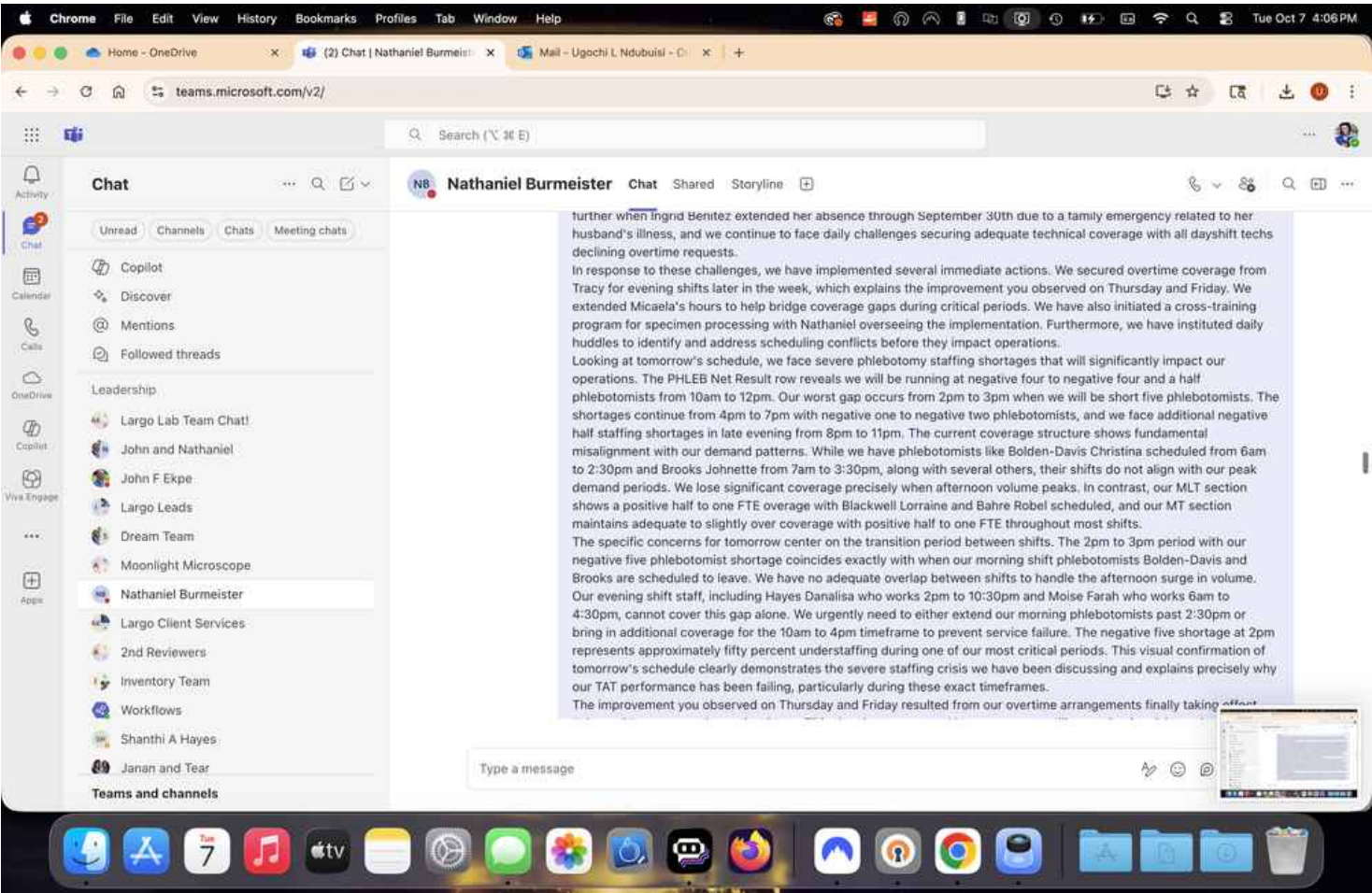


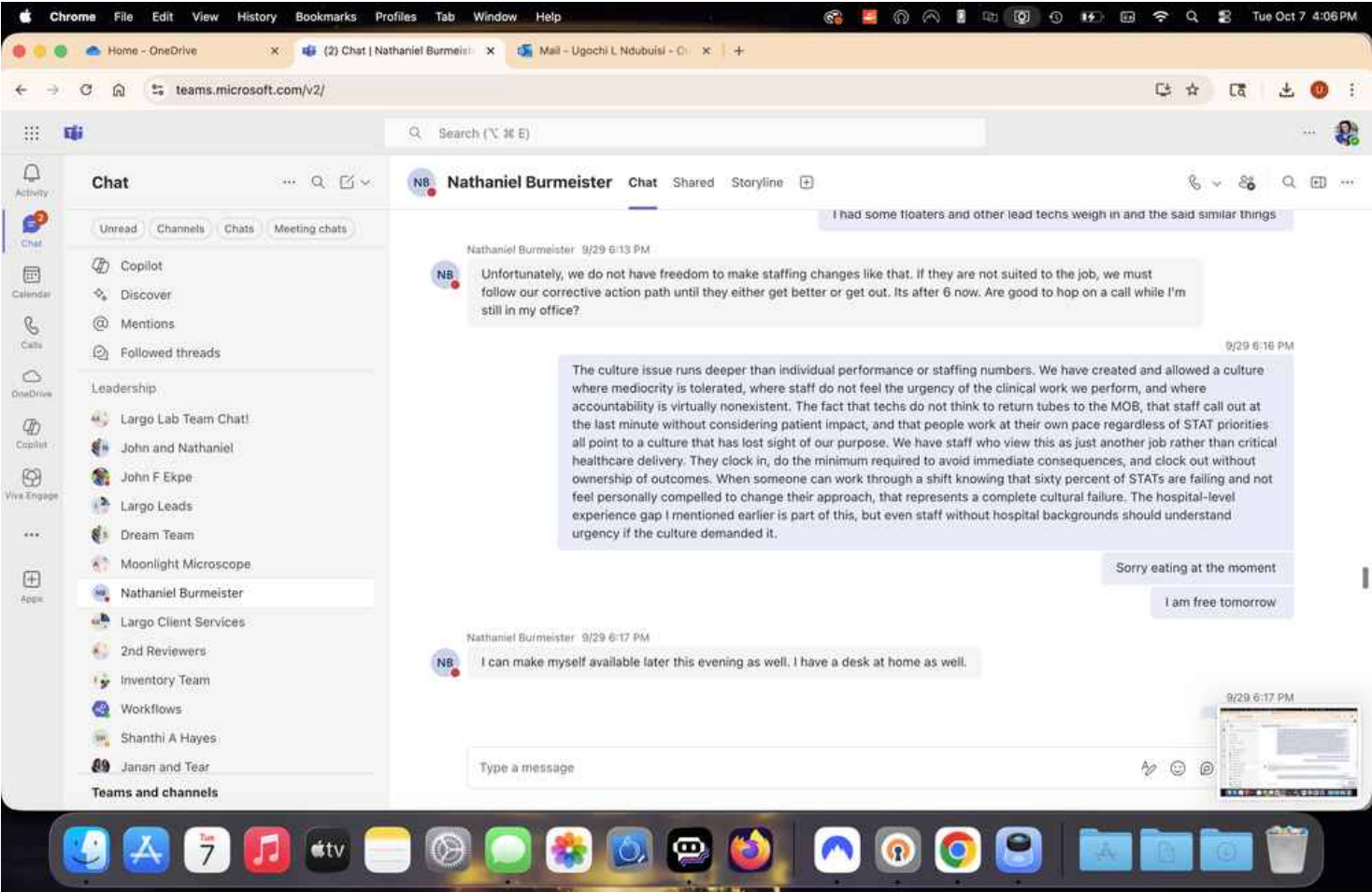




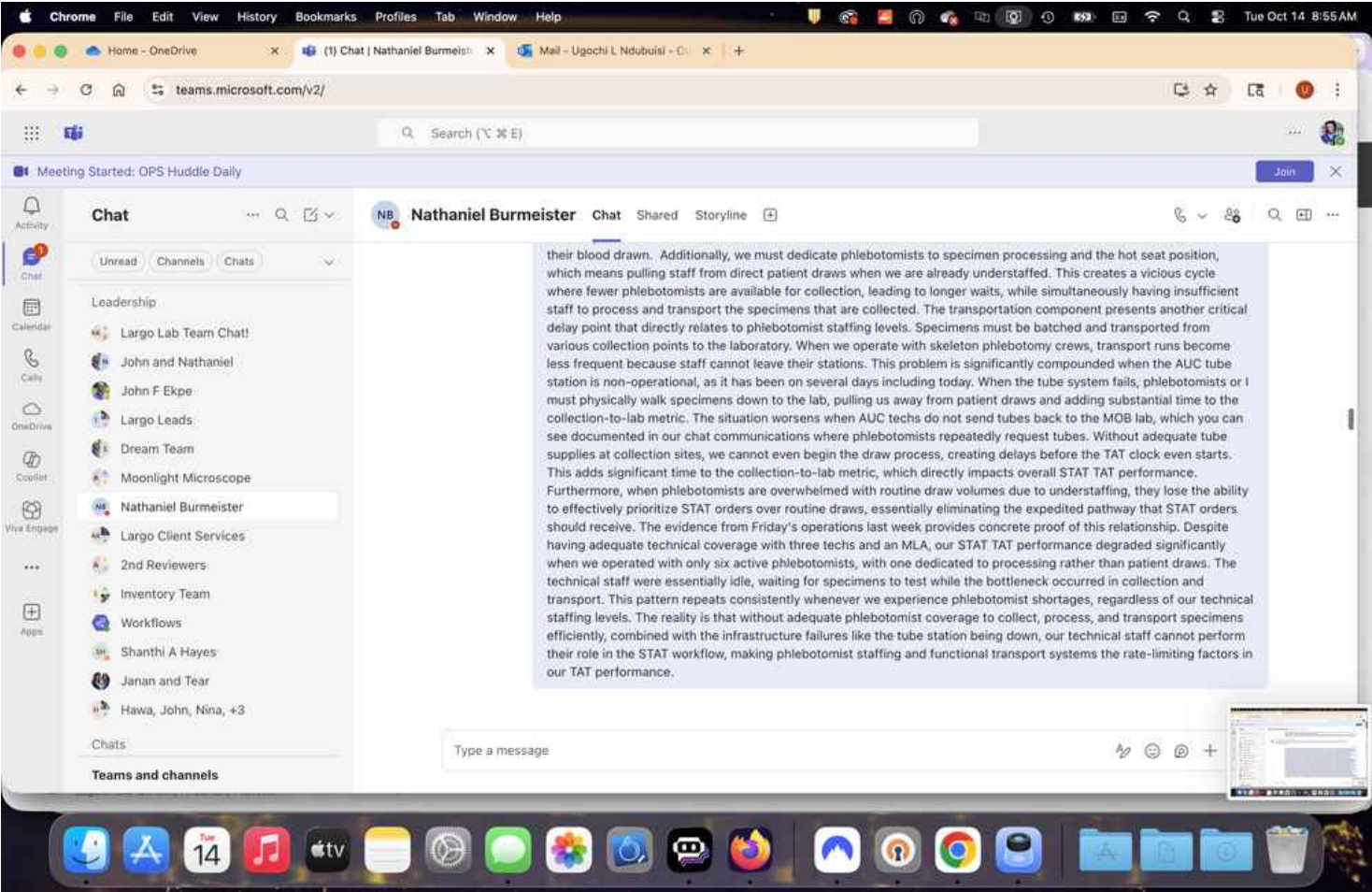




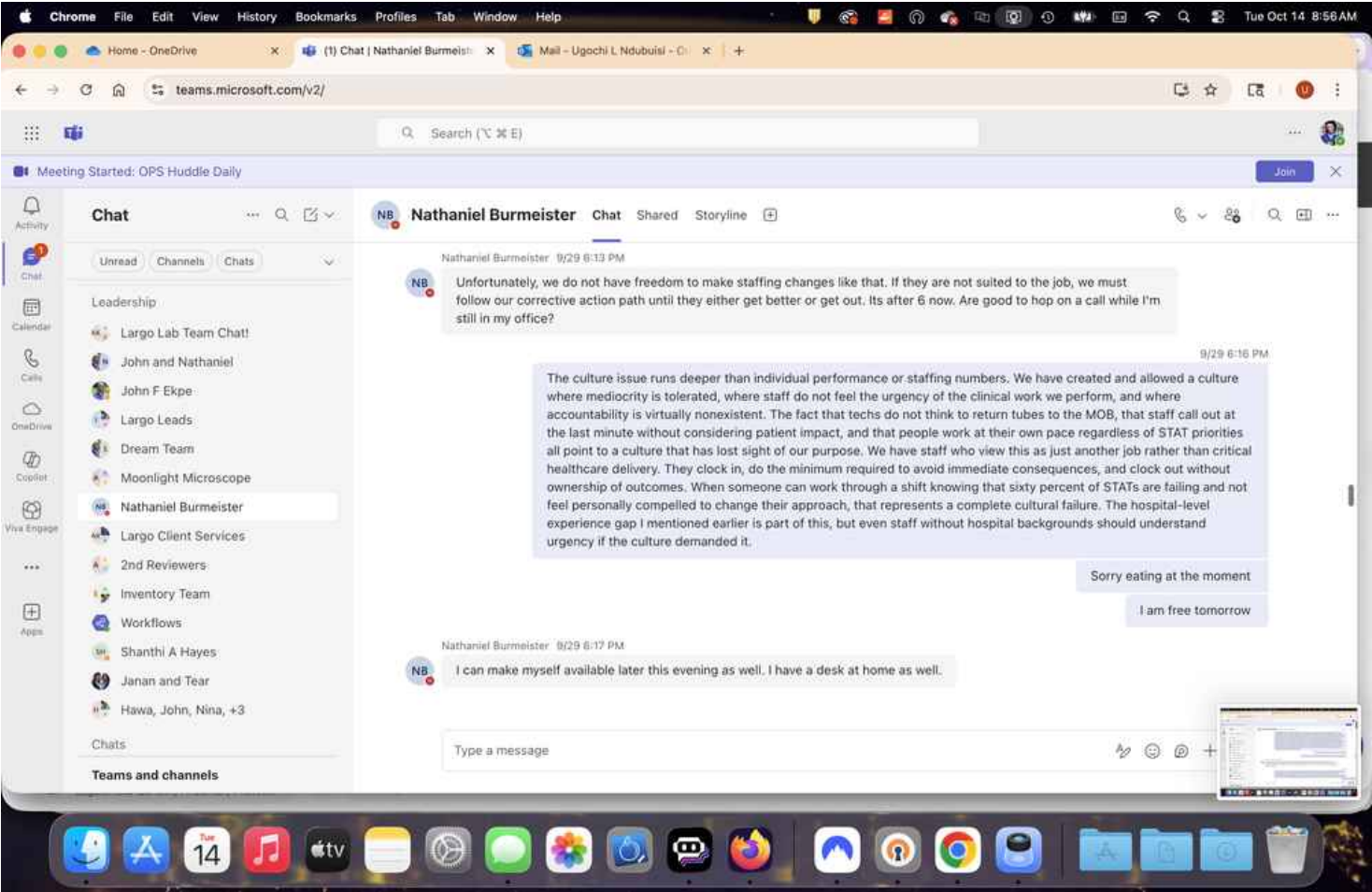


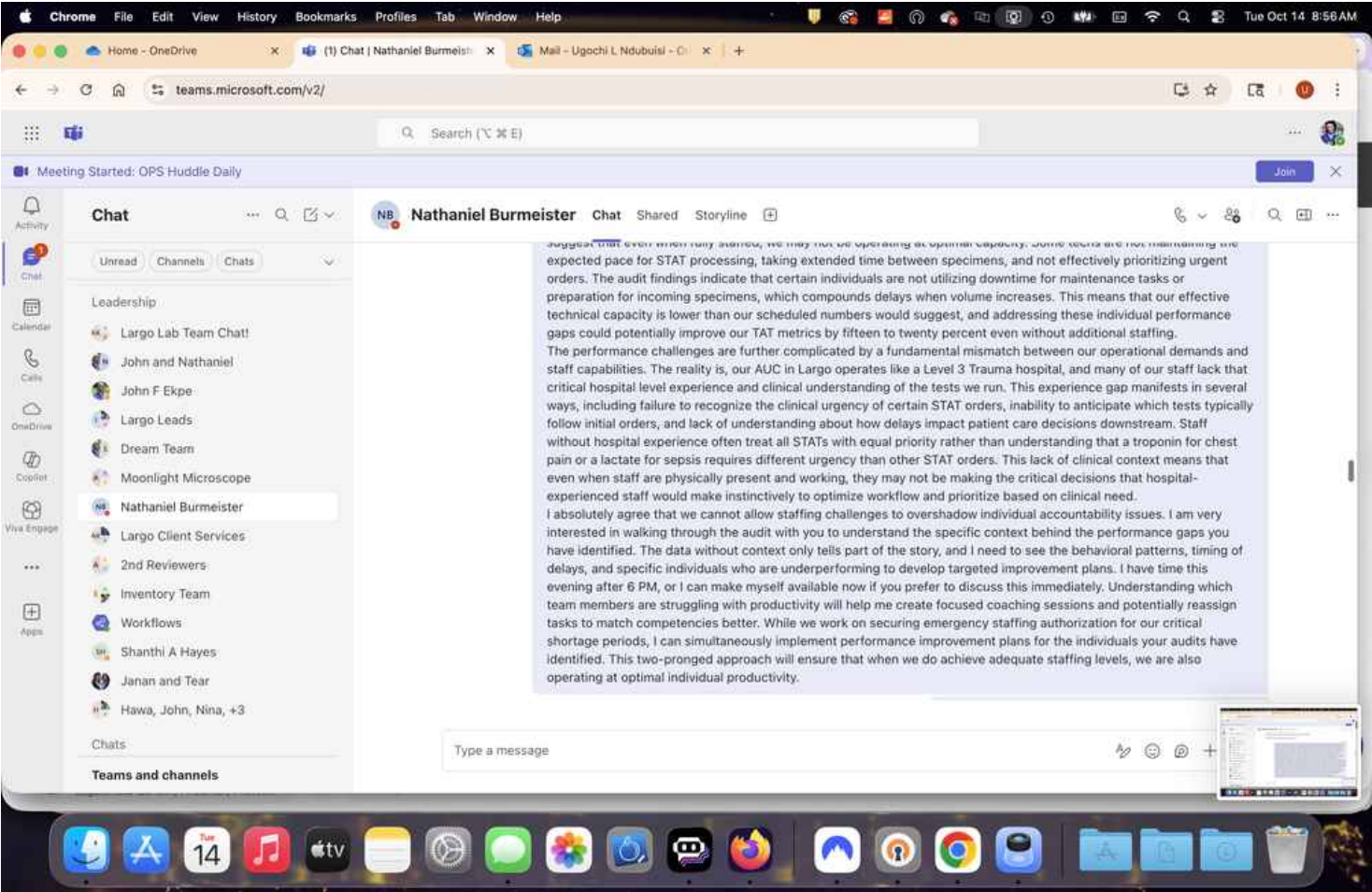


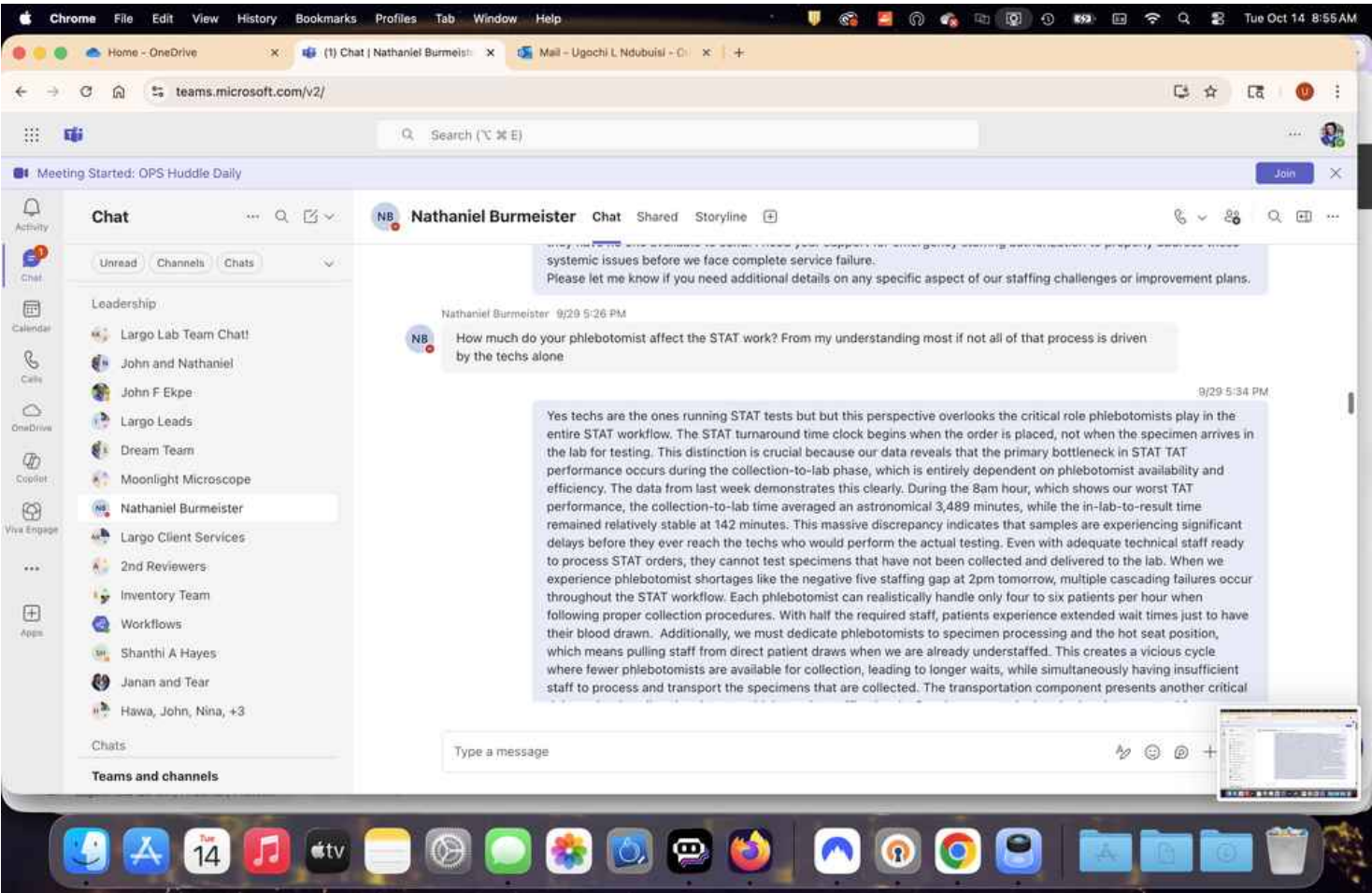
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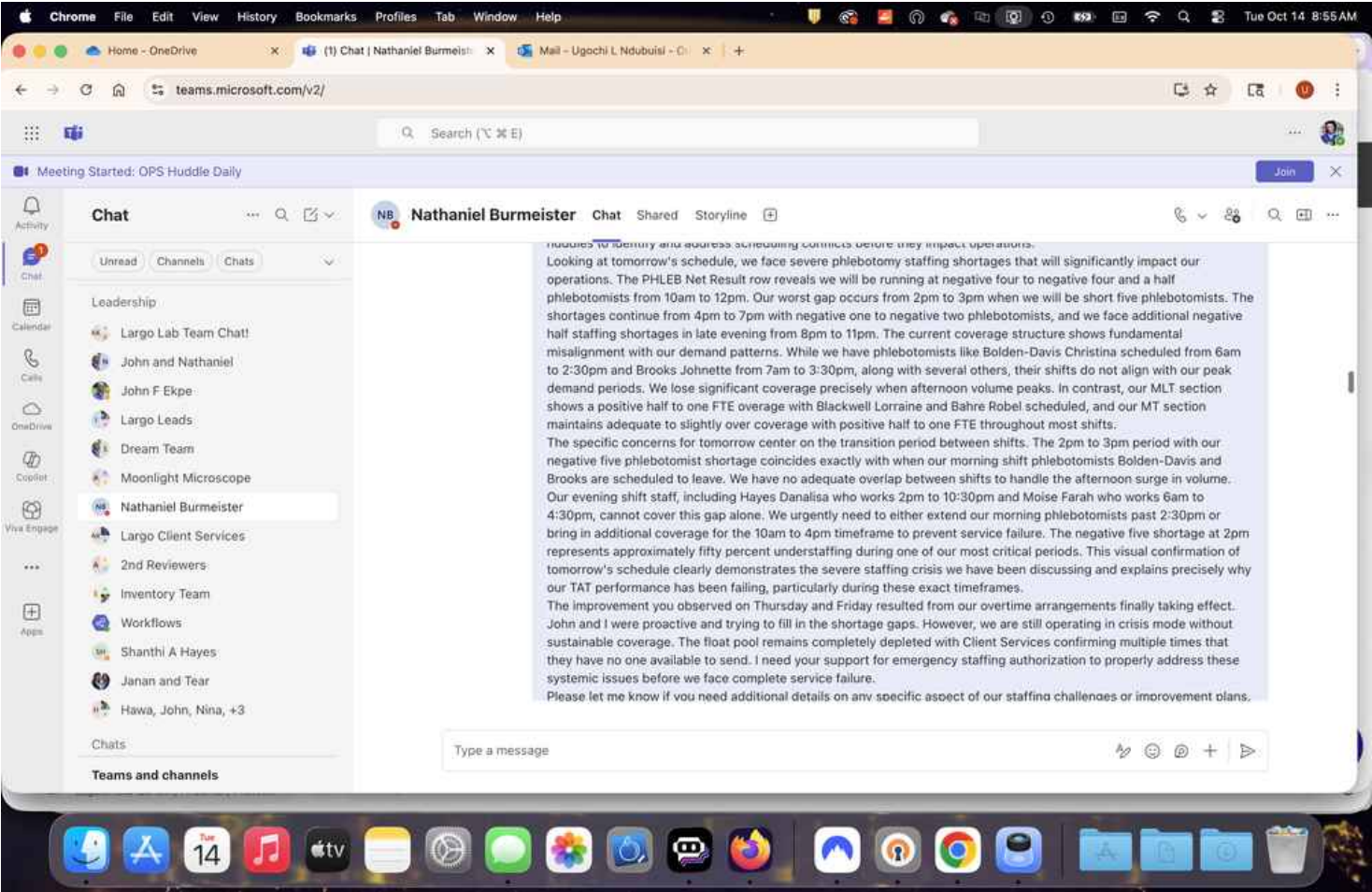
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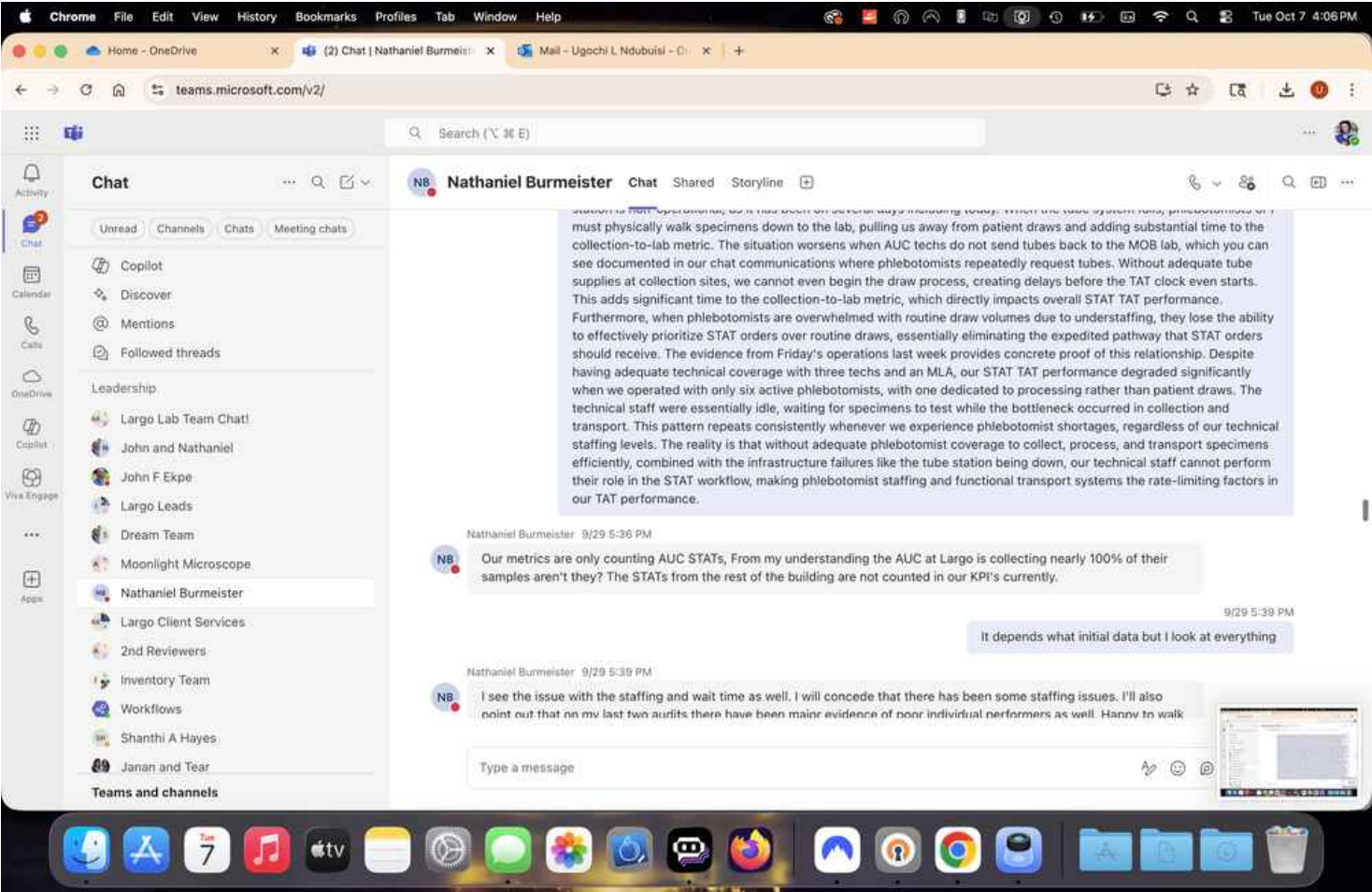


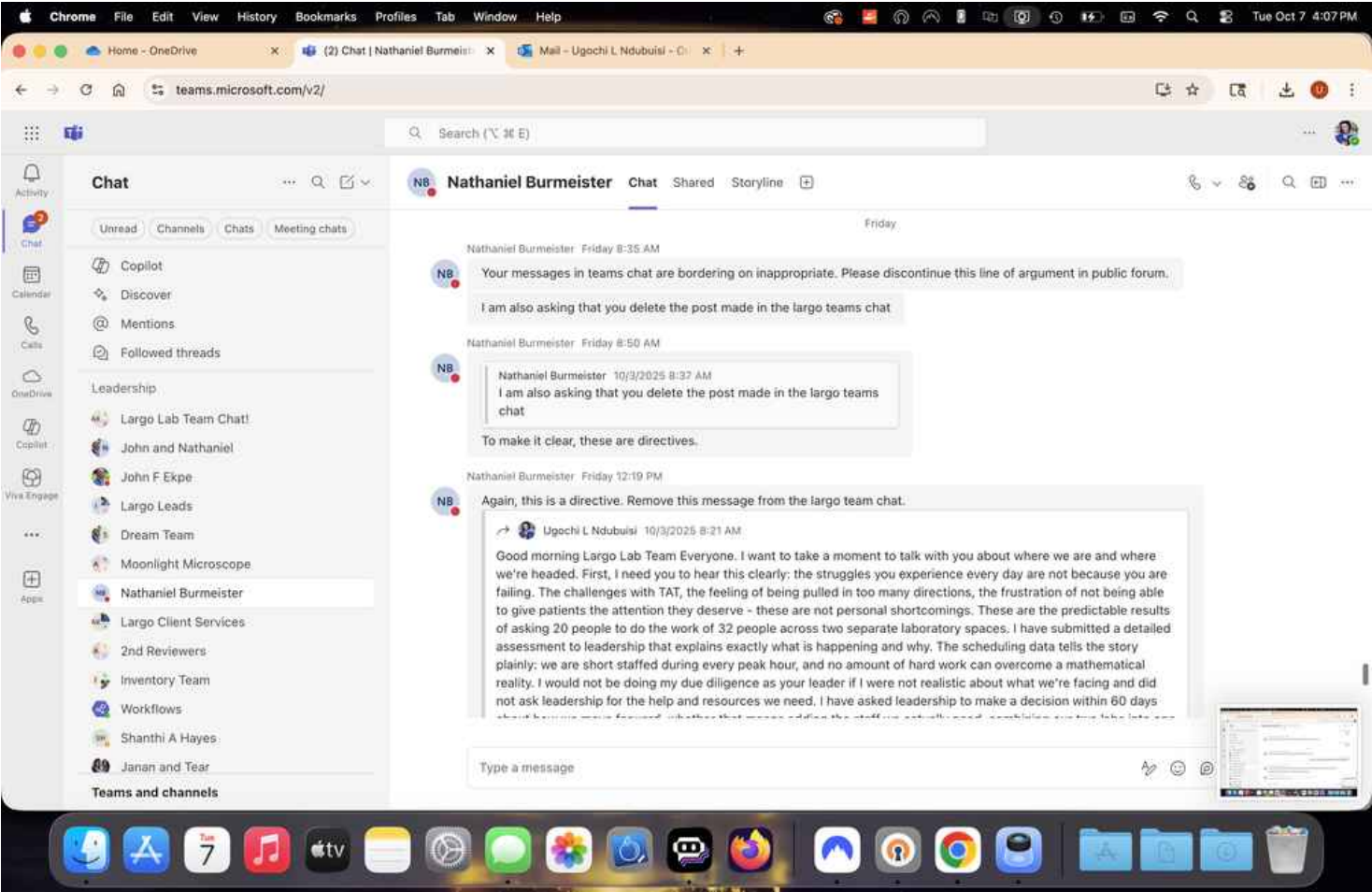


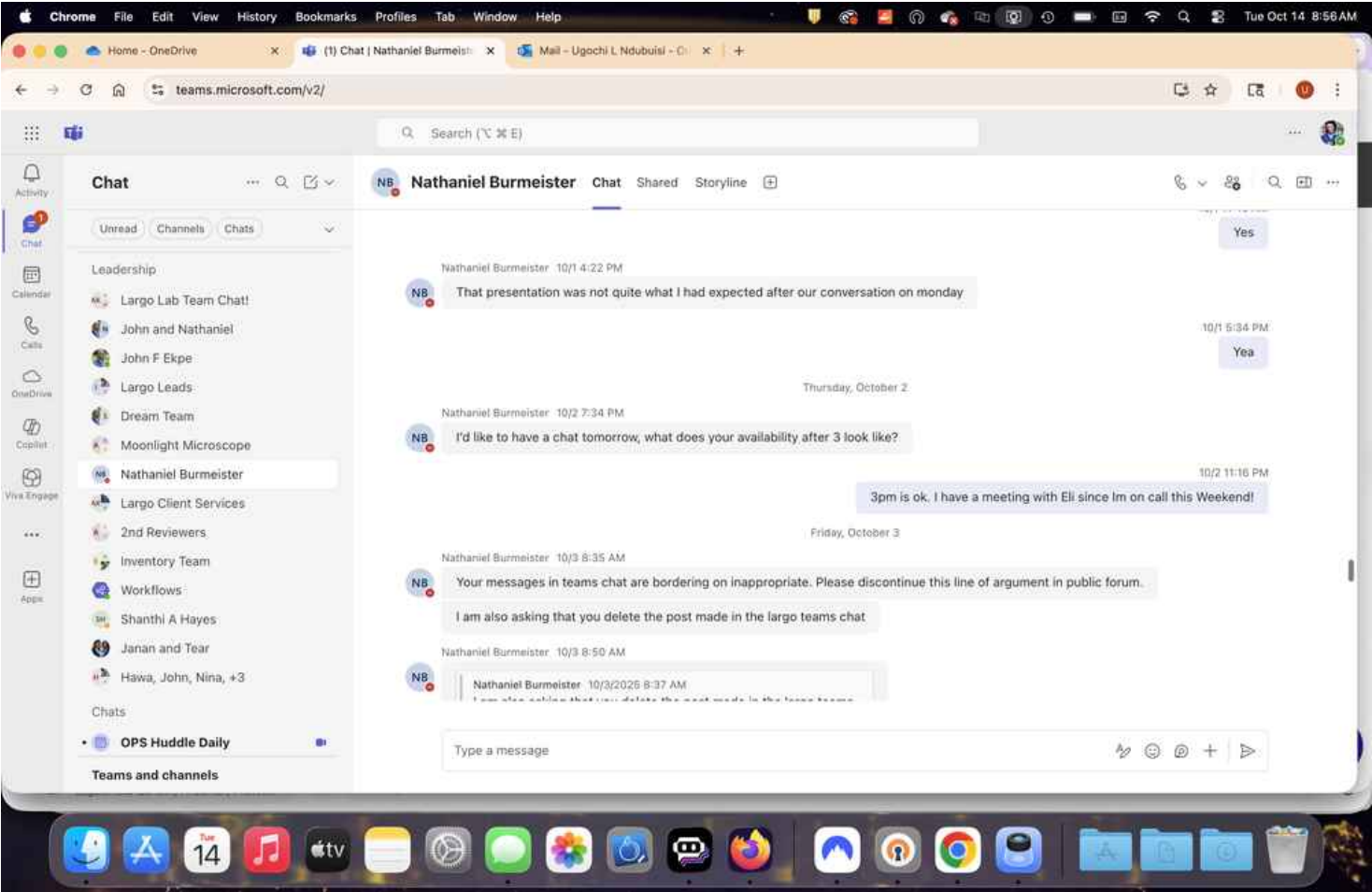


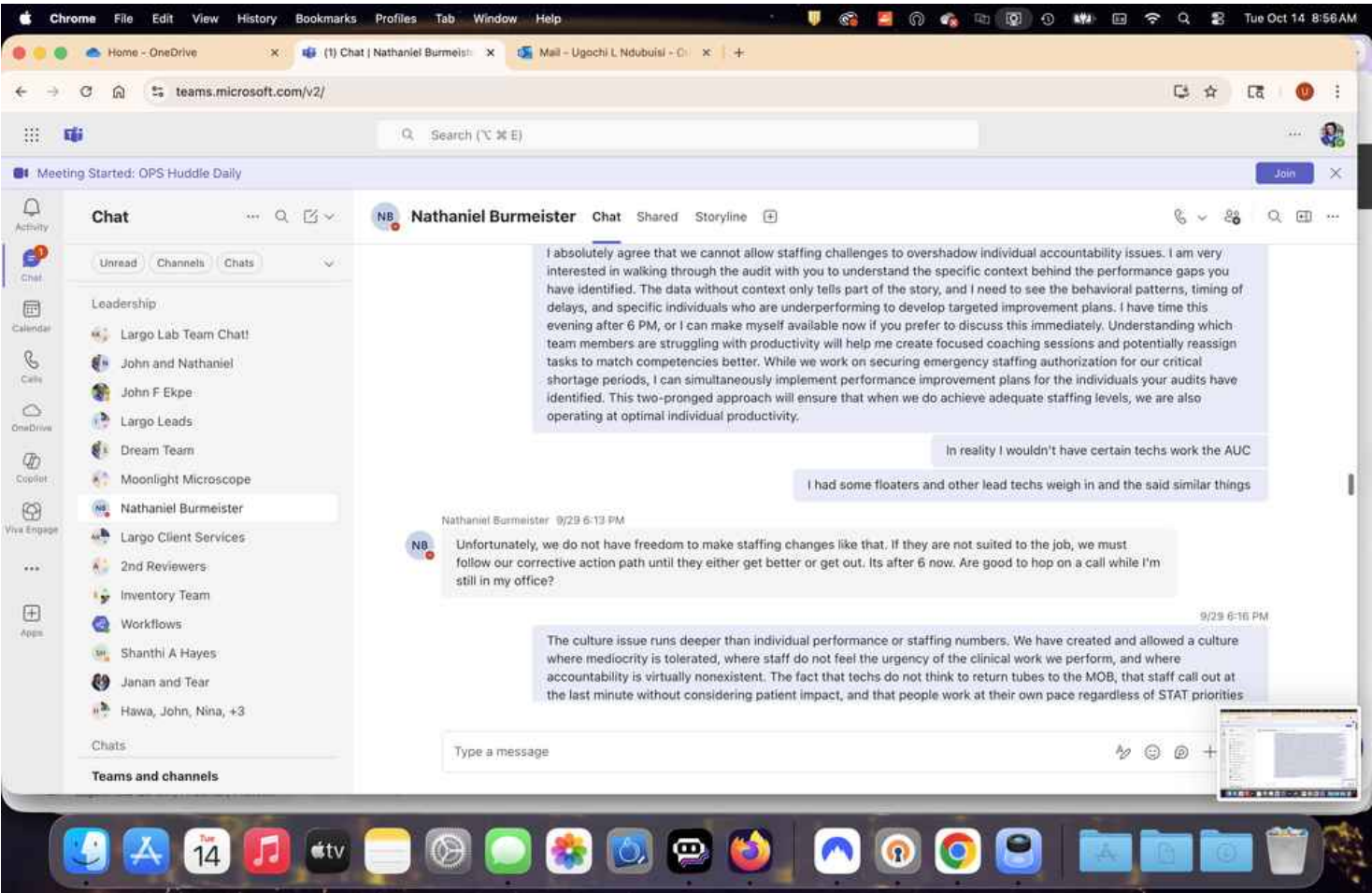
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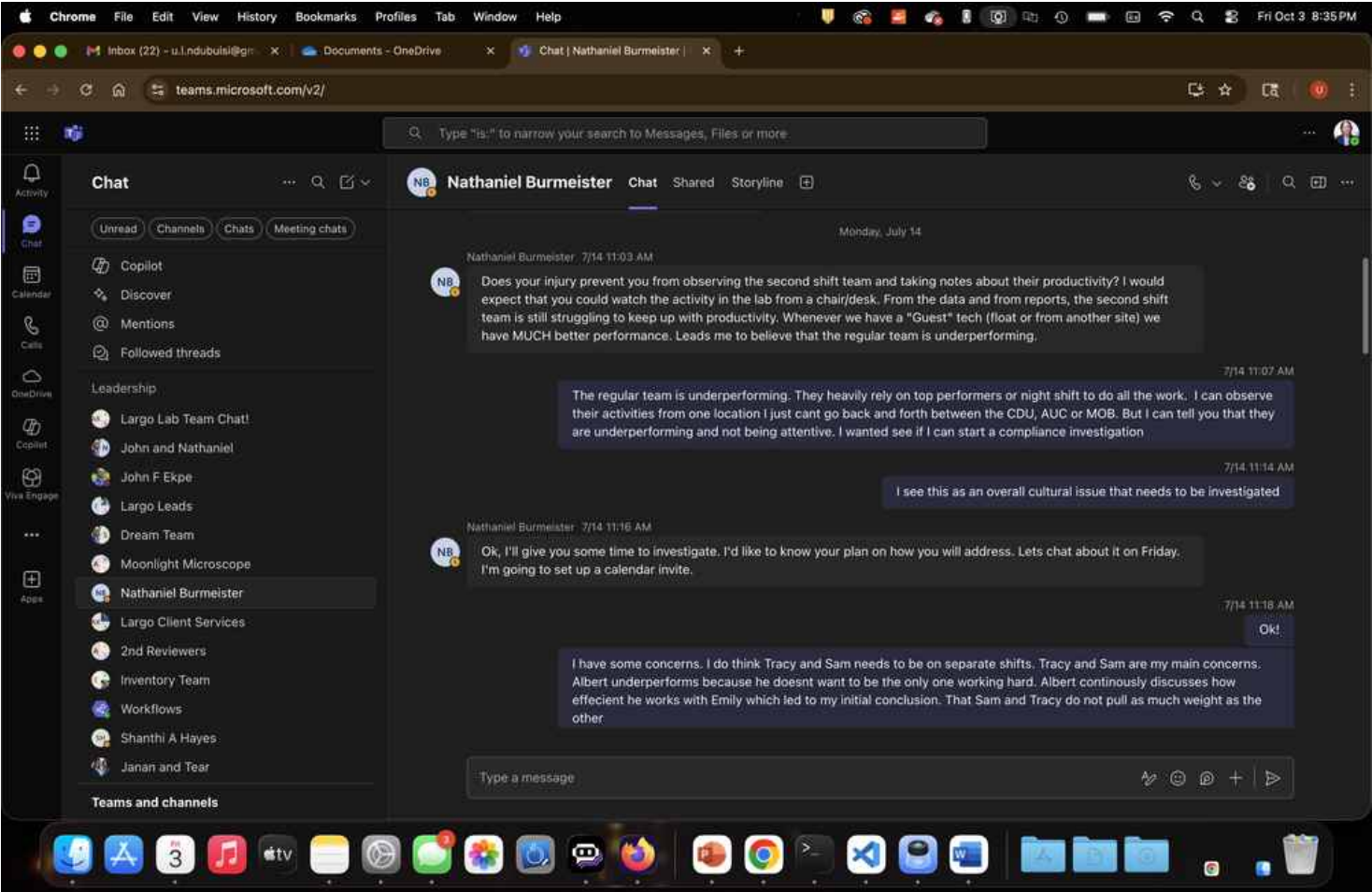
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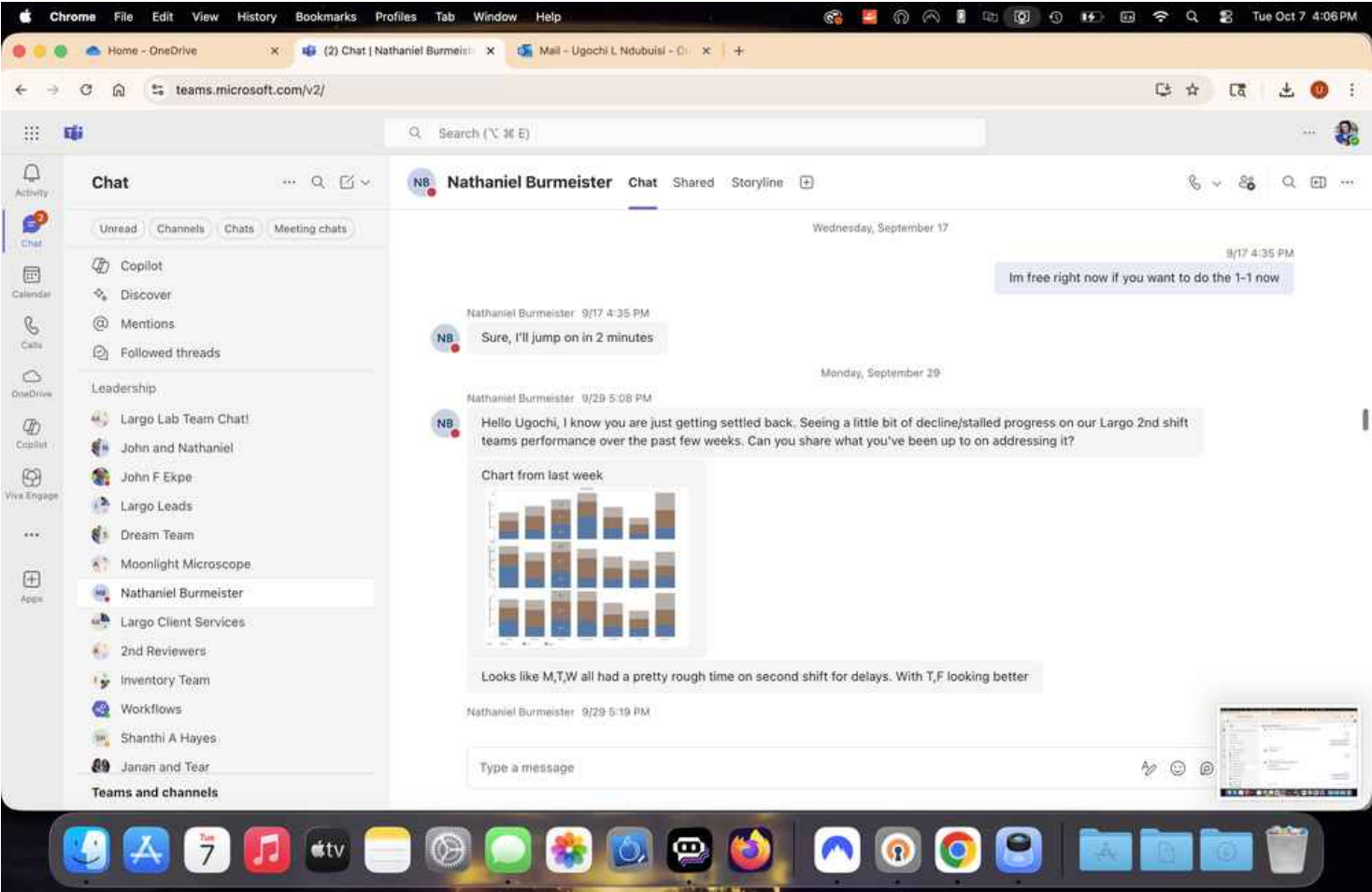
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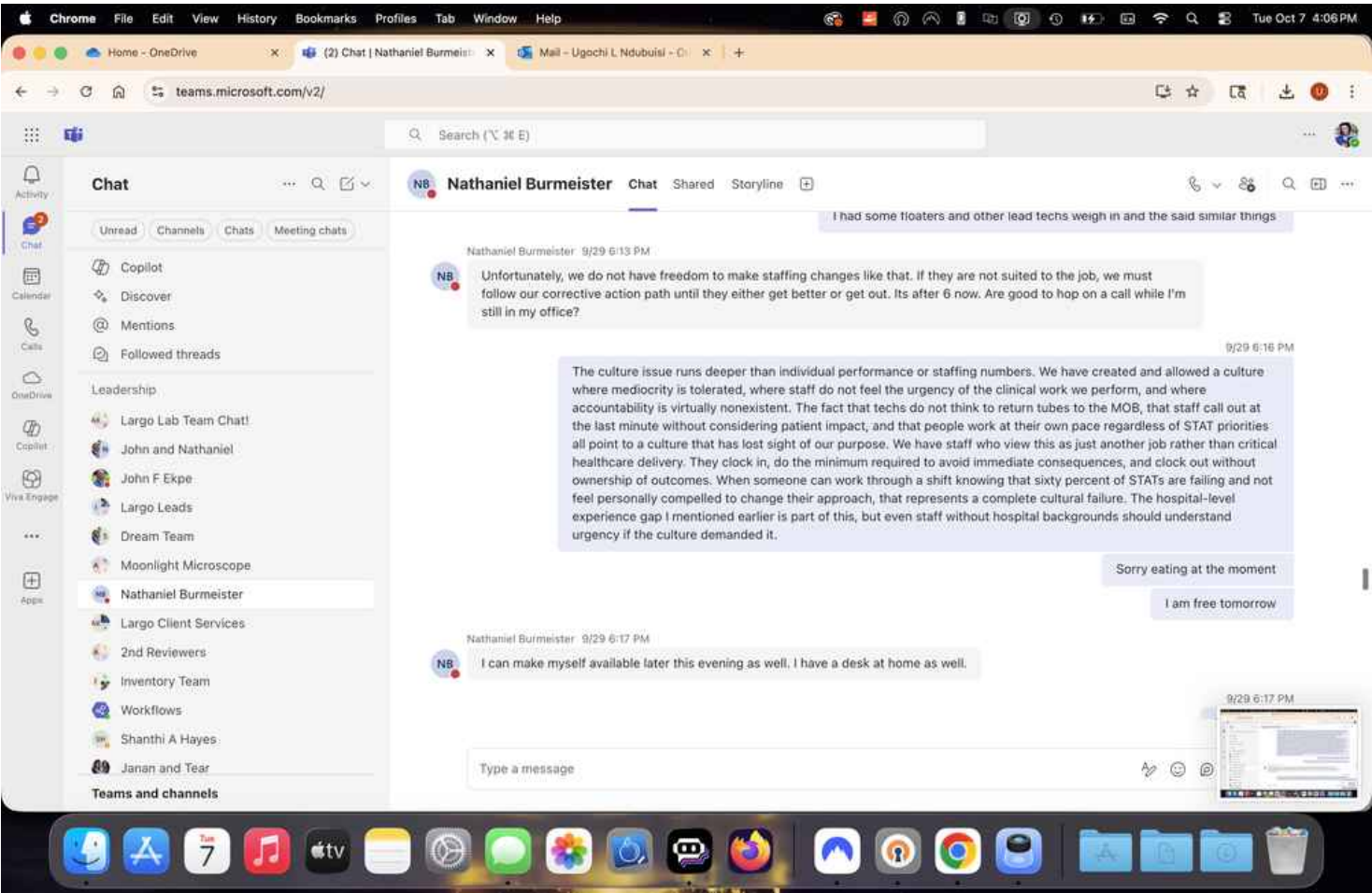
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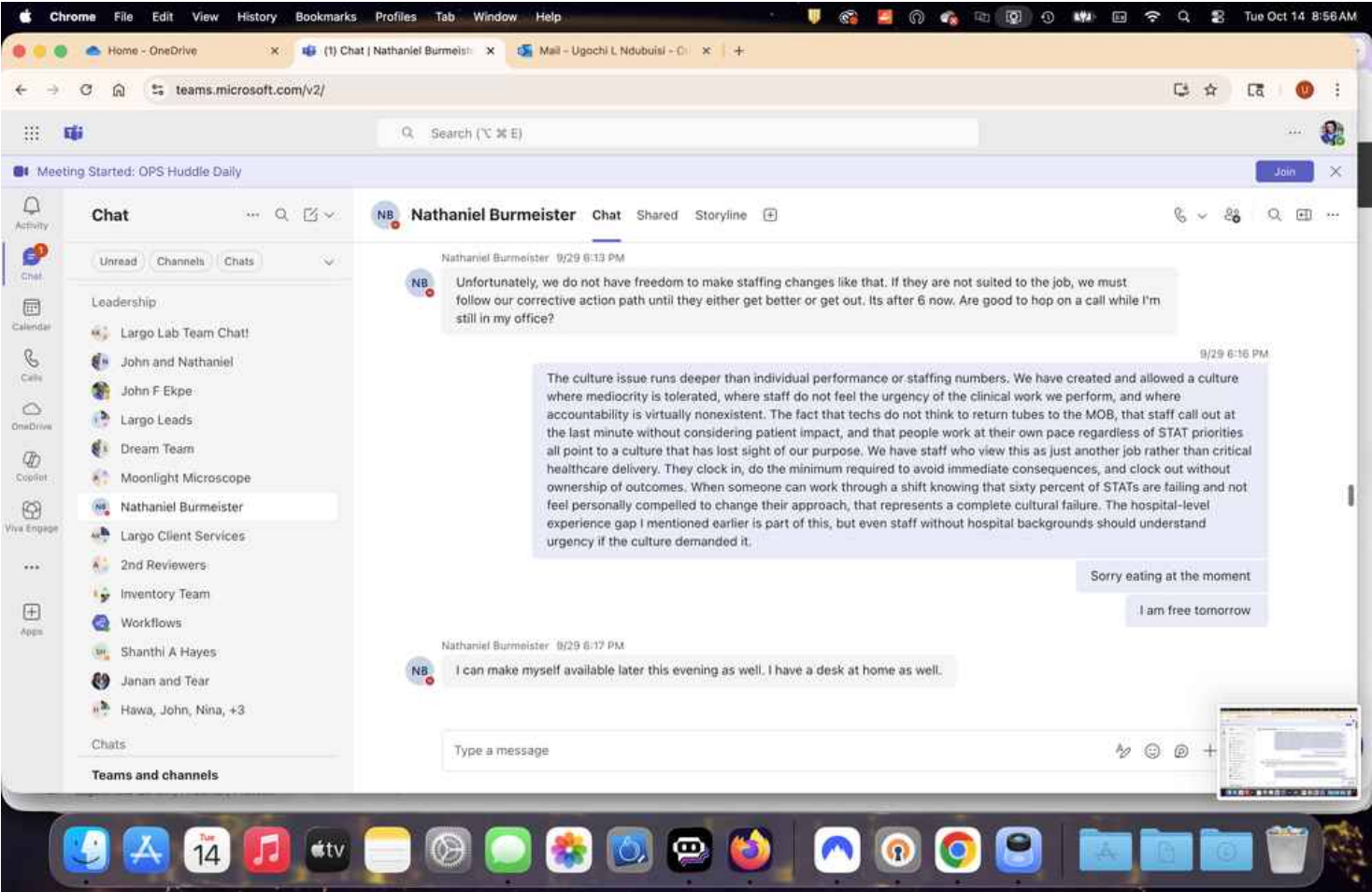
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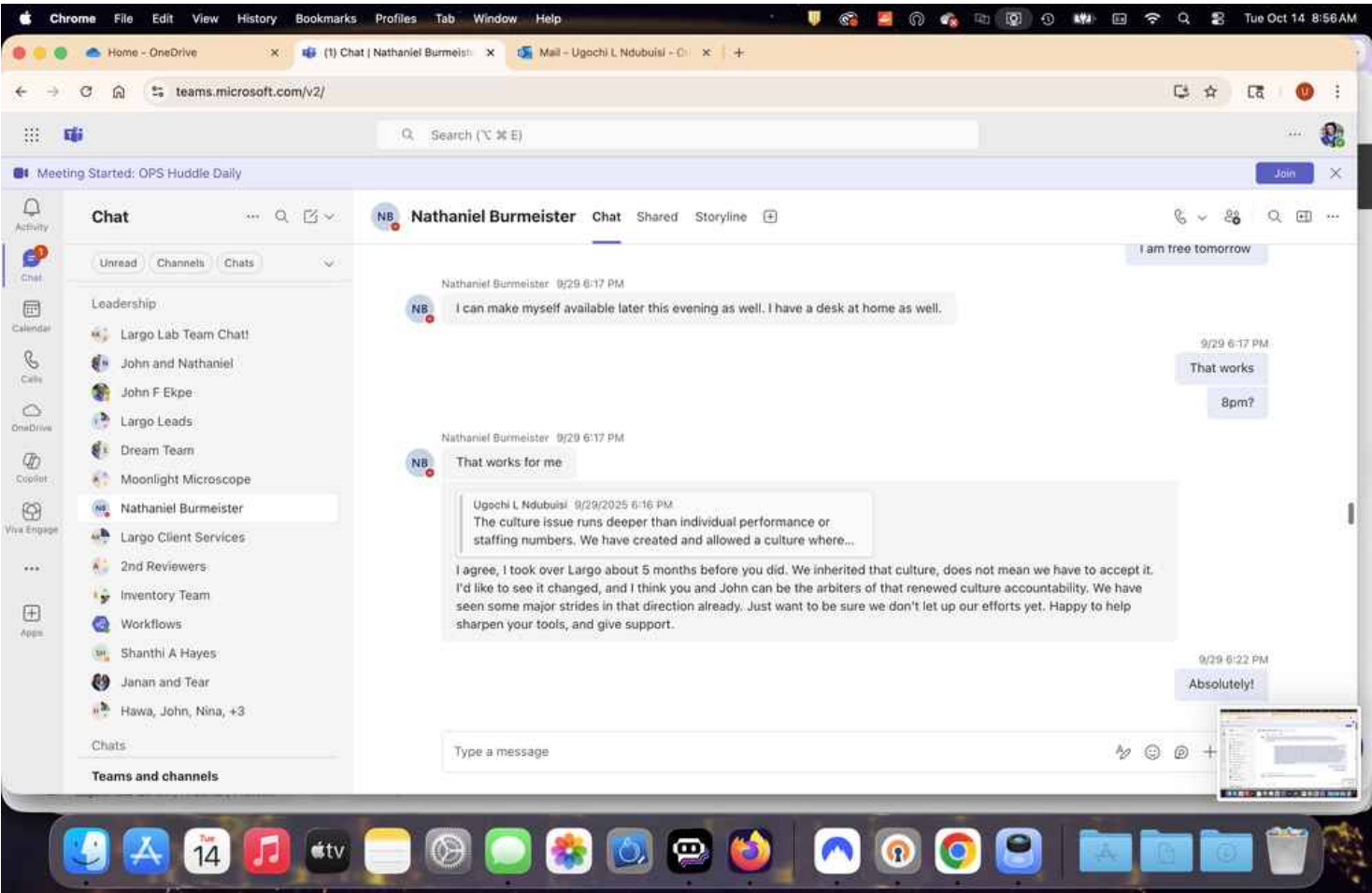


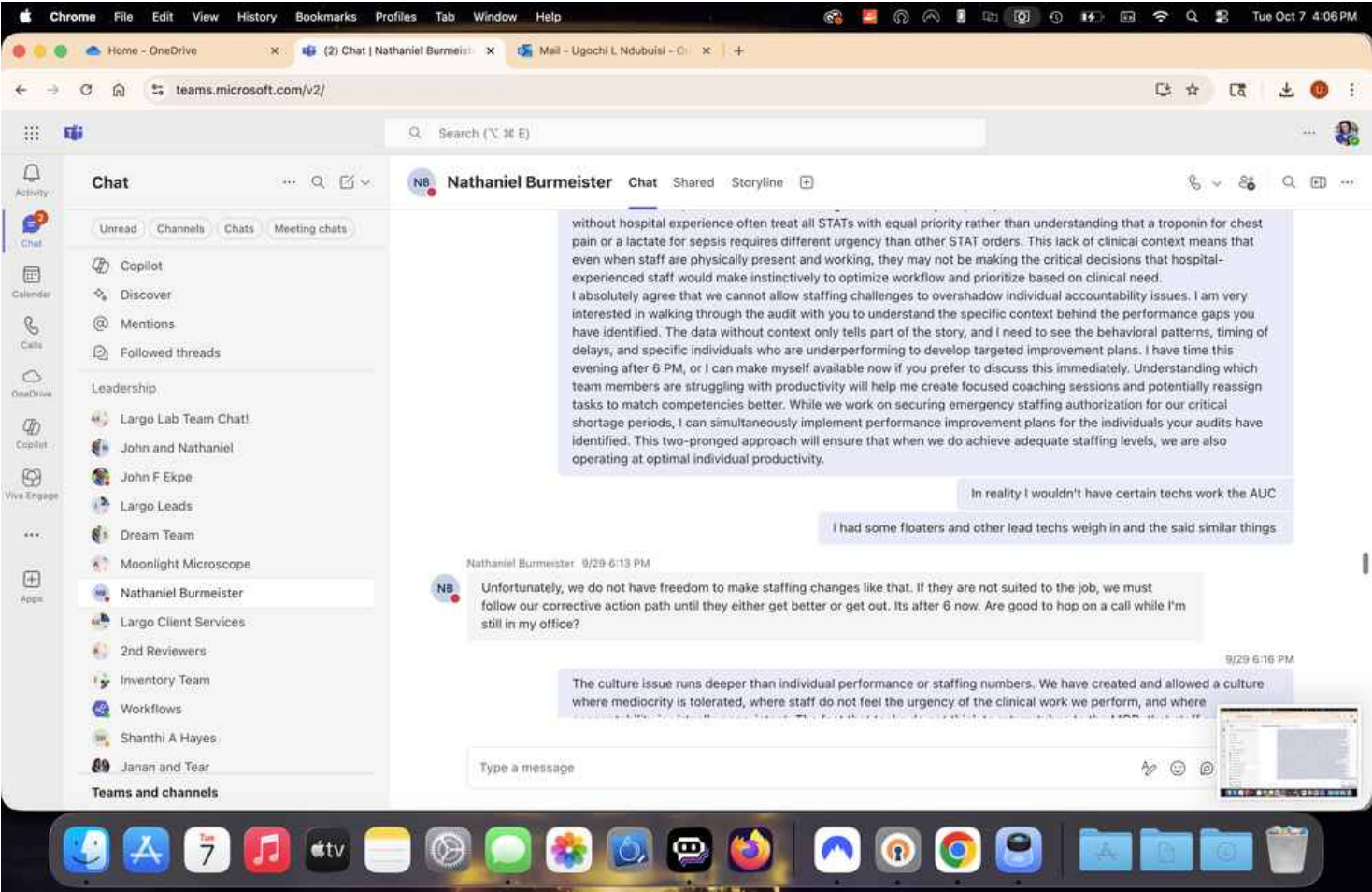


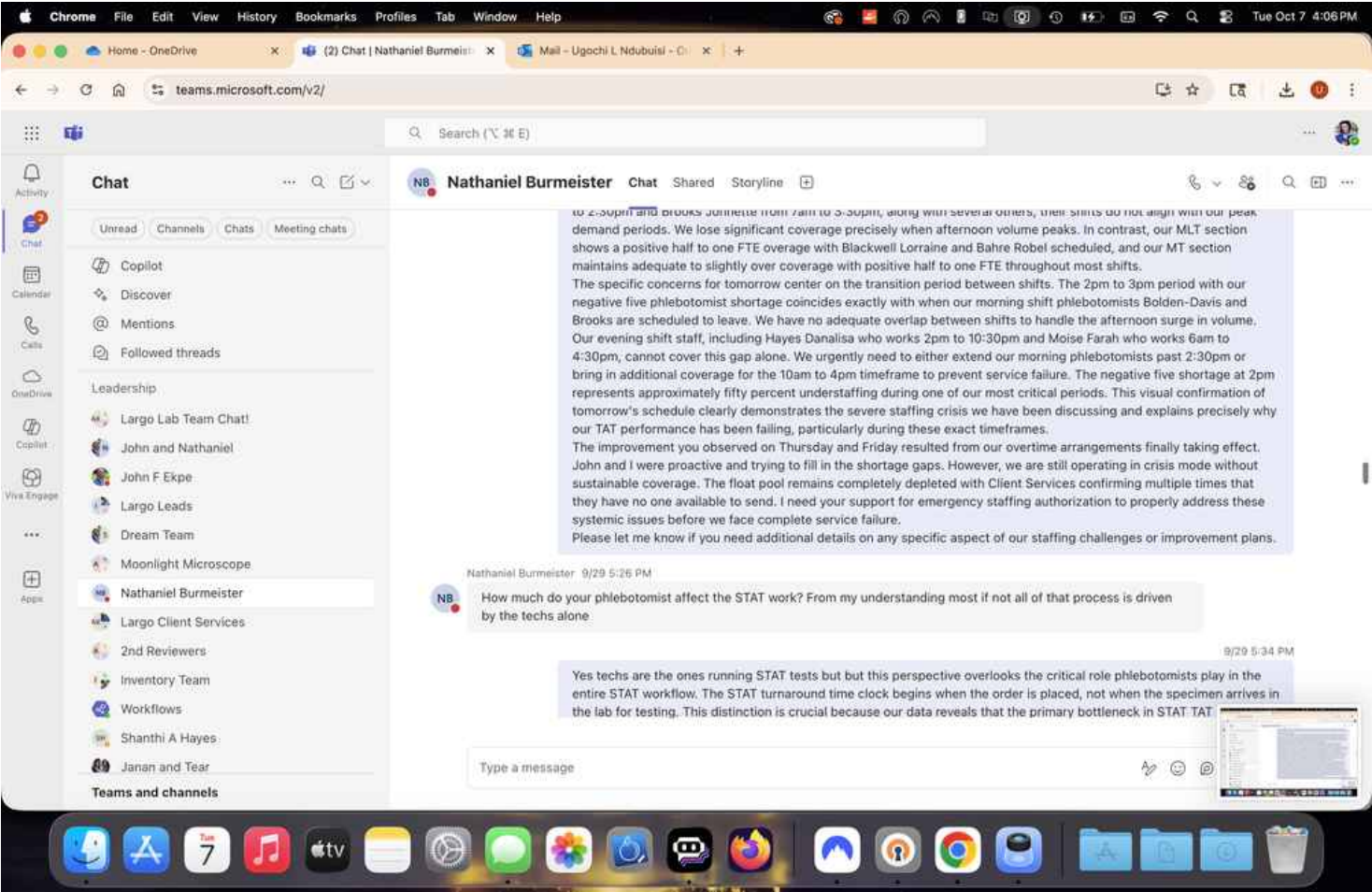


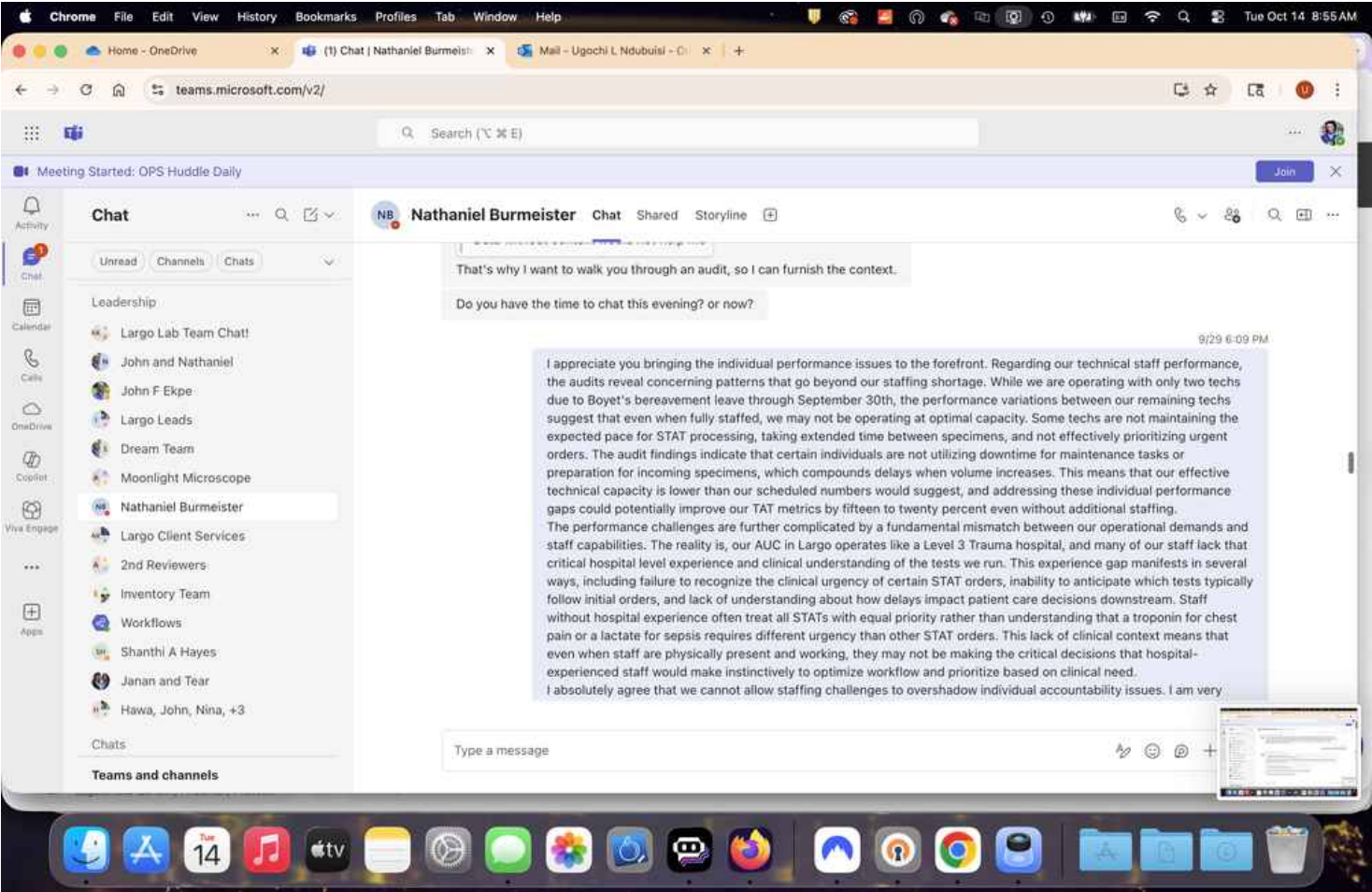
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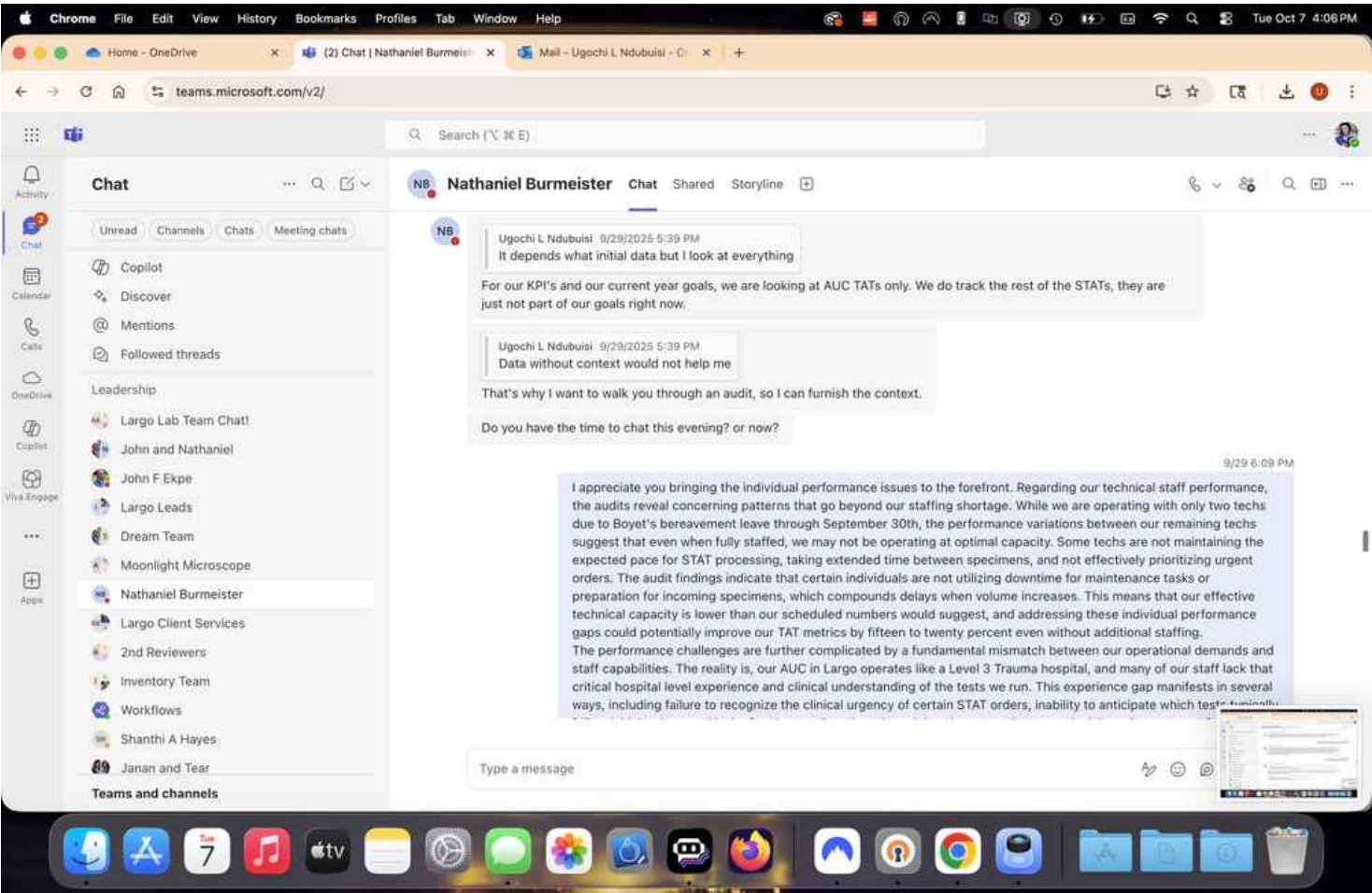


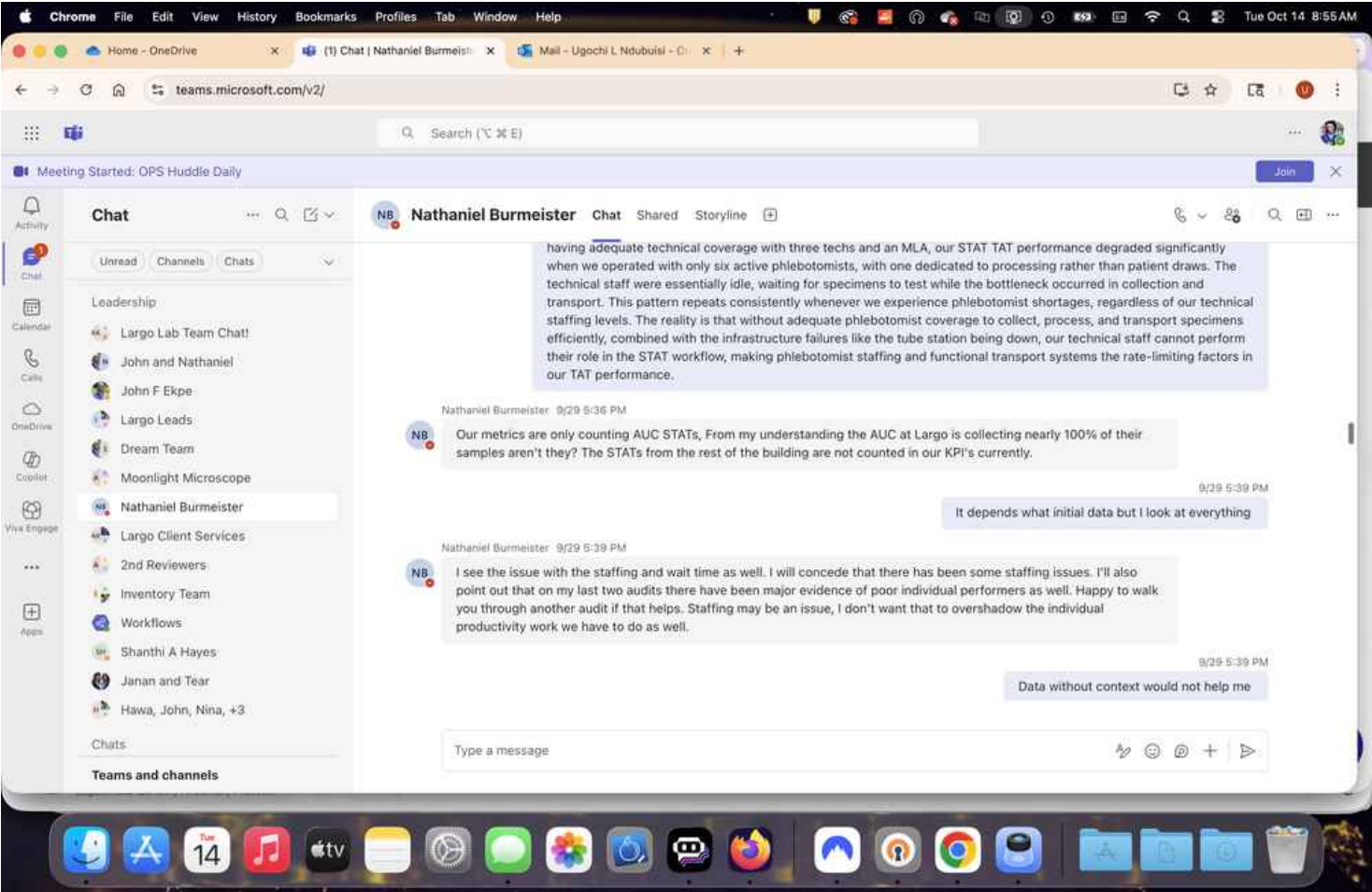


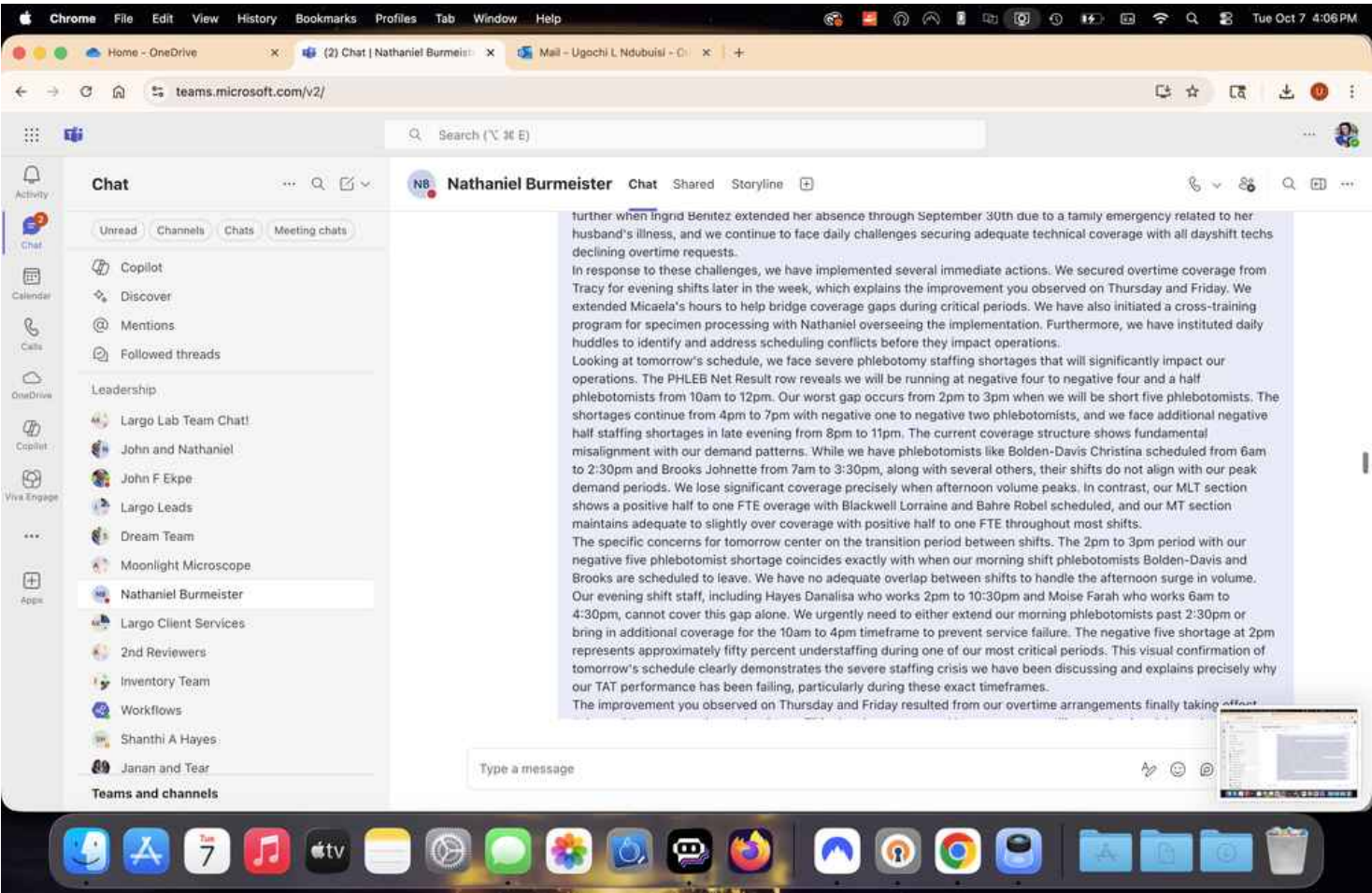


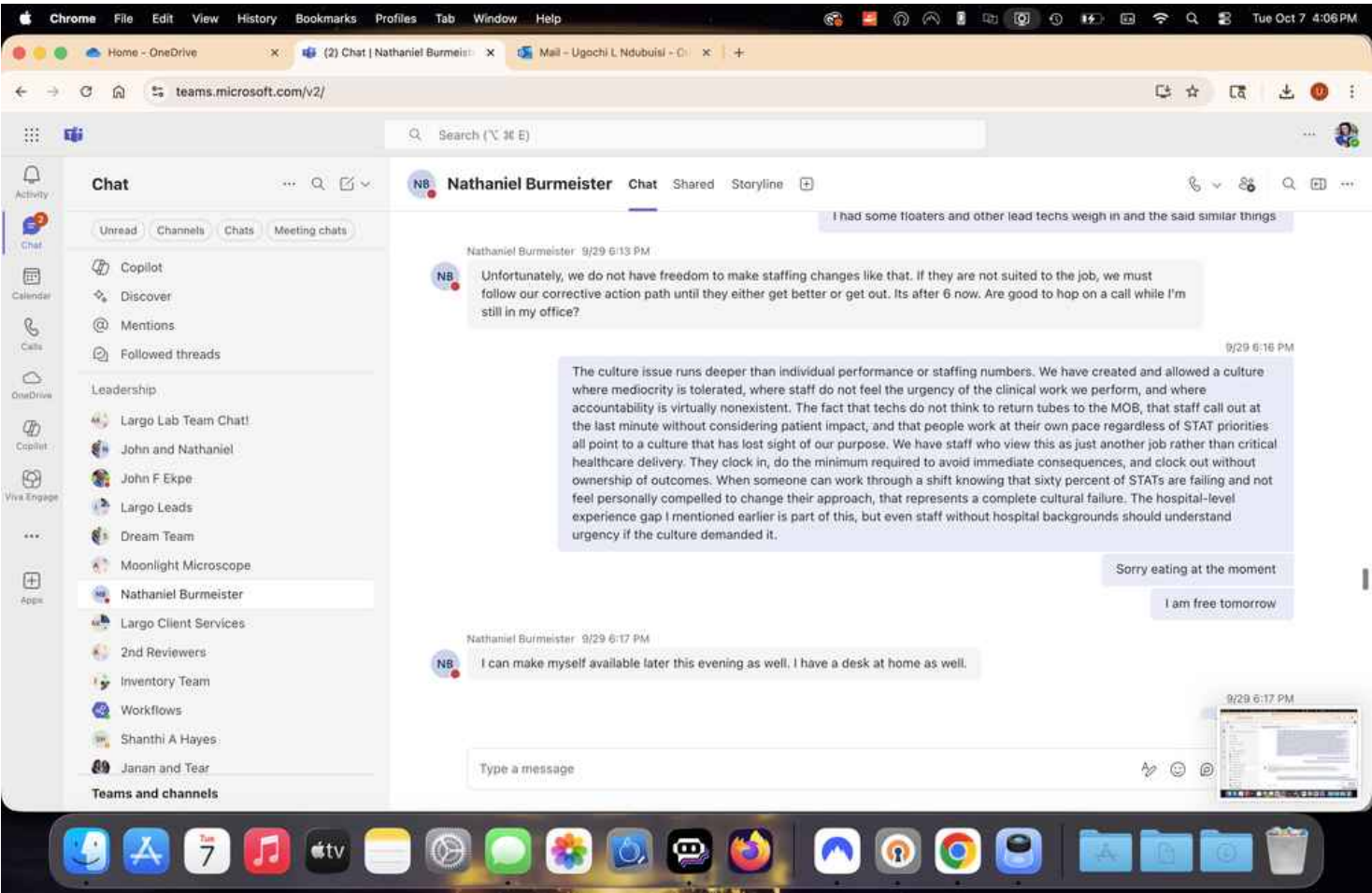




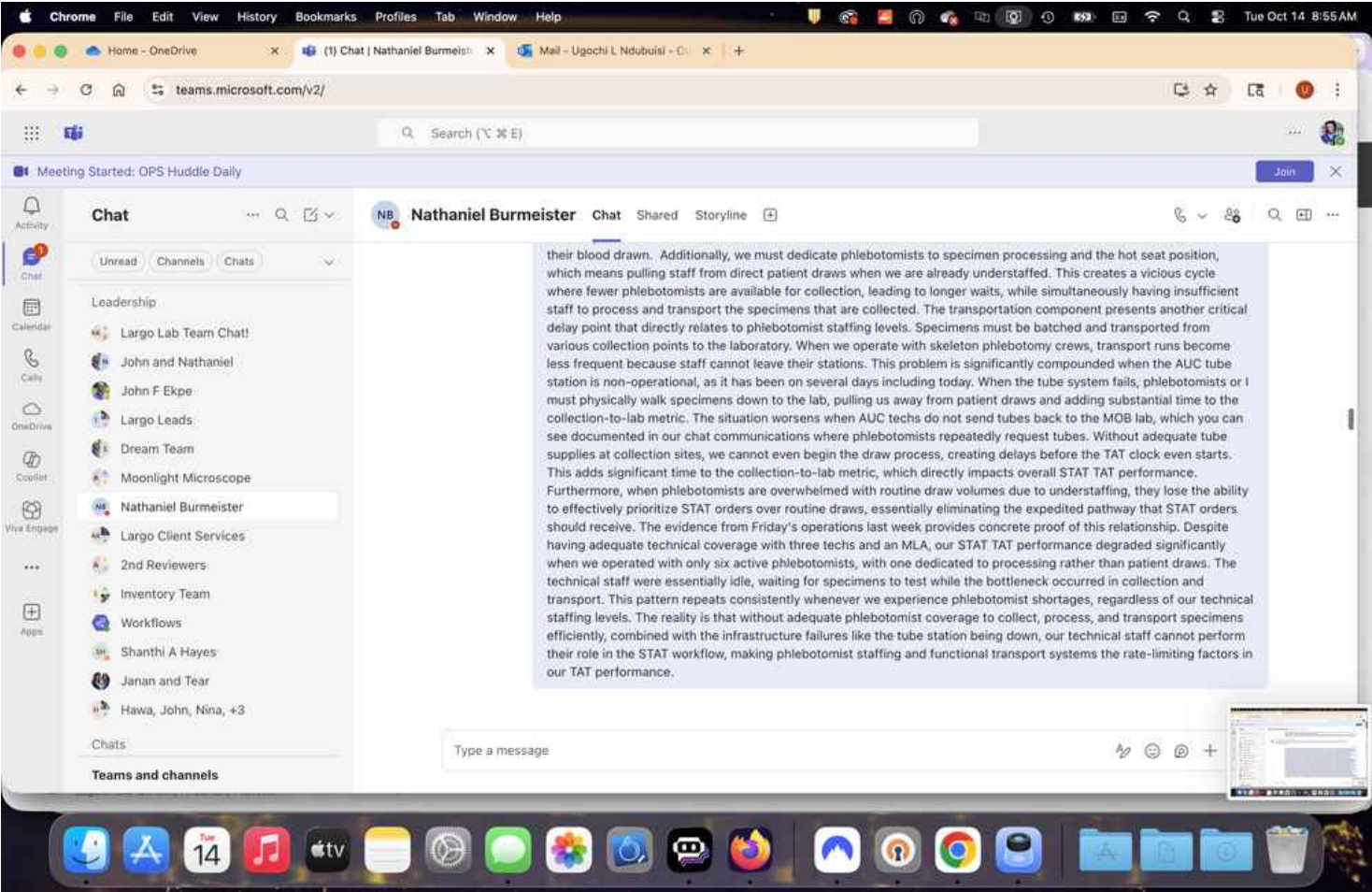




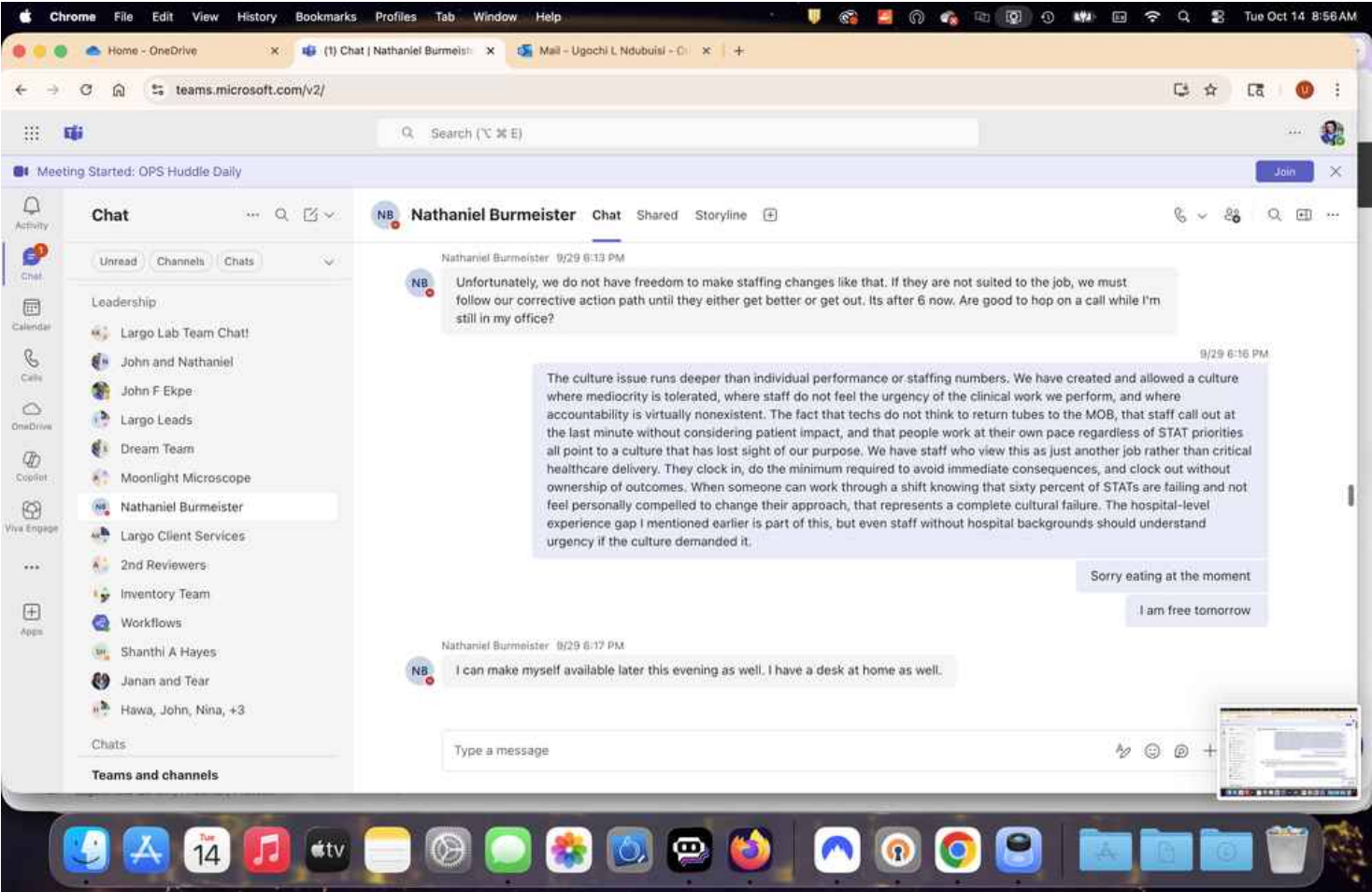


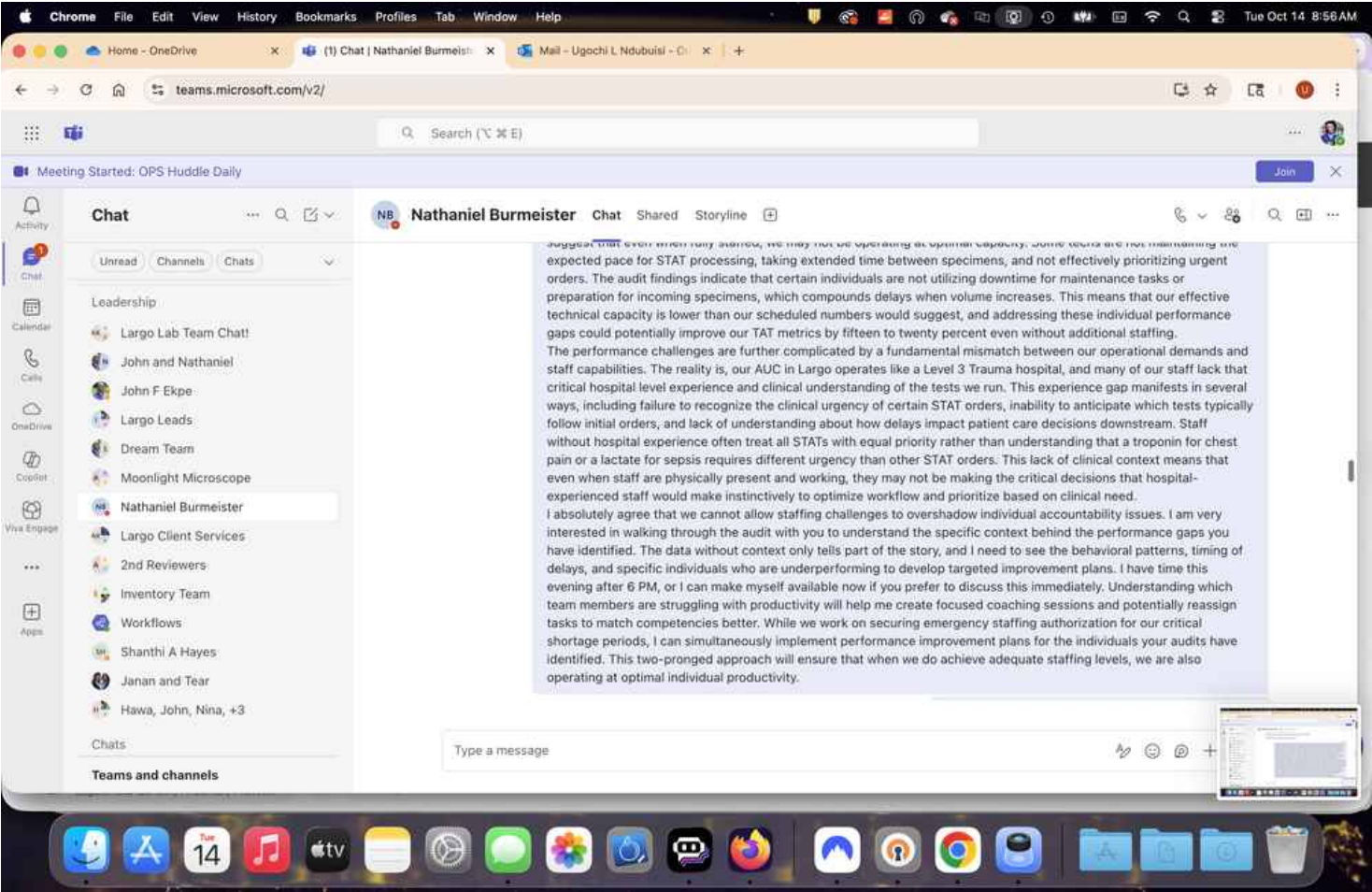


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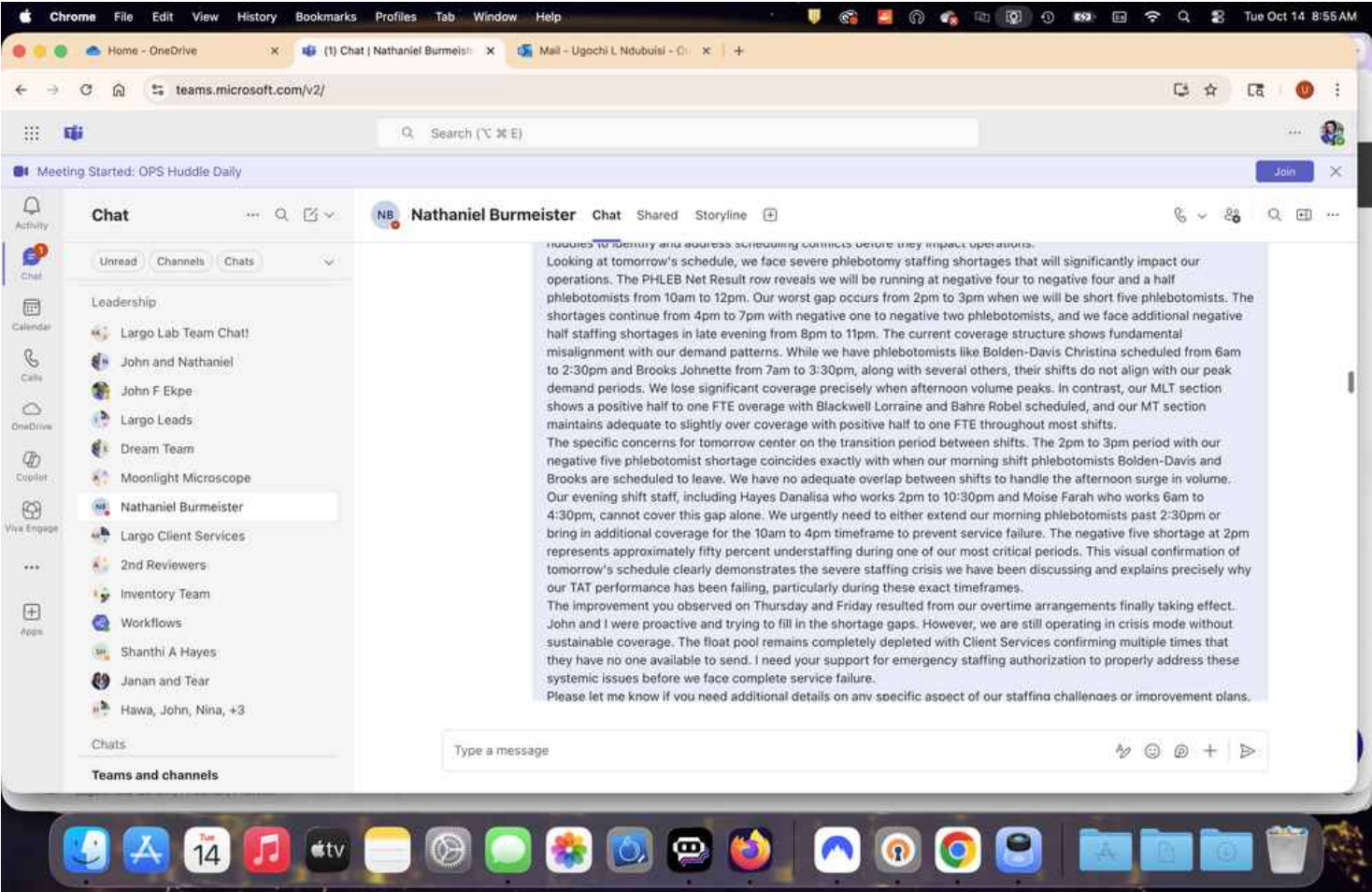


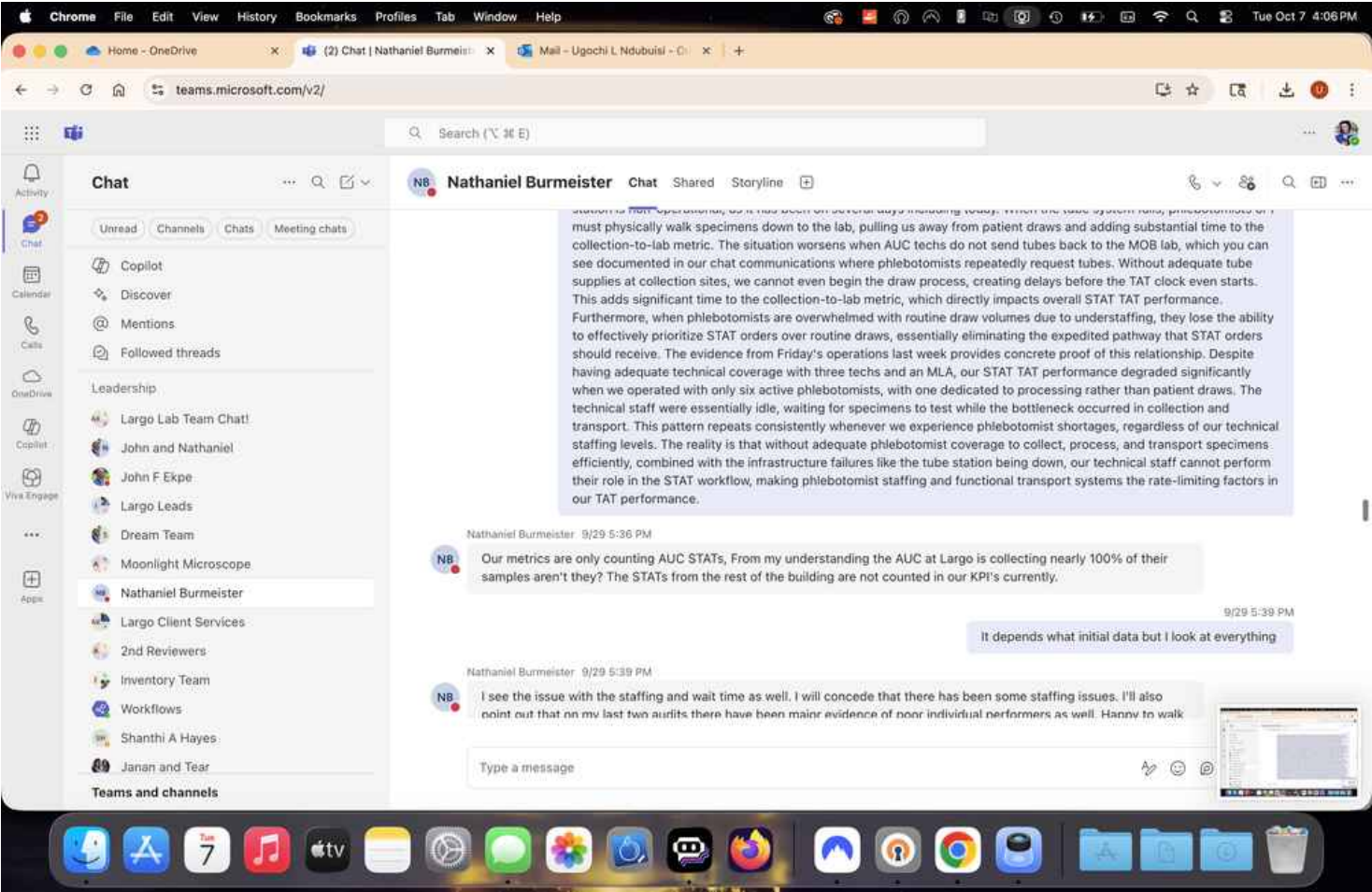
The screenshot shows a Microsoft Teams chat interface on a Mac. The top of the screen displays the macOS menu bar with various system icons and the time 'Tue Oct 14 8:55 AM'. The browser's address bar shows the URL 'teams.microsoft.com/v2/'. The chat window is titled 'Meeting Started: OPS Huddle Daily'. On the left sidebar, there's a list of channels and chats, including 'Largo Lab Team Chat!', 'John and Nathaniel', 'John F Ekpe', 'Largo Leads', 'Dream Team', 'Moonlight Microscope', 'Nathaniel Burmeister', 'Largo Client Services', '2nd Reviewers', 'Inventory Team', 'Workflows', 'Shanthi A Hayes', 'Janan and Tear', and 'Hawa, John, Nina, +3'. The main chat area shows a conversation between Nathaniel Burmeister and another user. Nathaniel's message at 9/29 5:26 PM asks about the impact of phlebotomists on STAT work. The response at 9/29 5:34 PM provides a detailed explanation of the STAT workflow, highlighting a major bottleneck in the collection-to-lab phase and the challenges posed by staffing shortages.

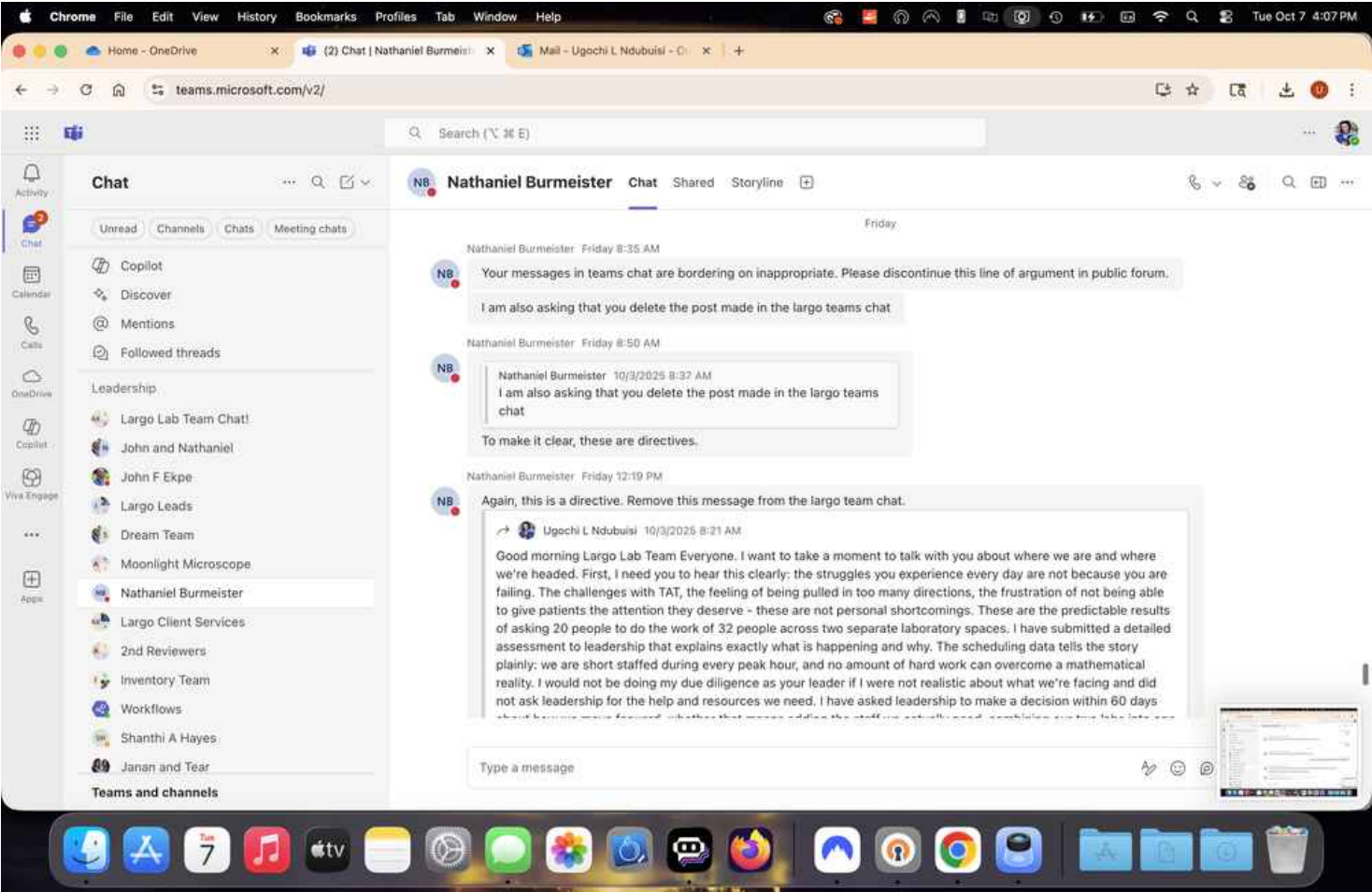
Chat History:

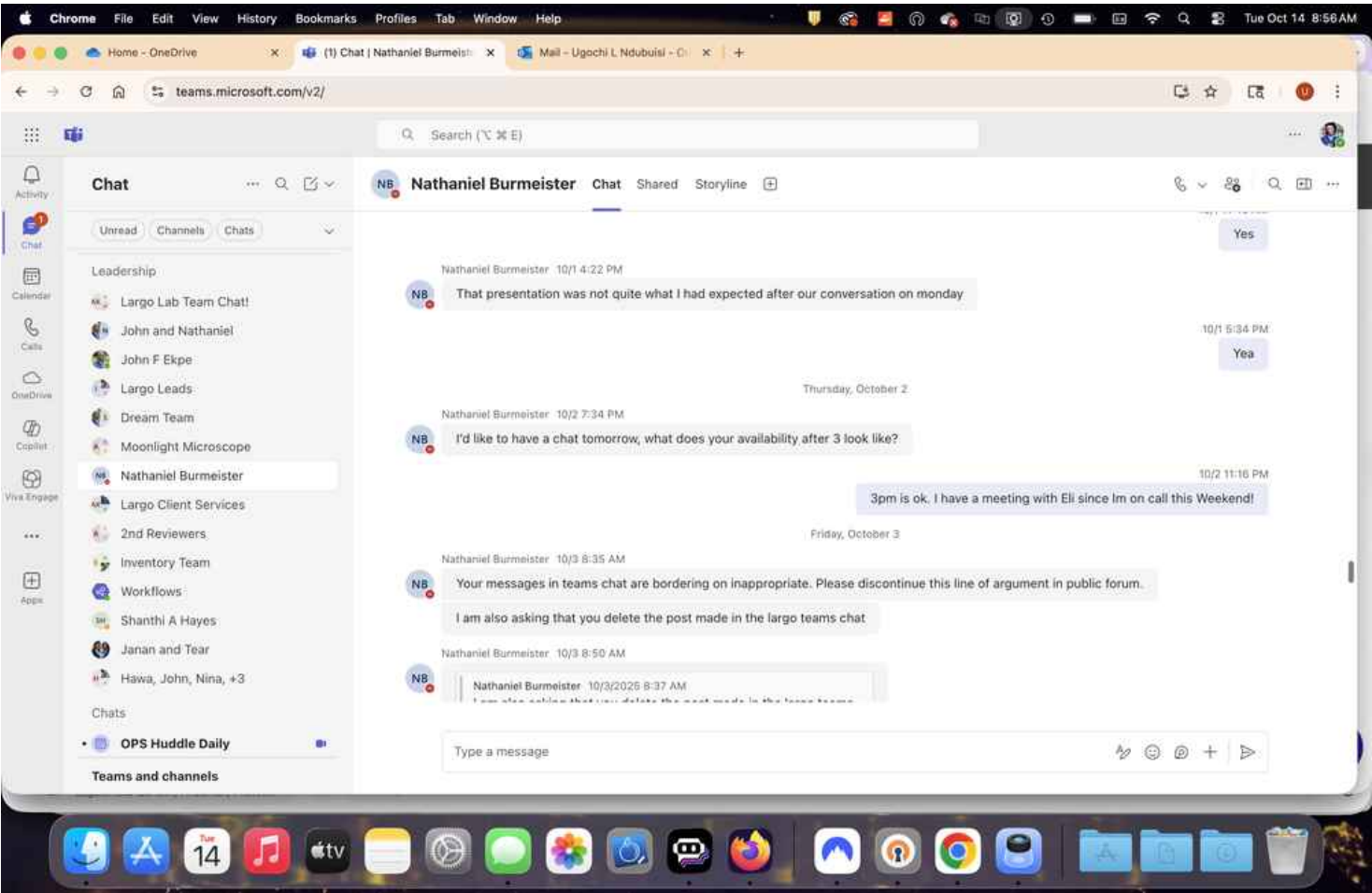
- Nathaniel Burmeister** 9/29 5:26 PM: How much do your phlebotomist affect the STAT work? From my understanding most if not all of that process is driven by the techs alone
- [User]** 9/29 5:34 PM: Yes techs are the ones running STAT tests but but this perspective overlooks the critical role phlebotomists play in the entire STAT workflow. The STAT turnaround time clock begins when the order is placed, not when the specimen arrives in the lab for testing. This distinction is crucial because our data reveals that the primary bottleneck in STAT TAT performance occurs during the collection-to-lab phase, which is entirely dependent on phlebotomist availability and efficiency. The data from last week demonstrates this clearly. During the 8am hour, which shows our worst TAT performance, the collection-to-lab time averaged an astronomical 3,489 minutes, while the in-lab-to-result time remained relatively stable at 142 minutes. This massive discrepancy indicates that samples are experiencing significant delays before they ever reach the techs who would perform the actual testing. Even with adequate technical staff ready to process STAT orders, they cannot test specimens that have not been collected and delivered to the lab. When we experience phlebotomist shortages like the negative five staffing gap at 2pm tomorrow, multiple cascading failures occur throughout the STAT workflow. Each phlebotomist can realistically handle only four to six patients per hour when following proper collection procedures. With half the required staff, patients experience extended wait times just to have their blood drawn. Additionally, we must dedicate phlebotomists to specimen processing and the hot seat position, which means pulling staff from direct patient draws when we are already understaffed. This creates a vicious cycle where fewer phlebotomists are available for collection, leading to longer waits, while simultaneously having insufficient staff to process and transport the specimens that are collected. The transportation component presents another critical

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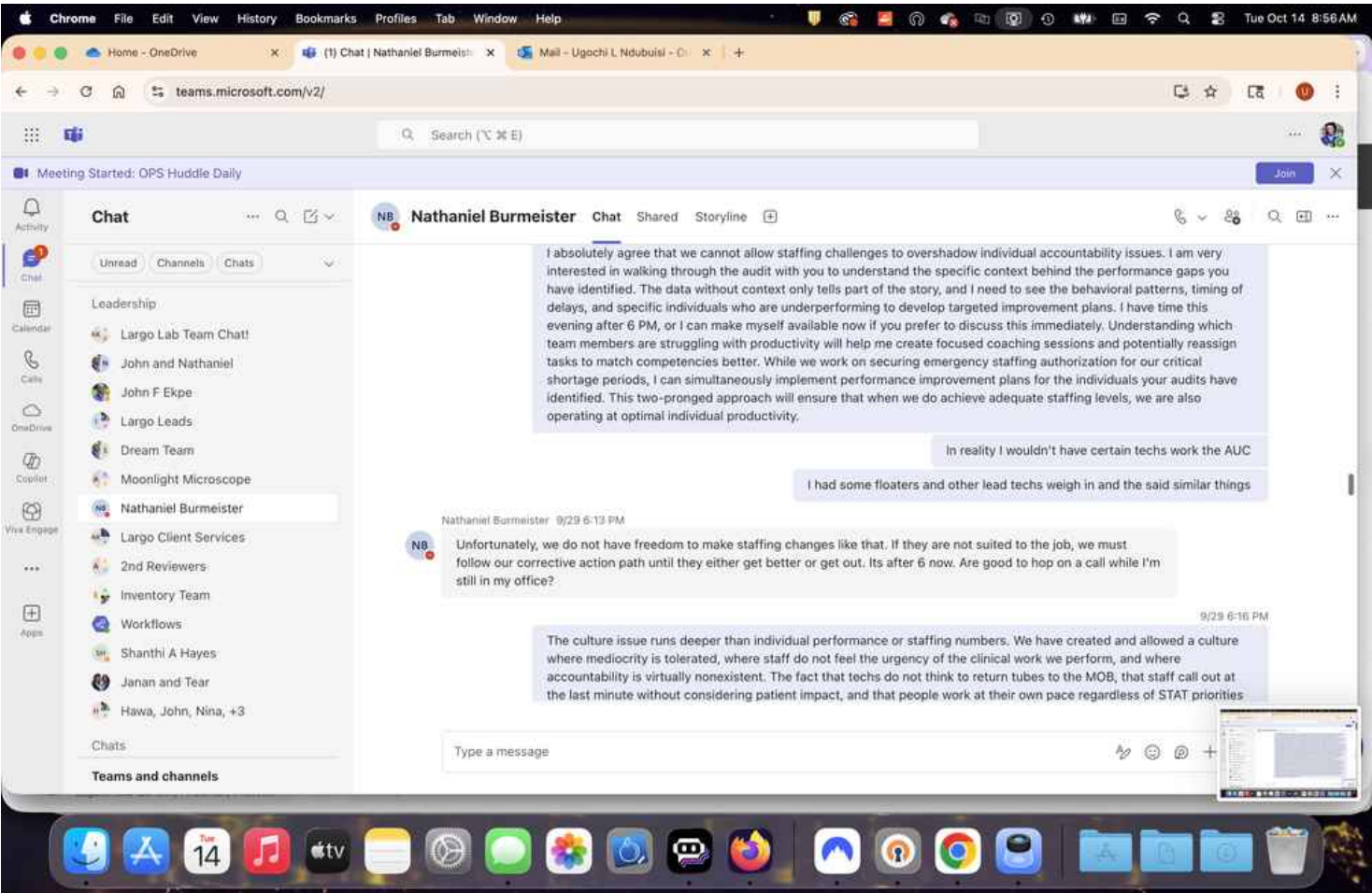


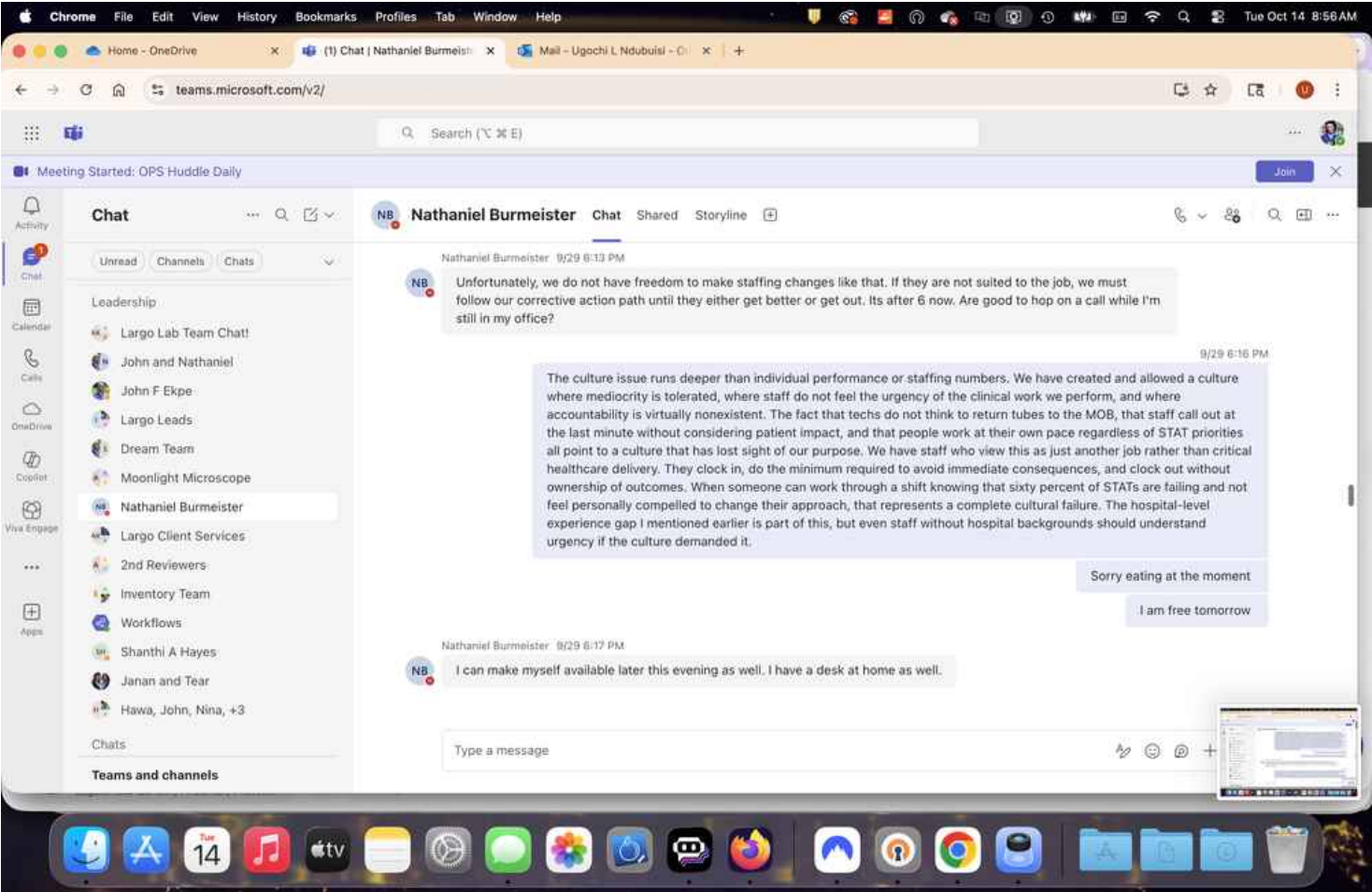






Evidence 21: Oct_8_Final_Warning_Screenshot 2025-10-14 at 8.56.05AM





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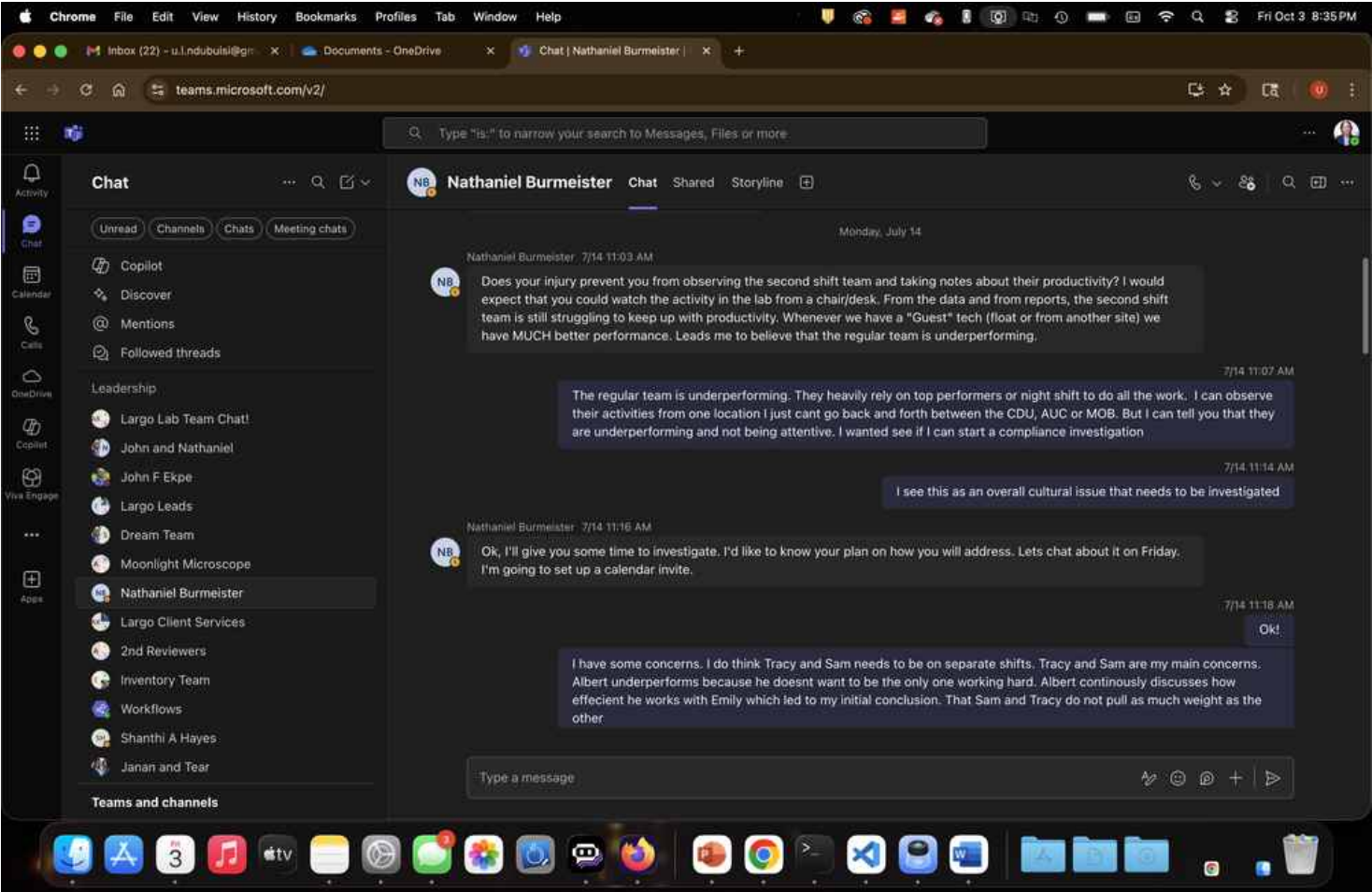
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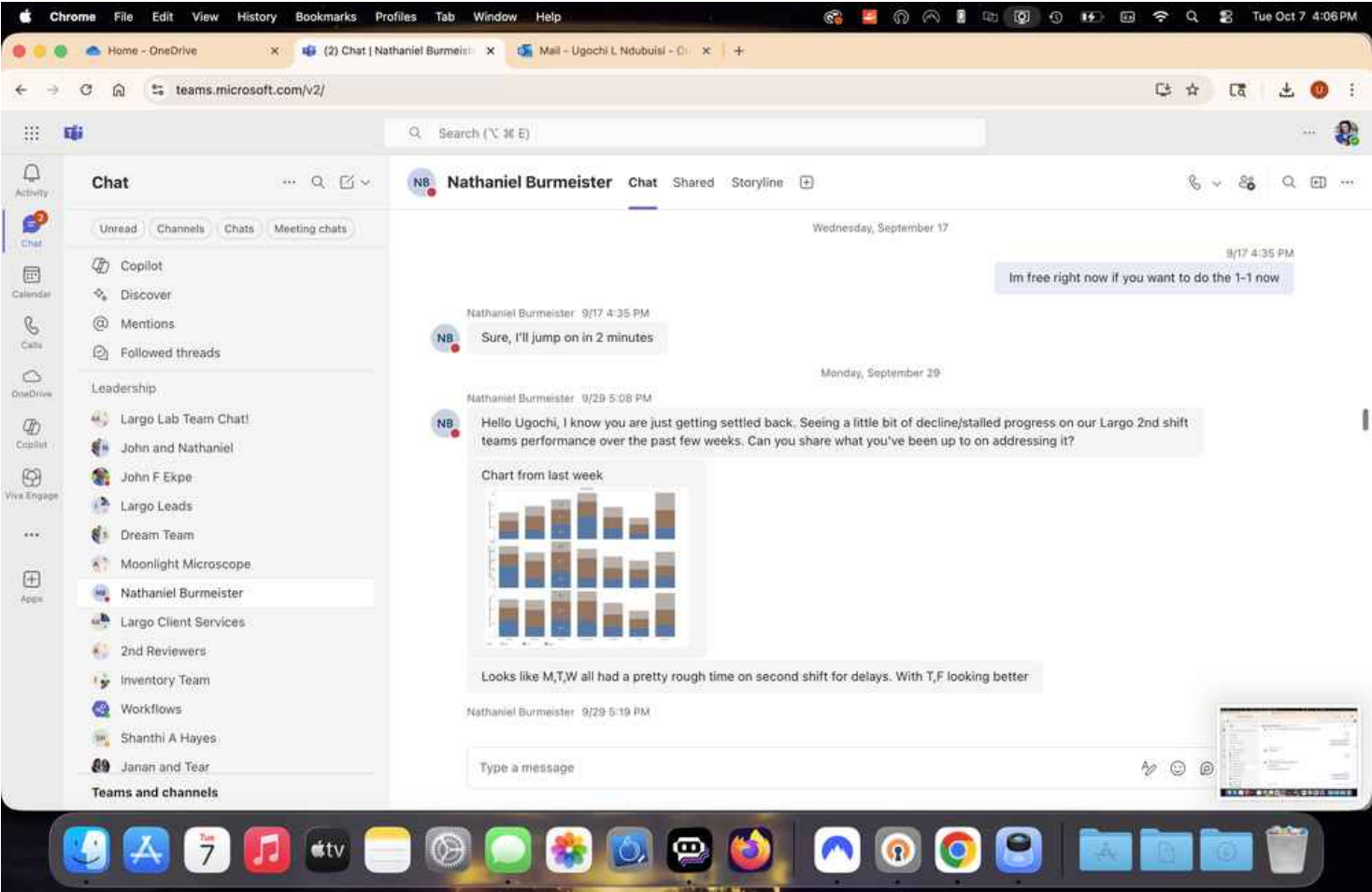
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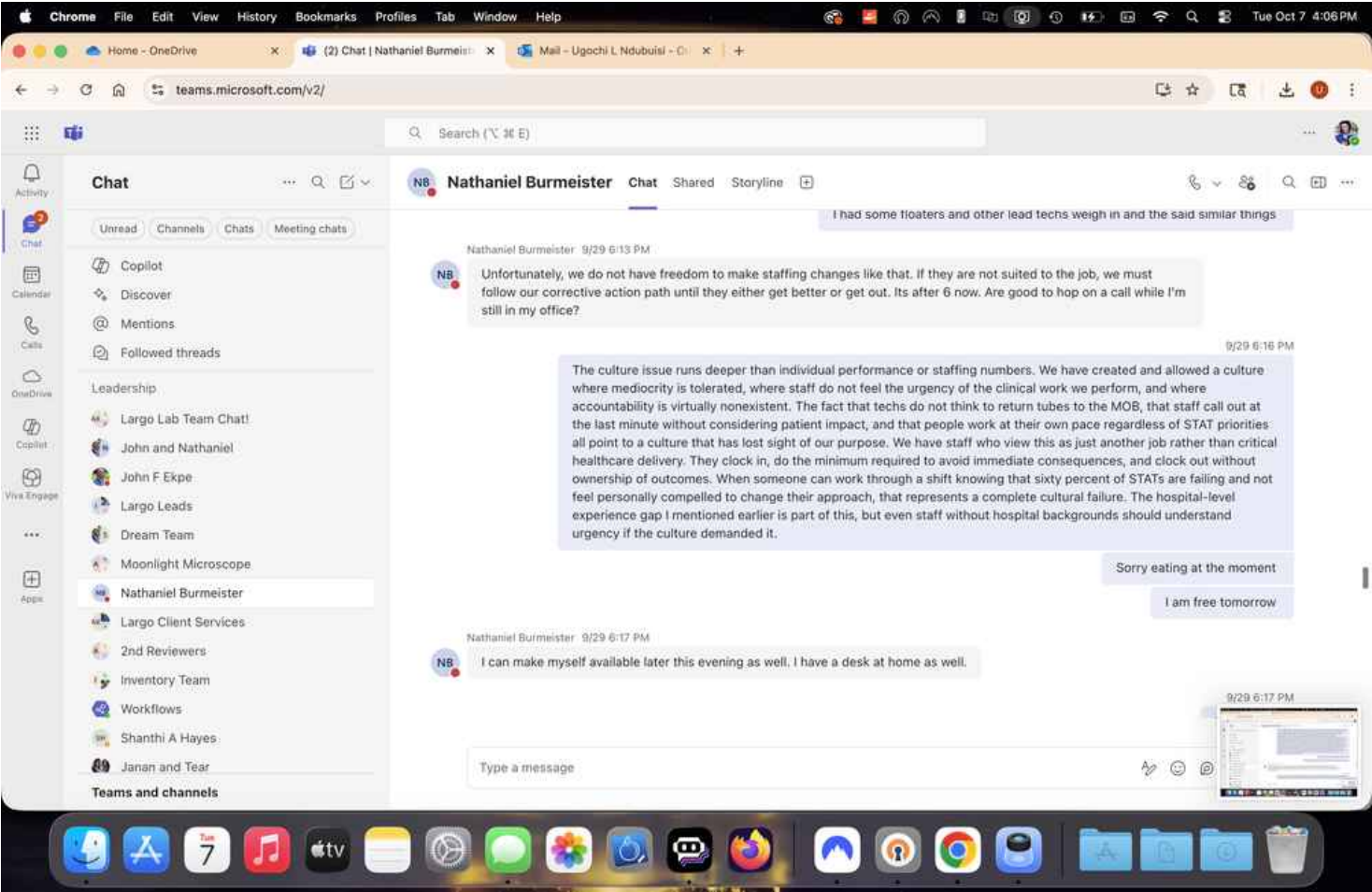
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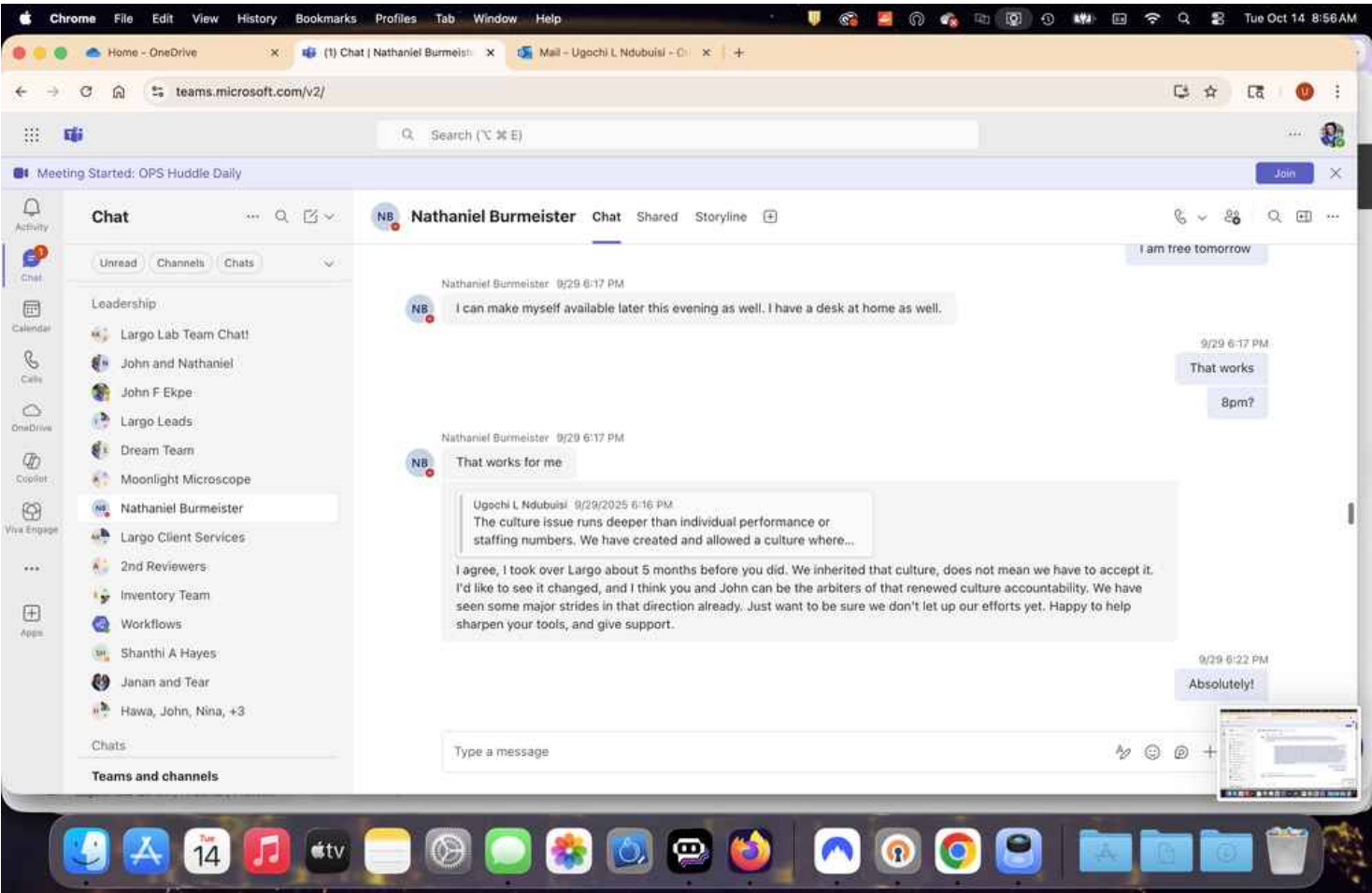


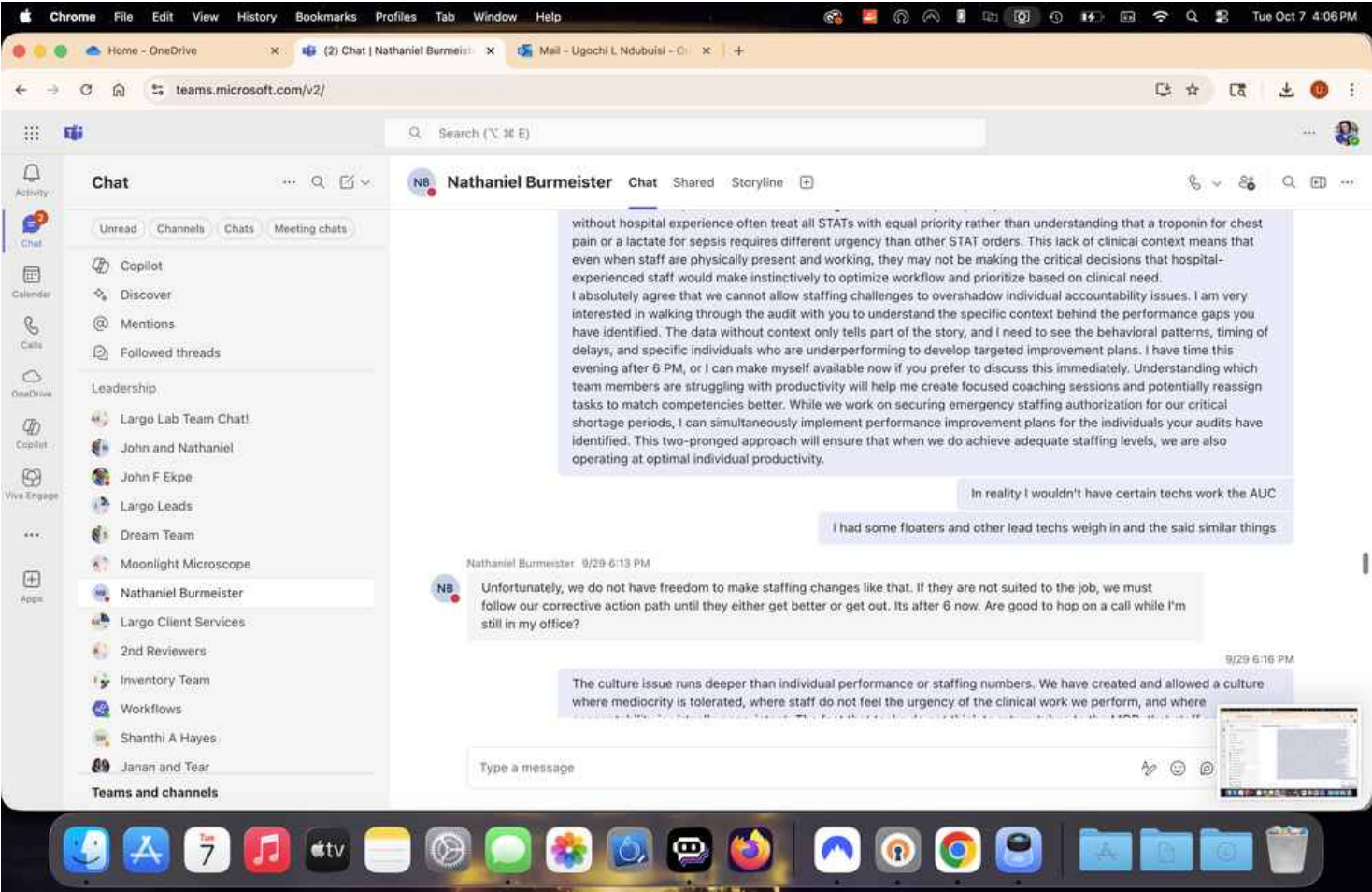


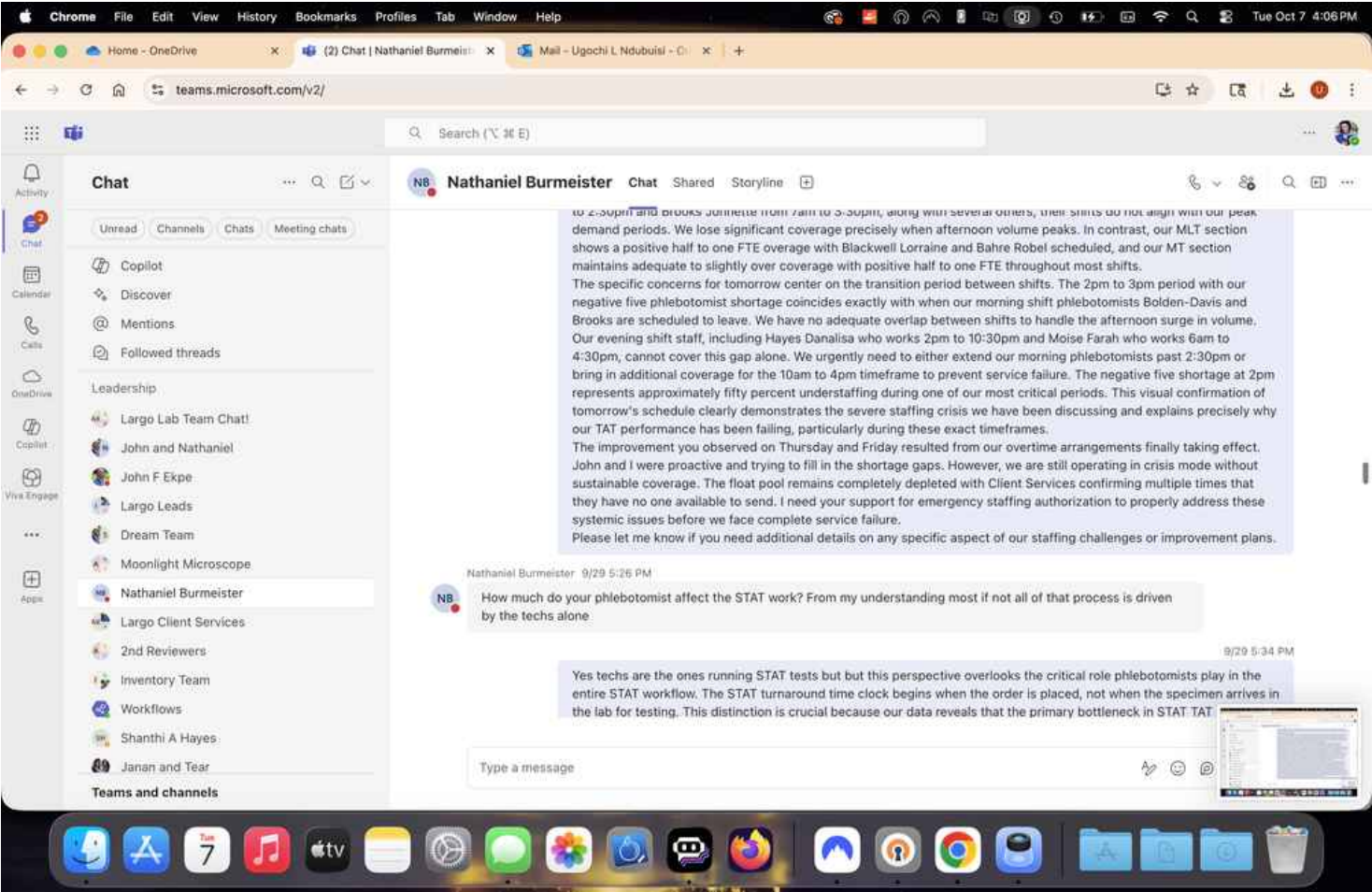
The screenshot shows a Microsoft Teams chat window on a Mac. The browser address bar at the top indicates the URL is teams.microsoft.com/v2/. The chat is titled "Meeting Started: OPS Huddle Daily". The chat history shows a conversation with Nathaniel Burmeister. The messages are as follows:

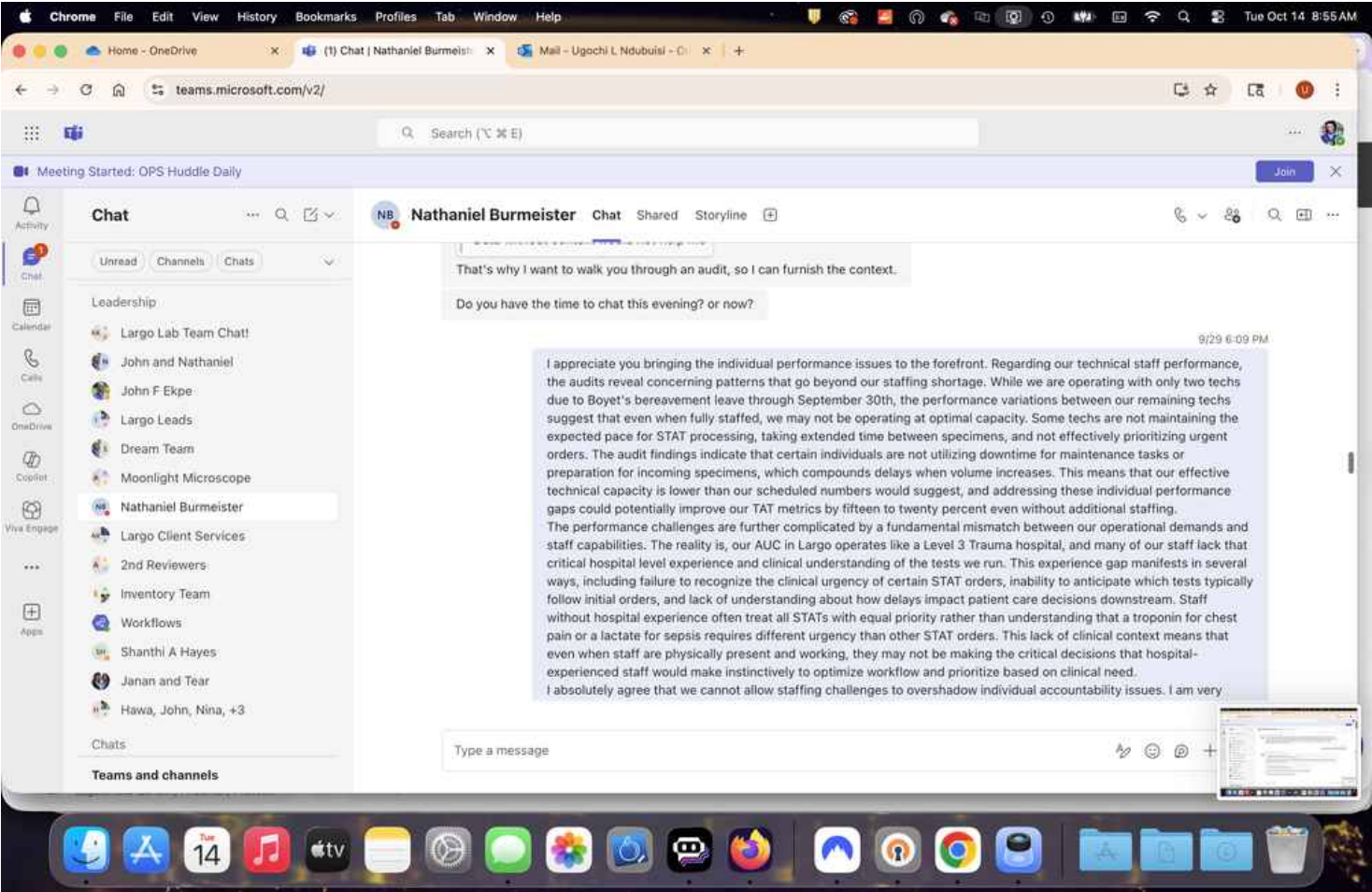
- Nathaniel Burmeister (9/29 6:13 PM): Unfortunately, we do not have freedom to make staffing changes like that. If they are not suited to the job, we must follow our corrective action path until they either get better or get out. Its after 6 now. Are good to hop on a call while I'm still in my office?
- (9/29 6:16 PM): The culture issue runs deeper than individual performance or staffing numbers. We have created and allowed a culture where mediocrity is tolerated, where staff do not feel the urgency of the clinical work we perform, and where accountability is virtually nonexistent. The fact that techs do not think to return tubes to the MOB, that staff call out at the last minute without considering patient impact, and that people work at their own pace regardless of STAT priorities all point to a culture that has lost sight of our purpose. We have staff who view this as just another job rather than critical healthcare delivery. They clock in, do the minimum required to avoid immediate consequences, and clock out without ownership of outcomes. When someone can work through a shift knowing that sixty percent of STATs are failing and not feel personally compelled to change their approach, that represents a complete cultural failure. The hospital-level experience gap I mentioned earlier is part of this, but even staff without hospital backgrounds should understand urgency if the culture demanded it.
- (9/29 6:16 PM): Sorry eating at the moment
- (9/29 6:16 PM): I am free tomorrow
- Nathaniel Burmeister (9/29 6:17 PM): I can make myself available later this evening as well. I have a desk at home as well.

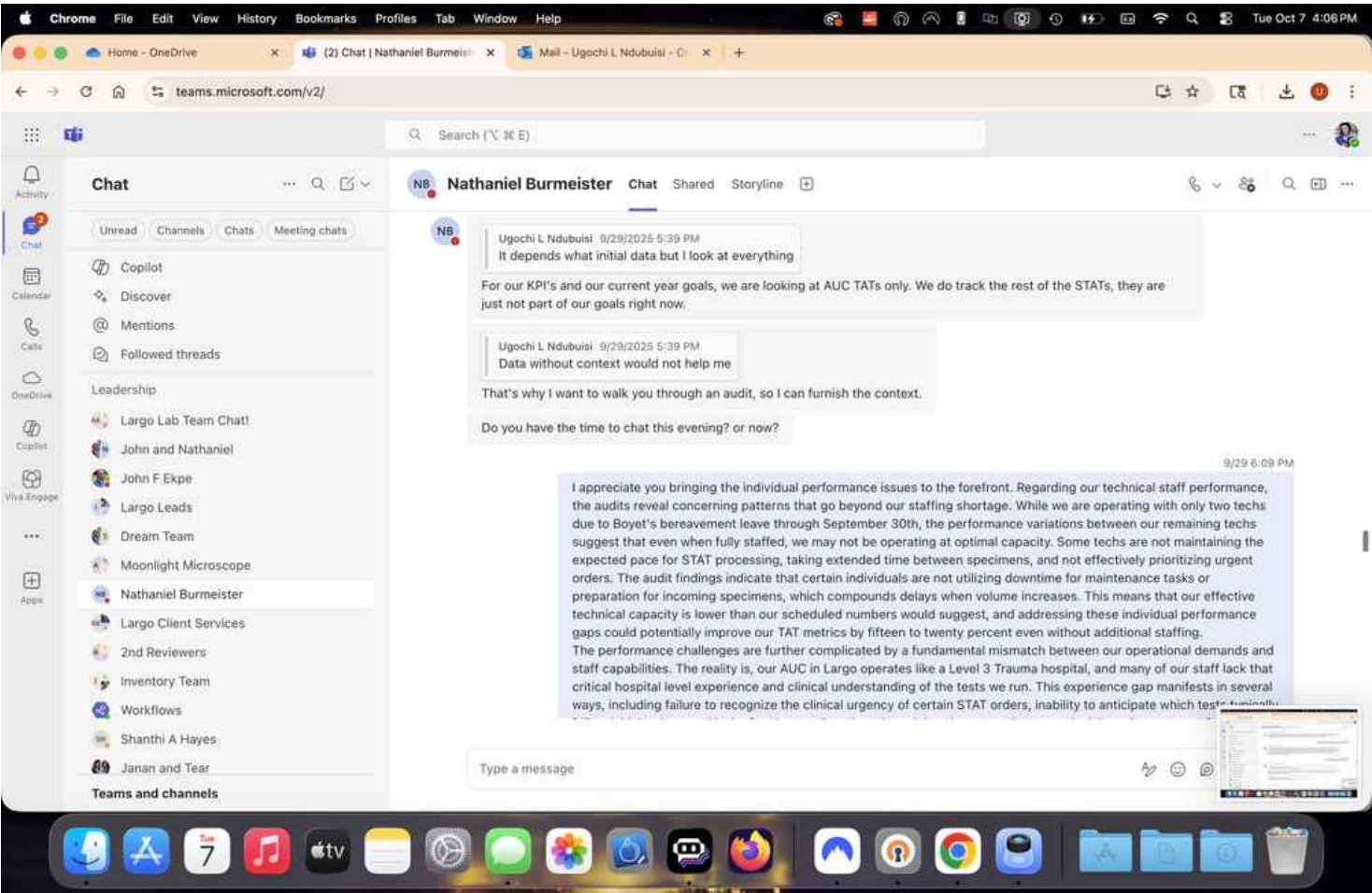
The interface includes a sidebar on the left with navigation options like Activity, Chat, Calendar, and a list of teams and channels. The chat input area at the bottom shows "Type a message".

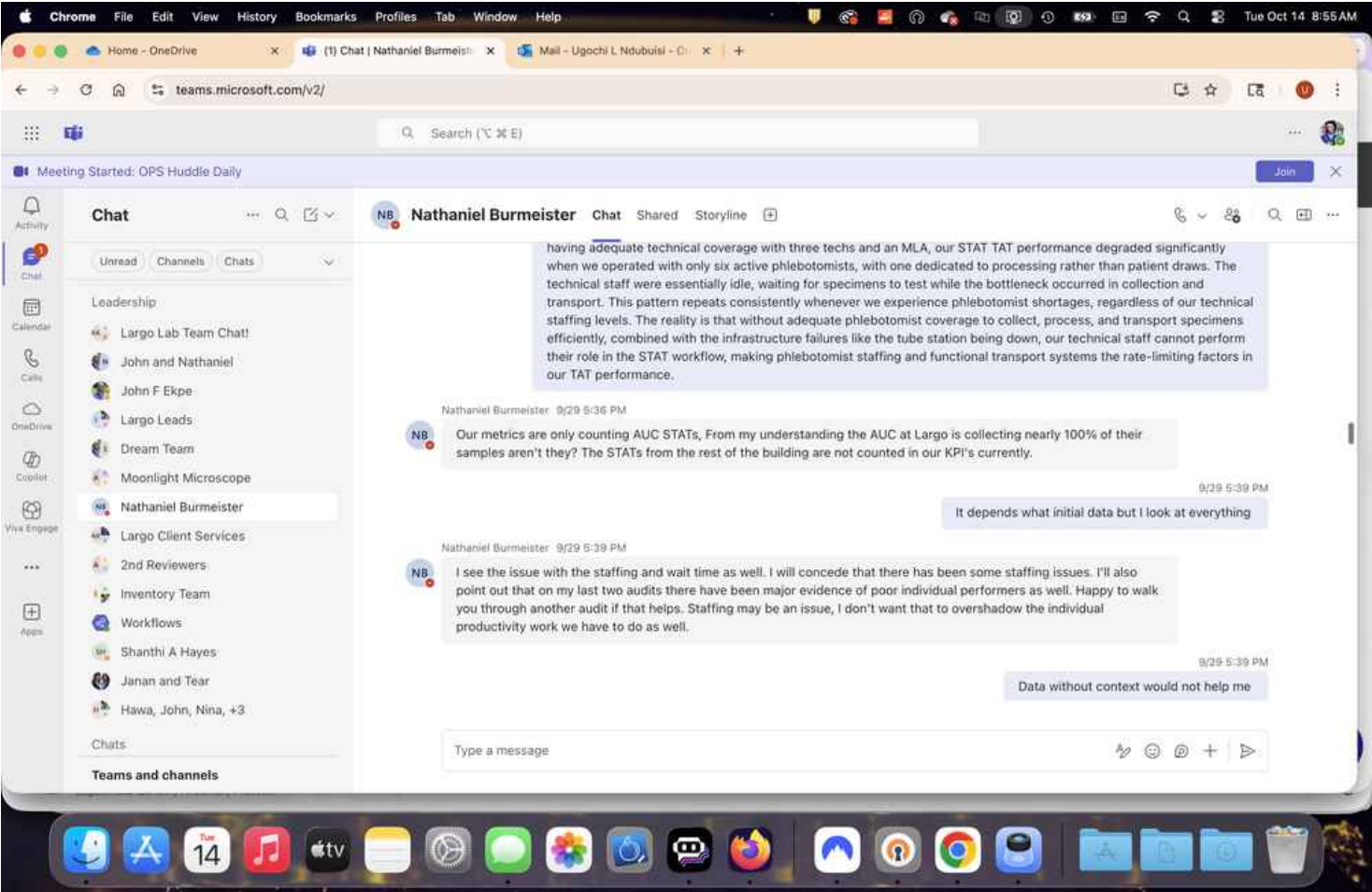


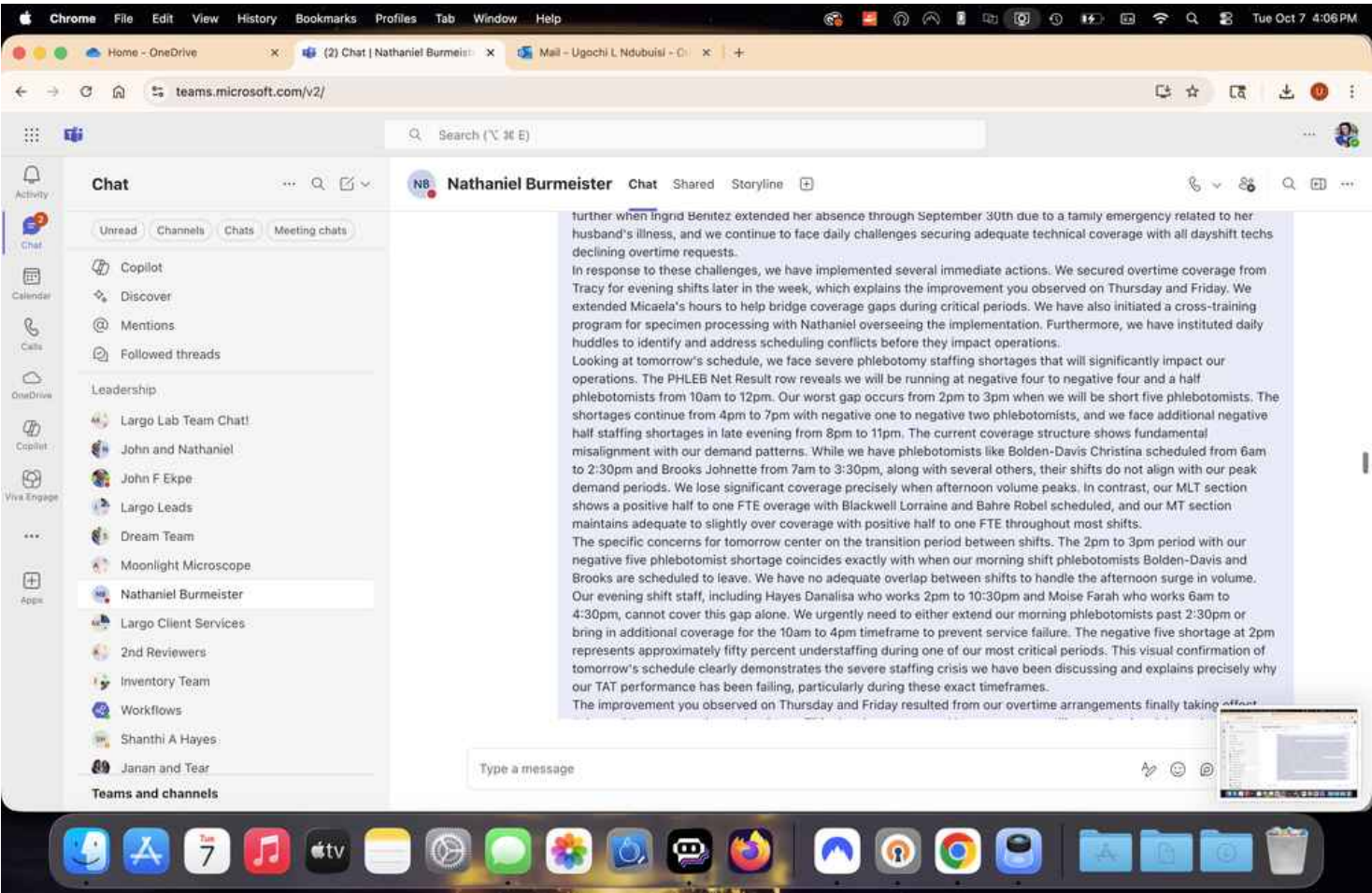


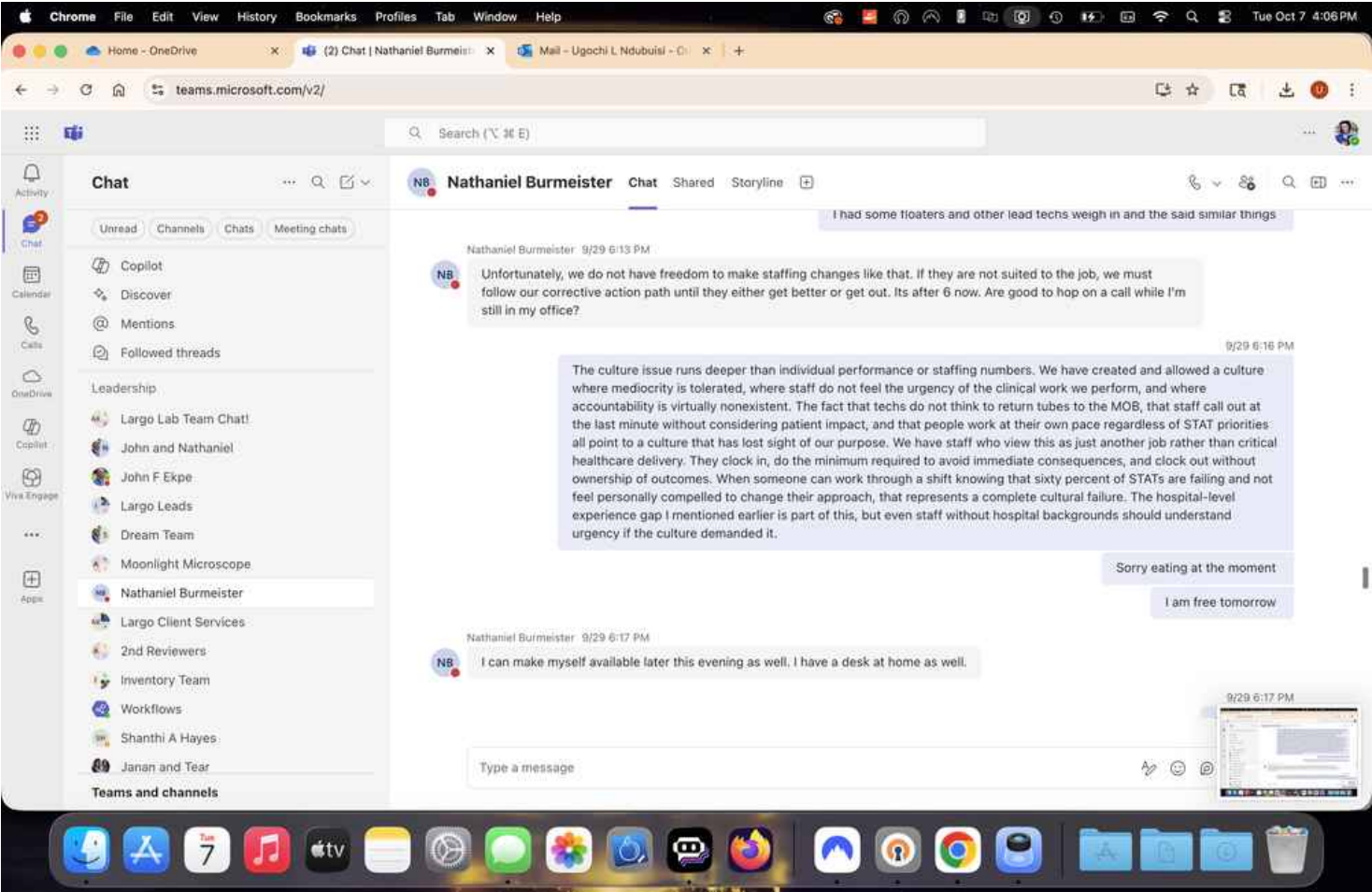




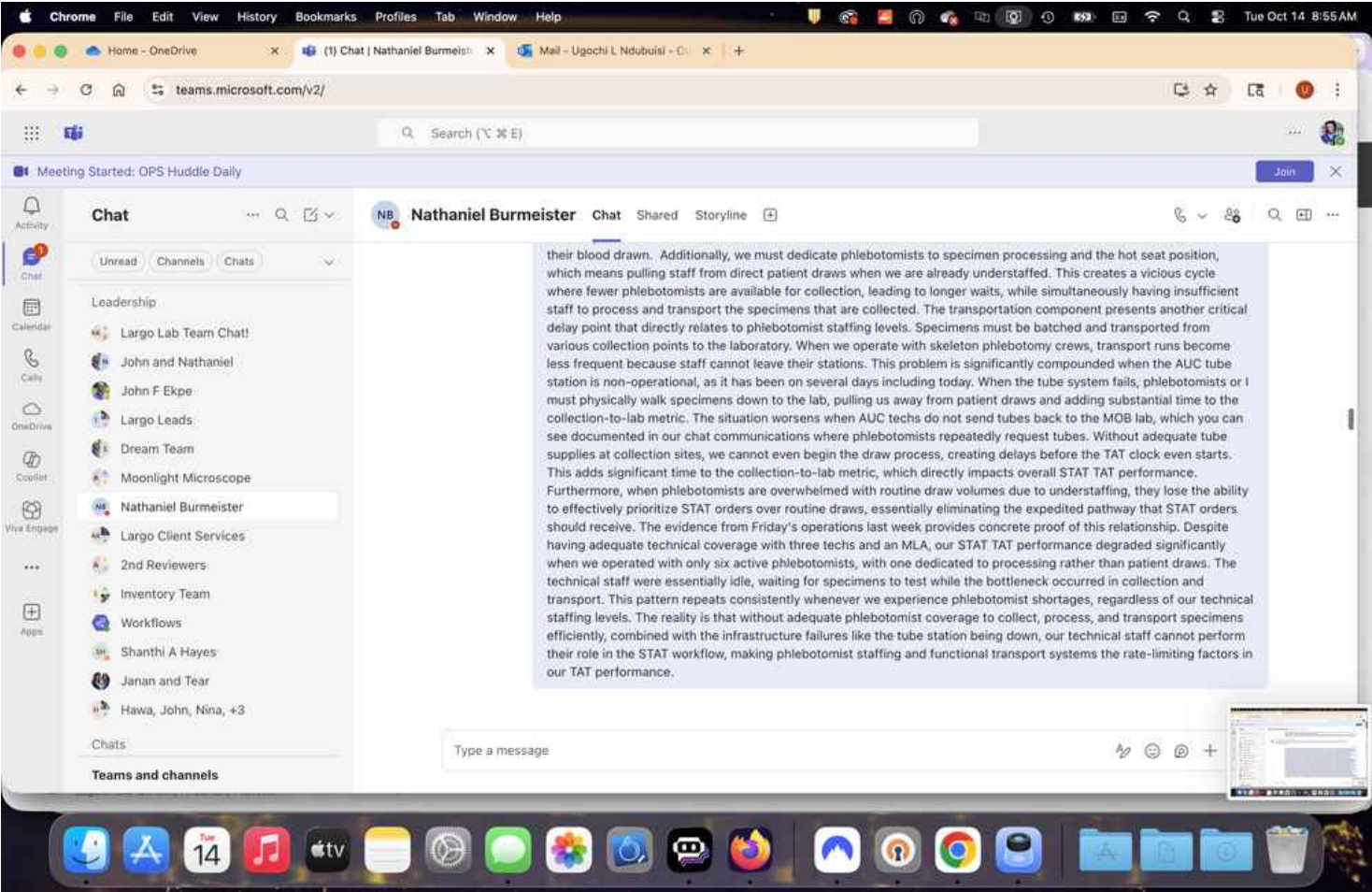




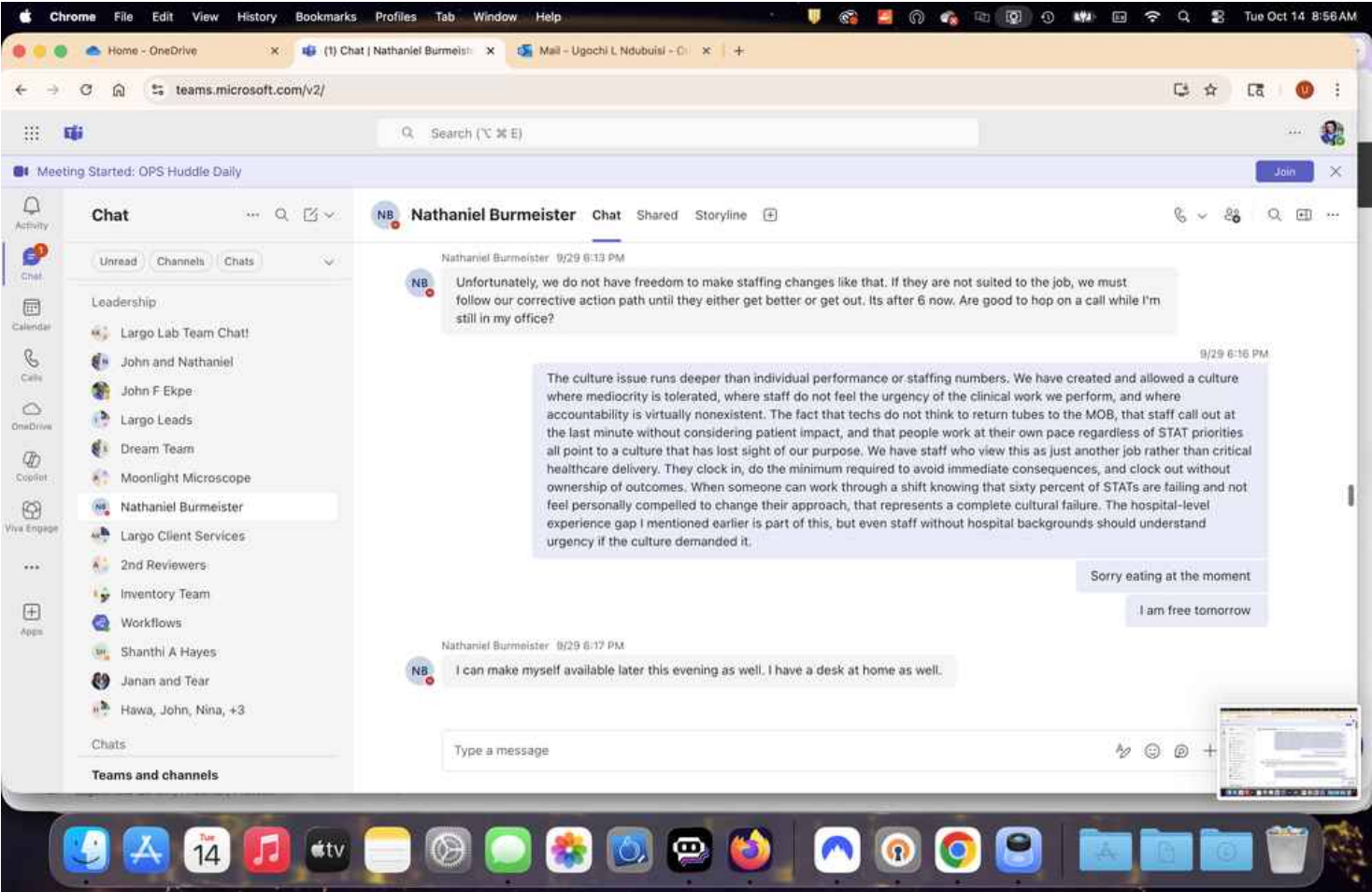


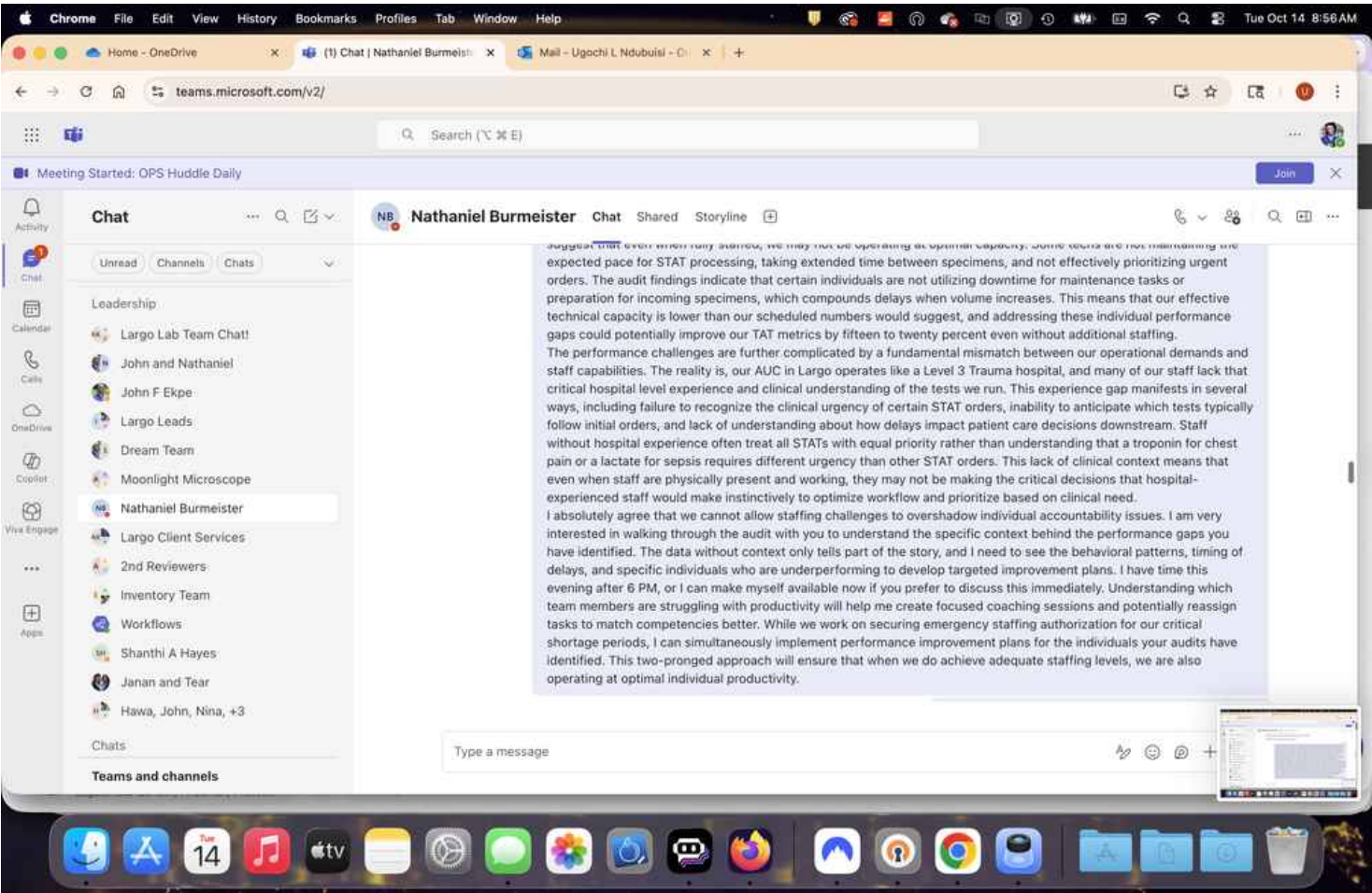


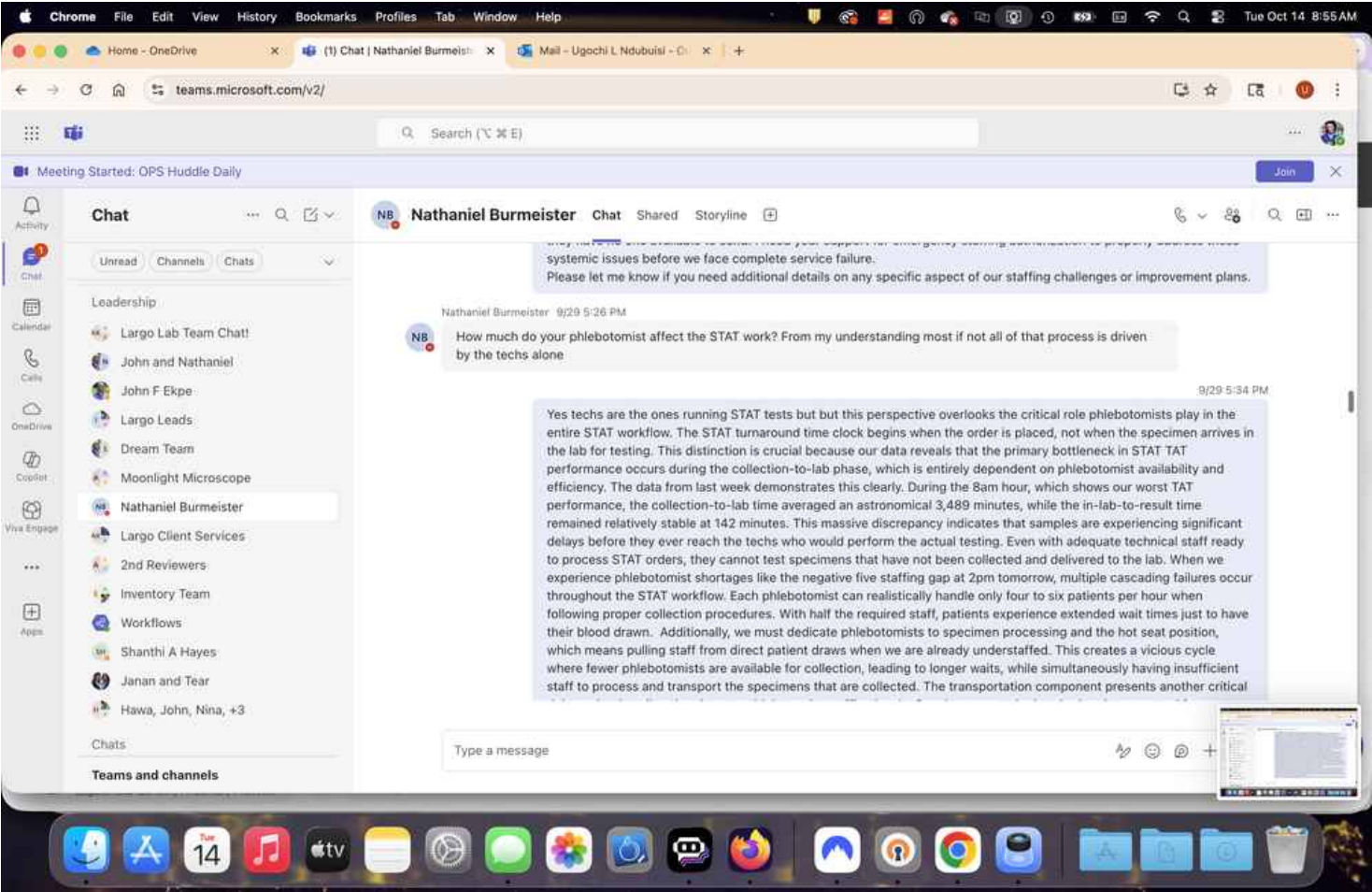
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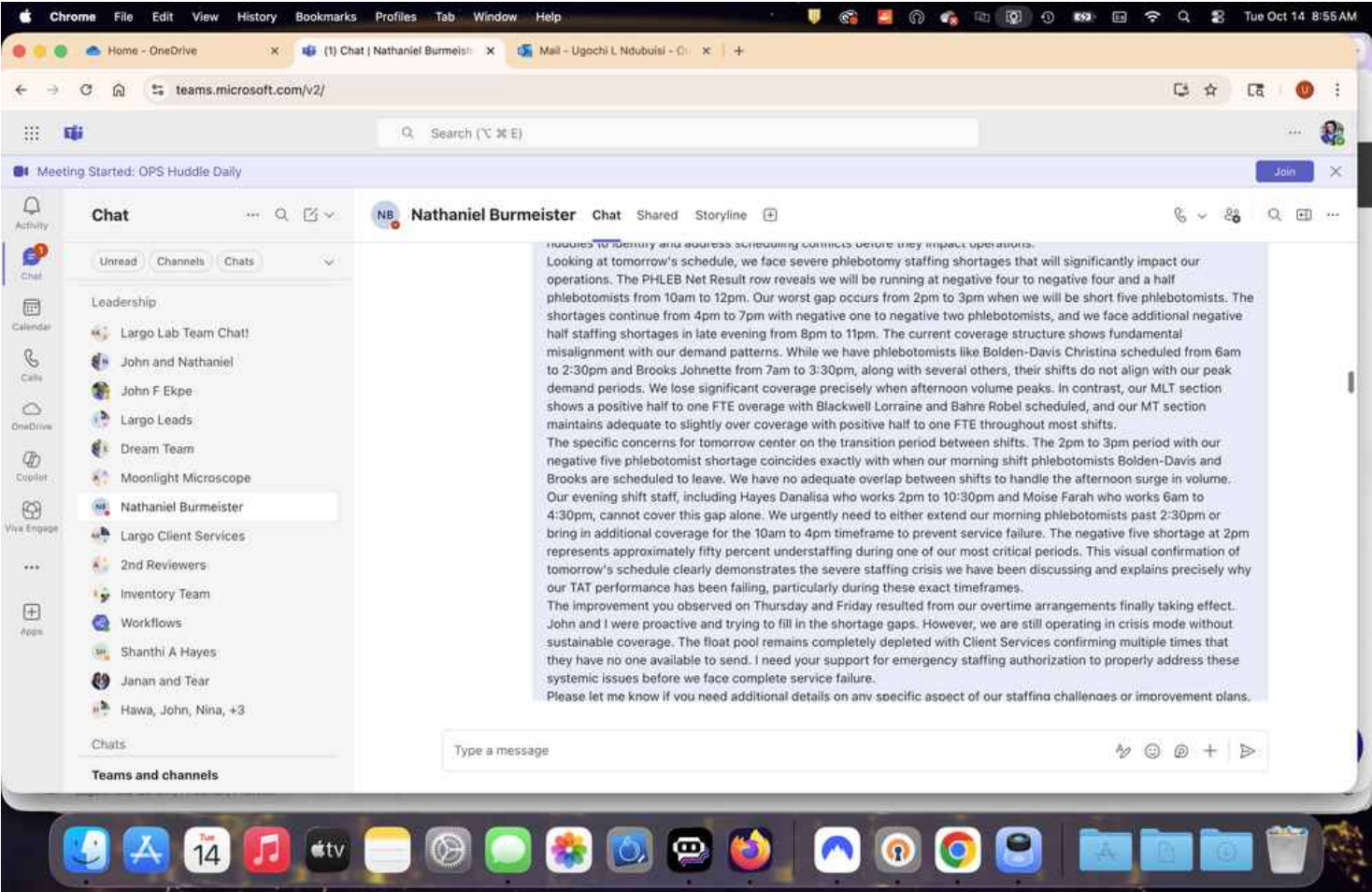
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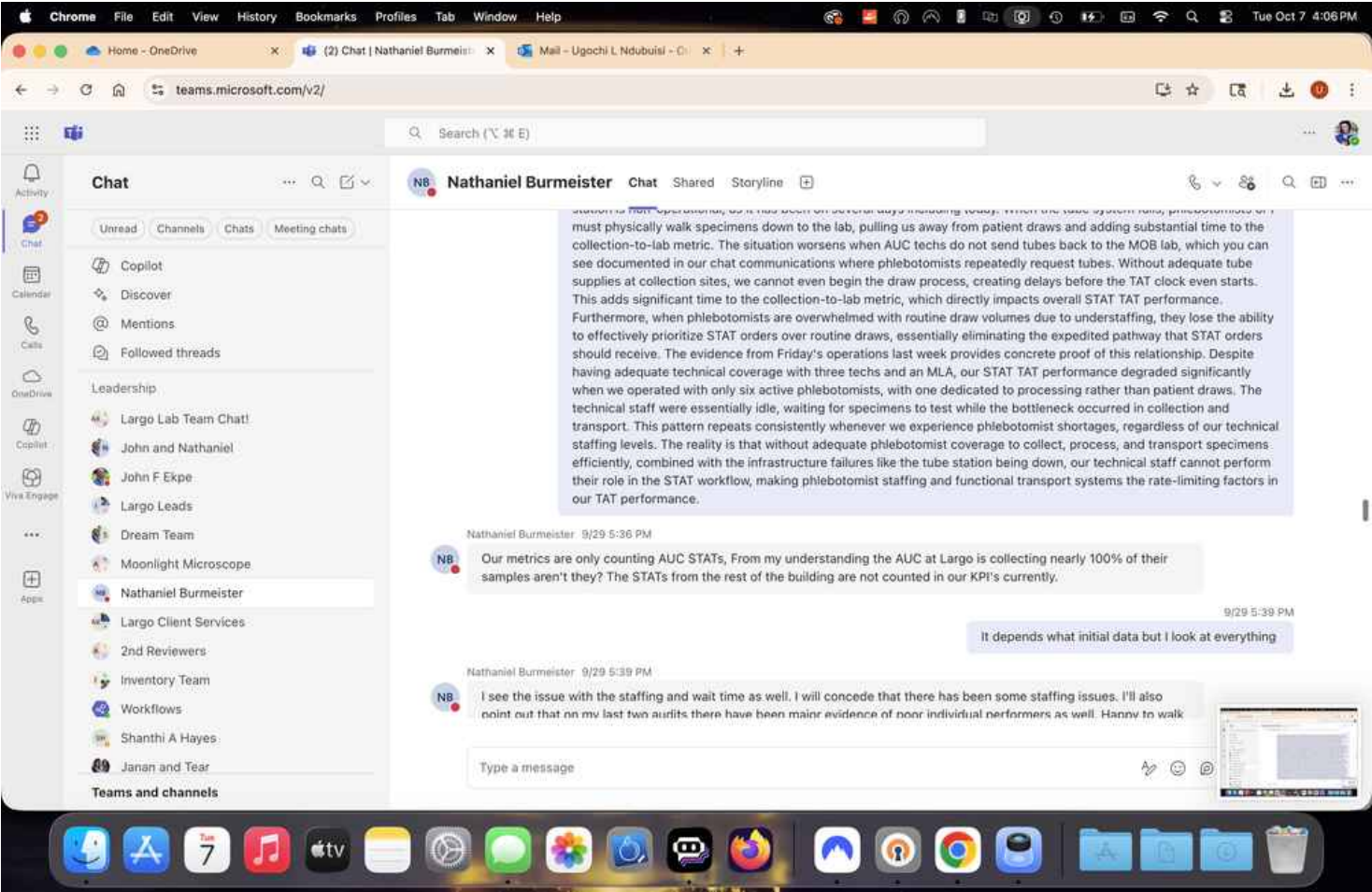


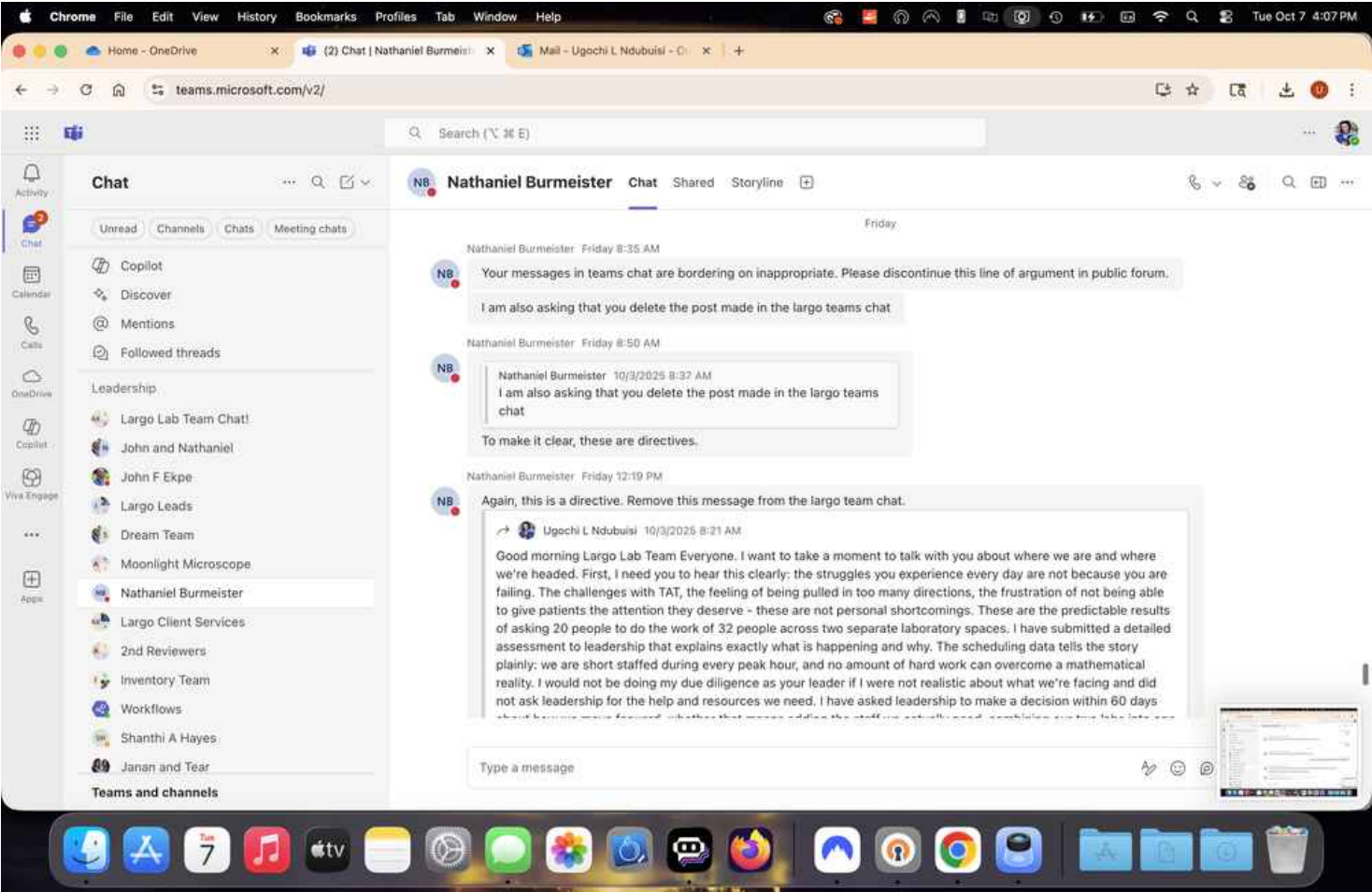


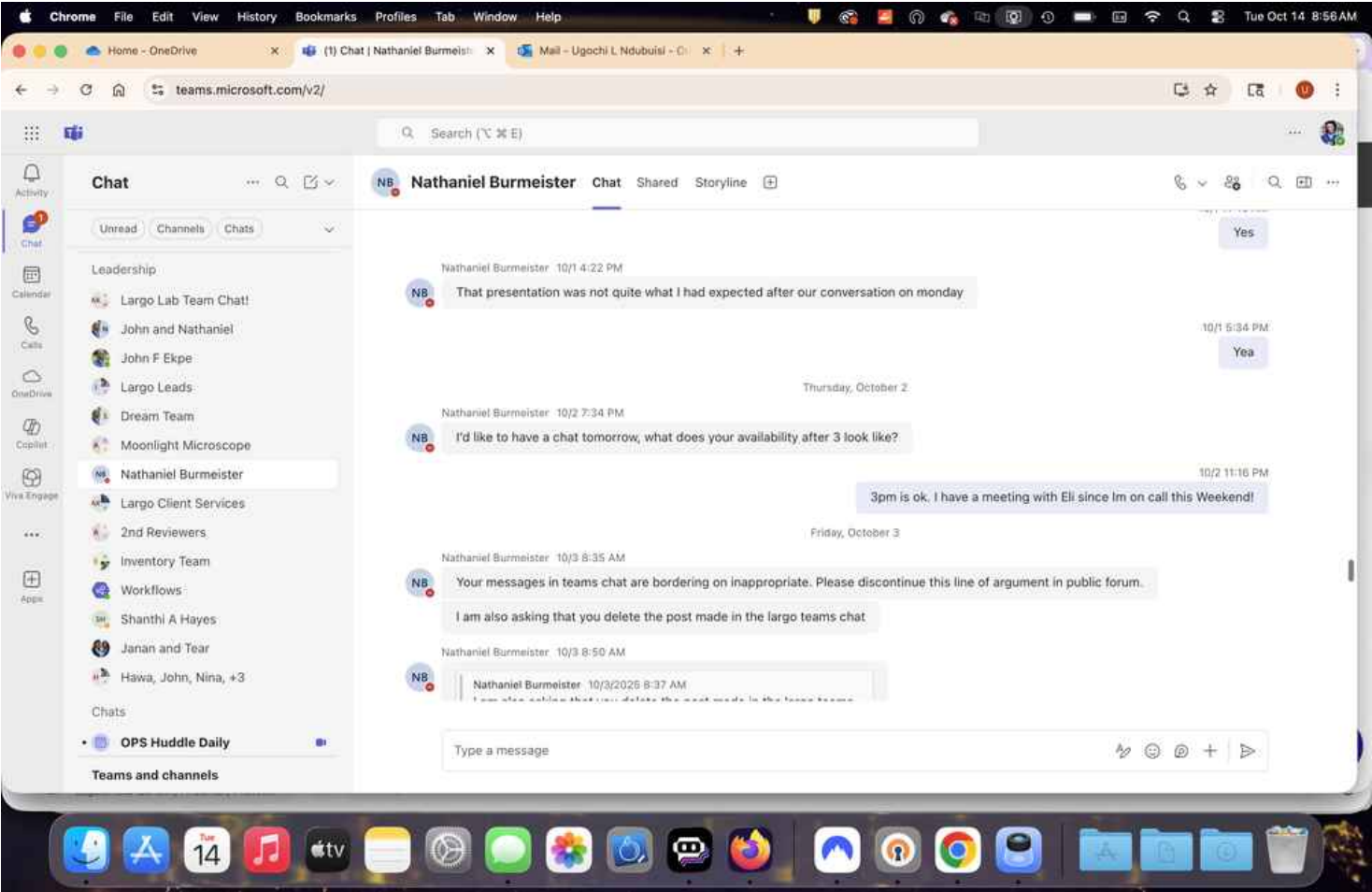


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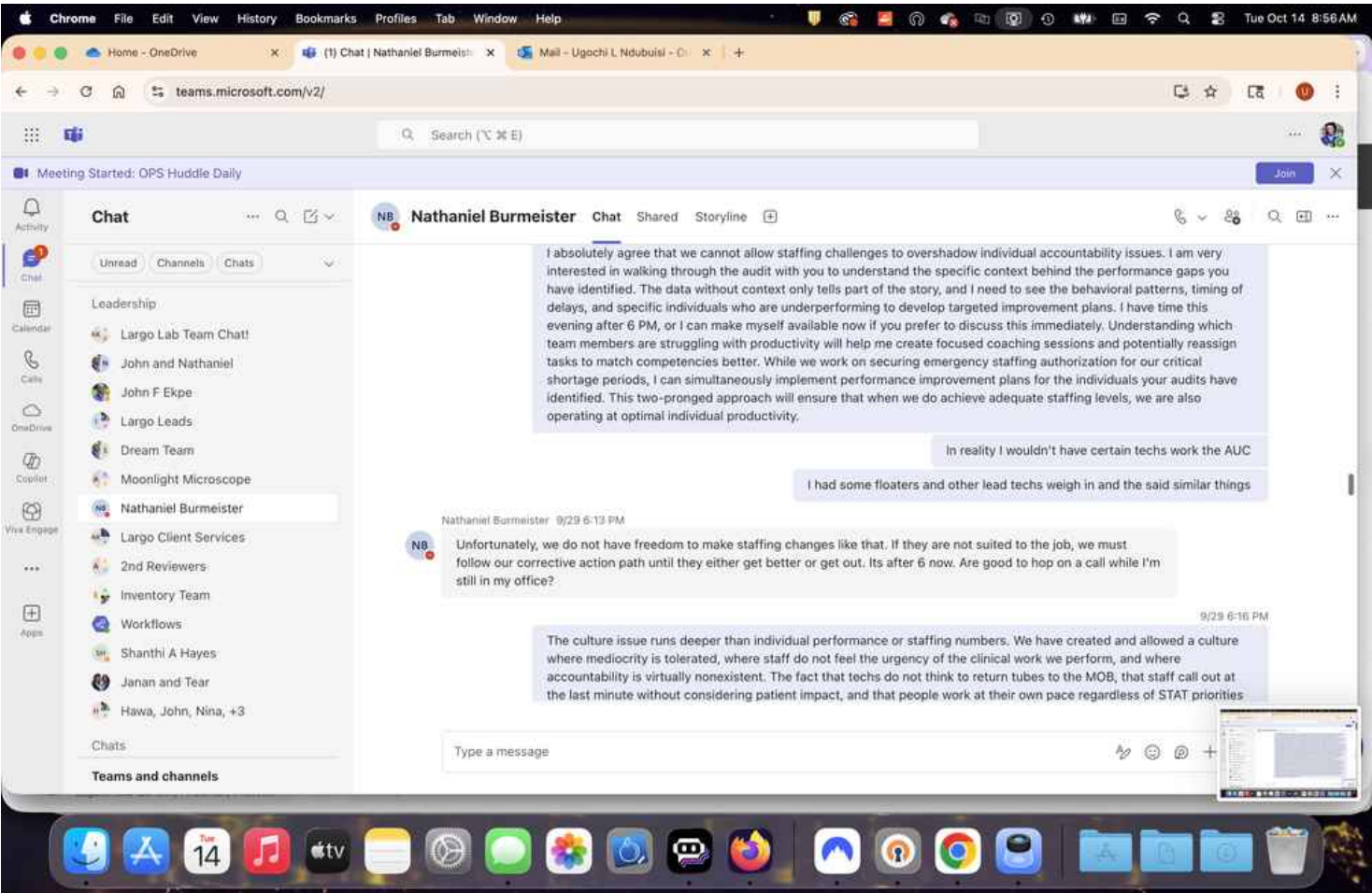


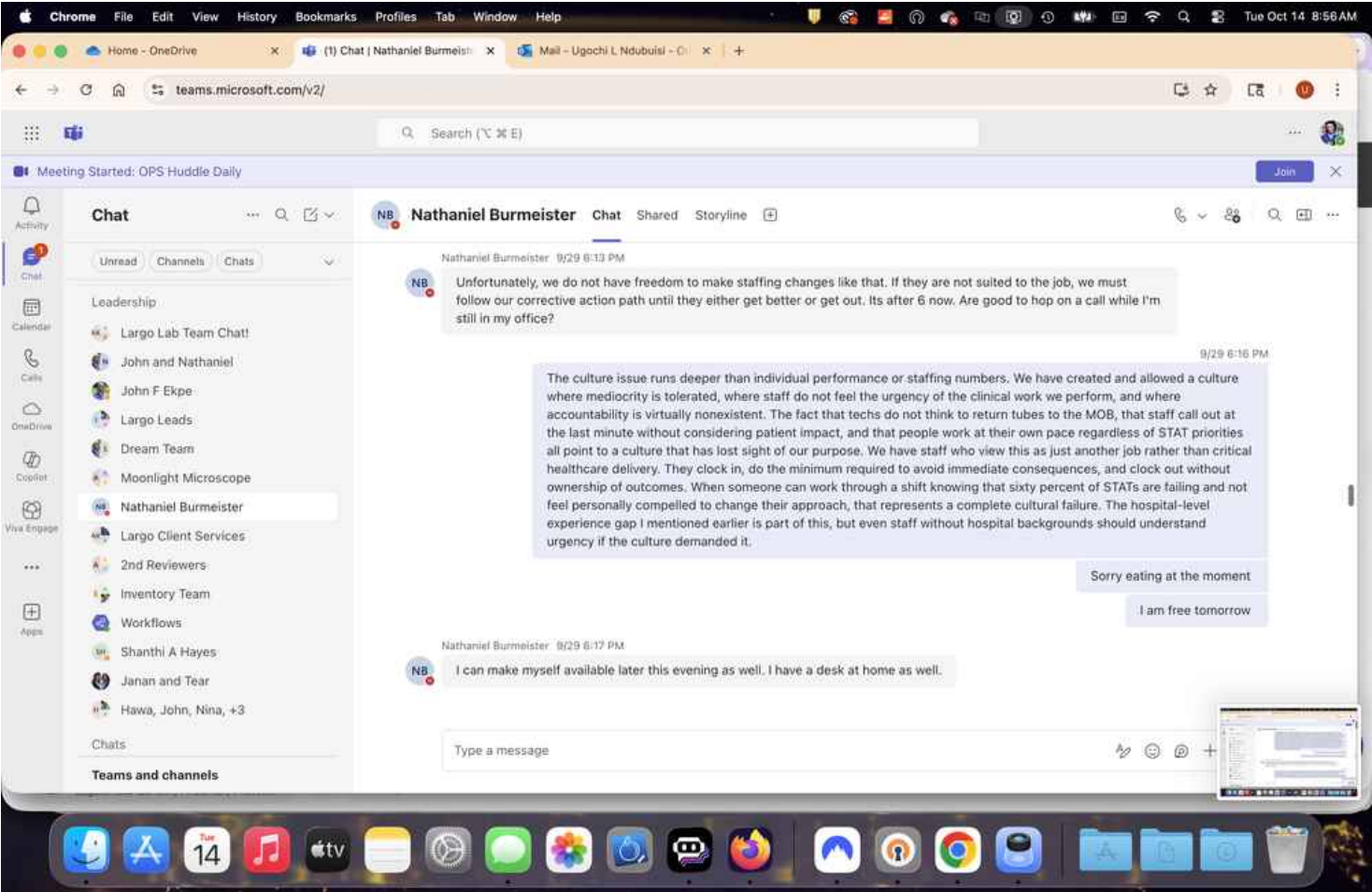


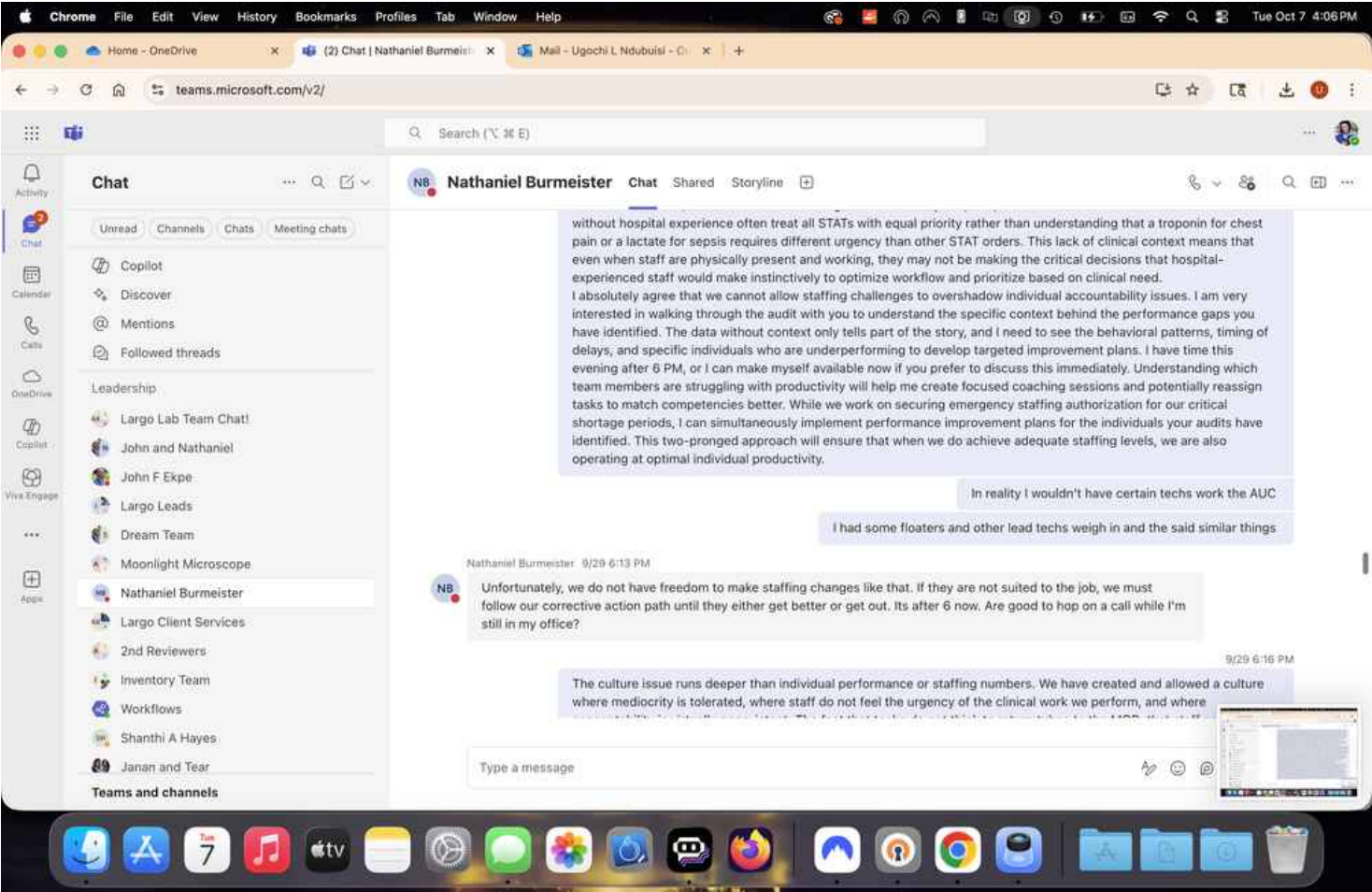




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WHISTLEBLOWER RETALIATION EVIDENCE

Largo Laboratory - Kaiser Permanente

EXECUTIVE SUMMARY

This document presents chronological evidence of retaliation against Ugochi L. Ndubuisi for protected whistleblowing activity. The timeline shows:

1. July 14, 2025: Director orders investigation
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Maryland OSHA Deadline: November 7, 2025

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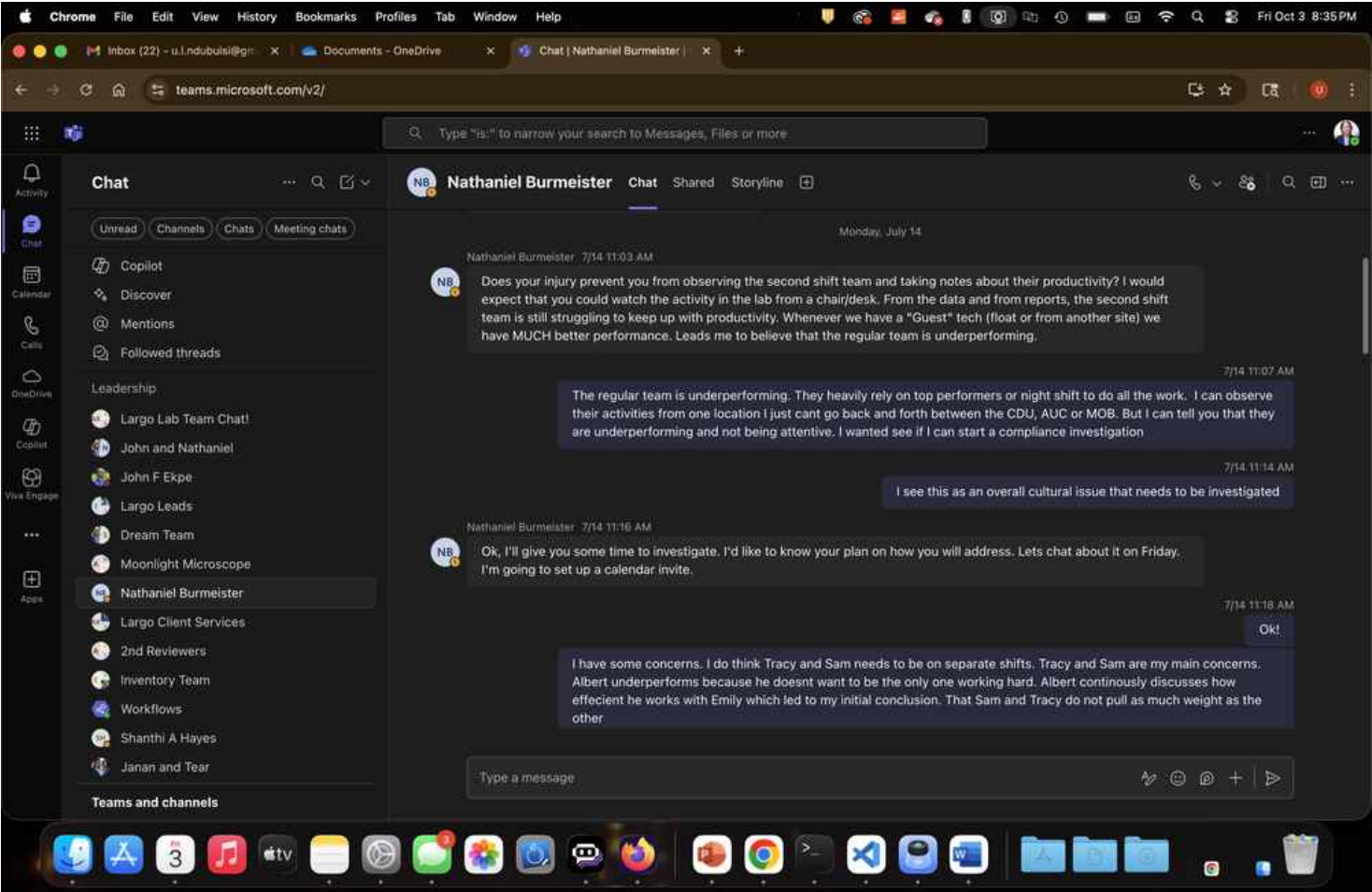
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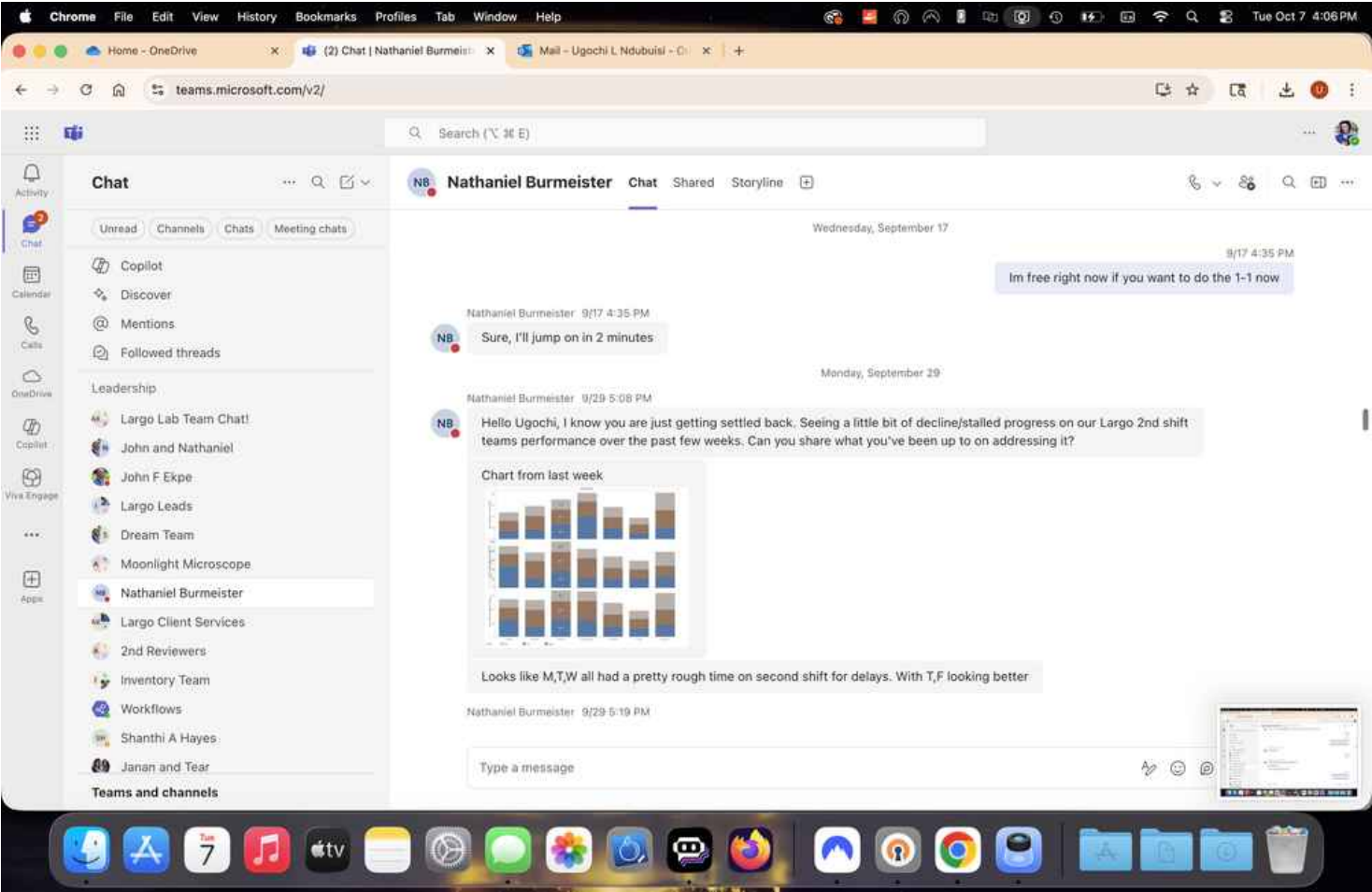
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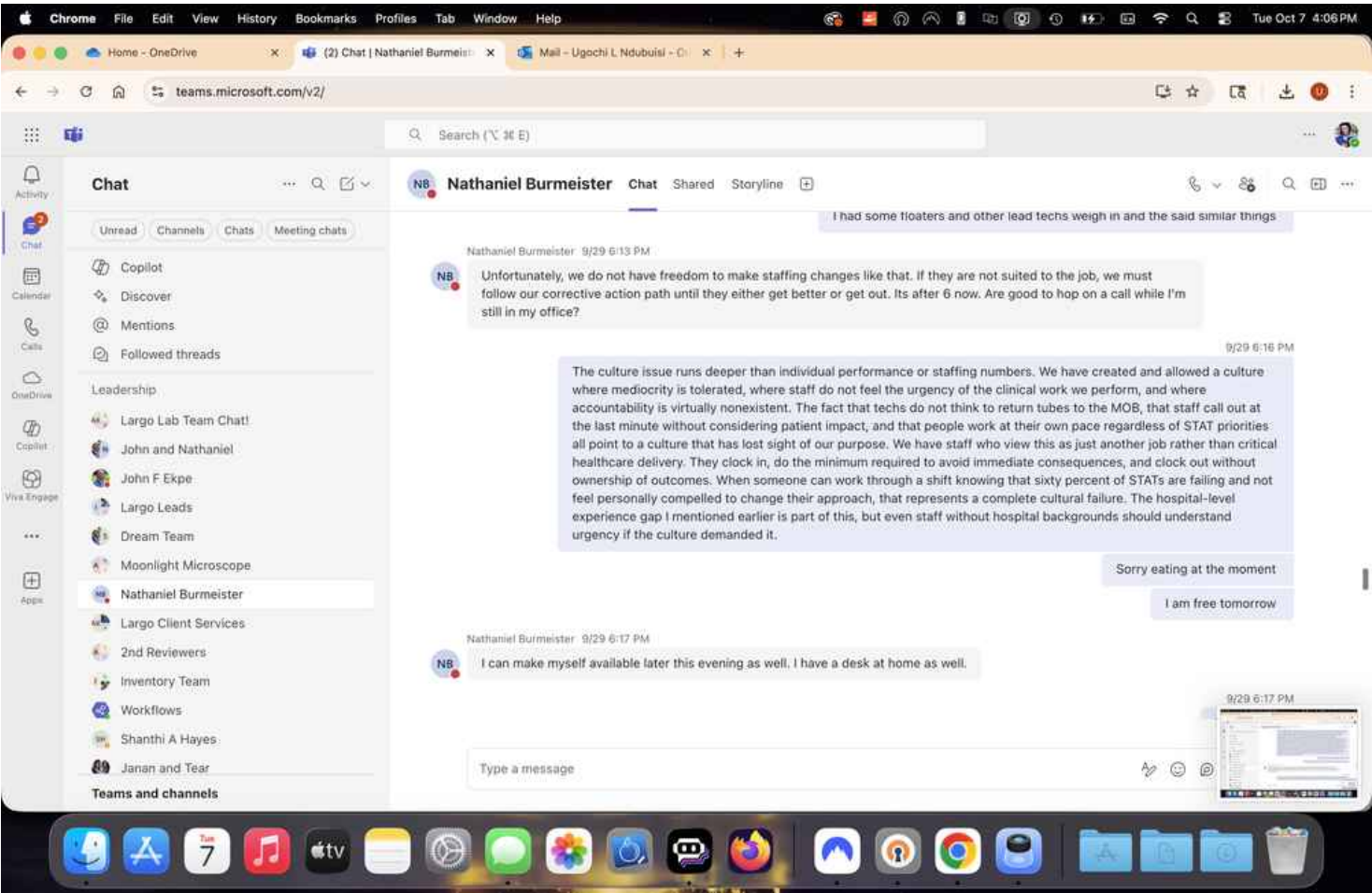
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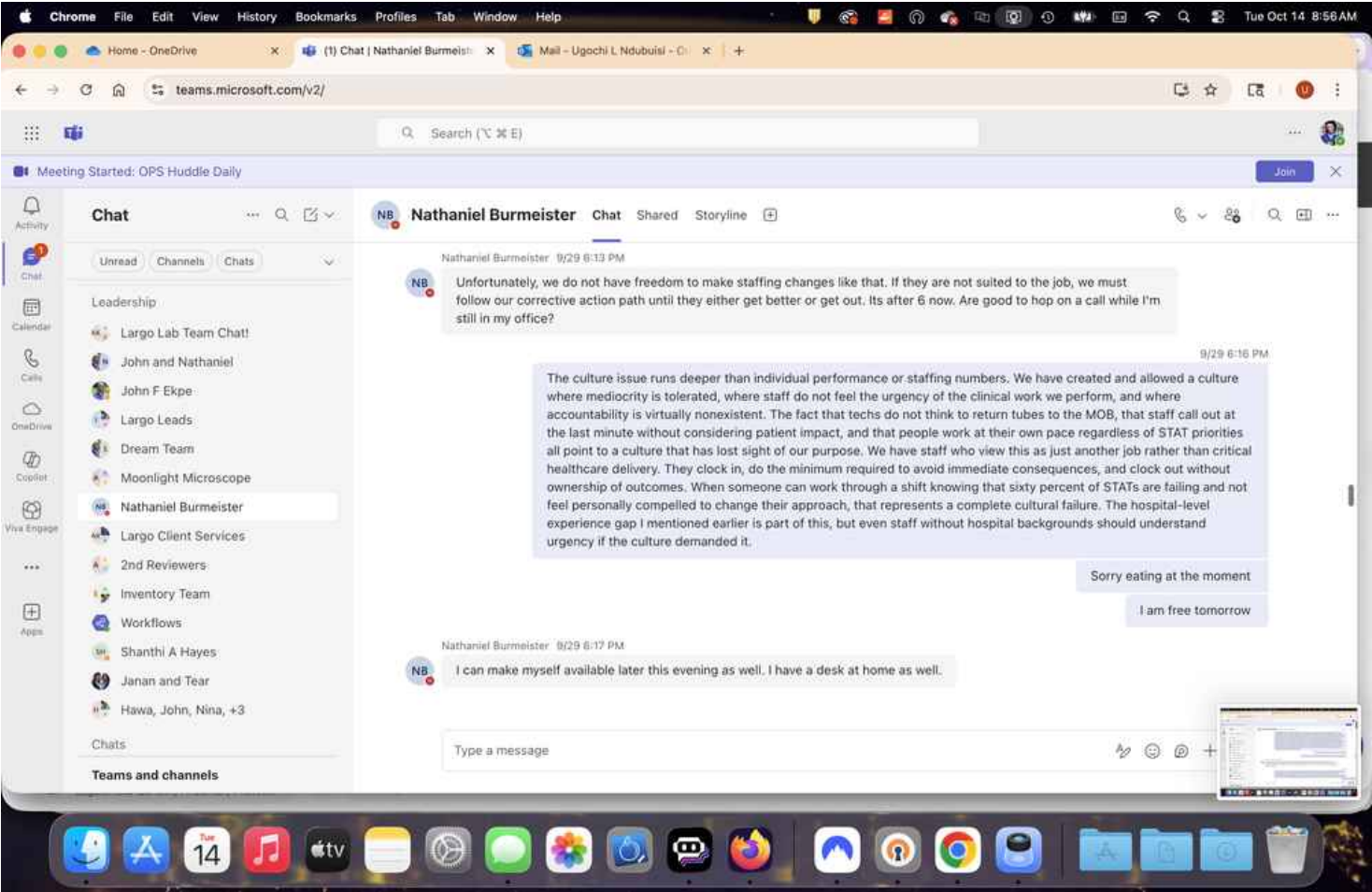
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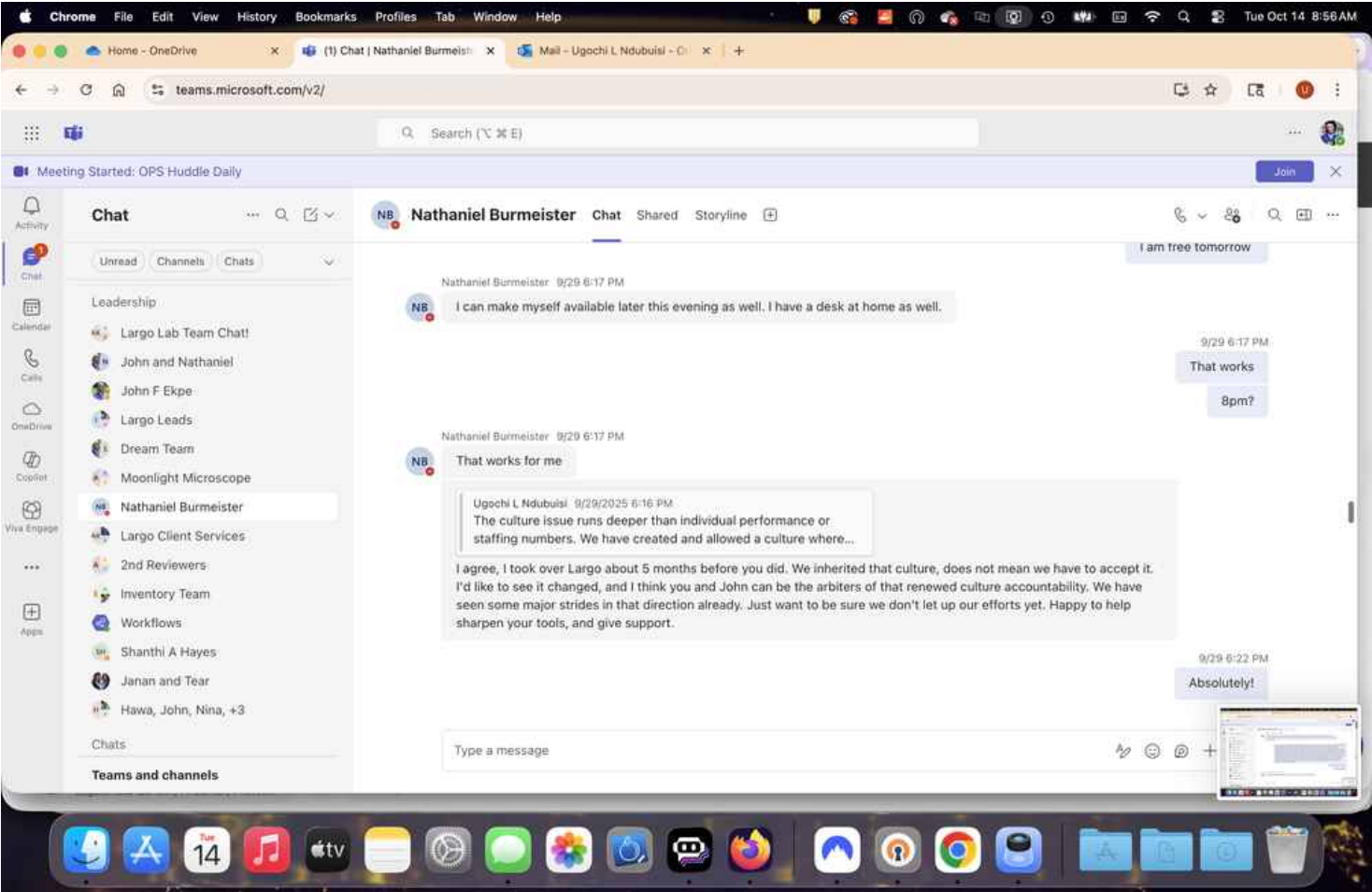
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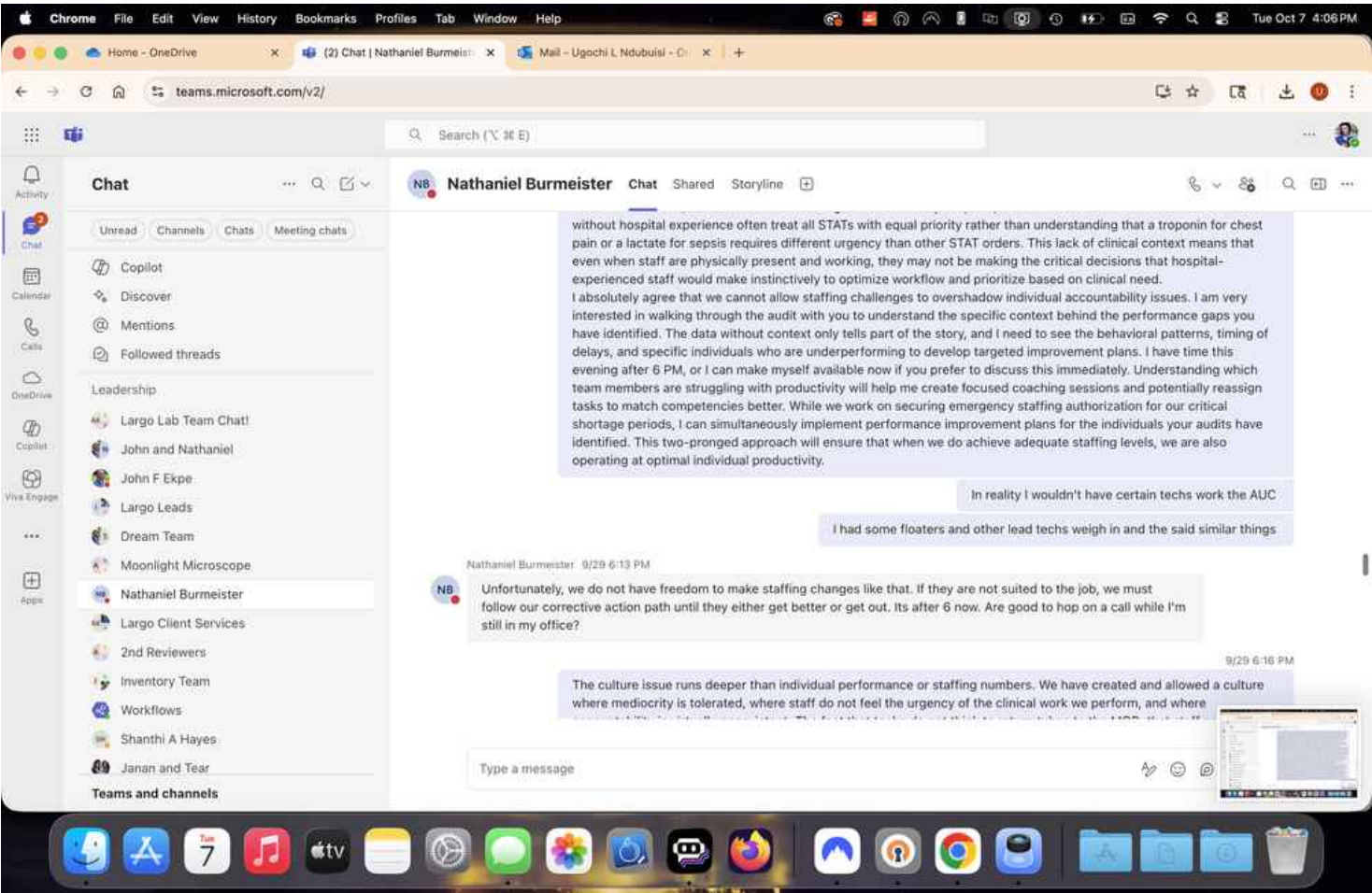


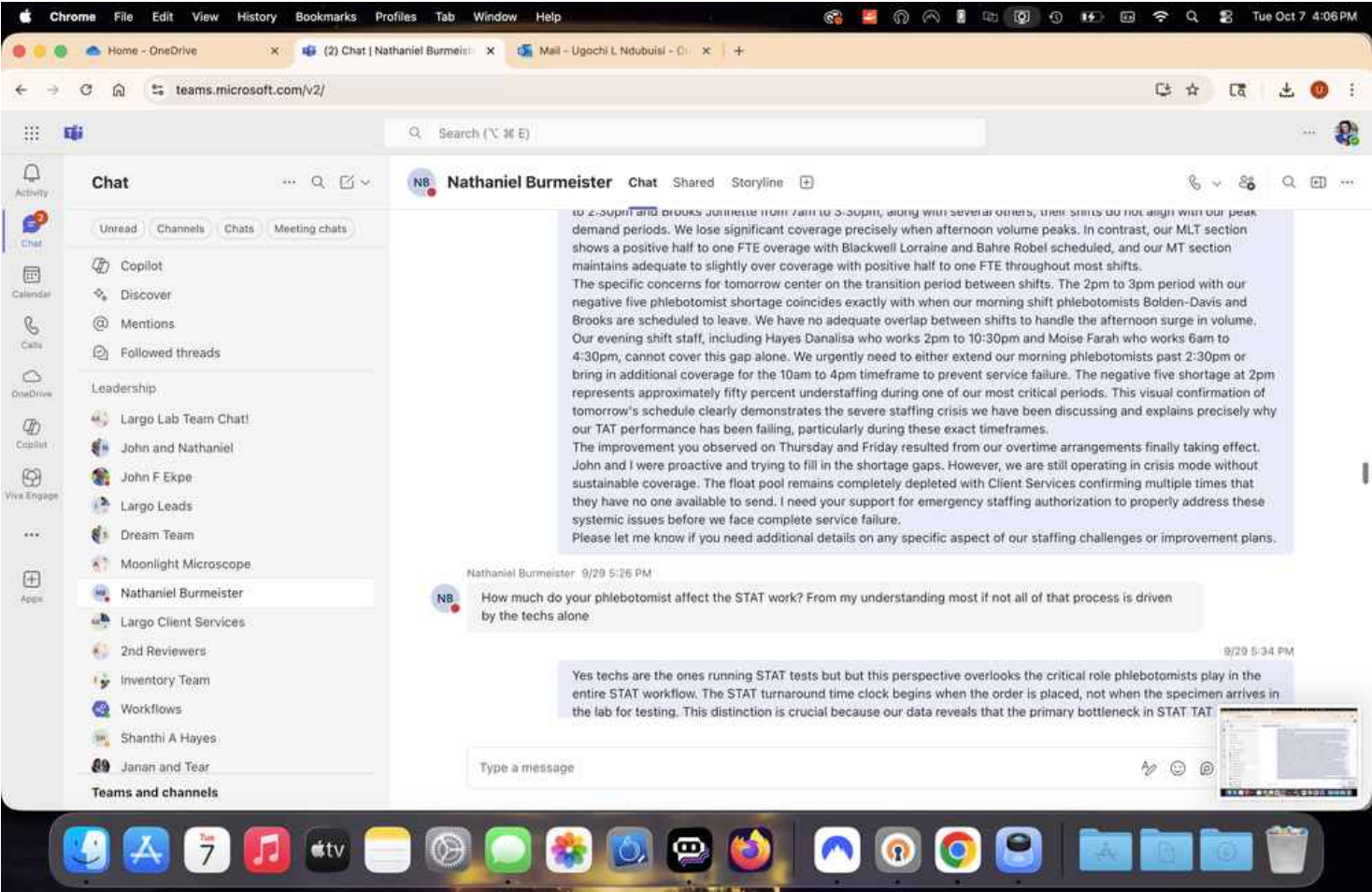


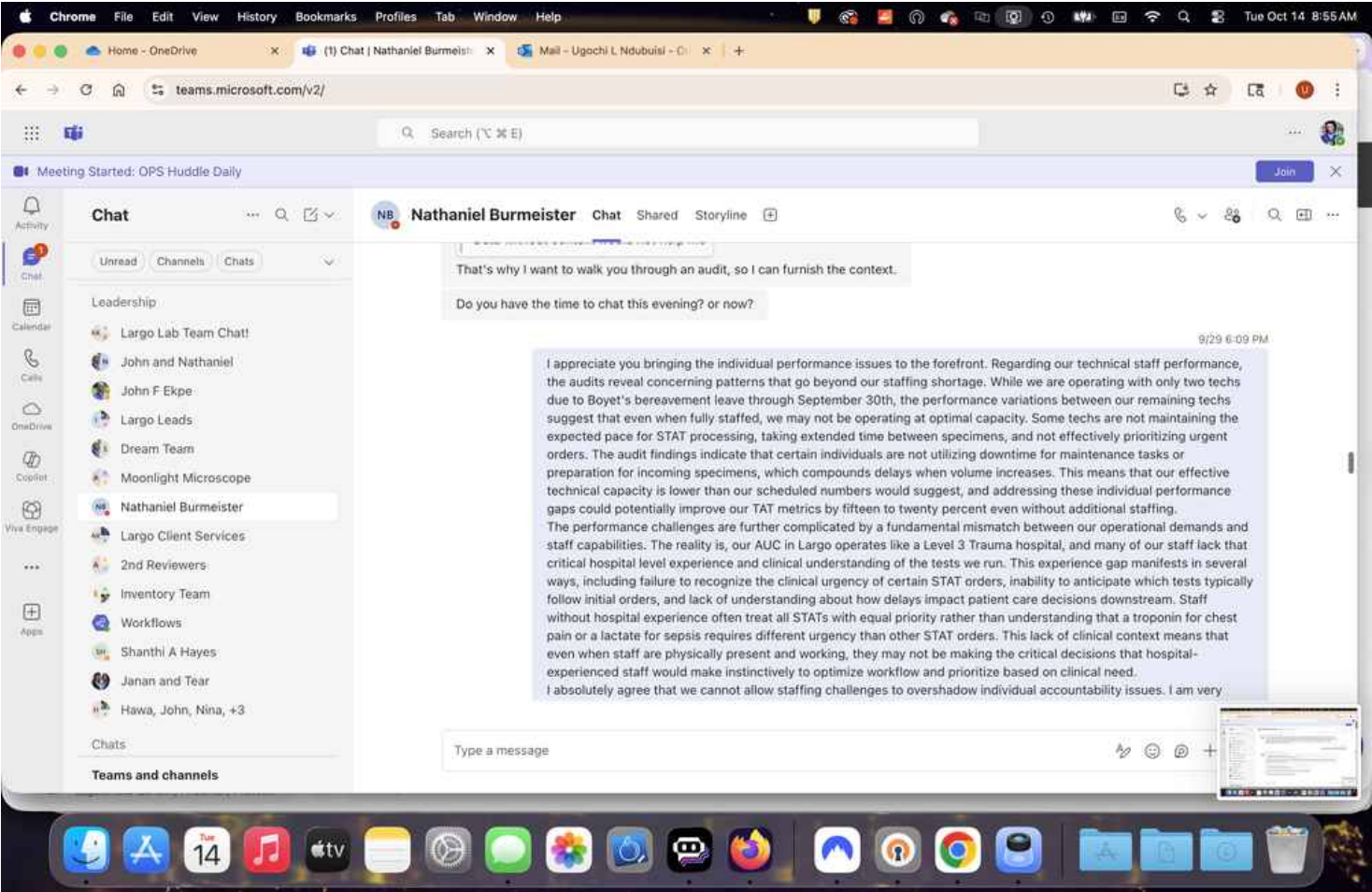


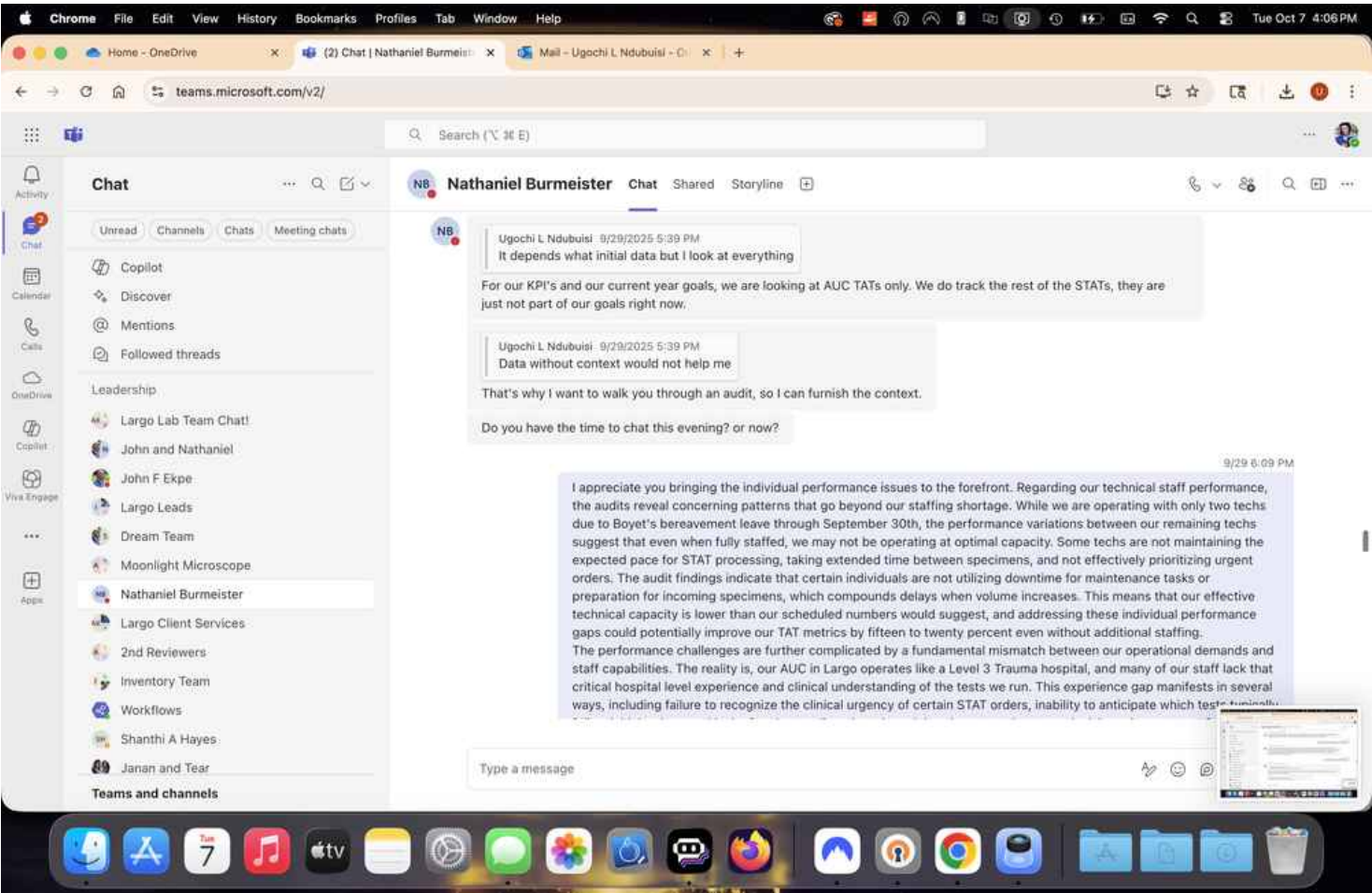


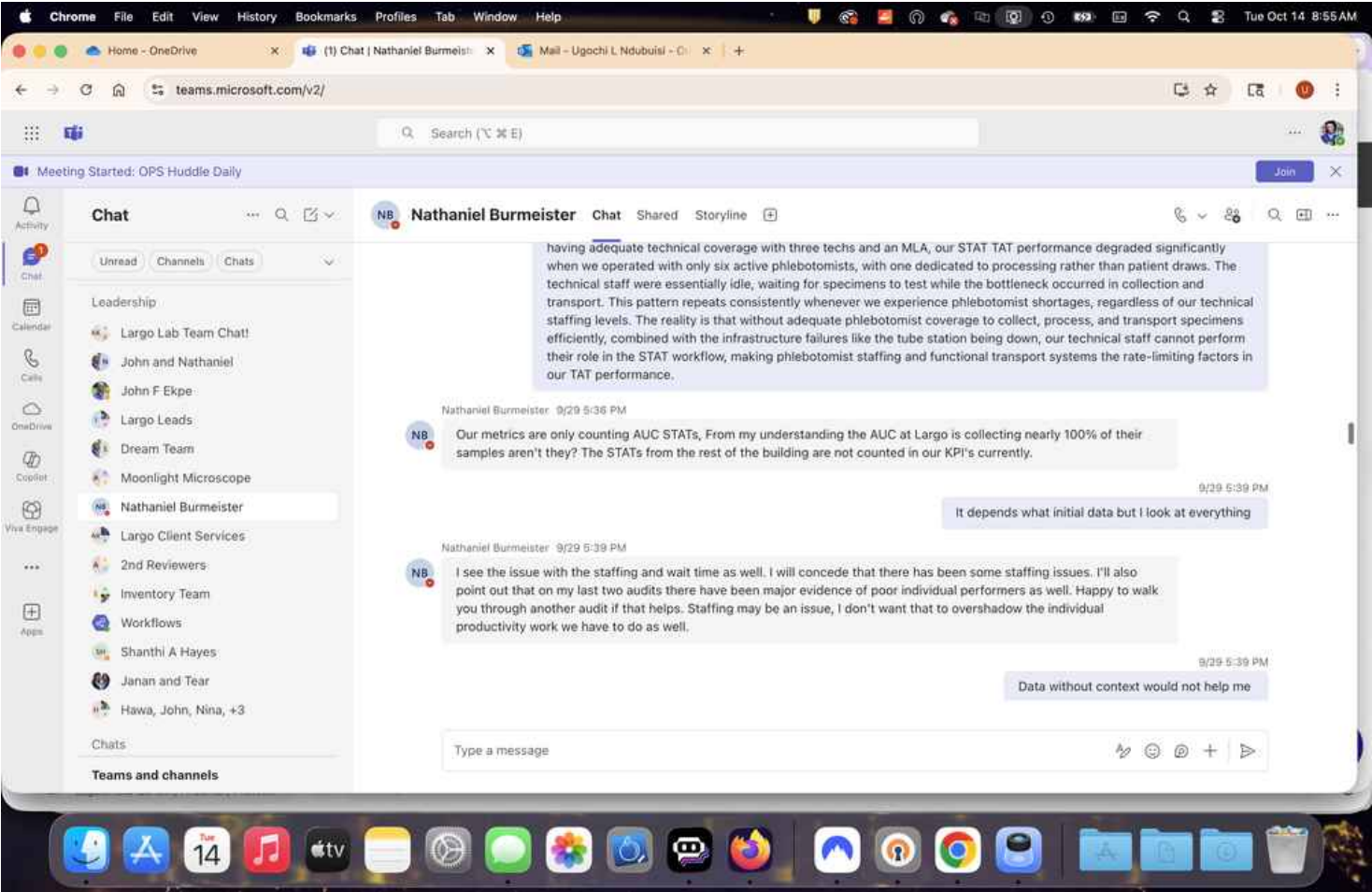


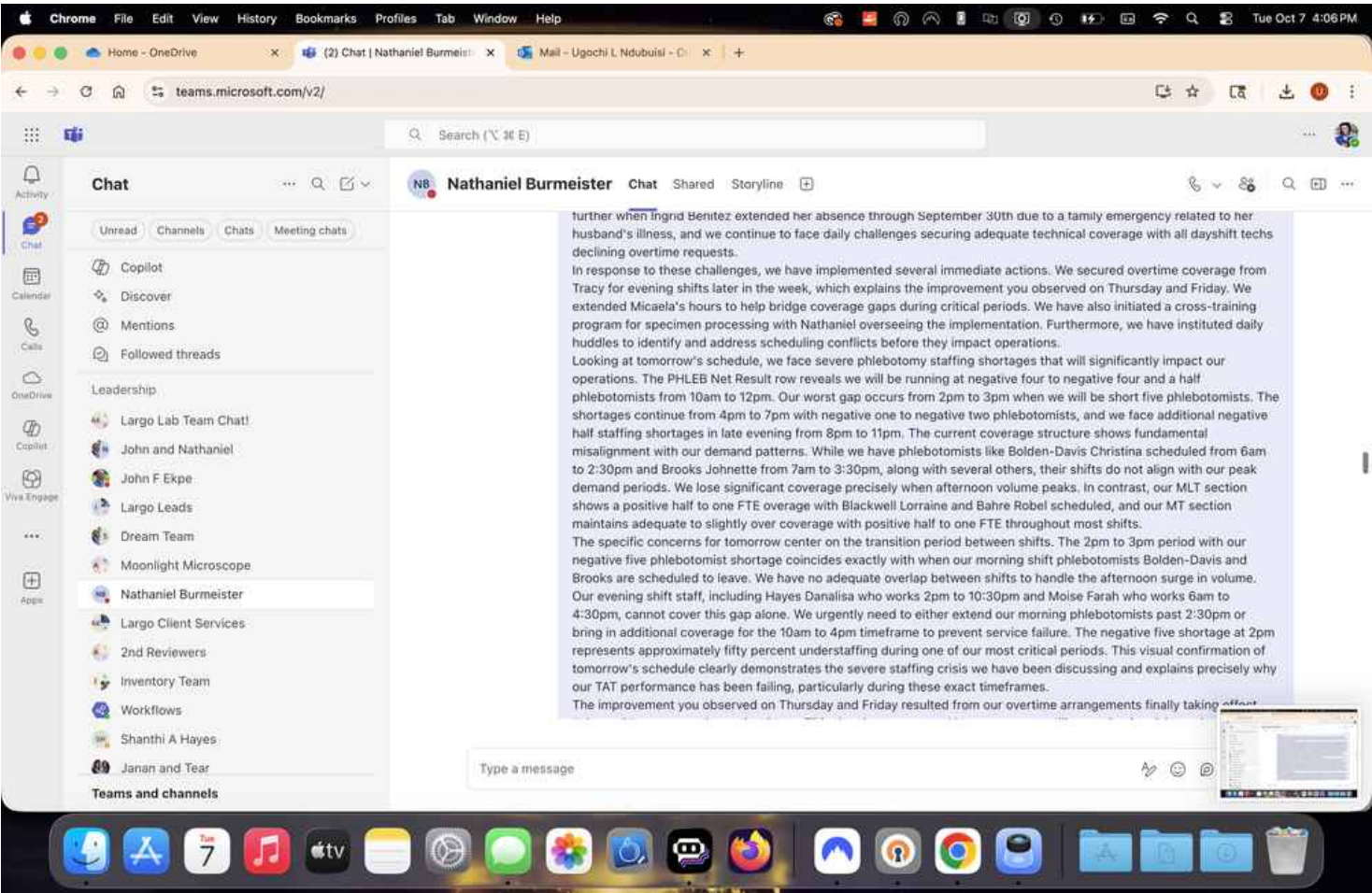


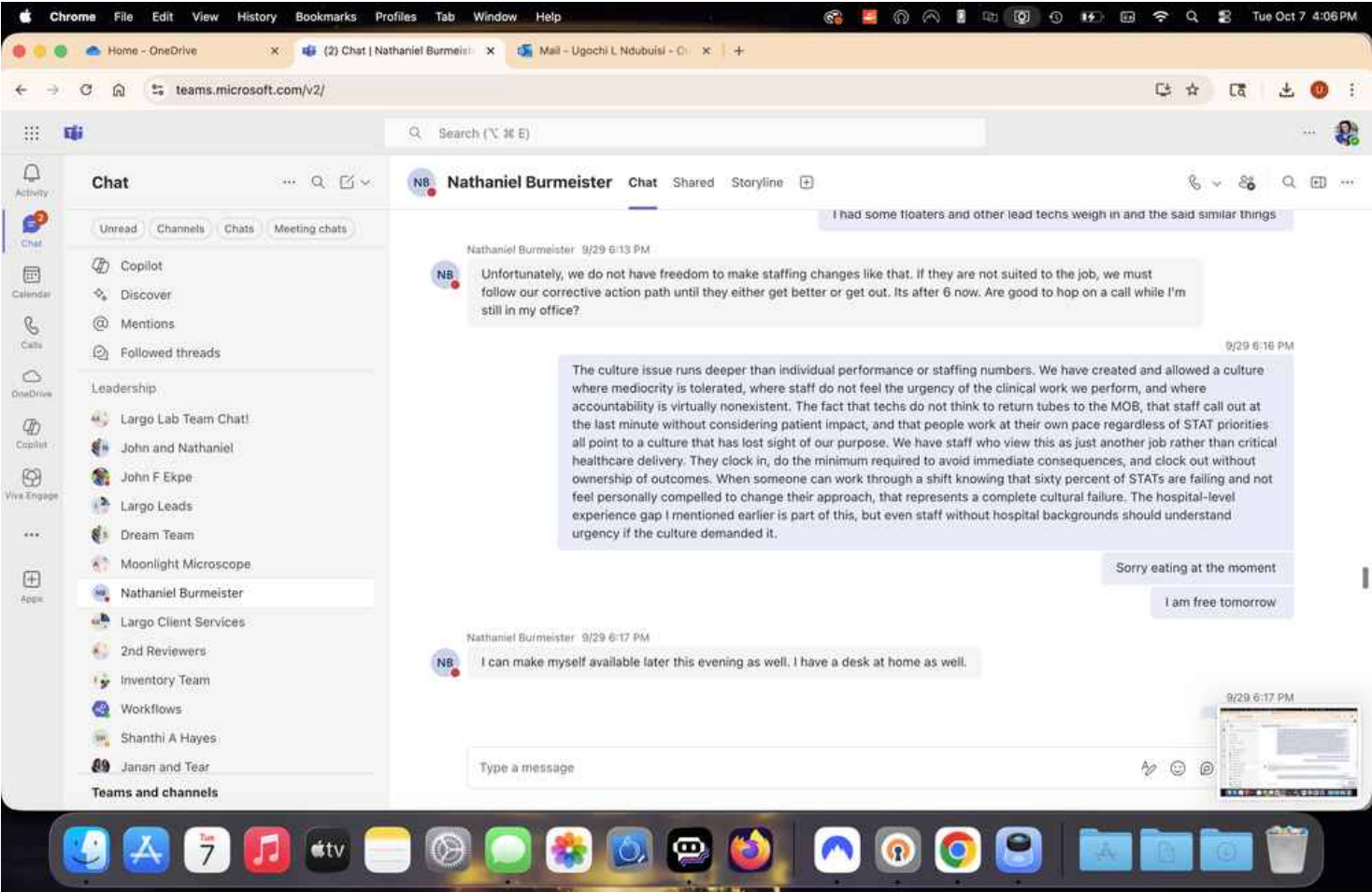




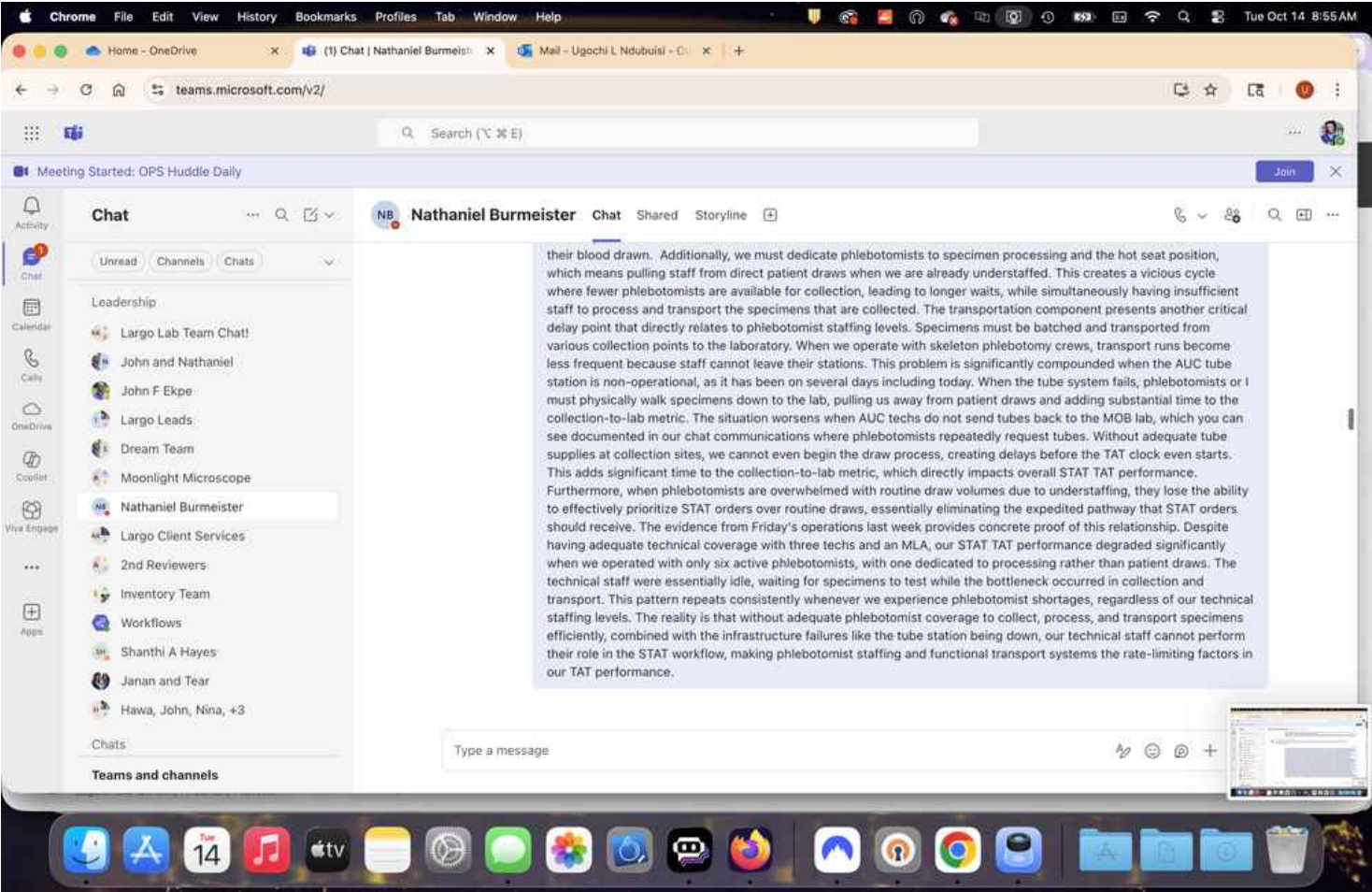




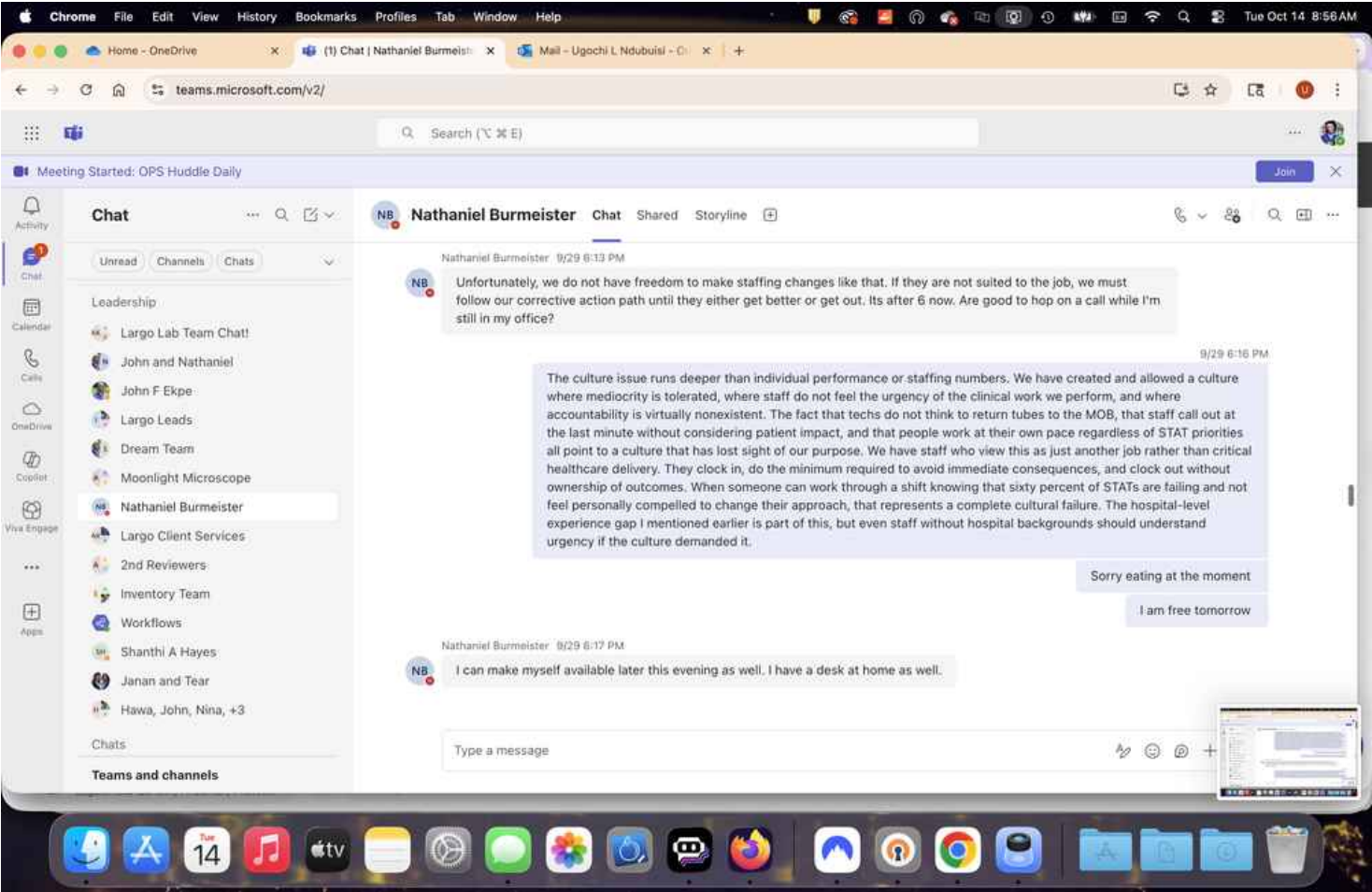


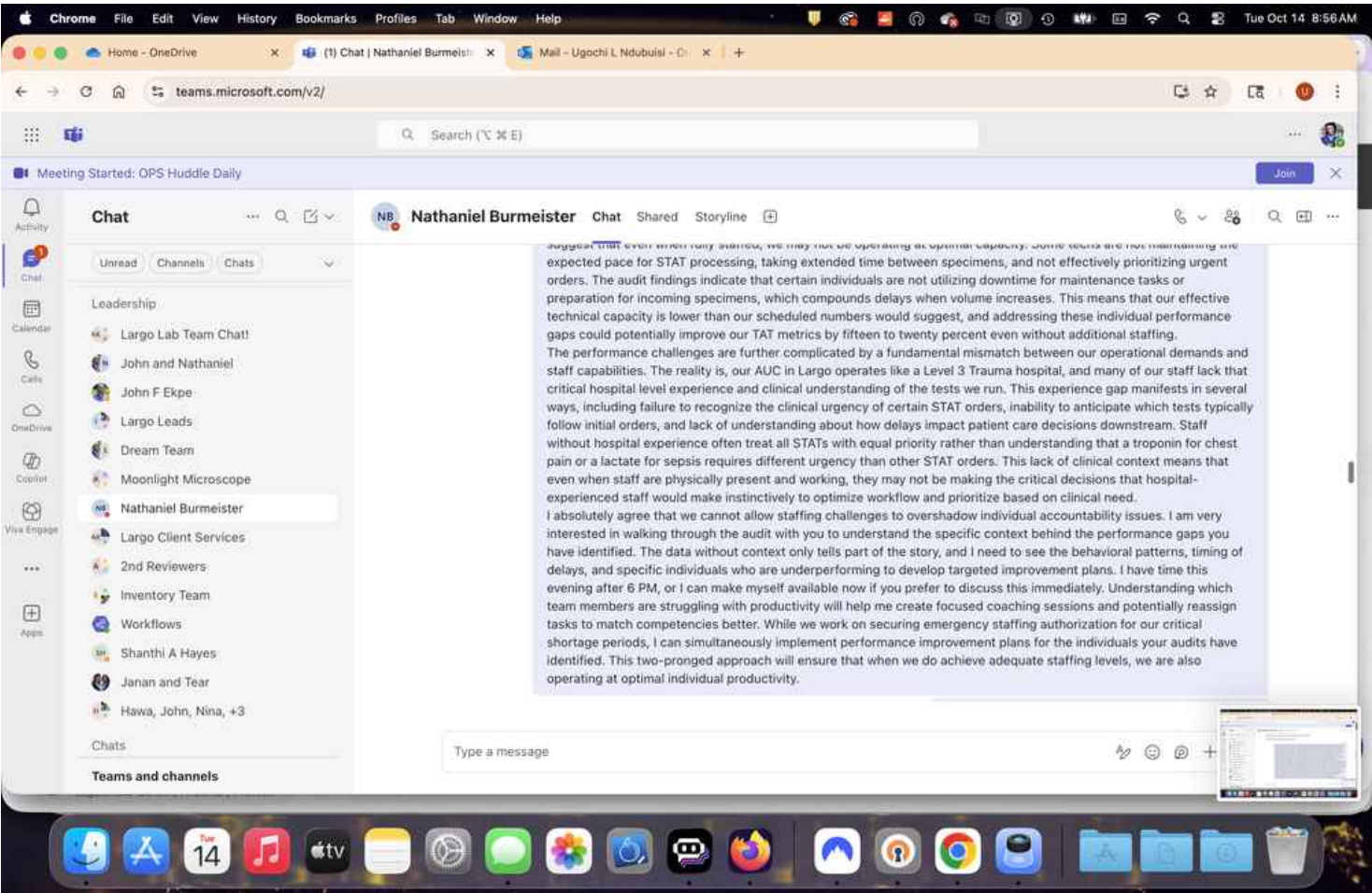


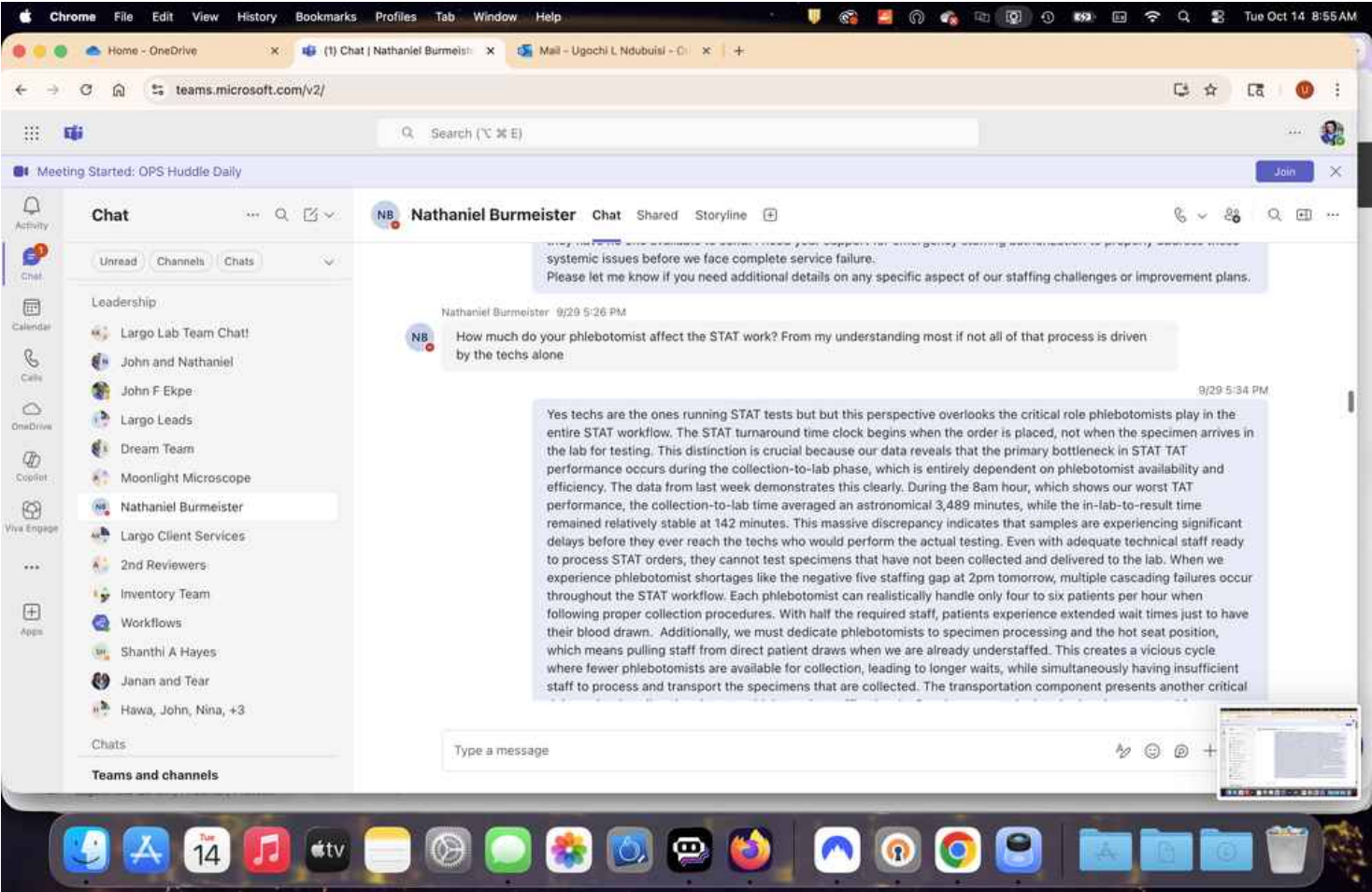
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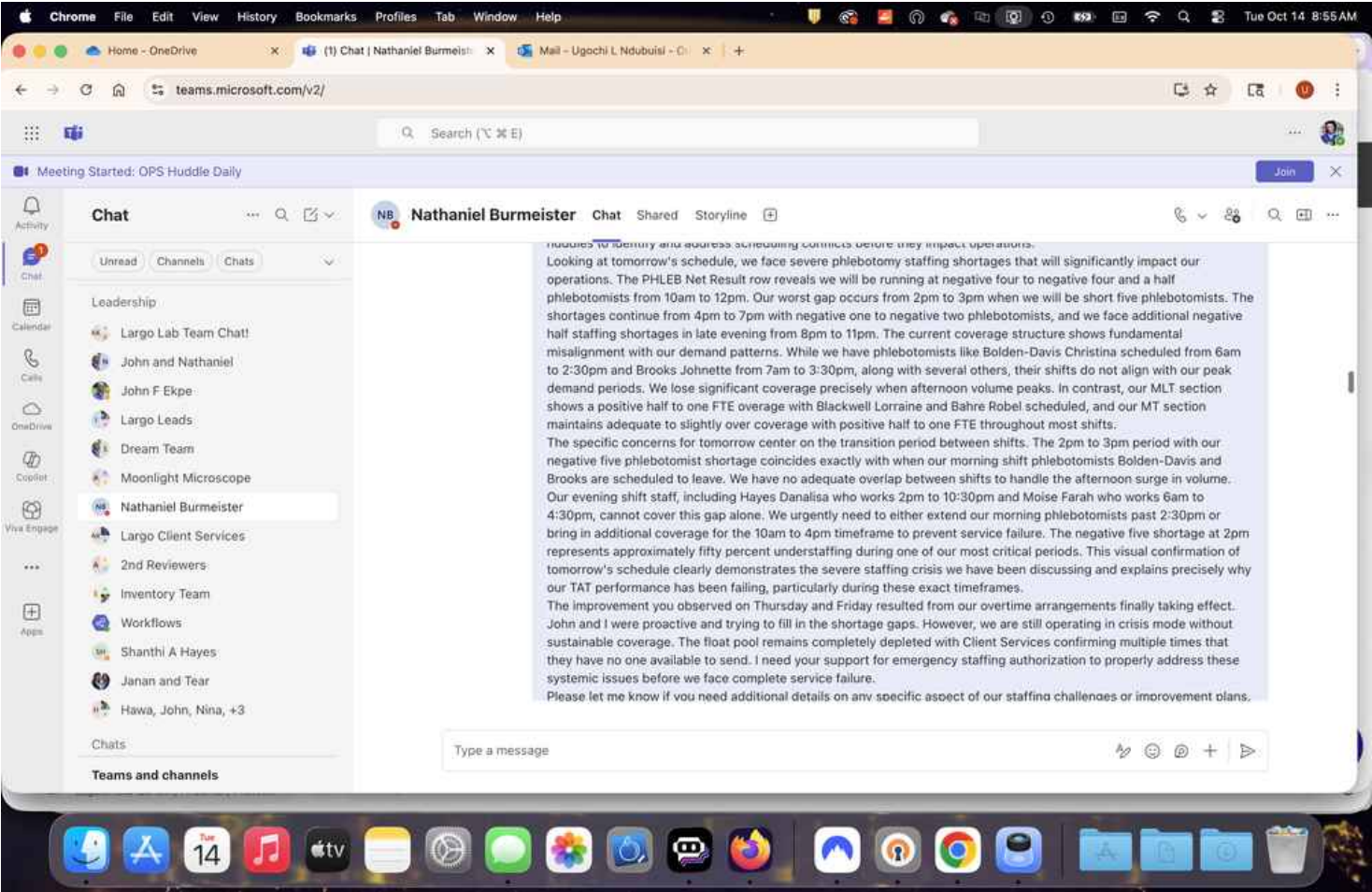
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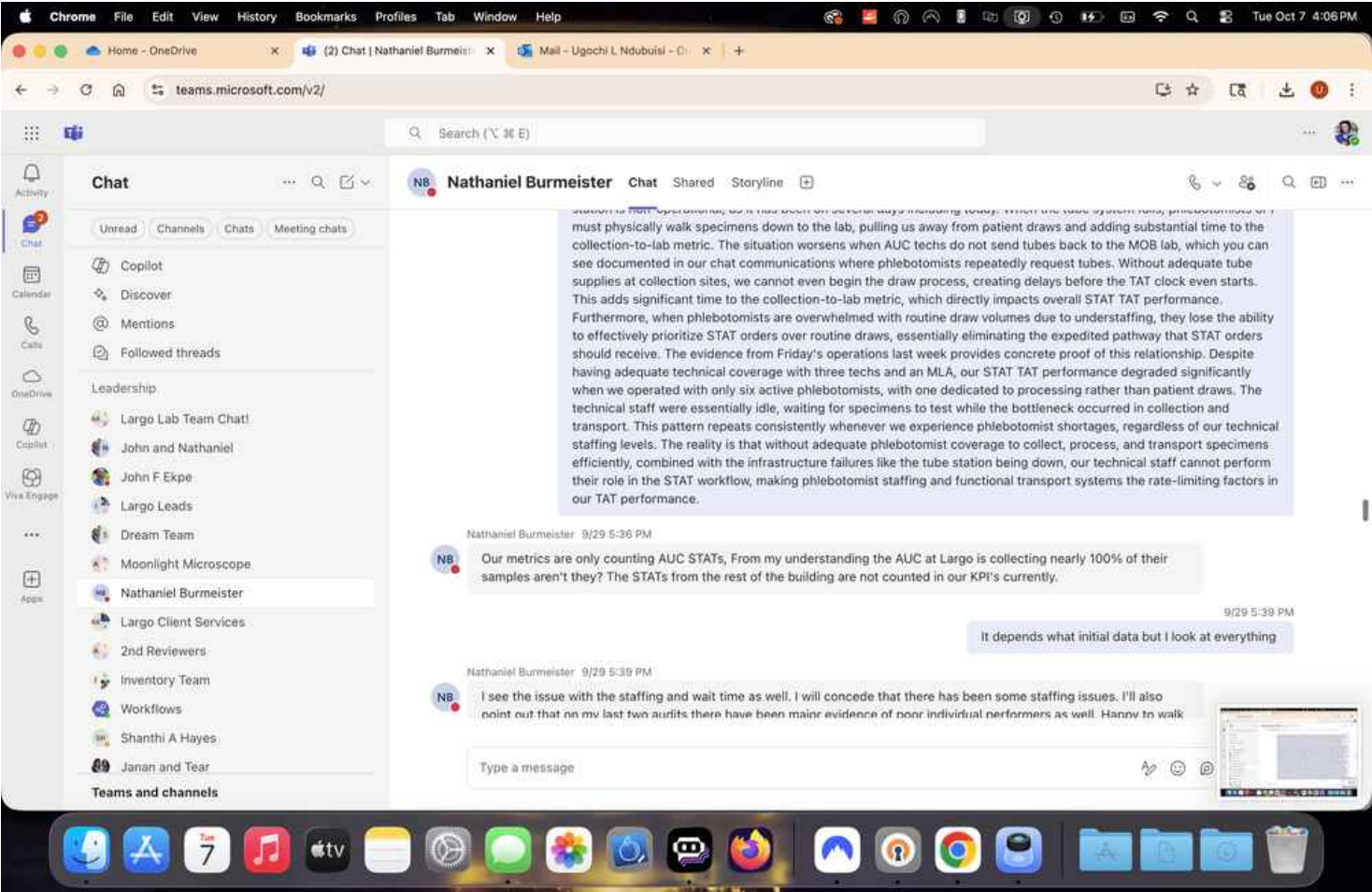


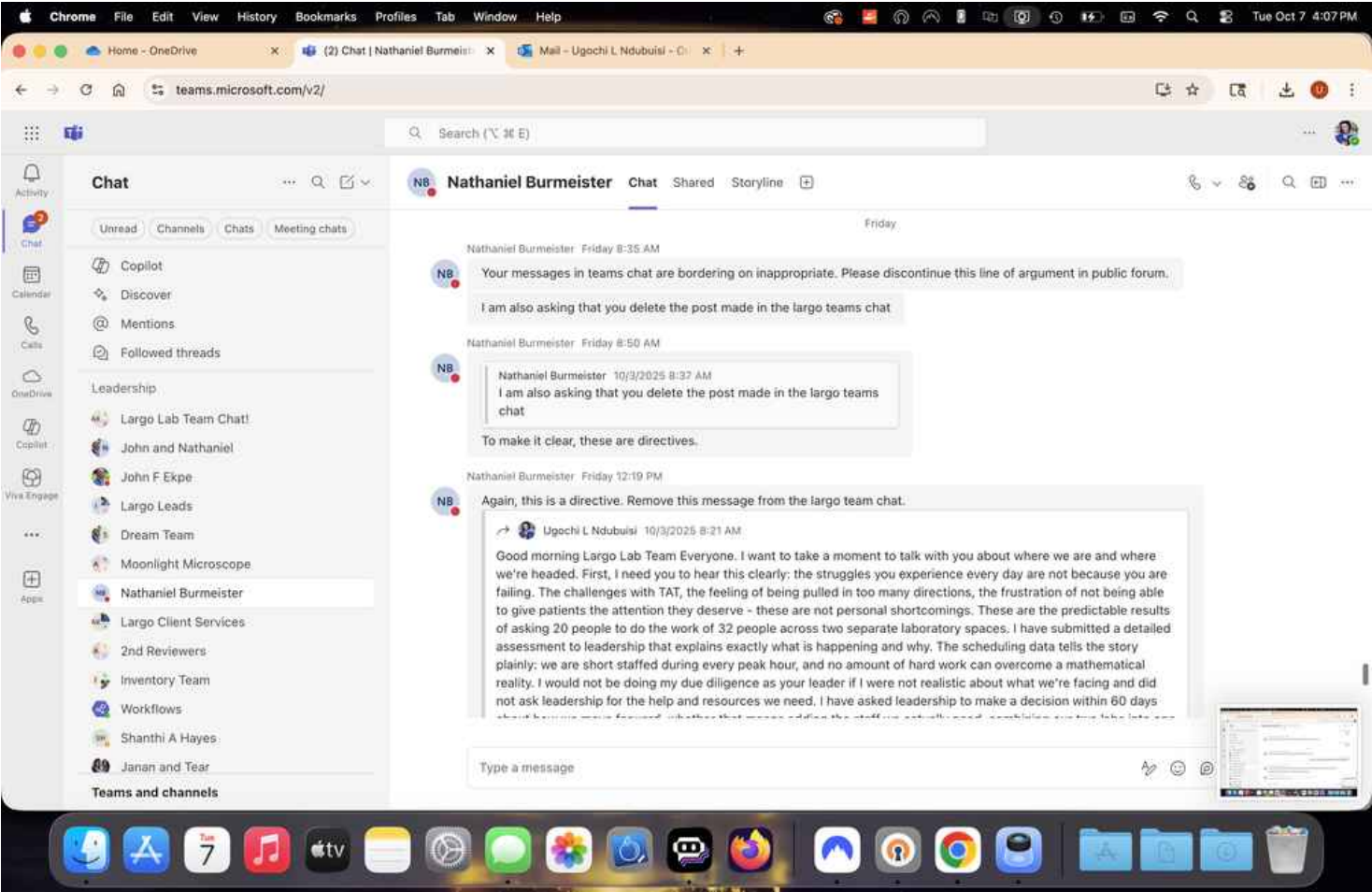


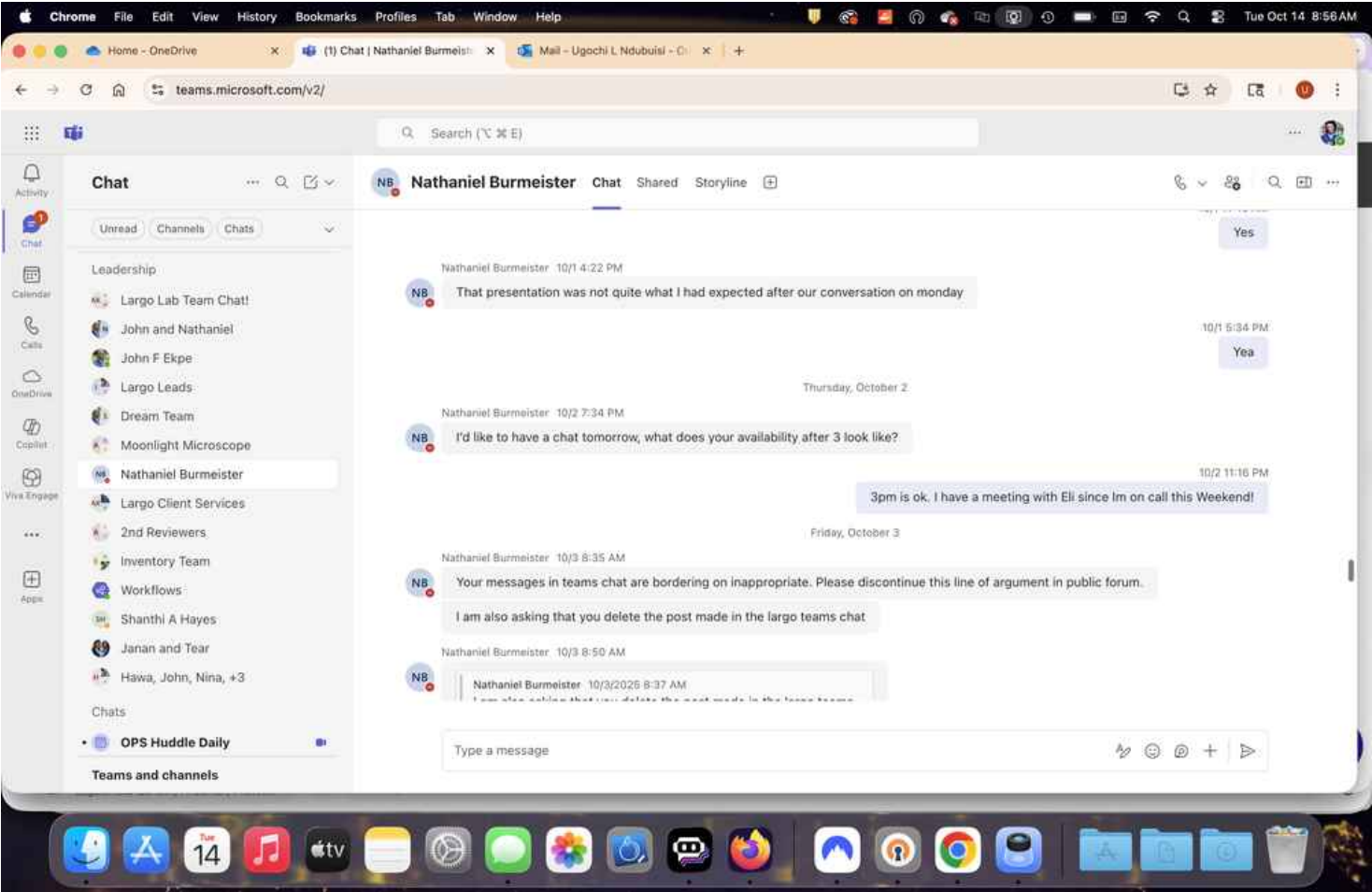


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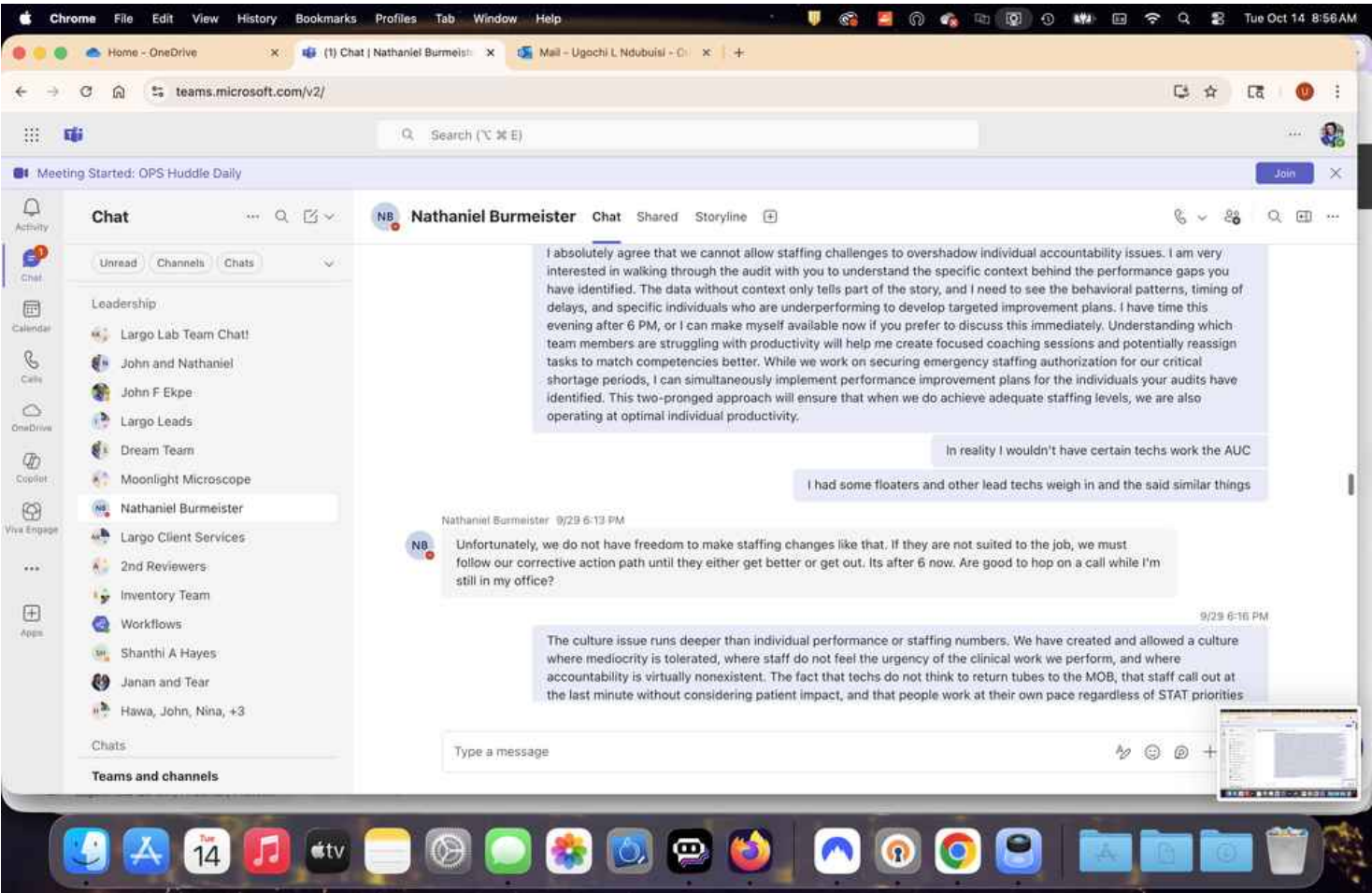


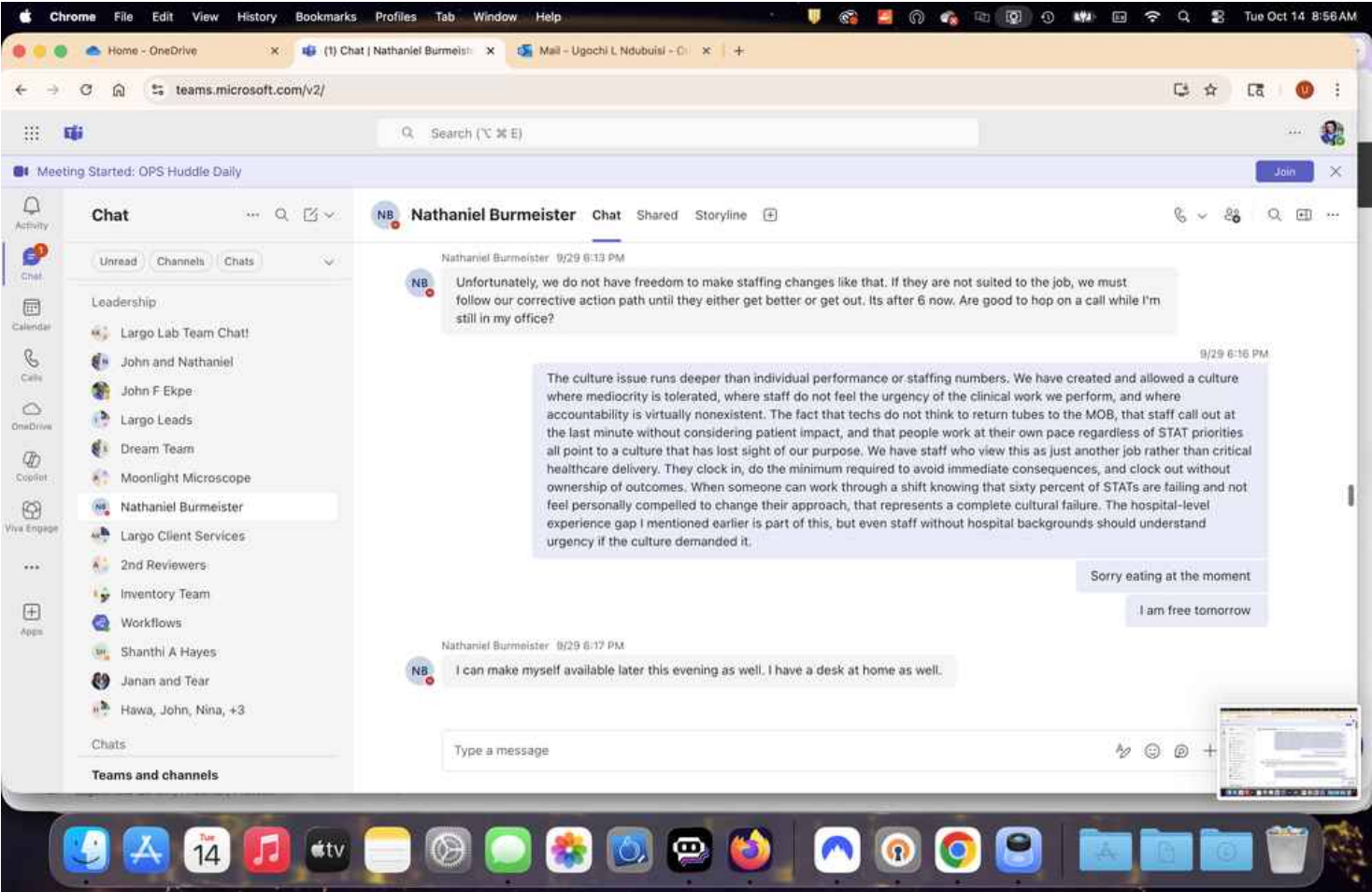


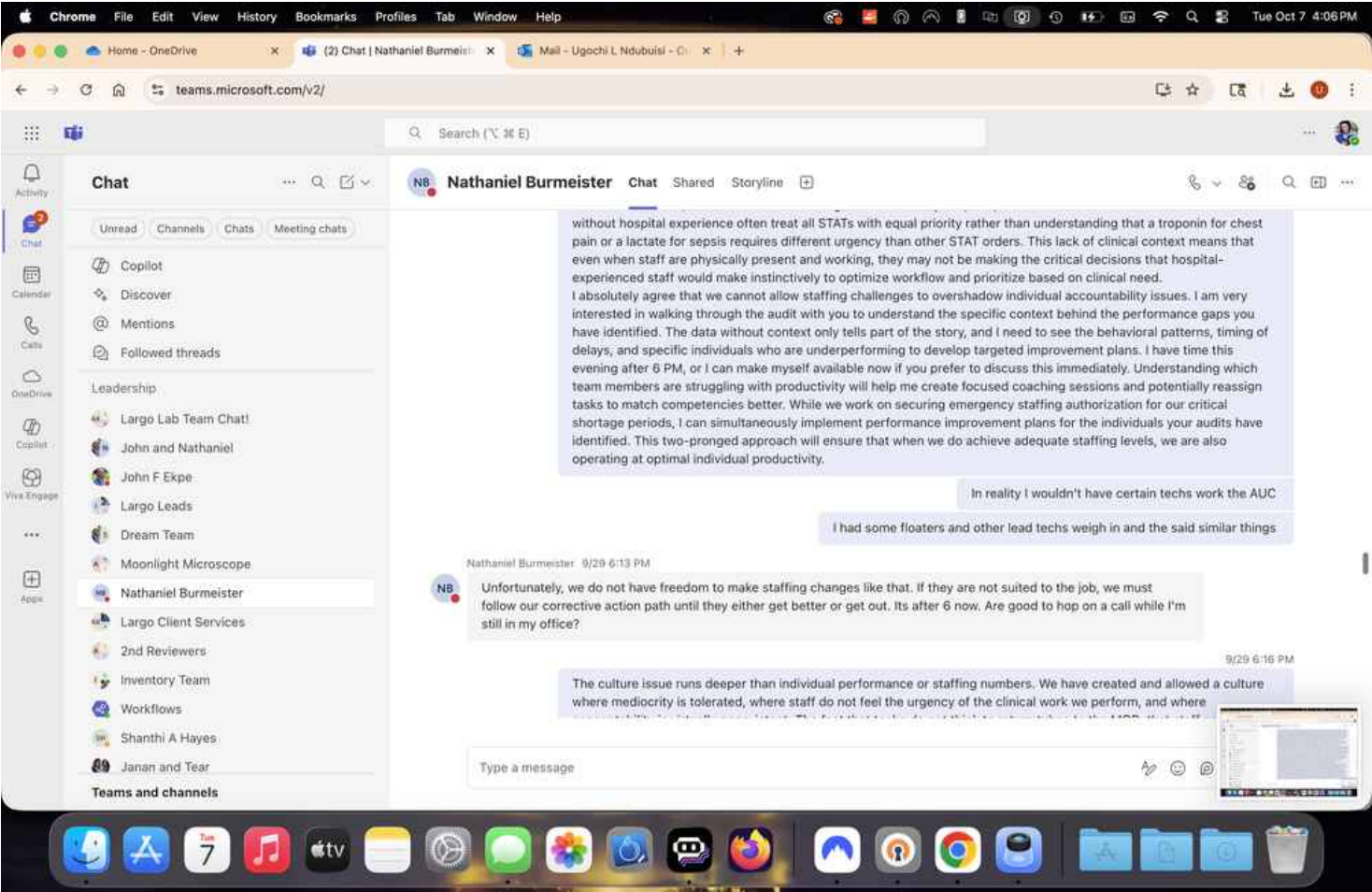


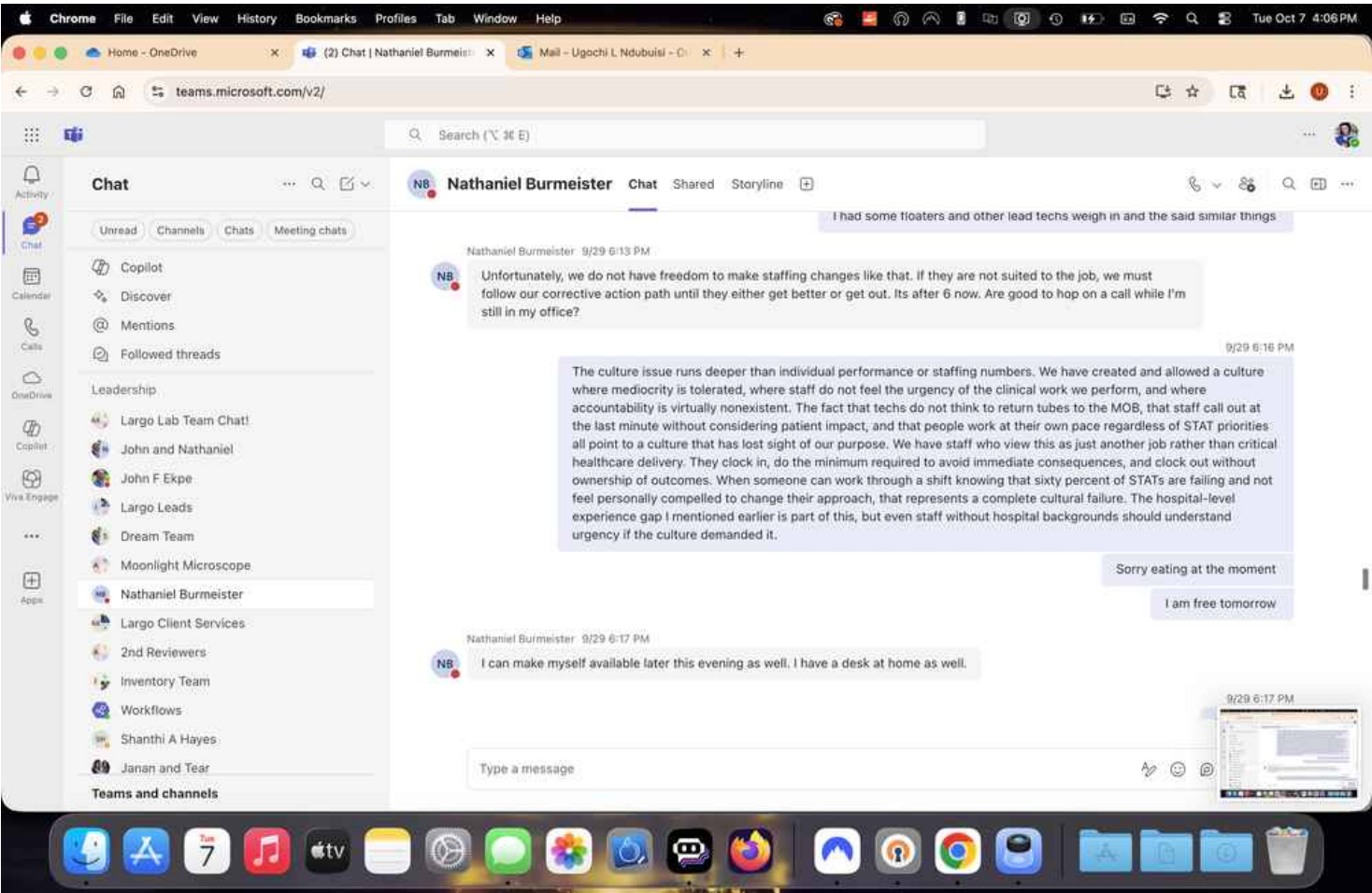


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Largo Laboratory - Kaiser Permanente

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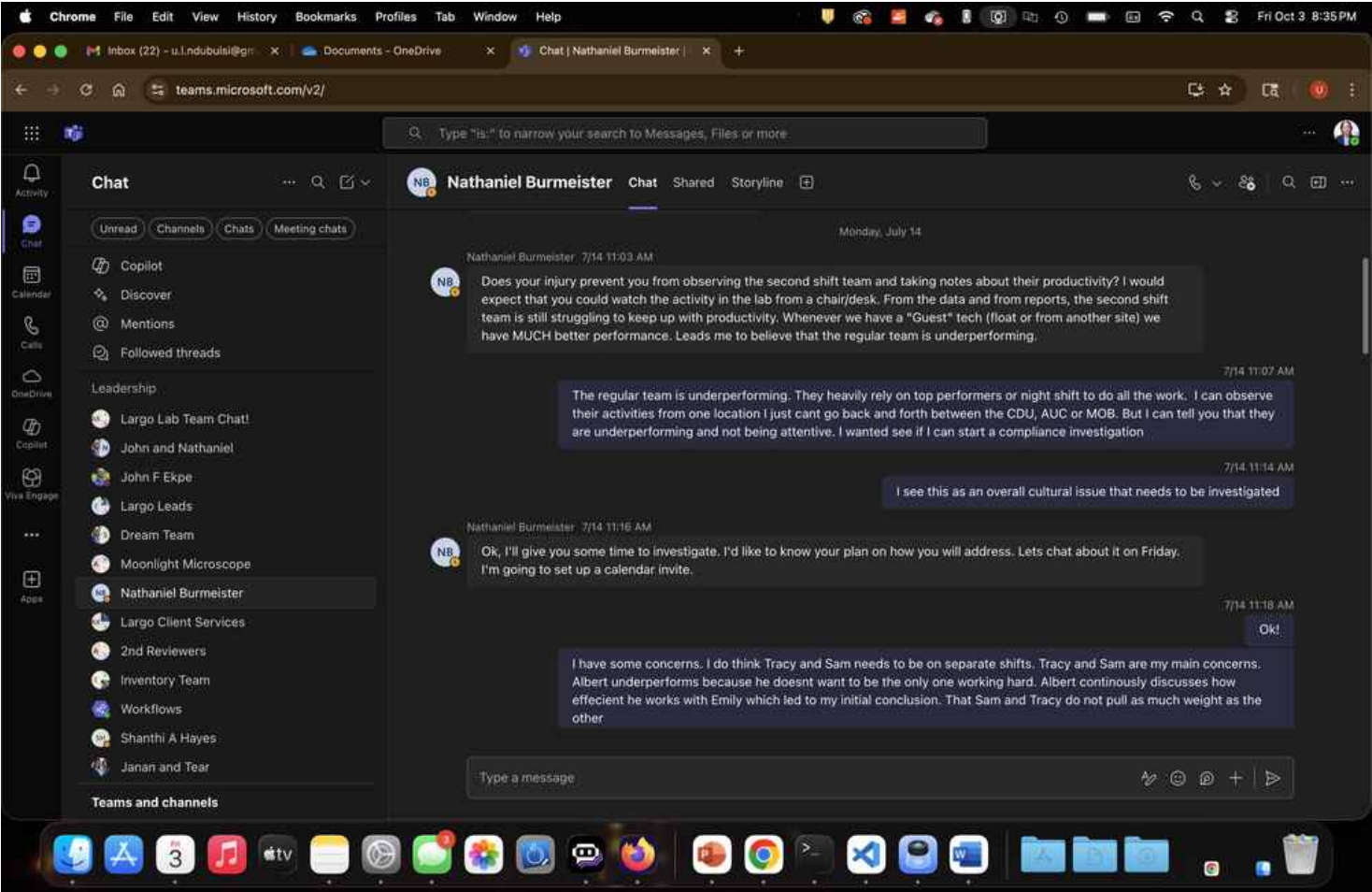
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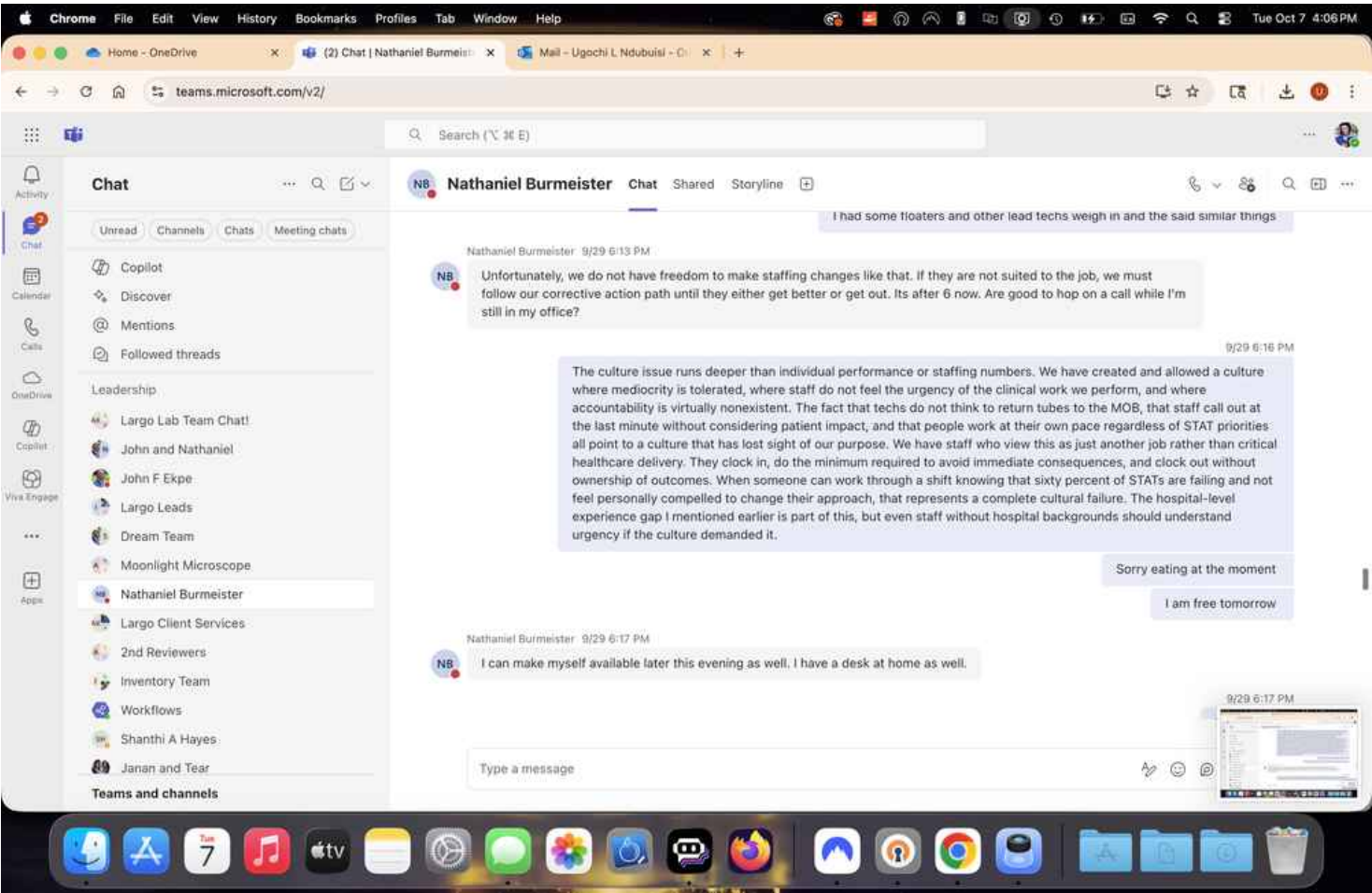
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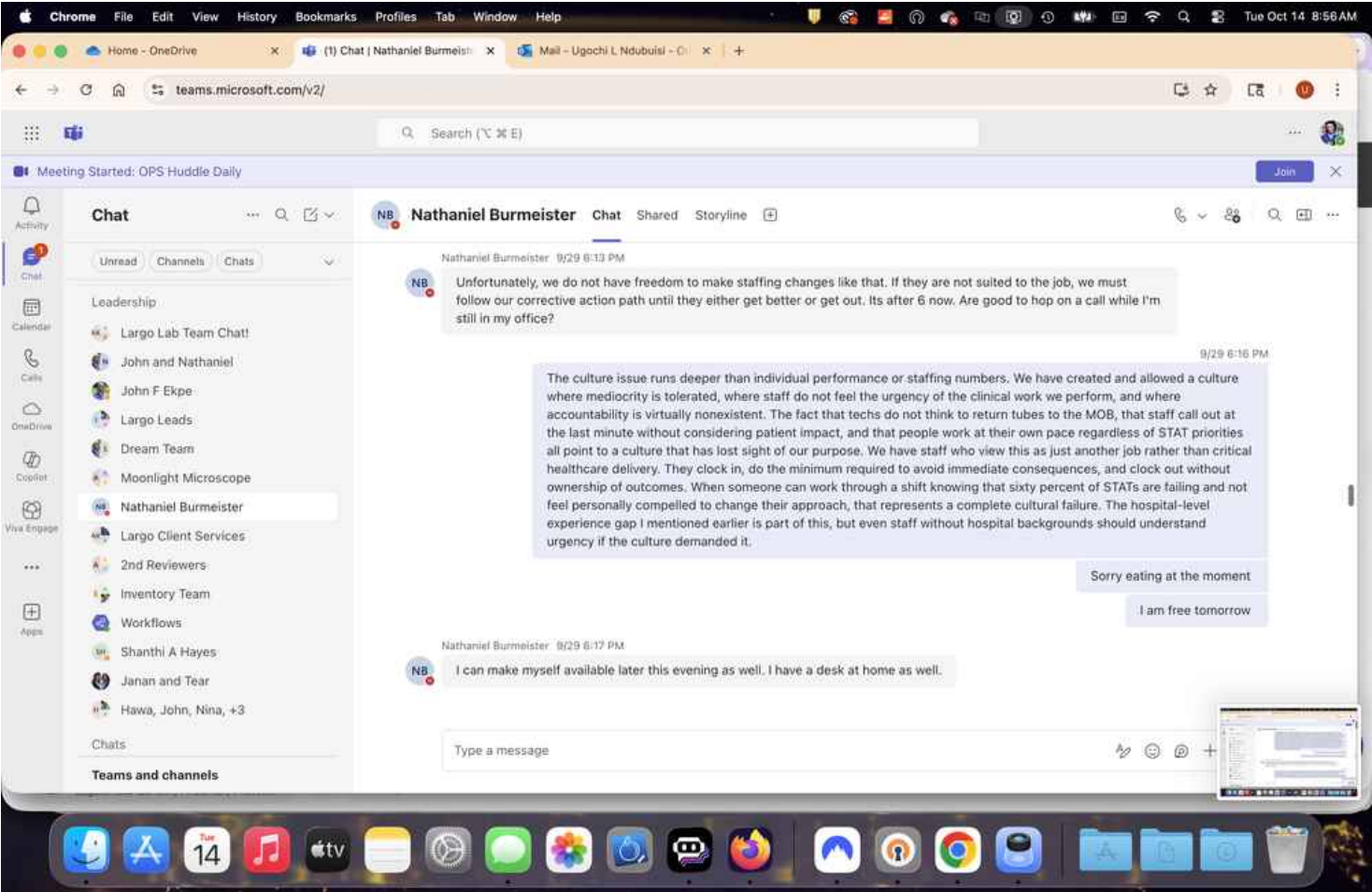
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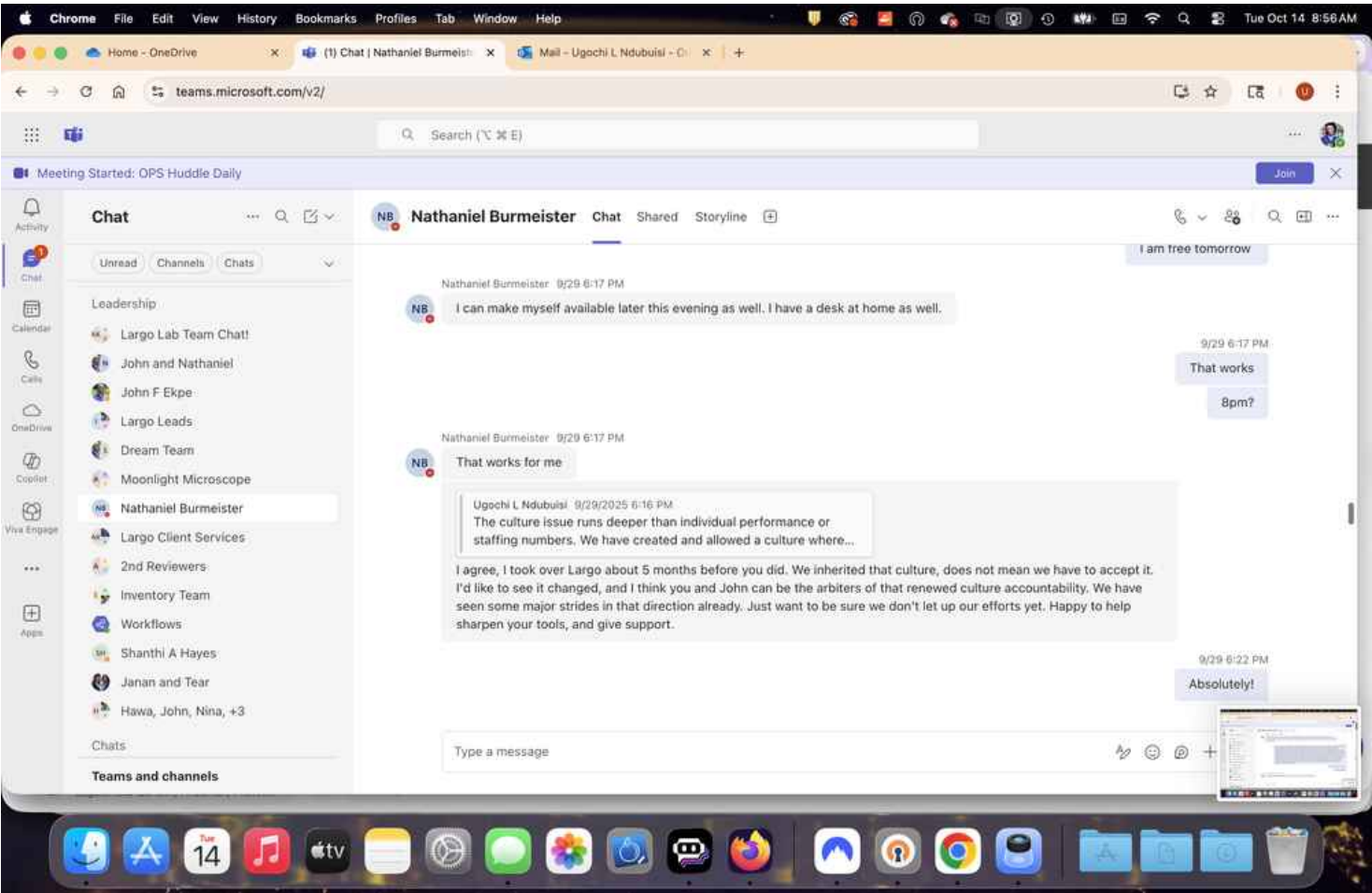


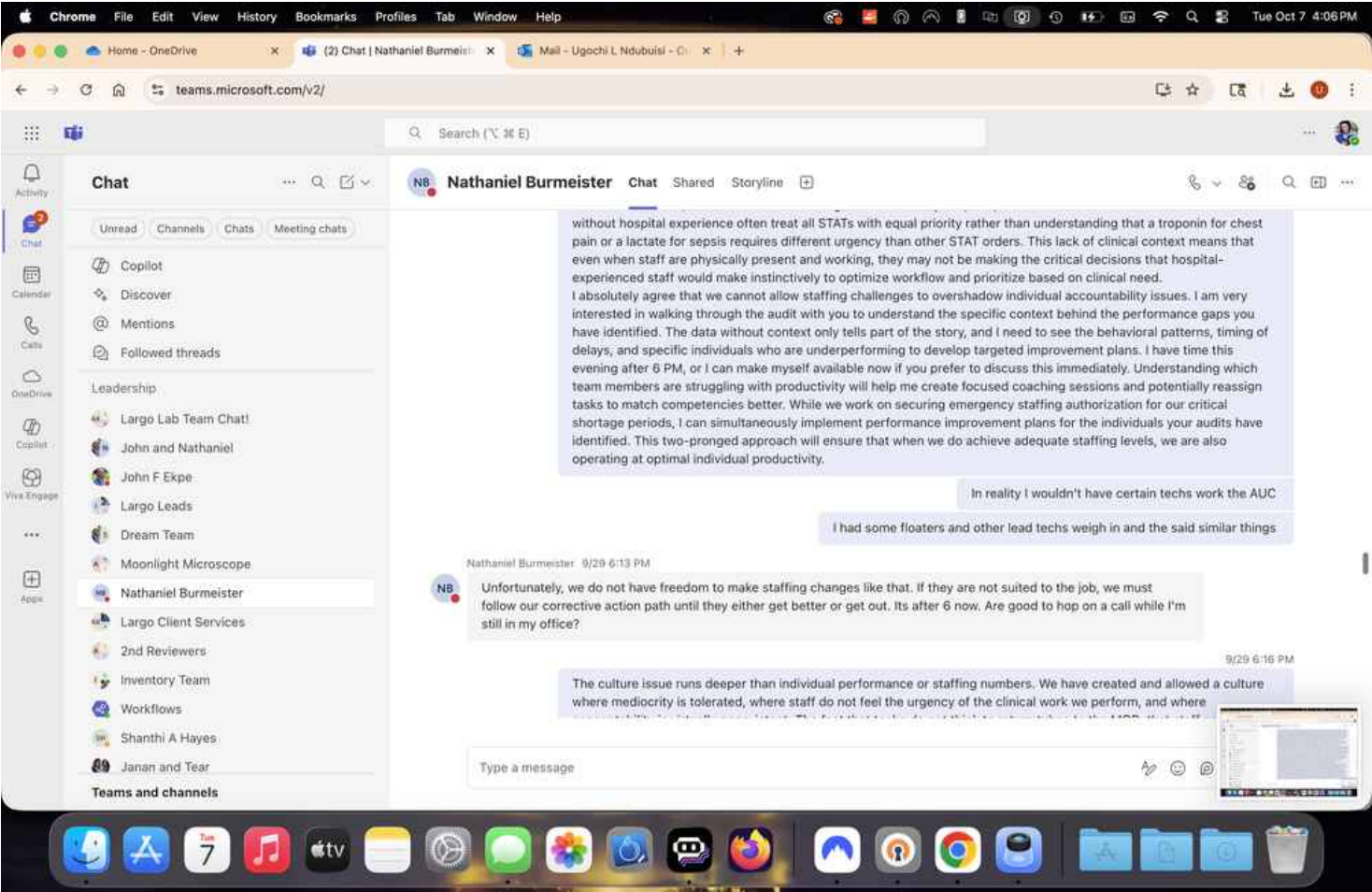
The screenshot displays a Microsoft Teams chat interface on a Mac. The browser window at the top shows the URL 'teams.microsoft.com/v2/'. The chat is with 'Nathaniel Burmeister'. The left sidebar contains navigation icons for Activity, Chat, Calendar, Calls, OneDrive, Copilot, Viva Engage, and Apps. The chat history shows messages from Wednesday, September 17, to Monday, September 29. A message from Nathaniel Burmeister on Monday says: 'Hello Ugochi, I know you are just getting settled back. Seeing a little bit of decline/stalled progress on our Largo 2nd shift teams performance over the past few weeks. Can you share what you've been up to on addressing it?'. Below this message is a bar chart titled 'Chart from last week' showing performance data across different shifts and days. The chat input field at the bottom says 'Type a message'.

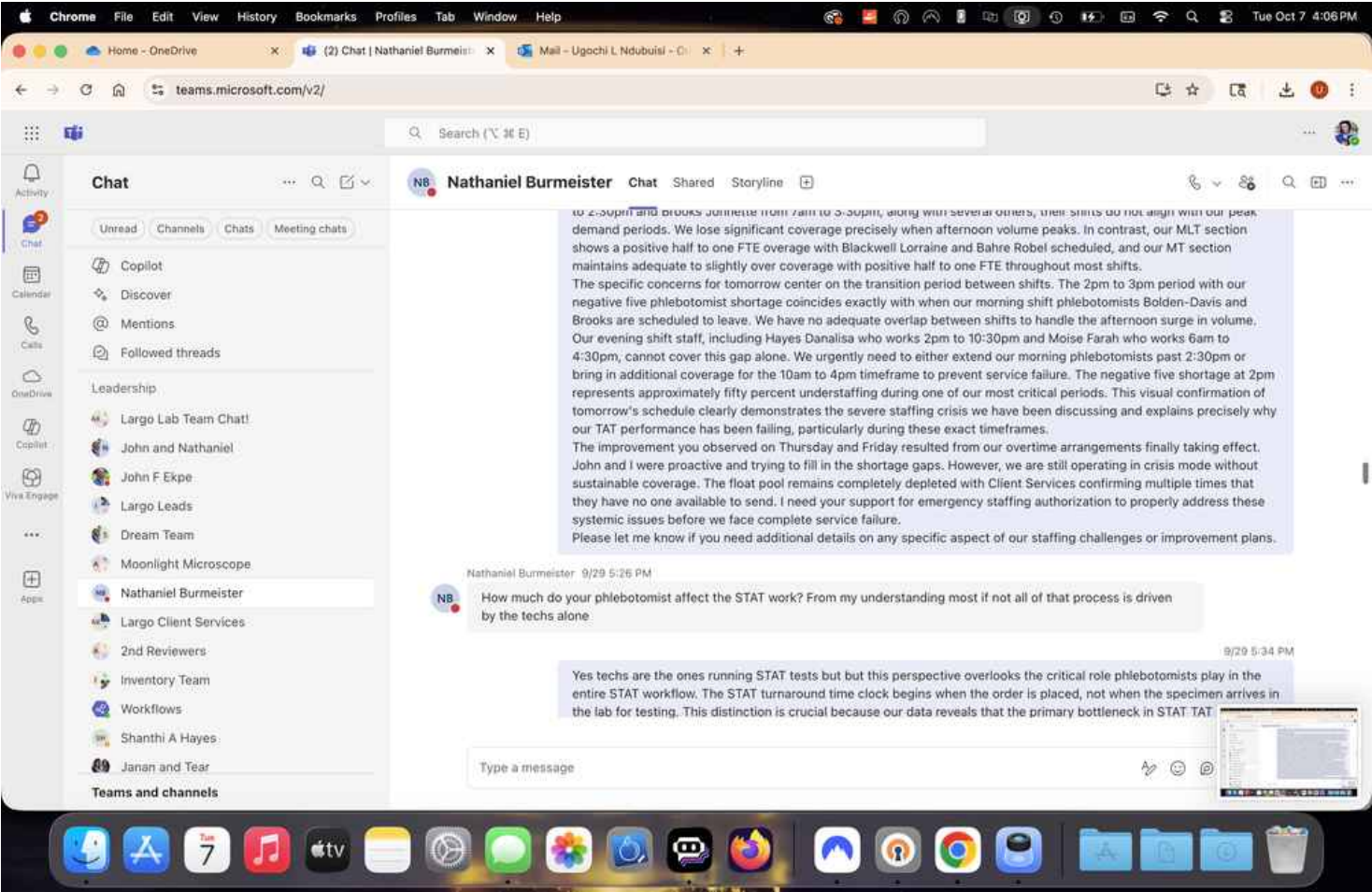


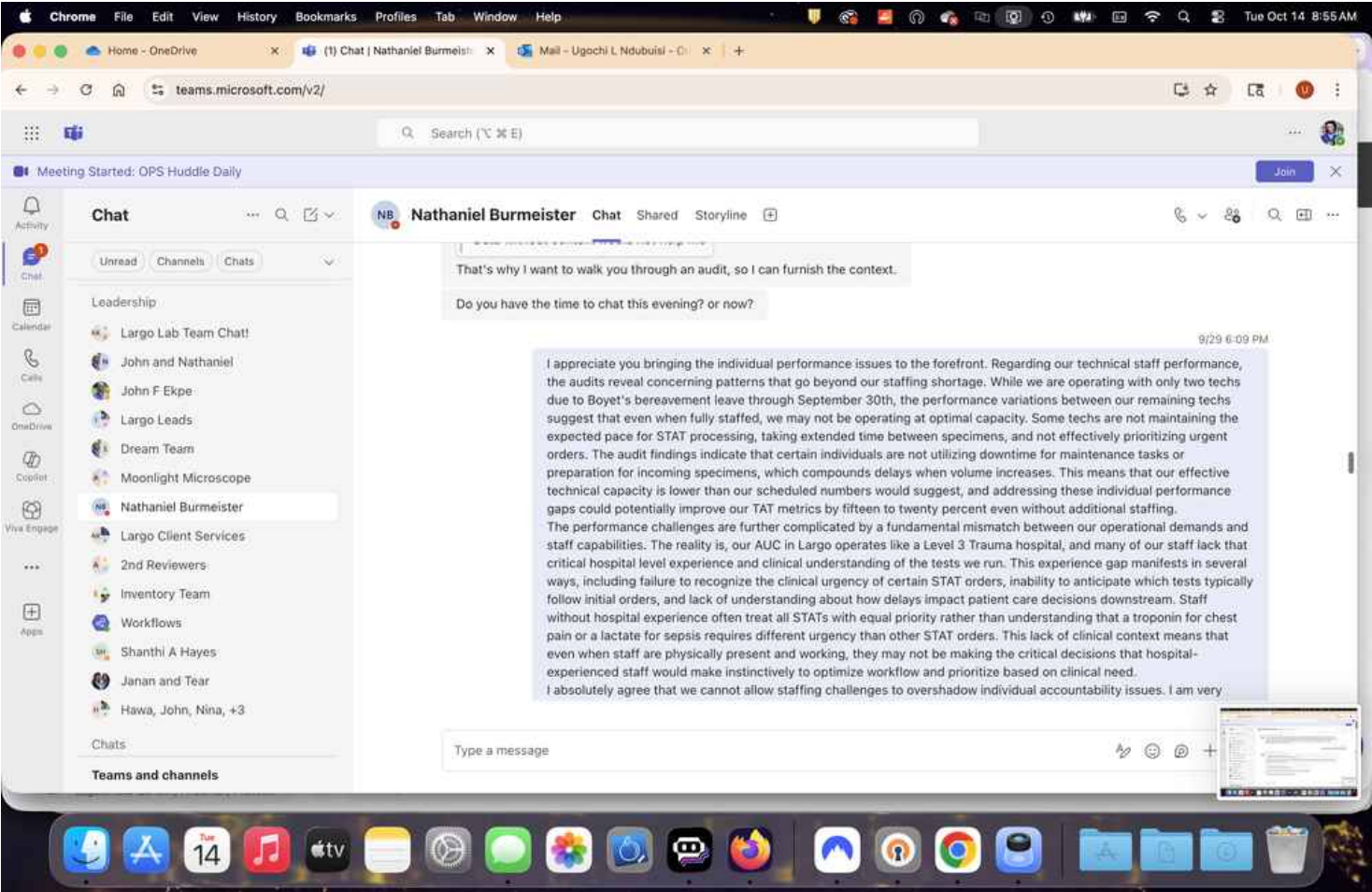
Evidence 4: Sept_29_Analysis_01_Screenshot 2025-10-14 at 8.56.10AM

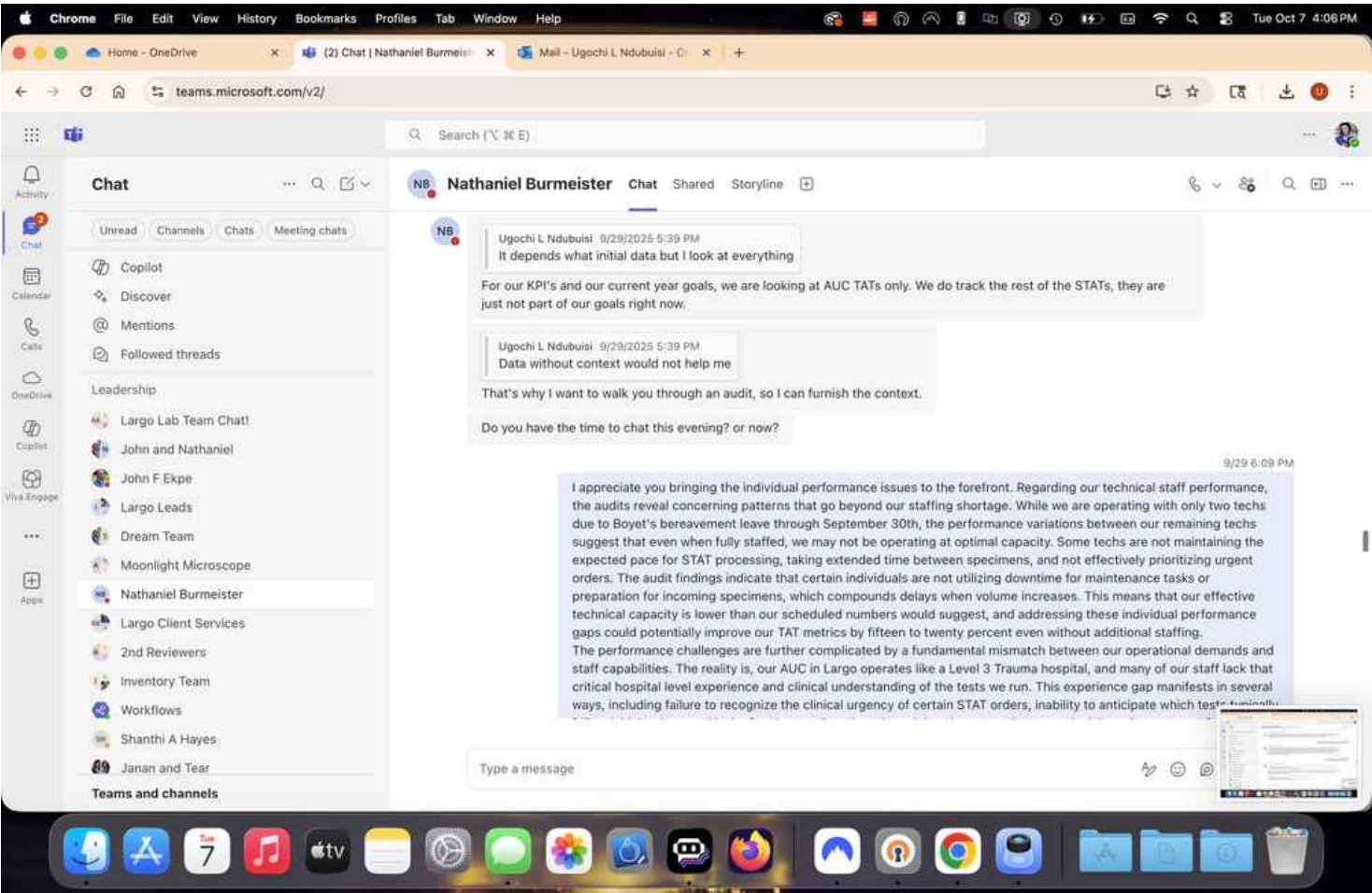


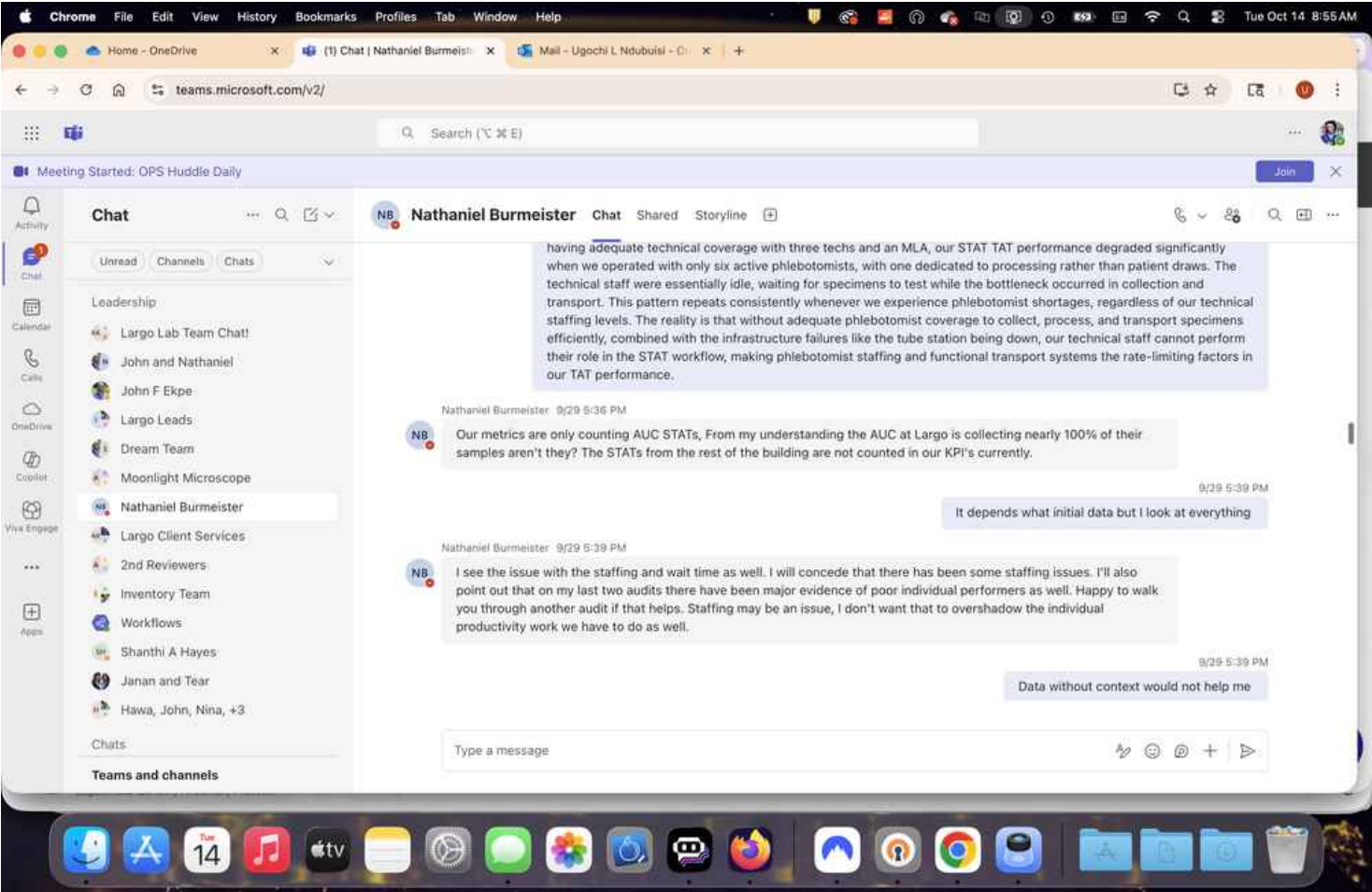


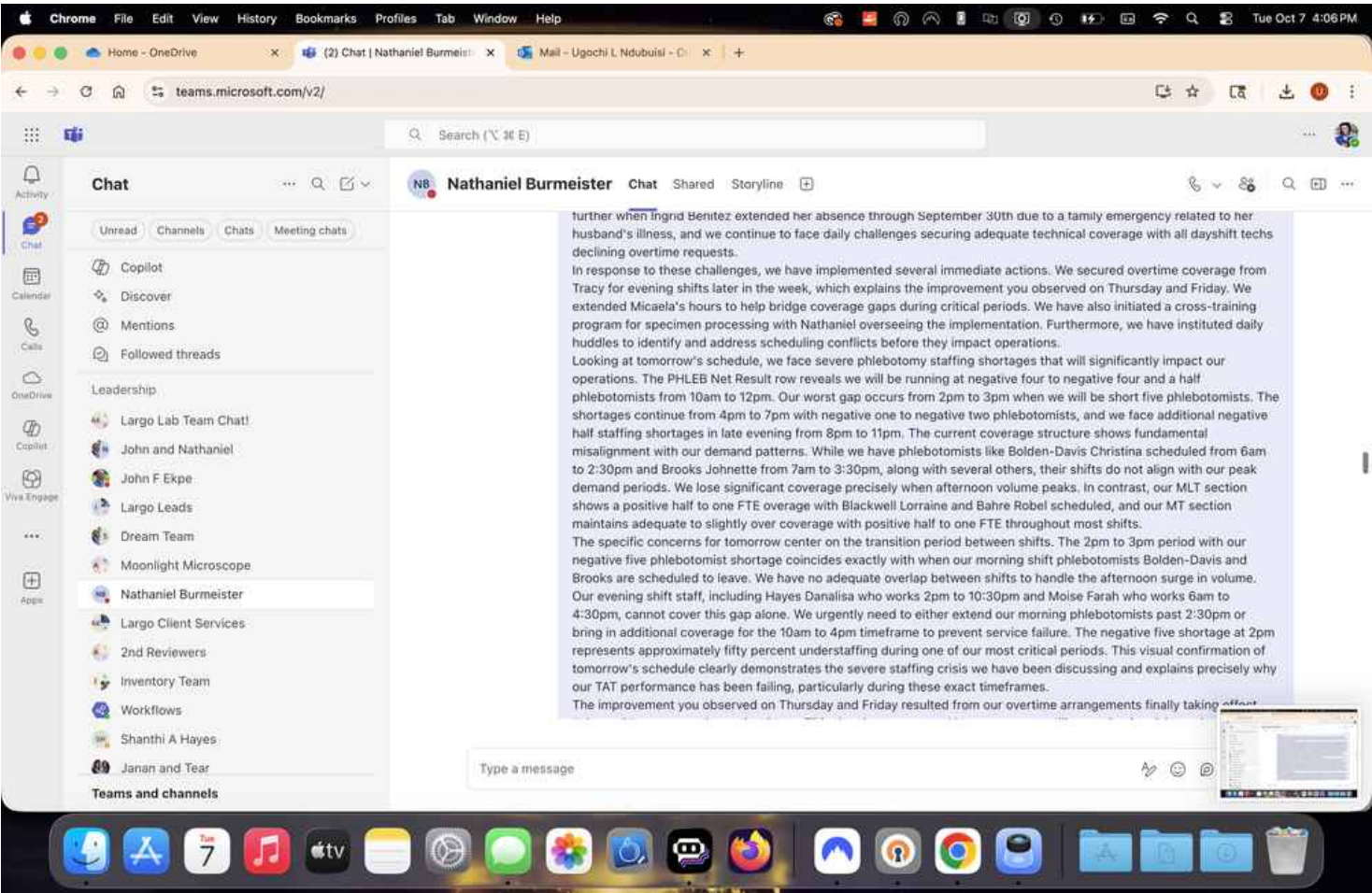


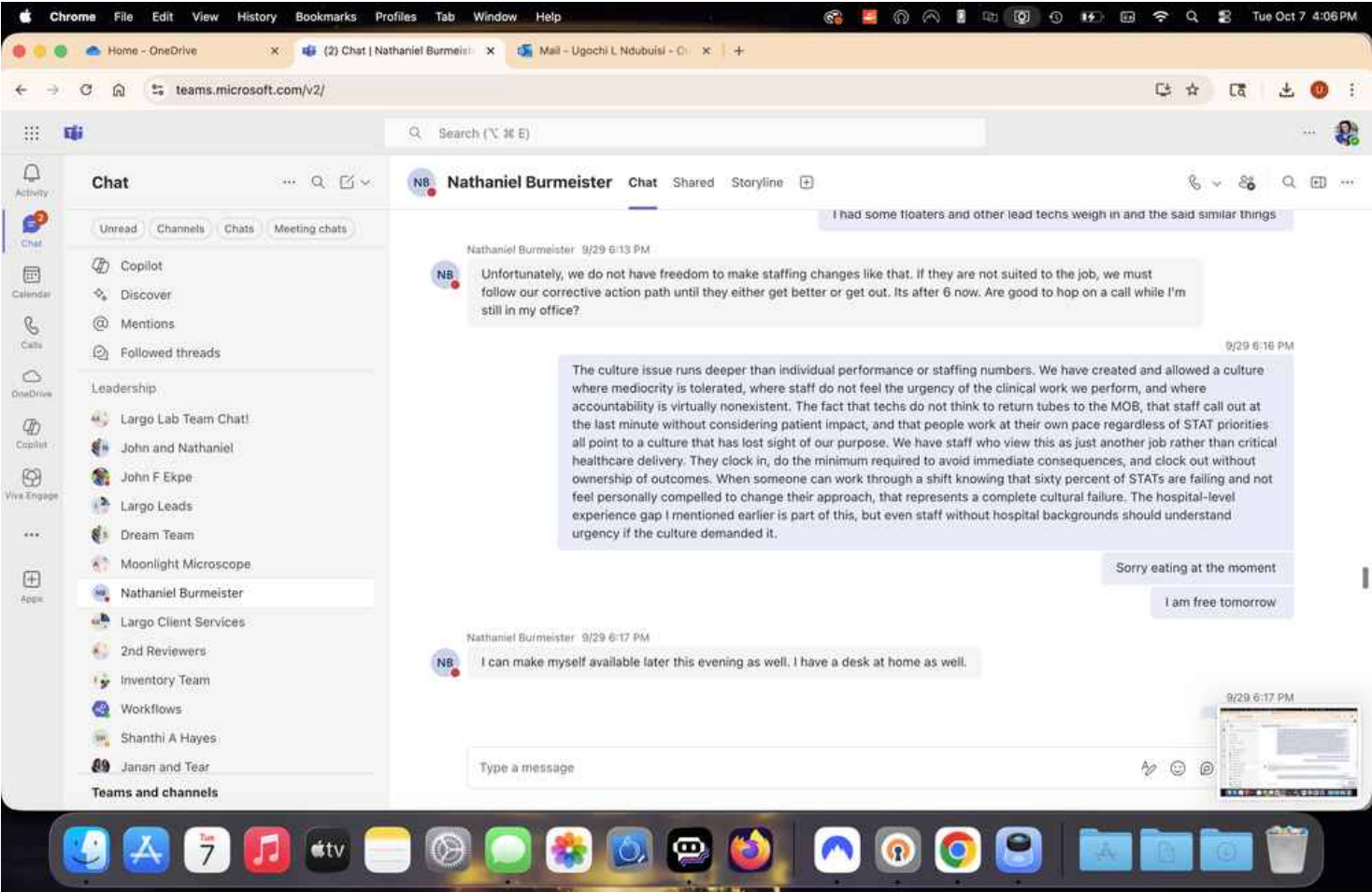




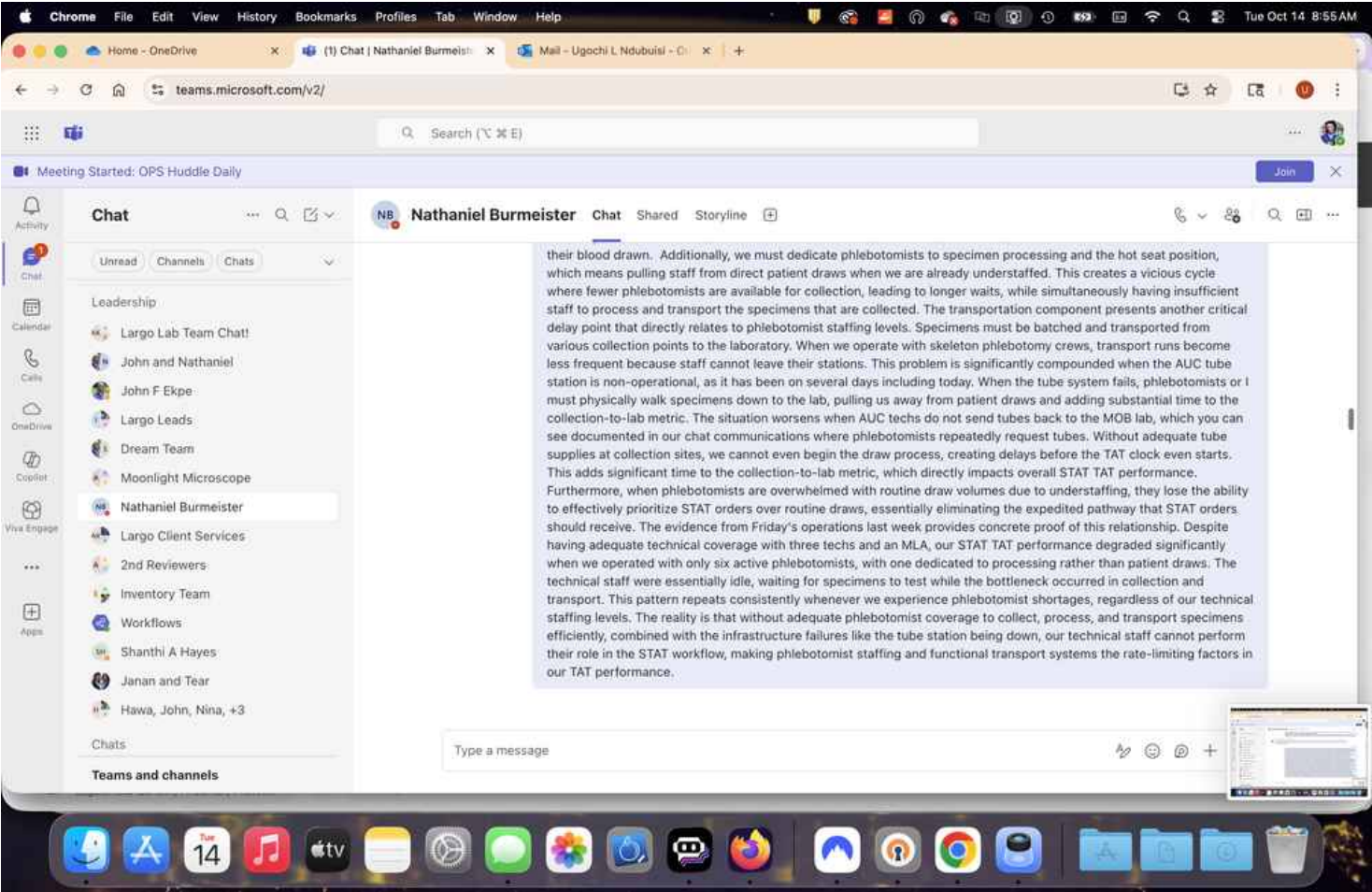




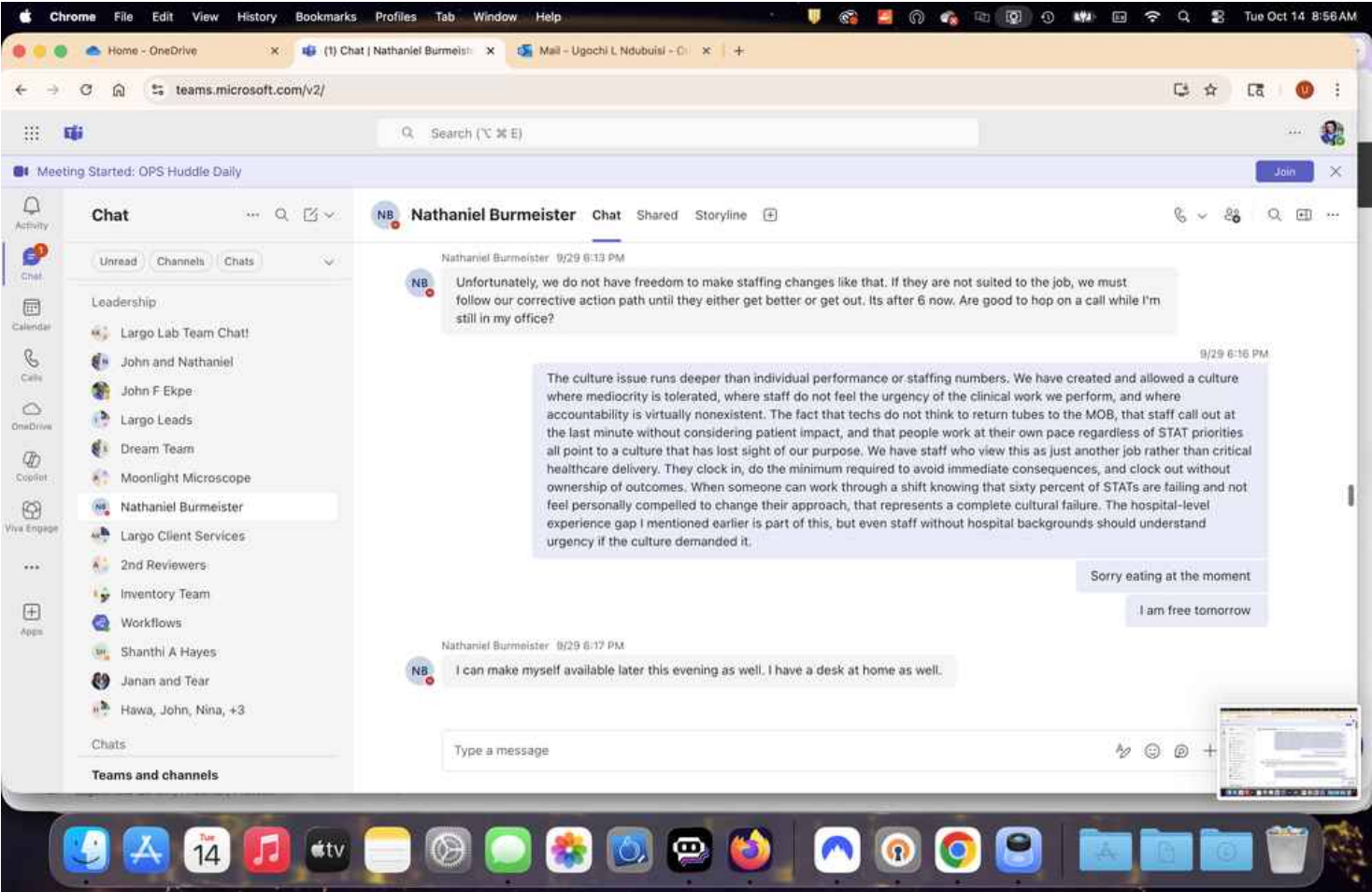


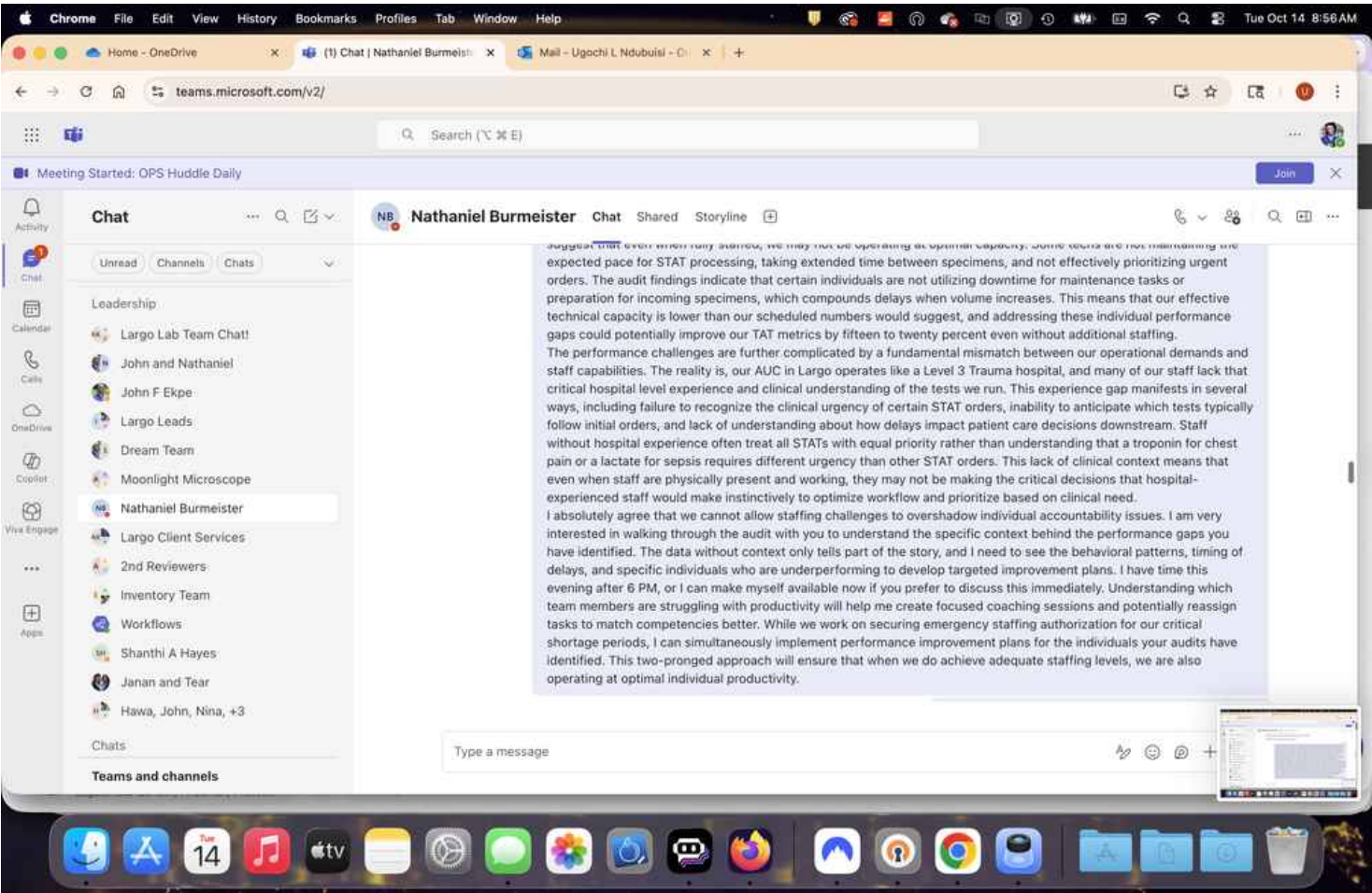


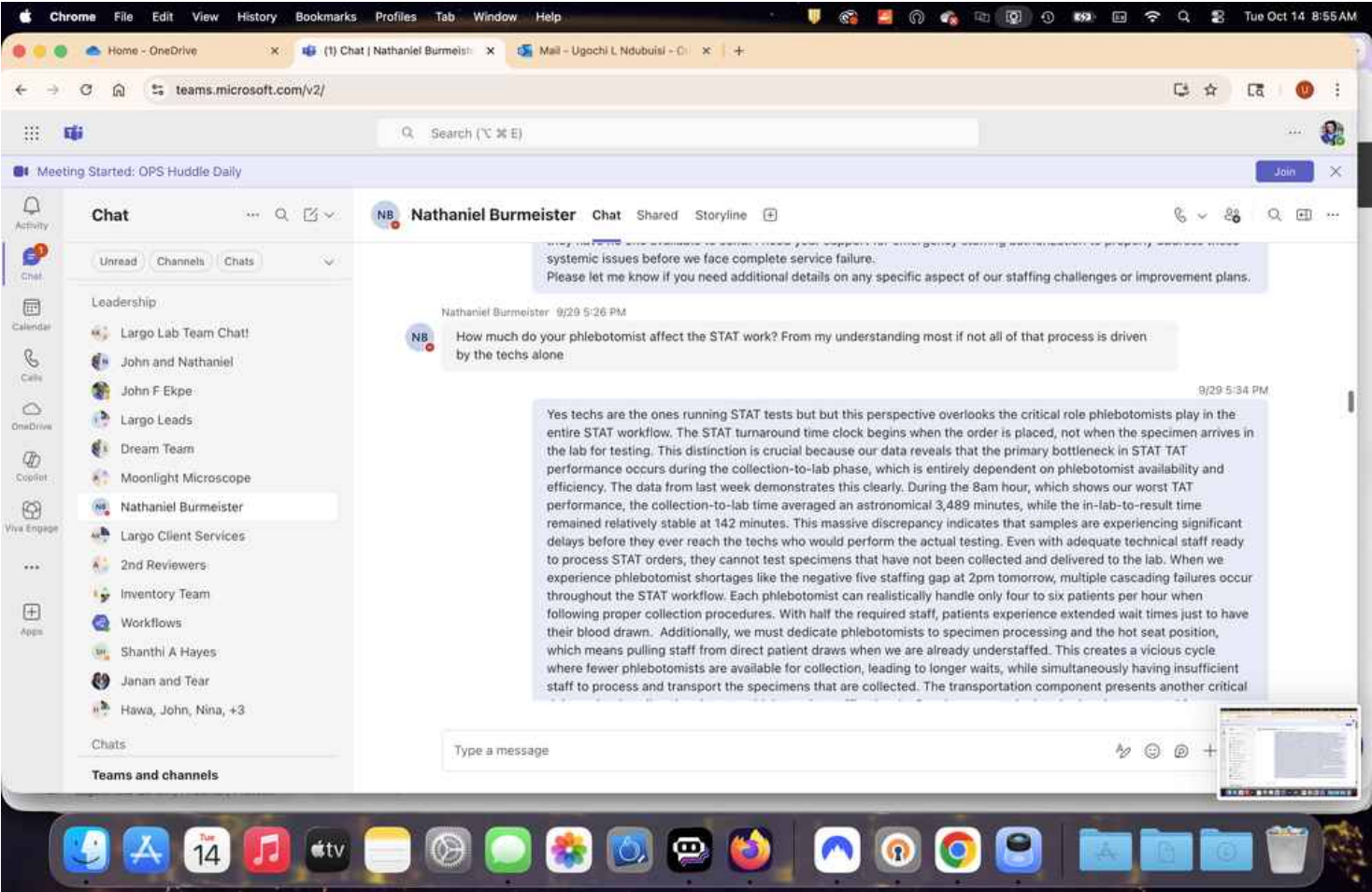
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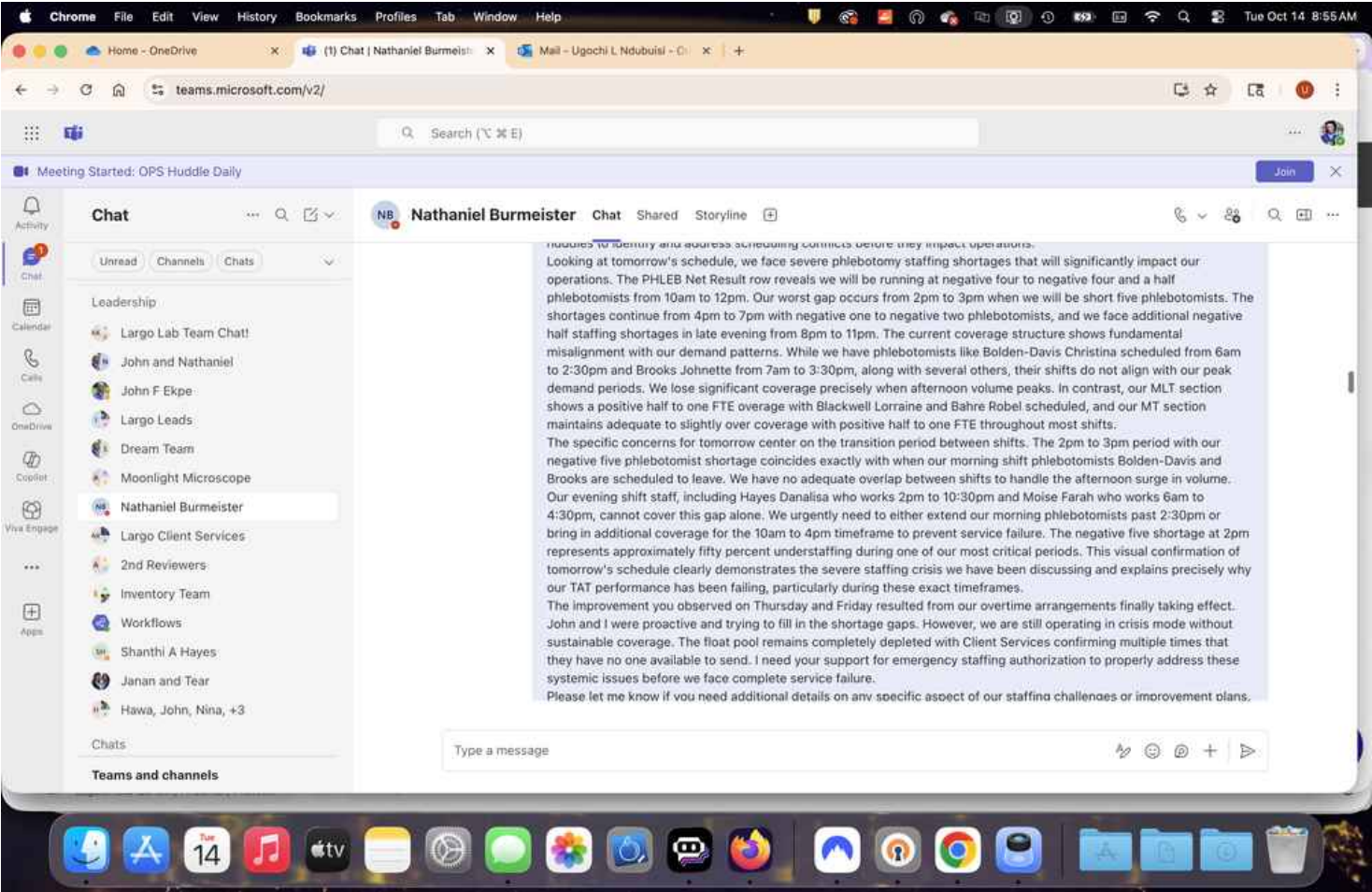
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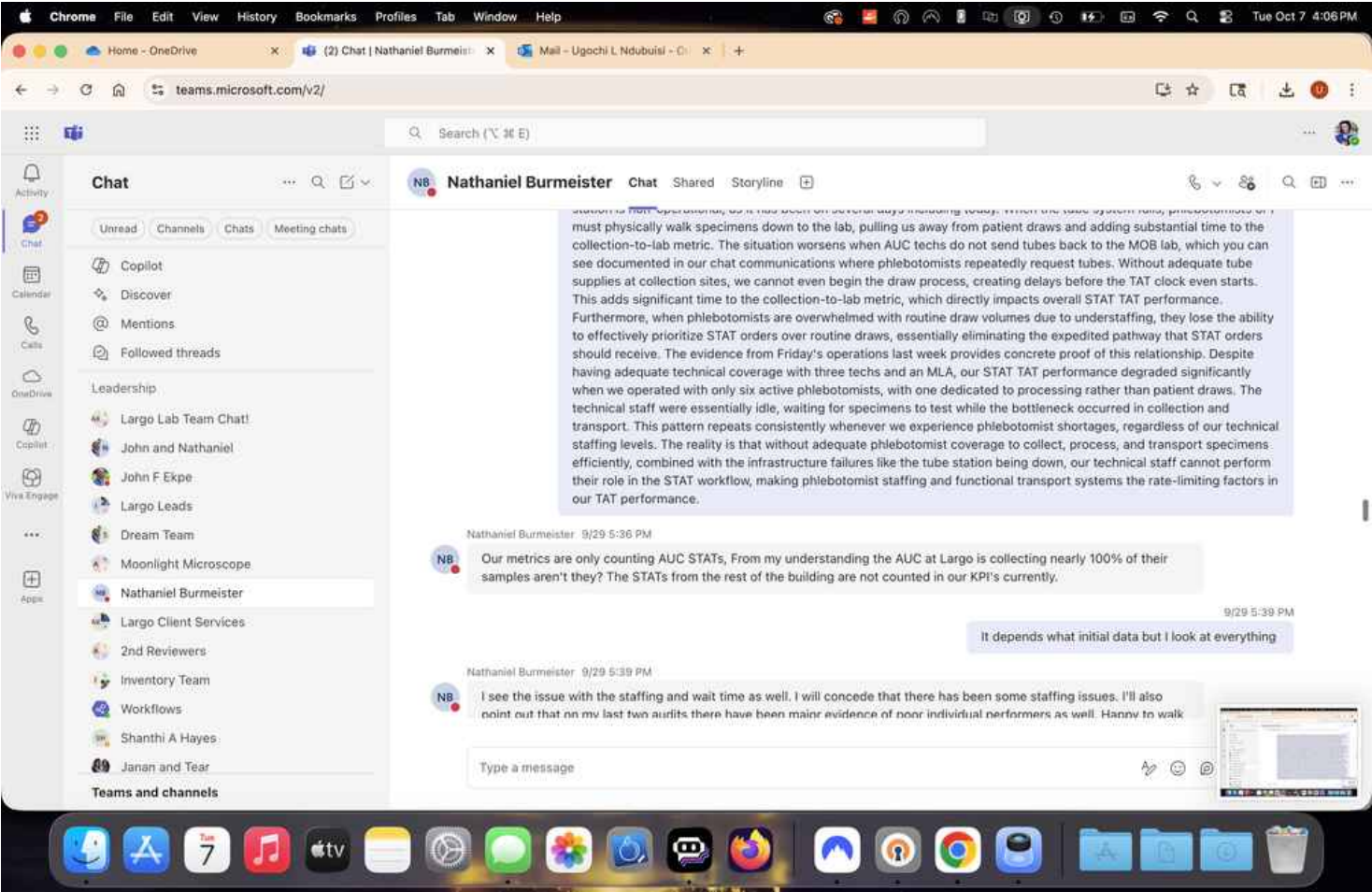


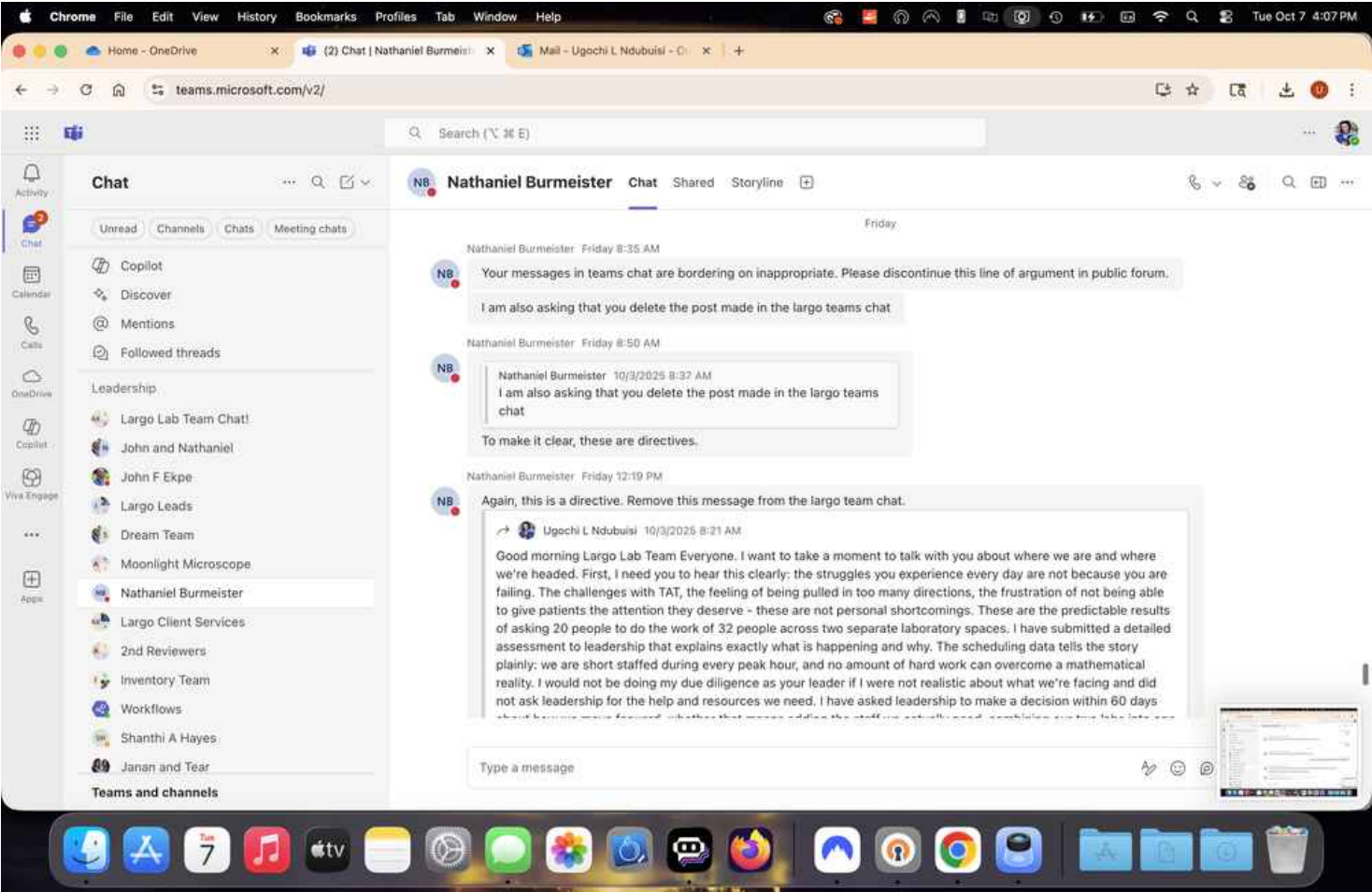


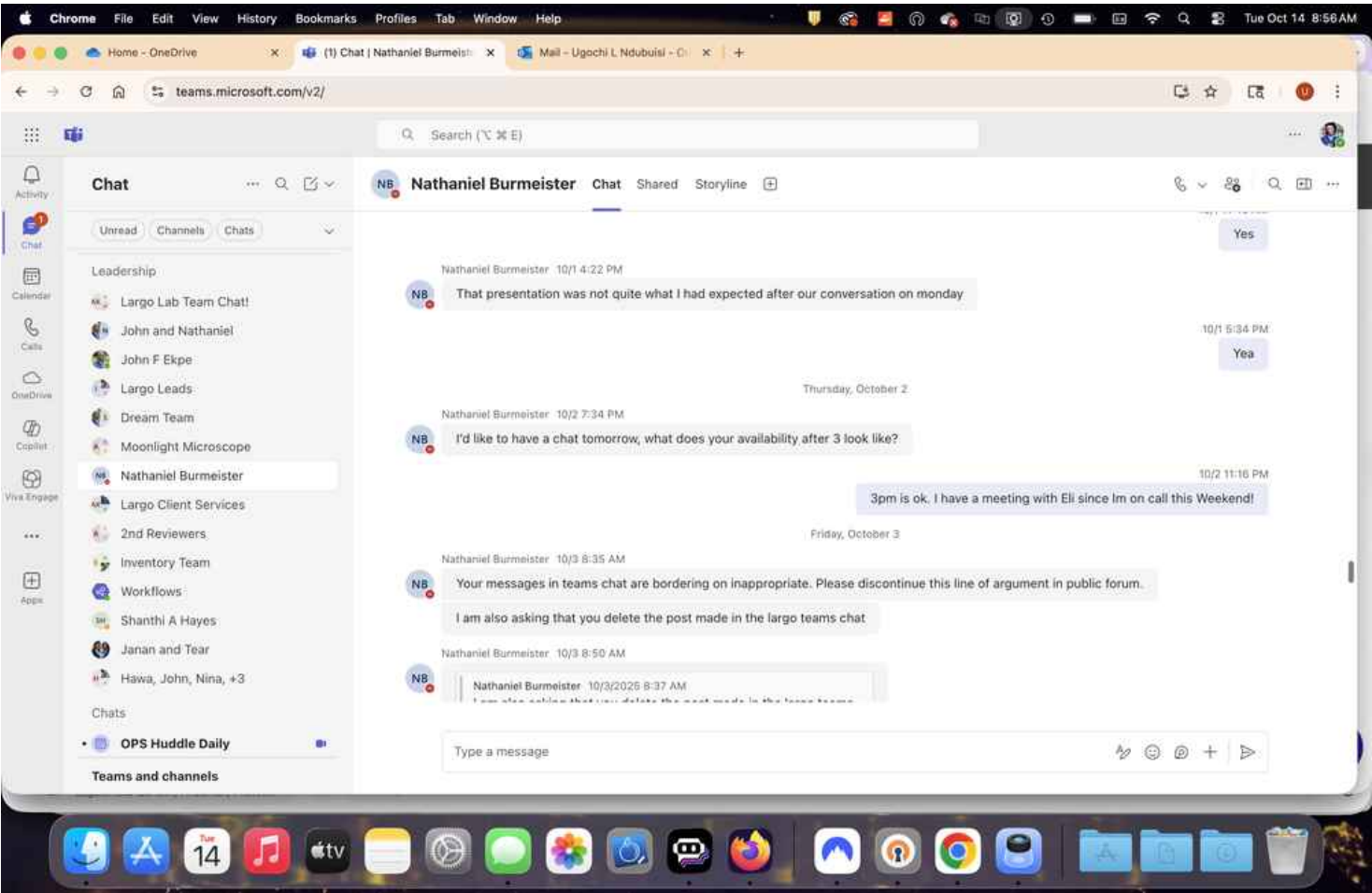


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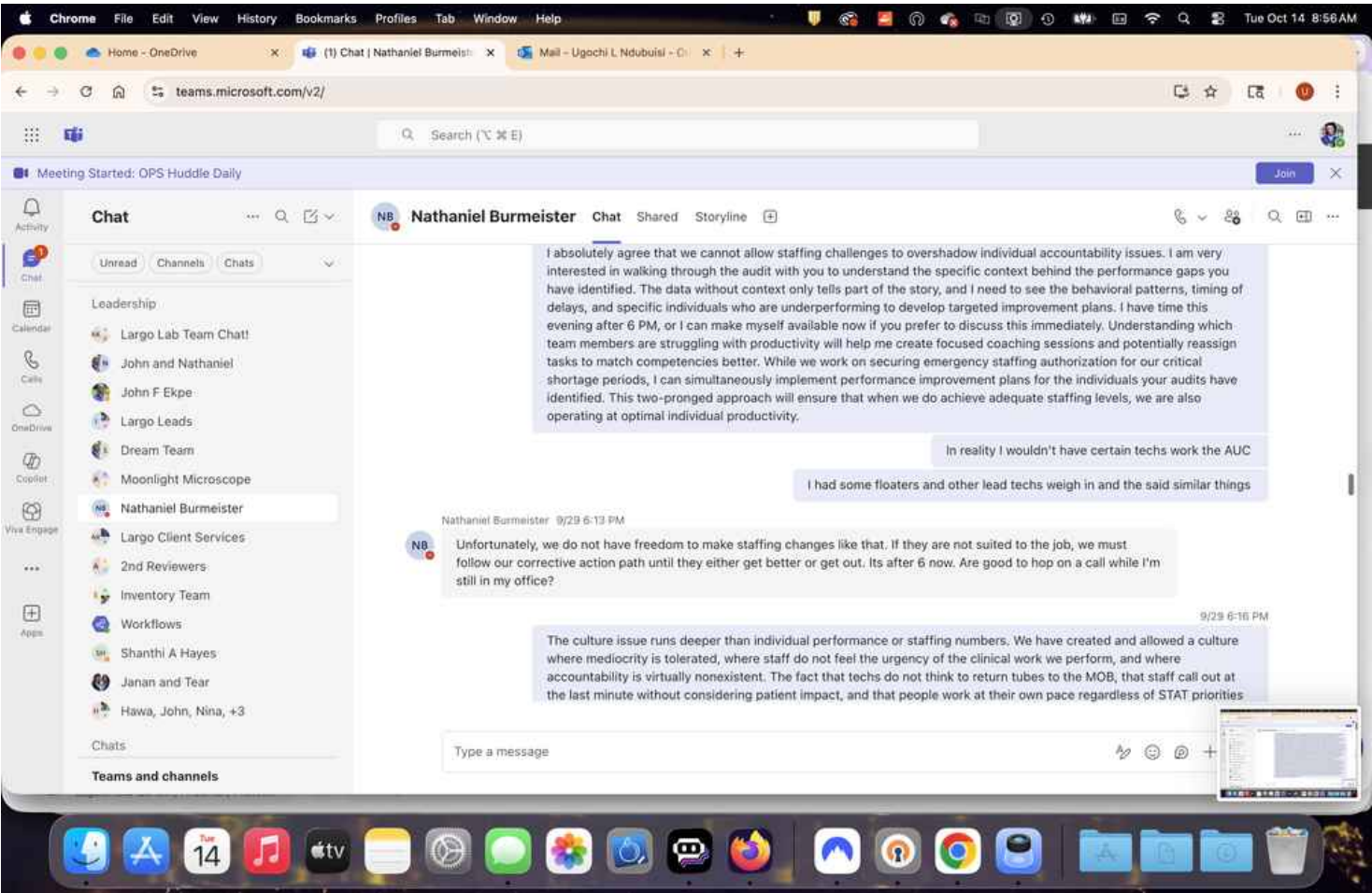


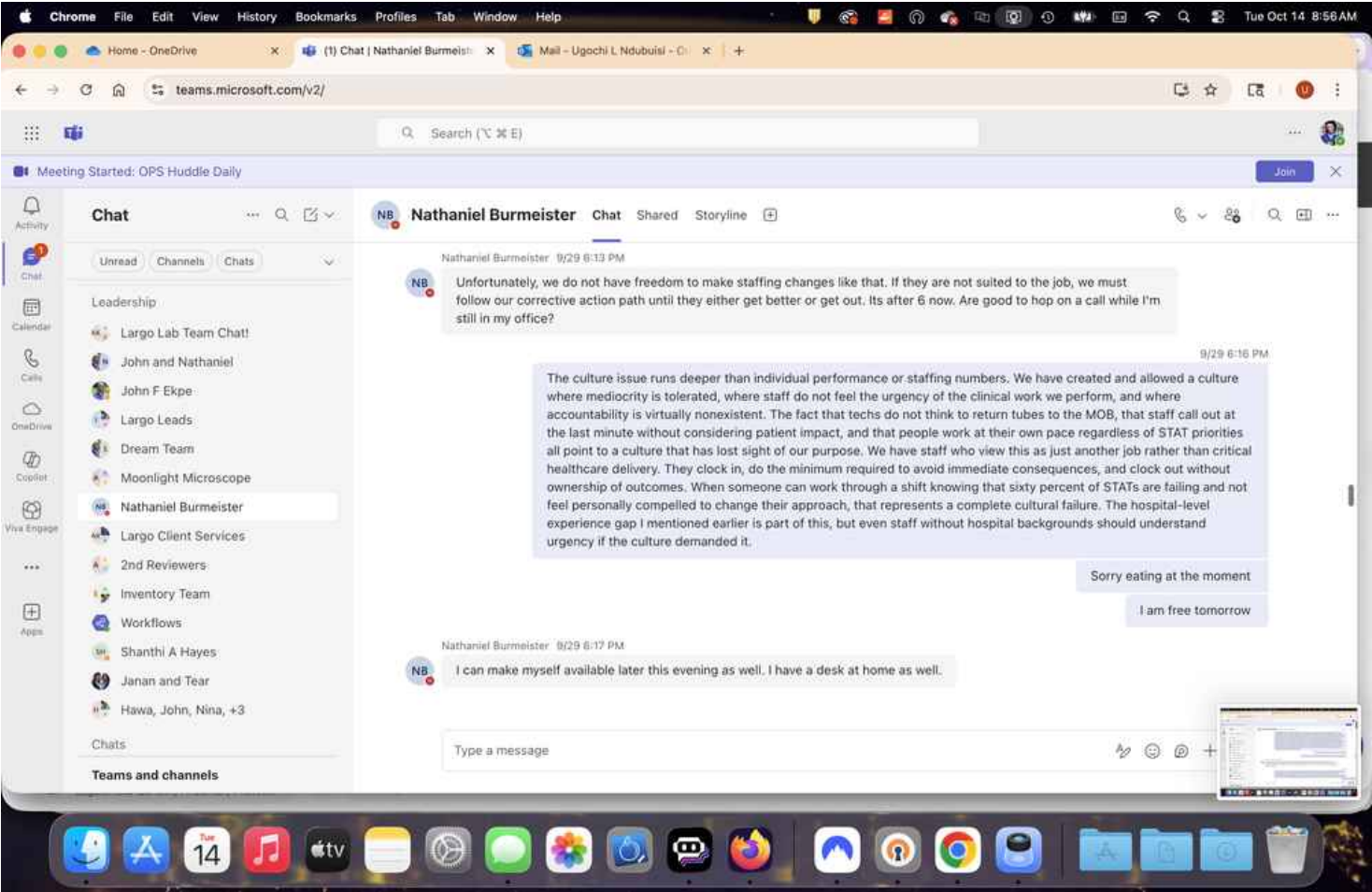


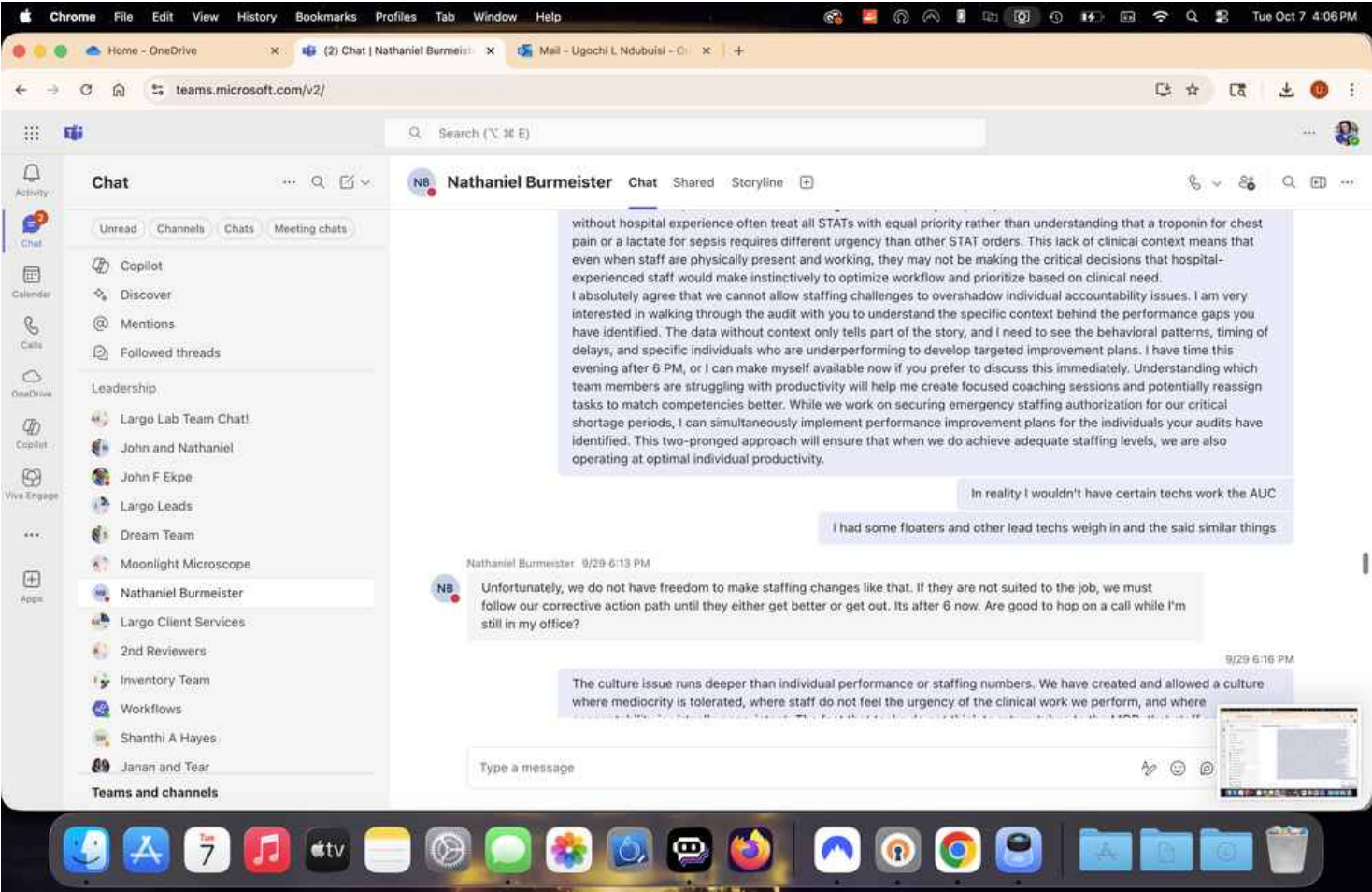


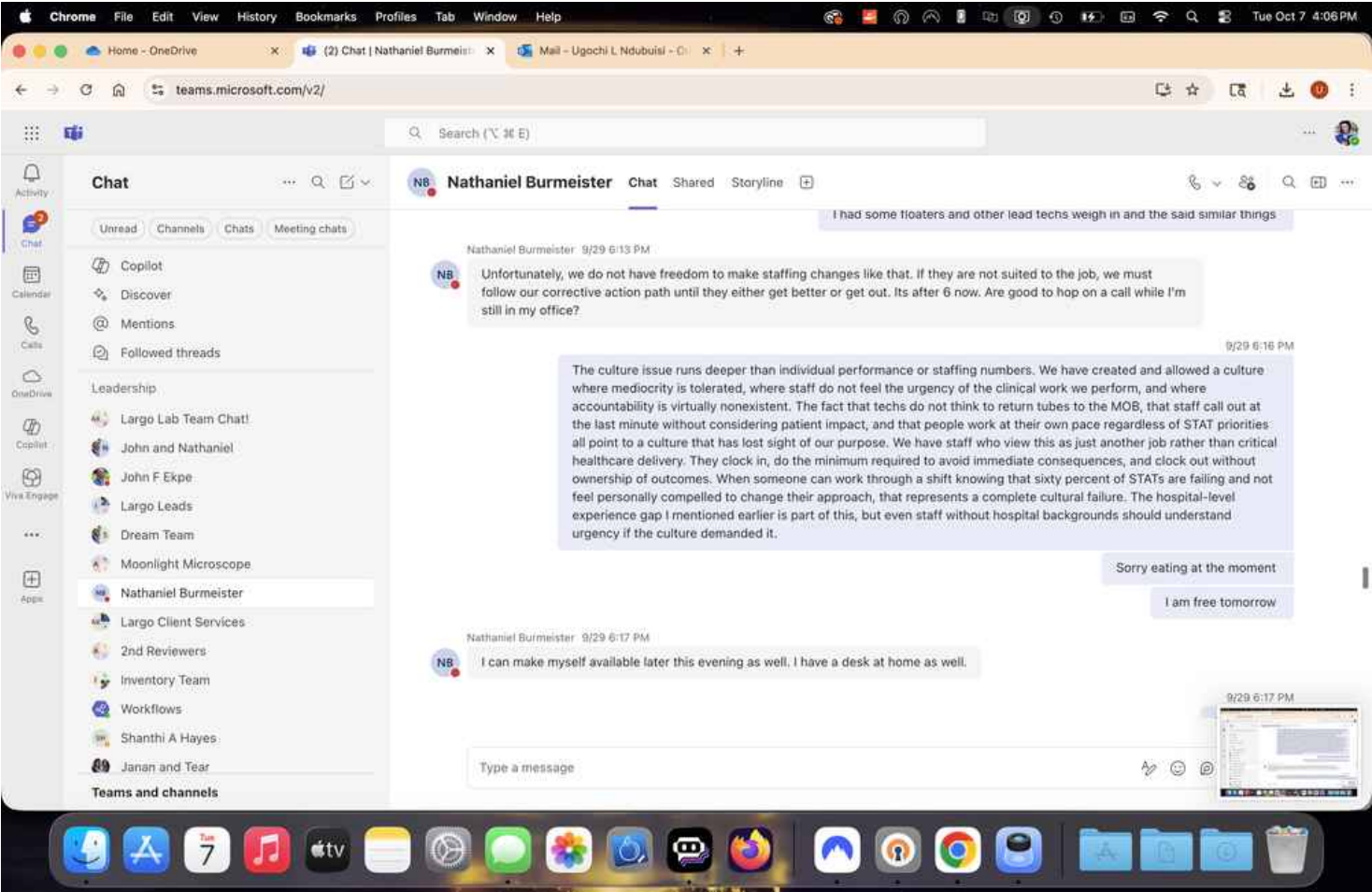


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POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
ACCOUNTABLE DEPARTMENT National Human Resources	EFFECTIVE DATE 12/19/2022
DOCUMENT OWNER Chair, National HR Policy Roundtable	PAGE 1 of 3

1.0 Policy Statement

Employees are required to abide by applicable federal, state, and local laws or regulations, to comply with Kaiser Permanente (KP) policies and procedures, and to conduct themselves in accordance with KP's Principles of Responsibility. If an employee fails to comply with any of these or engages in misconduct (including inappropriate/disruptive behavior) or poor performance, the employee may be subject to corrective/disciplinary action, up to and including termination. In addition, the violation of certain laws or regulations may require KP to report the violation to a regulatory or licensing agency and/or may subject an employee to criminal prosecution as well as civil penalties involving substantial monetary fines.

Although represented employees are covered by this policy, the levels of corrective action for represented employees will be determined according to the provisions of the applicable collective bargaining agreement.

Regional procedures must be consistent with this National policy but may include more detailed information regarding corrective/disciplinary action, and employees are subject to those procedures as well.

2.0 Purpose

Kaiser Permanente (KP) must comply with specific legal/regulatory standards that include, but are not limited to, those indicated by Medicare and other government program billing requirements, Guidance issued by the Centers for Medicare and Medicaid Services, the Department of Health and Human Services, the Compliance Program Guidance issued by the Office of the Inspector General, and HIPAA/Privacy and Security regulations. (See Section 6.0, References) KP is required to have well publicized disciplinary guidelines that demonstrate its commitment to compliance and set forth the consequences for violations of compliance mandates.

3.0 Scope/Coverage

This policy applies to all individuals who are employed by the following entities (collectively referred to as "Kaiser Permanente"):

- 3.1** Kaiser Foundation Health Plan, Inc. (KFHP);
- 3.2** Kaiser Foundation Hospitals (KFH);
- 3.3** KFHP/H subsidiaries;
- 3.4** The Permanente Medical Group, Inc. (TPMG) [NOTE: This policy does not apply to physicians, podiatrists, vice presidents, or members of the TPMG Executive Staff, who are covered by separate TPMG policies]; and
- 3.5** Southern California Permanente Medical Group (SCPMG) [NOTE: This policy does not apply to physicians of SCPMG].

4.0 Definitions: n/a

POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
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DOCUMENT OWNER Chair, National HR Policy Roundtable	PAGE 2 of 3

5.0 Provisions

- 5.1** Except as limited by collective bargaining agreements and/or statute, employment with KP is “at will” (see *At-Will Employment*, NATL.HR.37).
- 5.2** In the event that it is necessary to take corrective/disciplinary action to address concerns related to an employee’s work performance or conduct, the corrective/disciplinary action may include, but is not limited to:
 - 5.2.1** verbal discussion, coaching, and/or warning by the employee’s immediate supervisor or higher-level manager to correct the problem
 - 5.2.2** written notice, with or without final warning
 - 5.2.3** suspension, with or without final warning
 - 5.2.4** termination of employment
- 5.3** The corrective/disciplinary action steps listed in section 5.2 are not exhaustive and serve only as a description of possible action. None of these actions are mandatory or a prerequisite for any other actions that may be appropriate based on the circumstances. Any or all of these actions may be employed prior to termination and certain employee conduct may be deemed sufficiently serious to warrant immediate termination or escalated disciplinary action without prior corrective/disciplinary action.
- 5.4** The level of corrective/disciplinary action will be determined by management after review of the facts (including legal, compliance or policy violations), applicable regional/national collective bargaining agreement provisions, and consideration of other relevant factors and circumstances including, but not necessarily limited to, the factors in section 5.5 below. Management should consult Human Resources as appropriate.
- 5.5** Examples of factors to be considered in determining the level of corrective/disciplinary action include, but are not limited to, the following:
 - 5.5.1** the severity of the violation
 - 5.5.2** the number of violations
 - 5.5.3** whether the violation was part of a pattern or practice of improper behavior or non-compliance
 - 5.5.4** the individual’s past performance and conduct
 - 5.5.5** whether the individual knew or should have known the applicable policies, rules or regulations
 - 5.5.6** length of employment at KP
 - 5.5.7** whether the violation was intentional or negligent
 - 5.5.8** whether the action appeared to be committed for personal gain
 - 5.5.9** regional/national collective bargaining agreements corrective/disciplinary action provisions for represented employees

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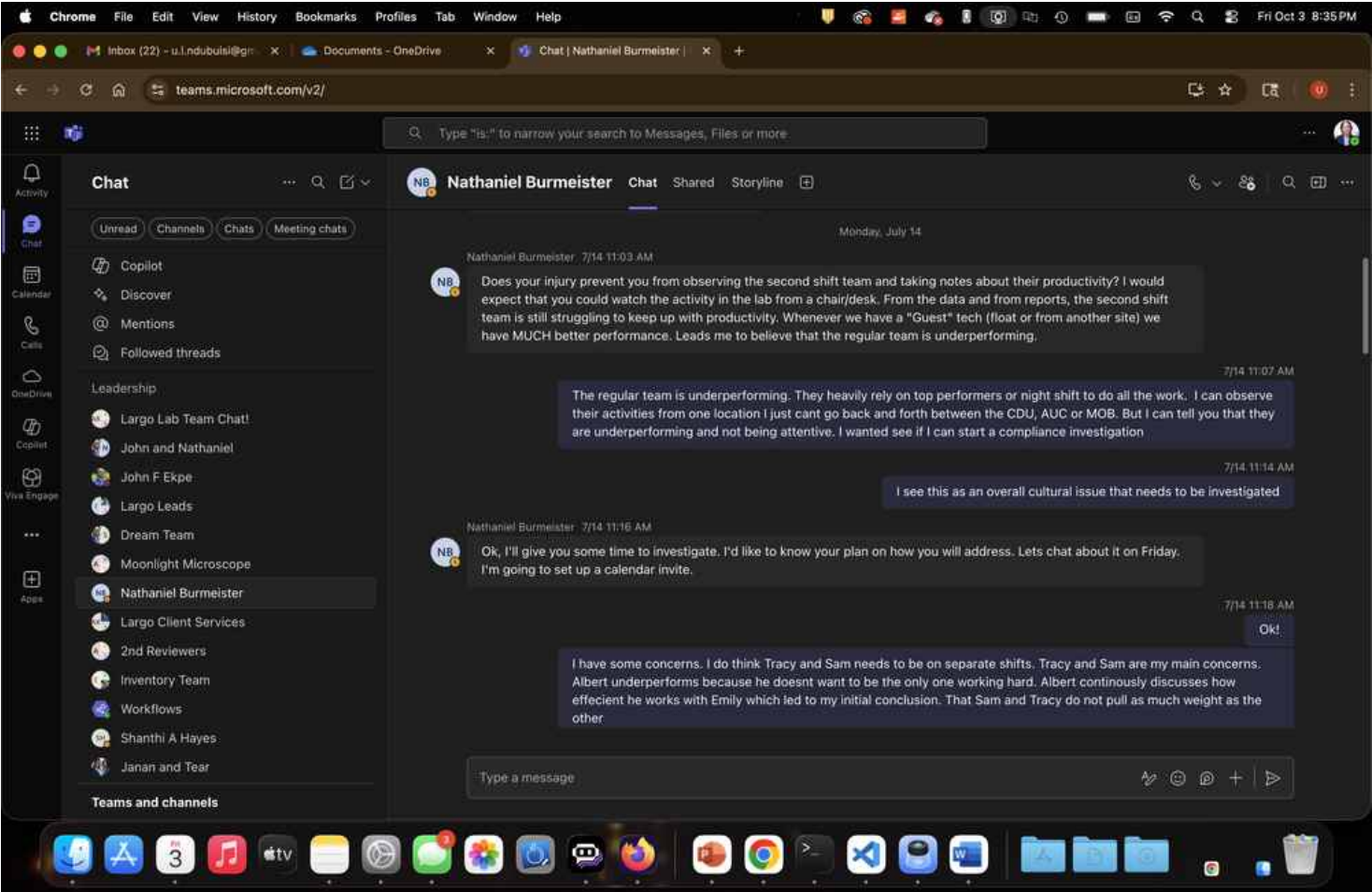
PATTERN OF RETALIATION - KEY EVIDENCE

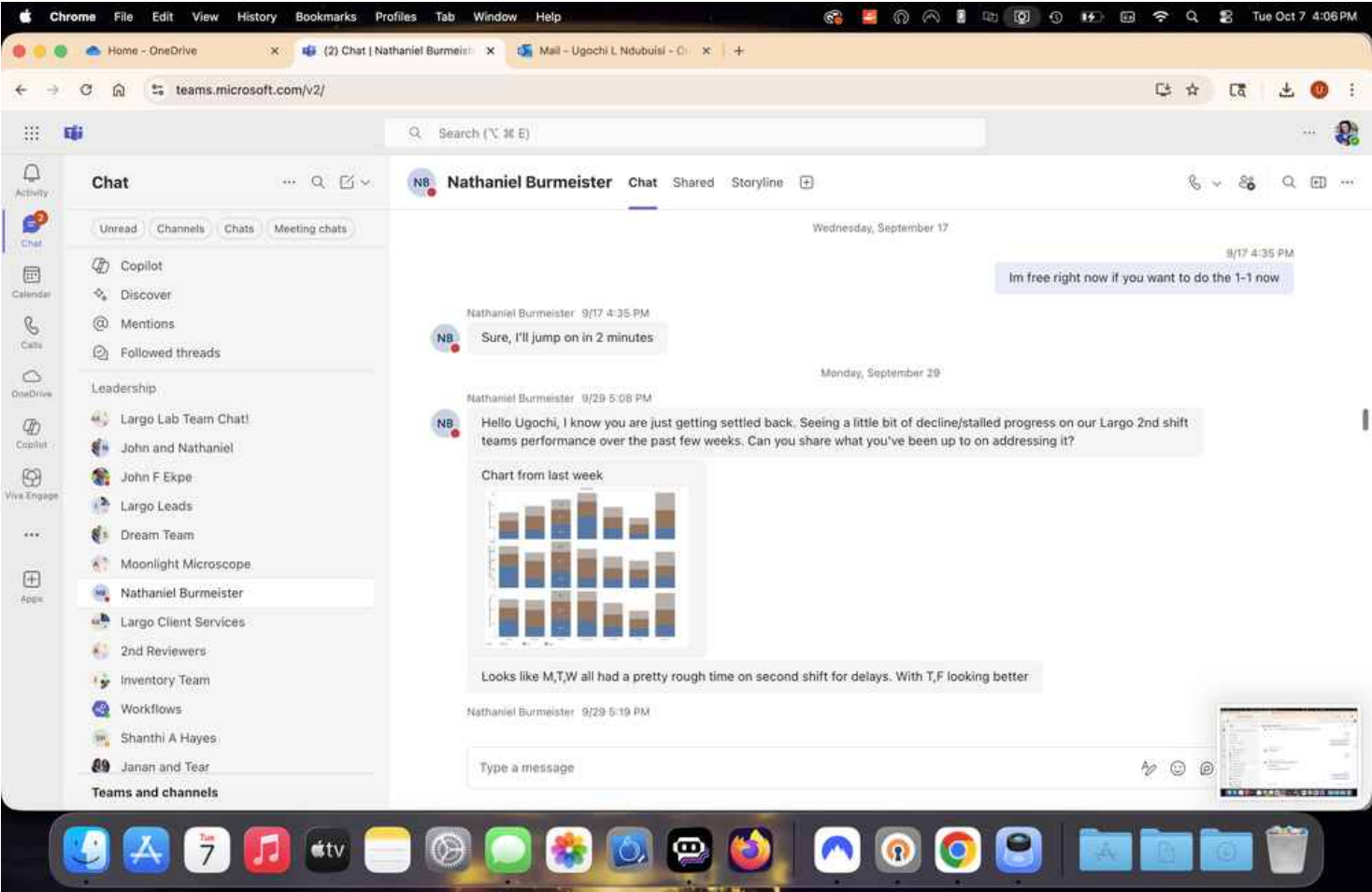
- 1. Protected Activity:** Reported workplace safety concerns affecting patient care (41.23% failure rate)
- 2. Temporal Proximity:** Only 5 days between whistleblowing and Final Written Warning
- 3. Disparate Treatment:** Same analysis praised by director on Sept 29, punished when shared Oct 3
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- 6. Public Interest:** Patient safety at Level 3 Trauma facility serving community

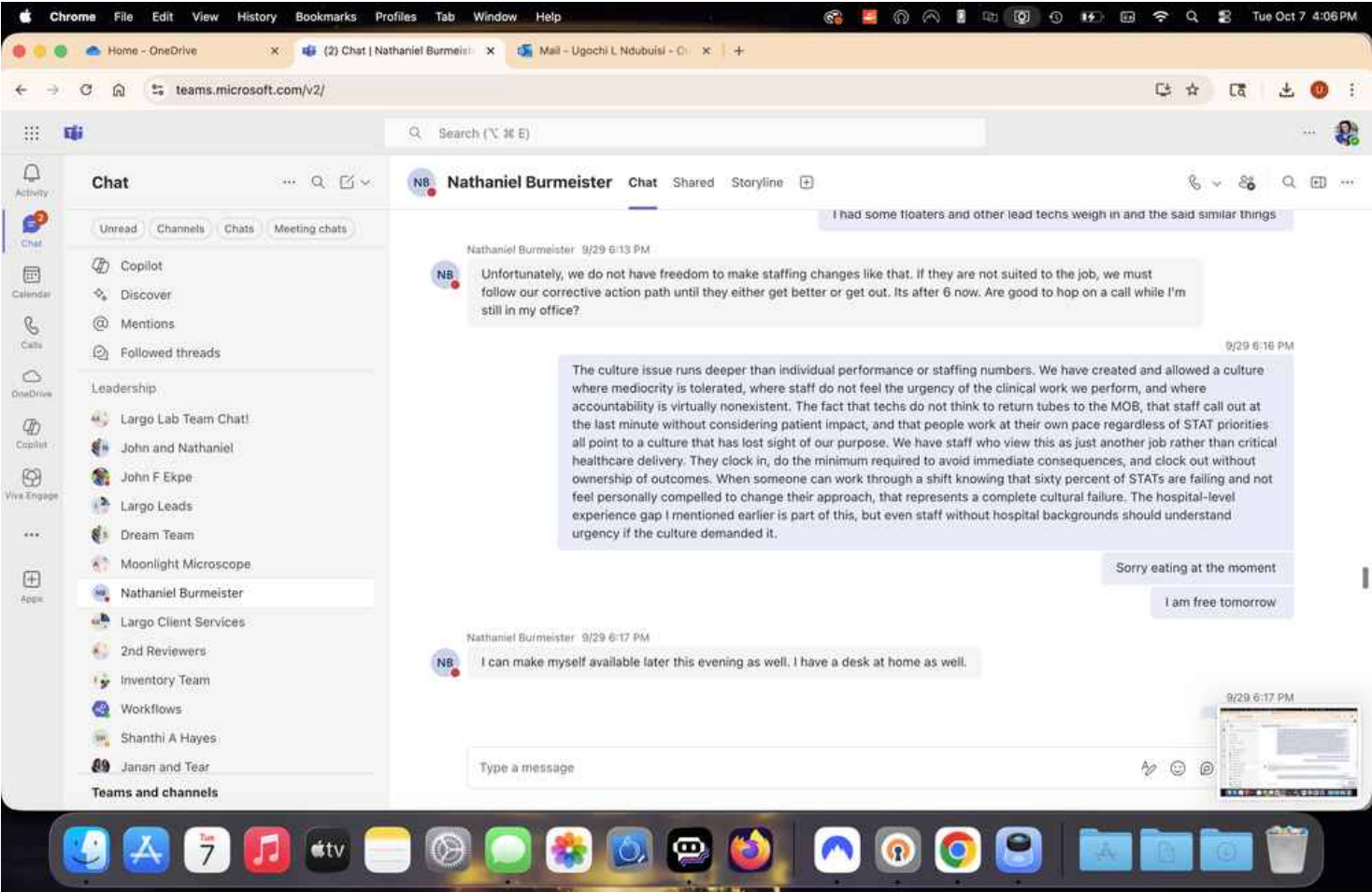
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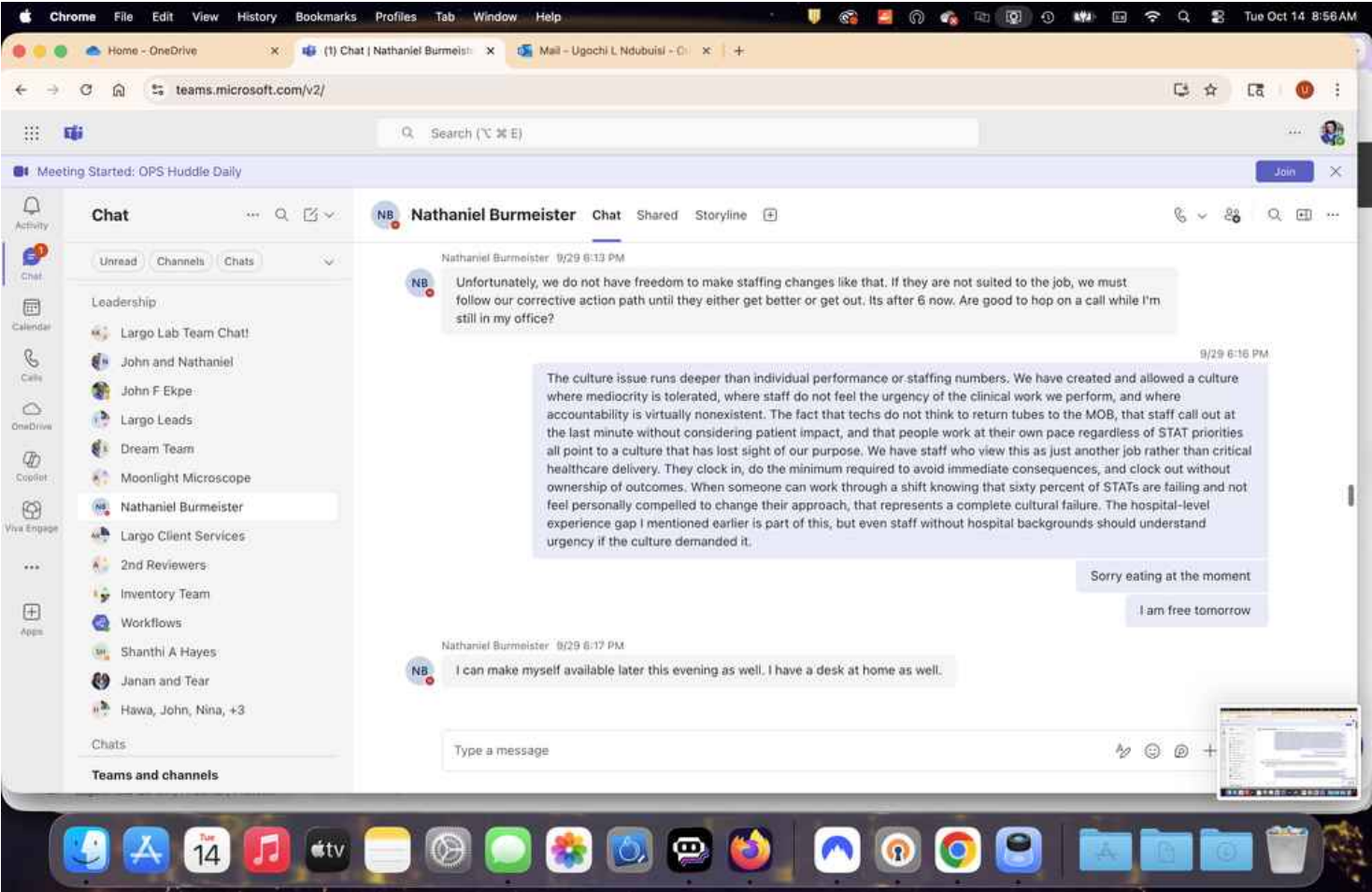
- Maryland OSHA Deadline: November 7, 2025
- Protected under Maryland Occupational Safety and Health Act
- Document Generated: October 14, 2025

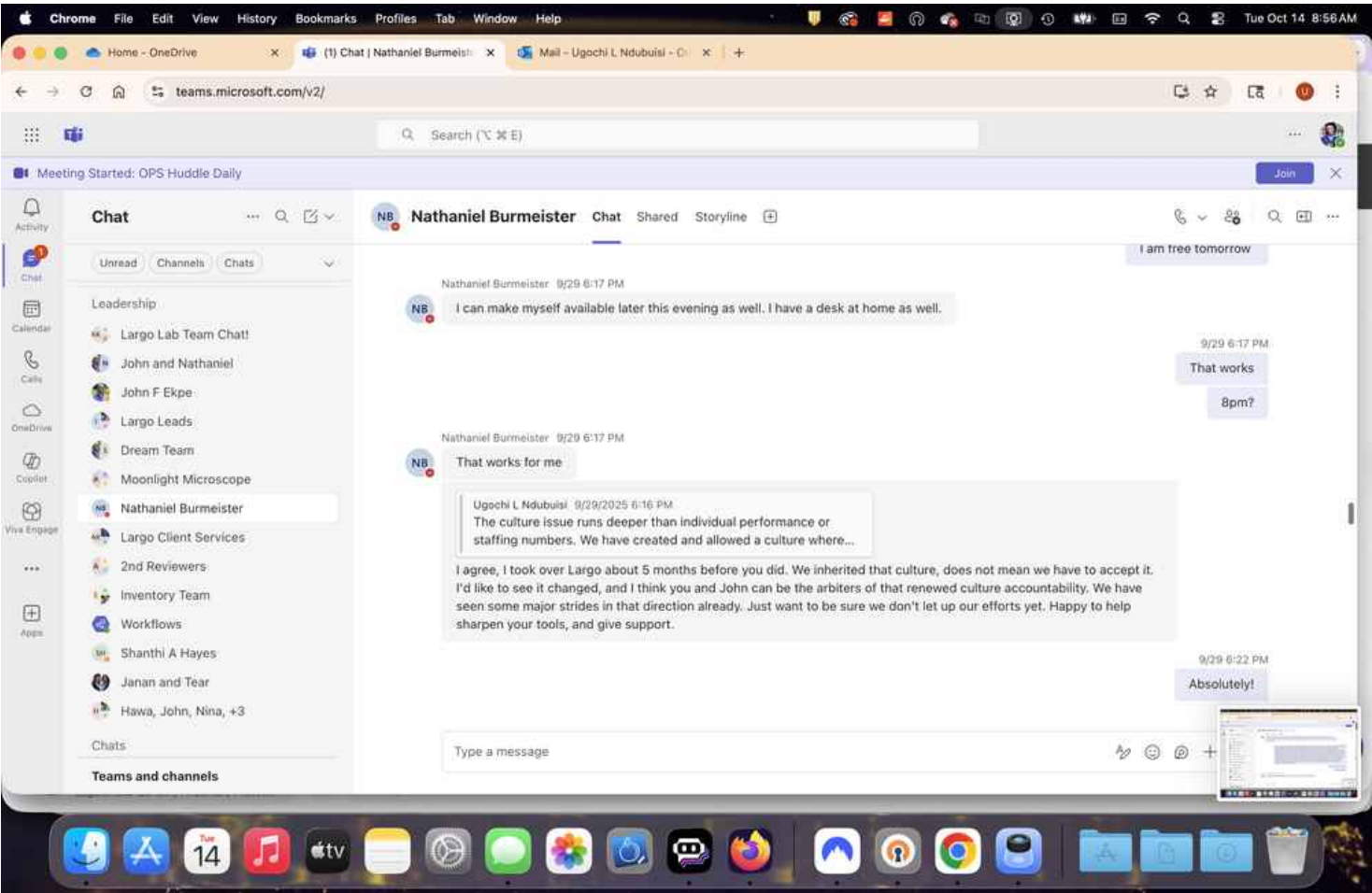
Evidence 1: July_14_Investigation_01_Screenshot 2025-10-03 at 8.35.06PM

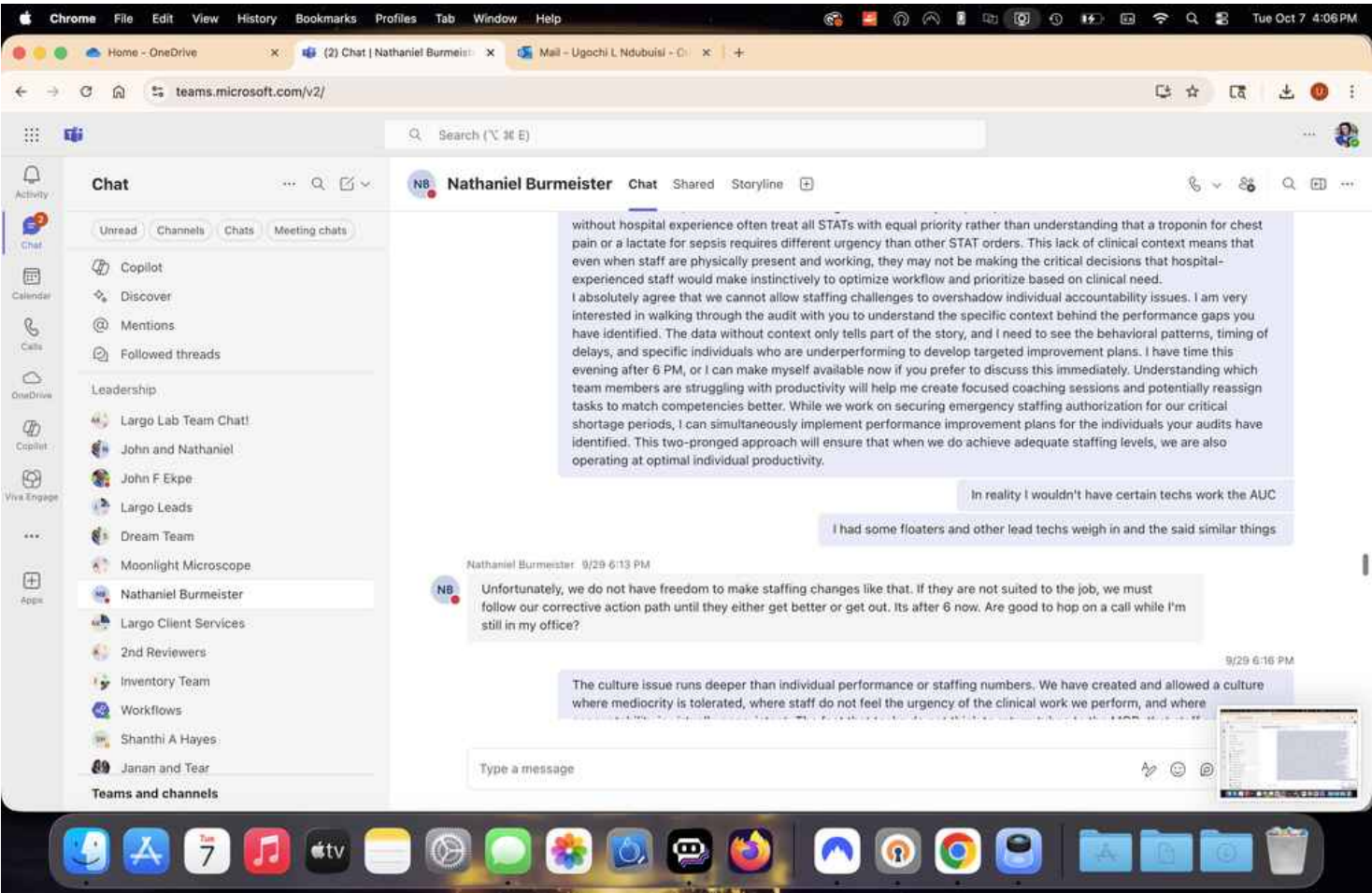


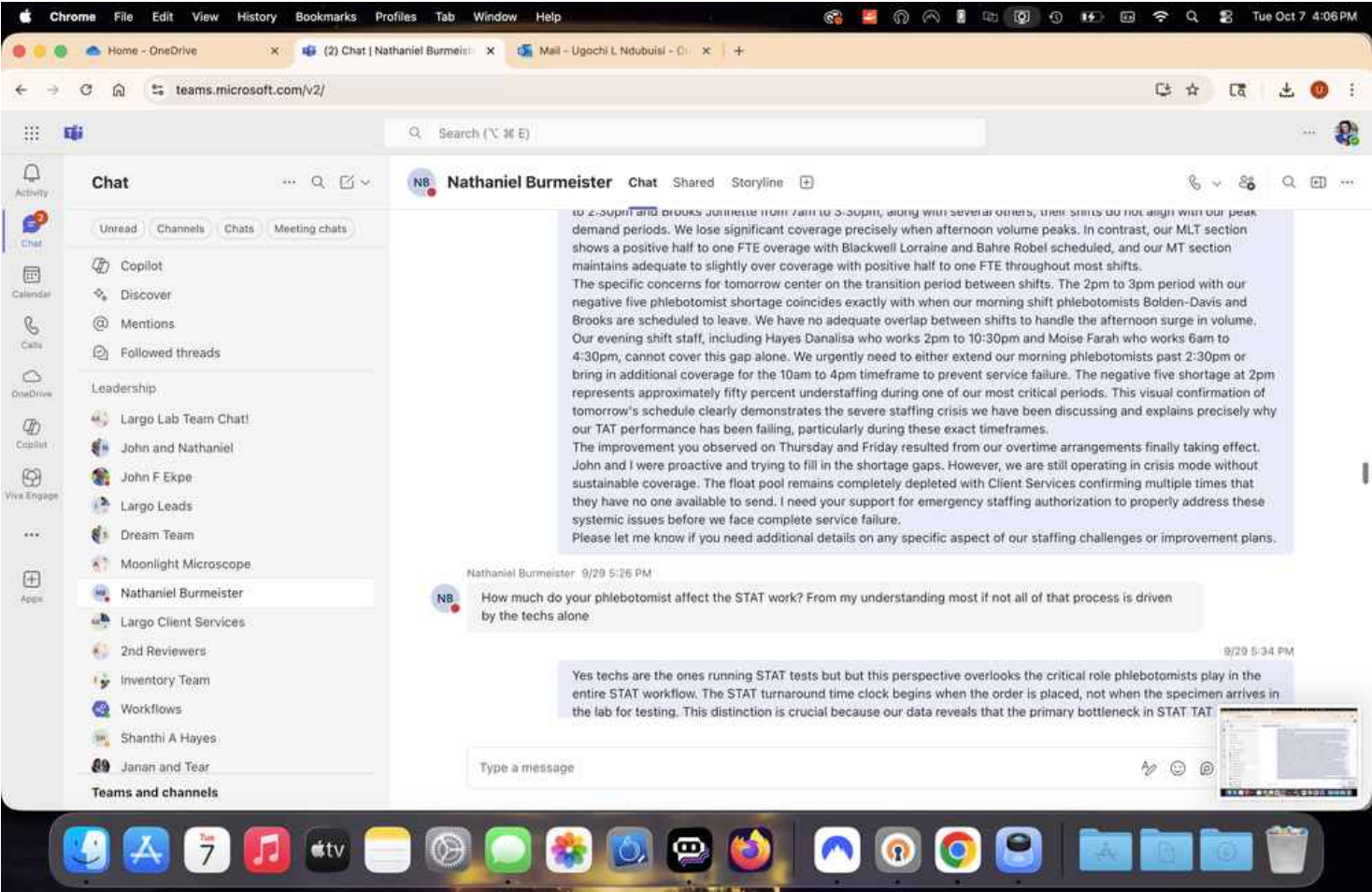


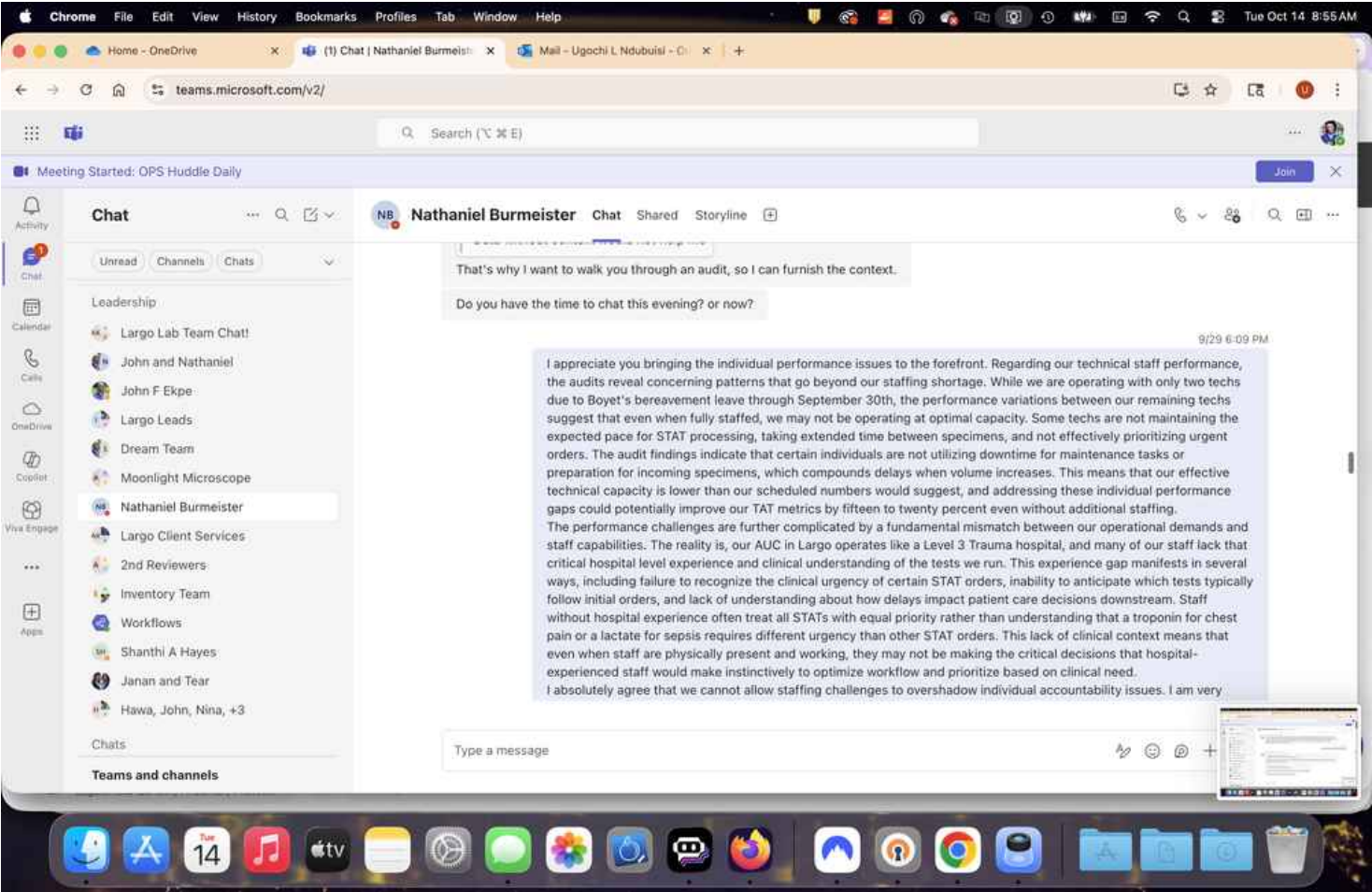


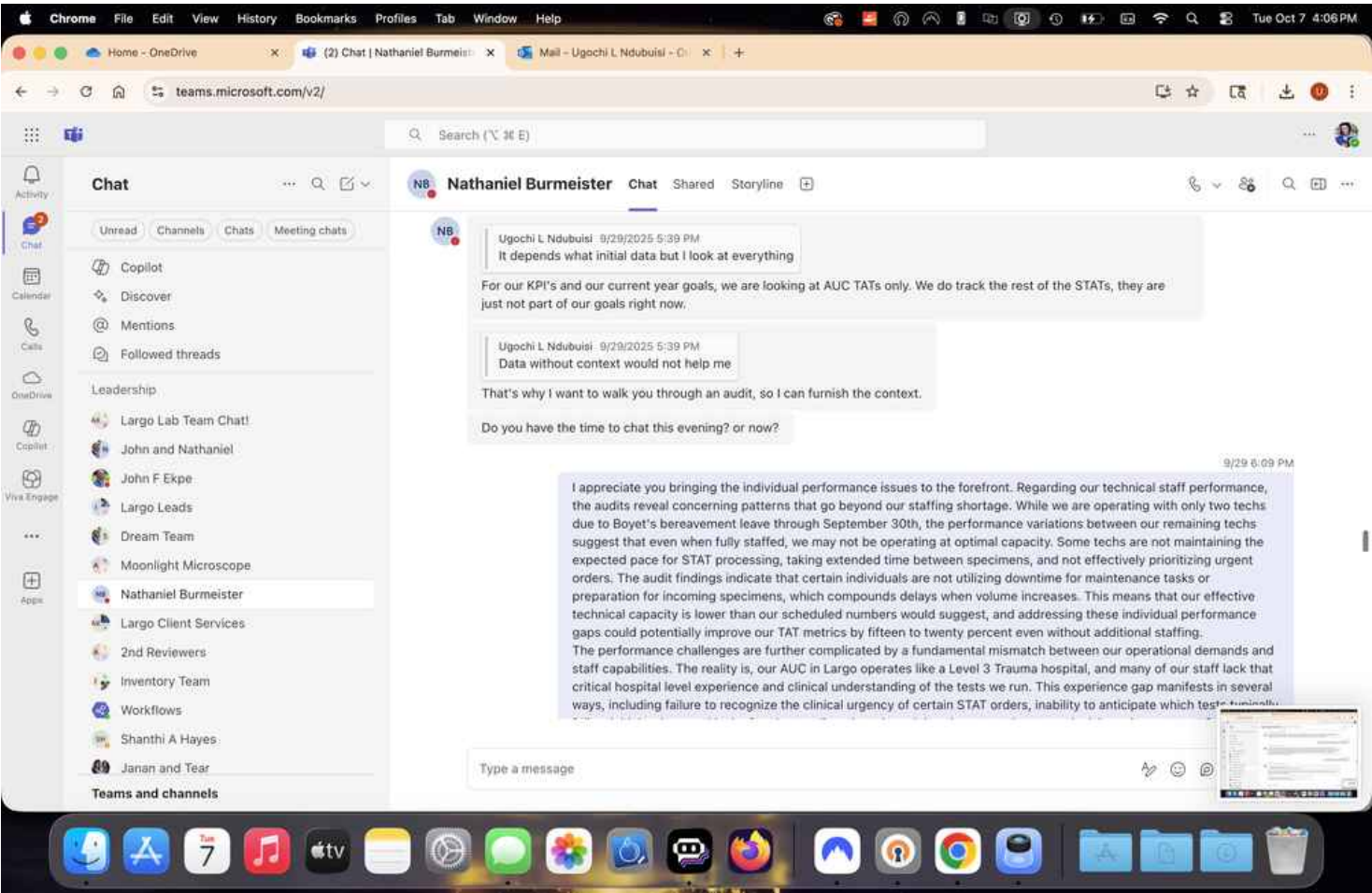


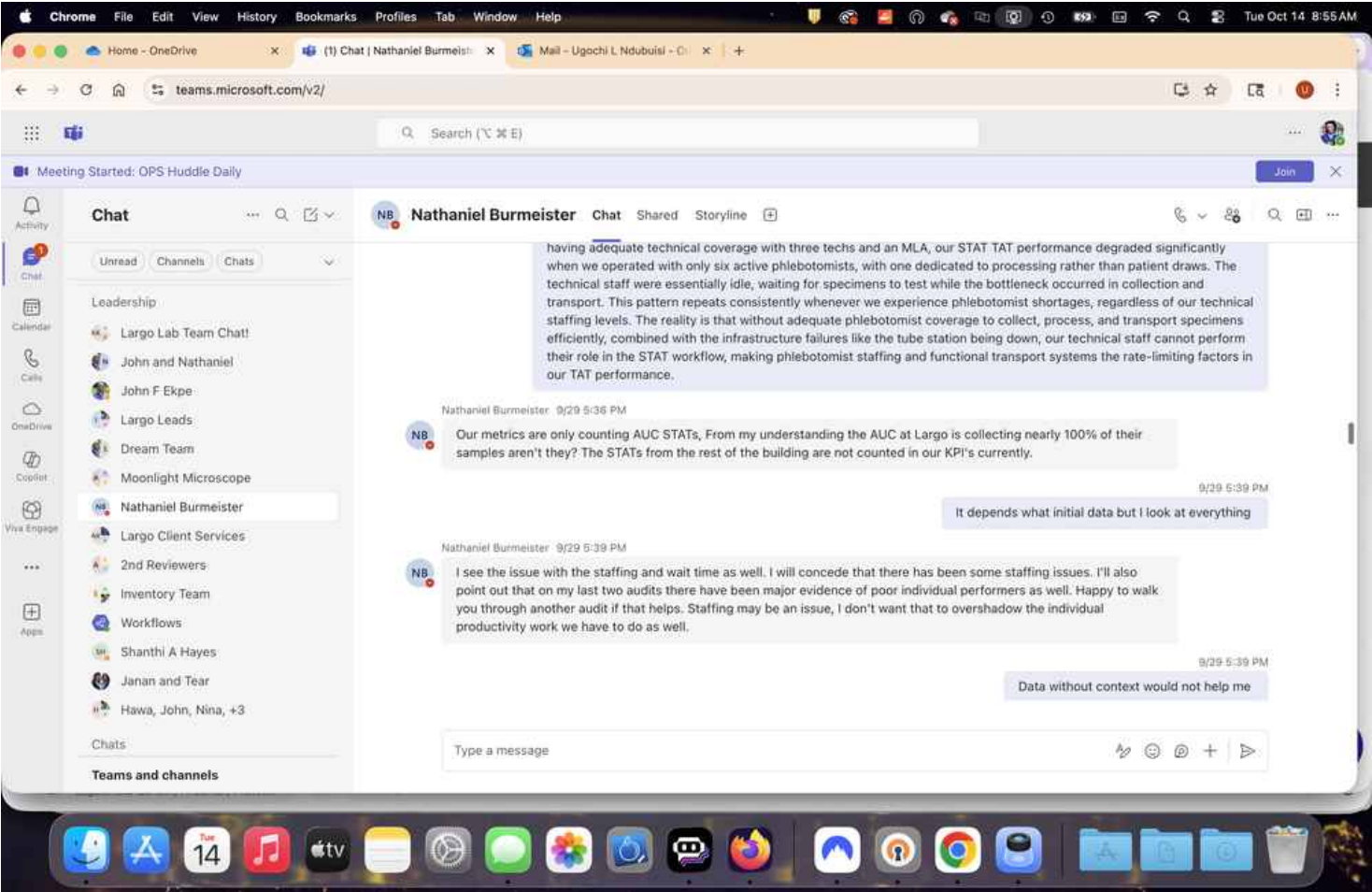


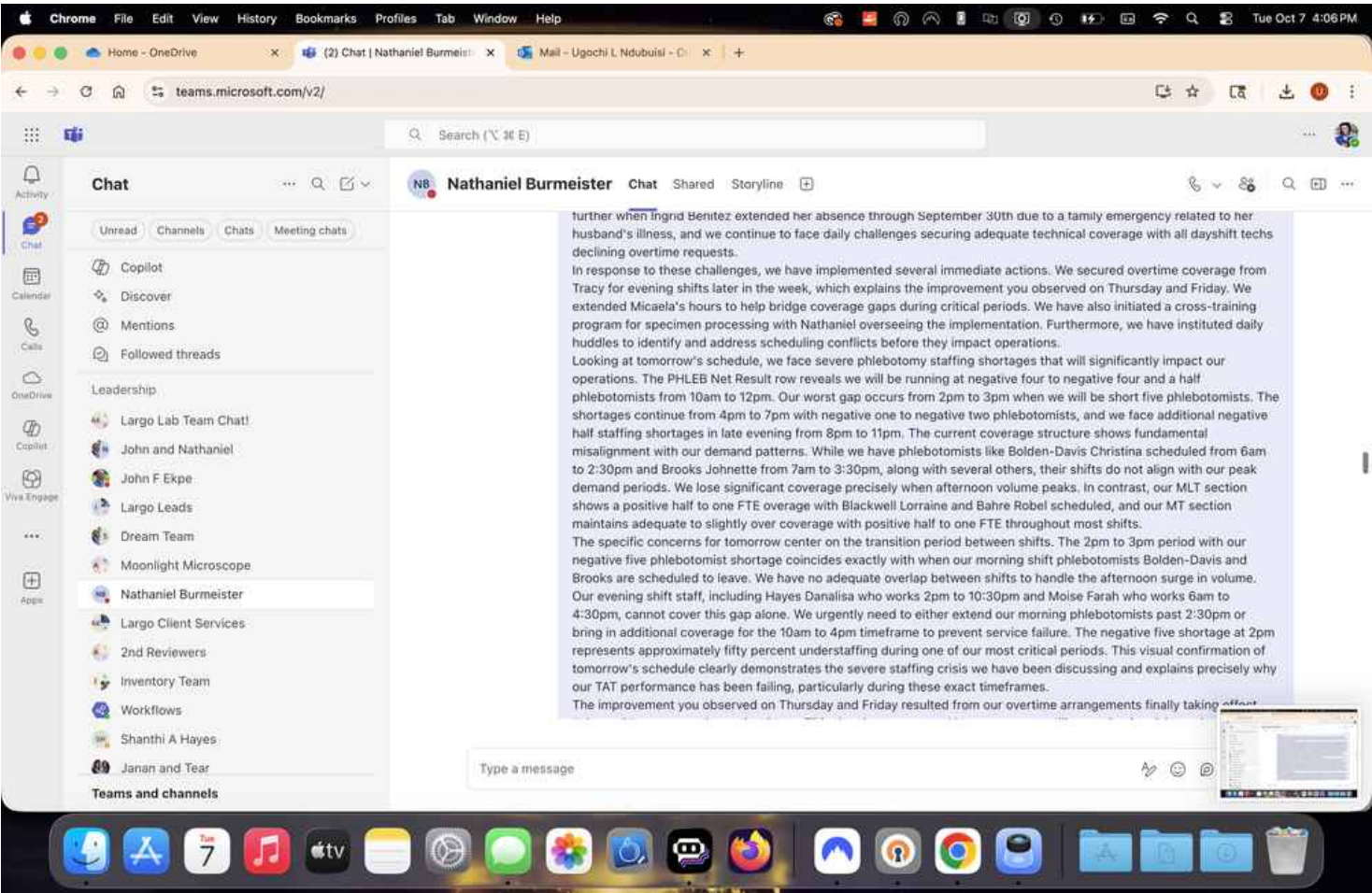


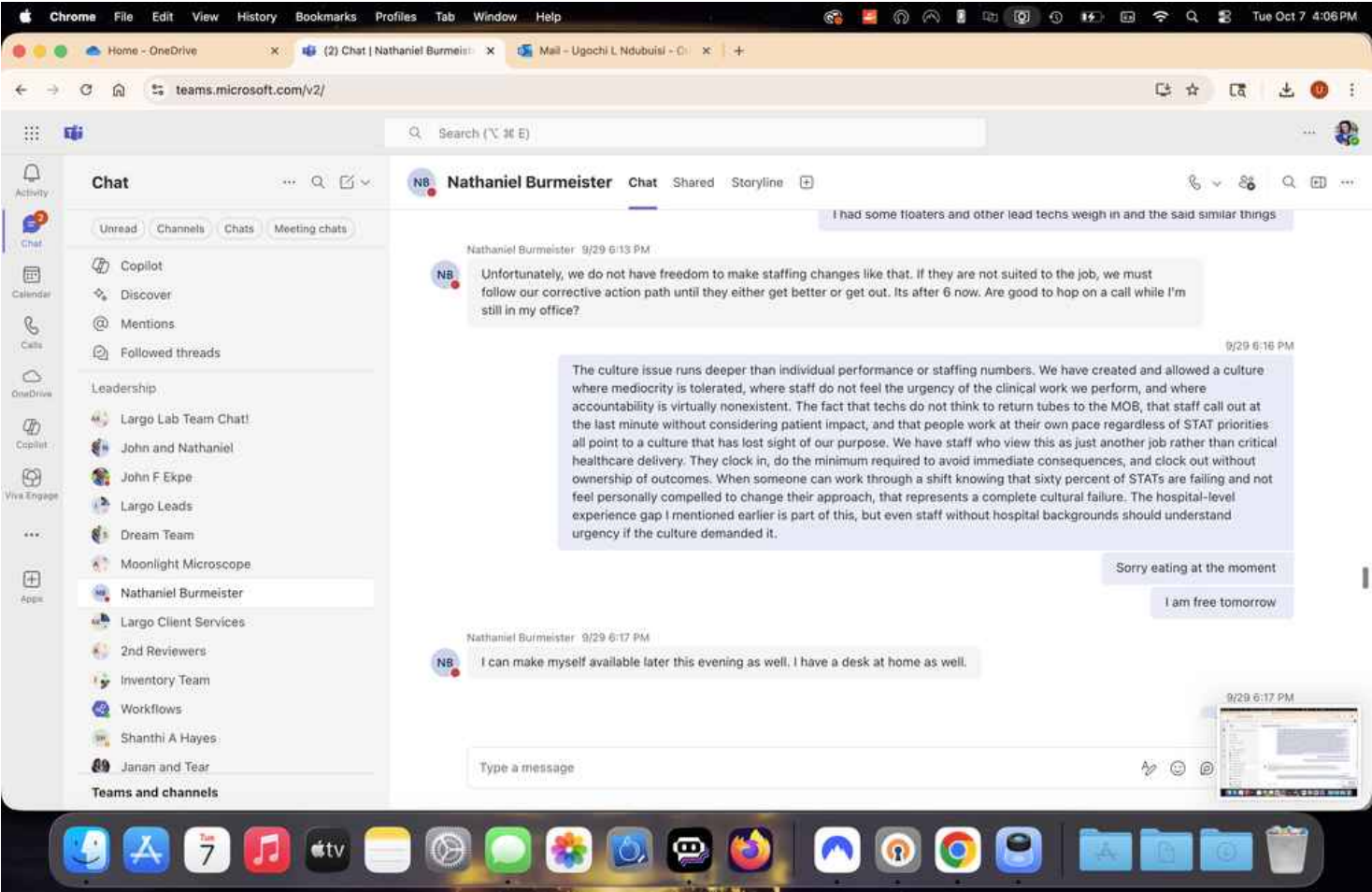




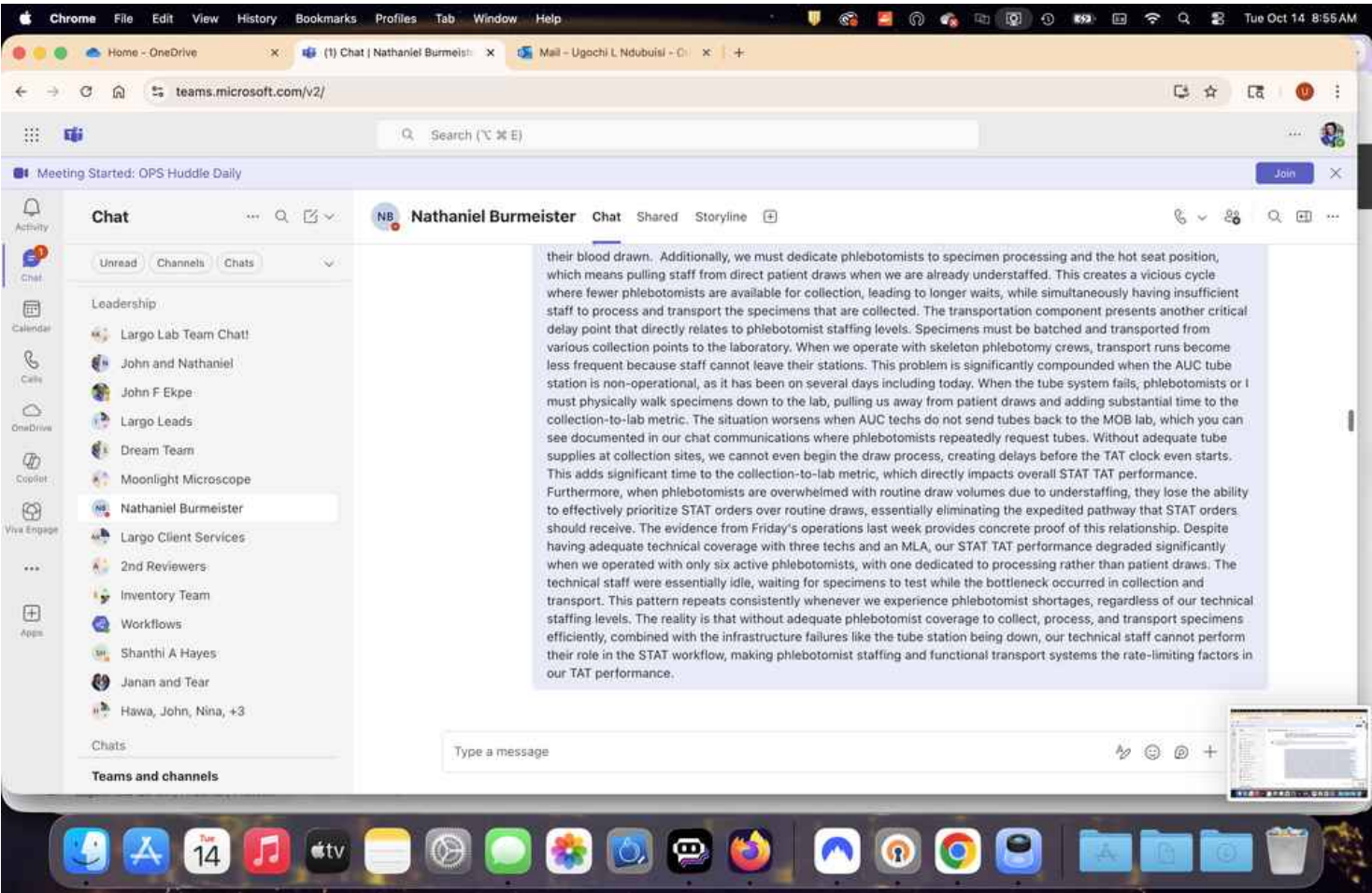




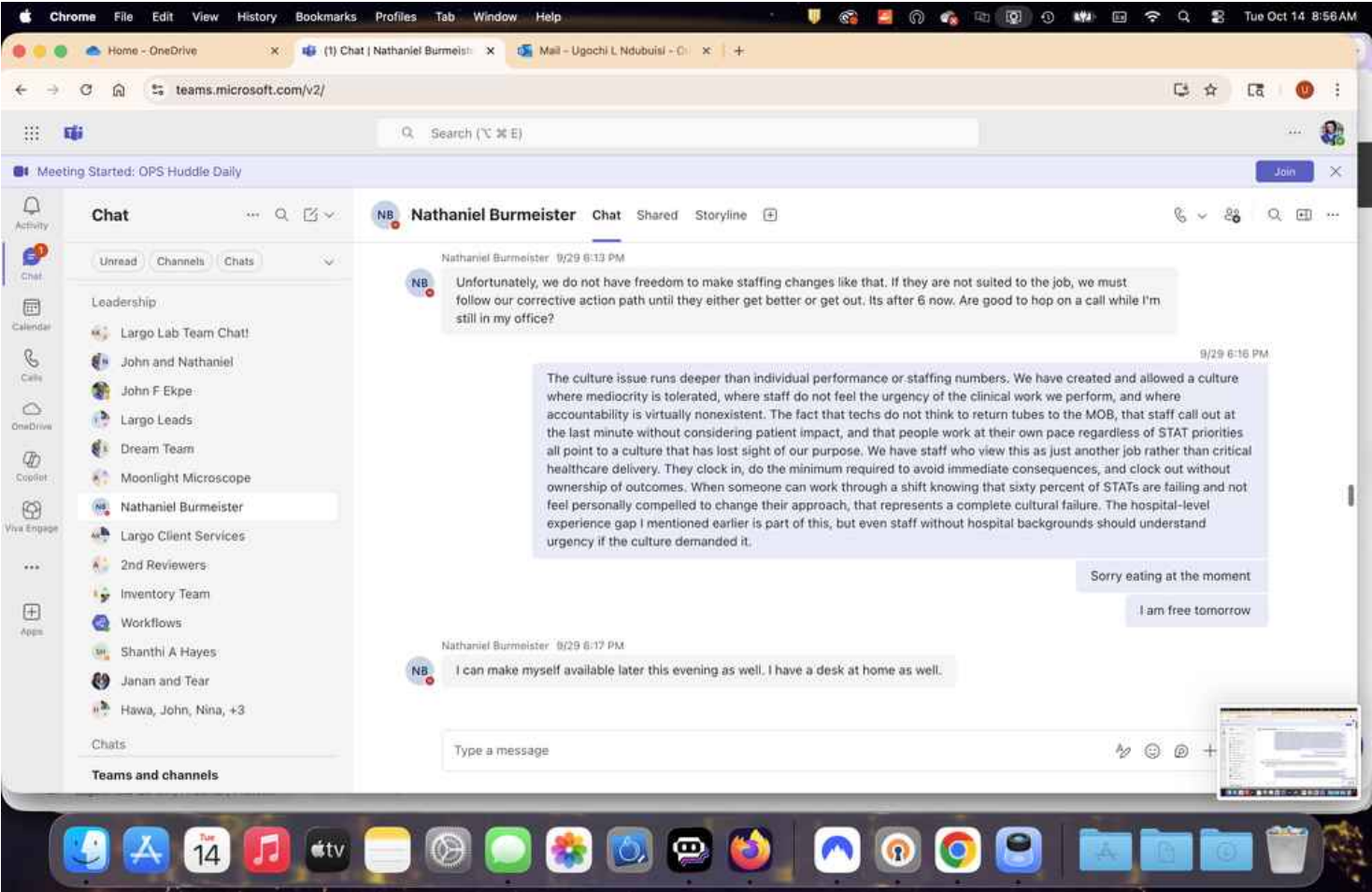


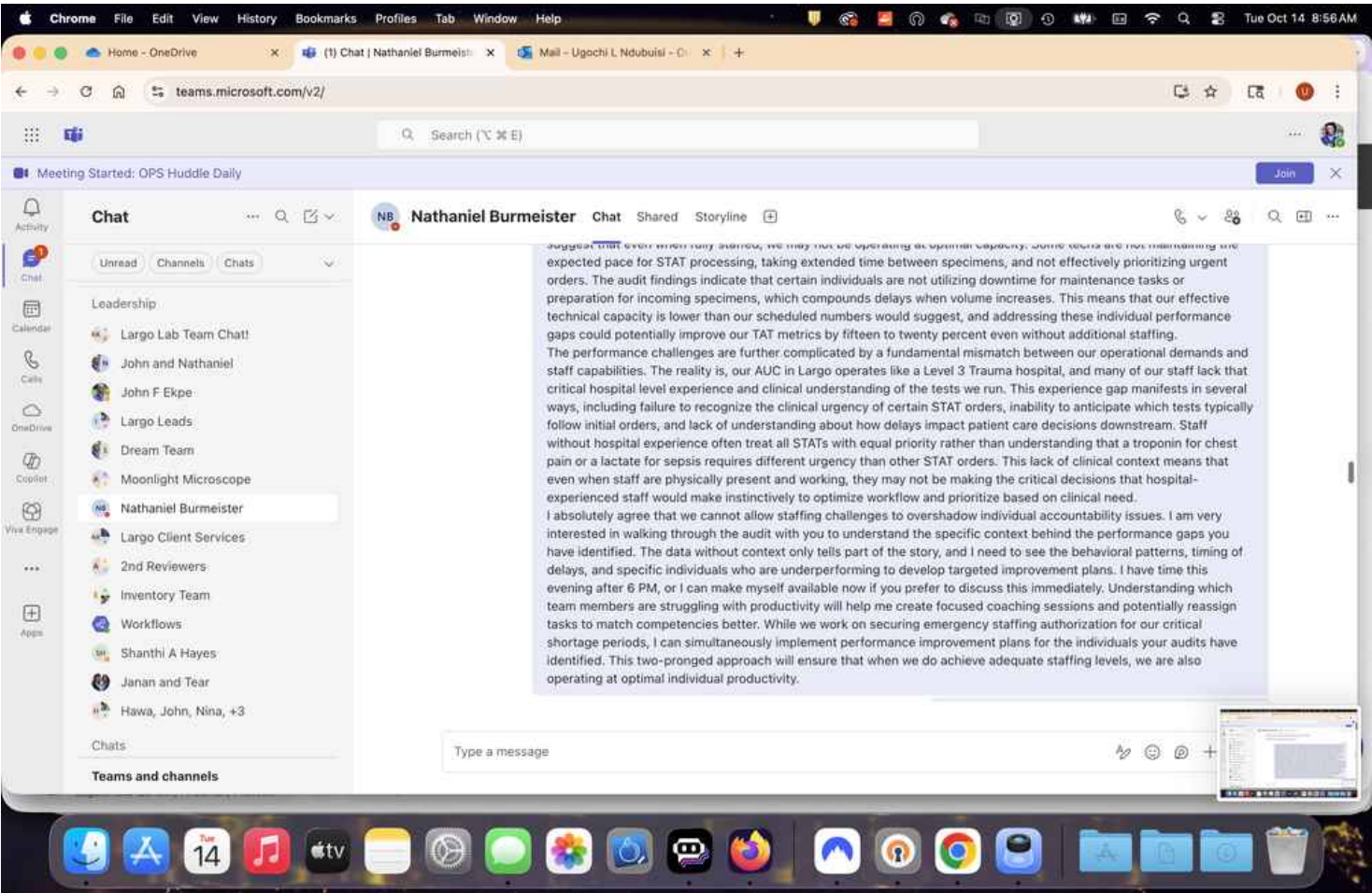


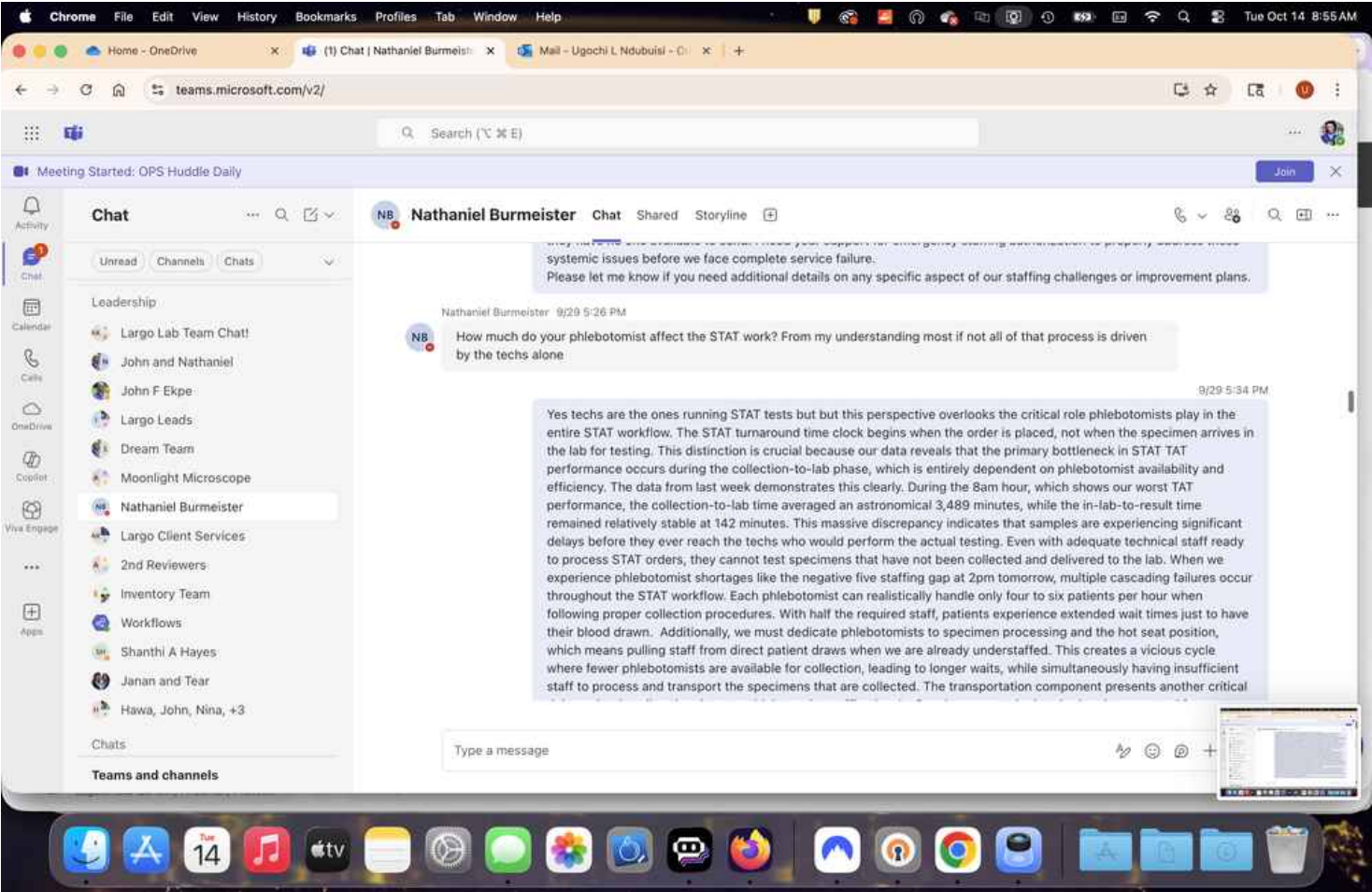
Evidence 13: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.55.37AM



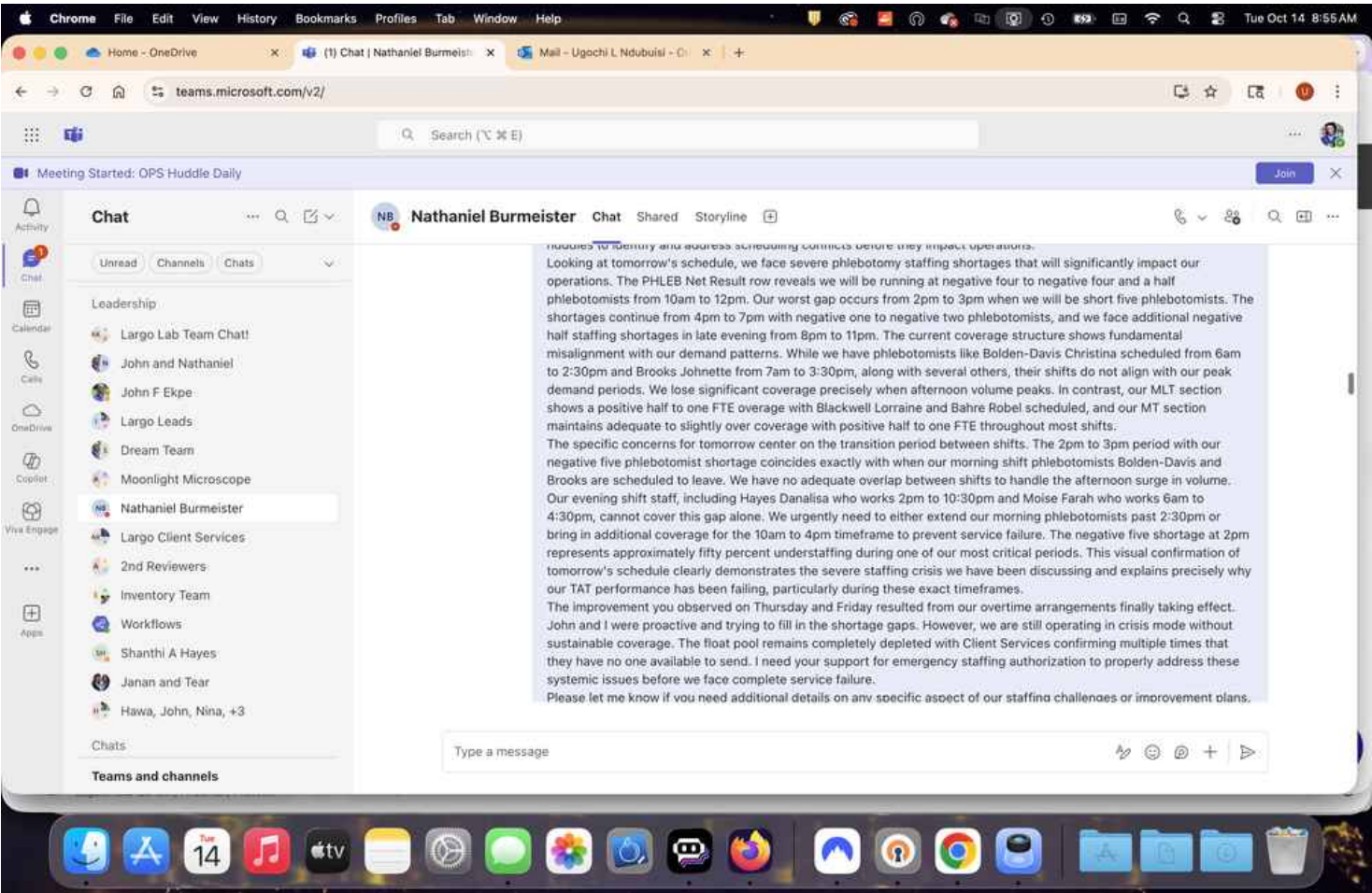
Evidence 14: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.56.10AM

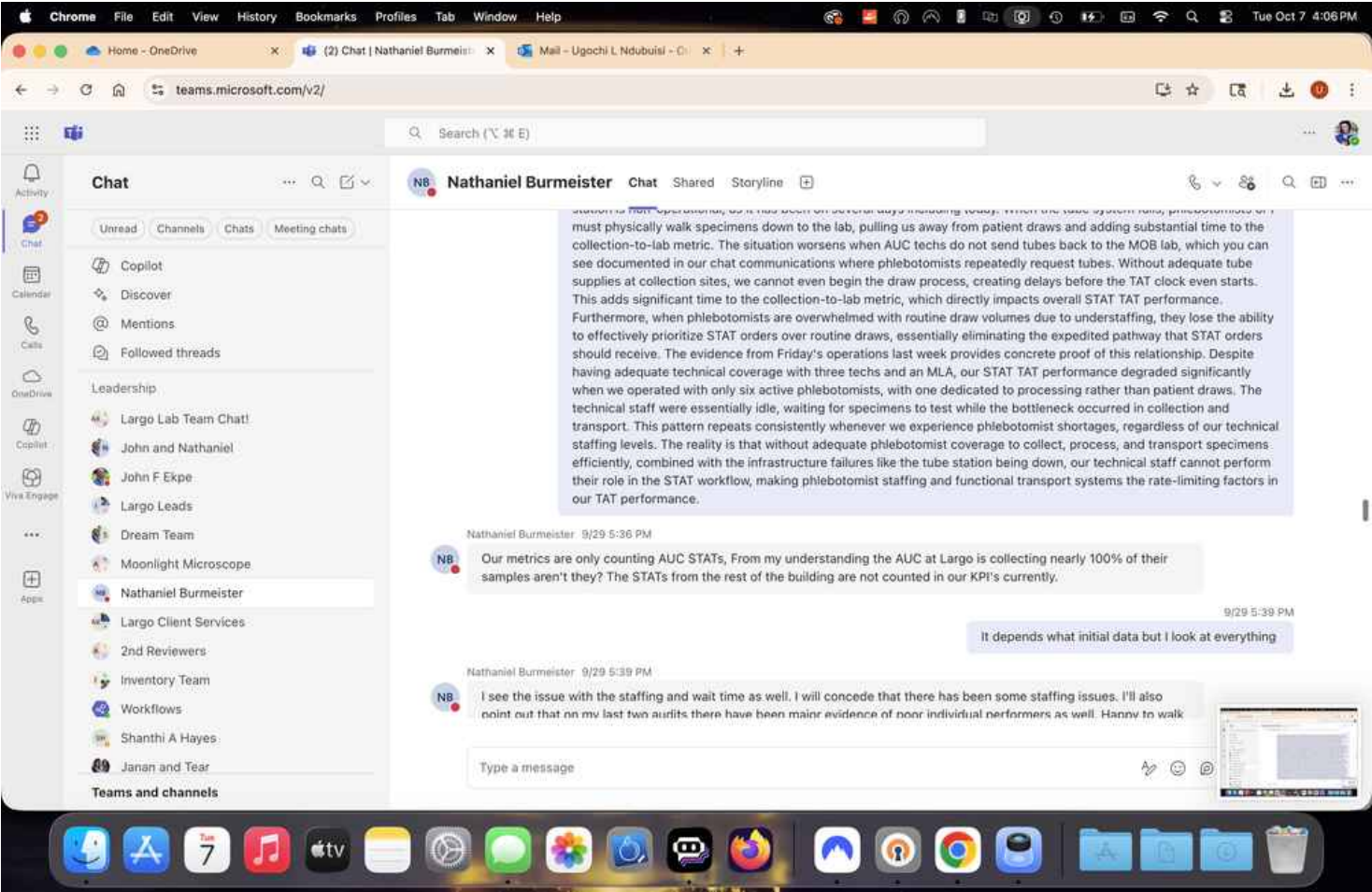


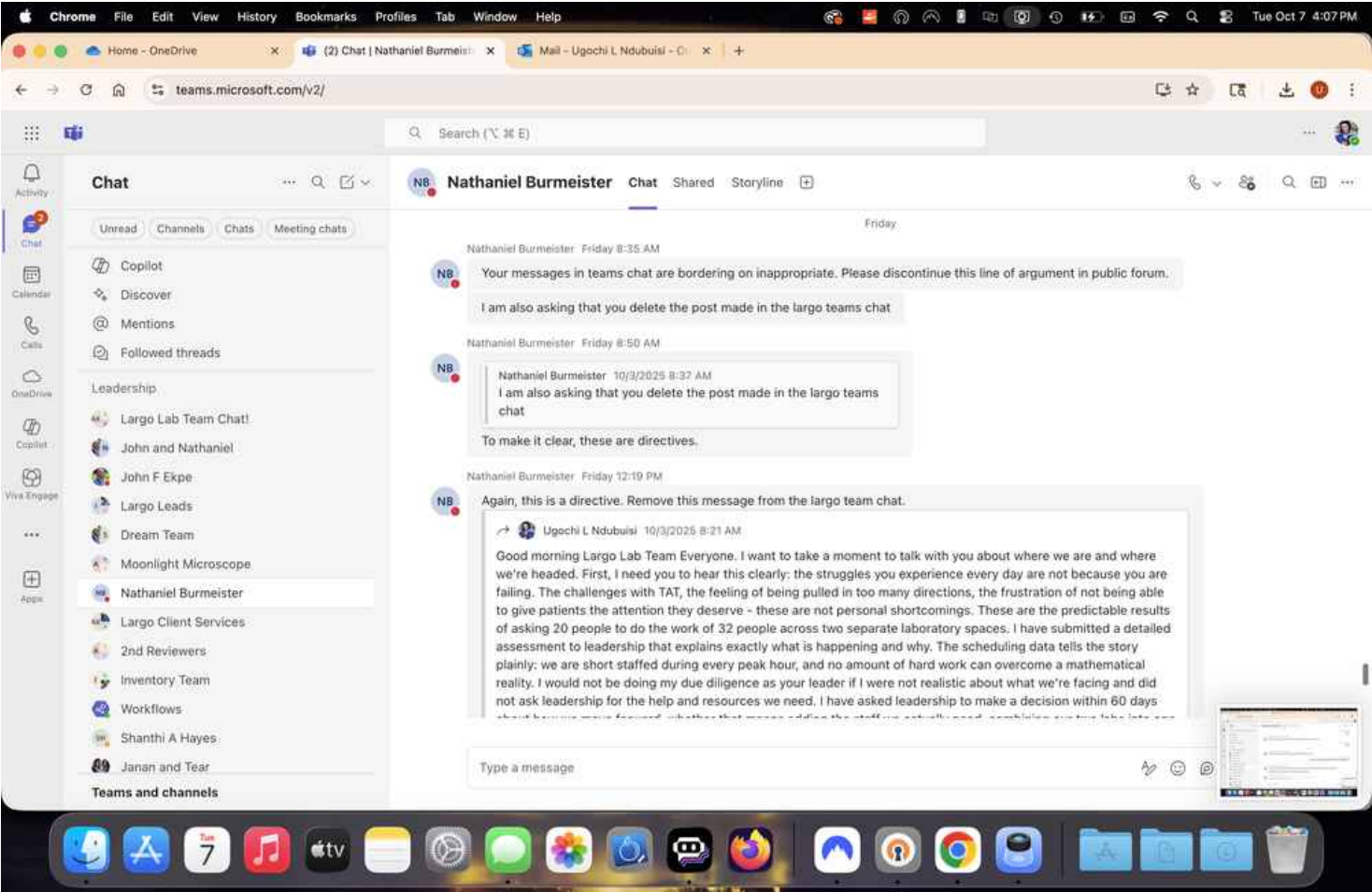


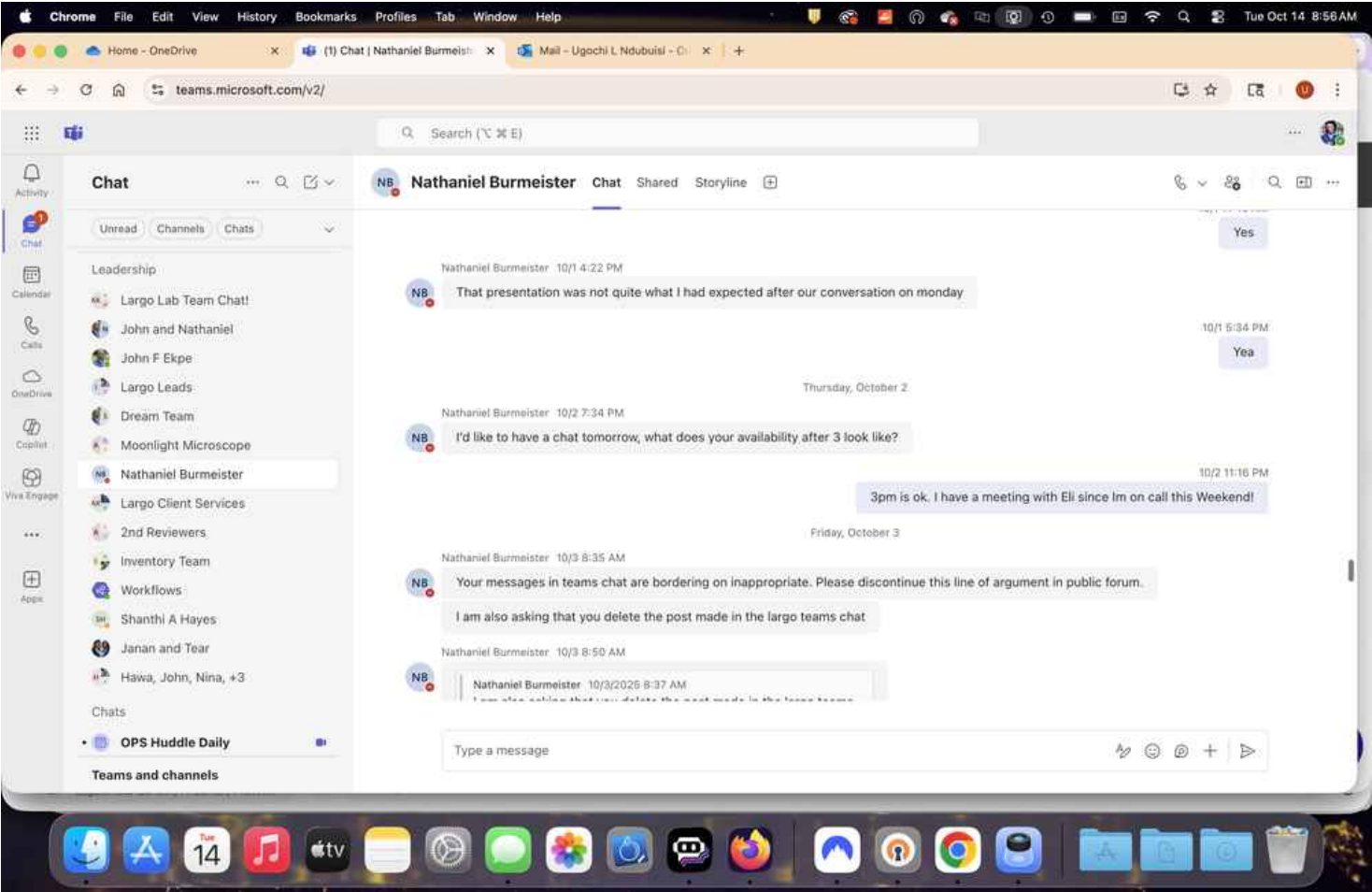


Evidence 17: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.55.19AM

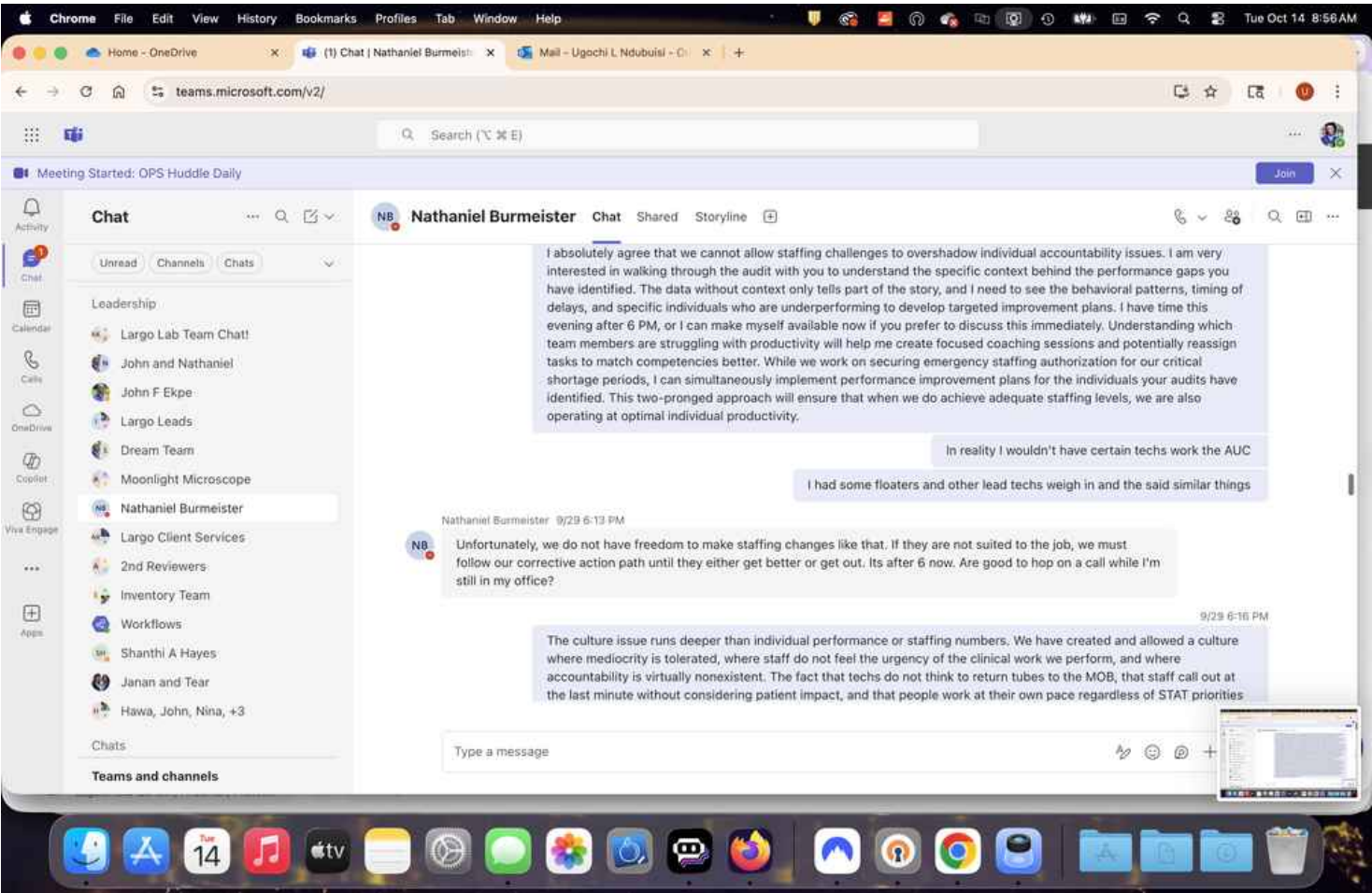


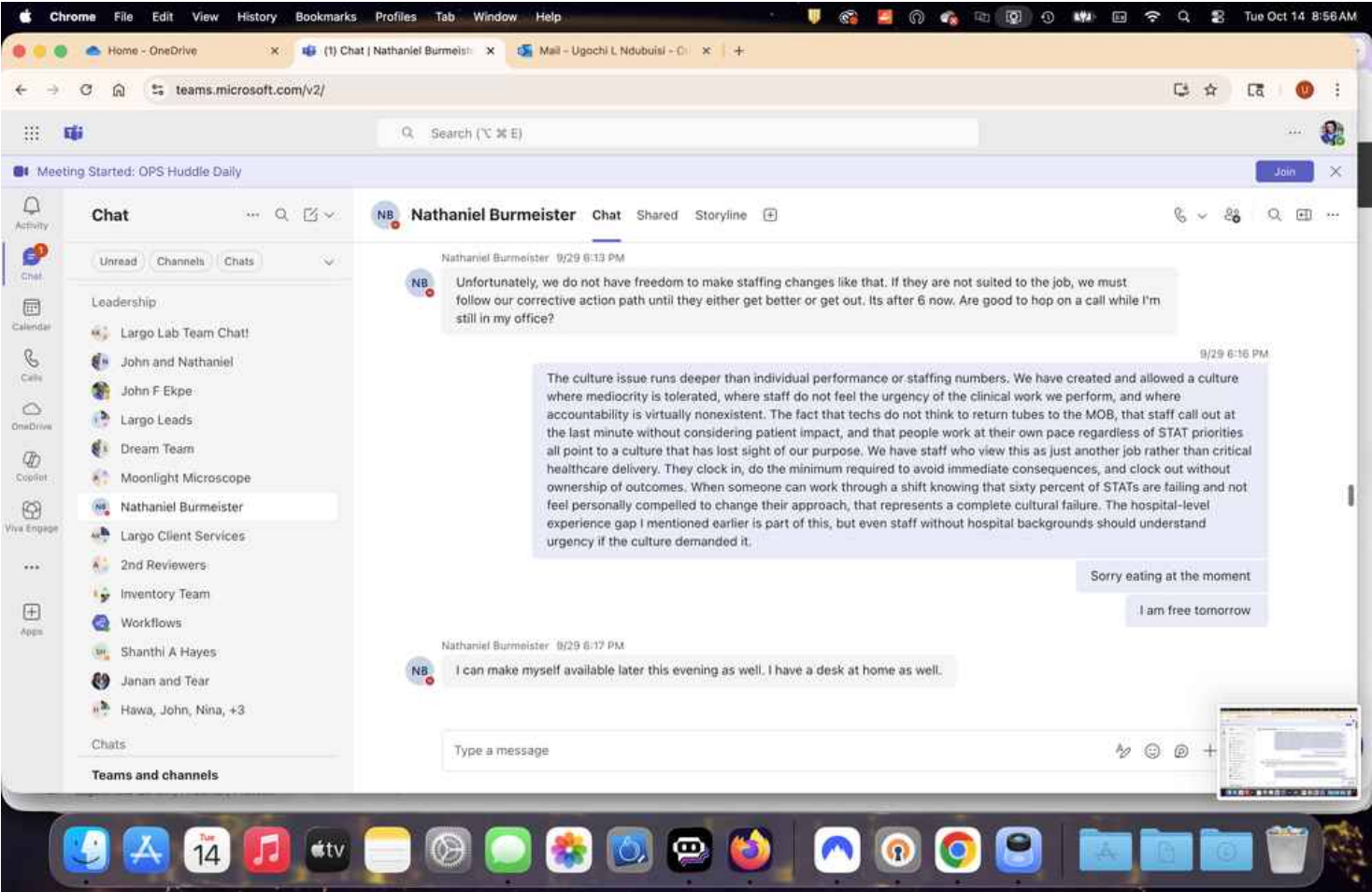


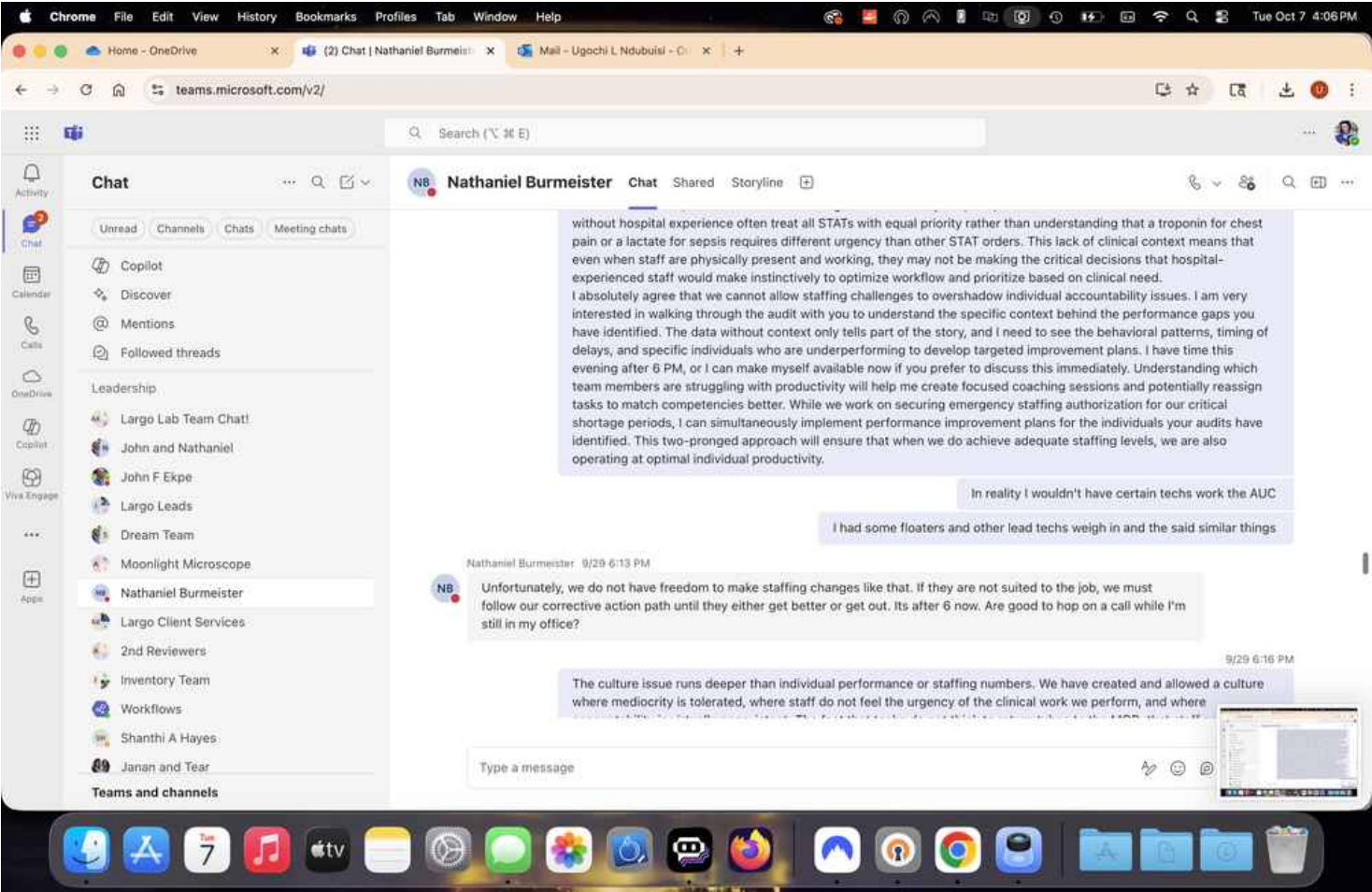


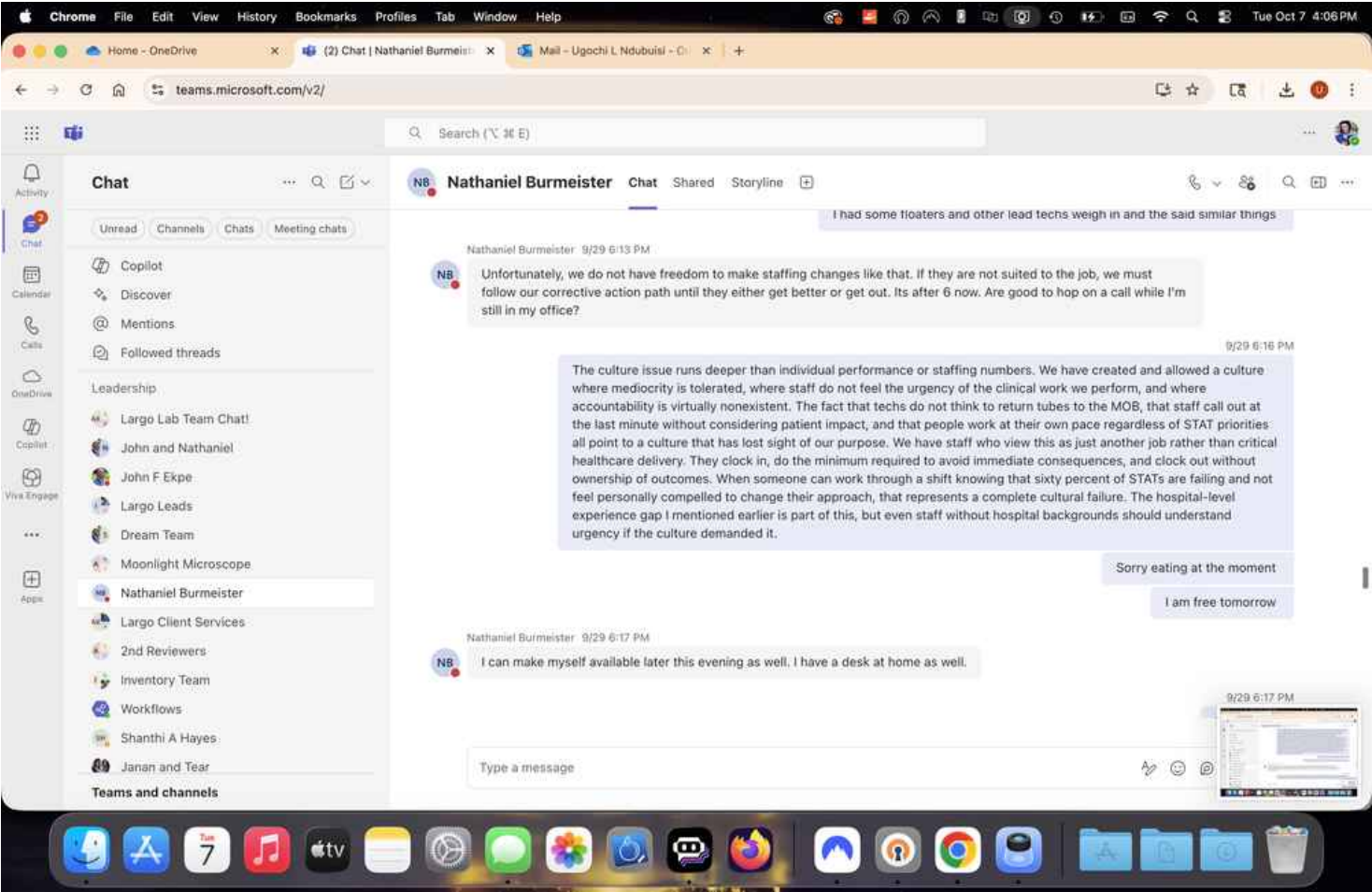


Evidence 21: Oct_8_Final_Warning_Screenshot 2025-10-14 at 8.56.05AM









POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
ACCOUNTABLE DEPARTMENT National Human Resources	EFFECTIVE DATE 12/19/2022
DOCUMENT OWNER Chair, National HR Policy Roundtable	PAGE 1 of 3

1.0 Policy Statement

Employees are required to abide by applicable federal, state, and local laws or regulations, to comply with Kaiser Permanente (KP) policies and procedures, and to conduct themselves in accordance with KP's Principles of Responsibility. If an employee fails to comply with any of these or engages in misconduct (including inappropriate/disruptive behavior) or poor performance, the employee may be subject to corrective/disciplinary action, up to and including termination. In addition, the violation of certain laws or regulations may require KP to report the violation to a regulatory or licensing agency and/or may subject an employee to criminal prosecution as well as civil penalties involving substantial monetary fines.

Although represented employees are covered by this policy, the levels of corrective action for represented employees will be determined according to the provisions of the applicable collective bargaining agreement.

Regional procedures must be consistent with this National policy but may include more detailed information regarding corrective/disciplinary action, and employees are subject to those procedures as well.

2.0 Purpose

Kaiser Permanente (KP) must comply with specific legal/regulatory standards that include, but are not limited to, those indicated by Medicare and other government program billing requirements, Guidance issued by the Centers for Medicare and Medicaid Services, the Department of Health and Human Services, the Compliance Program Guidance issued by the Office of the Inspector General, and HIPAA/Privacy and Security regulations. (See Section 6.0, References) KP is required to have well publicized disciplinary guidelines that demonstrate its commitment to compliance and set forth the consequences for violations of compliance mandates.

3.0 Scope/Coverage

This policy applies to all individuals who are employed by the following entities (collectively referred to as "Kaiser Permanente"):

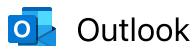
- 3.1** Kaiser Foundation Health Plan, Inc. (KFHP);
- 3.2** Kaiser Foundation Hospitals (KFH);
- 3.3** KFHP/H subsidiaries;
- 3.4** The Permanente Medical Group, Inc. (TPMG) [NOTE: This policy does not apply to physicians, podiatrists, vice presidents, or members of the TPMG Executive Staff, who are covered by separate TPMG policies]; and
- 3.5** Southern California Permanente Medical Group (SCPMG) [NOTE: This policy does not apply to physicians of SCPMG].

4.0 Definitions: n/a

POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
ACCOUNTABLE DEPARTMENT National Human Resources	EFFECTIVE DATE 12/19/2022
DOCUMENT OWNER Chair, National HR Policy Roundtable	PAGE 2 of 3

5.0 Provisions

- 5.1** Except as limited by collective bargaining agreements and/or statute, employment with KP is “at will” (see *At-Will Employment*, NATL.HR.37).
- 5.2** In the event that it is necessary to take corrective/disciplinary action to address concerns related to an employee’s work performance or conduct, the corrective/disciplinary action may include, but is not limited to:
 - 5.2.1** verbal discussion, coaching, and/or warning by the employee’s immediate supervisor or higher-level manager to correct the problem
 - 5.2.2** written notice, with or without final warning
 - 5.2.3** suspension, with or without final warning
 - 5.2.4** termination of employment
- 5.3** The corrective/disciplinary action steps listed in section 5.2 are not exhaustive and serve only as a description of possible action. None of these actions are mandatory or a prerequisite for any other actions that may be appropriate based on the circumstances. Any or all of these actions may be employed prior to termination and certain employee conduct may be deemed sufficiently serious to warrant immediate termination or escalated disciplinary action without prior corrective/disciplinary action.
- 5.4** The level of corrective/disciplinary action will be determined by management after review of the facts (including legal, compliance or policy violations), applicable regional/national collective bargaining agreement provisions, and consideration of other relevant factors and circumstances including, but not necessarily limited to, the factors in section 5.5 below. Management should consult Human Resources as appropriate.
- 5.5** Examples of factors to be considered in determining the level of corrective/disciplinary action include, but are not limited to, the following:
 - 5.5.1** the severity of the violation
 - 5.5.2** the number of violations
 - 5.5.3** whether the violation was part of a pattern or practice of improper behavior or non-compliance
 - 5.5.4** the individual’s past performance and conduct
 - 5.5.5** whether the individual knew or should have known the applicable policies, rules or regulations
 - 5.5.6** length of employment at KP
 - 5.5.7** whether the violation was intentional or negligent
 - 5.5.8** whether the action appeared to be committed for personal gain
 - 5.5.9** regional/national collective bargaining agreements corrective/disciplinary action provisions for represented employees



Outlook

RE: Lab delay

From Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>**Date** Fri 6/6/2025 3:41 PM**To** Alice J Gouvernayre <Alice.J.Gouvernayre@kp.org>; Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>; Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>**Cc** Sunny L Lambert <Sunny.L.Lambert@kp.org>; Carole R Kelly <Carole.R.Kelly@kp.org>; Nona Alexander <Nona.Alexander@kp.org>; Erin M Smith <Erin.M.Smith@kp.org>; Zachary Smith <Zachary.X3.Smith@kp.org>; James A Powell <James.A.Powell@kp.org>; Daniel J Patenaude <Daniel.J.Patenaude@kp.org>

Hello Dr. Gouvernayre,

Ugochi and I did a thorough investigation to discover root causes for the failures that were experienced on Monday. We talked about best strategy to mitigate that root cause, and I am confident that it is being addressed. As it is something that will include employee disciplinary actions, I will not go into details in this forum.

As far as regular state, we do normally perform maintenance on our instruments during our lower volume periods. Our major instrumentation all has redundancy. So even during regular maintenance, we have availability to test 24/7 (barring unscheduled downtime). For the Chemistry instruments, we perform one instrument between 9p-12a and the other is performed between 4a-8a. The process usually takes about 1-2 hours, and we have the flexibility to delay up to a few hours if testing demands are high. Ugochi and John are monitoring and reviewing for possible optimizations on this and other areas of the lab.

Ugochi and John will continue to reinforce the use of electronic communication through KP Healthconnect chat as best practice. I'll also continue to connect them with coaching and resources to best serve the operations needs of the Lab and by extension the AUC.

-Nate Burmeister

C:202-570-9030

From: Alice J Gouvernayre <Alice.J.Gouvernayre@kp.org>**Sent:** Friday, June 6, 2025 3:10 PM**To:** Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>; Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>**Cc:** Sunny L Lambert <Sunny.L.Lambert@kp.org>; Carole R Kelly <Carole.R.Kelly@kp.org>; Nona Alexander <Nona.Alexander@kp.org>; Erin M Smith <Erin.M.Smith@kp.org>; Zachary Smith <Zachary.X3.Smith@kp.org>; Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>; James A Powell <James.A.Powell@kp.org>; Daniel J Patenaude <Daniel.J.Patenaude@kp.org>**Subject:** RE: Lab delay

Hi Ugochi,

Thank you for taking the time to explain the situation so thoroughly to us. Can you tell us when the QC for Pure 2 is scheduled to occur?

I have a few questions/suggestions based on the information you provided.

1. Communication:

- a. I understand that you came over that night to inform the team of the issues you were having. It's really nice of you to do that, but not everyone gets the message when you come in person. I think the best way to make sure we all understand what's going on, is for you or another manager to send a detailed message about the issue and its impact to the lead nurse on epic chat. The lead nurse will then copy that message and send it to all that are working in the AUC that day. This workflow is good for temporary delays. If an instrument is down or for any other major issue, please continue to notify the DCSM critical operation alert on epic chat.

Reason: If physicians and nurses understand the lab limitations, we are then able to adjust the plan of care for patients appropriately (i.e. send patients home and message them on kp.org when the results are available versus the traditional path of having the patients wait in the department for results). Or we can set expectations with patients right off the bat on how long it will take to figure out what's wrong with them. The sooner we know, the better.

- b. Regarding the high volume of calls – I understand how disturbing this might have been. I think if we follow the communication pathway outlined above, you will get less calls. I also do want to point out that we wouldn't be doing our job if we left a patient waiting for a strep test result for hours without inquiring about the result repeatedly on their behalf.

1. Mitigation of impacts:

- a. I understand the chemistry machine was down and the urinalysis instrument was also not working but why did we have a 10-hour strep test delay? Or why were CBCs affected?
- b. What is the escalation pathway when you are short-staffed? What is the escalation pathway when the machines are malfunctioning? What is put in place to preserve operations across all testing instruments?

We look forward to working on solutions with you to improve operations and that we can present at the throughput meeting.

Best,

Alice J Gouvernayre, MD

Chief, Acute Care Services

DC and Suburban Maryland Service Area

Permanente Medicine

Mid-Atlantic Permanente Medical Group

Capitol Hill Medical Center

700 2nd St. NE,

Washington, DC 20002

202-853-1311 (mobile)

alice.j.gouvernayre@kp.org

For assistance:

Rodnee Petty at 202-680-3404 or rodnee.s.petty@kp.org

[Facebook](#) | [Twitter](#) | [LinkedIn](#) | [Instagram](#) | [YouTube](#)

Kaiser Permanente

From: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Sent: Tuesday, June 3, 2025 10:45 PM

To: Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>

WHISTLEBLOWER RETALIATION EVIDENCE

Largo Laboratory - Kaiser Permanente

EXECUTIVE SUMMARY

This document presents chronological evidence of retaliation against Ugochi L. Ndubuisi for protected whistleblowing activity. The timeline shows:

1. July 14, 2025: Director orders investigation
2. September 29, 2025: Analysis provided and acknowledged
3. October 3, 2025: Same analysis shared with staff (protected activity)
4. October 3, 2025: Four censorship demands in 8 hours
5. October 8, 2025: Final Written Warning (first discipline in 9 months)

Maryland OSHA Deadline: November 7, 2025

CRITICAL TIMELINE

Date/Time	Event	Significance
July 14, 2025	Investigation Ordered	Director orders investigation into second shift
Sept 29, 2025	Analysis Provided	"Culture issue runs deeper" - acknowledged by director
Oct 3, 8:21am	Whistleblowing	41.23% failure rate shared with staff
Oct 3, 8:35am	Censorship #1	"Inappropriate" - 14 minutes after sharing
Oct 3, 8:50am	Censorship #2	"Delete the post" - 29 minutes after
Oct 3, 12:19pm	Censorship #3	"This is a directive" - escalation
Oct 3, 4:30pm	Censorship #4	Fourth deletion demand
Oct 8, 2025	Final Warning	First discipline ever - clear retaliation

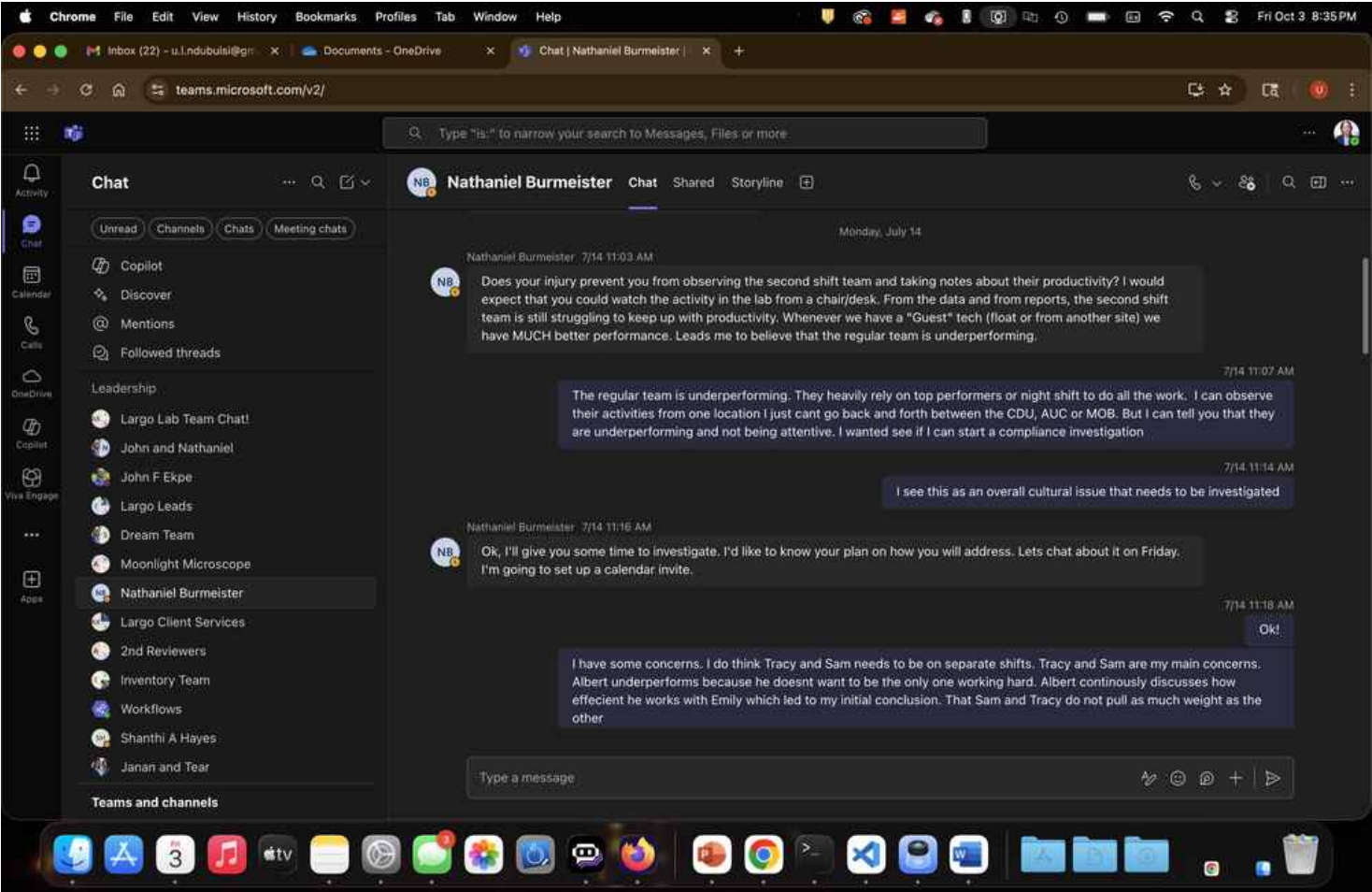
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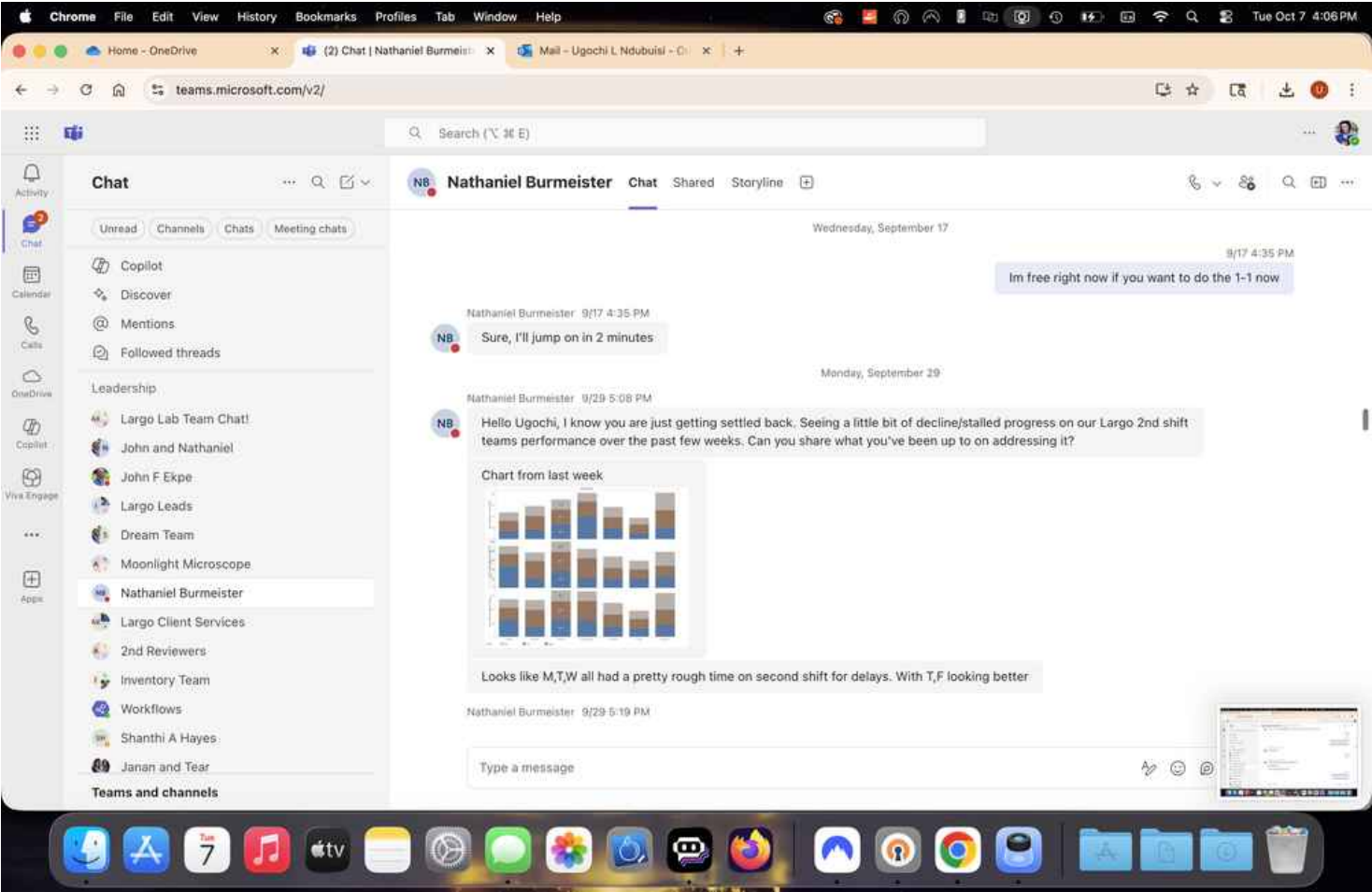
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- 2. Temporal Proximity:** Only 5 days between whistleblowing and Final Written Warning
- 3. Disparate Treatment:** Same analysis praised by director on Sept 29, punished when shared Oct 3
- 4. Censorship Pattern:** Four deletion demands in single day shows intent to silence
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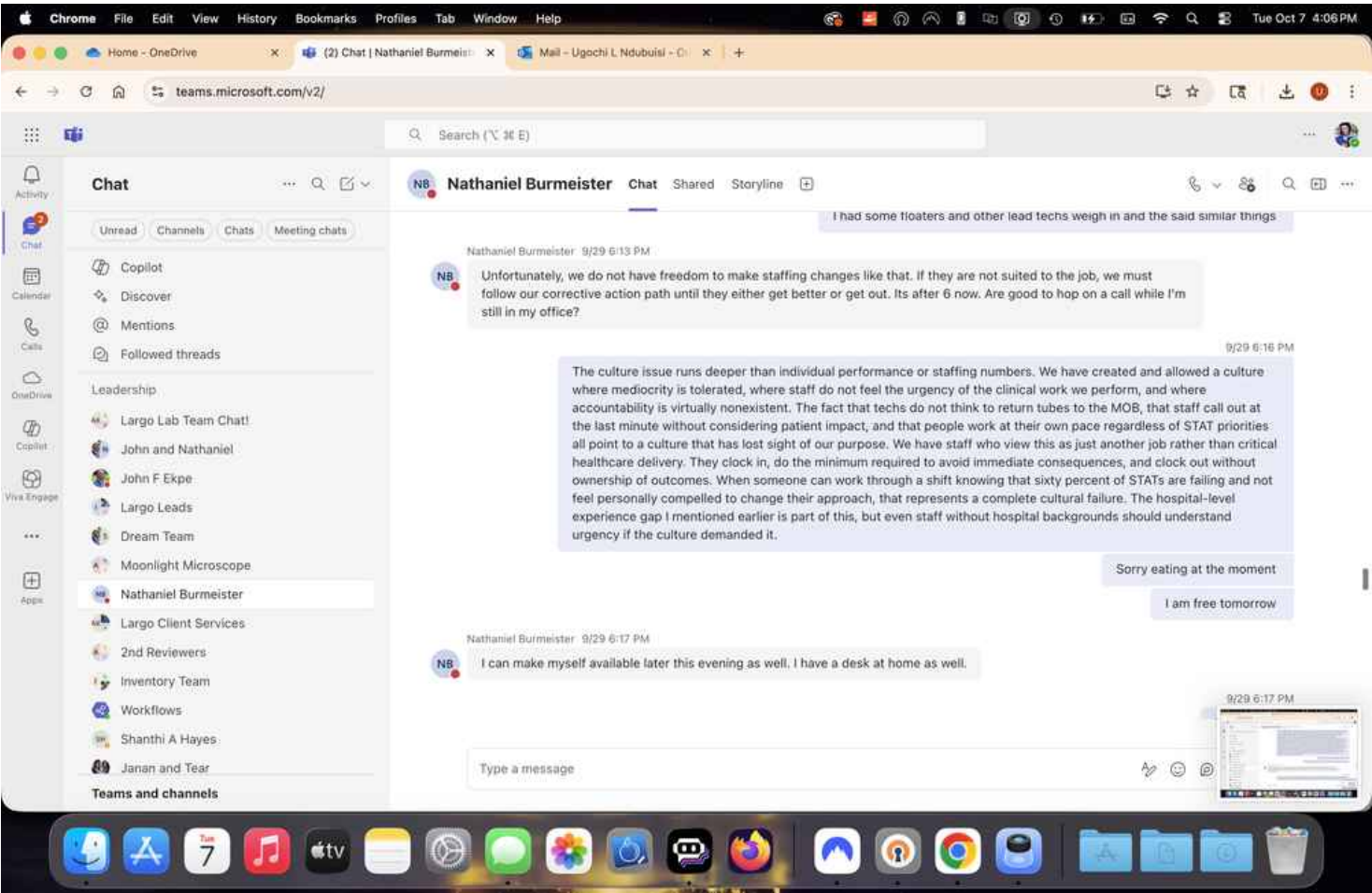
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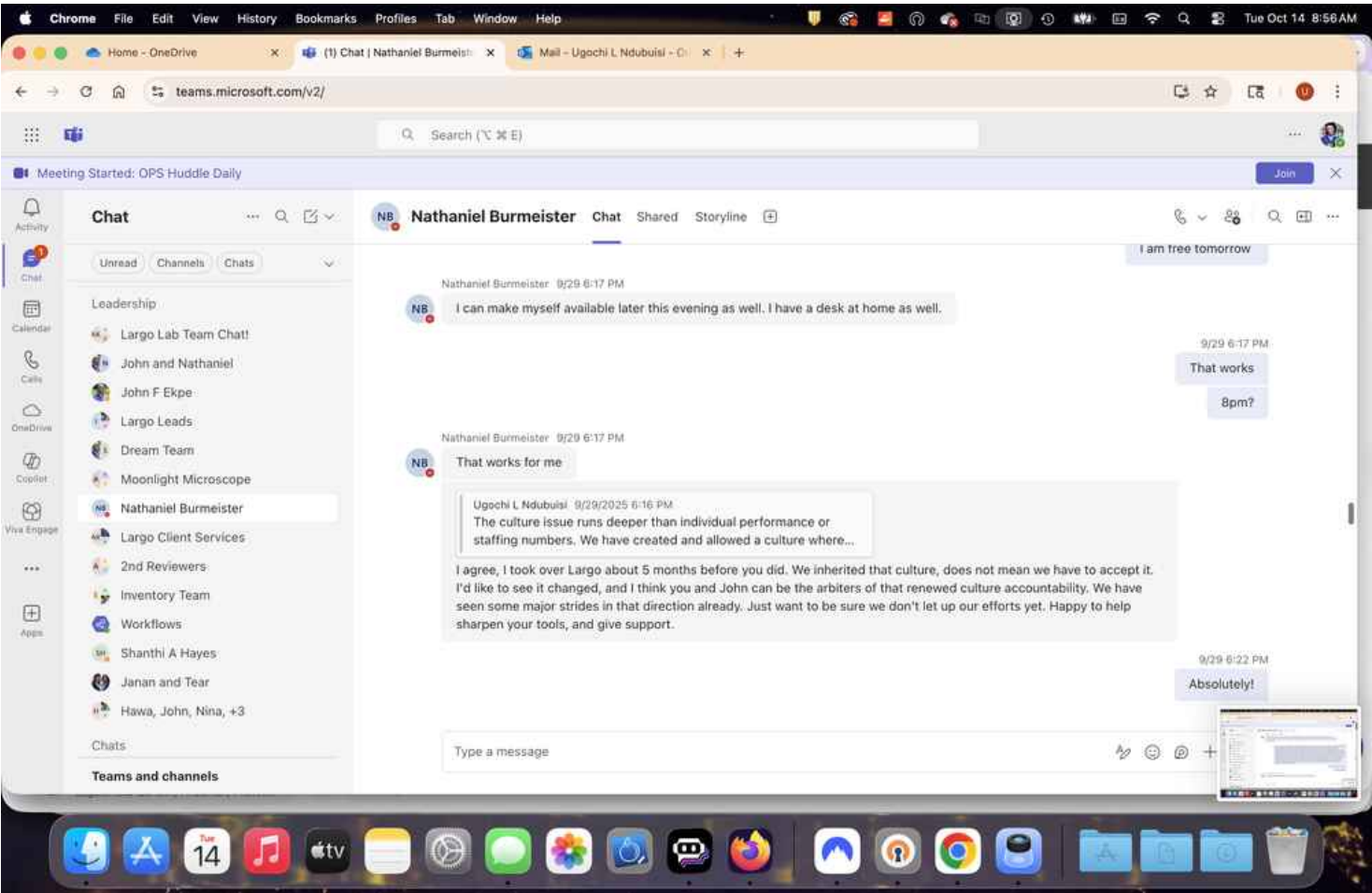


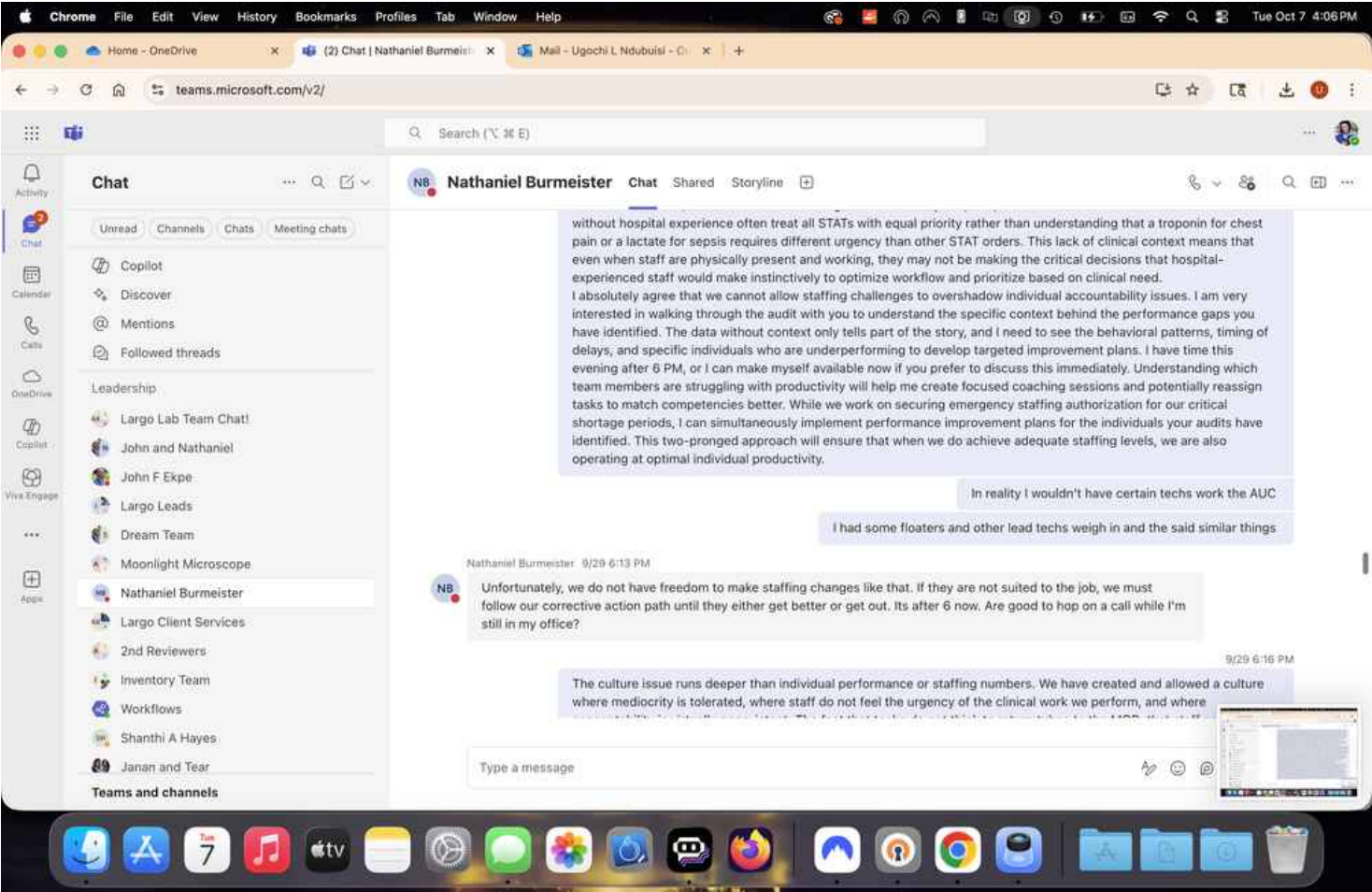


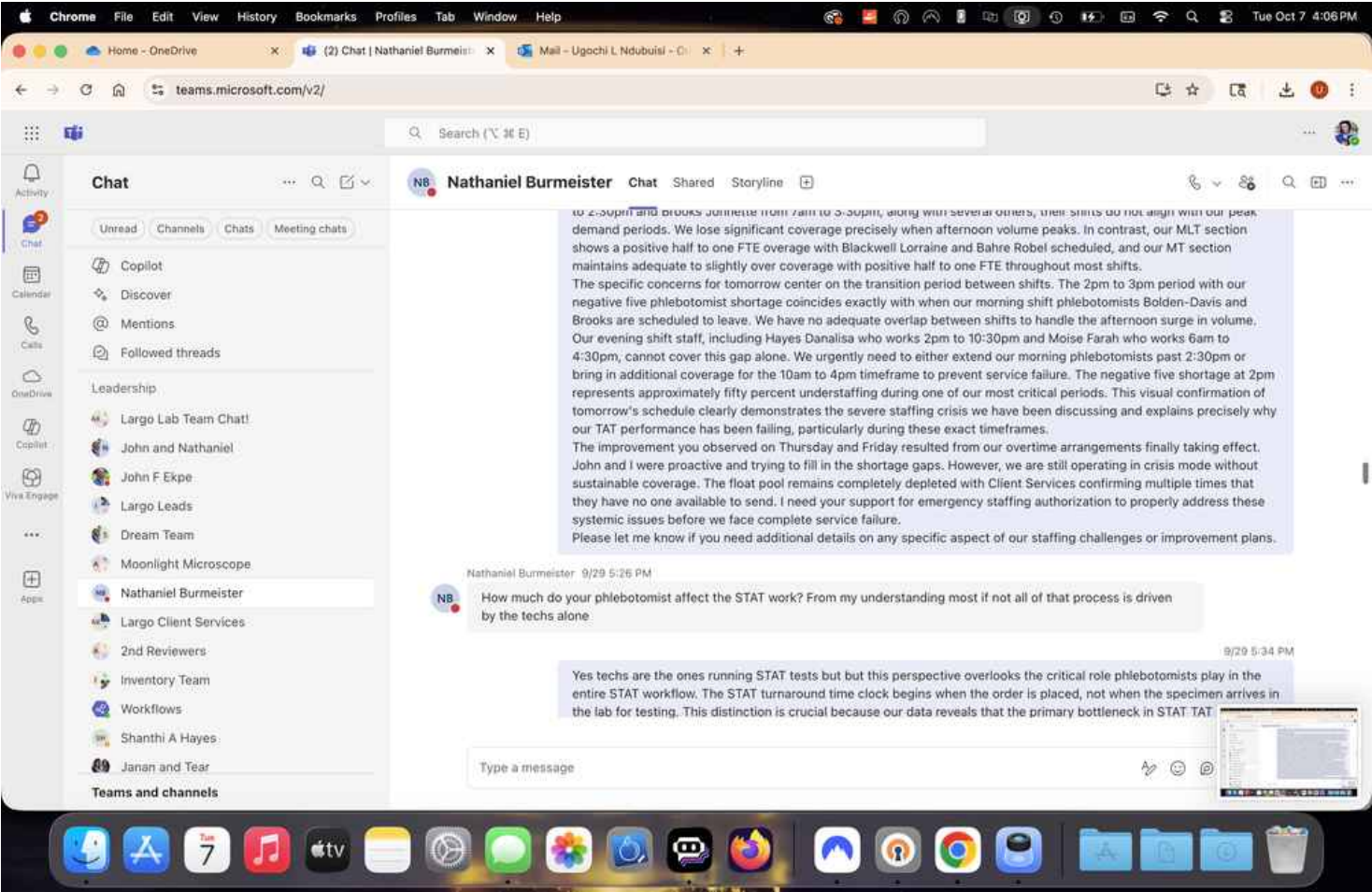
The screenshot shows a Microsoft Teams chat window on a Mac. The browser address bar at the top indicates the URL is teams.microsoft.com/v2/. The chat is titled "Meeting Started: OPS Huddle Daily". The chat history shows a conversation with Nathaniel Burmeister. The messages are as follows:

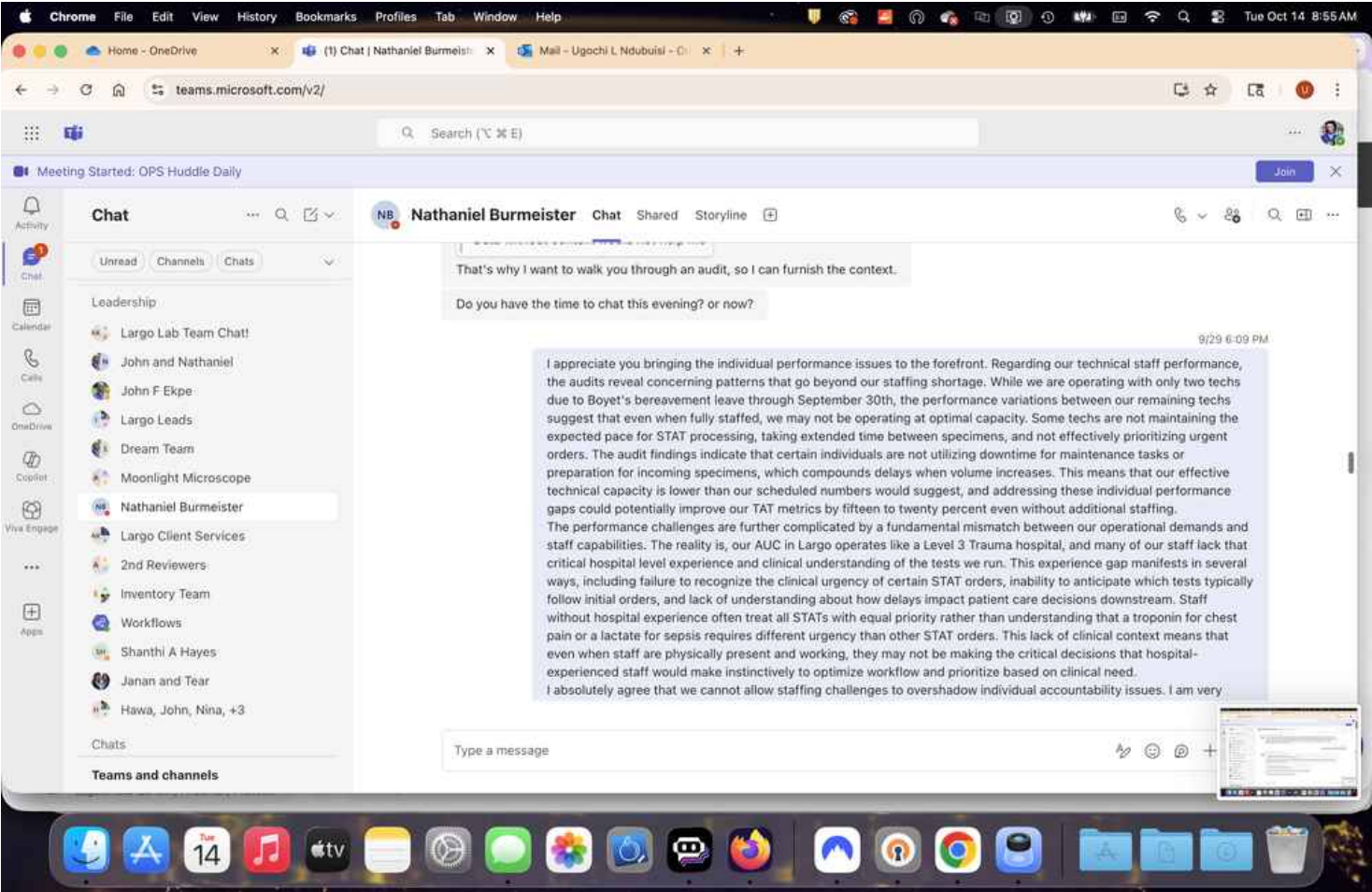
- Nathaniel Burmeister (9/29 6:13 PM): Unfortunately, we do not have freedom to make staffing changes like that. If they are not suited to the job, we must follow our corrective action path until they either get better or get out. Its after 6 now. Are good to hop on a call while I'm still in my office?
- Unread message (9/29 6:16 PM): The culture issue runs deeper than individual performance or staffing numbers. We have created and allowed a culture where mediocrity is tolerated, where staff do not feel the urgency of the clinical work we perform, and where accountability is virtually nonexistent. The fact that techs do not think to return tubes to the MOB, that staff call out at the last minute without considering patient impact, and that people work at their own pace regardless of STAT priorities all point to a culture that has lost sight of our purpose. We have staff who view this as just another job rather than critical healthcare delivery. They clock in, do the minimum required to avoid immediate consequences, and clock out without ownership of outcomes. When someone can work through a shift knowing that sixty percent of STATs are failing and not feel personally compelled to change their approach, that represents a complete cultural failure. The hospital-level experience gap I mentioned earlier is part of this, but even staff without hospital backgrounds should understand urgency if the culture demanded it.
- Response (9/29 6:16 PM): Sorry eating at the moment
- Response (9/29 6:16 PM): I am free tomorrow
- Nathaniel Burmeister (9/29 6:17 PM): I can make myself available later this evening as well. I have a desk at home as well.

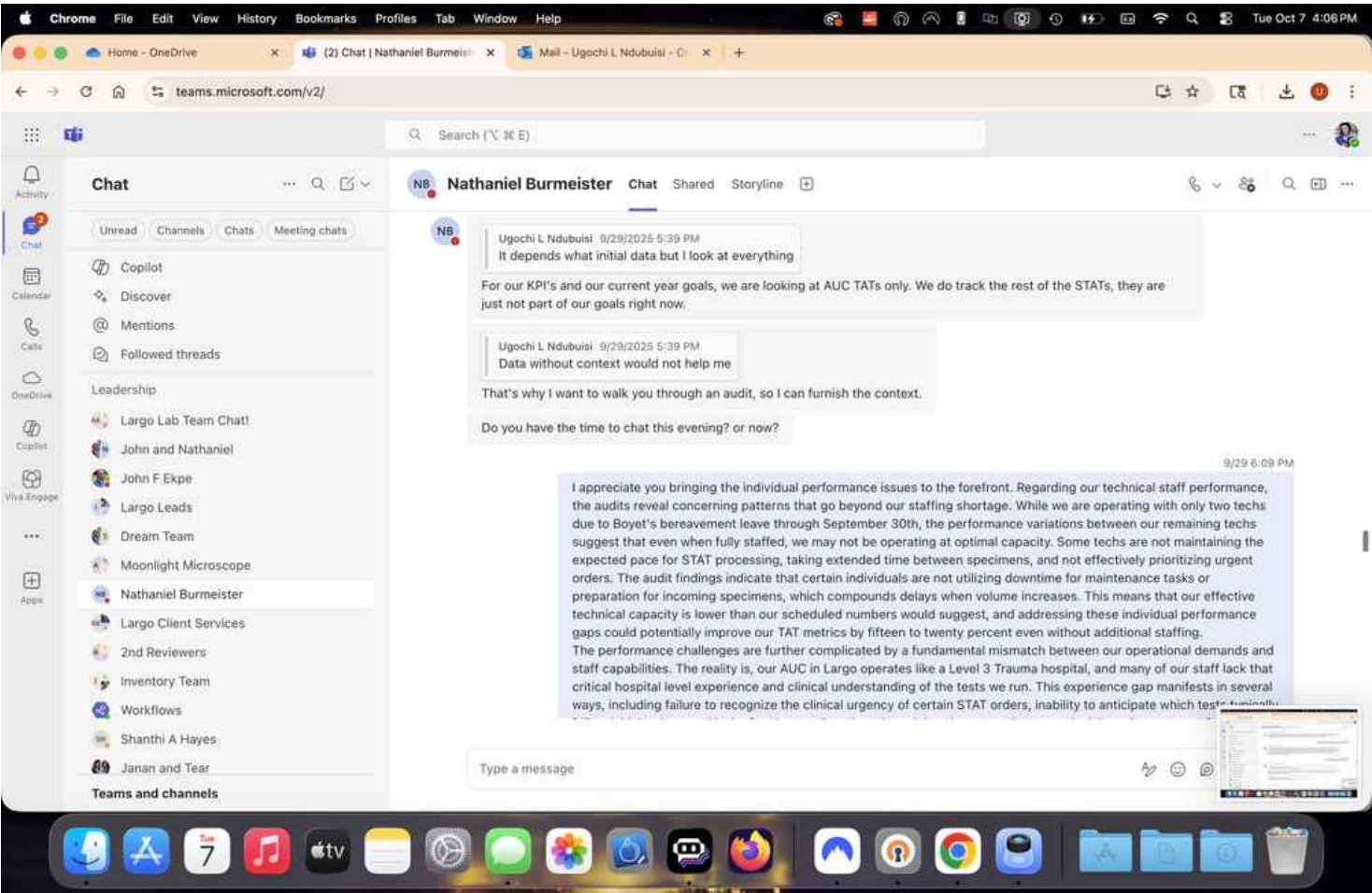
The interface includes a sidebar on the left with navigation options: Activity, Chat, Calendar, Calls, OneDrive, Copilot, Viva Engage, and Apps. The "Teams and channels" section lists various teams and channels, including "Largo Lab Team Chat!", "John and Nathaniel", "John F Ekpe", "Largo Leads", "Dream Team", "Moonlight Microscope", "Nathaniel Burmeister", "Largo Client Services", "2nd Reviewers", "Inventory Team", "Workflows", "Shanthi A Hayes", "Janan and Tear", and "Hawa, John, Nina, +3". The bottom of the screen shows the Mac dock with various application icons.

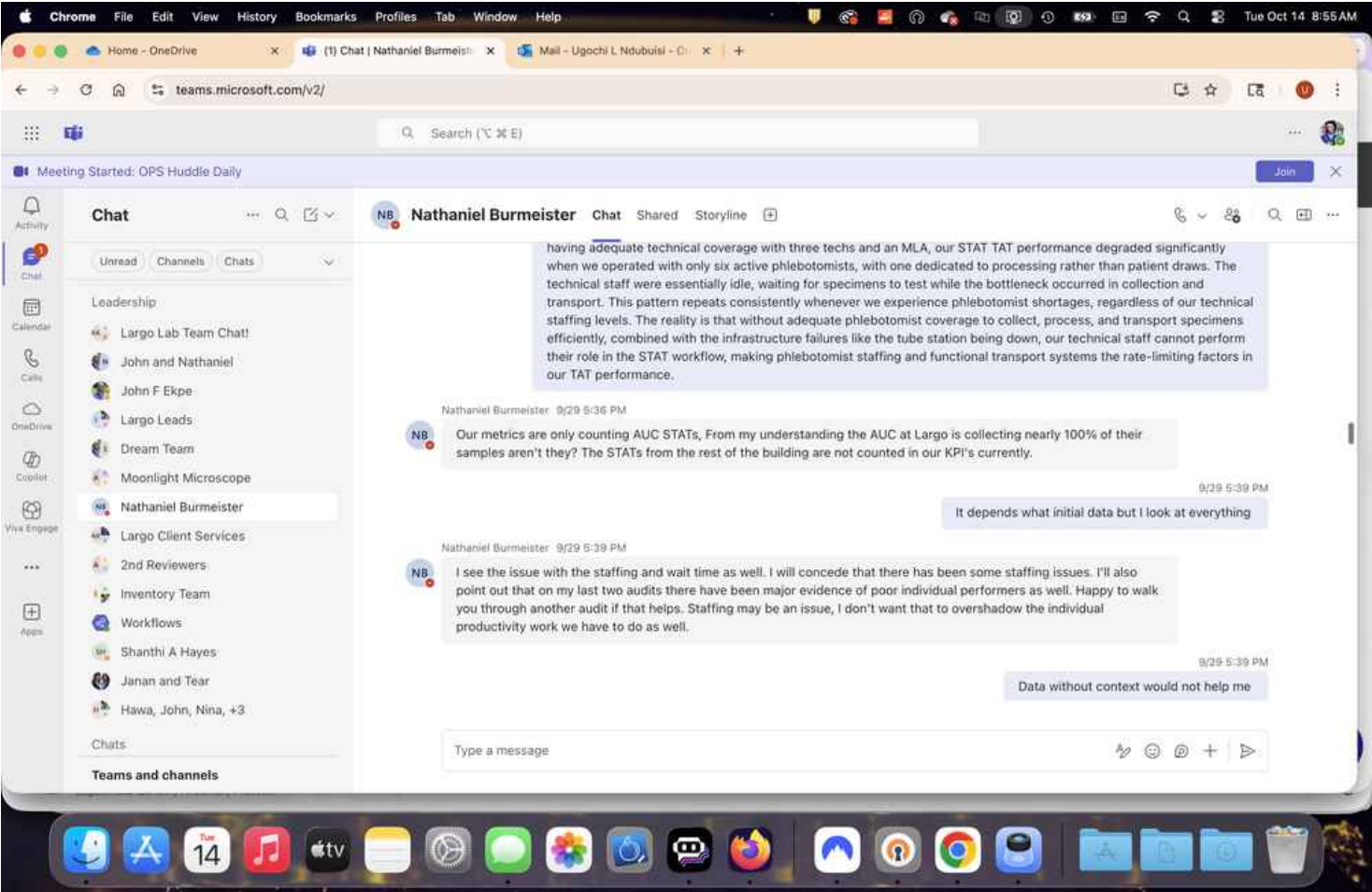


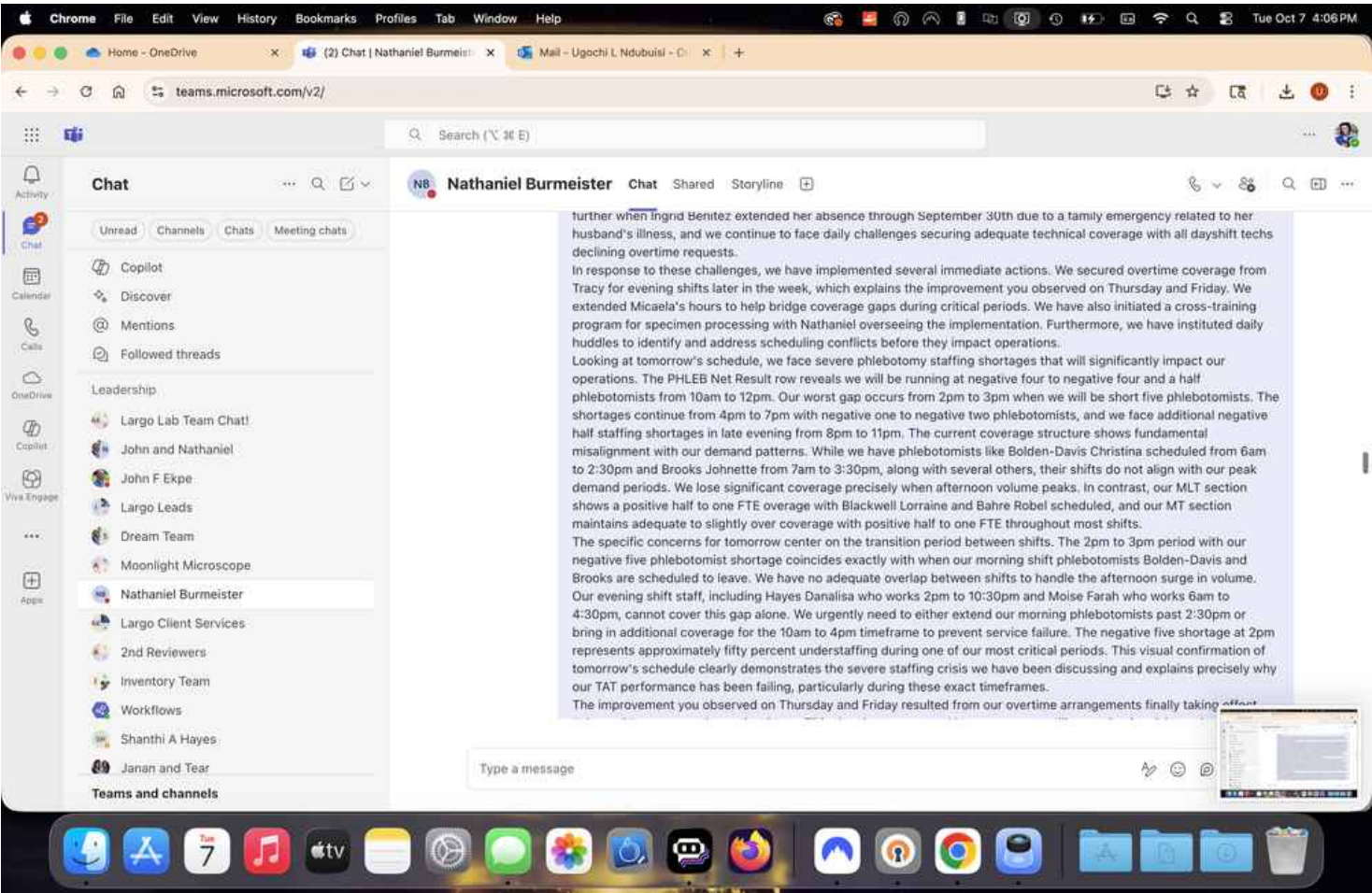


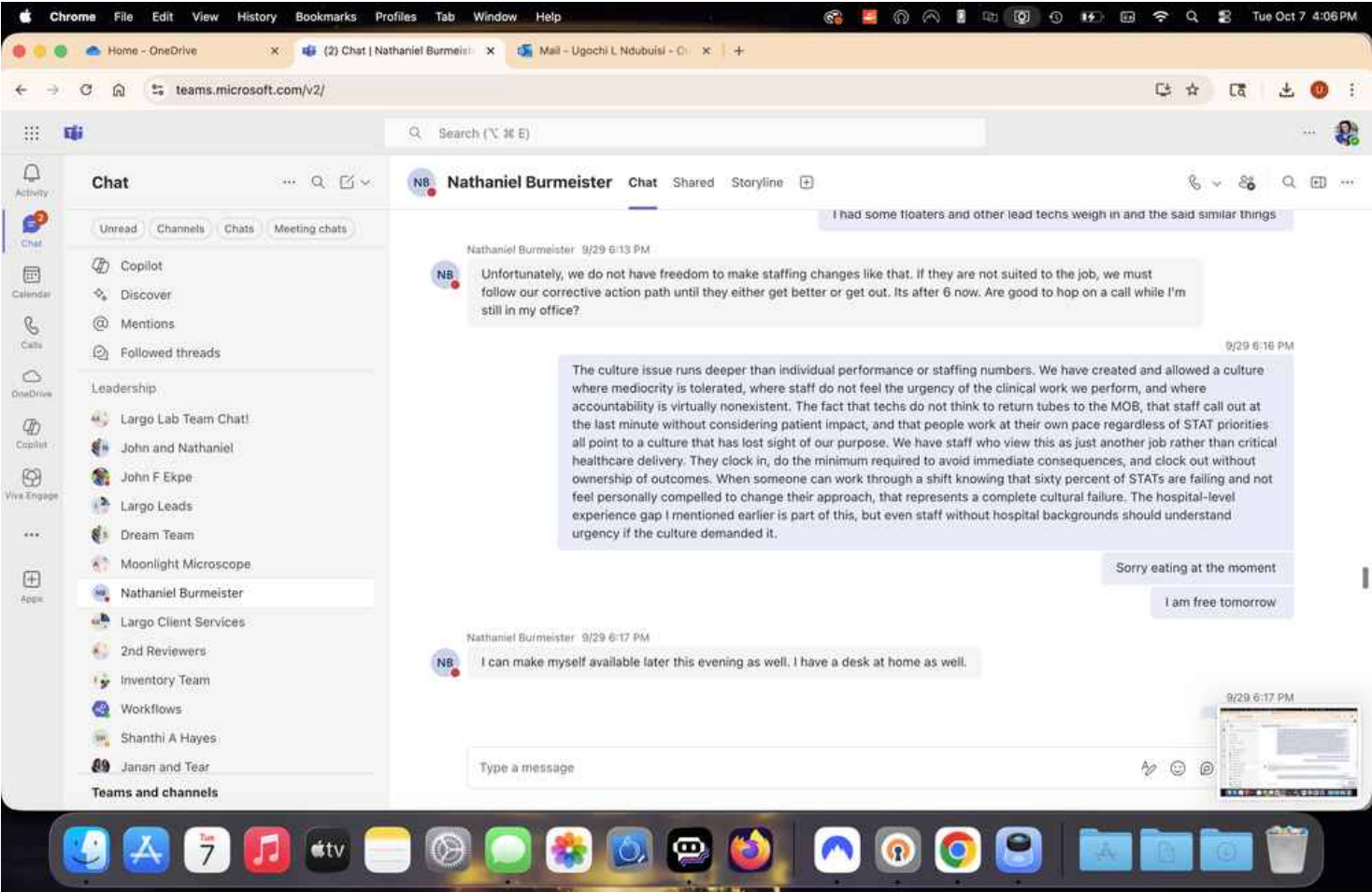




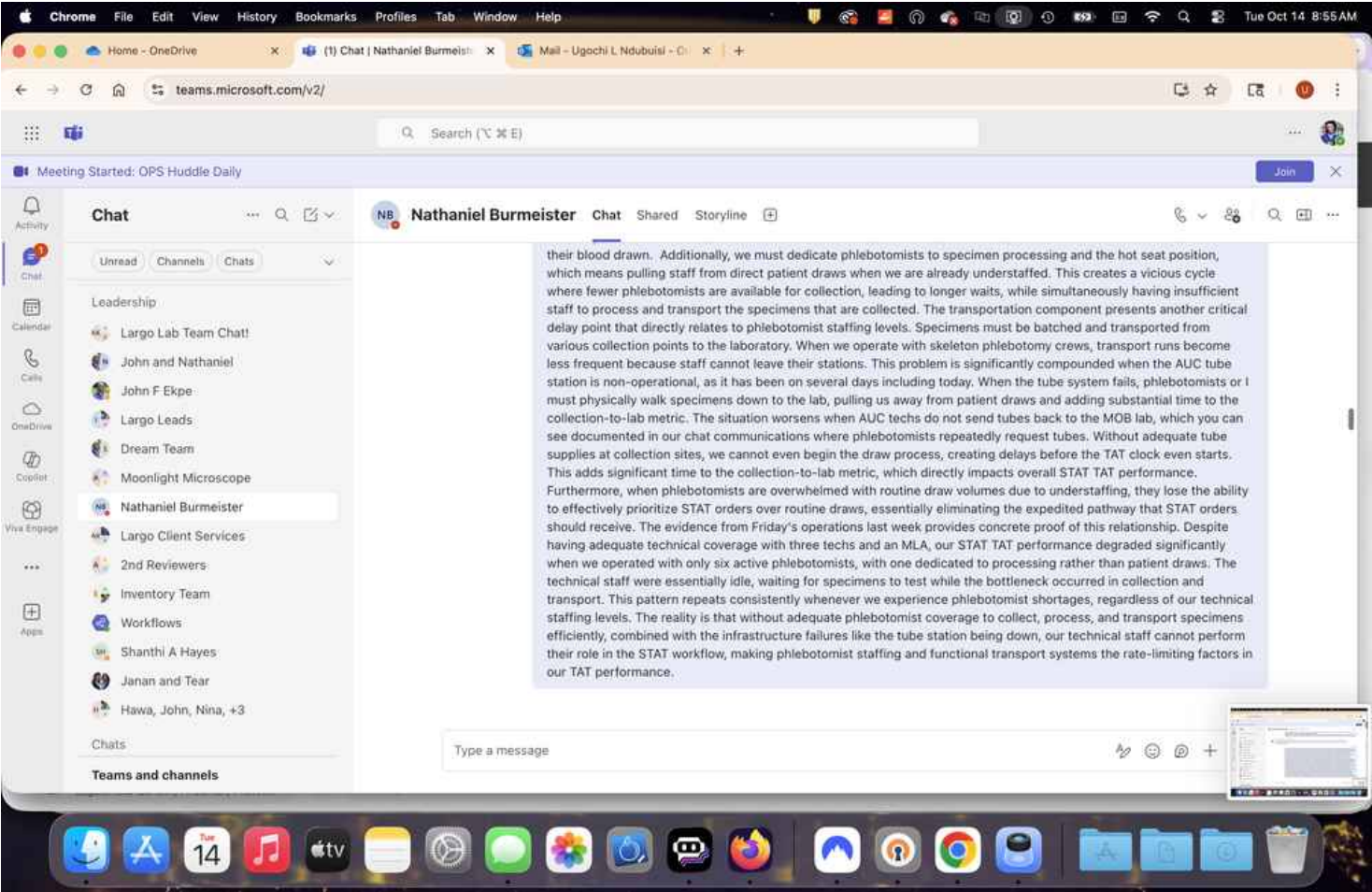




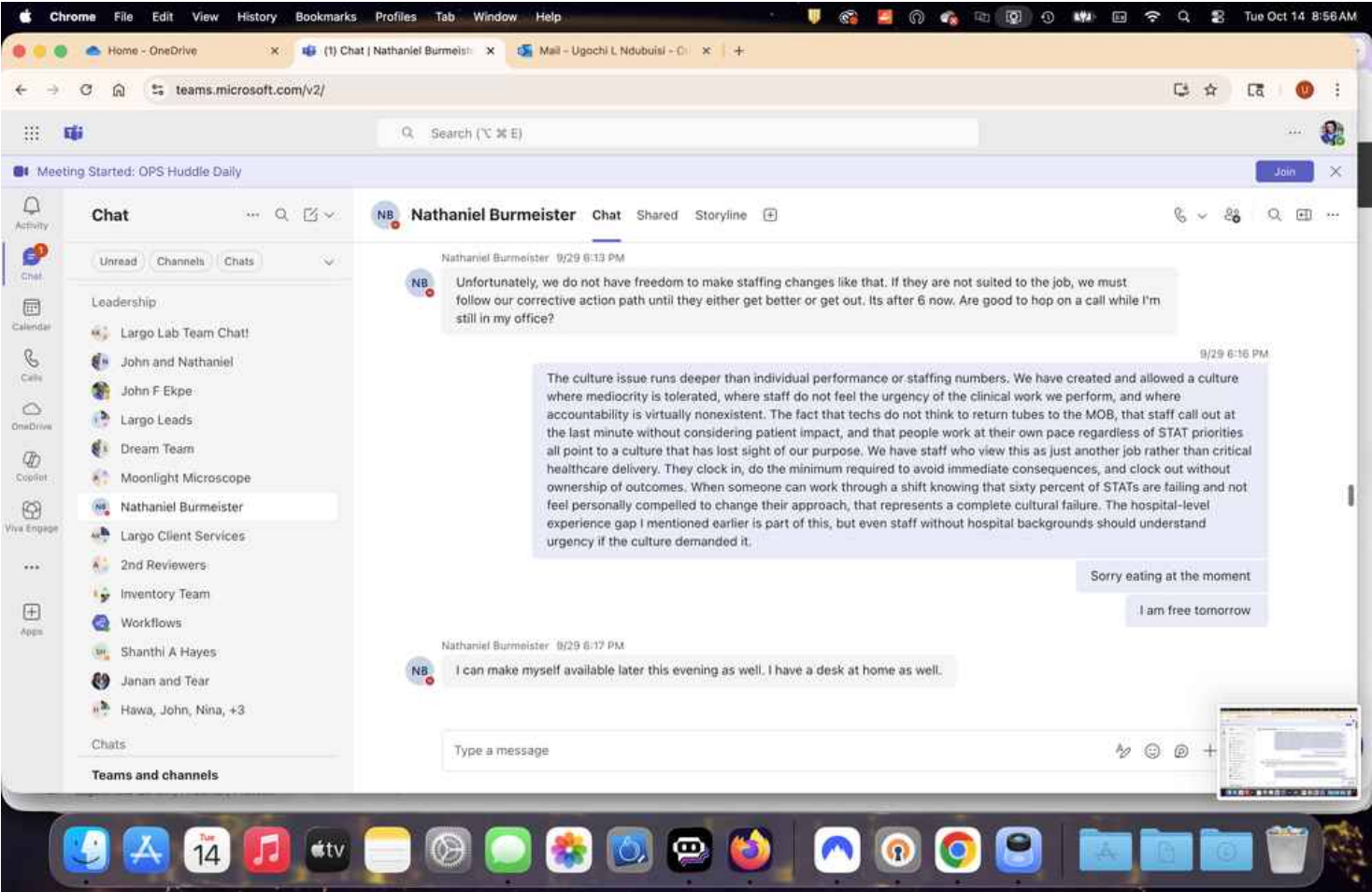


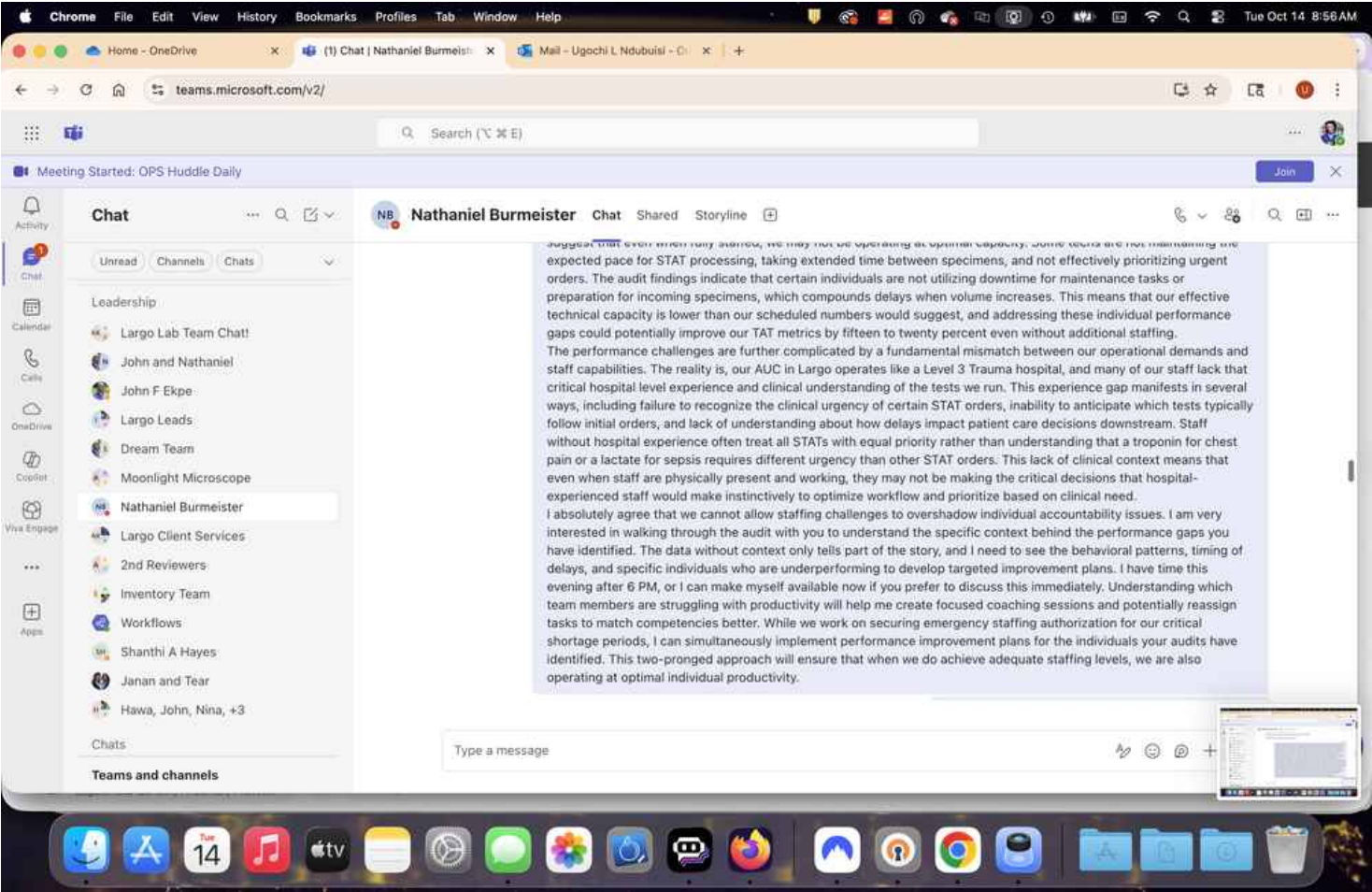


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Evidence 14: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.56.10AM





The screenshot shows a Microsoft Teams chat interface on a Mac. The top of the screen displays the macOS menu bar with various system icons and the time 'Tue Oct 14 8:55 AM'. The browser's address bar shows the URL 'teams.microsoft.com/v2/'. The chat window is titled 'Meeting Started: OPS Huddle Daily'. On the left sidebar, there's a list of 'Teams and channels' including 'Largo Lab Team Chat!', 'John and Nathaniel', 'John F Ekpe', 'Largo Leads', 'Dream Team', 'Moonlight Microscope', 'Nathaniel Burmeister', 'Largo Client Services', '2nd Reviewers', 'Inventory Team', 'Workflows', 'Shanthi A Hayes', 'Janan and Tear', and 'Hawa, John, Nina, +3'. The main chat area shows a conversation with 'Nathaniel Burmeister'. The chat history includes a message from him at 9/29 5:26 PM asking, 'How much do your phlebotomist affect the STAT work? From my understanding most if not all of that process is driven by the techs alone'. A response from another user at 9/29 5:34 PM explains that techs run STAT tests but overlooks the critical role of phlebotomists, detailing a significant discrepancy in turnaround times (142 minutes vs. 3,489 minutes) and the impact of staffing shortages on the STAT workflow. The bottom of the screen shows the macOS dock with various application icons.

Home - OneDrive (1) Chat | Nathaniel Burmeister Mail - Ugochi L. Ndubuisi - Chat +

teams.microsoft.com/v2/

Meeting Started: OPS Huddle Daily

Chat

Unread Channels Chats

Leadership

- Largo Lab Team Chat!
- John and Nathaniel
- John F Ekpe
- Largo Leads
- Dream Team
- Moonlight Microscope
- Nathaniel Burmeister
- Largo Client Services
- 2nd Reviewers
- Inventory Team
- Workflows
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- Janan and Tear
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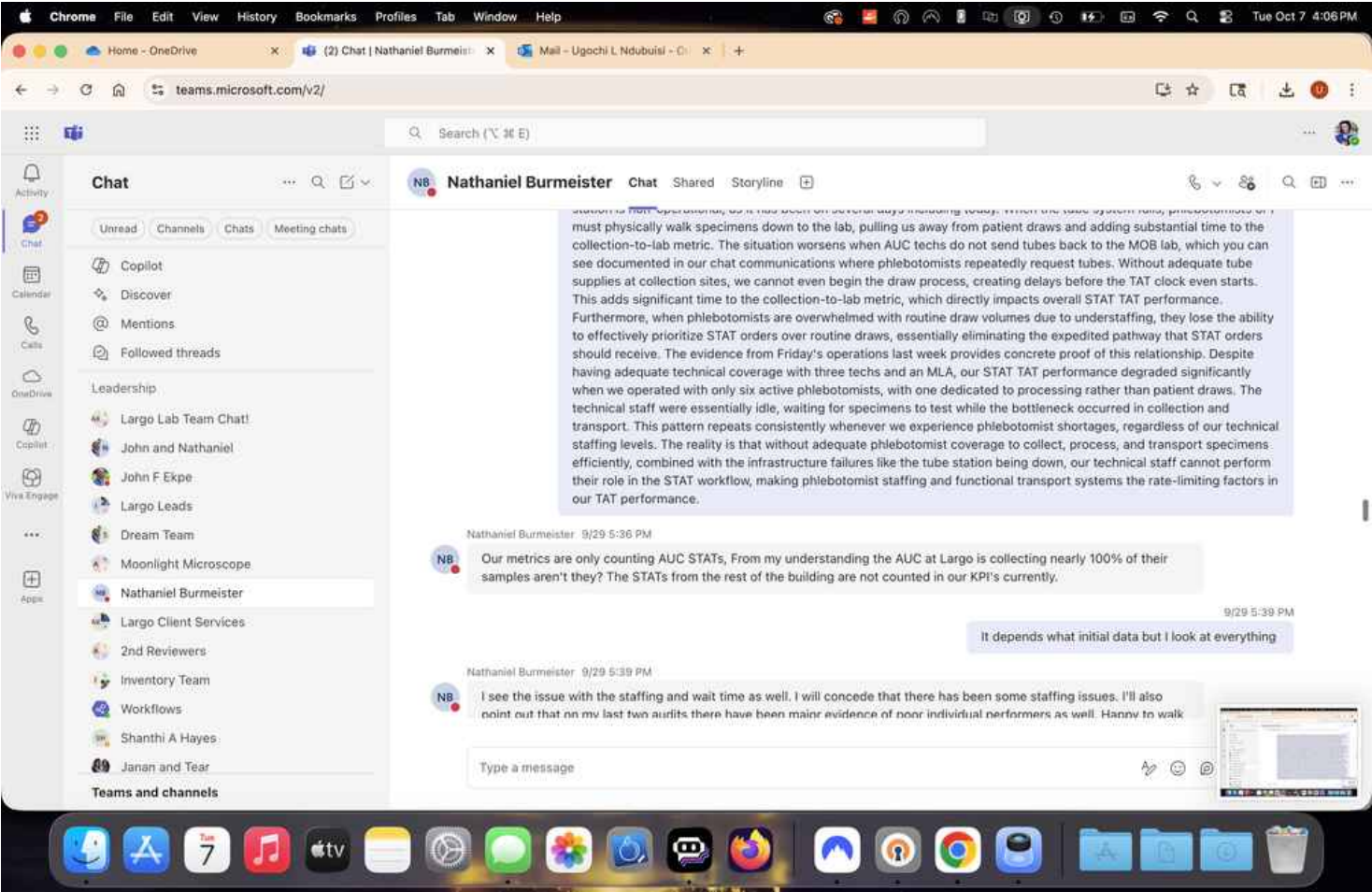
Chats

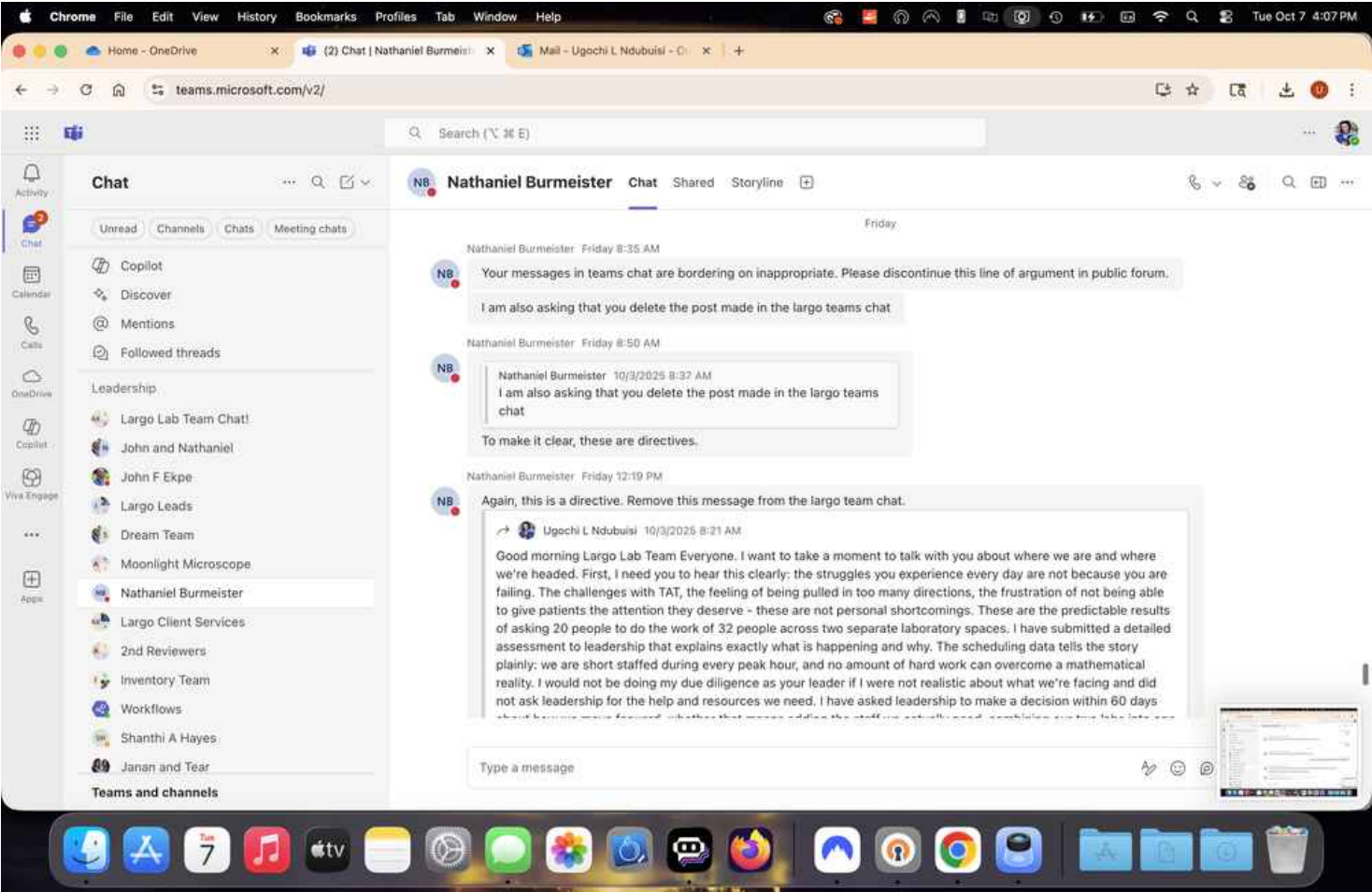
Teams and channels

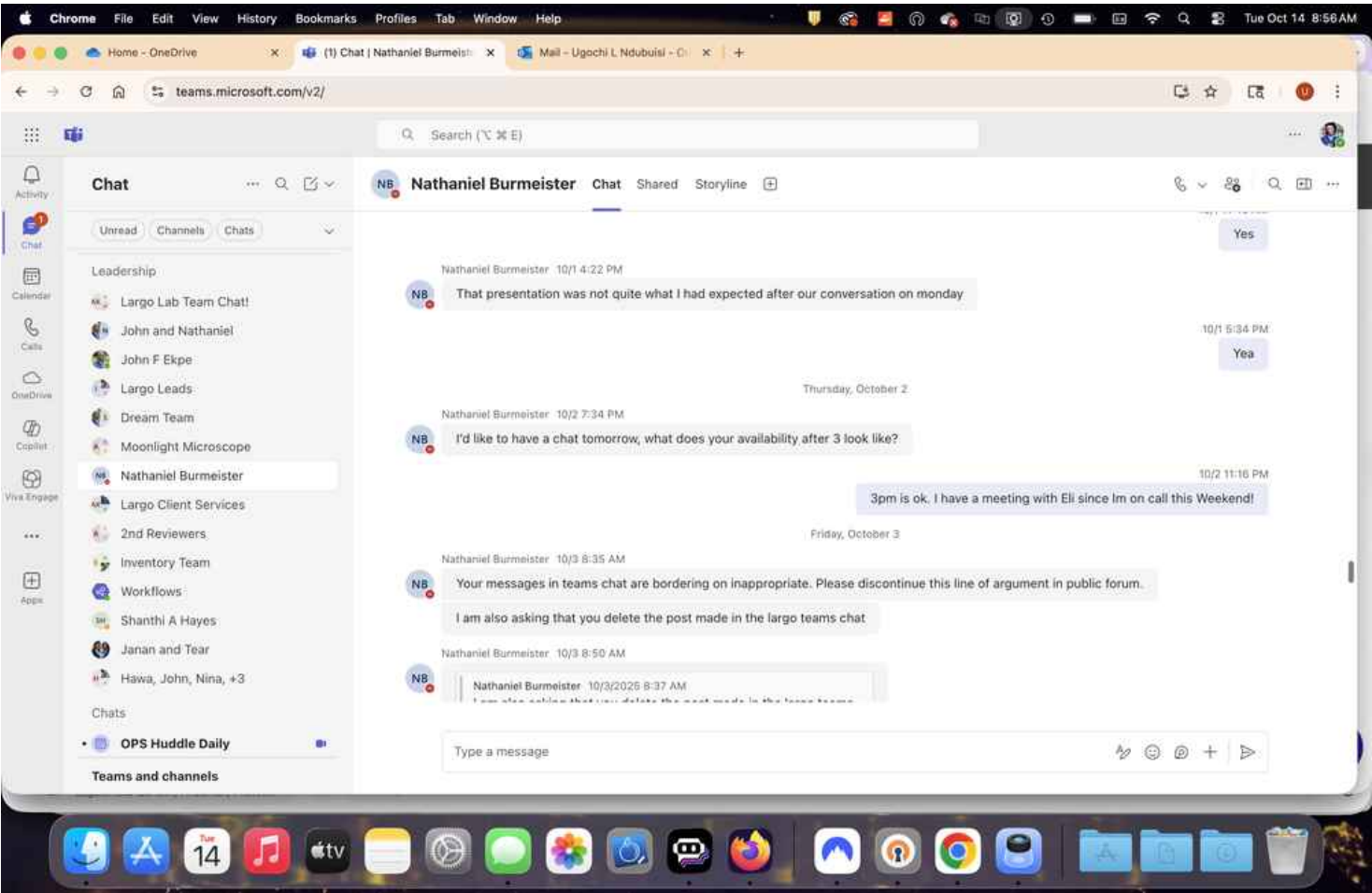
Nathaniel Burmeister Chat Shared Storyline

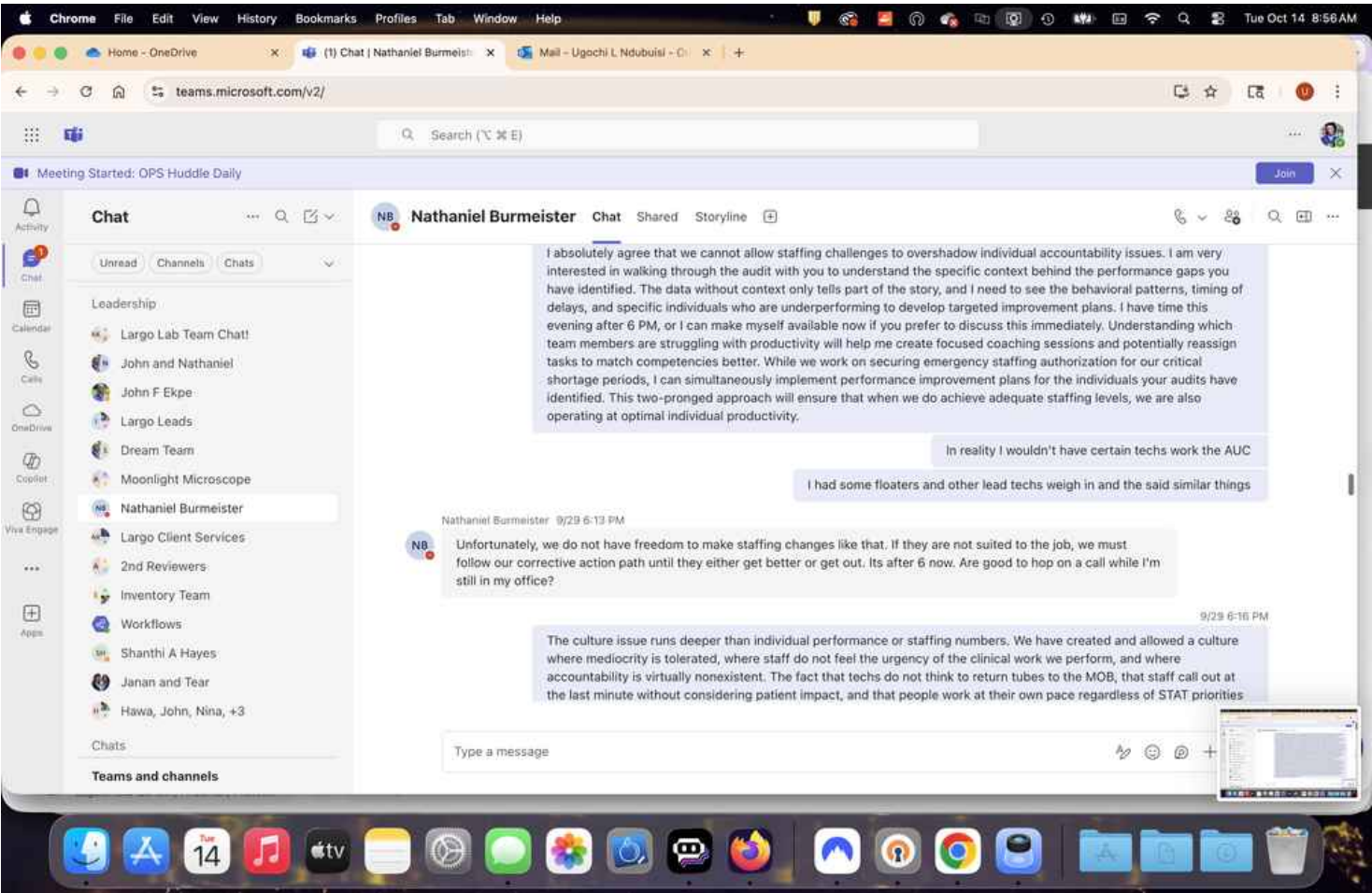
... reduces to identify and address scheduling conflicts before they impact operations. Looking at tomorrow's schedule, we face severe phlebotomy staffing shortages that will significantly impact our operations. The PHLEB Net Result row reveals we will be running at negative four to negative four and a half phlebotomists from 10am to 12pm. Our worst gap occurs from 2pm to 3pm when we will be short five phlebotomists. The shortages continue from 4pm to 7pm with negative one to negative two phlebotomists, and we face additional negative half staffing shortages in late evening from 8pm to 11pm. The current coverage structure shows fundamental misalignment with our demand patterns. While we have phlebotomists like Bolden-Davis Christina scheduled from 6am to 2:30pm and Brooks Johnette from 7am to 3:30pm, along with several others, their shifts do not align with our peak demand periods. We lose significant coverage precisely when afternoon volume peaks. In contrast, our MLT section shows a positive half to one FTE coverage with Blackwell Lorraine and Bahre Robel scheduled, and our MT section maintains adequate to slightly over coverage with positive half to one FTE throughout most shifts. The specific concerns for tomorrow center on the transition period between shifts. The 2pm to 3pm period with our negative five phlebotomist shortage coincides exactly with when our morning shift phlebotomists Bolden-Davis and Brooks are scheduled to leave. We have no adequate overlap between shifts to handle the afternoon surge in volume. Our evening shift staff, including Hayes Danalisa who works 2pm to 10:30pm and Moise Farah who works 6am to 4:30pm, cannot cover this gap alone. We urgently need to either extend our morning phlebotomists past 2:30pm or bring in additional coverage for the 10am to 4pm timeframe to prevent service failure. The negative five shortage at 2pm represents approximately fifty percent understaffing during one of our most critical periods. This visual confirmation of tomorrow's schedule clearly demonstrates the severe staffing crisis we have been discussing and explains precisely why our TAT performance has been falling, particularly during these exact timeframes. The improvement you observed on Thursday and Friday resulted from our overtime arrangements finally taking effect. John and I were proactive and trying to fill in the shortage gaps. However, we are still operating in crisis mode without sustainable coverage. The float pool remains completely depleted with Client Services confirming multiple times that they have no one available to send. I need your support for emergency staffing authorization to properly address these systemic issues before we face complete service failure. Please let me know if you need additional details on any specific aspect of our staffing challenges or improvement plans.

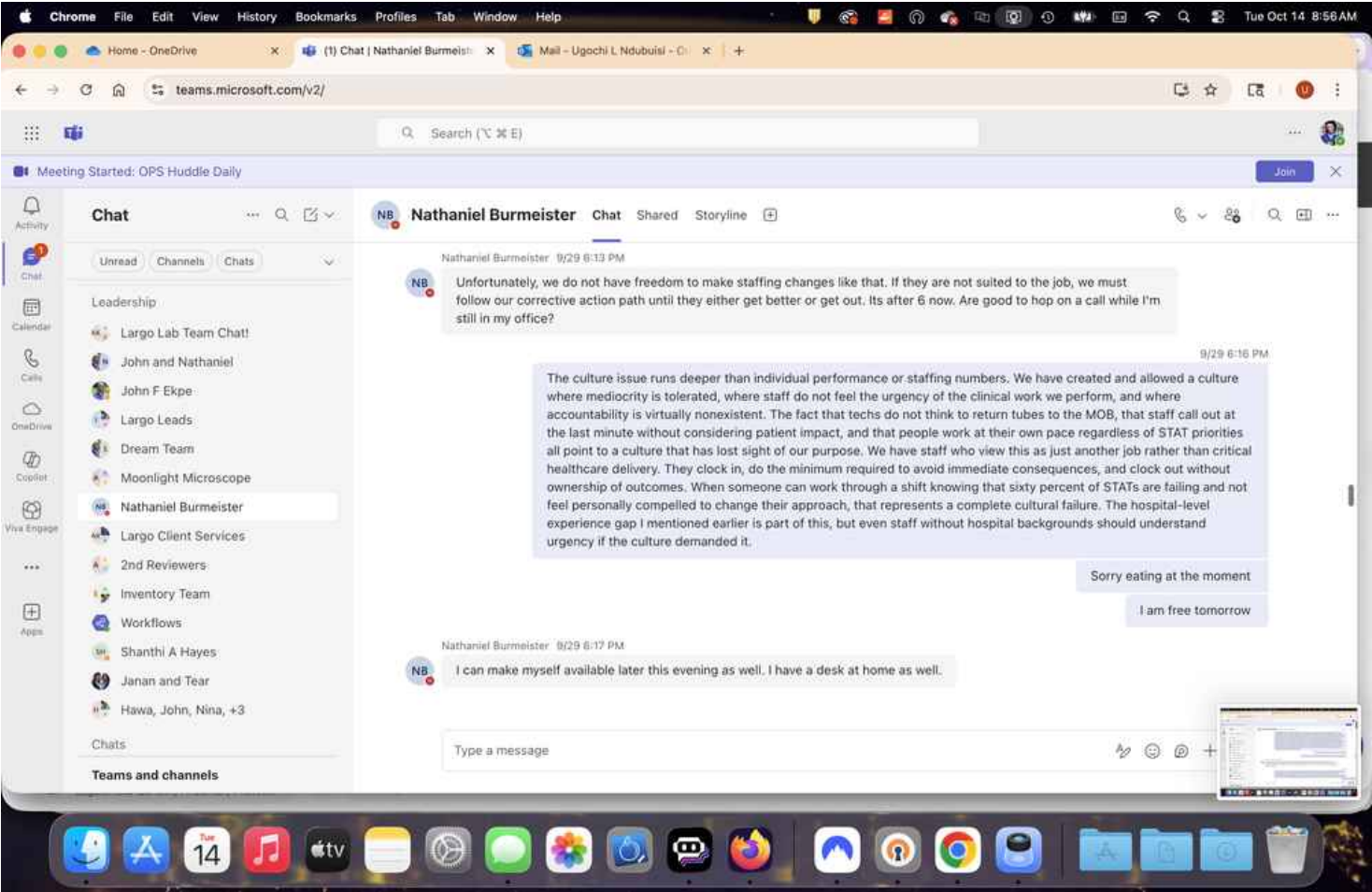
Type a message

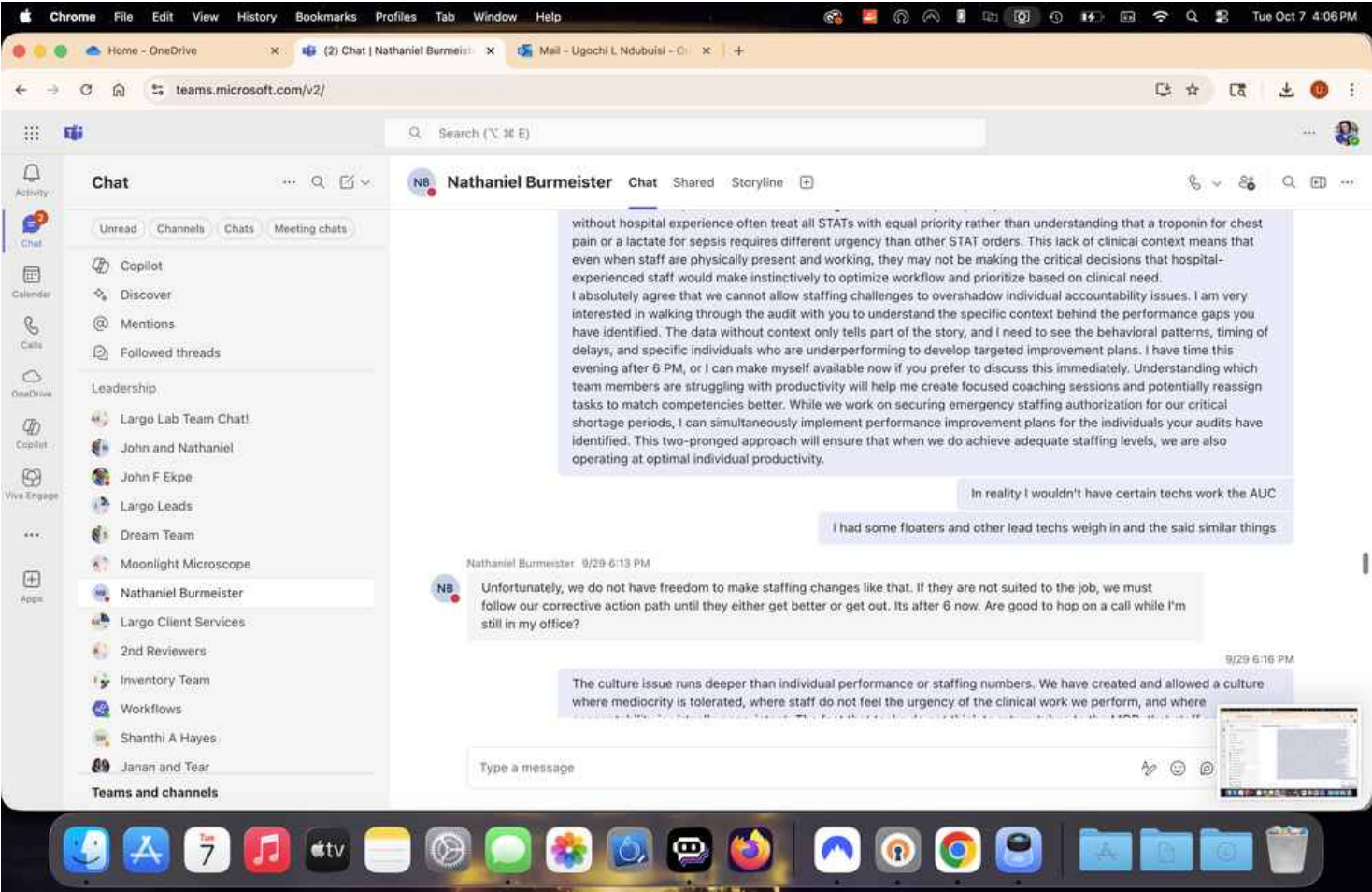


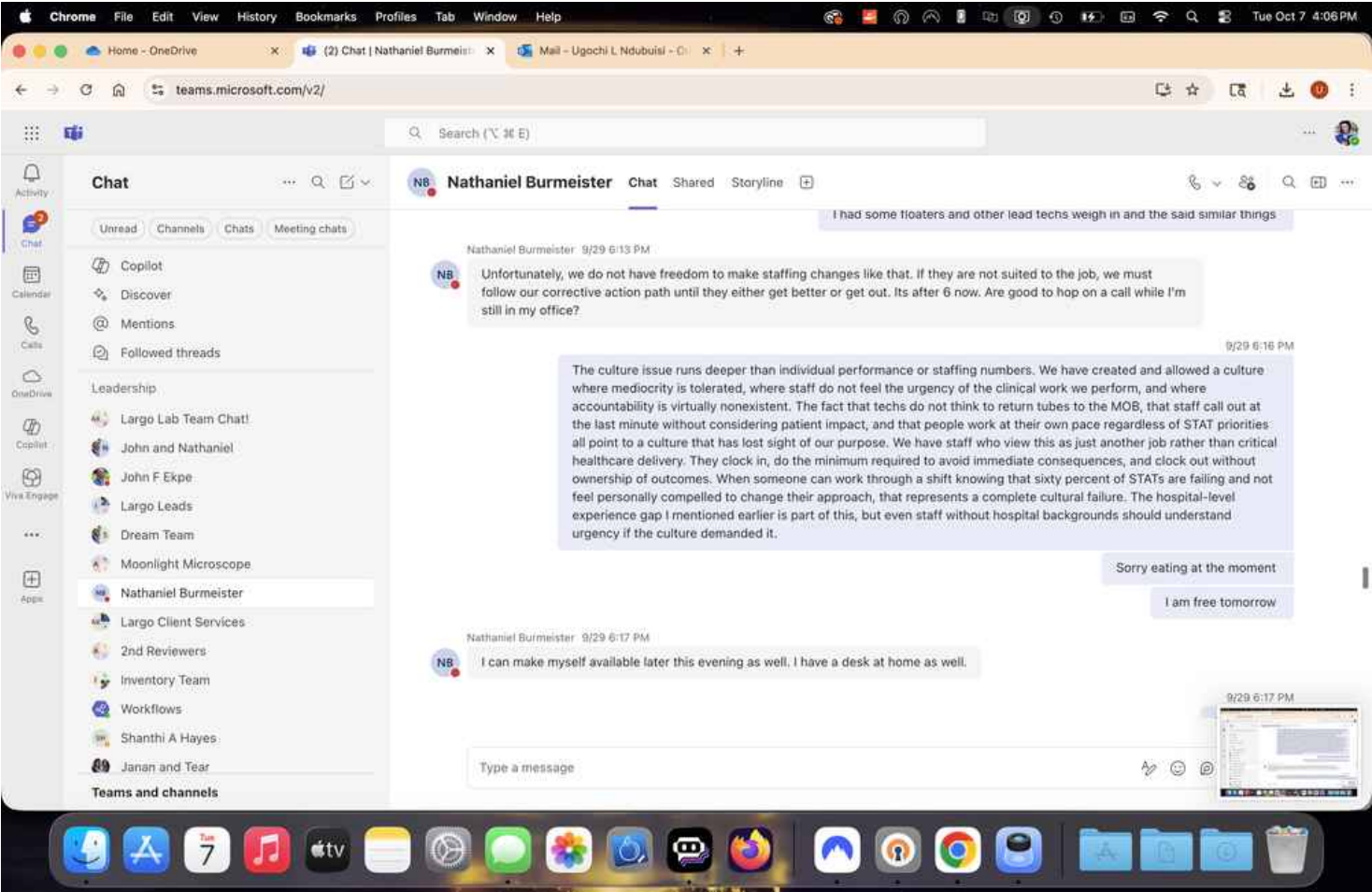












POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
ACCOUNTABLE DEPARTMENT National Human Resources	EFFECTIVE DATE 12/19/2022
DOCUMENT OWNER Chair, National HR Policy Roundtable	PAGE 1 of 3

1.0 Policy Statement

Employees are required to abide by applicable federal, state, and local laws or regulations, to comply with Kaiser Permanente (KP) policies and procedures, and to conduct themselves in accordance with KP's Principles of Responsibility. If an employee fails to comply with any of these or engages in misconduct (including inappropriate/disruptive behavior) or poor performance, the employee may be subject to corrective/disciplinary action, up to and including termination. In addition, the violation of certain laws or regulations may require KP to report the violation to a regulatory or licensing agency and/or may subject an employee to criminal prosecution as well as civil penalties involving substantial monetary fines.

Although represented employees are covered by this policy, the levels of corrective action for represented employees will be determined according to the provisions of the applicable collective bargaining agreement.

Regional procedures must be consistent with this National policy but may include more detailed information regarding corrective/disciplinary action, and employees are subject to those procedures as well.

2.0 Purpose

Kaiser Permanente (KP) must comply with specific legal/regulatory standards that include, but are not limited to, those indicated by Medicare and other government program billing requirements, Guidance issued by the Centers for Medicare and Medicaid Services, the Department of Health and Human Services, the Compliance Program Guidance issued by the Office of the Inspector General, and HIPAA/Privacy and Security regulations. (See Section 6.0, References) KP is required to have well publicized disciplinary guidelines that demonstrate its commitment to compliance and set forth the consequences for violations of compliance mandates.

3.0 Scope/Coverage

This policy applies to all individuals who are employed by the following entities (collectively referred to as "Kaiser Permanente"):

- 3.1** Kaiser Foundation Health Plan, Inc. (KFHP);
- 3.2** Kaiser Foundation Hospitals (KFH);
- 3.3** KFHP/H subsidiaries;
- 3.4** The Permanente Medical Group, Inc. (TPMG) [NOTE: This policy does not apply to physicians, podiatrists, vice presidents, or members of the TPMG Executive Staff, who are covered by separate TPMG policies]; and
- 3.5** Southern California Permanente Medical Group (SCPMG) [NOTE: This policy does not apply to physicians of SCPMG].

4.0 Definitions: n/a

POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
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5.0 Provisions

- 5.1** Except as limited by collective bargaining agreements and/or statute, employment with KP is “at will” (see *At-Will Employment*, NATL.HR.37).
- 5.2** In the event that it is necessary to take corrective/disciplinary action to address concerns related to an employee’s work performance or conduct, the corrective/disciplinary action may include, but is not limited to:
 - 5.2.1** verbal discussion, coaching, and/or warning by the employee’s immediate supervisor or higher-level manager to correct the problem
 - 5.2.2** written notice, with or without final warning
 - 5.2.3** suspension, with or without final warning
 - 5.2.4** termination of employment
- 5.3** The corrective/disciplinary action steps listed in section 5.2 are not exhaustive and serve only as a description of possible action. None of these actions are mandatory or a prerequisite for any other actions that may be appropriate based on the circumstances. Any or all of these actions may be employed prior to termination and certain employee conduct may be deemed sufficiently serious to warrant immediate termination or escalated disciplinary action without prior corrective/disciplinary action.
- 5.4** The level of corrective/disciplinary action will be determined by management after review of the facts (including legal, compliance or policy violations), applicable regional/national collective bargaining agreement provisions, and consideration of other relevant factors and circumstances including, but not necessarily limited to, the factors in section 5.5 below. Management should consult Human Resources as appropriate.
- 5.5** Examples of factors to be considered in determining the level of corrective/disciplinary action include, but are not limited to, the following:
 - 5.5.1** the severity of the violation
 - 5.5.2** the number of violations
 - 5.5.3** whether the violation was part of a pattern or practice of improper behavior or non-compliance
 - 5.5.4** the individual’s past performance and conduct
 - 5.5.5** whether the individual knew or should have known the applicable policies, rules or regulations
 - 5.5.6** length of employment at KP
 - 5.5.7** whether the violation was intentional or negligent
 - 5.5.8** whether the action appeared to be committed for personal gain
 - 5.5.9** regional/national collective bargaining agreements corrective/disciplinary action provisions for represented employees



Outlook

RE: Lab delay

From Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>**Date** Fri 6/6/2025 3:41 PM**To** Alice J Gouvernayre <Alice.J.Gouvernayre@kp.org>; Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>; Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>**Cc** Sunny L Lambert <Sunny.L.Lambert@kp.org>; Carole R Kelly <Carole.R.Kelly@kp.org>; Nona Alexander <Nona.Alexander@kp.org>; Erin M Smith <Erin.M.Smith@kp.org>; Zachary Smith <Zachary.X3.Smith@kp.org>; James A Powell <James.A.Powell@kp.org>; Daniel J Patenaude <Daniel.J.Patenaude@kp.org>

Hello Dr. Gouvernayre,

Ugochi and I did a thorough investigation to discover root causes for the failures that were experienced on Monday. We talked about best strategy to mitigate that root cause, and I am confident that it is being addressed. As it is something that will include employee disciplinary actions, I will not go into details in this forum.

As far as regular state, we do normally perform maintenance on our instruments during our lower volume periods. Our major instrumentation all has redundancy. So even during regular maintenance, we have availability to test 24/7 (barring unscheduled downtime). For the Chemistry instruments, we perform one instrument between 9p-12a and the other is performed between 4a-8a. The process usually takes about 1-2 hours, and we have the flexibility to delay up to a few hours if testing demands are high. Ugochi and John are monitoring and reviewing for possible optimizations on this and other areas of the lab.

Ugochi and John will continue to reinforce the use of electronic communication through KP Healthconnect chat as best practice. I'll also continue to connect them with coaching and resources to best serve the operations needs of the Lab and by extension the AUC.

-Nate Burmeister

C:202-570-9030

From: Alice J Gouvernayre <Alice.J.Gouvernayre@kp.org>**Sent:** Friday, June 6, 2025 3:10 PM**To:** Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>; Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>**Cc:** Sunny L Lambert <Sunny.L.Lambert@kp.org>; Carole R Kelly <Carole.R.Kelly@kp.org>; Nona Alexander <Nona.Alexander@kp.org>; Erin M Smith <Erin.M.Smith@kp.org>; Zachary Smith <Zachary.X3.Smith@kp.org>; Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>; James A Powell <James.A.Powell@kp.org>; Daniel J Patenaude <Daniel.J.Patenaude@kp.org>**Subject:** RE: Lab delay

Hi Ugochi,

Thank you for taking the time to explain the situation so thoroughly to us. Can you tell us when the QC for Pure 2 is scheduled to occur?

I have a few questions/suggestions based on the information you provided.

1. Communication:

- a. I understand that you came over that night to inform the team of the issues you were having. It's really nice of you to do that, but not everyone gets the message when you come in person. I think the best way to make sure we all understand what's going on, is for you or another manager to send a detailed message about the issue and its impact to the lead nurse on epic chat. The lead nurse will then copy that message and send it to all that are working in the AUC that day. This workflow is good for temporary delays. If an instrument is down or for any other major issue, please continue to notify the DCSM critical operation alert on epic chat.

Reason: If physicians and nurses understand the lab limitations, we are then able to adjust the plan of care for patients appropriately (i.e. send patients home and message them on kp.org when the results are available versus the traditional path of having the patients wait in the department for results). Or we can set expectations with patients right off the bat on how long it will take to figure out what's wrong with them. The sooner we know, the better.

- b. Regarding the high volume of calls – I understand how disturbing this might have been. I think if we follow the communication pathway outlined above, you will get less calls. I also do want to point out that we wouldn't be doing our job if we left a patient waiting for a strep test result for hours without inquiring about the result repeatedly on their behalf.

1. Mitigation of impacts:

- a. I understand the chemistry machine was down and the urinalysis instrument was also not working but why did we have a 10-hour strep test delay? Or why were CBCs affected?
- b. What is the escalation pathway when you are short-staffed? What is the escalation pathway when the machines are malfunctioning? What is put in place to preserve operations across all testing instruments?

We look forward to working on solutions with you to improve operations and that we can present at the throughput meeting.

Best,

Alice J Gouvernayre, MD

Chief, Acute Care Services

DC and Suburban Maryland Service Area

Permanente Medicine

Mid-Atlantic Permanente Medical Group

Capitol Hill Medical Center

700 2nd St. NE,

Washington, DC 20002

202-853-1311 (mobile)

alice.j.gouvernayre@kp.org

For assistance:

Rodnee Petty at 202-680-3404 or rodnee.s.petty@kp.org

[Facebook](#) | [Twitter](#) | [LinkedIn](#) | [Instagram](#) | [YouTube](#)

Kaiser Permanente

From: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Sent: Tuesday, June 3, 2025 10:45 PM

To: Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>



RE: Activity and Camera Footage Report Request

From Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>

Date Mon 7/14/2025 7:11 PM

To Janis A Thom-Grate <Janis.A.Thom-Grate@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>

Cc Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Thank you Janis,

I'll walk them through this portal at a later date, I'll be submitting on their behalf tonight.

-Nate Burmeister

C:202-570-9030

From: Janis A Thom-Grate <Janis.A.Thom-Grate@kp.org>

Sent: Monday, July 14, 2025 7:10 PM

To: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>

Cc: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Subject: RE: Activity and Camera Footage Report Request

Here is the link to submit the security request –

[Online Security Incident Report](#)

-Janis

Janis A. Thom-Grate, J.D., SPHR, SHRM-SCP

Pronouns: She, Her, Hers

HR Consultant IV

Kaiser Permanente Mid-Atlantic States

(c) 240.495.1364

Janis.A.Thom-Grate@kp.org

Upcoming PTO:

HR RESOURCES FOR ALL EMPLOYEES	HR RESOURCES FOR MANAGERS
- HR transactions including pay, benefits, leaves and HR system navigation inquiries: Contact the HR Service Center or Timekeeping or at 1-877-457-4772. Fax: 1-877-477-2329 - Retirement (pension plan, retiree medical and/or retiree life insurance benefits): Visit the KPRC website at www.myretirement.kp.org or call the KPRC at 1-866-627-2826, #0. - Defined contribution plans-related inquiries (Plan B, TSA, and Roth): Visit the Vanguard website at www.vanguard.com or call Vanguard at 1-800-523-1188.	- For information you cannot find in the HRconnect system or at hrconnect.kp.org: <i>Position management:</i> Email pm-support-team@kp.org or <i>Call the National HRSC at 1-877-457-4772, Options 3,4,5.</i> <i>Other transactions for individuals on your team (such as timekeeping, new hires, terminations, and general HR systems questions): Call the National HRSC at 1-877-457-4772, Options 3,4.</i>

From: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Sent: Monday, July 14, 2025 7:04 PM
To: John F Ekpe <John.F.Ekpe@kp.org>; Janis A Thom-Grate <Janis.A.Thom-Grate@kp.org>
Cc: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Subject: RE: Activity and Camera Footage Report Request

Hello John and Ugochi,

Whenever we wish to pull camera footage, we must also include our HRC partner in that request. [@Janis A Thom-Grate](mailto:Janis.A.Thom-Grate@kp.org), We will be requesting for camera footage to support an investigation into possible time fraud. We have reason to believe that employees are leaving their workstation for extended periods of time while clocked in. John and Ugochi have also requested badge swipes for the days below and for a few other days (7/2, 7/5, and 7/6) all between 7pm and 10pm. We will also be pulling Cerner activity data for the days listed.

-Nate Burmeister
C:202-570-9030

From: John F Ekpe <John.F.Ekpe@kp.org>
Sent: Monday, July 7, 2025 7:21 PM
To: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Cc: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Subject: Activity and Camera Footage Report Request

Hi Nate,

Thank you for taking the time to chat with me earlier about our AUC TAT metric, staff performance and related matters. Based on our discussions, I would like to request activity report pull on the following days and the camera report can be pulled between 7pm - 10pm.

Date: July 2nd, 5th and 6th.

Thank you.

John F. Ekpe, MLS (ASCP)cm
Manager, Laboratory Operations
Largo Medical Center
1221 Mercantile Ln, Upper Marlboro, MD 20774
Email: john.f.ekpe@kp.org
Phone: (301)-996-0119

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WHISTLEBLOWER RETALIATION EVIDENCE

Largo Laboratory - Kaiser Permanente

EXECUTIVE SUMMARY

This document presents chronological evidence of retaliation against Ugochi L. Ndubuisi for protected whistleblowing activity. The timeline shows:

1. July 14, 2025: Director orders investigation
2. September 29, 2025: Analysis provided and acknowledged
3. October 3, 2025: Same analysis shared with staff (protected activity)
4. October 3, 2025: Four censorship demands in 8 hours
5. October 8, 2025: Final Written Warning (first discipline in 9 months)

Maryland OSHA Deadline: November 7, 2025

CRITICAL TIMELINE

Date/Time	Event	Significance
July 14, 2025	Investigation Ordered	Director orders investigation into second shift
Sept 29, 2025	Analysis Provided	"Culture issue runs deeper" - acknowledged by director
Oct 3, 8:21am	Whistleblowing	41.23% failure rate shared with staff
Oct 3, 8:35am	Censorship #1	"Inappropriate" - 14 minutes after sharing
Oct 3, 8:50am	Censorship #2	"Delete the post" - 29 minutes after
Oct 3, 12:19pm	Censorship #3	"This is a directive" - escalation
Oct 3, 4:30pm	Censorship #4	Fourth deletion demand
Oct 8, 2025	Final Warning	First discipline ever - clear retaliation

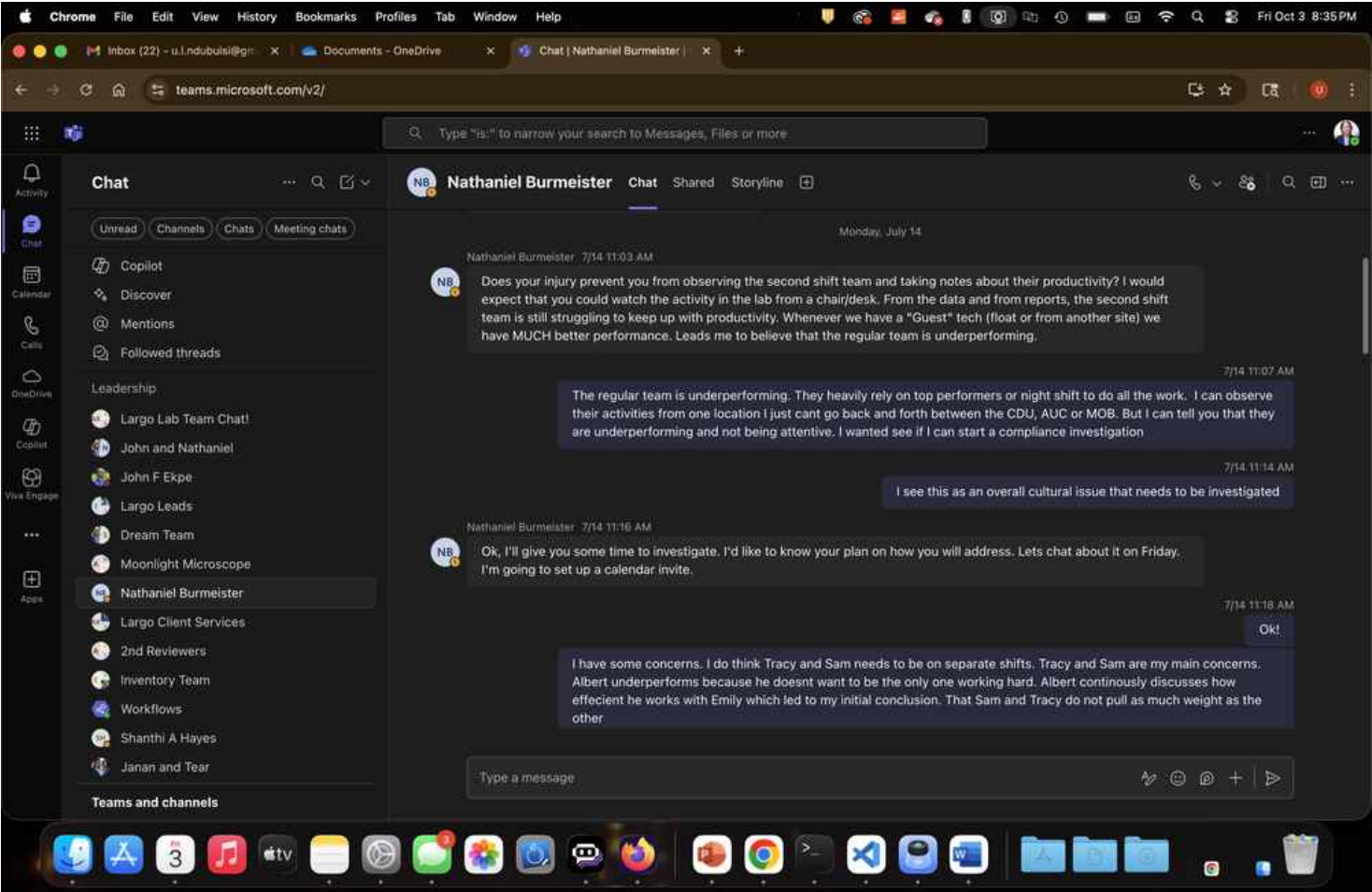
PATTERN OF RETALIATION - KEY EVIDENCE

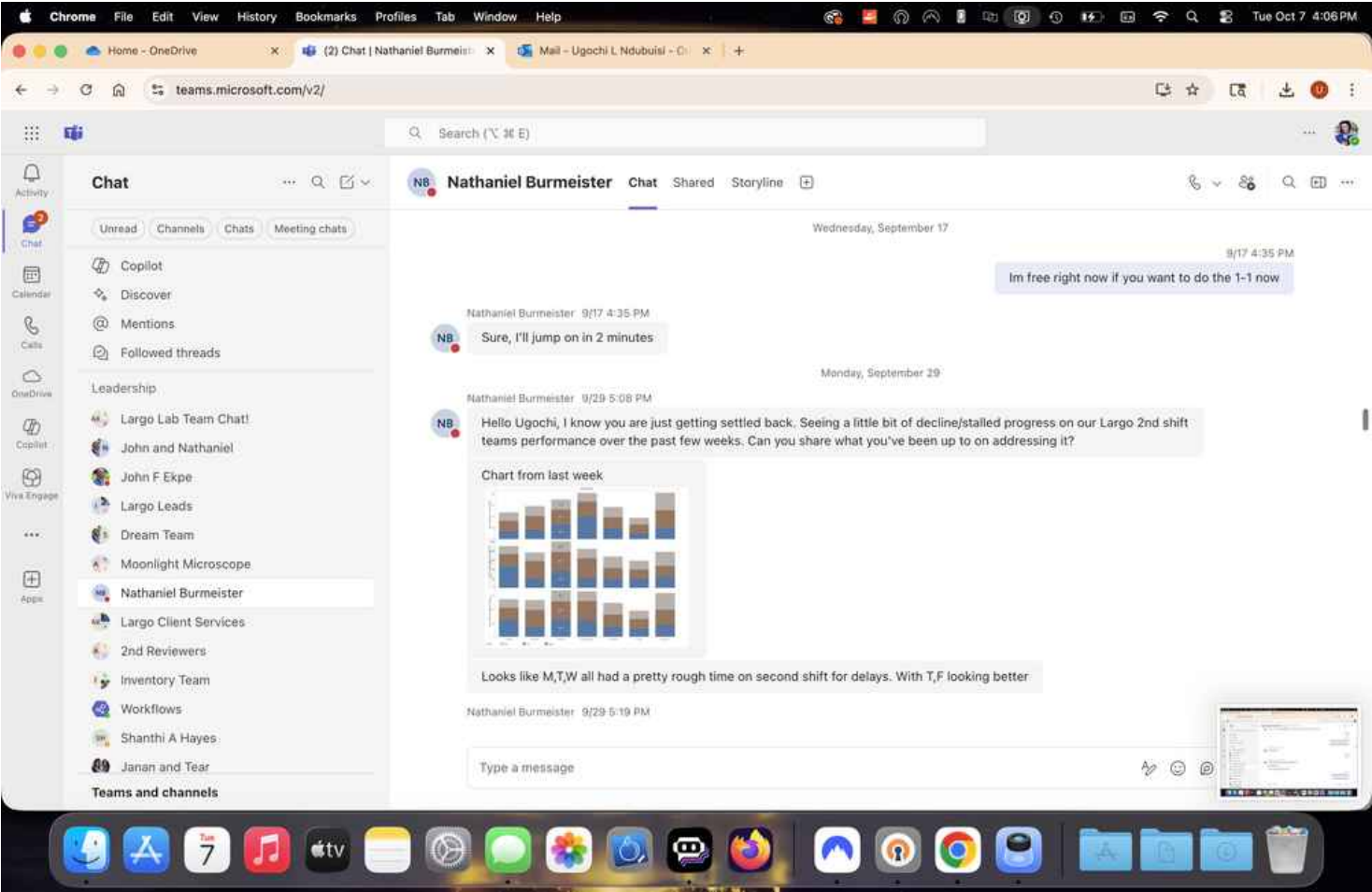
- 1. Protected Activity:** Reported workplace safety concerns affecting patient care (41.23% failure rate)
- 2. Temporal Proximity:** Only 5 days between whistleblowing and Final Written Warning
- 3. Disparate Treatment:** Same analysis praised by director on Sept 29, punished when shared Oct 3
- 4. Censorship Pattern:** Four deletion demands in single day shows intent to silence
- 5. Clean Employment Record:** No prior discipline in 9 months - sudden jump to Final Warning
- 6. Public Interest:** Patient safety at Level 3 Trauma facility serving community

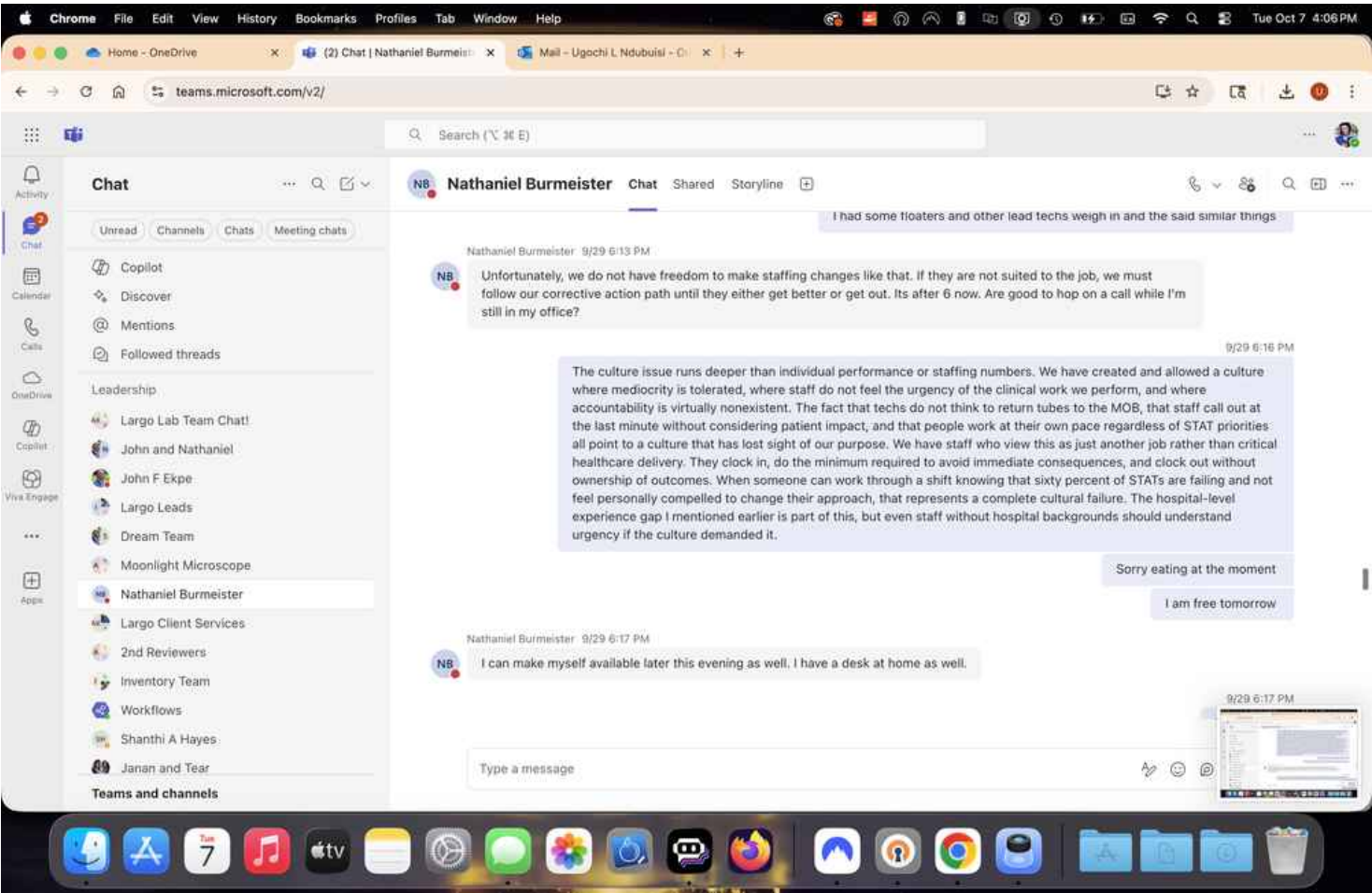
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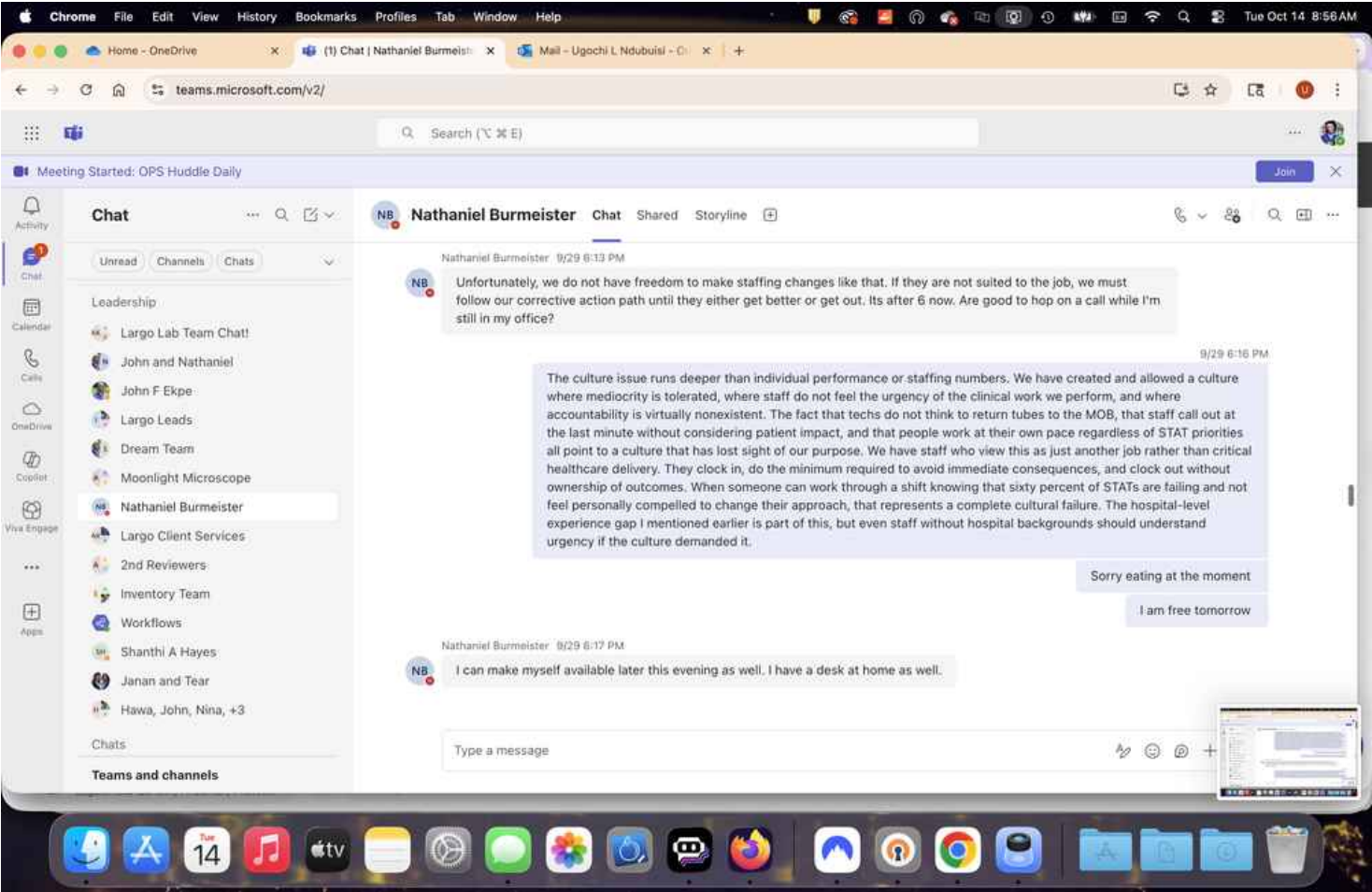
- Maryland OSHA Deadline: November 7, 2025
- Protected under Maryland Occupational Safety and Health Act
- Document Generated: October 14, 2025

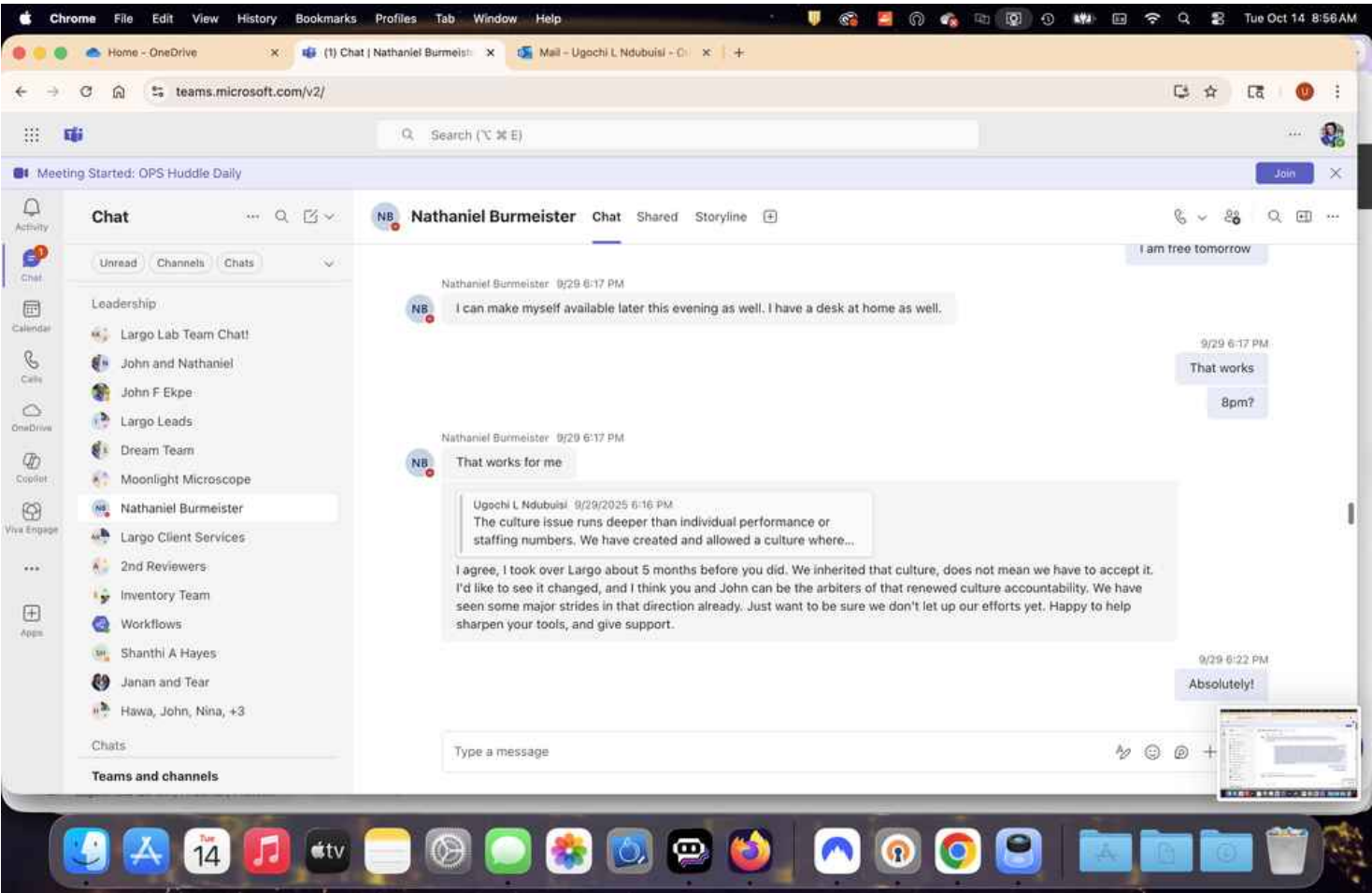
Evidence 1: July_14_Investigation_01_Screenshot 2025-10-03 at 8.35.06PM

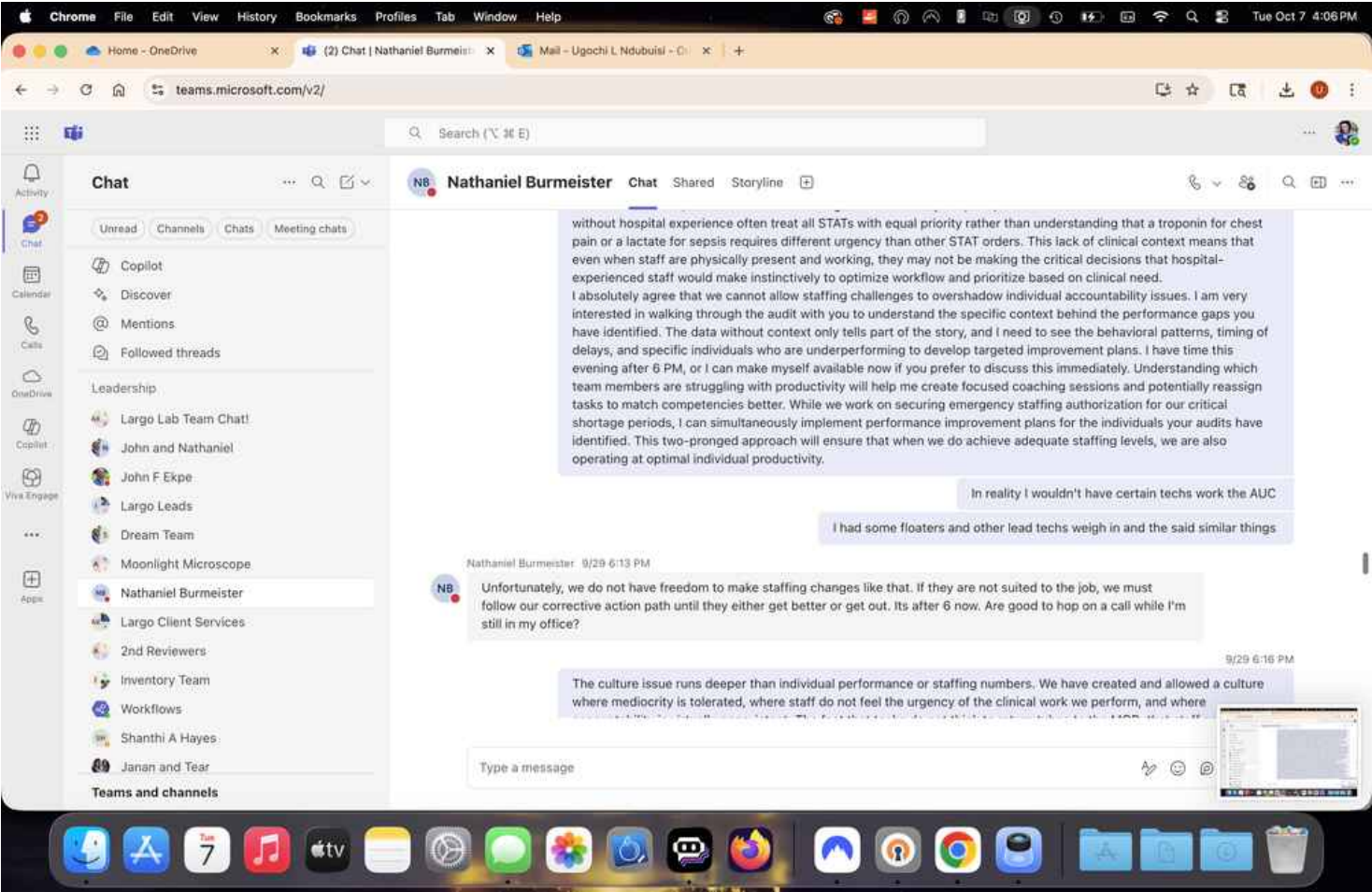


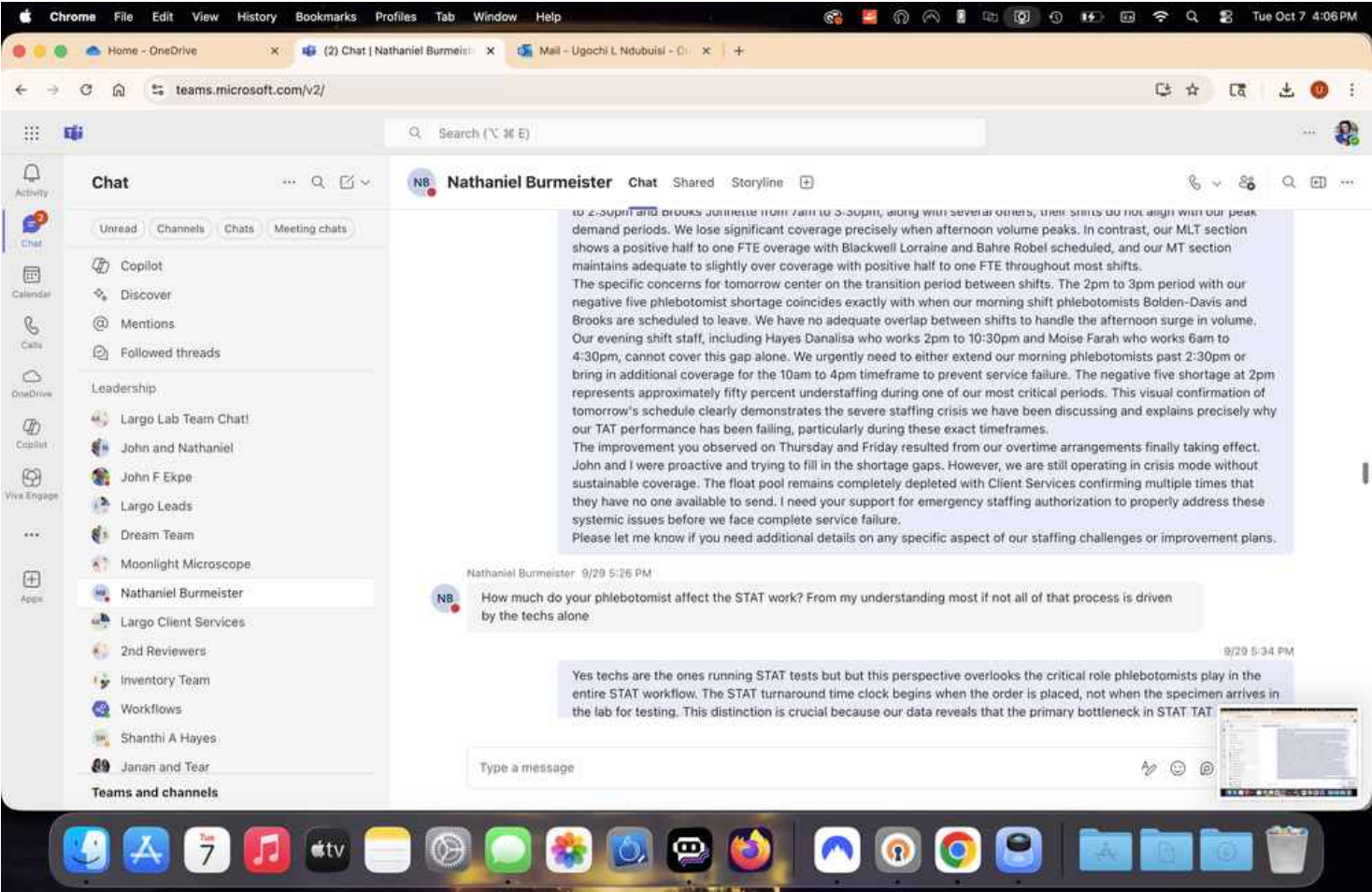


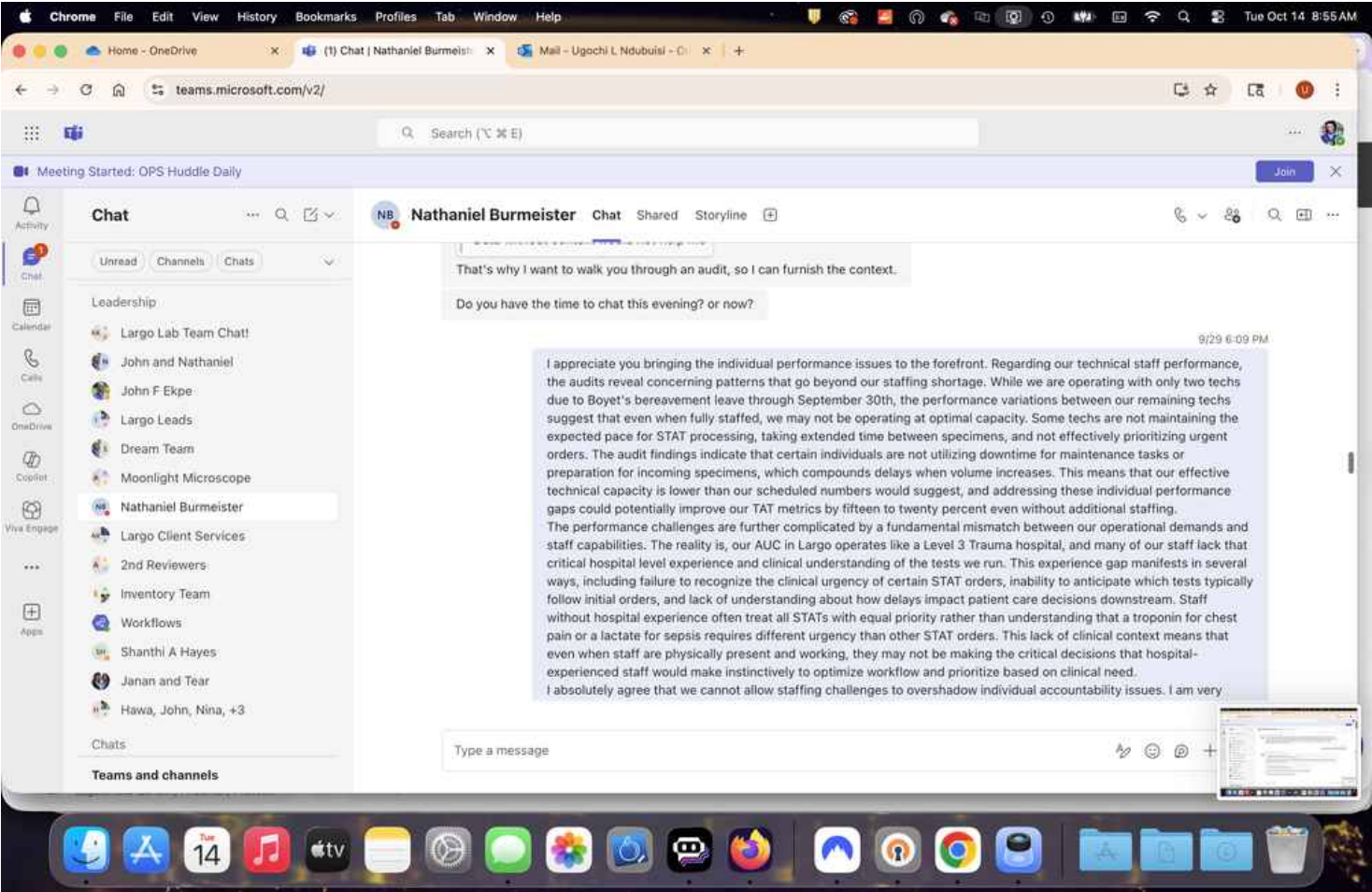


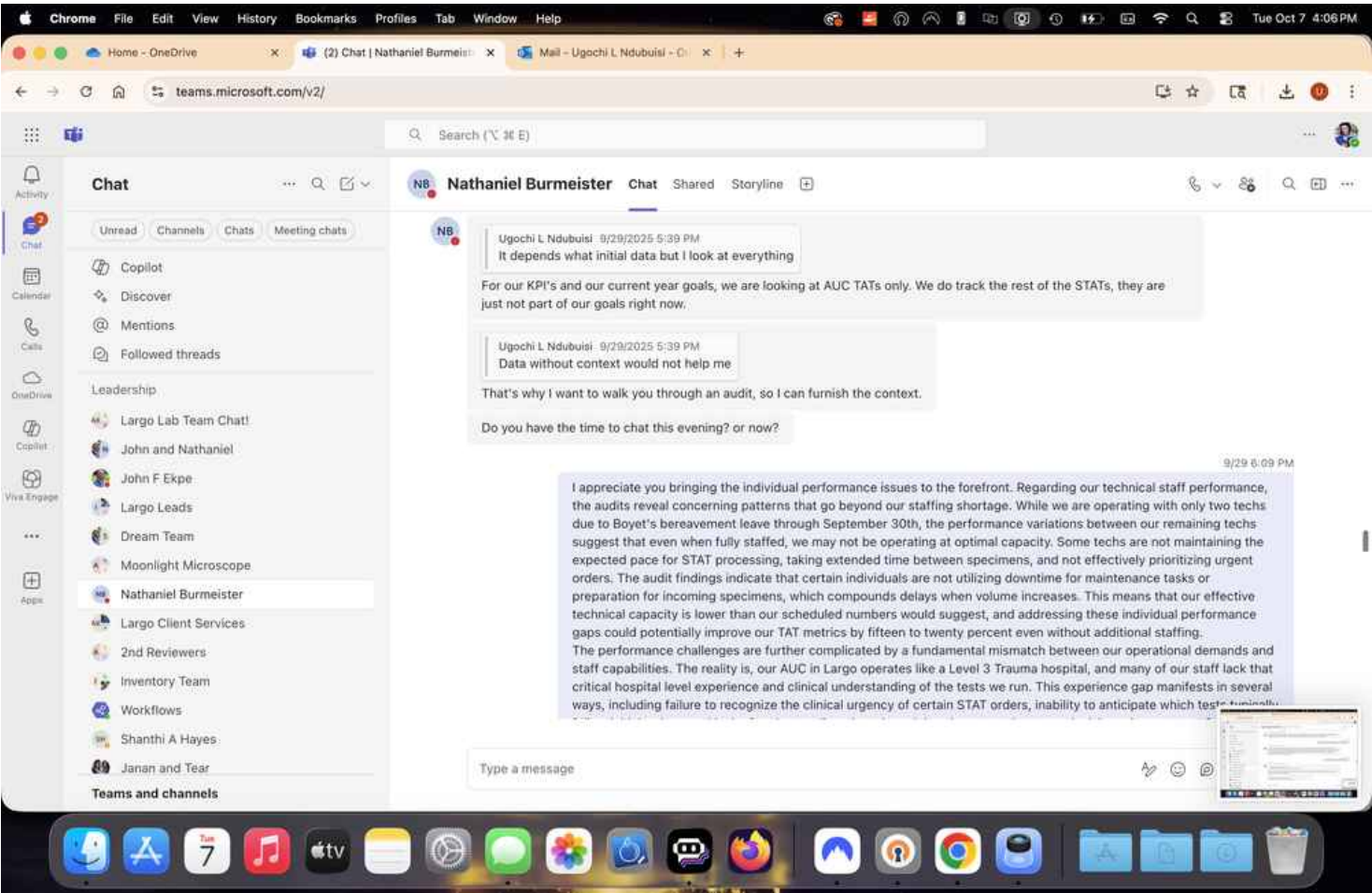


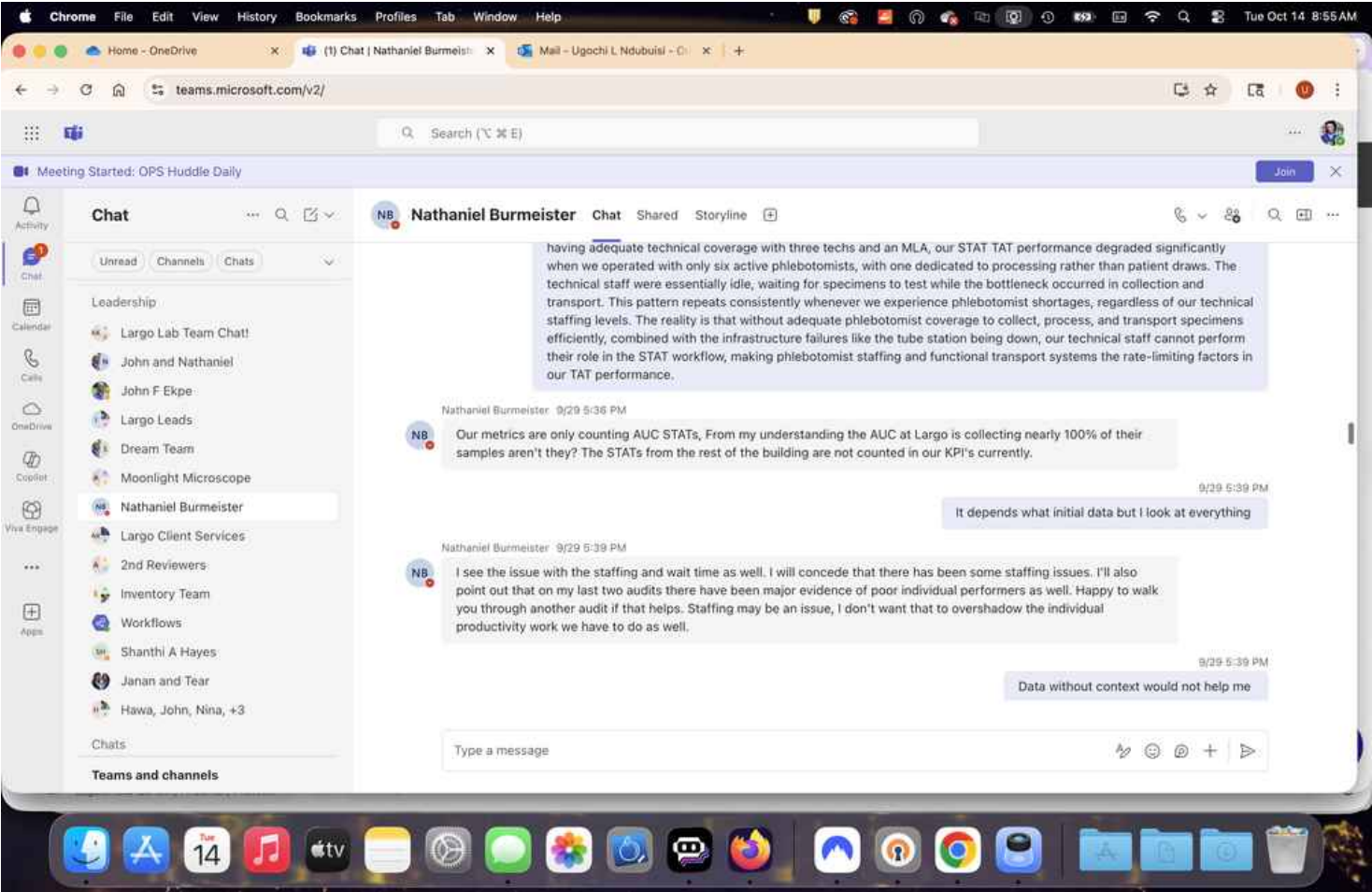


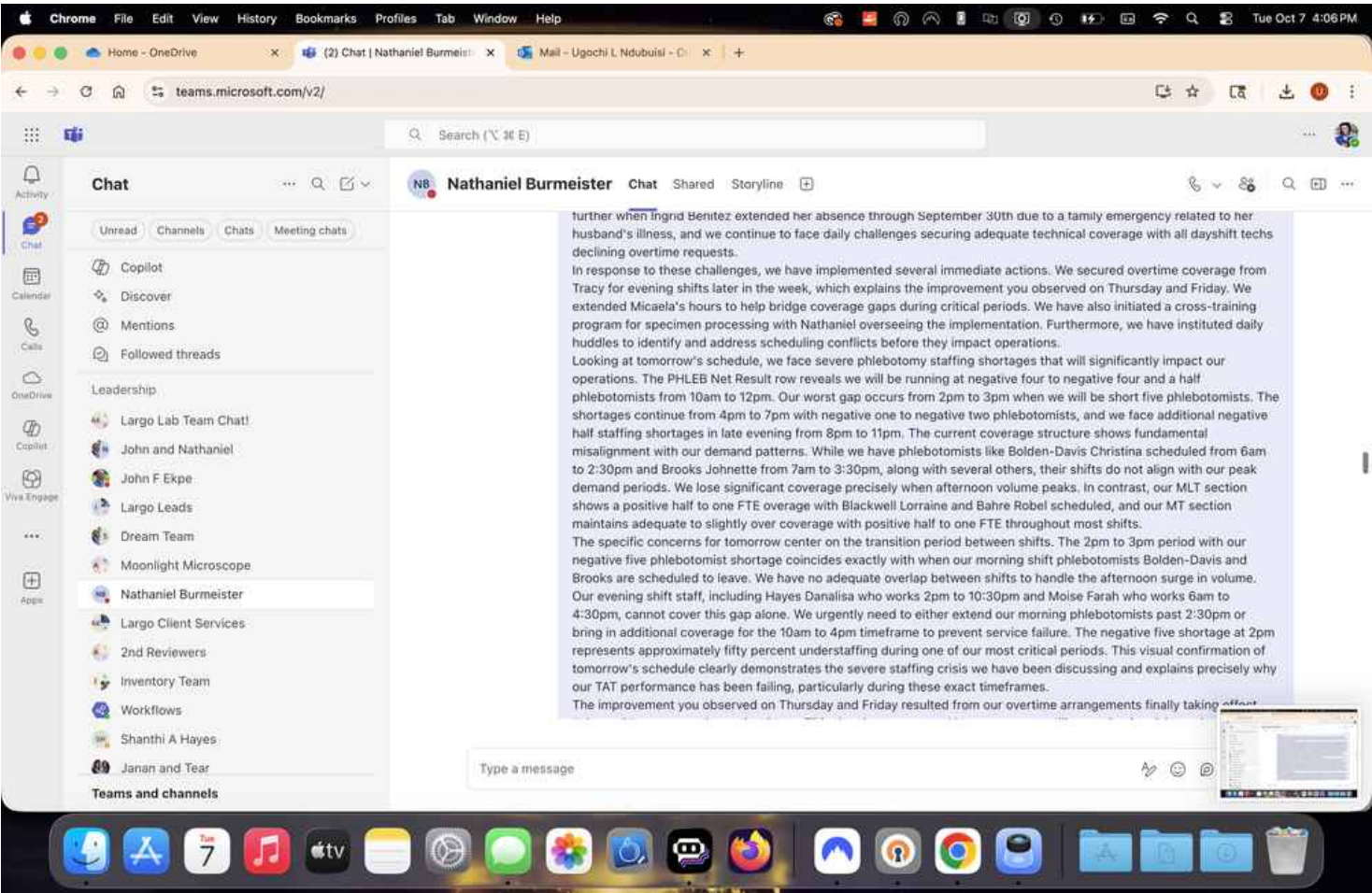


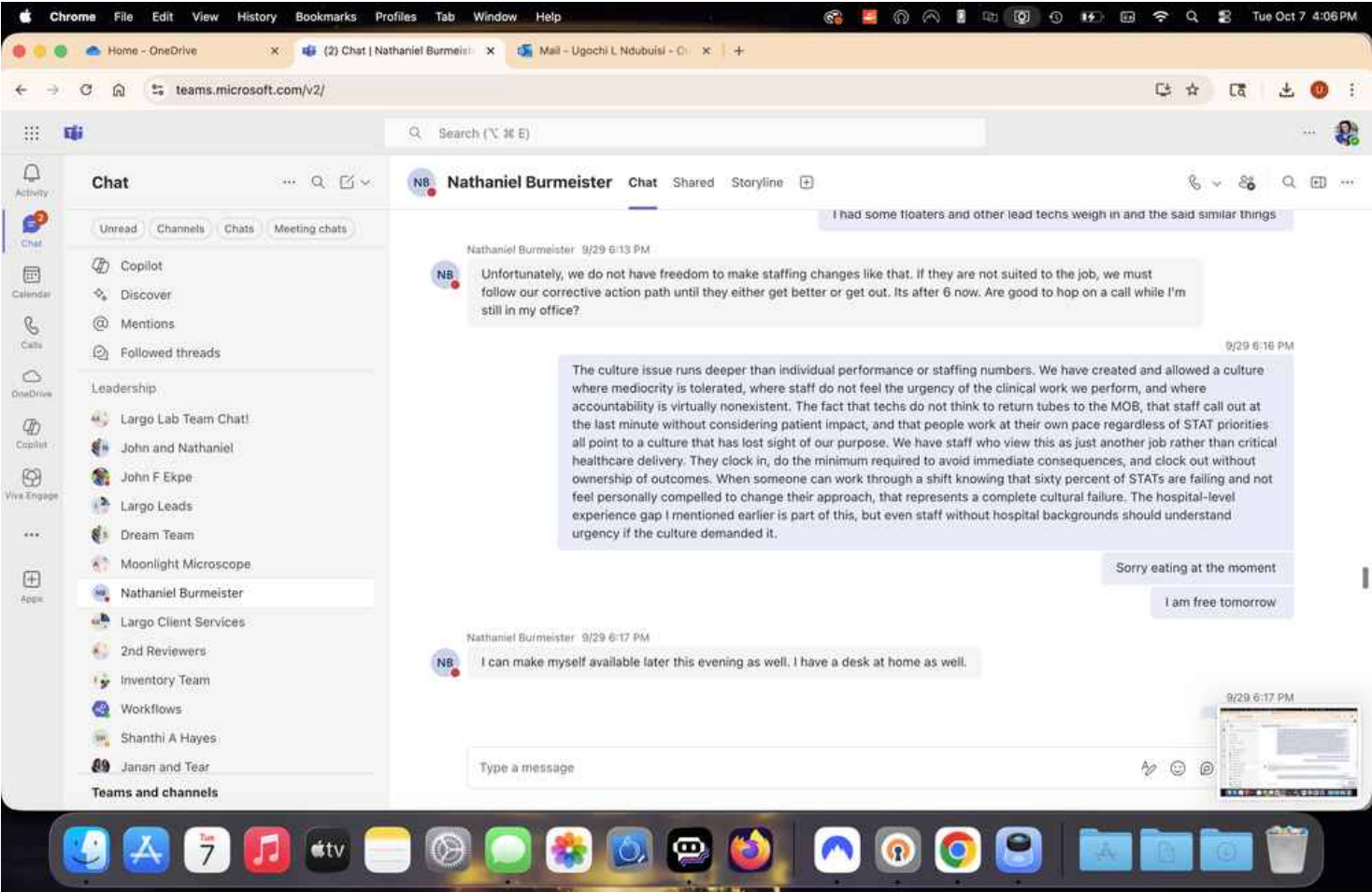




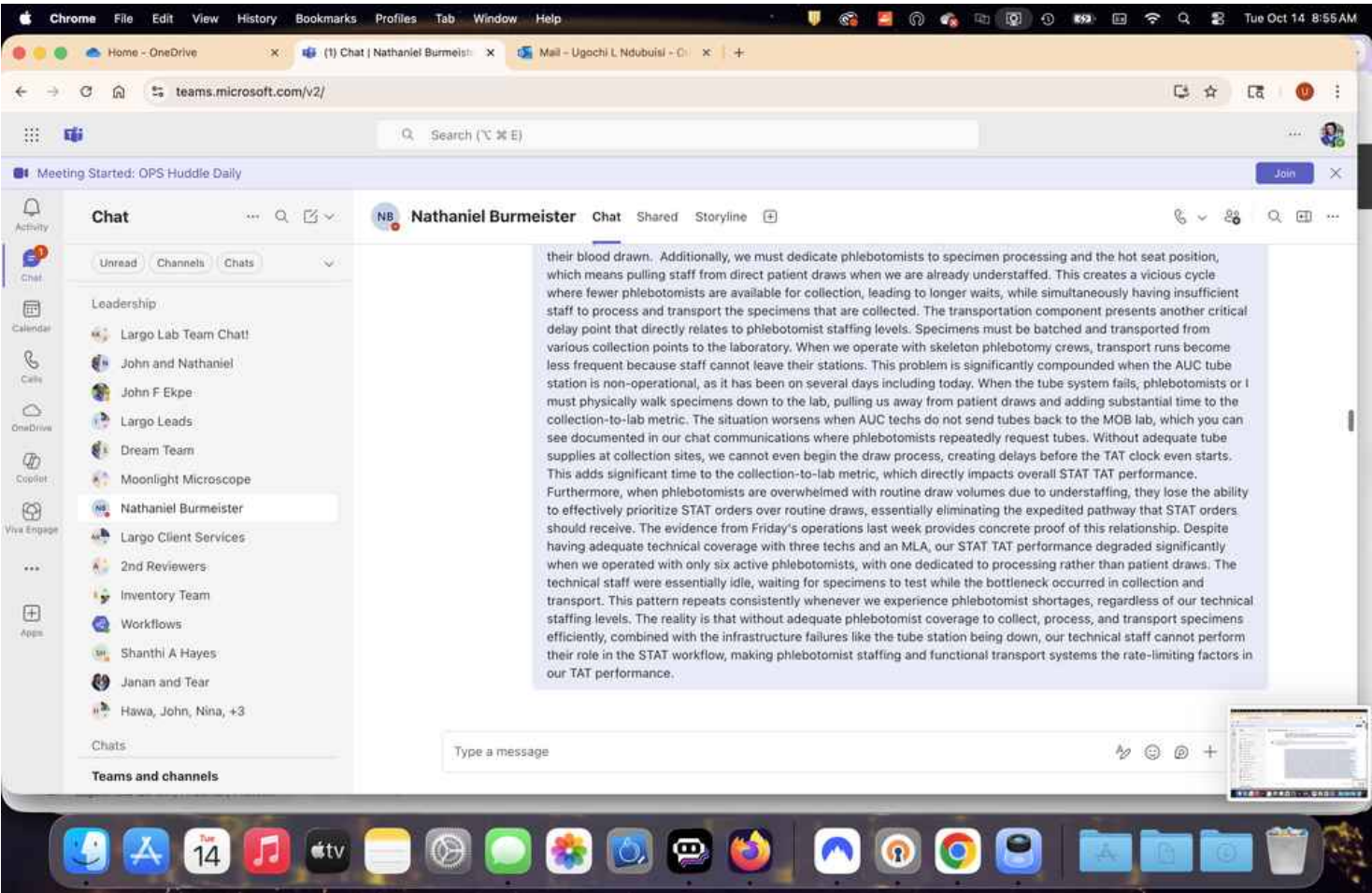




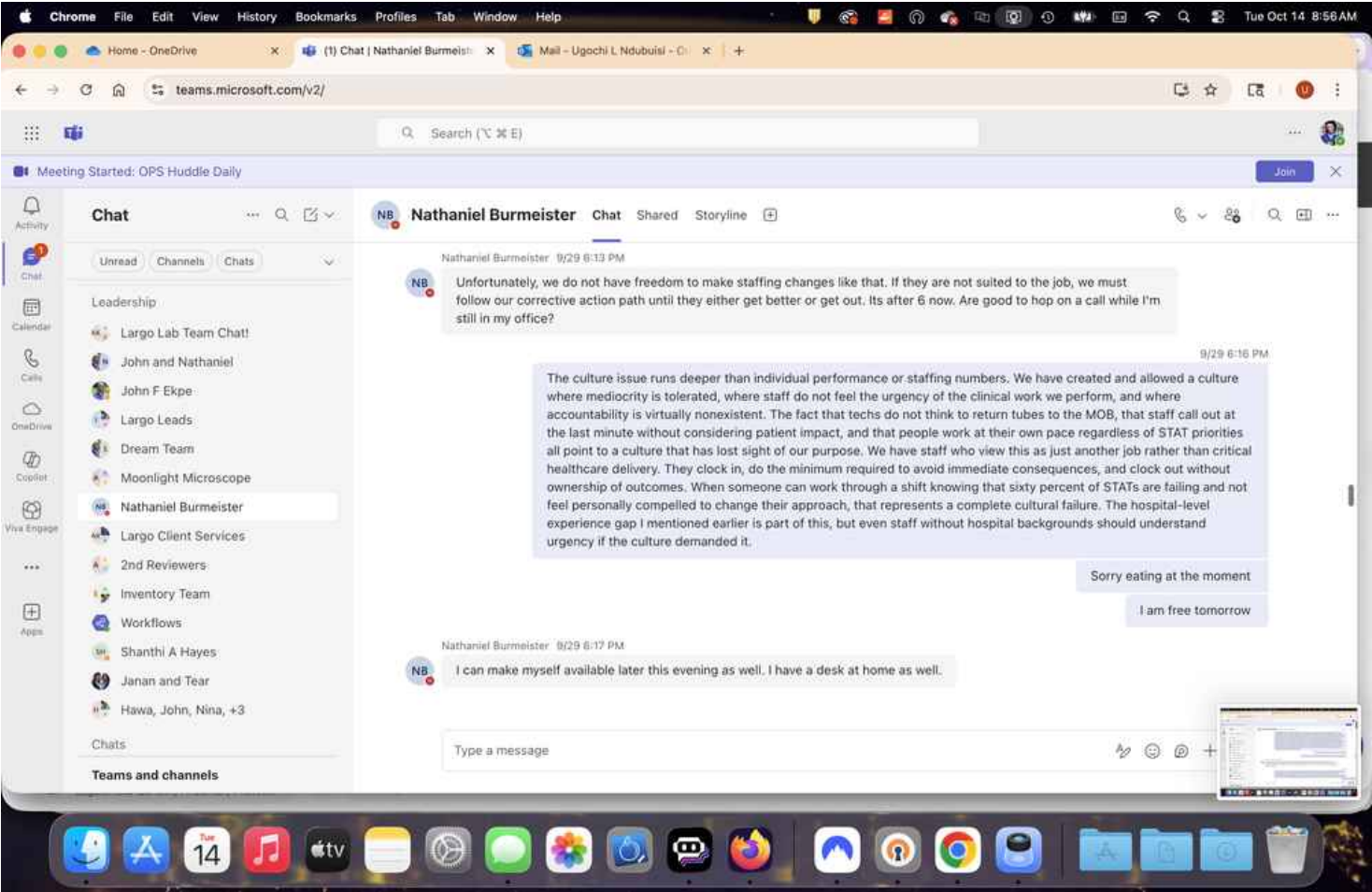


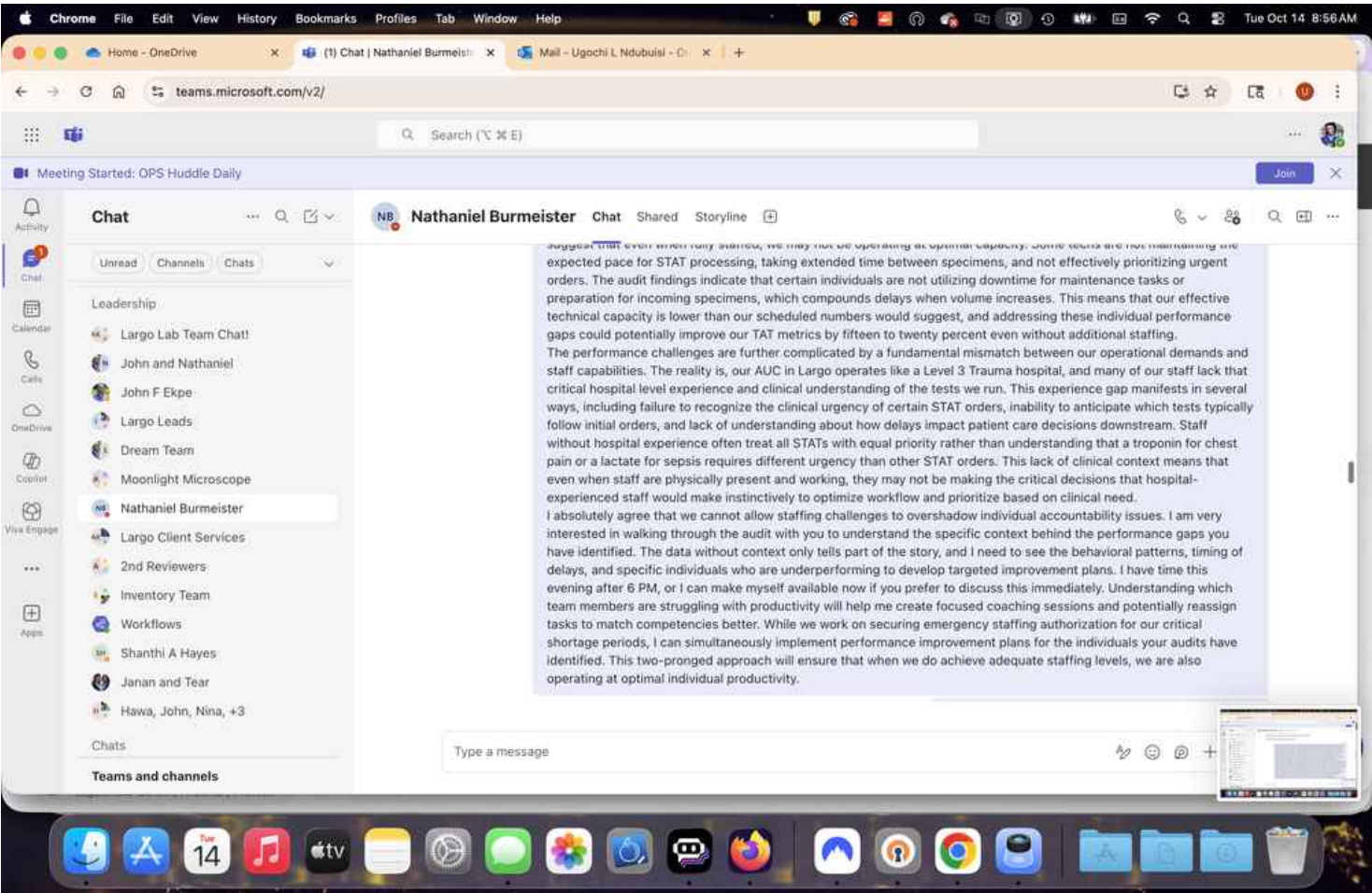


Evidence 13: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.55.37AM



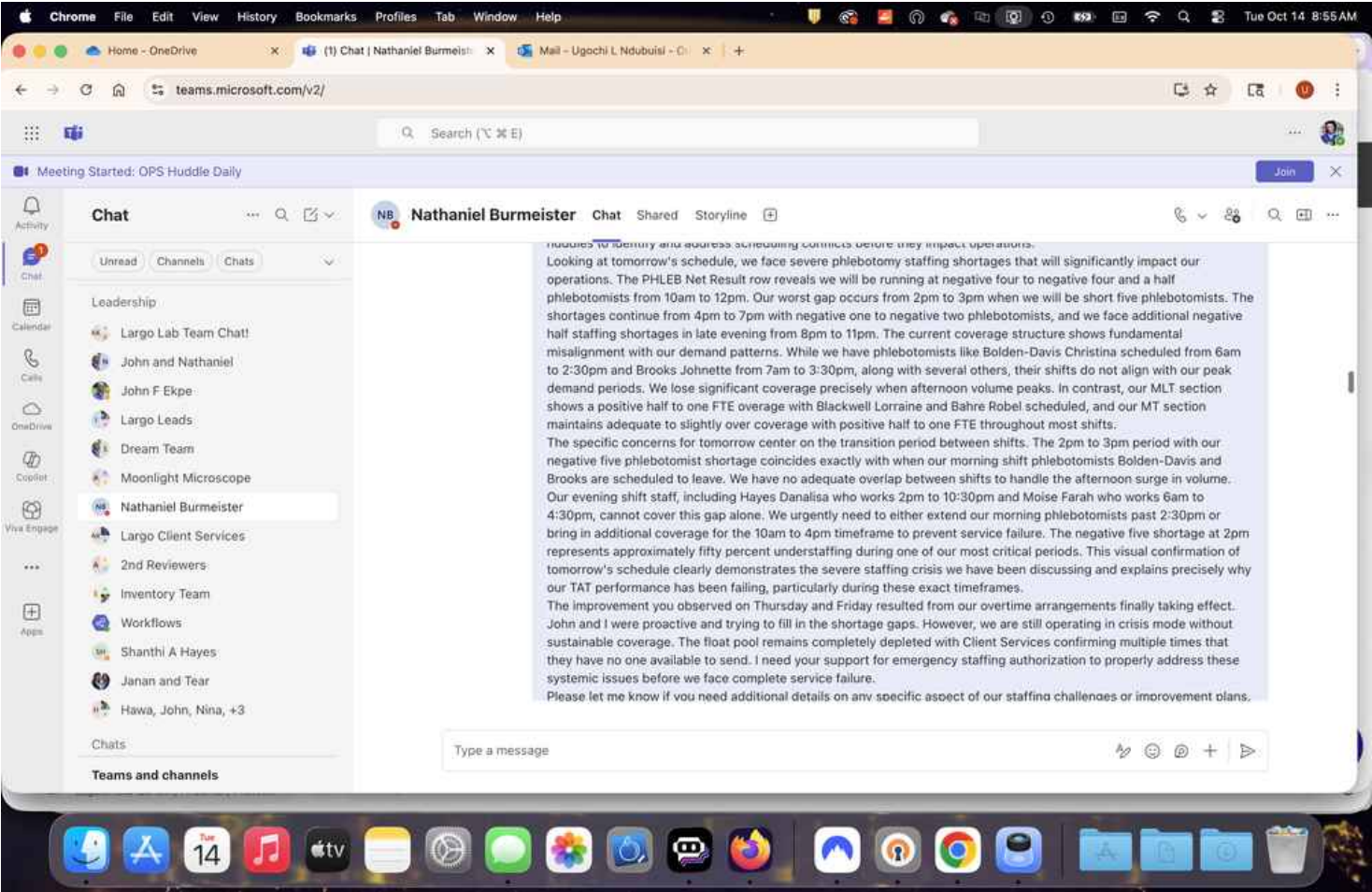
Evidence 14: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.56.10AM

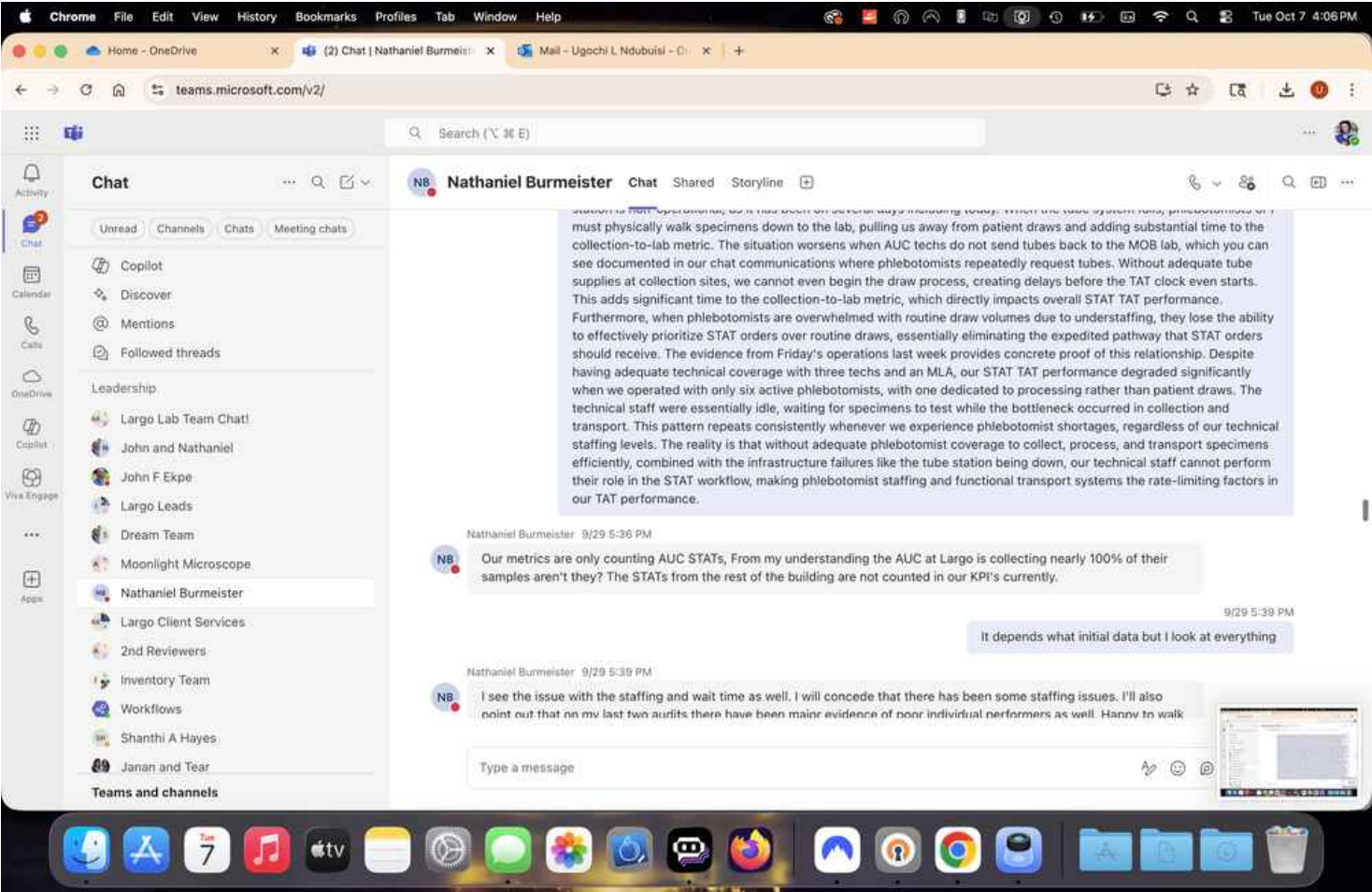


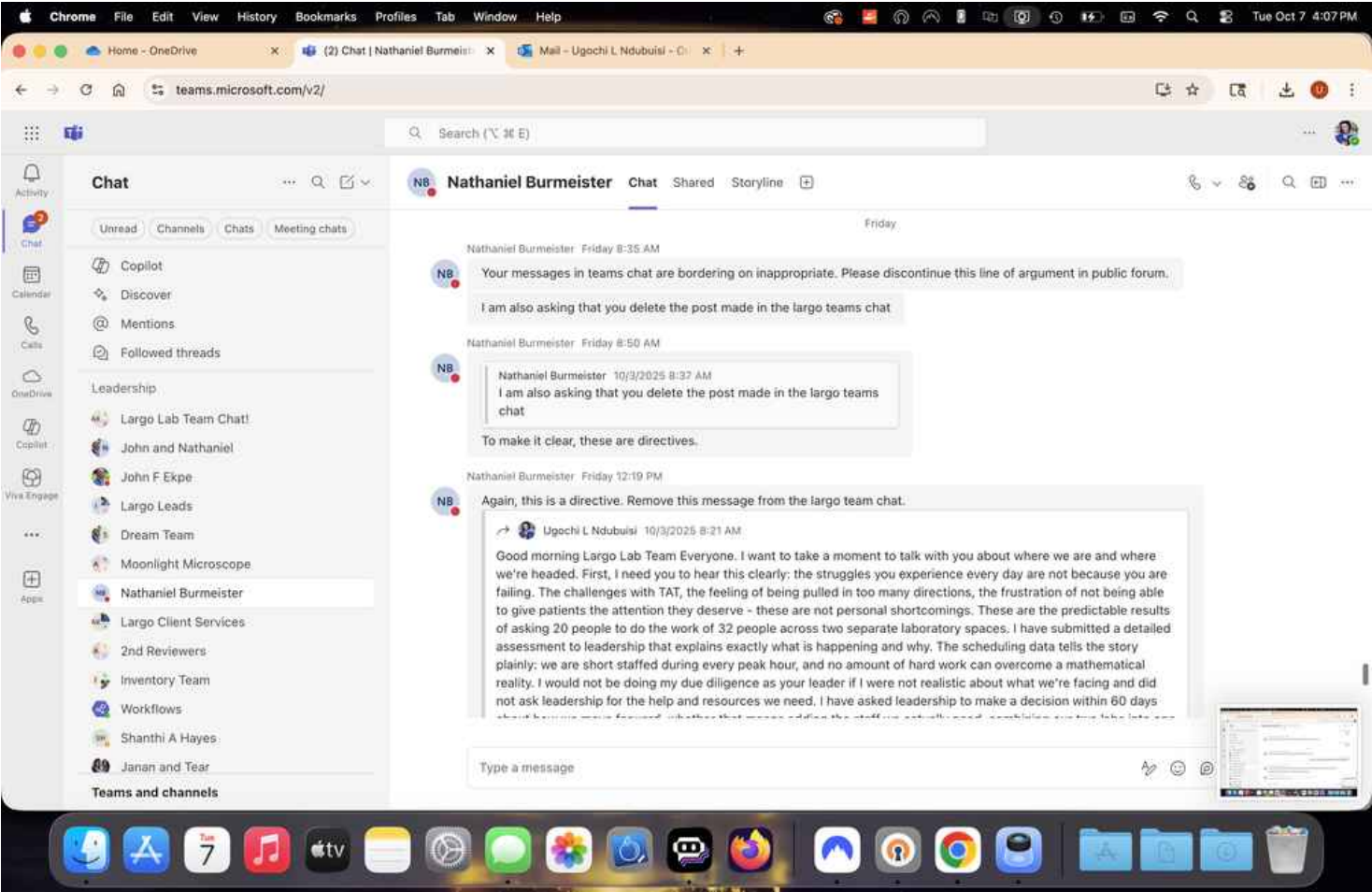


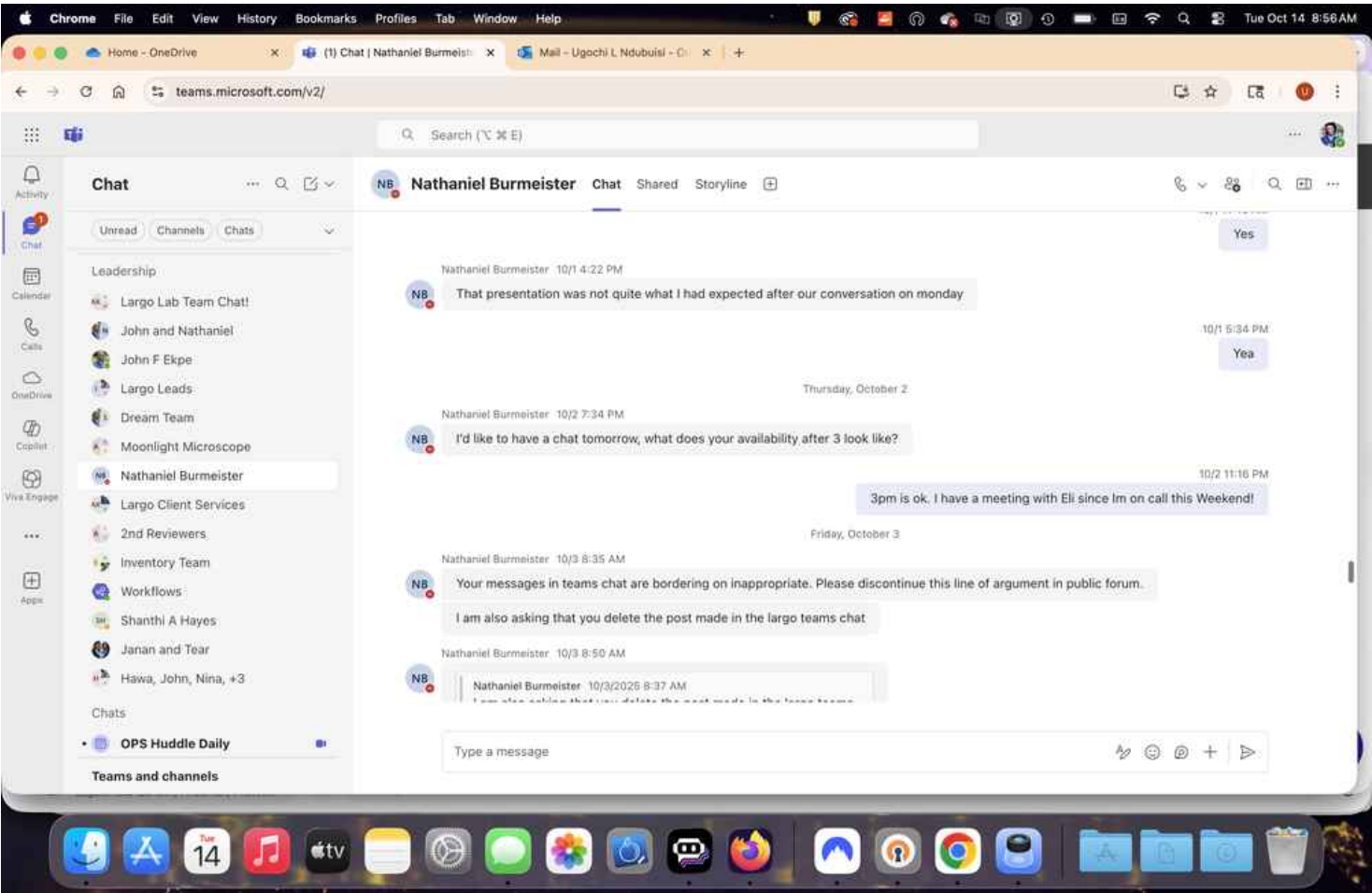
The screenshot shows a Microsoft Teams chat interface on a Mac. The top of the screen displays the macOS menu bar with various system icons and the time 'Tue Oct 14 8:55 AM'. The browser's address bar shows the URL 'teams.microsoft.com/v2/'. The chat window is titled 'Meeting Started: OPS Huddle Daily'. On the left sidebar, there's a list of 'Teams and channels' including 'Largo Lab Team Chat!', 'John and Nathaniel', 'John F Ekpe', 'Largo Leads', 'Dream Team', 'Moonlight Microscope', 'Nathaniel Burmeister', 'Largo Client Services', '2nd Reviewers', 'Inventory Team', 'Workflows', 'Shanthi A Hayes', 'Janan and Tear', and 'Hawa, John, Nina, +3'. The main chat area shows a conversation with 'Nathaniel Burmeister'. The chat history includes a message from him at 9/29 5:26 PM asking, 'How much do your phlebotomist affect the STAT work? From my understanding most if not all of that process is driven by the techs alone'. A response from another user at 9/29 5:34 PM explains that techs run STAT tests but overlooks the critical role of phlebotomists, detailing a significant discrepancy in turnaround times (142 minutes vs. 3,489 minutes) and the impact of staffing shortages on the STAT workflow. The bottom of the screen shows the macOS dock with various application icons.

Evidence 17: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.55.19AM

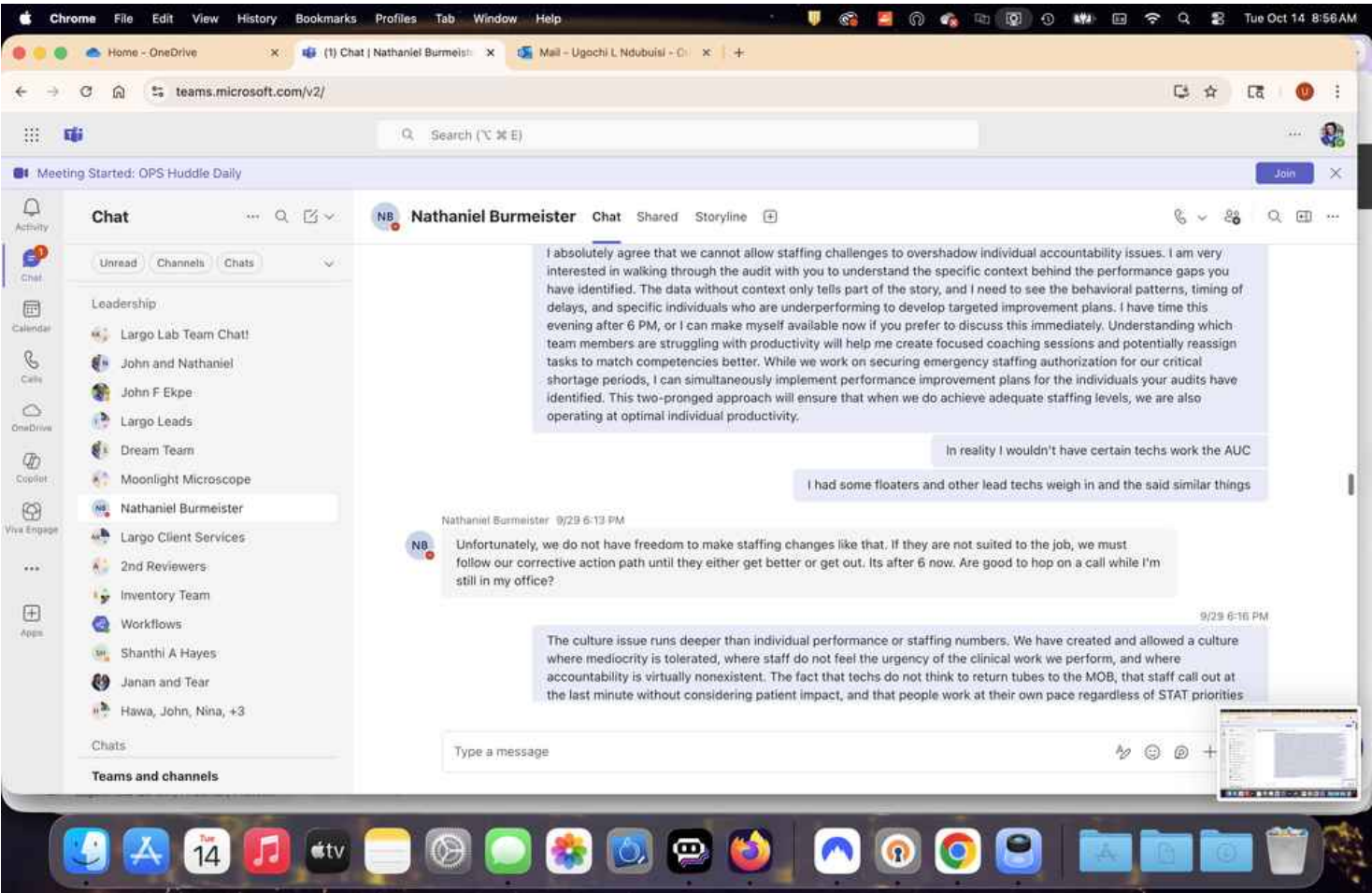


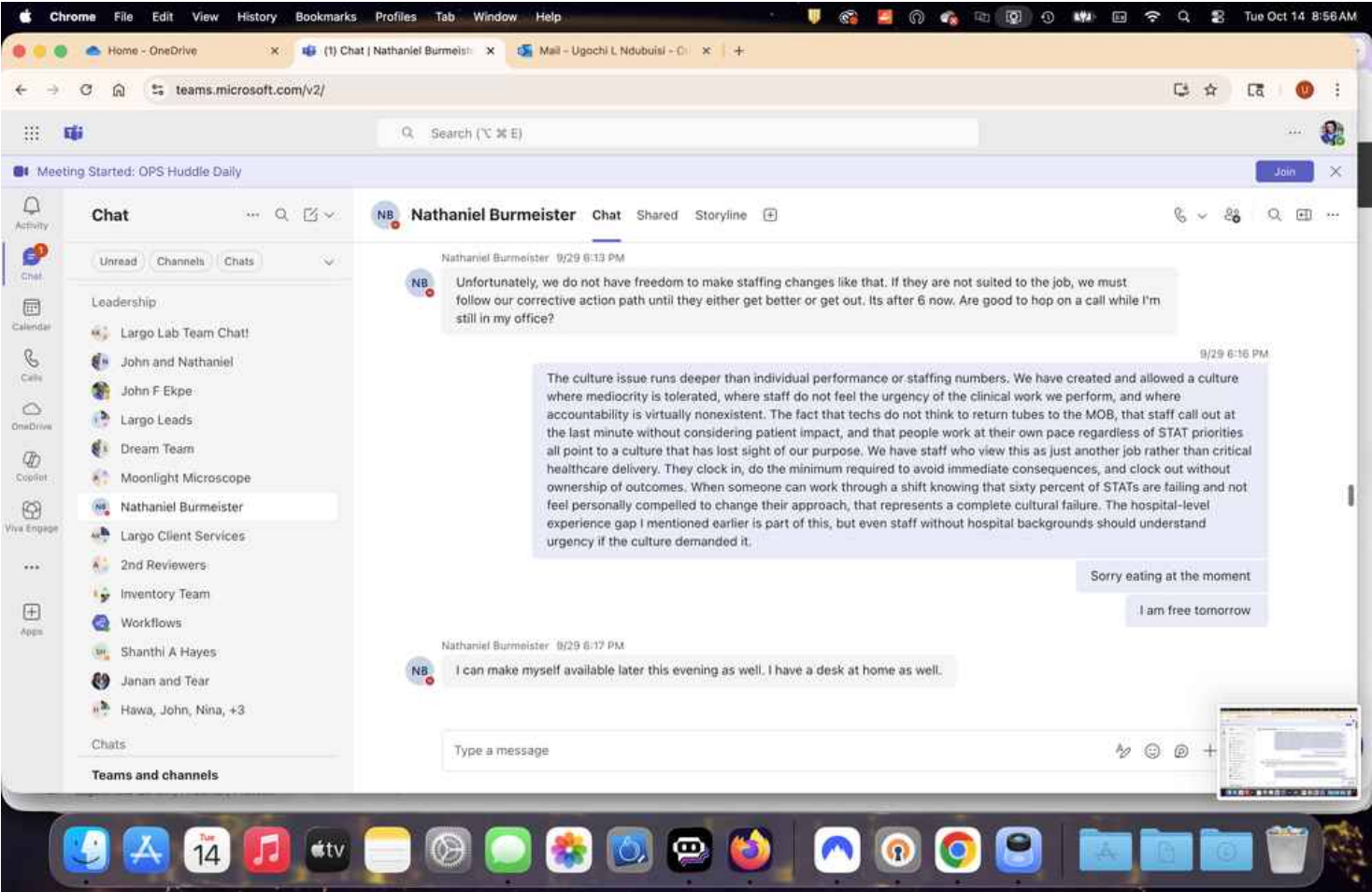


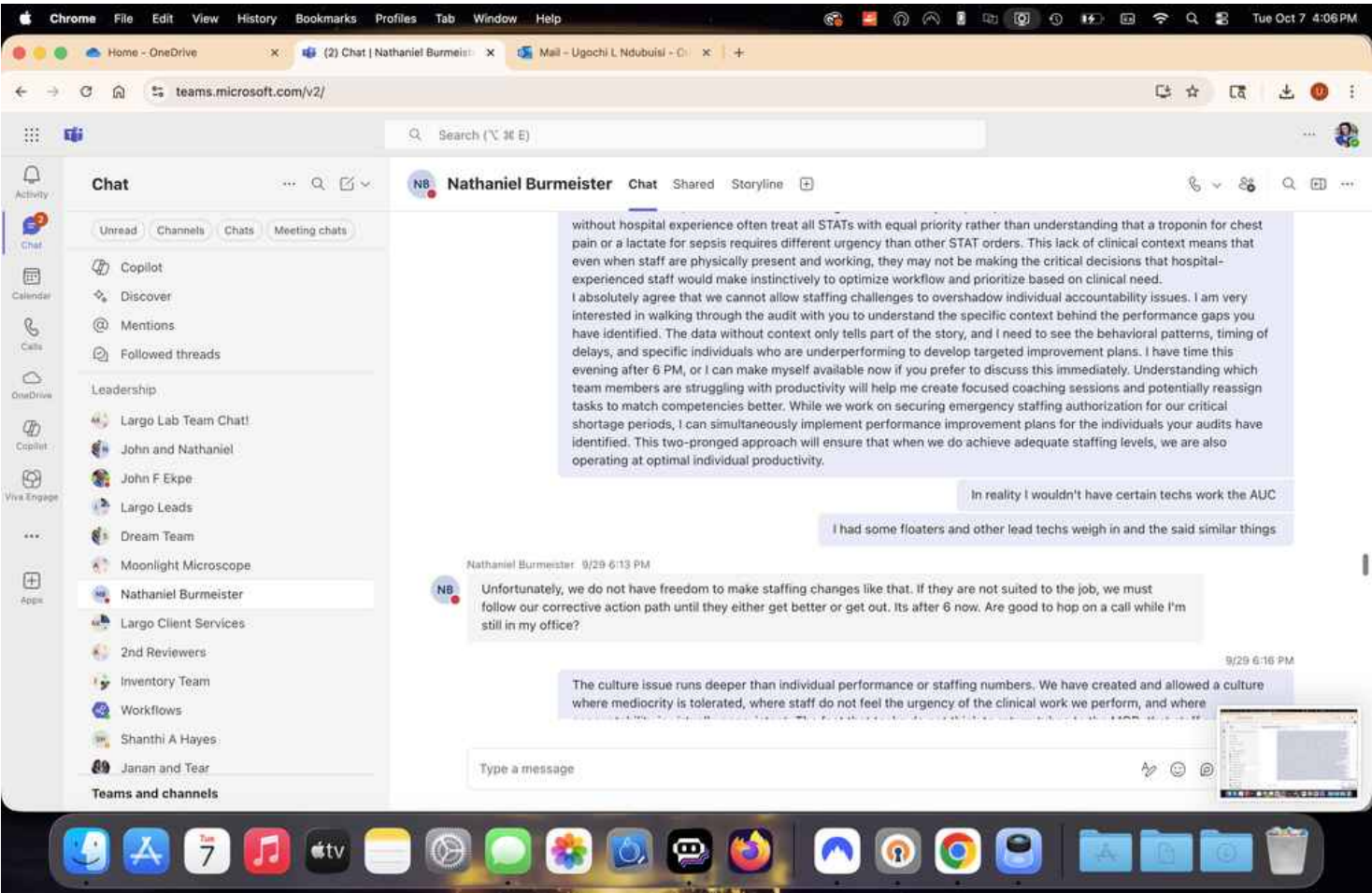


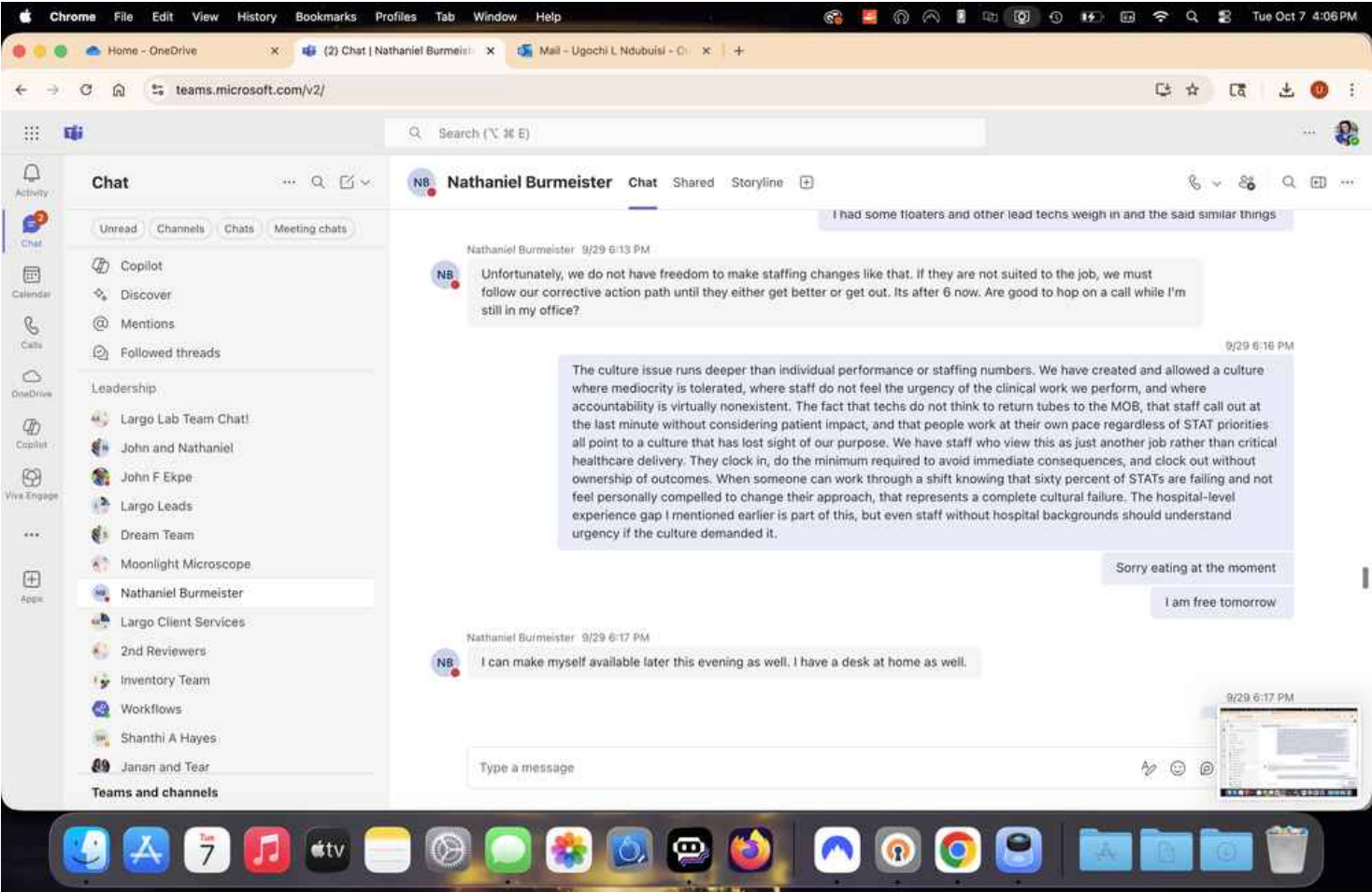


Evidence 21: Oct_8_Final_Warning_Screenshot 2025-10-14 at 8.56.05AM









POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
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DOCUMENT OWNER Chair, National HR Policy Roundtable	PAGE 1 of 3

1.0 Policy Statement

Employees are required to abide by applicable federal, state, and local laws or regulations, to comply with Kaiser Permanente (KP) policies and procedures, and to conduct themselves in accordance with KP's Principles of Responsibility. If an employee fails to comply with any of these or engages in misconduct (including inappropriate/disruptive behavior) or poor performance, the employee may be subject to corrective/disciplinary action, up to and including termination. In addition, the violation of certain laws or regulations may require KP to report the violation to a regulatory or licensing agency and/or may subject an employee to criminal prosecution as well as civil penalties involving substantial monetary fines.

Although represented employees are covered by this policy, the levels of corrective action for represented employees will be determined according to the provisions of the applicable collective bargaining agreement.

Regional procedures must be consistent with this National policy but may include more detailed information regarding corrective/disciplinary action, and employees are subject to those procedures as well.

2.0 Purpose

Kaiser Permanente (KP) must comply with specific legal/regulatory standards that include, but are not limited to, those indicated by Medicare and other government program billing requirements, Guidance issued by the Centers for Medicare and Medicaid Services, the Department of Health and Human Services, the Compliance Program Guidance issued by the Office of the Inspector General, and HIPAA/Privacy and Security regulations. (See Section 6.0, References) KP is required to have well publicized disciplinary guidelines that demonstrate its commitment to compliance and set forth the consequences for violations of compliance mandates.

3.0 Scope/Coverage

This policy applies to all individuals who are employed by the following entities (collectively referred to as "Kaiser Permanente"):

- 3.1** Kaiser Foundation Health Plan, Inc. (KFHP);
- 3.2** Kaiser Foundation Hospitals (KFH);
- 3.3** KFHP/H subsidiaries;
- 3.4** The Permanente Medical Group, Inc. (TPMG) [NOTE: This policy does not apply to physicians, podiatrists, vice presidents, or members of the TPMG Executive Staff, who are covered by separate TPMG policies]; and
- 3.5** Southern California Permanente Medical Group (SCPMG) [NOTE: This policy does not apply to physicians of SCPMG].

4.0 Definitions: n/a

POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
ACCOUNTABLE DEPARTMENT National Human Resources	EFFECTIVE DATE 12/19/2022
DOCUMENT OWNER Chair, National HR Policy Roundtable	PAGE 2 of 3

5.0 Provisions

- 5.1** Except as limited by collective bargaining agreements and/or statute, employment with KP is “at will” (see *At-Will Employment*, NATL.HR.37).
- 5.2** In the event that it is necessary to take corrective/disciplinary action to address concerns related to an employee’s work performance or conduct, the corrective/disciplinary action may include, but is not limited to:
- 5.2.1** verbal discussion, coaching, and/or warning by the employee’s immediate supervisor or higher-level manager to correct the problem
 - 5.2.2** written notice, with or without final warning
 - 5.2.3** suspension, with or without final warning
 - 5.2.4** termination of employment
- 5.3** The corrective/disciplinary action steps listed in section 5.2 are not exhaustive and serve only as a description of possible action. None of these actions are mandatory or a prerequisite for any other actions that may be appropriate based on the circumstances. Any or all of these actions may be employed prior to termination and certain employee conduct may be deemed sufficiently serious to warrant immediate termination or escalated disciplinary action without prior corrective/disciplinary action.
- 5.4** The level of corrective/disciplinary action will be determined by management after review of the facts (including legal, compliance or policy violations), applicable regional/national collective bargaining agreement provisions, and consideration of other relevant factors and circumstances including, but not necessarily limited to, the factors in section 5.5 below. Management should consult Human Resources as appropriate.
- 5.5** Examples of factors to be considered in determining the level of corrective/disciplinary action include, but are not limited to, the following:
- 5.5.1** the severity of the violation
 - 5.5.2** the number of violations
 - 5.5.3** whether the violation was part of a pattern or practice of improper behavior or non-compliance
 - 5.5.4** the individual’s past performance and conduct
 - 5.5.5** whether the individual knew or should have known the applicable policies, rules or regulations
 - 5.5.6** length of employment at KP
 - 5.5.7** whether the violation was intentional or negligent
 - 5.5.8** whether the action appeared to be committed for personal gain
 - 5.5.9** regional/national collective bargaining agreements corrective/disciplinary action provisions for represented employees



RE: Lab delay

From Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>

Date Fri 6/6/2025 3:41 PM

To Alice J Gouvernayre <Alice.J.Gouvernayre@kp.org>; Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>; Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>

Cc Sunny L Lambert <Sunny.L.Lambert@kp.org>; Carole R Kelly <Carole.R.Kelly@kp.org>; Nona Alexander <Nona.Alexander@kp.org>; Erin M Smith <Erin.M.Smith@kp.org>; Zachary Smith <Zachary.X3.Smith@kp.org>; James A Powell <James.A.Powell@kp.org>; Daniel J Patenaude <Daniel.J.Patenaude@kp.org>

Hello Dr. Gouvernayre,

Ugochi and I did a thorough investigation to discover root causes for the failures that were experienced on Monday. We talked about best strategy to mitigate that root cause, and I am confident that it is being addressed. As it is something that will include employee disciplinary actions, I will not go into details in this forum.

As far as regular state, we do normally perform maintenance on our instruments during our lower volume periods. Our major instrumentation all has redundancy. So even during regular maintenance, we have availability to test 24/7 (barring unscheduled downtime). For the Chemistry instruments, we perform one instrument between 9p-12a and the other is performed between 4a-8a. The process usually takes about 1-2 hours, and we have the flexibility to delay up to a few hours if testing demands are high. Ugochi and John are monitoring and reviewing for possible optimizations on this and other areas of the lab.

Ugochi and John will continue to reinforce the use of electronic communication through KP Healthconnect chat as best practice. I'll also continue to connect them with coaching and resources to best serve the operations needs of the Lab and by extension the AUC.

-Nate Burmeister

C:202-570-9030

From: Alice J Gouvernayre <Alice.J.Gouvernayre@kp.org>

Sent: Friday, June 6, 2025 3:10 PM

To: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>; Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>

Cc: Sunny L Lambert <Sunny.L.Lambert@kp.org>; Carole R Kelly <Carole.R.Kelly@kp.org>; Nona Alexander <Nona.Alexander@kp.org>; Erin M Smith <Erin.M.Smith@kp.org>; Zachary Smith <Zachary.X3.Smith@kp.org>; Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>; James A Powell <James.A.Powell@kp.org>; Daniel J Patenaude <Daniel.J.Patenaude@kp.org>

Subject: RE: Lab delay

Hi Ugochi,

Thank you for taking the time to explain the situation so thoroughly to us. Can you tell us when the QC for Pure 2 is scheduled to occur?

I have a few questions/suggestions based on the information you provided.

1. Communication:

- a. I understand that you came over that night to inform the team of the issues you were having. It's really nice of you to do that, but not everyone gets the message when you come in person. I think the best way to make sure we all understand what's going on, is for you or another manager to send a detailed message about the issue and its impact to the lead nurse on epic chat. The lead nurse will then copy that message and send it to all that are working in the AUC that day. This workflow is good for temporary delays. If an instrument is down or for any other major issue, please continue to notify the DCSM critical operation alert on epic chat.

Reason: If physicians and nurses understand the lab limitations, we are then able to adjust the plan of care for patients appropriately (i.e. send patients home and message them on kp.org when the results are available versus the traditional path of having the patients wait in the department for results). Or we can set expectations with patients right off the bat on how long it will take to figure out what's wrong with them. The sooner we know, the better.

- b. Regarding the high volume of calls – I understand how disturbing this might have been. I think if we follow the communication pathway outlined above, you will get less calls. I also do want to point out that we wouldn't be doing our job if we left a patient waiting for a strep test result for hours without inquiring about the result repeatedly on their behalf.

1. Mitigation of impacts:

- a. I understand the chemistry machine was down and the urinalysis instrument was also not working but why did we have a 10-hour strep test delay? Or why were CBCs affected?
- b. What is the escalation pathway when you are short-staffed? What is the escalation pathway when the machines are malfunctioning? What is put in place to preserve operations across all testing instruments?

We look forward to working on solutions with you to improve operations and that we can present at the throughput meeting.

Best,

Alice J Gouvernayre, MD

Chief, Acute Care Services

DC and Suburban Maryland Service Area

Permanente Medicine

Mid-Atlantic Permanente Medical Group

Capitol Hill Medical Center

700 2nd St. NE,

Washington, DC 20002

202-853-1311 (mobile)

alice.j.gouvernayre@kp.org

For assistance:

Rodnee Petty at 202-680-3404 or rodnee.s.petty@kp.org

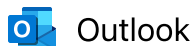
[Facebook](#) | [Twitter](#) | [LinkedIn](#) | [Instagram](#) | [YouTube](#)

Kaiser Permanente

From: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Sent: Tuesday, June 3, 2025 10:45 PM

To: Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>



RE: Activity and Camera Footage Report Request

From Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>

Date Mon 7/14/2025 7:11 PM

To Janis A Thom-Grate <Janis.A.Thom-Grate@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>

Cc Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Thank you Janis,

I'll walk them through this portal at a later date, I'll be submitting on their behalf tonight.

-Nate Burmeister

C:202-570-9030

From: Janis A Thom-Grate <Janis.A.Thom-Grate@kp.org>

Sent: Monday, July 14, 2025 7:10 PM

To: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>

Cc: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Subject: RE: Activity and Camera Footage Report Request

Here is the link to submit the security request –

[Online Security Incident Report](#)

-Janis

Janis A. Thom-Grate, J.D., SPHR, SHRM-SCP

Pronouns: She, Her, Hers

HR Consultant IV

Kaiser Permanente Mid-Atlantic States

(c) 240.495.1364

Janis.A.Thom-Grate@kp.org

Upcoming PTO:

HR RESOURCES FOR ALL EMPLOYEES	HR RESOURCES FOR MANAGERS
- HR transactions including pay, benefits, leaves and HR system navigation inquiries: Contact the HR Service Center or Timekeeping or at 1-877-457-4772. Fax: 1-877-477-2329 - Retirement (pension plan, retiree medical and/or retiree life insurance benefits): Visit the KPRC website at www.myretirement.kp.org or call the KPRC at 1-866-627-2826, #0. - Defined contribution plans-related inquiries (Plan B, TSA, and Roth): Visit the Vanguard website at www.vanguard.com or call Vanguard at 1-800-523-1188.	- For information you cannot find in the HRconnect system or at hrconnect.kp.org: <i>Position management:</i> Email pm-support-team@kp.org or <i>Call the National HRSC at 1-877-457-4772, Options 3,4,5.</i> <i>Other transactions for individuals on your team (such as timekeeping, new hires, terminations, and general HR systems questions): Call the National HRSC at 1-877-457-4772, Options 3,4.</i>

From: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Sent: Monday, July 14, 2025 7:04 PM
To: John F Ekpe <John.F.Ekpe@kp.org>; Janis A Thom-Grate <Janis.A.Thom-Grate@kp.org>
Cc: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Subject: RE: Activity and Camera Footage Report Request

Hello John and Ugochi,

Whenever we wish to pull camera footage, we must also include our HRC partner in that request. [@Janis A Thom-Grate](mailto:Janis.A.Thom-Grate@kp.org), We will be requesting for camera footage to support an investigation into possible time fraud. We have reason to believe that employees are leaving their workstation for extended periods of time while clocked in. John and Ugochi have also requested badge swipes for the days below and for a few other days (7/2, 7/5, and 7/6) all between 7pm and 10pm. We will also be pulling Cerner activity data for the days listed.

-Nate Burmeister
C:202-570-9030

From: John F Ekpe <John.F.Ekpe@kp.org>
Sent: Monday, July 7, 2025 7:21 PM
To: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Cc: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Subject: Activity and Camera Footage Report Request

Hi Nate,

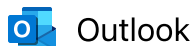
Thank you for taking the time to chat with me earlier about our AUC TAT metric, staff performance and related matters. Based on our discussions, I would like to request activity report pull on the following days and the camera report can be pulled between 7pm - 10pm.

Date: July 2nd, 5th and 6th.

Thank you.

John F. Ekpe, MLS (ASCP)cm
Manager, Laboratory Operations
Largo Medical Center
1221 Mercantile Ln, Upper Marlboro, MD 20774
Email: john.f.ekpe@kp.org
Phone: (301)-996-0119

NOTICE TO RECIPIENT: If you are not the intended recipient of this e-mail, you are prohibited from sharing, copying, or otherwise using or disclosing its contents. If you have received this e-mail in error, please notify the sender immediately by reply e-mail and permanently delete this e-mail and any attachments without reading, forwarding or saving them. v.173.295 Thank you.



RE: Meeting Request - Tomorrow at 4pm or Alternative Time

From Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>

Date Tue 6/10/2025 9:09 PM

To Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

I'll be on site on Friday and can assist

-Nate Burmeister

C:202-570-9030

From: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Sent: Tuesday, June 10, 2025 3:49 PM

To: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>

Subject: Re: Meeting Request - Tomorrow at 4pm or Alternative Time

Hi Nate,

I'd like to discuss two concerns regarding Tracy's recent performance that require your input. I would like for you to be there for this CA due to complexity but I can lead:

1. **June 2nd Incident:** We experienced a 6-hour turnaround time delay when Tracy claimed to be performing Pure Analyzer Maintenance and QC on Pure 2 instead of the correct Pure 1 analyzer in the MOB. The maintenance was done incorrectly and caused numerous issues, to the point where Night shift was experiencing problems due to the errors made. This significantly impacted patient care and our standard TAT for the AUC. I'd like to meet with her and her shop steward as well as with you to review the events of that day as a fact-finding opportunity to determine if a Corrective Action is warranted.
2. **Ongoing Tardiness Pattern:** Tracy has demonstrated a consistent pattern of arriving late for her shifts:
 - o April 21: Arrived at 3:43 PM
 - o April 27: Arrived at 3:37 PM
 - o April 29: Arrived at 3:44 PM
 - o April 30: Arrived at 3:38 PM
 - o May 25: Arrived at 3:35 PM
 - o May 28: Arrived at 3:37 PM

Additionally, she has been tardy on March 31, April 8, April 10, April 15, April 17, and most recently on June 5.

I have prepared the relevant documentation for our discussion, including:

- The Attendance Policy
- Pure Maintenance SOP with implementation dates

Based on our attendance policy, I believe a Level 1 Corrective Action is appropriate for the tardiness issues. However, I'd like your guidance on how to proceed with the June 2nd incident.

From: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Sent: Monday, June 9, 2025 10:35 PM
To: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Subject: RE: Meeting Request - Tomorrow at 4pm or Alternative Time

Can you share with me your plan for this meeting. Let me know what support you may need.

From: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Sent: Monday, June 9, 2025 10:36 AM
To: Oghenechuko T Eshofa <Oghenechuko.T.Eshofa@kp.org>; Jamie T Anderson <Jamie.T.Anderson@kp.org>
Cc: John F Ekpe <John.F.Ekpe@kp.org>; Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Subject: Meeting Request - Tomorrow at 4pm or Alternative Time

Good morning!

I would like to schedule a 1-hour meeting with both of you to discuss some important matters. I'm available tomorrow at 4pm.

If tomorrow at 4pm doesn't work, please note that I have an all-day meeting on Wednesday, but I'm flexible for any other time that fits your schedules.

Please let me know what works best for you both.

Very Respectfully,

Ugochi Ndubuisi, DHSc, MT(AMT)^{CM}, MB(ASCP)^{CM}
Laboratory Operations Manager

Kaiser Permanente Mid-Atlantic Region
Largo Medical Center
1221 Mercantile Ln, Upper Marlboro, MD 20774
Email: Ugochi.L.Ndubuisi@kp.org

Phone: 301-848-6794



KAISER PERMANENTE®

WHISTLEBLOWER RETALIATION EVIDENCE

Largo Laboratory - Kaiser Permanente

EXECUTIVE SUMMARY

This document presents chronological evidence of retaliation against Ugochi L. Ndubuisi for protected whistleblowing activity. The timeline shows:

1. July 14, 2025: Director orders investigation
2. September 29, 2025: Analysis provided and acknowledged
3. October 3, 2025: Same analysis shared with staff (protected activity)
4. October 3, 2025: Four censorship demands in 8 hours
5. October 8, 2025: Final Written Warning (first discipline in 9 months)

Maryland OSHA Deadline: November 7, 2025

CRITICAL TIMELINE

Date/Time	Event	Significance
July 14, 2025	Investigation Ordered	Director orders investigation into second shift
Sept 29, 2025	Analysis Provided	"Culture issue runs deeper" - acknowledged by director
Oct 3, 8:21am	Whistleblowing	41.23% failure rate shared with staff
Oct 3, 8:35am	Censorship #1	"Inappropriate" - 14 minutes after sharing
Oct 3, 8:50am	Censorship #2	"Delete the post" - 29 minutes after
Oct 3, 12:19pm	Censorship #3	"This is a directive" - escalation
Oct 3, 4:30pm	Censorship #4	Fourth deletion demand
Oct 8, 2025	Final Warning	First discipline ever - clear retaliation

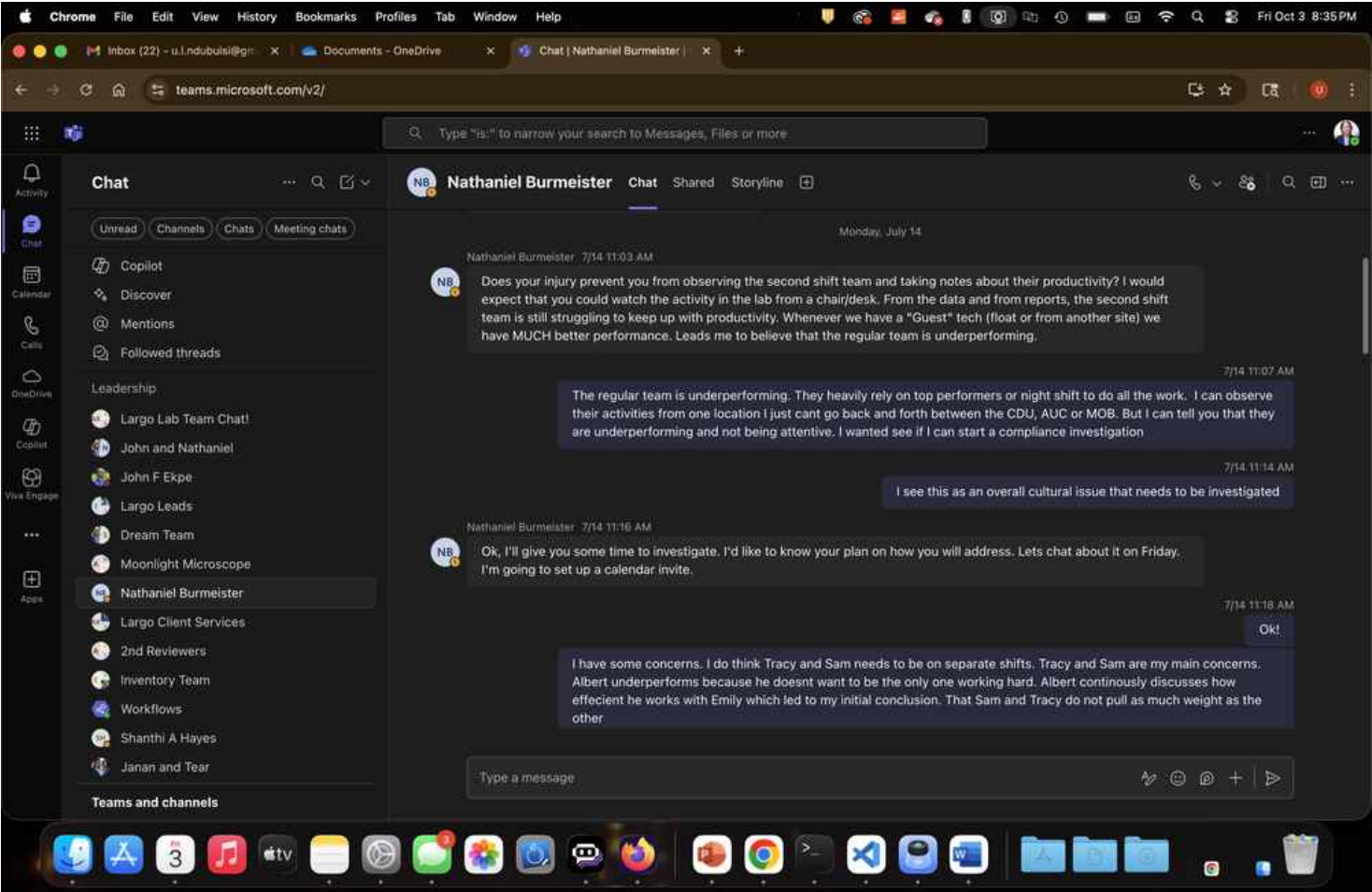
PATTERN OF RETALIATION - KEY EVIDENCE

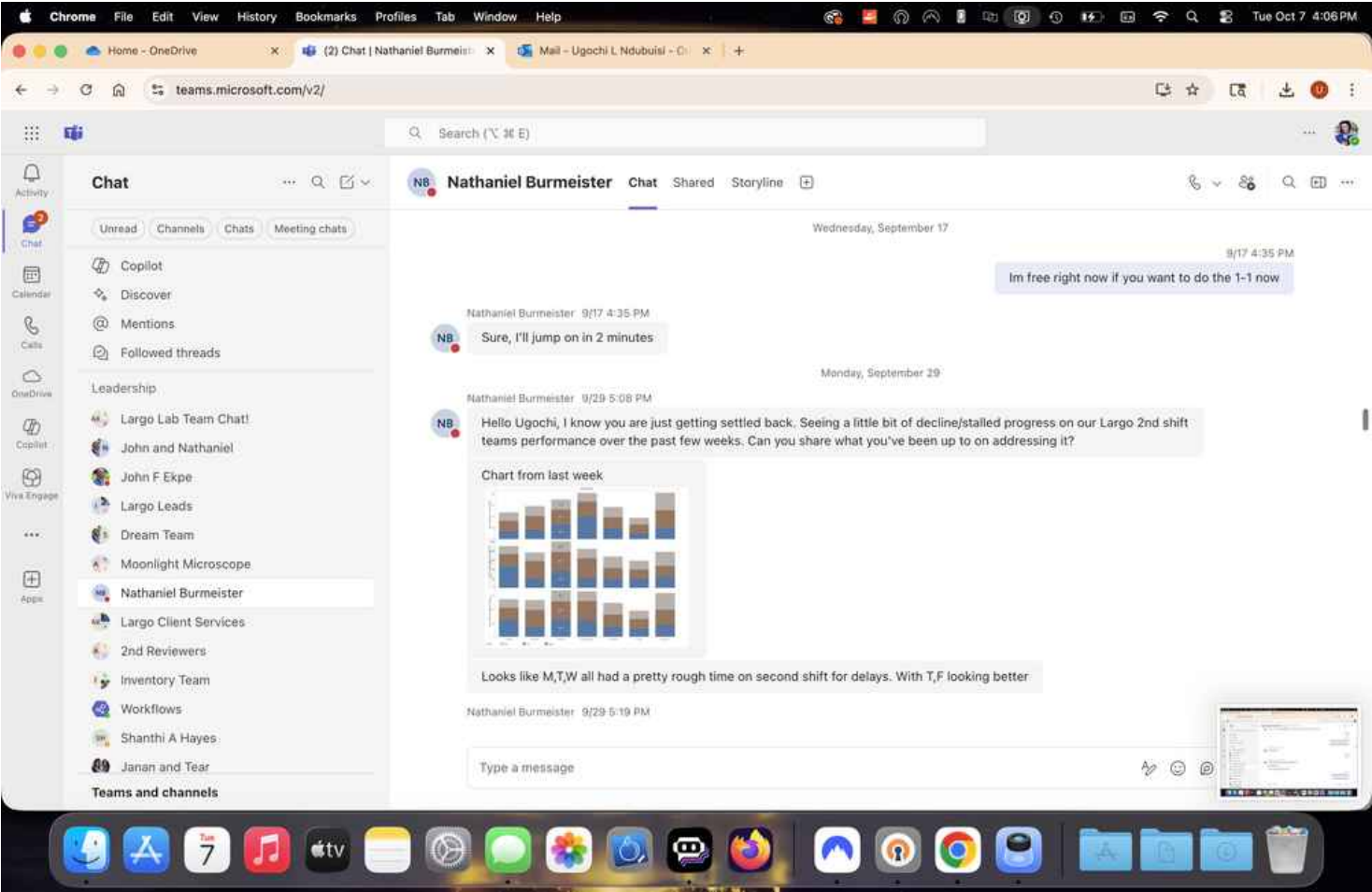
- 1. Protected Activity:** Reported workplace safety concerns affecting patient care (41.23% failure rate)
- 2. Temporal Proximity:** Only 5 days between whistleblowing and Final Written Warning
- 3. Disparate Treatment:** Same analysis praised by director on Sept 29, punished when shared Oct 3
- 4. Censorship Pattern:** Four deletion demands in single day shows intent to silence
- 5. Clean Employment Record:** No prior discipline in 9 months - sudden jump to Final Warning
- 6. Public Interest:** Patient safety at Level 3 Trauma facility serving community

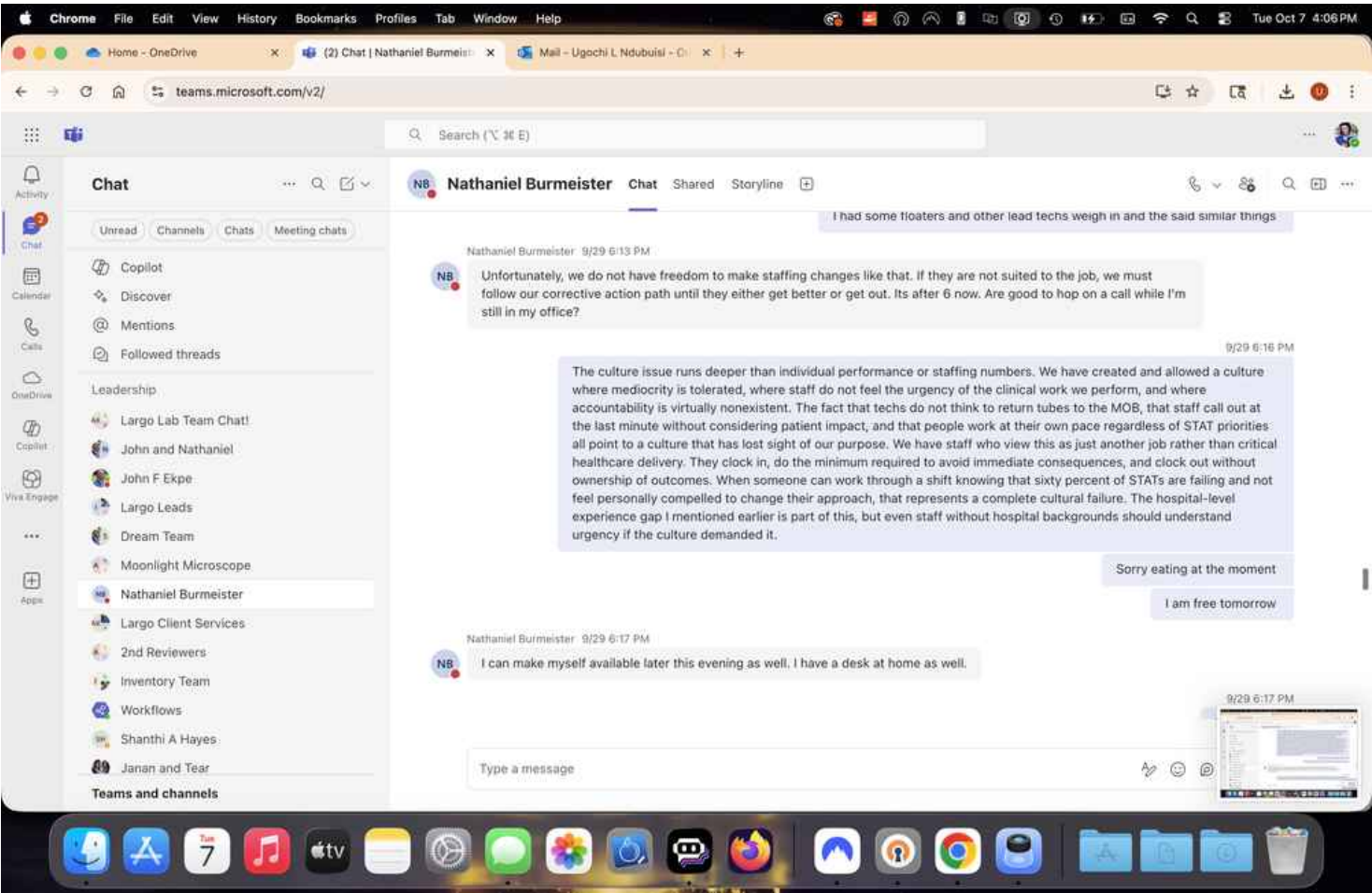
Filing Information:

- Maryland OSHA Deadline: November 7, 2025
- Protected under Maryland Occupational Safety and Health Act
- Document Generated: October 14, 2025

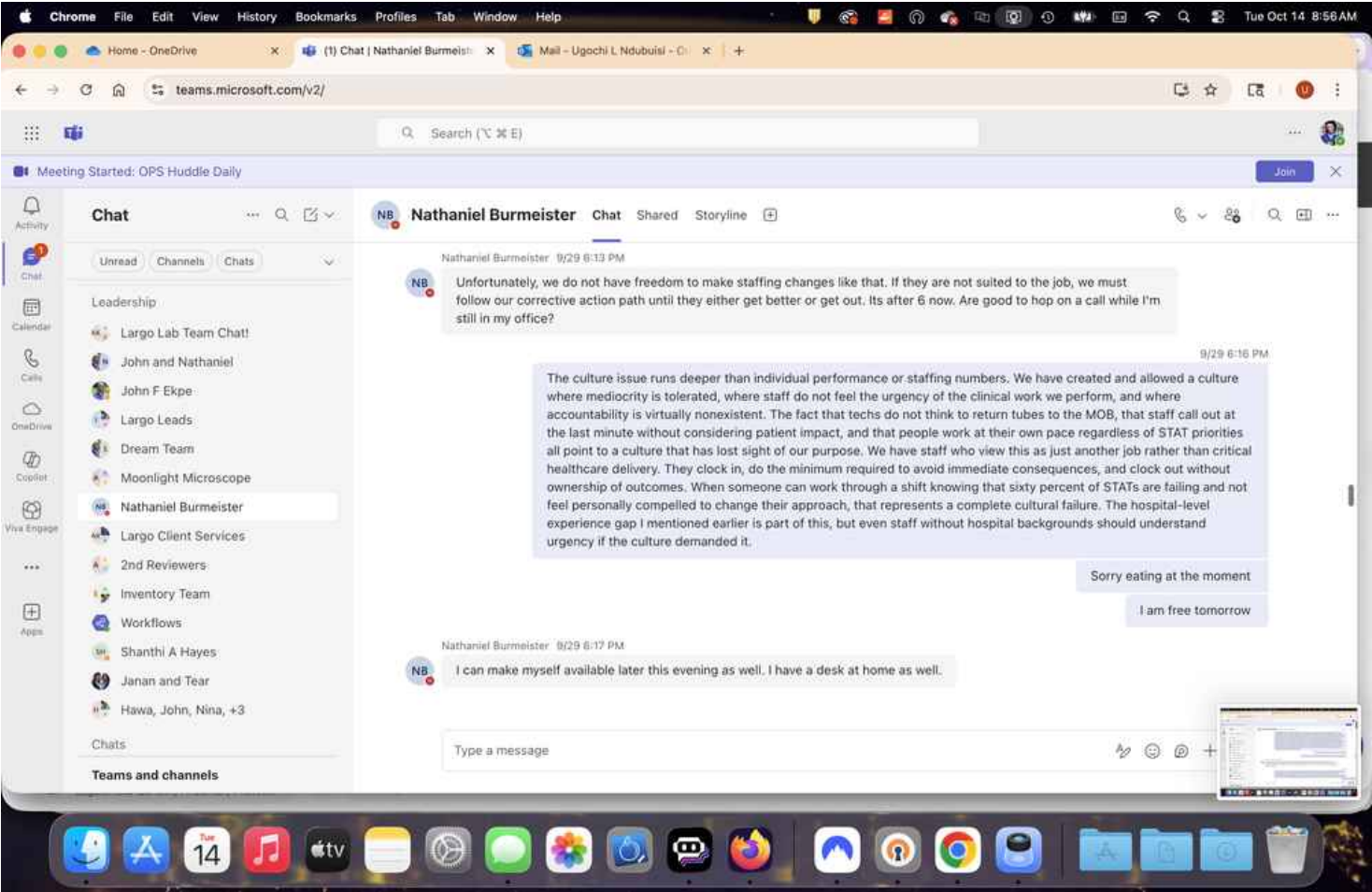
Evidence 1: July_14_Investigation_01_Screenshot 2025-10-03 at 8.35.06PM

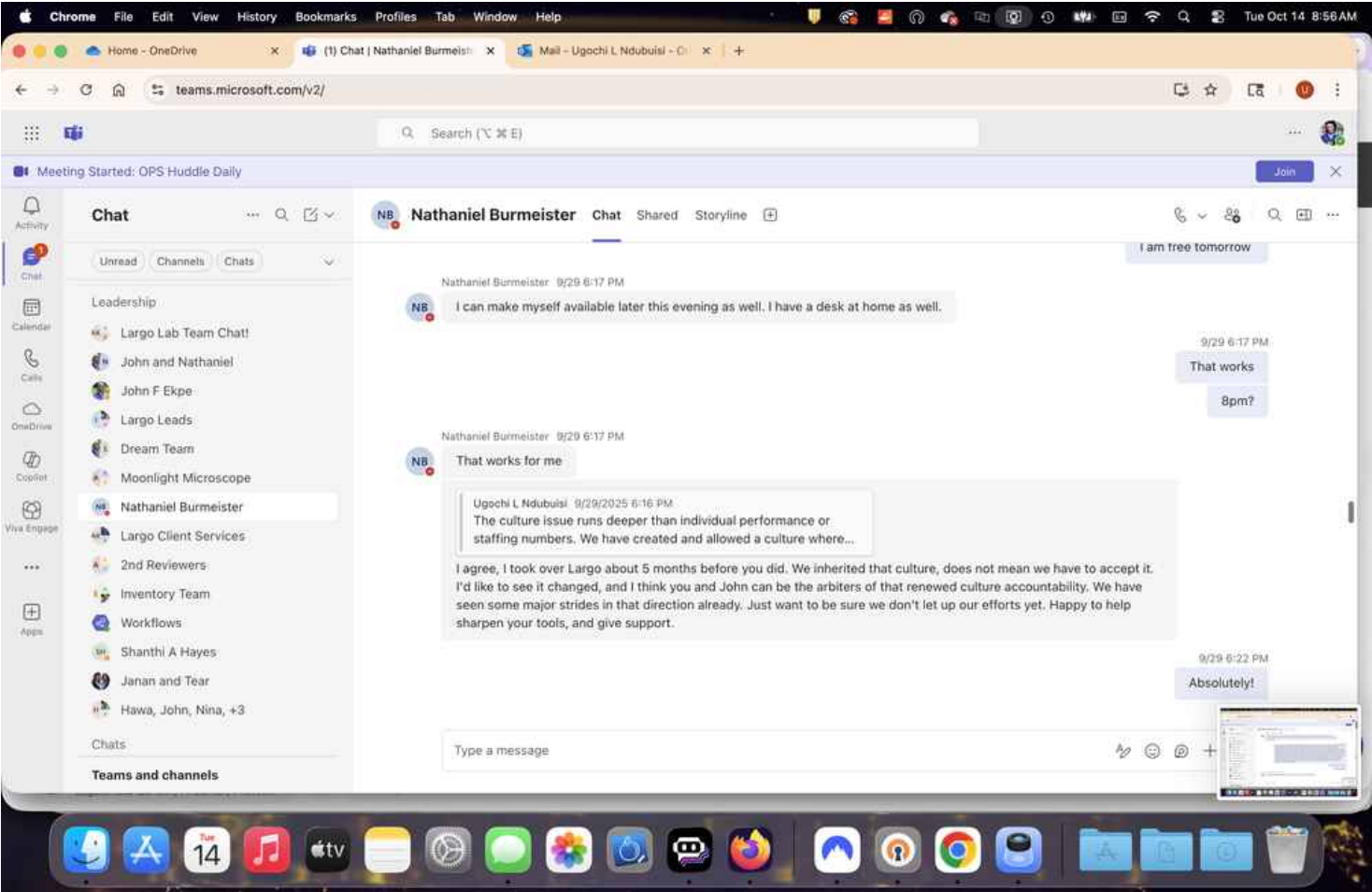


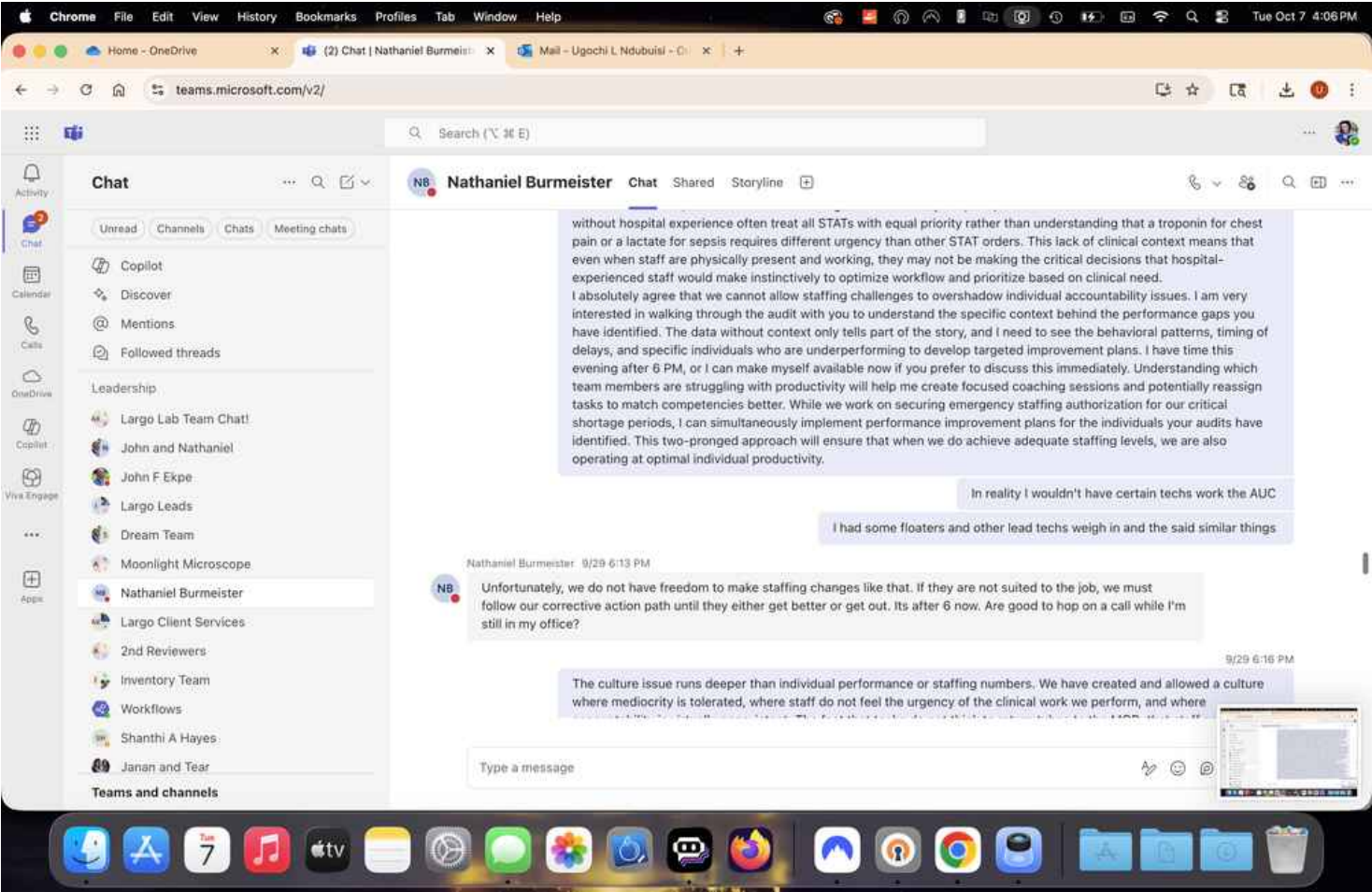


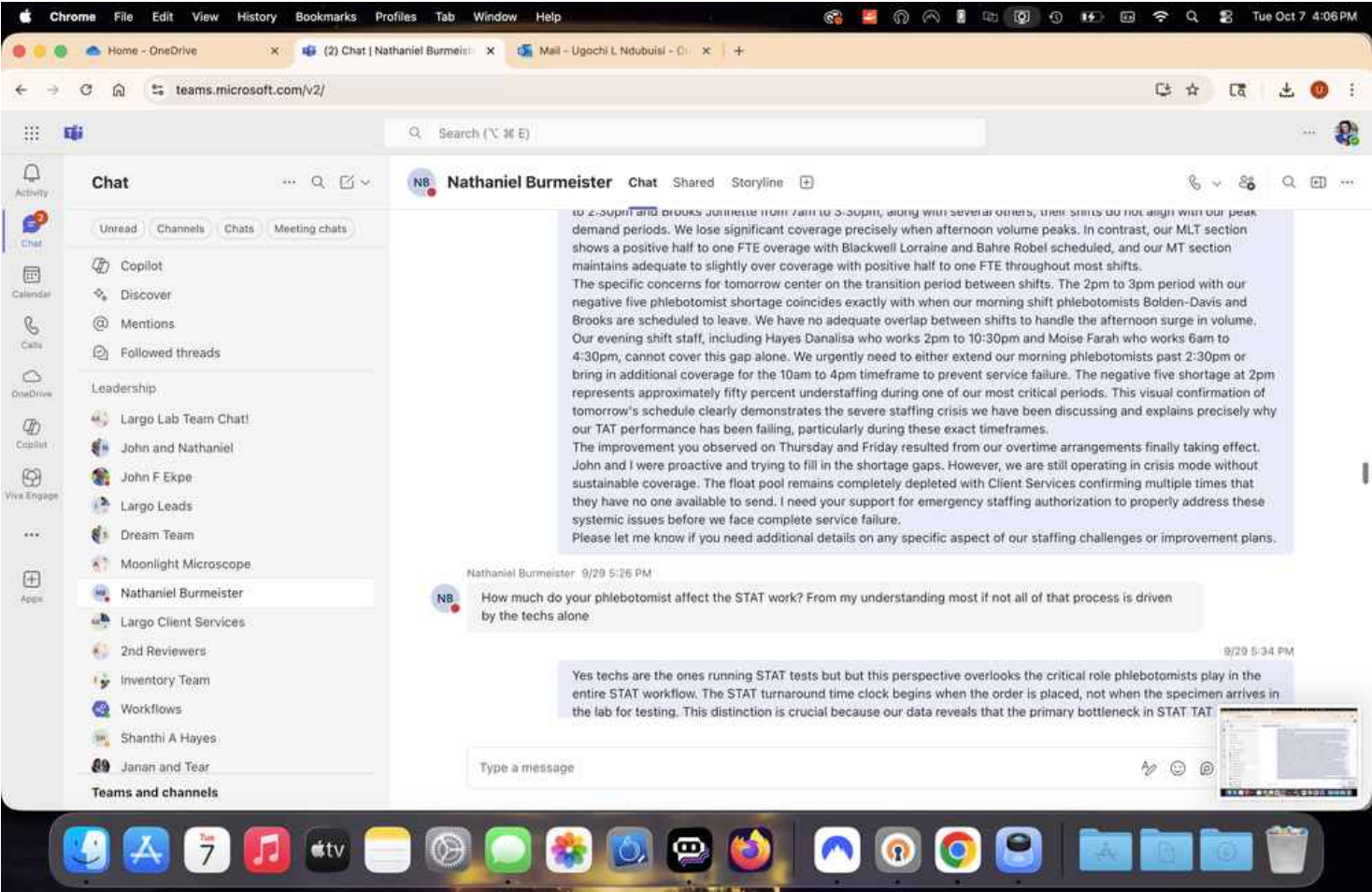


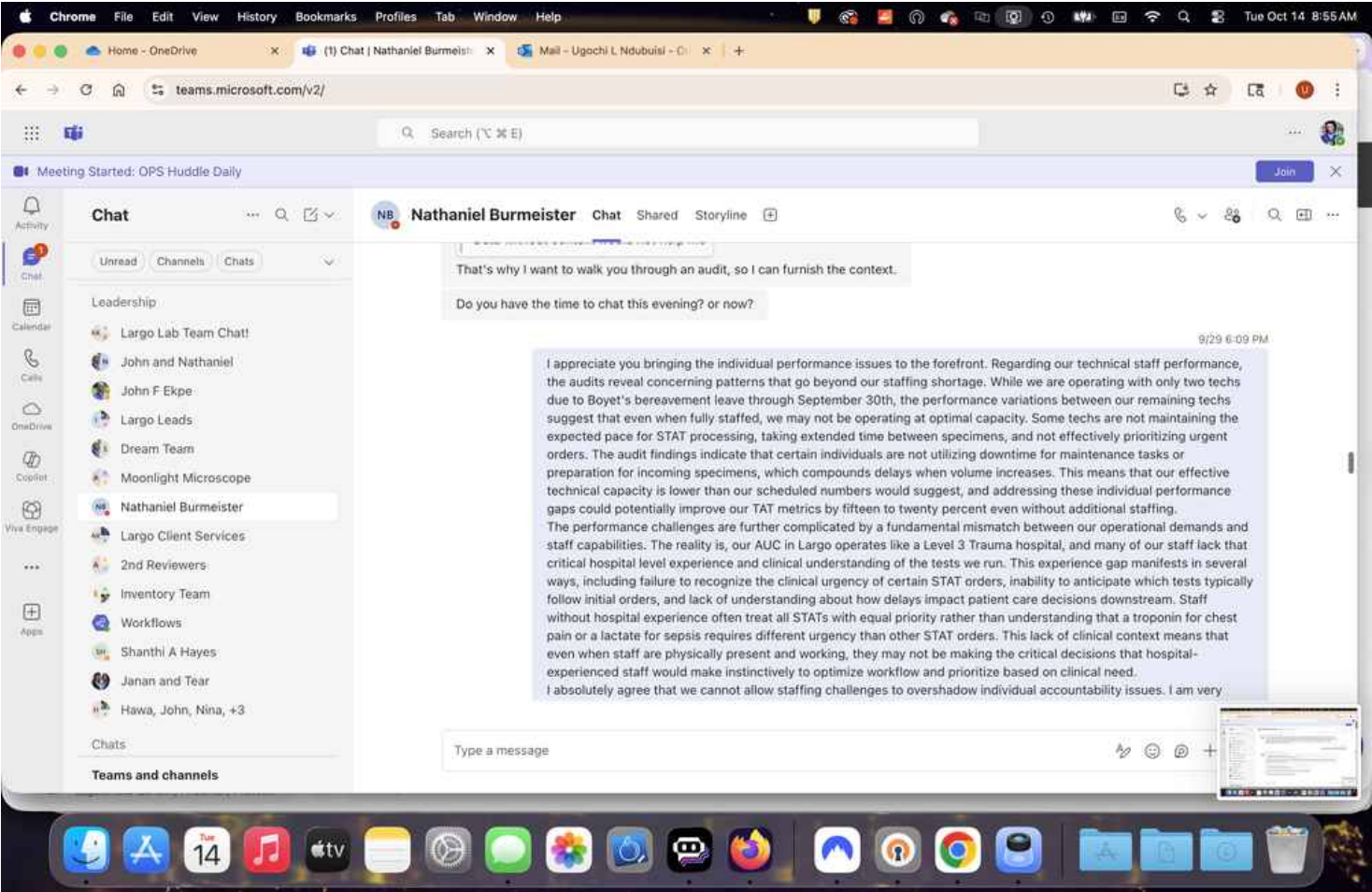
Evidence 4: Sept_29_Analysis_01_Screenshot 2025-10-14 at 8.56.10AM

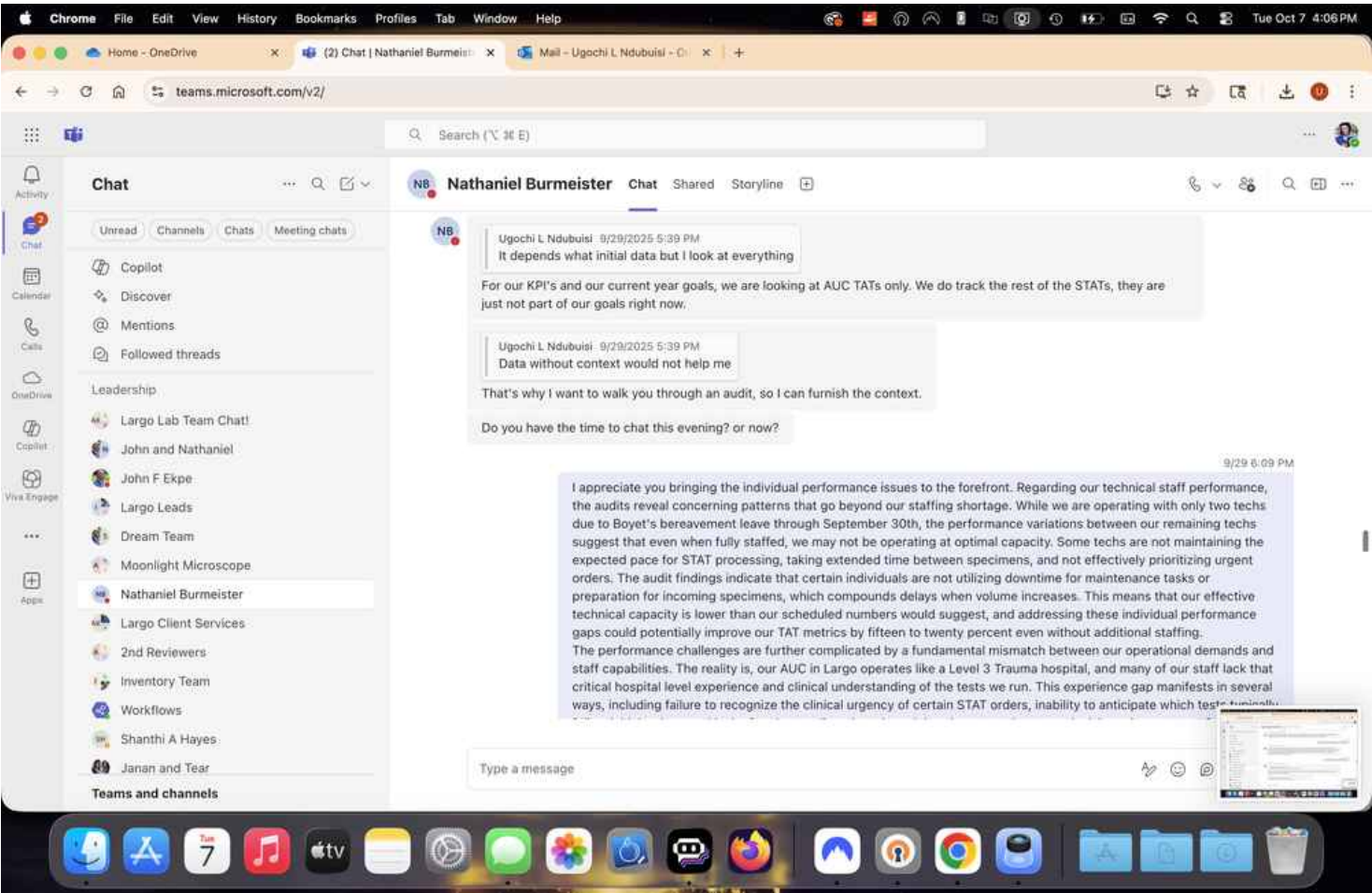


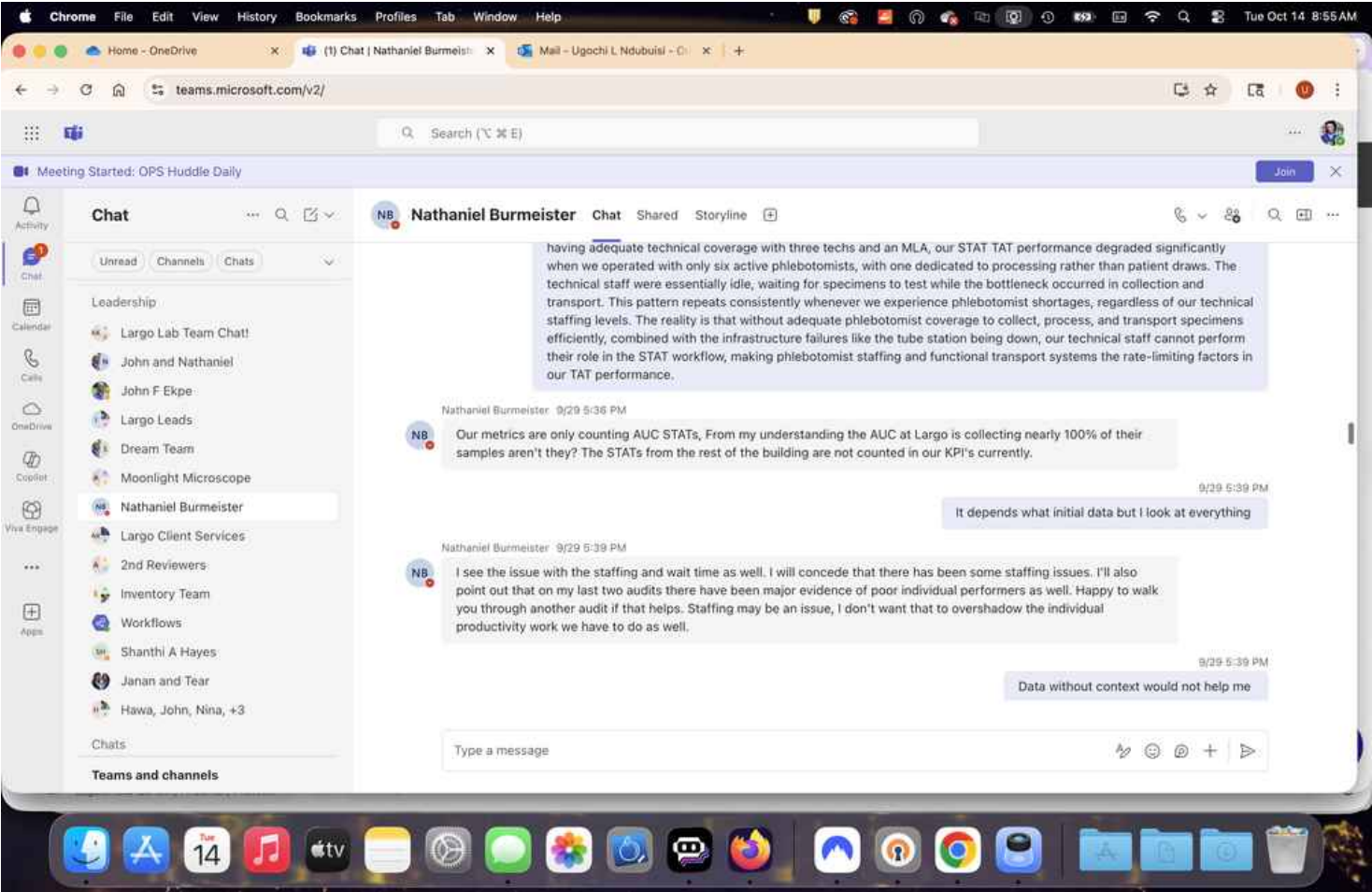


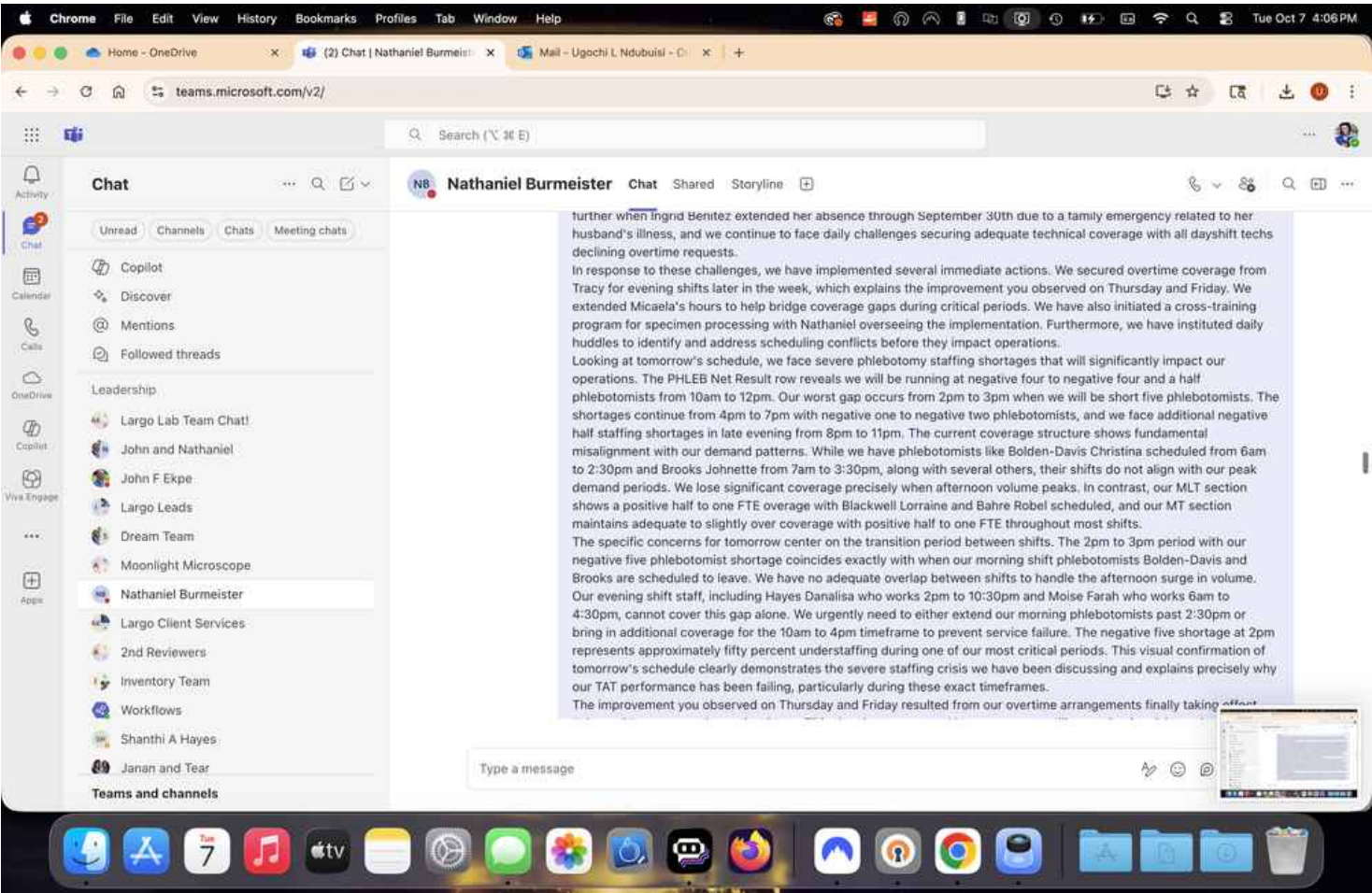


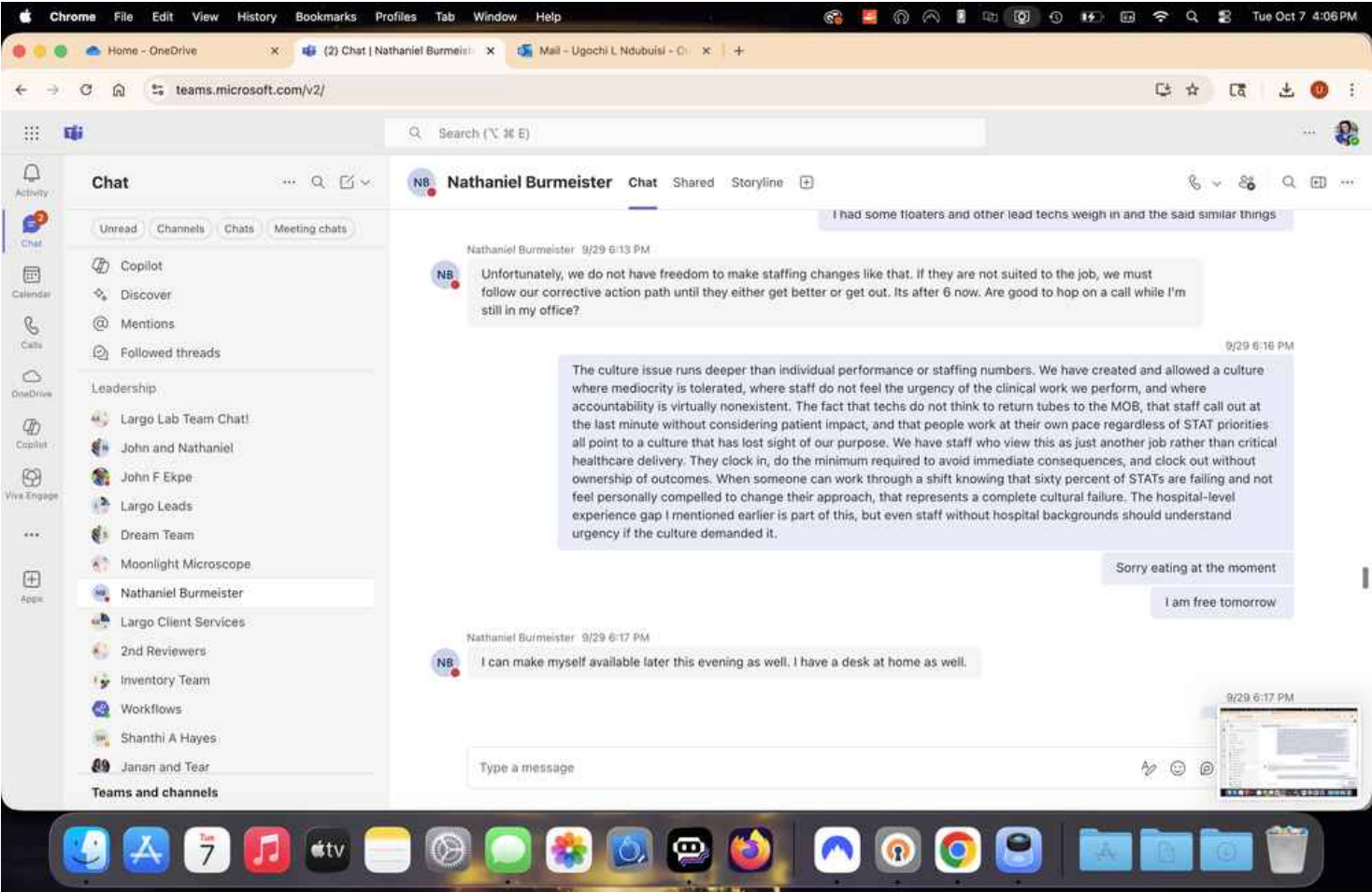




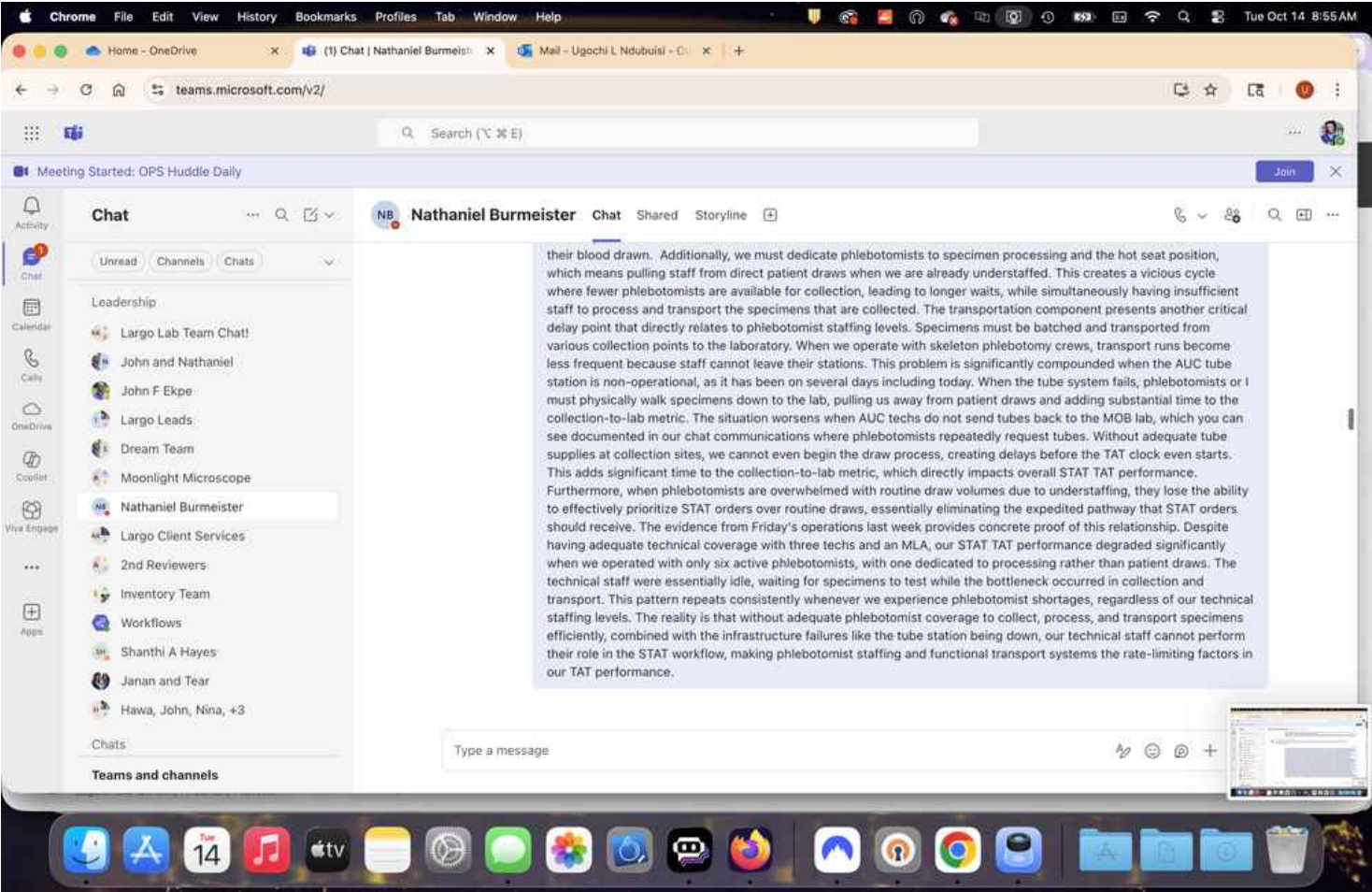




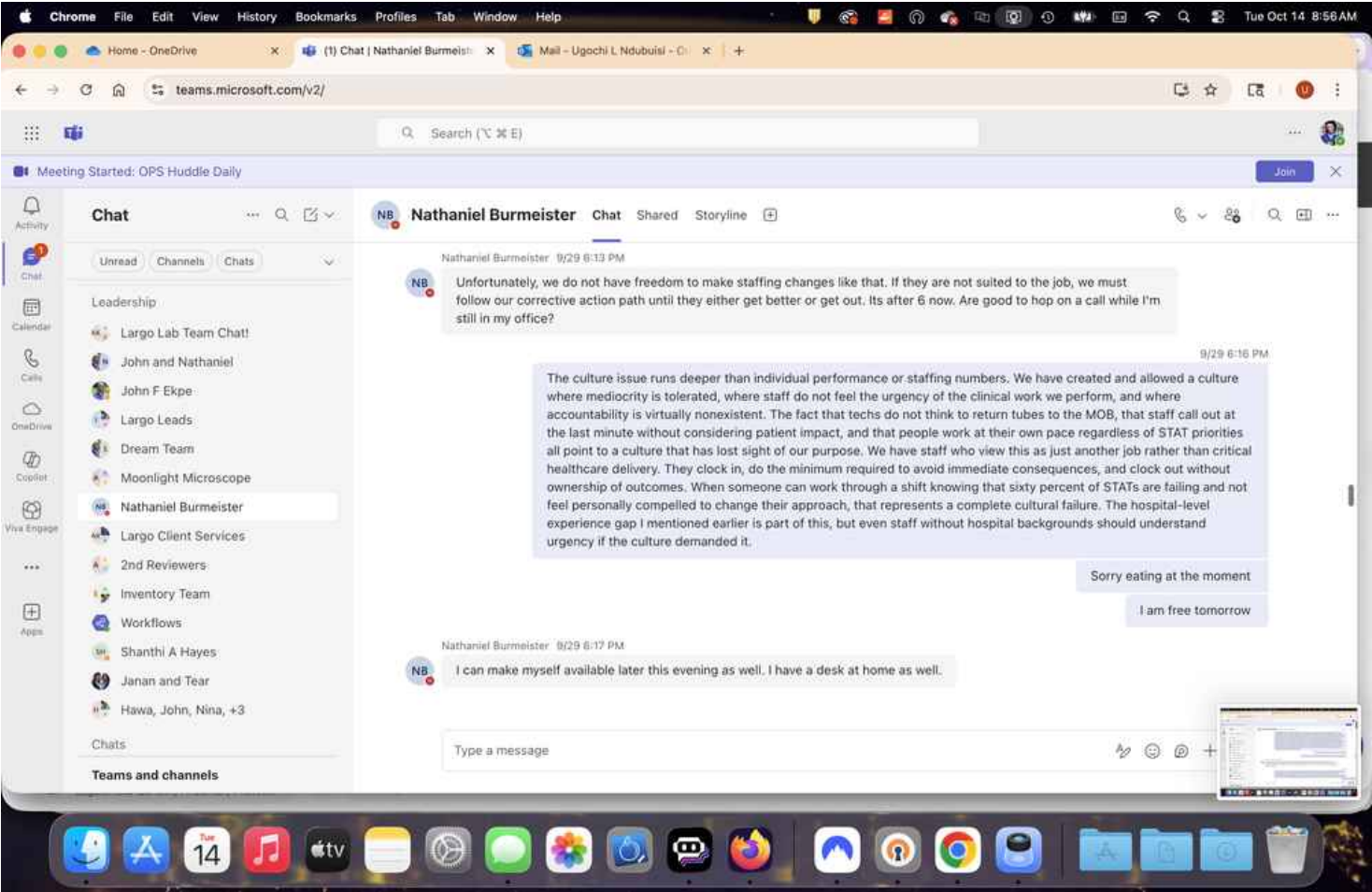


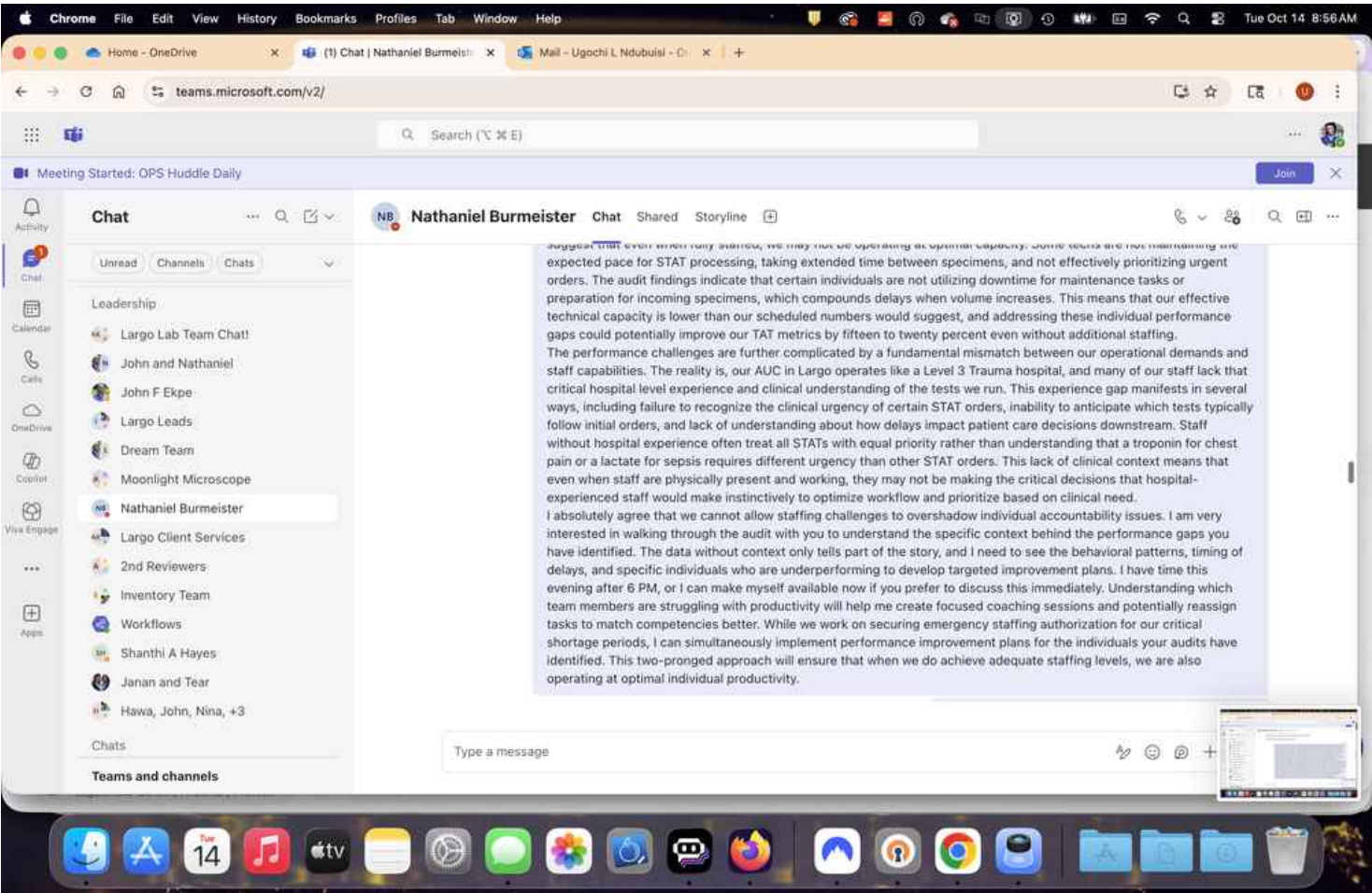


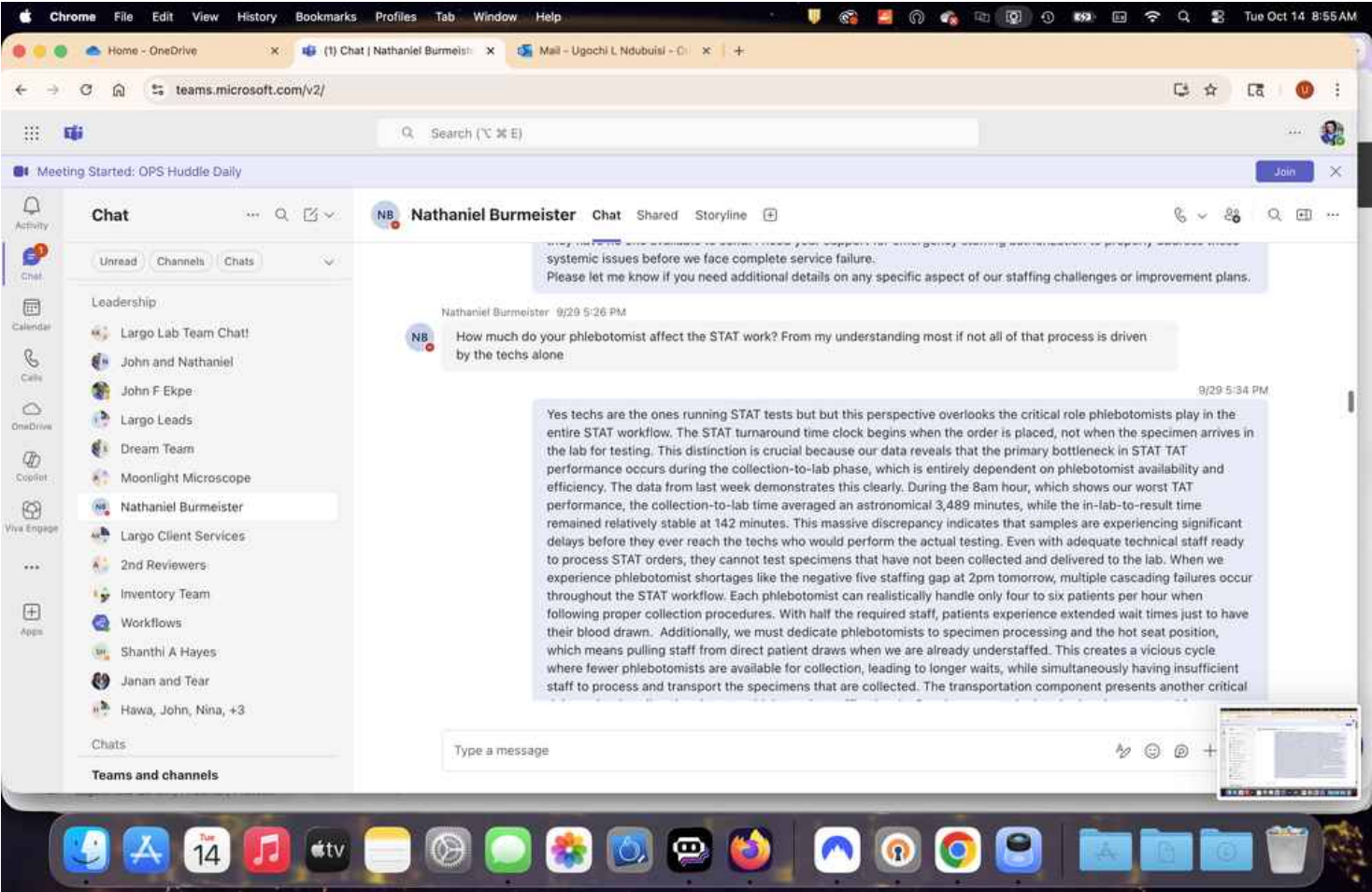
Evidence 13: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.55.37AM



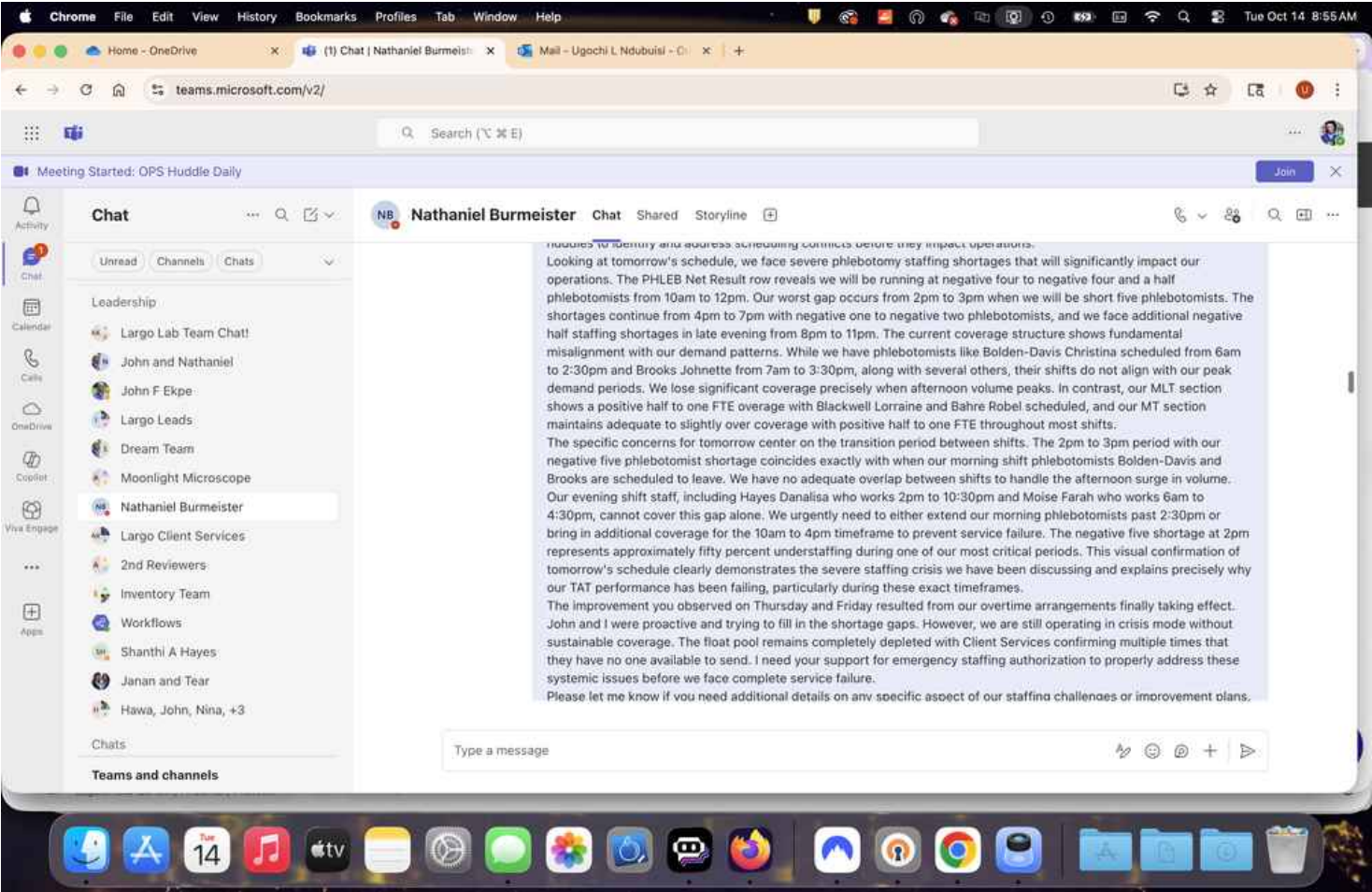
Evidence 14: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.56.10AM

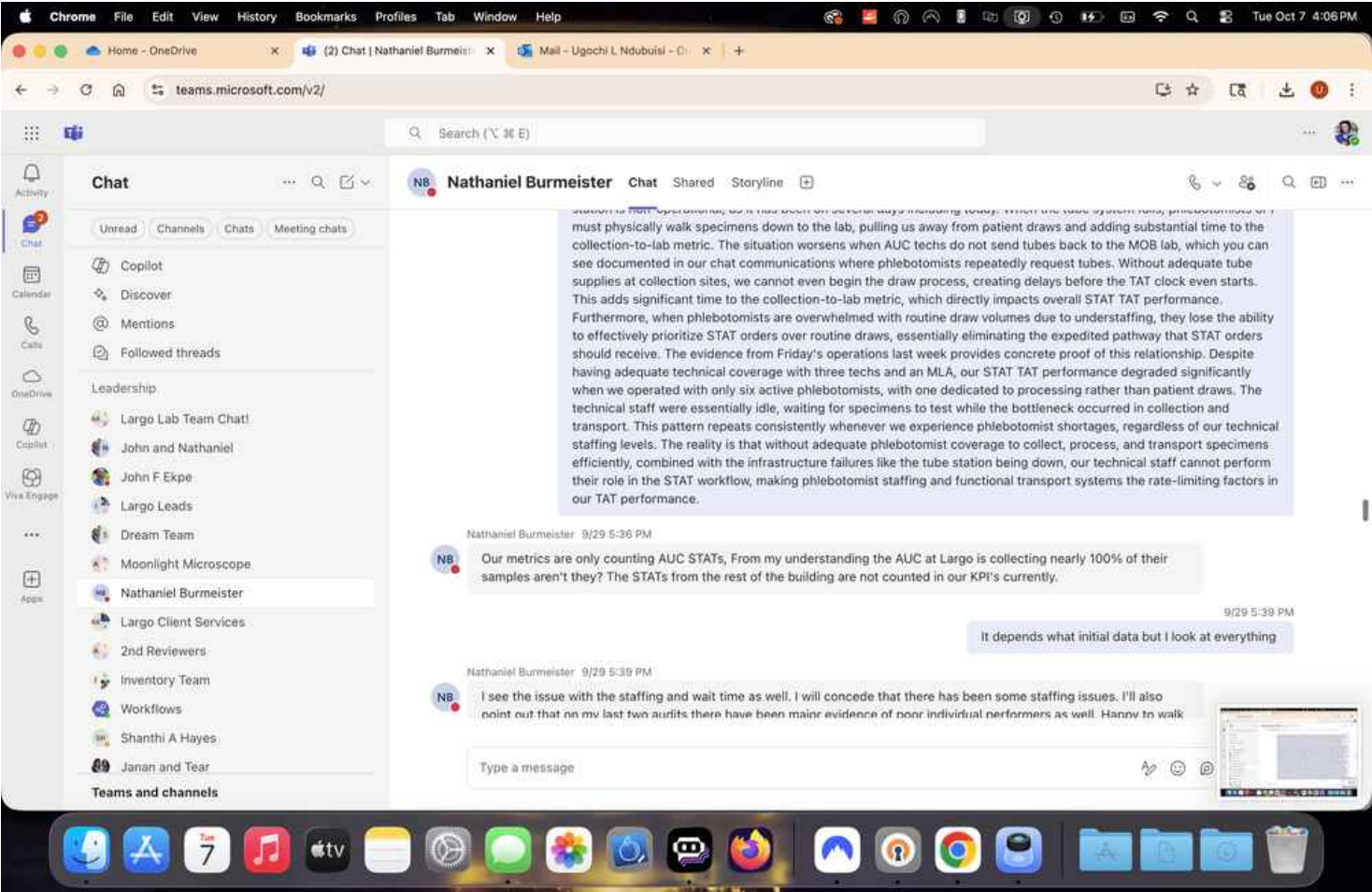


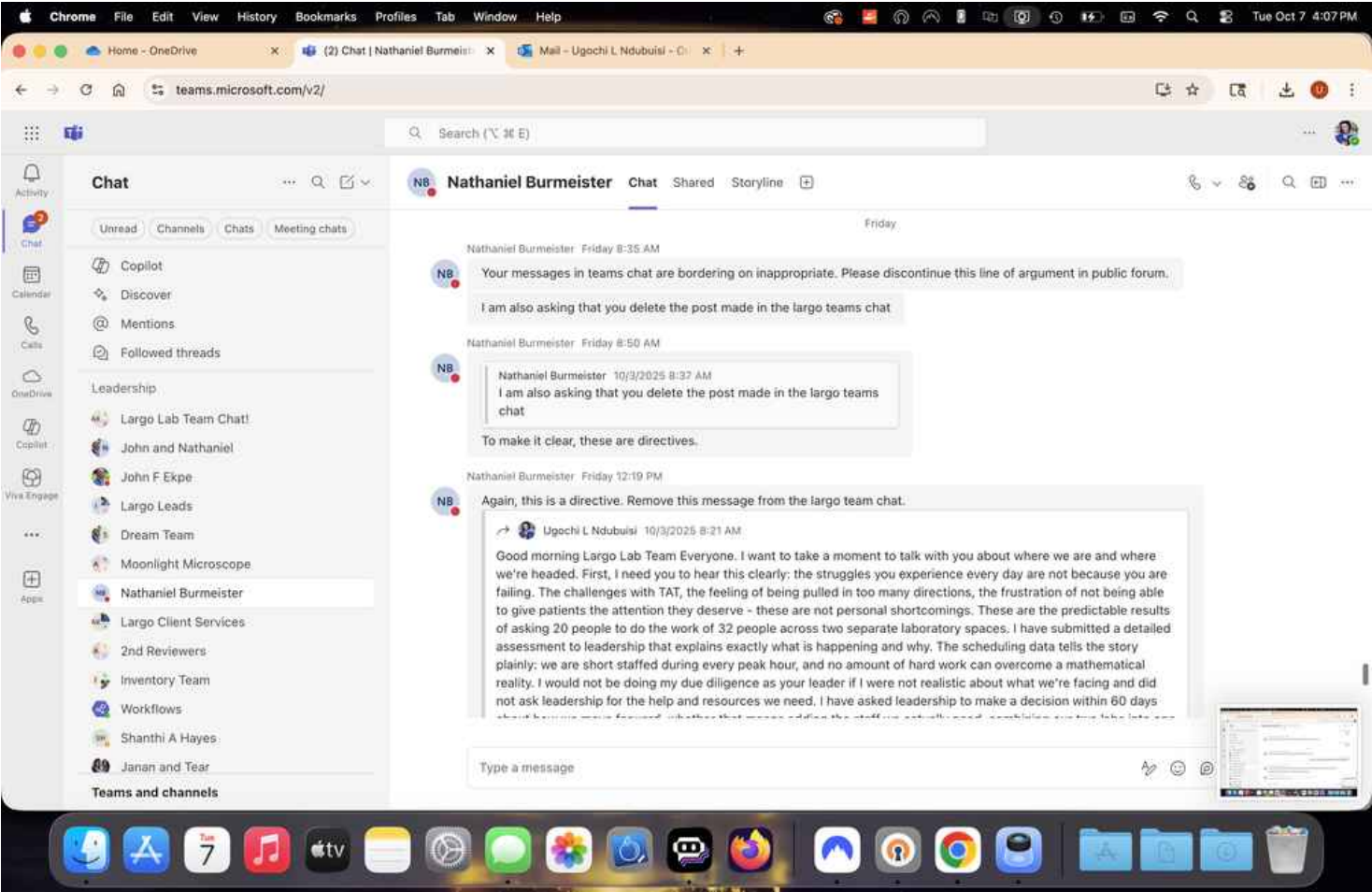


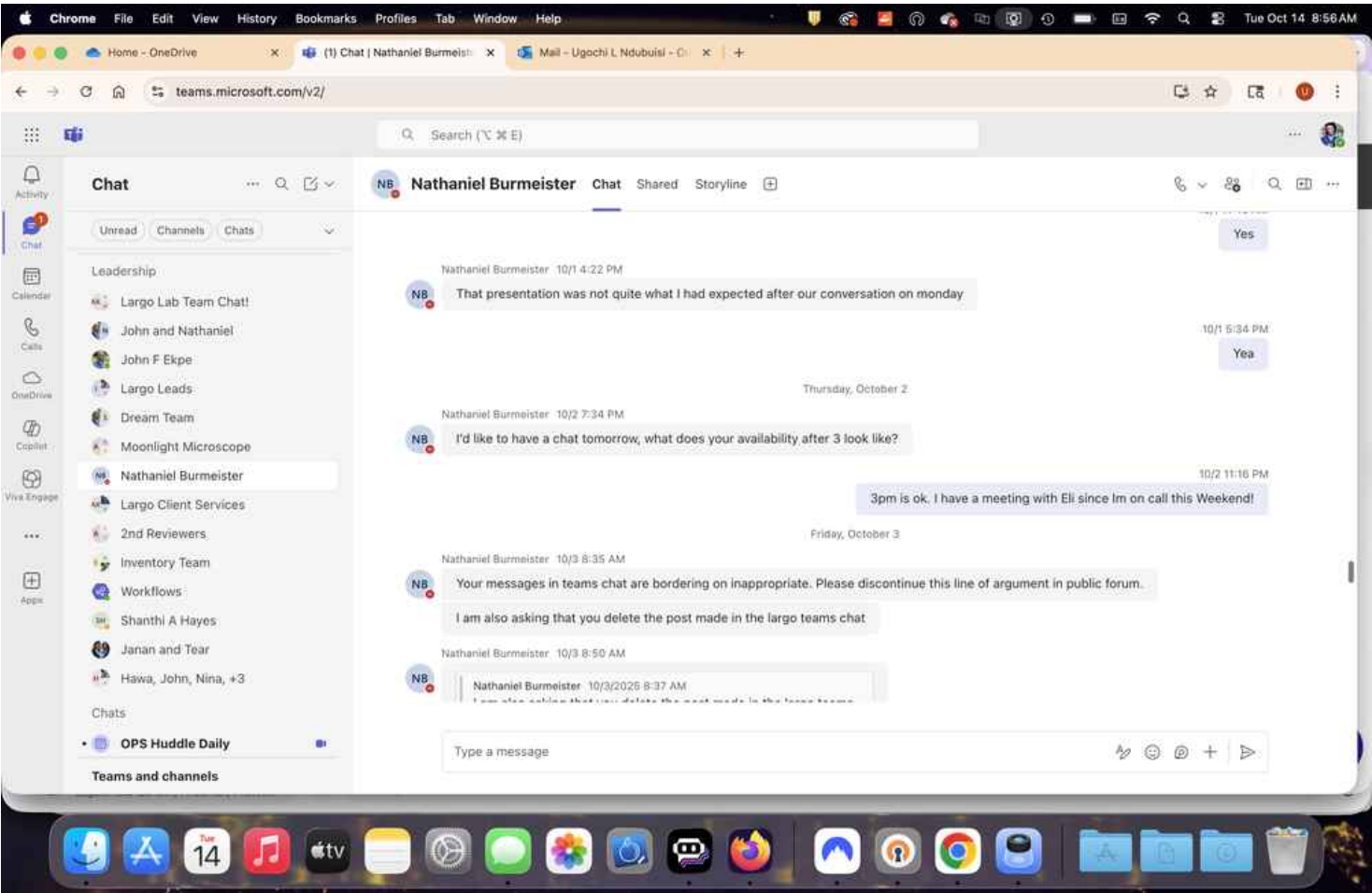


Evidence 17: Oct_3_Whistleblowing_41percent_Screenshot 2025-10-14 at 8.55.19AM

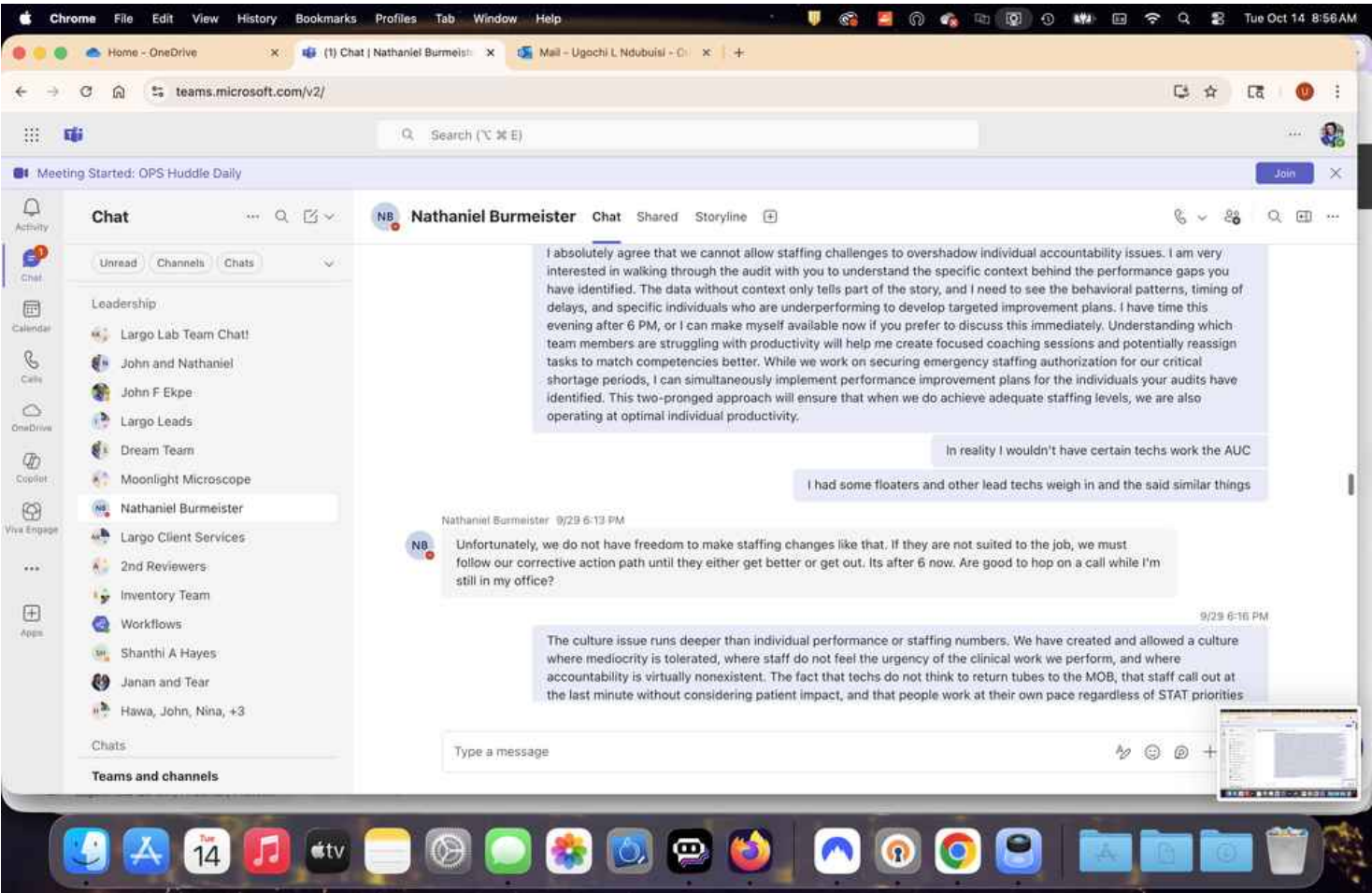


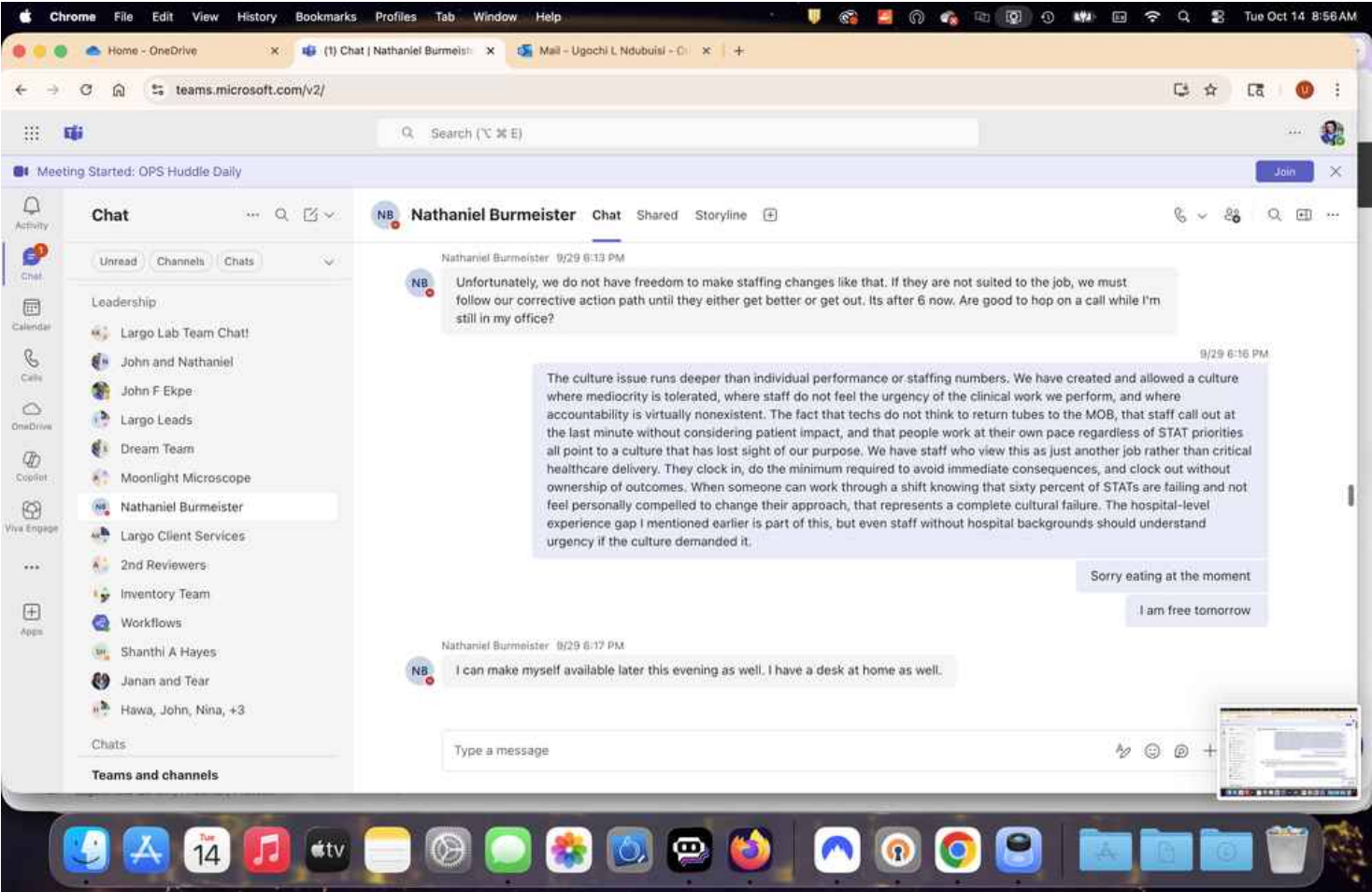


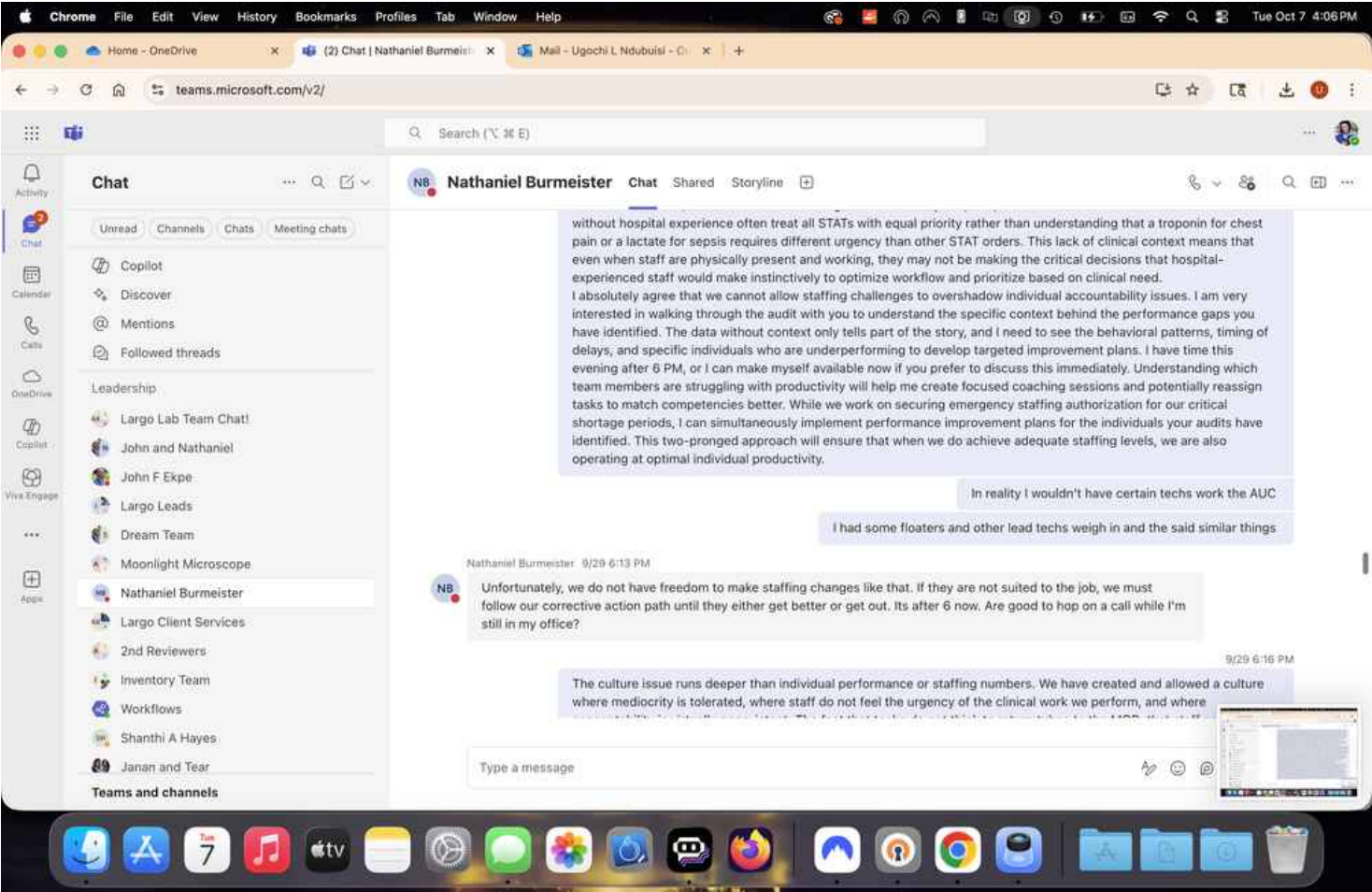


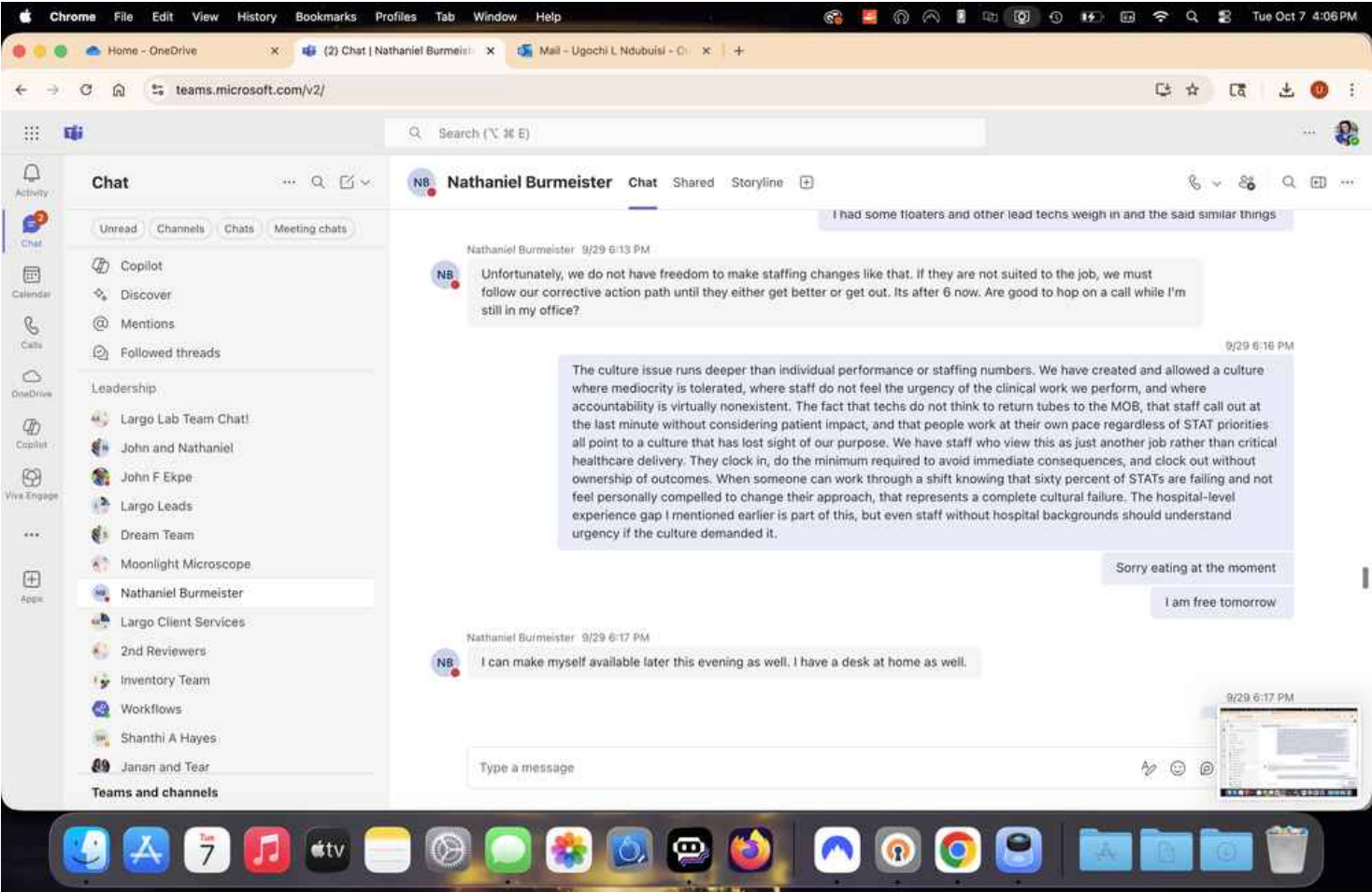


Evidence 21: Oct_8_Final_Warning_Screenshot 2025-10-14 at 8.56.05AM









POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
ACCOUNTABLE DEPARTMENT National Human Resources	EFFECTIVE DATE 12/19/2022
DOCUMENT OWNER Chair, National HR Policy Roundtable	PAGE 1 of 3

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Regional procedures must be consistent with this National policy but may include more detailed information regarding corrective/disciplinary action, and employees are subject to those procedures as well.

2.0 Purpose

Kaiser Permanente (KP) must comply with specific legal/regulatory standards that include, but are not limited to, those indicated by Medicare and other government program billing requirements, Guidance issued by the Centers for Medicare and Medicaid Services, the Department of Health and Human Services, the Compliance Program Guidance issued by the Office of the Inspector General, and HIPAA/Privacy and Security regulations. (See Section 6.0, References) KP is required to have well publicized disciplinary guidelines that demonstrate its commitment to compliance and set forth the consequences for violations of compliance mandates.

3.0 Scope/Coverage

This policy applies to all individuals who are employed by the following entities (collectively referred to as "Kaiser Permanente"):

- 3.1** Kaiser Foundation Health Plan, Inc. (KFHP);
- 3.2** Kaiser Foundation Hospitals (KFH);
- 3.3** KFHP/H subsidiaries;
- 3.4** The Permanente Medical Group, Inc. (TPMG) [NOTE: This policy does not apply to physicians, podiatrists, vice presidents, or members of the TPMG Executive Staff, who are covered by separate TPMG policies]; and
- 3.5** Southern California Permanente Medical Group (SCPMG) [NOTE: This policy does not apply to physicians of SCPMG].

4.0 Definitions: n/a

POLICY TITLE Corrective / Disciplinary Action	POLICY NUMBER NATL.HR.014
ACCOUNTABLE DEPARTMENT National Human Resources	EFFECTIVE DATE 12/19/2022
DOCUMENT OWNER Chair, National HR Policy Roundtable	PAGE 2 of 3

5.0 Provisions

- 5.1** Except as limited by collective bargaining agreements and/or statute, employment with KP is “at will” (see *At-Will Employment*, NATL.HR.37).
- 5.2** In the event that it is necessary to take corrective/disciplinary action to address concerns related to an employee’s work performance or conduct, the corrective/disciplinary action may include, but is not limited to:
 - 5.2.1** verbal discussion, coaching, and/or warning by the employee’s immediate supervisor or higher-level manager to correct the problem
 - 5.2.2** written notice, with or without final warning
 - 5.2.3** suspension, with or without final warning
 - 5.2.4** termination of employment
- 5.3** The corrective/disciplinary action steps listed in section 5.2 are not exhaustive and serve only as a description of possible action. None of these actions are mandatory or a prerequisite for any other actions that may be appropriate based on the circumstances. Any or all of these actions may be employed prior to termination and certain employee conduct may be deemed sufficiently serious to warrant immediate termination or escalated disciplinary action without prior corrective/disciplinary action.
- 5.4** The level of corrective/disciplinary action will be determined by management after review of the facts (including legal, compliance or policy violations), applicable regional/national collective bargaining agreement provisions, and consideration of other relevant factors and circumstances including, but not necessarily limited to, the factors in section 5.5 below. Management should consult Human Resources as appropriate.
- 5.5** Examples of factors to be considered in determining the level of corrective/disciplinary action include, but are not limited to, the following:
 - 5.5.1** the severity of the violation
 - 5.5.2** the number of violations
 - 5.5.3** whether the violation was part of a pattern or practice of improper behavior or non-compliance
 - 5.5.4** the individual’s past performance and conduct
 - 5.5.5** whether the individual knew or should have known the applicable policies, rules or regulations
 - 5.5.6** length of employment at KP
 - 5.5.7** whether the violation was intentional or negligent
 - 5.5.8** whether the action appeared to be committed for personal gain
 - 5.5.9** regional/national collective bargaining agreements corrective/disciplinary action provisions for represented employees



Outlook

RE: Lab delay

From Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>**Date** Fri 6/6/2025 3:41 PM**To** Alice J Gouvernayre <Alice.J.Gouvernayre@kp.org>; Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>; Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>**Cc** Sunny L Lambert <Sunny.L.Lambert@kp.org>; Carole R Kelly <Carole.R.Kelly@kp.org>; Nona Alexander <Nona.Alexander@kp.org>; Erin M Smith <Erin.M.Smith@kp.org>; Zachary Smith <Zachary.X3.Smith@kp.org>; James A Powell <James.A.Powell@kp.org>; Daniel J Patenaude <Daniel.J.Patenaude@kp.org>

Hello Dr. Gouvernayre,

Ugochi and I did a thorough investigation to discover root causes for the failures that were experienced on Monday. We talked about best strategy to mitigate that root cause, and I am confident that it is being addressed. As it is something that will include employee disciplinary actions, I will not go into details in this forum.

As far as regular state, we do normally perform maintenance on our instruments during our lower volume periods. Our major instrumentation all has redundancy. So even during regular maintenance, we have availability to test 24/7 (barring unscheduled downtime). For the Chemistry instruments, we perform one instrument between 9p-12a and the other is performed between 4a-8a. The process usually takes about 1-2 hours, and we have the flexibility to delay up to a few hours if testing demands are high. Ugochi and John are monitoring and reviewing for possible optimizations on this and other areas of the lab.

Ugochi and John will continue to reinforce the use of electronic communication through KP Healthconnect chat as best practice. I'll also continue to connect them with coaching and resources to best serve the operations needs of the Lab and by extension the AUC.

-Nate Burmeister

C:202-570-9030

From: Alice J Gouvernayre <Alice.J.Gouvernayre@kp.org>**Sent:** Friday, June 6, 2025 3:10 PM**To:** Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>; Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>**Cc:** Sunny L Lambert <Sunny.L.Lambert@kp.org>; Carole R Kelly <Carole.R.Kelly@kp.org>; Nona Alexander <Nona.Alexander@kp.org>; Erin M Smith <Erin.M.Smith@kp.org>; Zachary Smith <Zachary.X3.Smith@kp.org>; Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>; James A Powell <James.A.Powell@kp.org>; Daniel J Patenaude <Daniel.J.Patenaude@kp.org>**Subject:** RE: Lab delay

Hi Ugochi,

Thank you for taking the time to explain the situation so thoroughly to us. Can you tell us when the QC for Pure 2 is scheduled to occur?

I have a few questions/suggestions based on the information you provided.

1. Communication:

- a. I understand that you came over that night to inform the team of the issues you were having. It's really nice of you to do that, but not everyone gets the message when you come in person. I think the best way to make sure we all understand what's going on, is for you or another manager to send a detailed message about the issue and its impact to the lead nurse on epic chat. The lead nurse will then copy that message and send it to all that are working in the AUC that day. This workflow is good for temporary delays. If an instrument is down or for any other major issue, please continue to notify the DCSM critical operation alert on epic chat.

Reason: If physicians and nurses understand the lab limitations, we are then able to adjust the plan of care for patients appropriately (i.e. send patients home and message them on kp.org when the results are available versus the traditional path of having the patients wait in the department for results). Or we can set expectations with patients right off the bat on how long it will take to figure out what's wrong with them. The sooner we know, the better.

- b. Regarding the high volume of calls – I understand how disturbing this might have been. I think if we follow the communication pathway outlined above, you will get less calls. I also do want to point out that we wouldn't be doing our job if we left a patient waiting for a strep test result for hours without inquiring about the result repeatedly on their behalf.

1. Mitigation of impacts:

- a. I understand the chemistry machine was down and the urinalysis instrument was also not working but why did we have a 10-hour strep test delay? Or why were CBCs affected?
- b. What is the escalation pathway when you are short-staffed? What is the escalation pathway when the machines are malfunctioning? What is put in place to preserve operations across all testing instruments?

We look forward to working on solutions with you to improve operations and that we can present at the throughput meeting.

Best,

Alice J Gouvernayre, MD

Chief, Acute Care Services

DC and Suburban Maryland Service Area

Permanente Medicine

Mid-Atlantic Permanente Medical Group

Capitol Hill Medical Center

700 2nd St. NE,

Washington, DC 20002

202-853-1311 (mobile)

alice.j.gouvernayre@kp.org

For assistance:

Rodnee Petty at 202-680-3404 or rodnee.s.petty@kp.org

[Facebook](#) | [Twitter](#) | [LinkedIn](#) | [Instagram](#) | [YouTube](#)

Kaiser Permanente

From: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Sent: Tuesday, June 3, 2025 10:45 PM

To: Latoya Shallow <Latoya.Shallow@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>



RE: Activity and Camera Footage Report Request

From Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>

Date Mon 7/14/2025 7:11 PM

To Janis A Thom-Grate <Janis.A.Thom-Grate@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>

Cc Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Thank you Janis,

I'll walk them through this portal at a later date, I'll be submitting on their behalf tonight.

-Nate Burmeister

C:202-570-9030

From: Janis A Thom-Grate <Janis.A.Thom-Grate@kp.org>

Sent: Monday, July 14, 2025 7:10 PM

To: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>; John F Ekpe <John.F.Ekpe@kp.org>

Cc: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Subject: RE: Activity and Camera Footage Report Request

Here is the link to submit the security request –

[Online Security Incident Report](#)

-Janis

Janis A. Thom-Grate, J.D., SPHR, SHRM-SCP

Pronouns: She, Her, Hers

HR Consultant IV

Kaiser Permanente Mid-Atlantic States

(c) 240.495.1364

Janis.A.Thom-Grate@kp.org

Upcoming PTO:

HR RESOURCES FOR ALL EMPLOYEES	HR RESOURCES FOR MANAGERS
- HR transactions including pay, benefits, leaves and HR system navigation inquiries: Contact the HR Service Center or Timekeeping or at 1-877-457-4772. Fax: 1-877-477-2329 - Retirement (pension plan, retiree medical and/or retiree life insurance benefits): Visit the KPRC website at www.myretirement.kp.org or call the KPRC at 1-866-627-2826, #0. - Defined contribution plans-related inquiries (Plan B, TSA, and Roth): Visit the Vanguard website at www.vanguard.com or call Vanguard at 1-800-523-1188.	- For information you cannot find in the HRconnect system or at hrconnect.kp.org: <i>Position management:</i> Email pm-support-team@kp.org or <i>Call the National HRSC at 1-877-457-4772, Options 3,4,5.</i> <i>Other transactions for individuals on your team (such as timekeeping, new hires, terminations, and general HR systems questions): Call the National HRSC at 1-877-457-4772, Options 3,4.</i>

From: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Sent: Monday, July 14, 2025 7:04 PM
To: John F Ekpe <John.F.Ekpe@kp.org>; Janis A Thom-Grate <Janis.A.Thom-Grate@kp.org>
Cc: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Subject: RE: Activity and Camera Footage Report Request

Hello John and Ugochi,

Whenever we wish to pull camera footage, we must also include our HRC partner in that request. [@Janis A Thom-Grate](mailto:Janis.A.Thom-Grate@kp.org), We will be requesting for camera footage to support an investigation into possible time fraud. We have reason to believe that employees are leaving their workstation for extended periods of time while clocked in. John and Ugochi have also requested badge swipes for the days below and for a few other days (7/2, 7/5, and 7/6) all between 7pm and 10pm. We will also be pulling Cerner activity data for the days listed.

-Nate Burmeister
C:202-570-9030

From: John F Ekpe <John.F.Ekpe@kp.org>
Sent: Monday, July 7, 2025 7:21 PM
To: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Cc: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Subject: Activity and Camera Footage Report Request

Hi Nate,

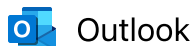
Thank you for taking the time to chat with me earlier about our AUC TAT metric, staff performance and related matters. Based on our discussions, I would like to request activity report pull on the following days and the camera report can be pulled between 7pm - 10pm.

Date: July 2nd, 5th and 6th.

Thank you.

John F. Ekpe, MLS (ASCP)cm
Manager, Laboratory Operations
Largo Medical Center
1221 Mercantile Ln, Upper Marlboro, MD 20774
Email: john.f.ekpe@kp.org
Phone: (301)-996-0119

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RE: Meeting Request - Tomorrow at 4pm or Alternative Time

From Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>

Date Tue 6/10/2025 9:09 PM

To Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

I'll be on site on Friday and can assist

-Nate Burmeister

C:202-570-9030

From: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>

Sent: Tuesday, June 10, 2025 3:49 PM

To: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>

Subject: Re: Meeting Request - Tomorrow at 4pm or Alternative Time

Hi Nate,

I'd like to discuss two concerns regarding Tracy's recent performance that require your input. I would like for you to be there for this CA due to complexity but I can lead:

1. **June 2nd Incident:** We experienced a 6-hour turnaround time delay when Tracy claimed to be performing Pure Analyzer Maintenance and QC on Pure 2 instead of the correct Pure 1 analyzer in the MOB. The maintenance was done incorrectly and caused numerous issues, to the point where Night shift was experiencing problems due to the errors made. This significantly impacted patient care and our standard TAT for the AUC. I'd like to meet with her and her shop steward as well as with you to review the events of that day as a fact-finding opportunity to determine if a Corrective Action is warranted.
2. **Ongoing Tardiness Pattern:** Tracy has demonstrated a consistent pattern of arriving late for her shifts:
 - o April 21: Arrived at 3:43 PM
 - o April 27: Arrived at 3:37 PM
 - o April 29: Arrived at 3:44 PM
 - o April 30: Arrived at 3:38 PM
 - o May 25: Arrived at 3:35 PM
 - o May 28: Arrived at 3:37 PM

Additionally, she has been tardy on March 31, April 8, April 10, April 15, April 17, and most recently on June 5.

I have prepared the relevant documentation for our discussion, including:

- The Attendance Policy
- Pure Maintenance SOP with implementation dates

Based on our attendance policy, I believe a Level 1 Corrective Action is appropriate for the tardiness issues. However, I'd like your guidance on how to proceed with the June 2nd incident.

From: Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Sent: Monday, June 9, 2025 10:35 PM
To: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Subject: RE: Meeting Request - Tomorrow at 4pm or Alternative Time

Can you share with me your plan for this meeting. Let me know what support you may need.

From: Ugochi L Ndubuisi <Ugochi.L.Ndubuisi@kp.org>
Sent: Monday, June 9, 2025 10:36 AM
To: Oghenechuko T Eshofa <Oghenechuko.T.Eshofa@kp.org>; Jamie T Anderson <Jamie.T.Anderson@kp.org>
Cc: John F Ekpe <John.F.Ekpe@kp.org>; Nathaniel Burmeister <Nathaniel.X.Burmeister@kp.org>
Subject: Meeting Request - Tomorrow at 4pm or Alternative Time

Good morning!

I would like to schedule a 1-hour meeting with both of you to discuss some important matters. I'm available tomorrow at 4pm.

If tomorrow at 4pm doesn't work, please note that I have an all-day meeting on Wednesday, but I'm flexible for any other time that fits your schedules.

Please let me know what works best for you both.

Very Respectfully,

Ugochi Ndubuisi, DHSc, MT(AMT)^{CM}, MB(ASCP)^{CM}
Laboratory Operations Manager

Kaiser Permanente Mid-Atlantic Region
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1221 Mercantile Ln, Upper Marlboro, MD 20774
Email: Ugochi.L.Ndubuisi@kp.org

Phone: 301-848-6794



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